

Centre Policy Handbook



CONTENTS

Categorical Index:

	Policy No.
Centre Culture Policies	
Family and Visitor Code of Conduct Policy.....	8
Behaviour Guidance Policy	9
Bullying Policy.....	68
Enrolment Policy	1
Gender Equity Policy	3
Environmental Sustainability Policy	90
Ethical Conduct and Code of Ethics Policy.....	70
Inclusion Policy	4
Open Door Policy.....	76
Complaints and Grievance Policy	7
Parent Involvement Policy	6
Communication Plan & Policy	5
Priority of Access Policy	2
Centre Safety & Security Policies	
Acceptance and Refusal of Authorisations	86
Delivery and Collection of Children Policy.....	65
Child Safe Standards Policy	12A
Child Safety, Protection & Wellbeing Policy	12B
Safeguarding Children: Look, Do & Tell Framework Policy	12C
Family Violence Safety Policy	99
Emergency Management and Evacuation Plan Policy and Procedure	20
Hazard Risk Management Policy Procedure and Program.....	81
Highchair Policy	26
Incident, Injury, Trauma and Illness Policy	19
Occupational Health, Safety & Wellbeing Policy	10
Family Law and Access Policy.....	71
Late Children Policy	18
Lost Child Policy.....	73
National Model Code Policy.....	100
Opening and Closing Policy	17
Privacy and Confidentiality Policy	16
Safety & Maintenance Policy	14
Security Policy	15
Staff Injury Management Policy and Procedure.....	80
Lockdown Policy.....	75
Severe Weather Policy	21
Supervision Policy	11
Interaction with Children Policy.....	72
Toxic & Potentially Dangerous Products Policy	23
Toxic Plant Policy	24
Valuables Policy	25
Water Safety Policy	13

General Policies

Centre Vehicle & Transportation Policy	63
Company Vehicle Policy	64
Exclusion Policy	43
Fee Policy	66
Fundraising Policy	61
Internet, Email and Social Network Policy	62
Policy and Procedure Review Policy.....	83
Technology & Digital Usage Policy	96
Recording Keeping and Retention Policy	84
Business Debit MasterCard Policy.....	77

Health Policies

Anaphylaxis Policy	35
Health and Wellbeing Policy.....	92
Asthma Policy	36
Diabetes Policy	85
Child Safe Environment Policy	88
Biting Policy	37
Clothing Policy	38
Coronavirus Policy.....	98
Cystic Fibrosis Policy.....	94
Dental and Oral Health Policy	30
Epilepsy Policy.....	91
First Aid Policy	34
Nutrition, Food, Beverage & Dietary Requirements Policy	28
Food Safety & Hygiene	29
Health & Hygiene Policy	27
Infectious Diseases and Immunisation Policy	32
Medical Conditions Policy	87
Medication Policy	33
Nappy Change and Toilet Support Policy	101
Physical Activity Promotion Policy.....	93
Rest and Sleep Policy	40
Illness Policy	31
Soiled Clothing Policy	39
Smoke, Vape, Alcohol & Drug Free Environment Policy	41
Sun Protection and Safety Policy	42

Curriculum & Program Policies

After School Care Collection Policy	50
Excursion and Incursion Policy	48
Family Grouping Policy	46
Indigenous and Multicultural Policy	47
Education, Curriculum and Learning Policy	89
Curriculum and Planning Policy	44
Child Self Reliance and Self Esteem In Children Policy.....	69
National Quality Framework	82
Transitions & Routines Policy	45

Transition to School and Other Organisations Policy	49
Vacation Care Policy	51
 Management and Team Member Policies	
Babysitting Policy	59
Employment Policy	52
Governance of Service, Management and Educator Policy	78
Media/Press Policy	60
Team Member Grievances	54
Staff to Staff Interactions.....	74
Staff Arrangement Policy	79
Team Member Maternity Policy	53
Team Member Self Protective Behaviour Policy	56
Team Member Training Policy	58
Student and Volunteer Policy	55
Back Care and Manual Handling Policy.....	67
OWNA App Rolls Management System	97
Protected Disclosures (Whistleblower) Policy	102

CONTENTS

<u>Alphabetical Index:</u>	Policy No.
Acceptance and Refusal of Authorisations	86
After School Care Collection Policy	50
Anaphylaxis Policy	35
Asthma Policy	36
Babysitting Policy	59
Back Care and Manual Handling Policy.....	67
Behaviour Guidance Policy	9
Biting Policy	37
Bullying Policy.....	68
Business Debit MasterCard Policy.....	77
Centre Vehicle & Transportation Policy	63
Child Safe Standards Policy	12A
Child Safety, Protection & Wellbeing Policy	12B
Child Safe Environment Policy.....	88
Child Self Reliance and Self Esteem In Children Policy.....	69
Clothing Policy	38
Communication Plan & Policy	5
Company Vehicle Policy	64
Complaints and Grievance Policy	7
Coronavirus Policy.....	98
Curriculum and Planning Policy	44
Cystic Fibrosis Policy	94
Delivery and Collection Policy of Children.....	65
Dental and Oral Health Policy	30
Diabetes Policy	85
Education, Curriculum and Learning Policy	89
Emergency Management and Evacuation Plan Policy and Procedure	20
Employment Policy	52
Enrolment Policy	1
Environmental Sustainability Policy.....	90
Epilepsy Policy.....	91
Ethical Conduct and Code of Ethics Policy.....	70

Exclusion Policy	43
Excursion and Incursion Policy	48
Expense and Travel Policy	57
Family and Visitor Code of Conduct Policy	8
Family Grouping Policy	46
Family Law and Access Policy.....	71
Family Violence Safety Policy	99
Fee Policy	66
First Aid Policy	34
Food Safety & Hygiene Policy	29
Fundraising Policy	61
Gender Equity Policy	3
Governance of Service, Management and Educator Policy	78
Hazard Risk Management Policy Procedure and Program	81
Health & Hygiene Policy	27
Health and Wellbeing Policy.....	92
Highchair Policy	26
Illness Policy	31
Incident, Injury, Trauma and Illness Policy	19
Inclusion Policy	4
Indigenous and Multicultural Policy	47
Infectious Diseases and Immunisation Policy	32
Interaction with Children Policy.....	72
Internet, Email and Social Networking Policy	62
Late Children Policy	18
Lockdown Policy.....	75
Lost Child Policy.....	73
Media/Press Policy	60
Medical Conditions Policy.....	87
Medication Policy	33
Nappy Change and Toilet Support Policy	101
National Model Code Policy.....	100
National Quality Framework Policy	82
Nutrition, Food, Beverage & Dietary Requirements Policy	28

Occupational, Health, Safety & Wellbeing Policy	10
Open Door Policy.....	76
Opening and Closing Policy	17
OWNA App Roll Management Policy	97
Parent Involvement Policy	6
Physical Activity Promotion Policy	93
Policy and Procedure Review Policy.....	83
Priority of Access Policy	2
Privacy and Confidentiality Policy	16
Protected Disclosures (Whistleblower) Policy	102
Record Keeping and Retention Policy	84
Rest and Sleep Policy	40
Safeguarding Children: Look, Do & Tell Framework Policy	12C
Safety & Maintenance Policy	14
Security Policy	15
Severe Weather Policy	21
Smoke, Vape, Alcohol & Drug Free Environment Policy	41
Soiled Clothing Policy	39
Staff to Staff Interactions.....	74
Staff Injury Management Policy and Procedure.....	80
Staffing Arrangements Policy.....	79
Student and Volunteer Policy	55
Sun Protection and Safety Policy	42
Supervision Policy	11
Team Member Grievances.....	54
Team Member Maternity Policy	53
Team Member Self Protective Behaviour Policy.....	56
Team Member Training Policy.....	58
Technology & Digital Usage Policy	96
Toxic & Potentially Dangerous Products Policy	23
Toxic Plant Policy	24
Transitions & Routines Policy	45
Transition to School and Other Organisations Policy	49
Vacation Care Policy	51

Valuables Policy	25
Water Safety Policy	13

1 - ENROLMENT POLICY

Position Statement

It is important for parents to feel welcome and relaxed at the centre. Parents should be able to meet team members and to understand the centre's policies and procedures. Enrolment forms need to be completed before a child can commence at the centre.

Objective

To conduct a comprehensive orientation for new families and children.

Implementation

Once a vacancy has been filled from the waiting list and parents have accepted the days; a convenient appointment time will be made so that parents can complete the orientation process.

Parents and guardians agree to comply with the Terms and Conditions of Enrolment, Service Policies and Procedures, and the Complying Written Agreements. These documents may be updated from time to time, and continued enrolment constitutes acceptance of any changes.

If a parent or guardian does not agree to or comply with these terms, they may choose to withdraw their child from the service in accordance with the notice period. The service also reserves the right to terminate the enrolment and care arrangement if the terms are not accepted or followed.

An orientation checklist will be completed for each new family that enrolls and for the centre as per the **Orientation Checklist Family** and the enrolment pack.

In brief the procedure will be as follows:

1. Parent completes the electronic enrolment form. Please follow instruction provided in the enrolment email. All questions and fields are required to be completed on the enrolment form. Being an online enrolment form if a field is left blank; it is intentional left blank by the parent. All required fields are required to be completed before submitting enrolment form.
2. Completion of CWA (via OWNA Parent App) before commencement at centre.
3. Completion of Direct Debit Request/Authorisation form via the OWNA Parent App.
4. Parents to be emailed Welcome Message (via Mail Chimp).
5. Upon offer of a place the family must pay the enrolment booking fee and fees in accordance with fee policy and fee schedule of the centre. All families need to apply to the Centrelink for claiming of Child Care Subsidy (CCS) entitlements. If the family have not applied for CCS, they will be billed at full fees until such time as the advice from Centrelink is received at the centre.
6. Families are required to pay a Enrolment Fee to secure enrolment. The enrolment booking fee is calculated as greater of \$100 or 2 weeks gap fee (use OWNA CCS calculator to work out balance due.) If child does not commence as planned at the service, the Enrolment Fee Paid is non-refundable. When this occurs a "Enrolment Fee Paid" transaction" will be added to the statement equals the same dollar value as the Enrolment Fee charged.
7. Families can view their Account Statement via the OWNA app to see charges and payments made.
8. Issue the family with a Family Handbook, Enrolment Email and a Welcome Email and explain the items as per the Orientation Procedure outlined above.
9. Ensure the family have been introduced to the contact team members who will be caring for their child.
10. Explain to families their options in regard to the acceptable methods of fee payment. Debit Success is our only payment method.
11. Show the family where their accounts and centre communication displayed in the OWNA App. We issue invoices and account transaction reports via the OWNA Parent App.
12. Confirm a start date for the child and organise an orientation sessions for the child (eg. The child may only come for 0.5 – 3 hours). This can make the settling in process easier for the child.

13. Explain the requirements for the first day – go through that page in the family handbook and family orientation check with the parent. The Family Orientation Checklist provides tasks and topics needing to discuss with parent and guardian of the child. The completed Family Orientation Checklist PDF needs to be uploaded to OWNA Child Documents as an enrolment document type. Title = Name the Family Orientation Checklist – Child Name: Date completed DD/MM/YY.

Manage Child's Documents Annabelle Amos

Home / Children / Manage Child's Documents Quick Links to: [Back to Profile](#) [Child's Documentation](#) [All Attendances](#)

Title

Document Type

Notes/Comments

Private? (i.e. not available on App)

Original document has been sighted by staff

Expiry Date (if any)

Image or PDF

URL

Add Document below

-select-

2 Months Vaccination

4 Months Vaccination

6 Months Vaccination

12 Months Vaccination

18 Months Vaccination

4 Years Vaccination

Consent Form

Emergency Action Plan

Enrolment

General

Medication

Dietary & Risk Minimisation

Legals

Medical Action Plans

Health Care Card

Pensioner Card

Veteran Healthcare Card

Refugee Visa

Filter: Current Documents - sort by: - Get Documents

#	Title	Doc Type	Sighted By Staff
1	4 Years Vaccination Record	Doc Type: 4 Years Vaccination	Link Archive
2	medical.cert	Doc Type: General	Link Archive

14. Menu – explain how the menu works (where applicable).
15. Show the family where the Parent Kiosks are located to sign in and sign out and explain how to complete them and why it is a requirement. Please refer to instructions on how parent use the OWNA Parent Kiosk to sign in and out their children to record attendance in the enrolment email.
16. Explain that the parent, a legal guardian, or authorised adult (and explain the meaning of authorised) must pick up and drop off the child every day. Show the parent where the Regulations Booklet, Child Care Act Booklet, Quality Improvement Plan, sign-in and sign-out Kiosk, Noticeboard and Centre Policy Handbook are located in the foyer.
17. Explain safety and security expectations – for example that parents enter and leave the centre carefully so that children do not follow them and that if there are door codes, that they keep the code confidential.
18. Explain the Open-Door policy. The use of phones and cameras by family members and visiting adults in the service (see service policy). If the parents have any queries, questions or concerns we would prefer that they direct them to the Centre Director or management. Also, show the parent the grievance procedure in the Parent Handbook and where the phone number is located for the State Licensing body and the support centre.
19. Explain Parent Participation and the Parent Consultative Group. Periodically parents are invited to participate in the program. If parents feel inclined to have input, we would strongly encourage it. Involvement in the parent committee is not onerous, and it allows them a forum to discuss issues at the centre in a child free forum.

Prior to commencement at the centre the Director must hold in their possession the following:

- Enrolment form; All fields and forms **MUST** be fully completed (**NO BLANK FIELDS ON ENROLMENT FORM ARE ALLOWED**). The enrolment form must include all child and parent information, emergency contact numbers, authorised person to collect the child and medical and doctors details. Further, consent to seek and carry out urgent medical attention if required is necessary. All enrolment form must include an Authorised Nominee (adults other than parent who have parent's authority to collect child, make decisions regarding child's wellbeing and health in absence of parents and are emergency contacts);
- Immunisation status certificate/immunisation history record);
- Medical Management, Risk and Communication Plans (if applicable) (For example: Asthma, Allergy, Anaphylaxis, Other Medical Conditions, etc);

- CWA (via OWNA App)
- Direct Debit Request/Authority (via OWNA App); and
- All other forms referred to in the enrolment form specific for each child.

Other information about our service's enrolment includes:

- We will try and accommodate families so that children from the same family can attend our service. This will be carried out in line with our obligations under the Priority of Access Policy.
- By Law(Public Health and Wellbeing Act 2008, to finalise enrolment for your child you must provide the service with an immunisation status certificate that shows your child is:
 - Up to date with vaccinations for their age; or
 - On a vaccine catch-up schedule; or
 - Has a medical condition preventing them from being fully vaccinated.
- Enrolment of educator's children at the service is generally not encouraged.
- In accordance with the National Law and Regulations, our educators will support each child to manage their own behaviour, respond appropriately to the behaviour of other children and communicate effectively to resolve conflicts. We will also work with each child's family to support any children with diagnosed behaviour and social difficulties. However, a child's enrolment at our service may be terminated if the nominated supervisor or approved provider decides the child's behaviour threatens the safety, health or wellbeing of any other child or staff member at the service.
- We will work with each child's family to support any children with a medical condition/s. However, a child's enrolment at our service may need to be terminated if the nominated supervisor or approved provider decides the child's, any other children's or staff member's safety, health or wellbeing is impacted or cannot be reasonably or effectively care for or be managed at the service.
- Parents and guardians authorise the service to obtain medical, dental, hospital and ambulance treatment for their child if required. In accordance with the enrolment terms and conditions, the service is not responsible for paying or covering any costs associated with medical, dental, ambulance, hospital or other treatment. Parents and guardians agree to reimburse Great Start Early Learning Centres and Service for any charges, costs or expenses incurred in arranging or obtaining such treatment.

Funded Kindergarten Programs

We offer the following Kindergarten programs which are delivered by Early Childhood Teachers and subsidised by the State Government. We can apply for kindergarten per capita funding or extension grants on families' behalf. These payments are made directly to our service. Please note children are only able to enrol in Government funded/subsidised Kindergarten programs at one Service. We will ask families to confirm in writing they are not accessing funded kindergarten programs elsewhere.

By way of background information our service receives Per Capita Kindergarten State Government Funding. The kindergarten per capita funding is paid directly to the service and our fees take into account receiving this funding. Kindergarten per capita funding is not received by parents. The per capita funding helps the service deliver a comprehensive kindergarten program. (Please note our service does not receive kindergarten program subsidy funding.)

Four-year-old Kindergarten

We offer children 15 hours of kindergarten per week in the year before they start school. Children may be able to access a second year of kindergarten if an early childhood teacher assesses that they have developmental delays in two or more areas. Our Early Childhood Teacher will be delivering the program on specific days and hours (please refer to our notice board) Children will need to attend for at least 2 or 3 days per week (depending on the number of hours and days the Early Childhood Teacher delivers the kindergarten program).

Three-year-old Kindergarten

We provide three-year-old Kindergarten programs between 5 to 15 hours per week to children who turn three years old by 30 April in the year they're enrolled in the program.

Please note all children in three-year-old kindergarten are expected to move to four-year-old Kindergarten the following year.

Kindergarten Fee Subsidy (3 and 4 year old kindergarten)

The Kindergarten Fee Subsidy enables eligible children to attend a Government funded kindergarten program at a service that does not provide long day care for 15 hours per week free of charge. Please note our service does not receive Kindergarten Fee Subsidy and only Per Capita Funding.

Early Start Kindergarten

We also offer lower-cost Early Start Kindergarten programs of 15 hours per week to children from a refugee or asylum seeker background, or who are Aboriginal or Torres Strait Islander, or who are known to Child Protection, where they will be at least three years old by 30 April in the year they're enrolled. Children can be enrolled in a three-year old group, a four-year-old group, a mixed age group or a combination of these.

These children can also access low cost Kindergarten in the year before school via an Early Start Kindergarten Grant irrespective of whether they were enrolled in Early Start Kindergarten the year before.

Victorian Government Free Kinder Offset

Families with a child enrolled in a funded kindergarten program may be eligible for additional Kinder funding – 'Free' Kinder Offset known as the *Victorian Government Free Kinder Offset*. This includes all children attending funded kindergarten programs in the year before school (Four-Year-Old Kindergarten) and Three-Year-Old Kindergarten program. Please refer to 66- Fee Policy for further information. Victorian Government Free Kindergarten Offset will be allocated to parent/guardian accounts on a monthly basis in arrears once it is paid by Department of Education.

Data Collection Privacy Policy

Funded Kindergarten services are required to collect information from all parents/guardians about their education and employment. Information collected includes:

- highest level of primary/secondary education and highest qualification completed (eg non-school qualification, bachelor degree). (Equivalent overseas education and qualifications are recognised.)
- Parents/guardians occupation group from the **Parental Occupation Index** for their main work.

The data is used to help calculate the amount of 'School Readiness Funding' we receive and we can use it to purchase a range of programs and resources including paying for professionals like speech therapists to work with children. All information provided during the enrolment process will be treated confidentially and will only be used to inform kindergarten funding.

Transition Learning and Development Statements

We complete Transition Learning and Development Statements for all children transitioning to school from a State Government funded Kindergarten program.

The Statement aims to help children's school teachers by summarising each child's abilities, individual approaches to learning, and how they can be supported to continue learning. Our Early Childhood Teachers will provide families with the Statement and advise that it will be shared with the child's school unless they don't want this to happen. Families will be encouraged to discuss sharing the Statement where they have concerns.

APPENDIX 1: Taking Enrolment Fee from Parent to secure enrolment (Using OWNA): As part of enrolment procedure

Access OWNA Portal > Families > Families / Accounts

Notes for enrolment fee:

- A) Parent can pay enrolment fee before they commence two ways:

SECTION 1: Direct Debit Bank Account or Credit Card (With bank account or credit card recorded in OWNA). NB: it takes up to 3 days for processing bank account direct debit. Credit card is immediate processing. Track Failed Payments and if failed

SECTION 2: A different credit card not recorded in OWNA.

- B) Enrolment Email: this includes a message to parent on the requirement to pay an Enrolment Fee and the amount before commencement to finalise enrolment. (until enrolment fee is paid enrolment is not held by service)
- C) Please let the parent know via email when you are planning to direct debit their bank account or credit card for the enrolment fee.
- D) Before commencement: When recording the **“Enrolment Fee Paid: Non-Refundable”** this will show as a payment as a “Credit/Payment” made by the parent in the family account for the amount current week plus 2 weeks. See further instructions below. Review Family Statement on completion: check enrolment fee charge and account balance.
- E) When child commences: attendance in roll will be recorded current week plus 2 weeks, this should be covered by the Enrolment Fee Paid. Ensure booking pattern and commence date is entered in child profile.
- F) ***If child does not commence at the service. The Enrolment Fee Paid is non-refundable. The Centre Director will need to “Add Transaction” (See OWINA Portal > Families Account > Click family > “+Action” > Add Transaction:
Add/select: Debit
Amount = Equals the same dollar value as the “Enrolment Fee Paid”
Description: Enrolment Fee Charged
Date: current date***

SECTION 1: Direct Debit (Bank Account Payment or Credit Card Method) Section: the bank account or credit card information will be set-up in the Family Account Profile. See below the details from the DDR form in OWINA

Families/Accounts

Quick Links to: [Parent/Child Left/Right](#) [Parent Test](#) [Child Test](#) [Families/Accounts](#) [Family Transactions](#) [Invoicing/Payments](#)

Home / Families / Families/Accounts

Family Details

Family Notes

Bond

Account Details

Account Name*

XTEST, SCOTT

Carers/Guardians*

Parent2First LastName X Parent Test3 X

Children*

Child Test X

Tags

Select...

Debit Limit (MAXIMUM amount to be charged)

Bank Details (if applicable) ☒

Bank Account Name

BSS & Account #

e.g. 062-000 e.g. 12345678

BPAY Reference ID (if any)

Preferred Debit Day

Wednesday

Credit Card Details (if applicable) ☐

Card Holder's Name

Card Number

Expiry

MM YY

CCV

Limits/Misc

Billing Start Date (if any)

Reminder Date

e.g. 2020-10-31

Reminder Notes

Reminders will be added to the Staff Communication Diary.

Pause Debiting on Invoices

☒ Family will NOT be debited their invoice amount. (Does not affect Regular Debit below).

SECTION 2: Take Payment via another Credit Card

(Ie another credit card that is not recorded in OWNA Family Account Profile)

Take Payment
Home / Families / Take Payment

Quick Links to: Families/Account Reconciliation Report Family Transactions Invoices/Payments

Enter Amount to be Direct Debited below:

SECTION 1 – bank account or credit card recorded in OWNA DDR Form

Family Account Name
Parents/Guardians
Children
Amount to Pay*
Transaction Description*
Override Surcharge

Enter: \$ Dollar amount of Enrolment fee – greater of \$100 or 2 weeks fee (use OWNA CCS calc)
ENTER "Enrolment Fee Paid: Non-refundable"
☐ Click if you want to override any transaction surcharge.
Take Payment via Direct Debit

SECTION 2 – a different credit card not in OWNA family profile

OR Take Payment via another Credit Card

Name on Card*
Card Number*
Expiry Date*
CCV*

Take Payment

VISA MasterCard

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

NQS

QA2	2.1	Each child's health and physical activity is supported and promoted.
	2.1.2	Steps are taken to control the spread of infectious diseases and to manage injuries and illness, in accordance with recognised guidelines.
	2.1.3	Healthy eating and physical activity are promoted and appropriate for each child.
	2.2.1	At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.
	2.2	Each child is protected.
	2.2.2	Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.
QA6	6.1	Respectful relationships with families are developed and maintained and families are supported in their parenting role.

6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions..</i>
6.2	<i>Collaborative partnerships enhance children's inclusion, learning and wellbeing.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	77	<i>Health, hygiene and safe food practices</i>
	78	<i>Food and beverages</i>
	79	<i>Service providing food and beverages</i>
	80	<i>Weekly menu</i>
	88	<i>Infectious diseases</i>
	90	<i>Medical conditions policy</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	96	<i>Self-administration of medication</i>
	97	<i>Emergency and evacuation procedures</i>
	99	<i>Children leaving the education and care service premises</i>
	100	<i>Risk assessment must be conducted before excursion</i>
	101	<i>Conduct of risk assessment for excursion</i>
	102	<i>Authorisation for excursions</i>
	157	<i>Access for parents</i>
	160	<i>Child enrolment records to be kept by approved provider and family day care educator</i>
	161	<i>Authorisations to be kept in enrolment record</i>
	162	<i>Health information to be kept in enrolment record</i>
	165	<i>Offence to inadequately supervise children</i>
	167	<i>Offence relating to protection of children from harm and hazards</i>
	168	<i>Education and care service must have policies and procedures</i>
	173	<i>Prescribed information is to be displayed</i>
	177	<i>Prescribed enrolment and other documents to be kept by approved provider</i>
	181	<i>Confidentiality of records kept by approved provider</i>
	183	<i>Storage of records and other documents</i>

Other References:

Public Health and Wellbeing Act 2008

The Child Wellbeing and Safety Act 2005

Children, Youth and Families Act 2005

Occupational Health & Safety Act 2004

Education and Care Services National Regulations 2011

National Quality Standard

A New Tax System (Family Assistance) Act 1999

2 - PRIORITY OF ACCESS POLICY

Position Statement

The centre has obligations to parents and funding bodies to provide care according to the Priority of Access guidelines set down by the Australian Government. The Government recognises that demand for child care sometimes exceeds supply and where a centre has a large waiting list and a number of parents competing for a limited number of vacancies, the guidelines will be adhered to.

Objective

To allocate available places to those families with the greatest need for childcare support as per the Australian Government Priority of Access Guidelines.

Implementation

Children who are enrolled at the centre or whose families are seeking a place at the centre will be given Priority of Access in accordance with the guidelines that have been established by the Department of Family and Community Services and Indigenous Affairs.

Below are the Priority of Access levels which the centre must follow when filling vacancies.

1. A child at risk of serious abuse or neglect.
2. A child of a single parent/guardian who satisfies, or of parents/guardians who both satisfy the work/training/study test under Family Assistance Legislation Amendment (Child Care) Act 2010.
3. Any other child.

Within these three categories priority is also given to the following children:

- Children in Aboriginal and Torres Strait Islander families.
- Children in families which include a disabled person.
- Children in families on low income.
- Children in families from culturally and linguistically diverse backgrounds.
- Children in socially isolated families.
- Children of single parents/guardian.

Upon enrolment families will be notified of their priority and advised that if the service has no vacancies and their child's position is a priority 3 under the Priority of Access Guidelines, it may be required that their child leave or reduce their days in order to make a place for a higher priority child.

When a service has no vacant places and is providing child care for a child who is a Priority 3 under the Priority of Access Guidelines, the service may require that child to leave the child care service in order for the service to provide a place for a higher priority child, but only if:

- the person who is liable to pay child care fees in respect of the child was notified when the child first occupied the child care place that the service followed this policy and
- the service gives that person at least 14 days notice of the requirement for the child to leave the child care service.

If the service cannot accommodate the immediate enrolment of Priority Level 1 and 2, the Centre Director will discuss with the child's parent/guardian (Priority 1 and 2) to review other children's services in the area to see if places are available to accommodate their immediate needs.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child.</i>
	2.2.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>
	2.2	<i>Each child is protected.</i>
	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
QA6	6.1	<i>Respectful relationships with families are developed and maintained and families are supported in their parenting role.</i>
	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions..</i>
	6.2	<i>Collaborative partnerships enhance children's inclusion, learning and wellbeing.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	77	<i>Health, hygiene and safe food practices</i>
	78	<i>Food and beverages</i>
	79	<i>Service providing food and beverages</i>
	80	<i>Weekly menu</i>
	88	<i>Infectious diseases</i>
	90	<i>Medical conditions policy</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	96	<i>Self-administration of medication</i>
	97	<i>Emergency and evacuation procedures</i>
	99	<i>Children leaving the education and care service premises</i>
	100	<i>Risk assessment must be conducted before excursion</i>
	101	<i>Conduct of risk assessment for excursion</i>
	102	<i>Authorisation for excursions</i>
	157	<i>Access for parents</i>
	160	<i>Child enrolment records to be kept by approved provider and family day care educator</i>
	161	<i>Authorisations to be kept in enrolment record</i>
	162	<i>Health information to be kept in enrolment record</i>
	165	<i>Offence to inadequately supervise children</i>
	167	<i>Offence relating to protection of children from harm and hazards</i>
	168	<i>Education and care service must have policies and procedures</i>
	173	<i>Prescribed information is to be displayed</i>
	177	<i>Prescribed enrolment and other documents to be kept by approved provider</i>

	181	<i>Confidentiality of records kept by approved provider</i>
	183	<i>Storage of records and other documents</i>

Other References:

Public Health and Wellbeing Act 2008

The Child Wellbeing and Safety Act 2005

Children, Youth and Families Act 2005

Occupational Health & Safety Act 2004

Education and Care Services National Regulations 2011

National Quality Standard

A New Tax System (Family Assistance) Act 1999

3 - GENDER EQUITY POLICY

Position Statement

It is the right of all children and adults to receive education which rewards, respects, and develops their individual needs, interests, and potential, without regard to gender, race or creed.

Objective

To encourage all children to reach their full potential regardless of gender through appropriate planning and implementation of a variety of activities.

Implementation

- Team members are to be conscious of the way they treat children and adults and to act in a non-biased way in regard to language, attitude, assumptions and experiences. Team members will not use derogatory or gender biased language towards others including team members and parents.
- Team members will provide a positive role model for children
- Team members will provide positive, non-stereotypical experiences for children eg. female team members climbing and running and male team members showing nurturing qualities.
- Team members will provide resource materials which portray non-stereotypical views of both genders.
- The centre will employ team members of either gender, provided they are the best applicant for the position.
- Team members will ensure that planned experiences are available to all children regardless of gender.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team members feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/12/2025

Reference:

NQS

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
QA7	7.2.3	Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.
	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

QIAS Principle 1.5, 1.6, 4.1
Anti-discrimination Act

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

4 - INCLUSION POLICY

Position Statement

All children and their families will be accepted into the Centre on an equal basis. It is a core focus of our philosophy that team members welcome and treat all children and their families equally as individuals. We are sensitive to the unique needs of children and their families and we respect their diverse backgrounds regardless of religion, gender, culture, physical needs and/or disabilities, and socio-economic conditions.

In the case of limited vacancies positions will be offered to children in accordance with the Priority of Access guidelines set down by the Family Assistance Office (See Priority of Access Policy).

Objective

To ensure that all children enrolled in the centre have equal opportunity to participate in the centre's program.

Implementation

- Enrolment and associated forms will request detailed information about the child's individual needs.
- Educational Room Leaders and other team members will make themselves aware of any individual needs that will need to be taken into account during the time that the child is at the centre.
- Extra curricula activities are offered to all children to ensure that equal opportunity exists for all children to participate in activities.
- Children with additional needs will need to provide detailed information to the centre regarding additional requirements for the child. Application will then be made to the relevant ancillary support agencies and other government agencies (where appropriate) for additional support and training to ensure the effective integration of children with additional needs.
- Inclusion extends beyond diagnosed additional needs. It takes into consideration diversity and equity. To ensure we create an inclusive environment for all children educators will work alongside and partnerships with families, Inclusion Professional – Victorian Inclusion Agency Region, Community Child Care Association, support agencies, Preschool Field Officers, Koori Pre-school Assistants (KPSA), orange door and child protection, local schools, OT and speech professionals, etc.
- Team members will encourage children to show respect for other cultures, races, religious beliefs, age, gender and differing abilities by role modelling appropriate practice. Team members will demonstrate an anti-bias approach through sensitivity, respect and appreciation for all individuals and their individual needs.
- The service environment will be set-up to encourage inclusion of all children in all experiences.
- The service and room displays and the equipment used within the centre will reflect the diversity of abilities, cultures, races, religions, gender, etc within Australian society.
- The educational curriculum and centre philosophy reflects an understanding of diversity and incorporates inclusion within all elements of centre operation.
- The centre aims to achieve positive outcomes for all children by enthusiastically embracing inclusive practices throughout daily centre operation.
- The centre works in partnership with families to provide an inclusive environment which meets the needs of both the child and the family.
- The centre reserves the right to limit the number of children attending the centre requiring additional support and resources. This is due to the financial hardship and stress placed upon the centre to provide the additional support and resources required to care for children with high support needs. We therefore are required to limit the number of children with high support needs at the centre at one time to ensure we are providing quality care to all children.

- Overcoming barriers to access and inclusion: Providing access for people with disabilities will also provide benefits to people who may be disadvantaged in terms of access. Examples include:
 - parents with prams, and seniors who find it difficult to negotiate steps or steep gradients;
 - people who have a temporary disability through accident or illness;
 - people from culturally and linguistically diverse backgrounds who may find it difficult to read signs or understand information; and
 - small children who have difficulty climbing steps or understanding information.

People with disabilities face barriers to everyday activities such as hearing what is said, seeing small print, climbing stairs and understanding documentation. The impact these barriers have on the life of the person can be major.

Our service aims to have an equitable, inclusive, and safe environment and for our workplace and services to be accessible to people with disability. The service will review barriers to inclusion and access as part of its reflective practices and internal review procedures (such as OHS&W reviews, policy reviews, Strategic Inclusion Plans and Early Ables.)

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/3/2026

Reference:

NQS

QA3	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
QA5	5.1.1	<i>Responsive and meaningful interactions build trusting relationships which engage and support each child to feel secure, confident and included.</i>
QA6	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
	6.2.2	<i>Effective partnerships support children's access, inclusion and participation in the program.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>
	157	<i>Access for parents</i>

EYLF

LO1	<i>Children feel safe, secure, and supported</i>
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	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency</i>
	<i>Children develop knowledgeable and confident self identities</i>
	<i>Children learn to interact in relation to others with care, empathy and respect</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 1.5, 2.2

Child Care Service Handbook 2003-2004

Legislation:

Occupational Health & Safety Act 2004

Disability Discrimination Act 1992

Racial Discrimination Act 1975

Sex Discrimination Act 1984

Equal Opportunity for Women in the Workplace Act 1999

Human Rights and Equal Opportunity Commission Act 1986

Racial and Religious Tolerance Act 2001

Sources and further reading:

- Creaser, B & Dau, E. (1997) The Anti-Bias Approach in Early Childhood. Australian Early Childhood Association, Inc
- Clarke, Dr Priscilla (2007) 'Respecting Diverse Philosophies – balancing parents wishes and promoting early childhood values' FKA Children's Services Inc, Resource 132
- Stonehouse, A. & Boschetti, C. (2006) A piece of cake? Inclusive practices in early childhood settings. Yooralla, Vic.

5 - COMMUNICATION PLAN & POLICY

Position Statement

Being Reggio Emilia inspired service, we believe it is essential to have strong parent involvement with clear channels for the sharing of information, communication, cooperation and partnership.

To enable us to provide the highest quality of care and education, it is imperative that parents and educator are able to communicate openly and with respect. Parents should feel comfortable in discussing any concerns or suggestions they may have.

Objective

To have open and clear communication between educators and parents/guardian.

The formulation of a communication plan mentioned in NQS, element 1.3.3 is about informing families about the curriculum and their child's progress 'to ensure that information for families is accessible, meaningful and useful'. The following communication plan provides the framework to achieve these objectives.

Implementation

- You will have received a copy of the Family Handbook that outlines our Philosophy statement and key policies and we welcome your feedback.
- On enrolment we will provide you a Welcome email that has a number of attachments and reference materials.
- We have a website: www.greatstartelc.com.au that contains a lot of useful materials including commonly used forms, our family handbook, reference to key policies, Centre Policy Handbook, links to useful websites and also frequently asked question page.
- As well as the information you have provided to us during enrolment, we would appreciate you sharing your knowledge about your child by completing our 'All About Me' form, Family Input Forms and Child Learning Plan. These includes questions such as your child's interests, likes, dislikes, favourite things to play with at home and celebrations that are meaningful in your family. Please share with us anything else that will help us to get to know your child.
- We value your input in establishing goals for your child's further learning and development. All children have a Child Learning Plans and parents have the opportunity to contribute to their child's learning goals. The Educational Room Leader and co-educators will communicate the child's learning outcomes associated with the learning goals. If you have a concern about your child's wellbeing, development or learning please discuss with us whenever you wish.
- There are daily opportunities for brief informal conversations at drop off and pick up times however if you would like more time to talk it is preferable to make a time for us to meet together.
- A communication book and/or diary is situated in each room. This allows for written messages or ideas to be passed from parent to team members and team members to team members. The use of this book is to be actively encouraged.
- Documentation about your child's participation in the educational program, including learning stories, learning observations, photos of your child's work will be included in your child's online learning portfolio. Your child's online learning portfolio can be accessed via OWNA. You may borrow this at any time to take home but please return the next day as the Regulations require that your child's records are at the service.
- The educational program is displayed on the curriculum noticeboard and on OWNA Parent App, so that you can see the experiences that have been planned to foster the children's learning and development as well as how educators respond to what children are interested in learning. Occasionally, specific plans for your child will be included in the daily educational program.
- Your comments and suggestions on the program are very welcome.
- Forming part of the Curriculum, Educators will seek Family Input (Parents, Siblings, Grandparent, friends, guardians) to promote and develop children's interested and growth areas. We encourage you to provide anything that reflects your life as a family.

- A daily journal via OWNA is provided to parents as a means of communication between the child's educator and parent. An notification will appear when the daily journey is published; please view via the OWNA Parent app or website
- Educator will prepare a mid-year and end of year summative assessments (Child Reflection Reports) providing a sum summary of your child's progress towards the five Learning Outcomes (that are part of the Early Years Learning Framework and the Victorian Early Years Learning and Development Framework). We value your input in establishing goals for your child's further learning and development.
- Kindergarten School Transitions Reports will be prepared by educators to communicate child's progress in relation to development and learning. These reports will be forward to your child's school to provide information to your child's teacher to assist in transition to school.
- If you have a concern about your child's wellbeing, development or learning please discuss with us whenever you wish.
- Meeting times can be requested by parents or team members to allow for uninterrupted discussion regarding the child's developmental level, progress and accomplishments, plus a time for sharing any queries, concerns, suggestions, expectations and practices.
- Parents are also informed about centre news through OWNA, newsletters, bulletin boards, telephone calls, memos, Facebook and emails.
- A family surveys will be conducted through the year using Survey Monkey.
- Parents will be encouraged to use the suggestion box to voice any concerns, queries or suggestions regarding the administration of the centre, policies and programs. These may be made anonymously. The box is situated near the reception area.
- In the foyer of the Service there is a display notice of a QR code to provide anonymous feedback, suggestions and complaints. We appreciate the opportunity to discuss any concerns in person but also respect people choice to provide anonymous feedback.

Interpreter and Translation Services and Protocols

We are able to support families with interpretation and translation service to promote communication and help understand information associated with their child enrolment, education and also build relationships.

On consultation with person needing interpreter or translation services, the service may seek support, advice and assistance from Bilingual Educators, staff, agency personnel and community leaders, FKA Children Services or DET: Interpreter and Translation Services in relation to interpreter and translation services.

Please refer to the following for procedures and protocols for delivering interpreter services:

- Department of Education: Guidelines for use of interpreting and translation services in Early Childhood services
- Policy Appendix 1: How to Work with an Onsite Interpreter
- Policy Appendix 2: How to work with a Telephone Interpreter

Foundation House has developed the following tip sheets to support work with interpreters and families from culturally and linguistically diverse (CALD) and refugee backgrounds:

- TIP SHEET 1: BUILDING PARTNERSHIPS WITH FAMILIES FROM REFUGEE BACKGROUNDS
- TIP SHEET 2: WORKING EFFECTIVELY WITH INTERPRETERS TO SUPPORT FAMILIES FROM REFUGEE BACKGROUNDS

For Enrolled Kindergarten Program Children and Families

Our interpreter and translation protocols are guided by the Department of Education: Guidelines for use of interpreting and translation services in Early Childhood services delivered or funded by DET.

Reference to DET website providing information on interpreter and translation services:

<https://www.education.vic.gov.au/childhood/professionals/families/Pages/interpreter.aspx>

DET provides funding for interpreting and translation services to support children and young people who are:

- enrolled in government schools;
or
accessing early childhood services or other support services delivered or funded by DET;
and
- whose parents are from a language background other than English and require communication to be undertaken in their first language.

Early Childhood Services and Support Services funded by DET include:

- Kindergarten Programs
- Early Childhood Intervention Services
- Specialist Children's Services
- School Focussed Youth Services
- Primary School Nursing
- Secondary School Nursing
- DET funded Occasional Child Care

With reference to the (DET) Guidelines for use of interpreting and translation services in Early Childhood services delivered or funded by DET. Extract from Guide:

1.1. Early Childhood Services and Support Services funded by DET - on-site and telephone interpreters are available for the following categories/assignments:

EC05 / EC06 - Funded Kindergarten Programs – telephone, service or home based discussion with parent/carer.

EC07 / EC08 – Funded Kindergarten Programs / Transition to Primary School – telephone, service or home based discussion with parent/carer.

EC09 / EC10 - Early Childhood Intervention - telephone, service or home based discussion with parent/carer.

Note:

The above categories and notional times (60mins/90mins) relate to interpreting assignments for approved categories.

While a parent/carer can initiate an interpreting service by contacting the school or DET funded service, all bookings must be made by the school or the DET funded service.

Booking an interpreter

It is advised by DET, for most interpreting services, schools, early childhood services and support services will not require prior permission from DET for booking interpreters. However, in some situations (see *Unusual situations* above); VITS LanguageLoop Interpreting and Translating Services will require permission from DET before accepting the booking.

To view an online version of the LanguageLoop welcome kit which has been distributed to all schools, see:

-  [VITS LanguageLoop Service Directory](#)
-  [How to work with an onsite interpreter \(pdf - 1.82mb\)](#)
-  [How to work with a telephone interpreter \(pdf - 4.77kb\)](#)
-  [Instructions for using VITS LanguageLoop telephone interpreting service](#)
-  [Language identifier \(pdf - 1.23mb\)](#)
-  [How to work with AUSLAN and deaf interpreters \(pdf - 517.99kb\)](#)

Contact details

Direct all queries concerning interpreting and translation services to:
language.services@edumail.vic.gov.au

Complaints about the quality of services provided by VITS LanguageLoop should be raised directly with VITS LanguageLoop in the first instance via email at feedback@languageloop.com.au. If this method does not resolve the matter, please contact DET via email at language.services@edumail.vic.gov.au.

Bilingual Educators, Staff and Community Leaders & Agencies

Our bilingual educators and staff will support, settle and engage the child and family in the service by facilitating communication through a shared language. We may also seek support from Community leaders and agencies on approval from the parent to support the family. Our aims are to:

- Create an environment in which the child's home language is spoken, thereby fostering a sense of identity, belonging and wellbeing
- Facilitate communication and support relationship development between key educators and the child
- Encourage communication between the child and their peers to support the child in building relationships with others; fostering their sense of wellbeing and belonging
- Facilitate communication between the service and family by providing a shared language
- Facilitate cultural awareness by enabling the exchange and sharing of information between educators and parents about the child, family life and participation within the learning environment

Appendix 1: How to Work with an Onsite Interpreter

(Source:

<https://www.education.vic.gov.au/Documents/childhood/providers/comms/vitsonsiteintertips.pdf>)

HOW TO WORK WITH AN ONSITE INTERPRETER



PREPARATION

- > Book the interpreter as early as practically possible according to the nature of the session. Do allow extra time for the session. As a general rule, sessions with interpreters take approximately half to double the time of a standard session.
- > Ensure the interpreter is provided with the name of the client, the expected duration and nature of the session, and any other relevant information to allow the interpreter to be familiar with the topic and/or terminology.
- > Organise an area where you can talk to the client undisturbed and arrange the seating to allow for easy communication.

BEFORE THE SESSION

- > It is recommended that the client and interpreter sit separately from each other to avoid the risk of the client asking the interpreter questions prior to session. If possible, have a designated waiting area for interpreters to ensure that they are visible and to ensure prompt commencement of sessions.

DURING THE SESSION

- > When the interpreter arrives, allow them to introduce themselves to you and the client.
- > Explain your role and the interpreter's role to the client and assure the client that what is discussed in the session will remain confidential between you, the interpreter and the client.
- > Always remember that you are in control of the session. Interpreters do not conduct the session.
- > During the session, speak directly to the client in the first person and not the interpreter. e.g. Say "How can I help you?", instead of "How I can help him/her?"
- > Maintain eye contact with the client throughout the session, speak clearly and avoid using jargon, metaphors, slang, idioms or proverbs.
- > Interpreters do not interpret any lengthy documents or provide any written services e.g. filling in forms or taking statements.
- > Speak in short intervals allowing time for the interpreter to relay the information. Let the interpreter control the length of each segment by signalling to you/the client when to stop.

- > Provide clear explanation when the interpreter seeks clarification and do not add additional information that will exclude the client as everything that is said during the session must be interpreted.
- > Check whether the client has understood the key messages and is ready to end the session. Also provide opportunity for the client to ask any questions.

AFTER THE SESSION

- > If the client requires another appointment, please make these arrangements with the client while the interpreter is present.
- > It is recommended that the client and interpreter do not leave at the same time to avoid the risk of the interpreter becoming involved with the client.
- > Offer a debriefing if you consider the interpreter is affected by a particularly stressful situation and/or clarify any question you both might have from the session.
- > If required, provide feedback to the interpreting service provider, as this will help VITS improve service quality. Also ensure that you provide positive feedback when the interpreter's service meets or exceeds your requirements.

Appendix 2: How to work with a Telephone Interpreter

Source:

<https://www.education.vic.gov.au/Documents/childhood/providers/comms/vitsteluserguide.pdf>

How to work with a Telephone Interpreter

- > Introduce yourself to the interpreter, explaining where you are and the nature of your call.
- > Clearly indicate the expected duration of the call, and limit conversation to the essentials to allow interpreter to assist clearly.
- > Try to make yourself clear as an interpreter can't rely on body language to understand and convey information to a Non-English speaking client.
- > Clearly indicate when the session has ended to everyone involved in the call.

Please note that Telephone Interpreting may not be appropriate if:

- > The client is under emotional, mental or physical stress
- > The client is deaf or hard of hearing
- > The interview will be greater than 45 minutes
- > Visual aids or documents need to be referred to
- > There may be legal or medical risks, such as in situations involving law enforcement or emergency services.



Evaluation and Review

This policy will be reviewed at least annually.

Family and team members feedback will be considered in the review process. Changes in legislation, regulations, NQF and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

Reference:
NQF

QA1	1.3.3	<i>Families are informed about the program and their child's progress.</i>
QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.3	<i>The services builds relationships and engages with their local community</i>

QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>
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National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

Reg	157	<i>Access For Parents</i>
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Other References:

Adapted from the 'Sharing a Picture Project' – Centre for Community Child Health, Royal Children's Hospital.
<https://www.education.vic.gov.au/childhood/professionals/families/Pages/interpreter.aspx>

6 - PARENT INVOLVEMENT POLICY

Position Statement

Parents are recognised as the most important teachers and caregivers in a child's life. Their continued involvement in their children's activities helps to optimise each child's development. For this reason, it is important to foster and support parental involvement within the centre. Recognition is also given to the fact that different families will have different levels of involvement they wish to maintain.

Objective

To structure the environment in such a way as to foster all levels of parental involvement.

Related Policies

8- Families and Visitor Code of Conduct

76 – Open Door Policy

Implementation

- A warm and caring attitude by team members goes a long way towards gaining parent's trust. Forming a positive relationship with parents helps to foster parental involvement with their child as well as with the centre.
- Families are welcome any time to attend the service. Please refer to our Open Door Policy. For effective parent involvement in the classroom please discuss and arrange with the educators the times of attendance to allow them to structure their curriculum and also discuss the parent involvement with children. This will also assist educators for planning strategies for children with Autism Spectrum Disorder (ASD).
- Each room will arrange to have a daily reflection journal for the parents to give them a clear idea of what their child has done during the course of the day. Contact team members who are present when the parent comes to pick up their child should also talk briefly to the parent about the child's day.
- Parents who wish to stay and spend time with their child should be welcomed and encouraged to do so.
- We work hard to achieve parent committee interest in the centre as a more formal forum for parental involvement. All parents are welcome to join this committee.
- Special events, such as Christmas parties, sausage sizzles, etc. are sometimes planned to invite parent and/or whole family participation.
- New or prospective parents are shown through the Centre and are made aware of our commitment to making them feel welcome as part of the centre family.
- Contribution of information from the Home Environment via Parent Input Forms, emails, verbal discussions, etc.

Arrival and Departure

- As a matter of safety, children must be brought into the centre and collected from the centre by an adult. The adult must sign the child "in" and "out" using the Kiosk and notify the educator that the child has arrived or is departing.

- The paper-based Wellbeing Charts (if used for room in addition to OWNA App Daily Chart) will be located where the children's activities are being conducted.
- Sign in and out Kiosk is not only a safety requirement but also a legal requirement linked to regulatory requirements. As such, the sign in and out protocols need to be strictly adhered to. Centre Directors are required to monitor parent's sign in and out on the Kiosk on a daily or weekly basis. From a CCS perspective all sign in and outs in Kiosk need to be confirmed by parents.
- If applicable - In relation Child Wellbeing Schedules, Team members should enter arrival times and departure times and cross reference to attendance absences to kiosk.

No child will be allowed to leave the centre with a person other than the parent/guardian unless:

- The parent has informed the Director, Responsible Person or Educator that another person will be picking the child up. Information will need to include the child enrolment form and child authorisations.
- That person is authorised on the enrolment form to collect the child.
- The authorised person shows centre team members identification that contains a photo and full name.
- Should a person arrive to collect a child who does not meet the above arrangements, the parents are to be called first to confirm the pick up arrangements for that day. If this will be a regular arrangement, the new person will need to be added to the authorised pick up list on the child's enrolment form. This includes the signing in and out on the Kiosk for dropping off and picking up children.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

References:

NQF

QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>

	6.2.3	<i>The services builds relationships and engages with their local community</i>
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QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	157	<i>Access For Parents</i>
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Other Reference:

CCB – Federal Funding Guidelines
Handbook on Child Care Licensing
QIAS Principle 2.2

Legislation

Anti-discrimination Act
Child Care Regulations 2004 & Act 1998 (NSW)
Privacy Act 1988
Sexual Harassment Act
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

7 – COMPLAINTS & GRIEVANCE POLICY

Position Statement

We recognise that the partnership between families, community and centre team members is most important. We also recognise our role in caring for a family's most precious treasures – their child/ren. It is therefore imperative that sound relationships are developed and that any parent or community concerns are addressed to achieve a positive resolution.

Objective

To enhance communication and co-operation between parties with mutually satisfactory solutions the desired outcomes.

Related Policies and Procedures

- Child Safe Standards Policy
- Child Safety, Protection and Wellbeing Policy
- Child Safeguarding: Look, Do & Tell Framework Policy
- Protected Disclosures (Whistleblowers) Policy
- Ethical Conduct and Code of Ethics Policy
- Governance of Service, Management and Educator Policy

Procedure

These guidelines explain the procedure for reporting and managing grievances, the roles and responsibilities of educators, staff and managers and the potential consequences of breaching our policies, procedures and Code of Conduct.

Should parents have any concerns in regard to their child's care it is essential that these concerns be raised so that the centre can address any issues and achieve a satisfactory resolution.

In the first instance concerns should be directed to the Room Leader in charge of the child's room. If it is not possible for the parent to address their concerns with the Room Leader, or they do not feel satisfied with the outcome, the parent should make their concerns known to the Centre Director (Nominated Supervisor or Responsible Person in Charge of Service) who will take steps to remedy the situation. To provide a written complaint or grievance please complete the form "Parent Concern & Suggestion Form" (OPR016). This form can be accessed also via the OWNA App. You can also complete an anonymous form via the QR code in the service foyer. The complaint is to be logged into the central complaint register. See appendix 1.

OWNA electronic form to respond to a complaint, concern, allegation or disclosure about the safety of a child, per the ACECQA NQF Child Safe Culture Guide – see Appendix 2. If the matter involves an incident, injury or trauma, also complete the [Incident, Injury, Trauma & Illness Record](#).

If the parent feels that their concern has not been adequately addressed, they may choose to contact either Management, the state Regulatory Authority (Victorian Early Childhood Regulatory Authority (VECRA)) or the Australian Children's Education and Care Quality Authority (ACECQA) whose contact numbers are displayed in the centre foyer for parent's convenience. The form to complete to report a formal complaint against the service is called "Notification of Complaints and Incidents (Other than Serious Incident)" – Form reference NL01. This form can be found on the ACECQA website.

The organisation has a strong commitment to child safety, wellbeing and protection, through implementing and adherence to the Child Safe Standards and the Information Sharing Scheme. This

commitment extends to all children in our service and community with an aim for children to feel safe, secure and supported. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations. We have a zero-tolerance approach to child abuse and harm. We acknowledge the importance of our organisation and community to be committed to cultural safety of aboriginal children, the cultural safety for culturally and/or linguistically diverse children and safety of children with a disability.

Child Focused Complaints Process

With reference to Child Safe Standard, Educators and staff will need to be attuned to complaints from children and will support them, and where appropriate their parents/guardians, to access and navigate our grievance/complaint process where this is reasonable. This includes for all complaints where a child alleges directly or indirectly their safety and wellbeing has been or could be harmed, including through the inappropriate behaviour of an adult or another child at the service.

Having a Child Focused Complaint Process includes:

- Empowering children to raise low-level concerns with an aim to improve the likelihood that they will feel comfortable making a disclosure or reporting abuse
- Process is publicly available and accessible
- Being culturally safe
- Ensuring complaints are taken seriously and responded to promptly and thoroughly
- Implementation of Child Safeguarding: Look, Do & Tell Framework Policy; the policy has a child centred and focused approach. (See Policy 12C).

Having an easily understandable policies and procedures such as our Child Safety, Protection & Wellbeing Policy and Child Safe Standards Policy supports the child focused complaints process. Please refer to this policy and procedures for guidelines.

Educators, staff, volunteers, families and visitors will:

- Please refer to the Child Safety, Protection and Wellbeing Policy for guidance on concerns or complaints relating to child safety, wellbeing or protection.
- raise the grievance/complaint directly with the person concerned. Both parties should try to resolve the issue and develop solutions to ensure the problem does not happen again. Discussions should remain private, confidential, respectful and open-minded, will not involve other educators, staff, volunteers or visitors (eg parents) and will take place away from children
- raise the grievance/complaint with the Approved Provider or Nominated Supervisor (or another manager/supervisor if the Approved Provider or Nominated Supervisor is involved) if they are unable to resolve the concern, or feel unable to raise the matter directly with the person concerned. The Approved Provider or Nominated Supervisor (or supervisor) may request the issue be put in writing. Employees should provide all relevant information, including what the problem is, any other person involved in the problem and any suggested solution. Educators are encouraged to communicate openly about the issue.
- raise any grievance involving suspected or actual unlawful activity (including bullying) with the Approved Provider or Nominated Supervisor immediately and privately
- be confident that their concerns will be thoroughly investigated, but aware that the outcome may not result in the action requested.

Educators, staff, volunteers, families and visitors will not:

- get involved in complaints/ grievances that don't concern them. This is not ethical or helpful in managing the complaint.
- raise complaints with an external complaints body, such as a court or Tribunal, without using our grievance procedures and appeal process first.

The Approved Provider or Nominated Supervisor will:

- discuss the issue with the complainant within 24 hours of receiving the verbal or written complaint
- identify and report any concerns and allegations of inappropriate conduct (observed or suspected or disclosures) by other staff members, volunteers or students to the approved provider and VECRA within 24 hours. Notify the Social Services Regulator within three (3) business days of becoming aware of reportable conduct made against an employee or volunteer and ensure they are investigated, and appropriate action taken.
- properly, fairly, confidentially and impartially investigate the issue including:
 - thoroughly investigating the circumstances and facts and inviting all affected parties (individually) to provide information or respond where appropriate. To encourage teamwork and respect, the issue may be discussed at an educator meeting if the privacy of the people involved can be protected.
 - inviting the complainant to have a support person present during an interview (eg health and safety representative, but not a lawyer acting in a professional capacity)
- provide all affected parties with a clear statement of the outcome of the investigation within seven working days of receiving the verbal or written complaint.
 - If the resolution of the complaint involves a written agreement, all parties must agree with the wording etc.
 - If the Approved Provider or Nominated Supervisor decides not to proceed with the investigation after initial enquiries, he or she will give the complainant the reason/s in writing.
- keep appropriate records of the investigation and outcome, and store those records in accordance with our Privacy and Confidentiality Policy and Record Keeping and Retention Policy. Unsubstantiated complaints against educators/staff may be retained on file if the person has been given the opportunity to record a comment on the documentation
- monitor ongoing behaviour and provide support as required
- ensure the parties are protected from victimisation
- offer external review by a Tribunal or alternate organisation where employees, visitors and volunteers are unhappy with the outcome of the grievance procedure. Workplace bullying matters may be referred to the Fair Work Commission which can direct employers to take specific actions against workplace bullies or the Work Health and Safety (WHS) Regulator which may investigate whether WHS duties have been contravened
- request feedback on the grievance process using a questionnaire
- track complaints to identify recurring issues within the Service
- notify the regulatory authority within 24 hours if a complaint alleges the safety, health or wellbeing of a child is being compromised. Refer Incident, Injury, Trauma and Illness Policy.

Outcomes may include:

- an apology and a commitment that certain behaviour will not be repeated (monitoring this over time)
- education and training in relevant laws, policies or procedures (eg bullying awareness, leadership skills)
- assistance in locating relevant counselling services
- disciplinary procedures including a verbal or written warning, termination of employment or transfer to a different position at the Service
- ensuring any inequality or inequity is remedied
- providing closer supervision
- modifying Service policies and procedures
- developing new policies and procedures.

Outcomes will take into consideration relevant industrial relations principles and guidelines and make provision for procedural fairness. The Approved Provider or Nominated Supervisor will consider:

- the number of complaints (or breaches)
- the opportunities given to adhere to a policy or procedure and/or change behaviour.
- the opportunities given to respond to the allegations.
- the seriousness of the complaint (or breach), and whether it impacted the safety and welfare of other employees, volunteers or visitors.
- whether a policy, procedure or complaint is reasonable.

Complaints that must be notified to Regulatory Authority

The Approved Provider or Nominated Supervisor will notify the **Victorian Early Childhood Regulatory Authority (VECRA)** using form [NL01 Notification of Complaints, Incidents and Additional Children in an Emergency](#):

- within 24 hours of any complaints alleging that the safety, health or wellbeing of a child is being compromised at the service
- within 24 hours of any complaints that the National Law has been breached
- notify the Social Services Regulator within three (3) business days of becoming aware of reportable conduct made against an employee or volunteer and ensure they are investigated, and appropriate action taken

We welcome parent and community feedback and parents are regularly encouraged to comment upon the standard of care provided at our centre both formally (through parent surveys) and informally (with regular verbal communication).

Please note that this service has a zero tolerance for adult behaviour that is verbally or physically threatening. Team members faced with such a situation are to direct parents to the Family and Visitor Code of Conduct Policy and contact the police for assistance if required.

Protected disclosures (whistleblowers) training

The approved provider and our service will provide regular training and awareness sessions to our staff (including volunteers and students) about the importance of, and protections provided to, persons making protected disclosures (*National Law* s 300(1)(b)). See *102 - Protected Disclosures (Whistleblowers) Policy*.

Appendix 1 - Feedback/Complaints Register

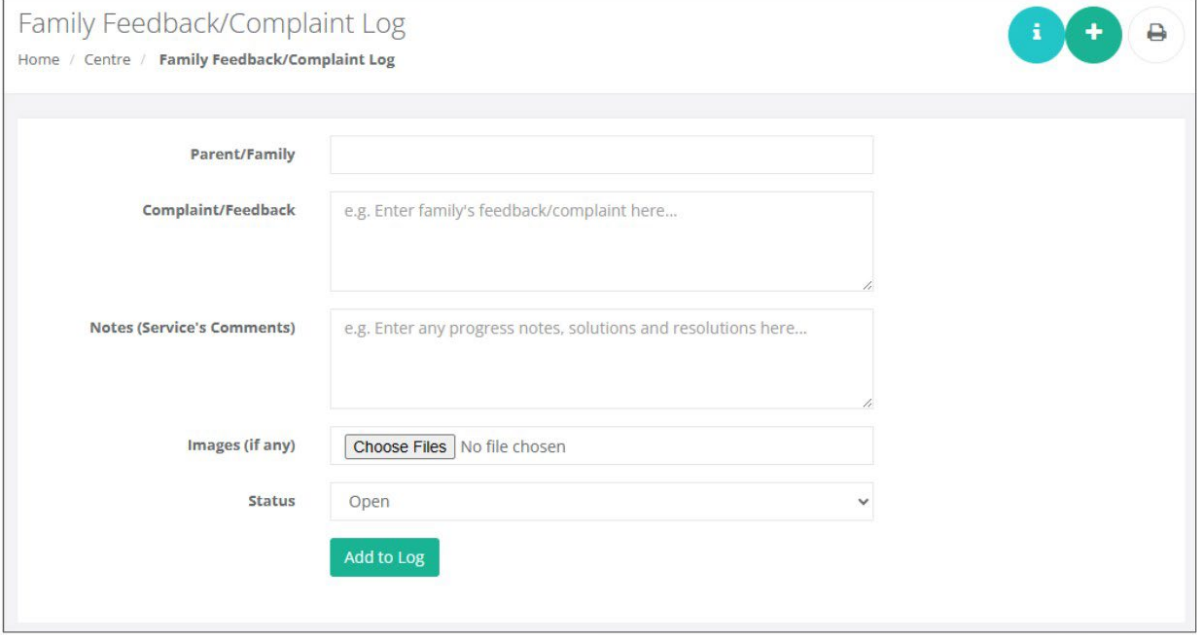
Overview

Family's at time may provide feedback and/or complaints to the Centre. It is a requirement that this type of information is recorded and retained.

Record Feedback/Complaint

 **Reports > Family Feedback**

1. Click on the plus **+** button in the top right corner
2. Fill in the relevant information
3. Click **Add to Log**



The screenshot shows a web form titled "Family Feedback/Complaint Log". At the top, there is a breadcrumb trail: "Home / Centre / Family Feedback/Complaint Log". In the top right corner, there are three circular icons: a teal circle with a white 'i', a green circle with a white '+', and a white circle with a grey printer icon. The form itself has a light grey border and contains several input fields:

- Parent/Family**: A single-line text input field.
- Complaint/Feedback**: A multi-line text area with placeholder text "e.g. Enter family's feedback/complaint here...".
- Notes (Service's Comments)**: A multi-line text area with placeholder text "e.g. Enter any progress notes, solutions and resolutions here...".
- Images (if any)**: A file upload section with a "Choose Files" button and the text "No file chosen".
- Status**: A dropdown menu currently showing "Open".

At the bottom of the form is a green button labeled "Add to Log".

Update Feedback/Complaint Log

 **Reports > Family Feedback**

- The log will display feedback and complaints that have been recorded prior.
- Click on the blue Edit icon to edit the complaint and enter Action Taken in the Service's Comments section. You can also adjust the status of the record.

Overview

Record respond to a complaint, concern, allegation or disclosure about the safety of a child

1. Click on the plus **+** button in the top right corner
2. Fill in the relevant information
3. Click **Save Incident Response**

Policy Handbook – P001 7.2026

5
Initial risk assessment

☐ A risk assessment is required for this matter
Complete to identify actions needed to mitigate ongoing risk to the child / children.

Hazard / risk identified

Likelihood

-select-

Consequence

-select-

Risk rating (before controls)

-select-

Controls / actions to mitigate risk

Residual rating (after controls)

-select-

6
Course of action

Actions decided, responsible person(s) and timeframes
What will be done, by whom, and by when.

Communication with family
☐ Family / parent notified

Date family notified

yyyy-mm-dd

Notes

7
Review & continuous improvement

Outcome of the matter

Changes to practice / policy to reduce recurrence

Review date

yyyy-mm-dd

8
Attachments

Upload supporting documents (statements, correspondence, photos, notification confirmations). You can select multiple files. Accepted types: PDF, JPG, JPEG, PNG, GIF.

Choose Files
No file chosen

Current attachments
No attachments uploaded yet.

Sign-off & status

Completed by

Scott Zannes

Position

Reviewed by (Approved Provider / Nominated Supervisor)

Status

Open

Cancel

Save incident response

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2027

Reference:

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

QA7	7.2.3	<i>Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

Education and Care Services National Law Act 2010
Education and Care Services National Regulations
Child Safe Standards – 1 July 2022
Child Wellbeing and Safety Act 2005 (Vic)
United Nations Convention on the Rights of the Child

8 – FAMILIES AND VISITOR CODE OF CONDUCT

Position Statement

In a service where there are many families with different backgrounds and values; team members, children and parents all need to feel safe and respected.

Objective

To state our standards and expectations for the conduct of families and visitors at our service. To outline the services expectations for provision of a safe and peaceful environment where children can come to play and learn. This Code helps us to have a safe, respectful and positive environment at our service.

Scope

This code applies to:

- 'Families' of children enrolled at our service: parents, grandparents, extended family, relatives through marriage, and care providers
- Visitors to our service, including allied health professionals

It applies to all behaviour and conduct that is reasonably related to, or connected with, our service's activities, including our day-to-day operations, service hosted events, functions, excursions, and use of technology. It is not limited to behaviour within the physical workplace or during normal business hours.

Related Policies

- 62 – Internet, Email & Social Networking Policy
- 12A - Child Safe Standards Policy
- 12B - Child Safety, Protection & Wellbeing Policy
- 12C - Child Safeguarding: Look, Do & Tell Framework Policy
- 88 - Child Safe Environment Policy
- 7 - Complaints and Grievance Policy
- 1 – Enrolment Policy
- 70 - Ethical Conduct and Code of Ethics Policy
- 71 - Family Law and Access Policy
- 78 - Governance of Service, Management and Educator Policy
- 72 - Interaction with Children Policy
- 62 - Internet, Email and Social Networking Policy
- 100 - National Model Code Policy
- 76 - Open Door Policy
- 6 - Parent Involvement Policy
- 16 - Privacy and Confidentiality Policy
- 41 - Smoke, Vape, Alcohol & Drug Free Environment Policy
- 55 - Student and Volunteer Policy

Definitions

The following definitions apply to this Code:

- 'Harm' and 'risk of harm' are used in this document as overarching terms that cover neglect and various forms of abuse towards children
- 'Parents' includes guardians and persons who have parental responsibilities for the child under a decision or order of court

- 'Staff' refers to the approved provider, nominated supervisor, paid employees, students on practicum placements, volunteers (but not occasional parent helpers), and third parties who carry out work on our behalf relevant to this Code
- 'Visitor' is any individual entering the service who is not an enrolled child or a staff member at the service
- 'Unacceptable behaviour' includes bullying, harassment, sexual harassment, sex-based harassment, unlawful discrimination, cyber harassment/bullying, vilification, victimisation, violence, aggression

Families and Visitor Code of Conduct

The Families and Visitors Code of Conduct is at **Appendix A**.

It provides examples of the behaviour we want families to engage in, and the behaviour that is unacceptable

Engaging in unacceptable behaviour is a breach of our Code and may result in action being taken against the person who is responsible

Code Communication, Training and Monitoring

- This Code of Conduct and related documents can be found in this service policy handbook
- Families have access to this Families Code of Conduct and related documents at the time of their child's enrolment
- The approved provider and nominated supervisor provide information, resources and support regarding the Families Code of Conduct
- The approved provider and nominated supervisor monitor behaviour at our service. Breaches of this Code are taken seriously
- Families are notified in line with our obligations under the *National Regulations* when changes are made to our policies and procedures

Implementation

- All adults and visitors to abide by the Families and Visitor Code of Conduct. (See Appendix 1).
- Speech, tone, and manner used while in the Centre and surrounds will be conducive to an educational and nurturing environment. Specifically, adults will refrain from yelling and shouting at others while in the Centre.
- Team members, parents, children and visitors will refrain from the use of obscenities, including but not limited to, abusive language or gestures while in the Centre or its surrounds.
- Adults will refrain from using any threatening or intimidating behaviour with other persons on the premises.
- Adults will refrain from any physical actions towards others that can be construed as aggressive and /or violent
- This Centre is a Smoke Free Zone. Cigarettes or similar will not be smoked while on Centre premises, or surrounds.
- When wearing service uniform (service logo and name is displayed) outside the service, please do not drink alcohol or smoke cigarettes. The reason for this policy is you are a role model in the children services industry and also represent the organisation when wearing the service uniform. Being a children service organisation we need to ensure our organisation presents positive health and wellbeing images while in uniform.
- All users of the Centre and surrounds will abide with Centre policies.
- Those who are unable or unwilling to abide by this policy will be asked to leave forthwith. Should the situation require, the police will be called.
- Should any parent be escorted from the premises, management is to be notified as soon as practical following the incident. An incident form will need to be completed and forwarded to management.

- Should an authorised adult arrive at the centre intoxicated or otherwise unfit to drive a vehicle, the senior team members (For example: Centre Director, Assistant Centre Director, Certified Supervisor, etc) on premises should ask if there is an alternate adult who can be called or if any assistance is required. Team members should not release children to authorised adults in such a state without first ringing the parents or guardians and informing them of the situation.
- Please note that if the parent or the legal guardian of a child insists on taking the child, team members cannot prevent this. In such cases team members are expected to call the police immediately and explain the situation.
- Inappropriate adult behaviour may result in termination of enrolment of child/ren from service or non-attendance by adult/s.

APPENDIX A

Families and Visitors Code of Conduct

Guiding principles

Families, we:

- Support the service's philosophy and values, and are committed to maintaining a positive, respectful, inclusive and child safe culture in which diversity is celebrated
- Uphold the rights and dignity of children, other families and staff members
- Prioritise and promote the safety, health and wellbeing of ourselves and others at the service
- Engage in open, accessible, respectful and effective two-way communication with others
- Have zero tolerance for bullying, harassment, sexual and sex-based harassment, unlawful discrimination, victimisation, vilification, violence or aggression
- Raise concerns appropriately, respectfully and productively

Positive relationships

DO:

- Speak and behave respectfully at all times
- Use polite spoken and written language at all times, including during disagreements or complaints
- Collaborate with educators in making decisions on children's care and education. Understand that, at times, compromises may be necessary
- Respect the privacy and confidentiality of families, children, visitors and staff
- Treat all members of our community equally, respectfully and fairly, regardless of race, gender, sexuality, disability, culture, religion, age or family structure
- Support us in providing an inclusive and culturally safe environment that celebrates the diverse backgrounds, abilities, circumstances, views, beliefs and values of everyone at our service
- Raise any concerns or complaints with your child's educators or through our service's 7 - Complaints and Grievance Policy

DON'T

- Engage in any unacceptable behaviour (either in the physical or online environment), such as using offensive, aggressive, threatening, intimidatory language or abusive language, or bullying, harassment, sexual harassment, unlawful discrimination, physical aggression
- Make negative comments or gossip with other members of our service's community (including on social media)

Child safe environment

DO:

- Support us in creating a child safe environment that upholds the rights of all children to be safe, heard, and respected
- Always act respectfully and appropriately around children and do not have physical contact with any child at the service who is not your own
- Report any concerns (including suspicions or disclosures) about the safety or wellbeing of a child at our service to a staff member or the nominated supervisor

- Respect our legal and ethical obligations to follow child protection laws, including the mandatory reporting of harm or risk of harm to a child
- Supervise your own child while on the premises, unless they are signed in and under our care
- Supervise any other children in your care who are not enrolled in the service (e.g. siblings), and don't allow them to use service equipment or be left unaccompanied at any time
- Follow all directions from staff regarding phone or other device use while at the service

DON'T:

- Condone or engage in any activity that harms or risks harm to a child
- Use physical discipline, corporal punishment or aggressive behaviour towards any child while at the service, including your own
- Take photographs, videos or audio recordings of children, staff or other families and visitors at the service without the relevant consent (noting that, in the case of a child, you need their parent's consent)
- Share (e.g., on social media, via email or messaging apps) images, videos or any identifying information about children, staff or other families without the relevant consent

Health and safety

DO:

- Take good care of your own health and safety, and that of others
- Respect our safety rules, hygiene protocols and illness exclusion periods
- Comply with any reasonable instruction that our staff give to you about health and safety

Alcohol, drugs, vapes and tobacco free environment

DO:

- Respect our legal and ethical obligations to have an alcohol, drug, vape and tobacco free environment for children

DON'T:

- Attend the service if you are affected by alcohol or illicit drugs (including misused prescription medication)
- Bring alcohol, illicit drugs, or tobacco or vaping products onto the premises
- Smoke or vape at our service at any time

Cooperate with policies and procedures

DO:

- Follow all relevant service policies and procedures, including for child safety, enrolment, attendance, delivery and collection of your child, fees, behaviour guidance, photography, medical conditions, infectious diseases, immunisation, medication, authorisations, clothing and sun protection, and food and nutrition
- Notify us as soon as possible of any change to your child or family's circumstances, such as contact details, authorised persons, court orders, medical conditions, immunisation status, health and wellbeing etc

Gifts, benefits and hospitality:

DON'T:

- Offer staff gifts, benefits or hospitality that exceed \$500
- Offer staff cash or cash vouchers (e.g., Visa gift cards) of any value

- Offer any gifts that may influence, or give the impression to influence, any decision by a staff member

Breaches of this Code of Conduct

Report breaches or suspected breaches of our Families and Visitors Code of Conduct to nominated supervisor and/or approved provider either in person or by telephone.

Staff must follow our Child Protection Policy and Procedures if the breach relates to harm or risk of harm to a child.

Breaches and suspected breaches will be taken seriously and dealt with quickly, fairly, transparently and according to our relevant policies and procedures, including our 7 - Complaints and Grievance Policy

Families or visitors who breach this Code may have action taken against them, such as formal warnings, denied access to our premises, suspension/termination of enrolment.

We may consider the following factors when deciding what action to take: the seriousness of the breach, the likelihood of the breach happening again, whether the person has committed the breach before, the risk of the breach to others health and safety, and other relevant circumstances.

Depending on the breach, we may need to refer the matter to the police and/or another relevant authority.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Policy Reviewed: 22/8/2025

References:

NQF

QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.3	<i>The services builds relationships and engages with their local community</i>
QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	157	Access For Parents
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Other References:

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

9 - BEHAVIOUR GUIDANCE POLICY

Position Statement

Young children are learning to become active members of society. As a natural part of their learning; children will explore the limits placed upon them. Guiding children's behaviour is an essential part of early childhood education. Consistent approaches in relation to management strategies are paramount to the overall success of a positive behaviour guidance policy.

Children face many challenges throughout their lives. Learning acceptable behaviours and being able to regulate their own behaviours in different social and emotional environments, or when interacting with their peers or adults, are two of those challenges.

Objective

To provide guidelines when dealing with a behavioural issue, so as to foster a consistent approach to managing children's behaviour. This policy purpose is to:

- Encourage acceptable forms of behaviour by using strategies that build children's confidence and self-esteem;
- Provide children with support, guidance and opportunities to manage their own behaviours
- Promote collaborative approaches to behaviour guidance between the service's stakeholders and/or external agencies;
- Establishes what is acceptable behaviours and unacceptable behaviour in our service.
- We are committed to the Behaviour Guidance Policy because it:
 - reflects the values, attitudes and current recommended strategies that promote positive play behaviours and patterns;
 - respects the importance of interactions and relationships between children, families, educators and staff;
 - understands why children behave in certain ways in specific circumstances;
 - promotes realistic play and behaviour limits that guide children's safety and security rather than curb their play experiences, curiosity or creativity;
 - defines clear and transparent caregiving strategies that communicate how behaviour guidance is implemented by the service;
 - informs the service's stakeholders about the procedures involved in behaviour guidance management plans; and
 - explains the service's commitment to professional development and utilisation of external agencies.
- The service recognises and understands that a child's behaviour may be affected by their:
 - age and development;
 - general health and wellbeing;
 - relationships with their family;
 - play and learning environments, which includes the physical indoor/outdoor settings, the weather, the time of year, the time of day;
 - educator's caregiving strategies and practices, which includes how those strategies are implemented;
 - relationship with other children and stakeholders, such as students, volunteers and visitors; and
 - external factors, such as family, home life, school or peer group experiences, or media coverage of traumatic events.
- Families and educators display respect and empathy towards children when they label behaviour and not the individual child. This means that behaviours are managed, not children. Staff/carers, other children and families should refrain from labelling a child's inappropriate or negative behaviour as 'naughty' or 'bad'. Similarly, traditional labels such as 'good boy' or 'good girl' identify the individual but not the positive behaviour. For example, when a child completes

a task directed by an adult, such as washing their hands before a meal, staff/carers should identify the behaviour that reinforces the achievement and not label the worth of the individual. Instead of 'good boy/girl', the staff/carer can respond with "thank you (child's name) for washing your hands with soap before lunch" or "thank you for remembering to clean your hands before you eat".

- While educators are aware and respect individual children's and families' backgrounds and beliefs, it may be necessary to balance the individual needs of stakeholders with educator's knowledge of developmentally appropriate practices and current best practice recommendations from recognised authorities.
- In accordance with the *Occupational Health and Safety Act*¹ our service has a duty of care to our employees to ensure that the working environment supports emotional and mental wellbeing. We have a duty of care to ensure that all our employees' mental and emotional wellbeing is considered, as well as the child's need for positive behaviour guidance strategies.

Related Policies:

- Family and Visitor Code of Conduct Policy – 8
- Bullying Policy – 68
- Enrolment Policy – 1
- Ethical Conduct and Code of Ethics Policy – 70
- Inclusion Policy – 4
- Open Door Policy – 76
- Parent Involvement Policy – 6
- Child Safety, Protection & Wellbeing Policy – 12
- Incident, Injury and Trauma Policy – 19
- Occupational Health & Safety Policy – 10
- Privacy and Confidentiality Policy – 16
- Staff Injury Management Policy and Procedure – 80
- Interaction with Children Policy – 72
- Exclusion Policy – 43
- Health and Wellbeing Policy – 92
- Child Safe Environment Policy – 88
- Education, Curriculum and Learning Policy – 89
- Child Self Reliance and Self Esteem In Children Policy – 69
- Transitions & Routines Policy – 45
- Governance of Service, Management and Educator Policy – 78
- Team Member Self Protective Behaviour Policy – 56
- Team Member Grievances - 54

Definitions

Behavioural guidance: A means of assisting children to self-manage their behaviour. It differs from traditional "behaviour management" or 'discipline' which generally implies that an adult is 'managing' children's behaviour or using punishment to control children. Behaviour guidance applies to all forms of behaviour, not just behaviours labelled as 'negative'.

Behaviour Guidance Management Plan: The behaviour guidance plan may be called or prepared using templates: Individual Learning Plan, Behavioural Guidance Management Plan or an Individual Support & Safety Plan. A plan that documents strategies to assist an educator in guiding a child with behavioural difficulties to self-manage his/her behaviour. The plan is developed in consultation with parents and families members, educators, the nominated supervisor, specialists and other professional support agencies, as applicable.

¹ There are legislative Acts and regulations for each state and territory that address the issue of Occupational Health and Safety.

Inclusion Support Facilitator (ISF): Employed by inclusion support agencies and funded by the Commonwealth Government to provide advice on inclusive practices in childcare services. ISFs also help services to access a range of practical support.

Normal Challenging Behaviour: Behaviour that disrupts other and causes disputes between children, but which is part of 'normal' social development.

Unacceptable Challenging Behaviour: Behaviour that:

- Disrupts other or causes disputes between children, but which is part of normal social development;
- Interferes on the rights of others;
- Causes harm or risk to the child, other children, adults or living things;
- Is destructive to the environment and/or equipment;
- Inhibits children's learning and relationships with others;
- Is 'inappropriate' relative to the child's developmental age and background.

(Source: DEECD National Quality Framework – Understanding Children's Behaviour, Fact Sheet, June 2014)

Examples of unacceptable challenging behaviours (Not limited to);

- Yelling and screaming (child is inconsolable by adult);
- Punching, hitting, slapping, pinching, pushing, kicking, etc (without provoking of behaviour);
- Excessive biting (taking into consideration 'normal' social developmental);
- Spitting
- Causing physical damage or disruption to equipment or materials; including hitting, kicking and throwing of equipment or items causing potential or actual harm or damage;
- Bullying;
- Self-harming;
- Use of inappropriate language and swearing;
- Aggressive behaviour that causes physical or emotional harm to other or threaten to; and
- Inappropriate sexual behaviours or actions (outside normal developmental age).

Discipline: The term discipline is often associated with punishment because it has previously been defined by what adults do to children to control behaviours (Stonehouse, 2004, 47). It is also used as a threat or consequence of inappropriate behaviour. The term 'behaviour guidance' is preferred because it includes all forms of behaviour and not just those behaviours labelled as 'negative'. Educators should remember that an environment which supports children to learn self-regulation and guide their own behaviour reflects the importance of a behaviour guidance policy.

Rationale

The following rationale represents a statement of reasons why the policy and procedures have been developed and are important to the service.

Important:

- The use of physical punishment² by educators/students/volunteers/visitors as a behaviour guidance strategy is not acceptable under any circumstances.
- The use of isolation, humiliation, intimidation or negative labelling by educators/students/volunteers/visitors as a behaviour guidance strategy is not acceptable under any circumstances.
- In meeting the service's duty of care there is a shared responsibility between the service and its stakeholders that the Behaviour Guidance Policy and procedures are adhered to.

² For the purpose of this policy, physical punishment includes smacking, hitting, slapping, kicking, pinching, pulling, pushing, shoving or the inappropriate manhandling of a child by an adult.

Implementation

General Strategies and Practices

- The use of physical punishment, isolation, humiliation, intimidation or negative labelling by team members as a behaviour guidance strategy is NOT acceptable under any circumstances.
- Plan to minimise inappropriate behaviour by ensuring there is a well-planned environment which includes the children's input for activities, actively participate in the children's play, role model appropriate behaviour, and ensure all adults practice consistent behaviour management techniques.
- Set and maintain appropriate limits for behaviour and make these expectations explicit within the room. Strategies include group discussions about behaviour and flash cards depicting the desired behaviour.
- When children's behaviour is inappropriate, apply an intervention strategy from the least intrusive to most intrusive continuum.
- Acknowledge children's feelings and behaviours and talk to them about the issues and the consequences of their actions. Strategies to use include directional feedback to the child about both positive and negative behaviour.
- It may be useful to help children relax or provide physical comfort to children who are experiencing stress in their lives.
- Provide children with choices and encourage decision making. This will encourage positive self-esteem. Strategies to use include selective choice wherever possible that allow the child to exercise autonomous decisions while keeping the child's choices within the boundaries and expectations set within the room.
- Educators to use the resources from Kimochi program lesson plans and service wellbeing tool bank.
- Let children share the decision making process by actively involving them in group discussions that affect them, use group meetings to discuss issues and make decisions, and encourage children to make their own decisions.
- Have realistic expectations of children's behaviour. If adult expectations are unreasonable, children are bound to be seen as misbehaving.
- Consequences must also be made explicit within the room in the same way as rules and expectations are. In this way children should always know that a particular behaviour would result in a known consequence.
- Consequences, target behaviour and room rules & expectations need to be taught at group time and practised in a group environment in the hope that children will be able to generalise the skill into a real social setting and also have a more concrete understanding of rules and expectations.
- Team members are expected to remain calm when dealing with children's inappropriate behaviour. Children respond to adult reactions to behaviour. Better results will be forthcoming if the adult maintains a calm authoritative manner. Using a calm tone and manner assists in keeping a potentially difficult behaviour from escalating. Team members should recognise that their own values and stresses will affect how they respond to children's behaviour.
- Team members are expected to respond positively to any anti-social behaviour displayed by children. Each situation is seen as a 'teachable moment' where children can learn, develop empathy, understand the consequences of their behaviour and actions and build their own strategies for responding to challenging situations.
- Role model appropriate behaviour. Eg. If there is a problem with pack away time, model that you as the adult pack up your share of things too. Children will often follow a good role model.
- Time away as a strategy. This is a more positive alternative to 'time out'. Depending on the age of child and the behavioural incident circumstances the child is re-directed to an experience or the child is given the opportunity to find a space away from the group and consider how their behaviour has impacted on others. This strategy allows the child to calm down before discussion can be held with child regarding their behaviour and to re-engage with their peers.
- If a child displays behaviour which is likely to result in serious self-harm or injury to others a crisis response may be required. The response should involve the minimum amount of intrusion and be applied only for as long as is necessary to manage the risk.
- Behaviour Guidance and Management at our centre is based on positive reinforcement techniques. Any form of verbal, emotional, or physical punishment will not be tolerated.

- Educators will document a Behaviour Management Plan, Individual Learning Plan or a Individual Support & Safety Plan, where required.
- Please refer to the 19 - Incident, Injury and Trauma Policy and 12 – Child Safety and Protection Policy for further guidance on reviewing, managing and reporting child incidents.
- Please also refer to the extract below from Child Wise “*Understanding “Normal” Versus “Problem Sexual Behaviour” (0-4 years old children)*” for identifying ‘normal’ and ‘Problem Sexual Behaviour’.
- Should you require any further information on our Behaviour Management strategies please consult with your Centre Director.

Procedures for the guidance of normal challenging behaviours in young children

For all children, educators and parents should:

- Incident, Injury and Trauma forms will be completed documenting the normal challenging behaviour for all children involved in the incident. Please refer to the 19 - Incident, Injury and Trauma Policy. The form will be required to be signed by children parents involved in incident.
- Discuss the behaviour with parent/s. Providing parents resources and information on the behaviour.
- Use reflective practices to assess children’s behaviours.
- Understand the needs of individual children and those in a group and acknowledge many factors could influence behaviour.
- Based their expectations on the child’s individual level of development. Respect the individual differences in children.
- Respond to children proactively, rather than reactively.
- Assist children to control their behaviours and eliminate situations and physical arrangements that may encourage inappropriate behaviour.
- Reinforce children’s positive behaviours and provide positive role models. Encourage children to resolve potential conflicts for themselves, but step in when needed.
- Acknowledge a child’s good intentions, even if they were carried out inappropriately.
- Acknowledge that it is the behaviour that is inappropriate and not the child.
- Allow choices in decisions making and be prepared to accept the child’s decision if appropriate.
- Be consistent in the guidance of all children.
- Work cooperatively with parents on issues relating to the guidance of their child’s behaviour, keeping them informed of the techniques and methods used.
- Respect the confidentiality of both parents and child when dealing with issues of behaviour guidance concerning an individual child.
- Endeavour to ensure that children are not put in a position where they feel frightened, insecure or isolated.
- Provide a set of basic developmentally appropriate behaviour rules which:
 - Emphasis positive actions
 - Give children reasons for limits
 - Are implemented in a consistent manner. Are reinforced regularly.
- Educators should seek assistance, or support, whenever necessary.

Please refer to the 19 - Incident, Injury and Trauma Policy and 12 – Child Safety and Protection Policy for further guidance on reviewing, managing and reporting child incidents.

Procedure for unacceptable challenging behaviour:

Where a child has performed an unacceptable challenging behaviour the following procedure will be followed:

- The educator to perform strategies and practices per the Behavioural Guidance Policy to maintain the safety of the child, other children and educators (see above).
- Incident, Injury and Trauma forms will be completed documenting the unacceptable challenging behaviour for all children involved in the incident. Please refer to the 19 - Incident, Injury and Trauma Policy. The form will be required to be signed by children parents involved in incident.
- Discuss child’s behaviour with the Centre Director.

- Assessment of the behaviour and incident/s will be made by the Educator and the Centre Director/Responsible Person in Charge of Service. The behaviour of the child is deemed to be unacceptable if:
 - Interferes on the rights of others;
 - Causes harm or risk to the child, other children, adults or living things;
 - Is destructive to the environment and/or equipment;
 - Inhibits children's learning and relationships with others;
 - Is 'inappropriate' relative to the child's developmental age and background.

(See examples of unacceptable challenging behaviours in definition section of policy)

- The service has a duty of care to all children and educators to provide a safe environment. If, Centre Director and Approved Provider deems that the safety of children and/or educators to be at risk as result of the behaviour of a child, the service will invoke the Exclusion Policy.
- Invoking the Exclusion Policy will require a parent to collect the child from the service. If the service is unable to be contacted the parent, the parents authorised nominee/s will be contacted to collect the child from the service.
- Depending on the nature and severity of the incident and the child behaviour the service may invoke the Exclusion Policy for an extended period until a meeting can be held with the parent/s and the preparation and agreement of a Behavioural Guidance Management Plan, Individual Learning Plan or a Individual Support & Safety Plan.
- If the Centre Director and Approved Provider deems the incident to be serious nature and the safety of the child, other children and/or educators is at risk; the service will invoke the Exclusion Policy and terminate the enrolment of the child.
- Call made to parent advising of unacceptable behavioural incident. This is to be made by Centre Director or Responsible Person in Charge of Service.
- Discuss with the parent on the telephone or pick-up of child the unacceptable behaviours shown by child.
- Provide parent with any resource or guides to provide information regarding behaviour.
- Arrange a meeting time with parent to discuss child's behavioural, proactively seeking information to manage behaviour, possible strategies to manage behaviour and provide resources or information.

If the child's unacceptable behaviour continues please refer below "Procedure for Guidance on Ongoing Unacceptable Behaviour".

Procedure for guidance of ONGOING unacceptable behaviour

When dealing with unacceptable behaviour, the educators should:

1. Assess and Consult

Discuss child's behaviour with the Centre Director

- Prior to discussions with Centre Director collate child observations
- Access the Individual Learning Plan, Individual Support & Safety Plan or Behaviour Management Plan to document child behaviour guidance management plan.

Discuss with the parent (A parent meeting may be called with Centre Director and Parent/s):

- The unacceptable behaviour shown by the child.
- Strategies that could be implemented.
- Consulting with other professional, specialists and agencies, as appropriate.
- Providing information and guides.
- Parents / guardians are required to seek help and advice from support agencies, specialised practitioners, doctors on strategies to implement to support their child. This support must be sought within a reasonable timeframe.
- Resources available such as an Inclusion Support Facilitator (ISF), Kindergarten Inclusion Support(KIS), Flexible Support Packages (FSP), Preschool Field Officer (PSFO) and a referral for Specialist Assessment (eg Inter-reach: see <https://www.intereach.com.au/>).

2. Develop a Behaviour Guidance Management Plan / Individual Learning Plan / Individual Support & Safety Plan which is:

- Based on the consensus reached with the parent on the strategies used.

- Clear and easily to follow by all educators, parents caring and educating the child.
- Discuss in detail with other educators. Review as required.
- Educators will keep child observations in Individual Learning Plan, Running Record schedule or an Observation Record, Incident Form, etc.
- Behavioural guidance management plan, Individual Support & Safety Plan or Individual Learning Plan will be provided to parent/guardian. (It is recommended for parent/guardian to sign the form).

The Behavioural Guidance Management Plan and Individual Support & Safety Plan needs to:

Document the Child Behaviours and Strategies:

- Brief and concise detail of the service's strategy being implemented by educators
- Brief and concise detail of the service's strategy being implemented by parents at home.
- How the service and its stakeholders observe, plan, implement, evaluate and document strategies that reflect when a child is non-compliant to the established limits of play and interactions. For example, swearing; hitting, smacking, or kicking other children or adults; or potentially causing harm to themselves.
- The behavioural guidance management plans:
 - are based on evidence that the displayed behaviour is inappropriate;
 - can be observed and documented over a period of time that suggests a pattern is emerging;
 - include inappropriate behaviours that occur consistently;
 - include inappropriate behaviours that occur with consistent triggers;
 - identify that the behaviour could possibly harm another child or adult;
 - define the context within which the behaviour occurs;
 - reflect a collaborative approach with the child's family; and
 - The services may define specific behaviours that are not acceptable.

Other information the plan needs to cover:

- When parents are obtaining advice from other professionals or specialists. Including undertaking a behavioural assessment of the child; including observations of the child's behaviours.
- Parent to seek advice from professionals providing strategies the service can use to assist the child to regulate their own behaviours and prevent unacceptable behaviours.
- Where appropriate;
 - Agreeing on behavioural intervention program or obtaining specialist advice (i.e. ISP, Preschool Field Officer).
 - Agreeing on referral to a Parent Support Program to obtain assistance or other support service for a family.
 - Agreeing on seeking to obtain government funding for additional staff assistant, where available.
- Discussing and parents signing necessary forms for service to access support agencies to obtain advice and support for child.
- Establishing timelines and responsibilities for the above to be completed by parents and service.
- Agreeing on behavioural guidance strategies
- Where required, agreeing on changes child routine at the service; this may include (but not limited to):
 - A parent remaining with the child for a pre-determined period of time;
 - Changing drop-off and pick-up times of child;
 - Reducing the hours attending the service each day; and
 - Reducing the days attending the service each week.
- Agreeing the need for ongoing consultation with parents: Set date for next meeting.
- Parents agreeing to behavioural guidance management plan.

3. Refer the matter to the Centre Director / Responsible Person in Charge of Service:

- The educator is concerned that the child's behaviour may put themselves, other children, staff or other at risk and the consultation with the parent and other and the behavioural guidance plan / individual learning plan has not resolved the unacceptable challenging behaviour.

4. The Centre Director/Responsible Person will become involved when:

- Requested by Educators.
- A parent lodges a complaint concerning another child's behaviour that is threatening the safety of other children. Complaints must follow the Complaints and Grievance Policy and Procedure.
- The Centre Director/Responsible Person has concerns about the unacceptable behaviour of a child and the effect of this on other children, staff, environment, etc.
- There is an occupational health and safety issue involved for staff or others at the service.

The service will attempt to resolve the issue as soon as possible if the child's ongoing unacceptable behaviour is putting themselves and/or other children and adults in danger or harm.

Please refer to the 19 - Incident, Injury and Trauma Policy and 12 – Child Safety and Protection Policy for further guidance on reviewing, managing and reporting child incidents.

Summary of Behaviour Guidance Approach and Levels

Support & Safety Plan for Child Displaying the following Support Needs/ Behaviours / Concerns:
▪ Inability to regulate and make the bigger emotions/feeling smaller. (E.g. mad, anger, frustration, disappointed) ▪ Displaying Challenging Behaviours: yelling, punching, pinching, pushing, kicking, spitting ▪ Causes harm or risk to the child, other children and adults. ▪ Causes physical damage to equipment / furniture / walls. ▪ Unpredictable, disproportionate and unsafe behaviours that are inappropriate relative to the children developmental age. ▪ Educators not able predict the moment before a serious incident occurs. Child behaviour escalates from normal play to aggression quickly.
Signals to watch for:
▪ Volume of voice is louder. ▪ Screams. ▪ Grunting. Grunting gets louder. ▪ Picks up objects. Throws. ▪ Up ends furniture. ▪ Throws or displaces toys off shelves and tables. ▪ Creates a physical zone around child through the above and seeks an object to pickup to throw to distance people. ▪ If someone enters child's physical space the above behaviours are more aggressive.
Response strategies to guide the child's needs and behaviour when evident in moment:
▪ Educator identifies the behaviours of the child that could cause harm or safety concern. ▪ Analysing the safety risk and potential injury to children or educators. (Assess Level 1, 2 or 3) ▪ The immediate priority is to ensure the safety of all children and adults. Remove any children from harm's way (if assessed as needed) and create a safe environment.
<u>Educator and Centre Director/Responsible Person In Charge of Service to Assess</u> Educators will use available tools and strategies to mitigate/reduce chance of occurrence or progression of behaviours that are level at 2 or 3. (Also make level 1 smaller.) Tools to consider: - Review Individual learning plan or support plan for specific strategies. - H.A.L.T

- Regular connection
- Re-direction to another area of interest or passive/calm space.
- Emotional coaching (educator should know general script to deliver emotional coaching. I.e. reference to kimochi and wellbeing tools.
- Offering your P.E.A.C.E
- Deliver Kimochi tools/lesson that has been delivered and known by child. Eg Calm Down Breath, Use a Talking Voice, Disappointed Shrug, etc.

Implementation of wellbeing tools and strategies requires consistent and persistent implementation to achieve positive outcomes. This is best achieved through educators using the above tools and strategies to create connection and trusting relationships in teachable moments.

Behaviour is Level 1, 2 or 3:

Level 1: Child is able to respond to instructions and guidance.

- An educator is nominated to support the child and diffuse the situation using strategies.
- Educator maintains at a minimum arm length safe distance to assess the situation.
- Educator approaches the child calmly, acknowledging the behaviour and explaining why it's not acceptable. Use clear and simple language to communicate expectations. (Eg. "What you are doing is unsafe and you please keep the shovel low to the ground")
- Model Calmness: Stay composed and model appropriate behaviour for the child. Your calm demeanour can help de-escalate the situation and reassure other children.
- Educator assists children to negotiate play. I.e sharing, turn taking, return object, etc.
- Educators agree on strategies and timing when a different educator needs to step and swap over to support child.
- Educator provides support and guidance. (eg. I am here if you want to talk? Position at a distance – with a watchful eye.
- Educator team agree on strategies to inter-change the nominated person. I.e tapping out and supporting each other. When tapping out the initial nominate continues to support the children and educator team in a different capacity.

Level 2: Child is unable to regulate, is safe for other children and educator (no immediate risk):

- Educator maintains at a minimum arm length safe distance.
- Educator approaches the child calmly, acknowledging the behaviour and explaining why it's not acceptable. Use clear and simple language to communicate expectations.
- Model Calmness: Stay composed and model appropriate behaviour for the child. Your calm demeanour can help de-escalate the situation and reassure other children.
- Other children involved are re-directed to another activity or area to create a safe zone. OR
- Redirect Attention: Offer the child an alternative activity or provide them with a safe outlet for their emotions, such as a designated calming corner or sensory materials or a space away from other children to calm down.
- Educator informs Centre Director or Responsible Person to notify of situation and support in room.
- Educator direct active supervision of child. (maintain safety of child and other people. Be available when child is able to regulate and communicate.
- Use emotional coaching tools to support child.
- Assess if a call needs to be made to parent to advise of incident, obtain any background information and also pre-emptive communication that child may need to be collected if unsafe behaviours continue.
- Document and Communicate: Record the incident, including what happened, how it was addressed, and any follow-up actions taken. Communicate with parents/guardians about the incident and collaborate on strategies for addressing the behaviour.

Level 3: Child is unable to regulate, and behaviours are unsafe to other children and educators (assessed risk = potential immediate harm to self and/or children and educator):

- Educator maintains at a minimum arm length safe distance.
- Educator approaches the child calmly, acknowledging the behaviour and explaining why it's not acceptable. Use clear and simple language to communicate expectations.
- Model Calmness: Stay composed and model appropriate behaviour for the child. Your calm demeanour can help de-escalate the situation and reassure other children.

- Educators decide what is the safe zone. Indoor or outdoor environment. All other children are directed to the safe environment area. I.e (child is outdoors, other children are moved indoors).
 - Most experienced and qualified educator maintains safe zone and supervision of the child.
 - Behaviour Guide Policy (9) is followed, in relation to informing parent or guardian of incident, needing to collect child from care (maximum of 30 minutes). If parent is not available next available authorised emergency contact will be contact.
 - Parent is informed of the nature of the incident and advise actions needing to take place on collection. I.e they are coming to collect child.
- Collection of Child at time of incident:
- Parent is met by a Centre Director or nominated educator.
 - Parent or guardian enters room.
 - Parent leaves with child within 15 minutes.

With reference to Behaviour Guidance Policy, the services has a duty of care to all children and educators to provide a safe environment. If the Centre Director, Responsible Person or Approved Provider deems that the safety is at risk, the service will invoke the Exclusion Policy.

The extension of the Exclusion Period will continue until a meeting with parent is conducted, behavioural guidance / individual support plan in place that will result in a safe environment.

Forming part of the individual support plan of the child is advice and support from external professionals (OT, GP, other specialists) and agencies.

If the service deems due to nature and severity of the incident and behaviours and is an ongoing safe risk to all children and educators then the service will evoke the Exclusion Policy and evaluate ongoing care arrangements.

Our Centre and Families working in Partnership – Open Communication and Expectations

Crucial to the success of behaviour guidance is the role family's play. Families will be provided with regular opportunities to contribute to the development and review of their child's behaviour plan, along with the service's overall strategies to promote positive outcomes for the child.

At any time parents can discuss their child/ren with classroom educational leaders for brief periods (5 minutes maximum). During these discussion educators may need to continue to supervisor and engage with children and may not be able to provide you their full attention. For an in-depth discussion, the service asks that parents make an appointment with the Centre Director and Educator.

Upon enrolment at our centre families are provided with information about our behaviour guidance policy and we will record any feedback provided by families about the behaviour guidance strategies used at home. Please refer to the service's Enrolment and Orientation Policy.

In order to maintain open lines of communication, families are encouraged to express their thoughts, expectations and feelings openly with staff/carers in relation to all aspects of their child's care and education.

Team members will also extend open lines of communication with families in regards to maintaining behaviour guidance strategies and practices.

Families should recognise that some behaviour guidance strategies or practices established in the home cannot be enforced in the service's environment.

In cases where a child's behaviour causes disruption to the smooth operation of the classroom or places children or team member's safety in jeopardy an individual behaviour plan will be developed for the child. To develop a plan that meets the needs of the child a meeting will be held with both management and the child's family to agree upon consistent strategies which will be implemented in both the home and centre environments. It is essential that families and team members work in partnership to ensure the best possible outcomes for all children.

In instances where behaviour guidance management plan does not meet the needs of the child and other children, further discussion will be held with families to seek a suitable resolution.

Should the Centre Director raise a concern with a family who refuses to accept the services observations, for example, their child does unacceptable behaviours, and the family refuses to engage in a Behaviour Guidance Policy, Strategies and Behavioural Management Plan / Individual Learning Plan in conjunction with the service educators and centre director, then the safety of other children and staff is deemed to be at risk. This also extends, if a parent does not make themselves available to meet with educators or centre director to discuss their child behaviour or does not comply with requests of the service in relation to the Behavioural Guidance Management Plan or Individual Learning Plan. In such situations, the family is considered non-compliant with service policies and therefore the Non-compliance Policy and Exclusion Policy will be invoked. Please refer to our Exclusion policy and the Non-Compliance Policy.

EXTRACT

Understanding “Normal” Versus “ Problem Sexual Behaviour” (0-4 years old children)

(Source: Child Wise)

Website References: www.childwise.net

Reference 1: <http://www.childwise.net/page/39/fact-sheets>

Reference 2:

http://childwise.blob.core.windows.net/assets/uploads/files/Fact%20Sheets/Sexual_behaviour_0to4yearsold.pdf

Reference 3: <http://www.nctsn.org/products/sexual-development-and-behavior-children-information-parents-and-caregivers-2009>

Extract from Fact Sheet: Sexual Behaviour of Children Aged 0 to 4 years Old

To know what sexual behaviour is problematic, we need to first understand what is normal for each age and stage of child development.

Considerations

- *Is the observed sexual behaviour age appropriate or concerning?*
- *What is the context of the sexual play?*
- *Age difference and relationship between children/young people*
- *Do not over or under react **Gil (1993)***

Age appropriate sexual behaviours

0 – 4 years; Pre-school

- *Touching/rubbing his/her own genitals*
- *Likes to be rude*
- *Showing others his/her genitals*
- *Playing doctors and nurses*
- *Playing house; Mummies and Daddies*
- *Touching or looking at private parts of familiar children or adults*
- *Using slang words/dirty language for bathroom and sexual functions*

Very Concerning Sexual Behaviours

- *Persistently touches or rubs self to the exclusion of normal childhood activities; hurts own genitals by rubbing or touching*
- *Simulating sex with other children with or without clothes on*
- *Oral sex*
- *Sexual play between children involving forceful anal or vaginal penetration with objects.*

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/03/2026

Reference:

NQS

QA5	5.1.1	<i>Each child's health and physical activity is supported and promoted.</i>
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	2.1.1	Effective illness and injury management and hygiene practices are promoted and implemented.
	5.2.1	Children are supported to collaborate, learn from and help each other.
	5.2.2	Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.
	5.1.2	The dignity and the rights of every child are maintained.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	Interactions with children
	156	Relationships in groups

EYLF

LO1	Children feel safe, secure, and supported.
	Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.
	Children develop knowledgeable and confident self identities.
	Children learn to interact in relation to others with care, empathy and respect.
LO2	Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.
	Children respond to diversity with respect.
	Children become aware of fairness.
	Children become socially responsible and show respect for the environment.

Other References:

Handbook on Child Care Licensing
QIAS Principle 1.2

Legislation:

Anti-discrimination Act
Occupational Health and Safety Act

Sources and Further Reading:

* **Sample Behaviour Guidance Policy.** Source: ncac.acecqa.gov.au/educator-resources/policy.../behaviour_guidance.doc

* **Guiding children's behaviour in child care, NCAC Factsheet for Families**

* **Rodgers, Dr William A (2000) The Art of Balancing: behaviour management for parents**

* **Tebyani, Vida – NCAC (2009) Guiding children's behaviour in child care**

* **Cole, Debbie – Behaviour Management training notes, May 2010**

* **Biddulph, S (1989) The Secret of Happy children Bay Books, Sydney.**

* **National Assoc for the Prevention of Child Abuse and Neglect (NAPCAN)**
<http://www.napcan.org.au/helpfulhints.htm> 13 October 2008

10 - OCCUPATIONAL HEALTH, SAFETY & WELLBEING POLICY

Position Statement

This centre has a duty of care to provide all persons (including team members, families, children and any other individual on centre premises) with a safe and healthy environment. This centre is committed to fostering and incorporating Occupational Health and Safety (OHS&W) / Risk Management initiatives that provide and maintain a safe work environment.

Objective

To significantly reduce or eliminate accidents, injuries and work-related illnesses occurring within this centre.

To increase the profile and importance of Occupational Health and Safety and safe work practices through raising the awareness and commitment level of all employees.

To develop systems that allows employees and work groups to take responsibility for Occupational Health and Safety / Risk Management within the workplace.

To encourage and implement Occupational Health and Safety systems and safe work practices that foster organisational ownership and independence.

To establish realistic benchmarks and agreed objectives against which future performances can be evaluated.

In summary, our aim is to achieve full involvement of all personnel to ensure the safe and efficient provision of a quality service.

Organisational Responsibilities for OHS&W

To achieve these objectives through a shared commitment a comprehensive set of OHS&W policies and procedures have been developed. These are committed to all members of the organisation through effective induction and ongoing training programs. The Health and Safety program operates in a consultative basis, and in accordance with legislation, to ensure that all aspects of health and safety and any areas of concern are dealt with in a cooperative manner.

Occupational Health, Safety & Wellbeing Committee

The role of our OHS&W committee is to advise on matters relating to health and safety, draw attention to any legal obligations relating to health and safety and make recommendations designed to protect the health and safety of team members, visitors and other persons entering our premises.

Health and Safety Committee representation is governed by the "Occupational Health and safety Committee Policy." The appointed Health and Safety are to be given every assistance, including administrative support, in order that they may perform the duties as described in the state legislation.

All requests for information and/or improvement should, where practicable, be in writing and directed to the appropriate manager, having due regard to the relevant clauses within legislation and this manual.

Occupational Health, Safety & Wellbeing Officer

Our OHS&W Officer's role includes the coordination of the centre's general safety requirements, providing advice to Managers, Coordinators, Team Leaders and Health & Safety Committees advising and assisting the elected representatives as required.

Managers, Coordinators and Team Leaders will;

- Be accountable for ensuring that the activities conducted within their area of responsibility comply with centre/company policy on Health and Safety.
- In association with appropriate team members, formulate and disseminate health and safety rules and safe working procedures.
- Promote and monitor safe working practices for team members within their centre/area of responsibility.
- Familiarise themselves with all safety procedures relevant to their groups activities.
- Team members pregnant – will receive a team member pregnancy and parental information pack.
- Ensure that team members are adequately trained, equipped and supervised.
- Carry out regular safety audits/inspections and risk assessments for areas under their control. Where hazards or potential risks are brought to the attention of the centre Director or Supervisor, immediate correction action should be taken.
- Report and investigate all accidents and incidents, including near misses, for areas under their control.
- Encourage team member participation in all health and safety matters

Team Members will;

- Follow safe working practices, rules and regulations, including the wearing of appropriate protective clothing, when required.
- Encourage other team members to observe safe working procedures and to refrain from activities which could adversely affect our actions in health and safety matters.
- Bring to the attention of the OHS&W Team Leader any issue which could be a potential hazard or risk.

Health and Safety representatives represent all team members within their designated centre/work group and attend OHS&W committee meetings as required. Regular discussions with all team members regarding OHS&W concerns and inspections of work spaces should be undertaken (team member meetings and team member inductions are crucial times when OHS&W issues should be discussed).

All visitors and other persons are required to comply with all instructions given in order to promote their health and safety whilst on centre/company premises.

Hazard Identification and Control

The effectiveness of the Accident Prevention Program depends on the continuing involvement of team members and encouragement of safe working practices and hazard identification and control by team members at all levels.

- **Inspections and Job Analysis**
As part of their routine responsibilities, all Managers, Coordinators and Team leaders should carry out regular safety audits/inspections of their workplaces. This is also part of the role of the elected OHS&W representatives. The OHS&W Officer will assist in these activities as well as carrying out independent inspections on a programmed basis.
- **Material Safety Data Sheets (MSDS)**
To provide team members with safety information about the chemical products and materials that they are using, Material Safety Data Sheets should be obtained from the suppliers of these products.
These should be assessed by the relevant supervisor in conjunction with the OHS&W Officer and the OHS&W representative in accordance with the Toxic & Potentially Dangerous Products Policy. Material Safety Data Sheets should be readily available to all employees.
- **Substitution of Harmful Substances**

Where potentially harmful substances exist in the workplace, every avenue must be explored in attempting to substitute these substances with non-harmful ones.

- **Removal of Hazards at the Source**

Wherever possible, potential hazards such as noise, dust and vapours should be eliminated by job and workplace design rather than relying on the wearing of protective clothing.

- **Hygiene/Housekeeping**

Poor cleanliness/housekeeping is a significant factor in many accidents. It is the relevant supervisor's responsibility to ensure that his or her work group are operating in a safe and efficient environment. All team members have a responsibility to maintain their workplace in a clean and orderly condition.

- **Job Design and Room/Workplace Layout**

Particular attention should be given to ergonomics and health and safety matters in every job design and room/workplace layout. The additional needs of disabled team members or those undergoing rehabilitation must be addressed with care. The OHS&W Officer is available to assist in these matters.

- **Handling of Hypodermic Syringes**

Procedures have been developed to reduce the risk to team members when safely disposing of used hypodermic syringes that they may encounter. These incorporate information on the potential hazards, the supply of suitable handling equipment and training in its use, the establishment of a safe disposal system of the containers and the vaccination of potentially exposed team members against Hepatitis B.

Protective Clothing and Equipment

Personal protective equipment is the least satisfactory method of injury prevention or control of hazards. This is primarily due to a lack of compliance, the sometimes inappropriate nature of the items, poor maintenance and/or lack of availability of equipment.

It is therefore the responsibility of supervisory team members to ensure that appropriate protective clothing and equipment is available, adequately maintained and is used by team members when required. Team members must be informed of the reasons for these requirements and trained in the equipment's use.

Accident and "Near Miss" Reporting

Accident and incident reporting, investigation and analysis are vital components of our Accident Prevention Program. By learning from past experience, we can effectively have an impact on the future.

- **Reporting Procedures**

All injuries and "near miss" incidents are to be reported to the supervisor prior to injured team members leaving the centre/workplace. Incident report Forms (HEI007) and WorkCover Claim Forms are to be completed as necessary, immediately following the reporting of an accident. Unless the injury precludes them, it is the team member's responsibility to ensure that a WorkCover Claim Form is completed correctly. If in doubt, check with your centre Director, Supervisor or OHS&W Officer.

The process of submitting and substantiating claims is assisted when team members follow the correct reporting procedures.

- **Investigating and Corrective Action**

Centre Directors/Supervisors should urgently investigate each and every accident or "near miss" incident reported by completing the Follow-up Action portion of the Incident report form (HEI007). This should be done in consultation with the reporting person and, where appropriate, the OHS&W Officer and OHS&W representative. The OHS&W Committee also reviews the reports of significant accidents and incidents.

- **Analysis and Management Reporting**

The OHS&W Officer's role includes the analysis of information from the Incident Forms and routine and specific reporting of the results and recommendations to management.

Safety Training Program

A systematic Safety Training program has been developed in conjunction with the Accident Prevention program.

Team Member Induction

Each team member will undertake the centre/company induction process on commencing their position. This will include being provided with information on OHS&W which included information and details on;

- The Occupational Health & Safety Regulations and Act for your state, regarding the responsibilities of employees and employers
- NQF – National Quality Framework
- Occupational Health & Safety Policy
- The role of the Safety Officer / Representative / Committee
- The identity of the Safety Officer / Representative / Committee
- The procedures for reporting any Occupational Health & Safety concerns
- Hazard Identification
- Manual Handling
- Emergency Evacuations
- Computer Workstations and Ergonomics
- Electrical Safety, procedures for dealing with any problems
- First Aid Procedures and Infectious Diseases
- Maternity provisions
- Recommended Periods of Exclusion
- Hazardous Substances
- Playground Safety
- Sun Safety
- Fire extinguisher use and location

These points are explained in further detail in the Workplace Health and Safety booklet which is available in the centre office.

First Aid

Medical treatment and advice for injured or ill team members can be arranged through the OHS&W Officer.

- **First Aid Kits**
First Aid kits are located in each centre/workplace. The contents of each kit must meet the WorkCover approved content list, a copy of which can be found in the centre Director's Resource Manual. The contents of First Aid kits are to be checked at least monthly by the OHS&W representative and the stock levels maintained. In addition to the centre/workplace kits there should also be a kit that is available for excursions, for bus travel (ie before and after school pick-up, if applicable) and also for emergency evacuations.
- **Persons able to Administer First Aid**
Our centre/company is supportive of all team members obtaining First Aid skills.

For further information please refer to the First Aid Policy.

Fire Prevention and Evacuation Procedure

An established evacuation procedure is displayed throughout the centre/workplace for all team members and visitors. It is constantly being reviewed and updated in accordance with best practice guidelines and information from the state Fire Safety authority.

- **Fire Prevention**
Steps must be taken to eliminate various causes of fire. This is achieved by training team members in the use of fire fighting equipment including extinguishers and hose reels and the provision of adequate warning devices (ie smoke alarms). Emphasis must be paid to the maintenance of the centre/workplace environment and storage areas to reduce the possibility of a fire occurring.

- **Notification of Fire Brigade**

The Fire Brigade should be notified immediately if any fire occurs; regardless of the size. The Fire Brigade should be met on arrival by a designated person who is familiar with the area to act as a guide. Please also refer to the Emergency Evacuation Policy and the Emergency Procedure Policy.

- **Regular Evacuation Exercises**

Regular evacuation drills are to be held in each centre/workplace to familiarise team members of the evacuation procedure. Drills are to be held as specified within state regulations or if not specified at least monthly.

Team Members who are Pregnant

In an industry employing a majority of female employees, it is not unusual to have team members become pregnant during the course of their employment. Pregnancy may place additional strain on the team member's energy reserves as they continue to perform duties as per their job description. There are also specific requirements related to the health and safety of pregnant women including the risk of injury or illness. Having a clear written policy that sets down management expectations of pregnant team members is important to ensure that no confusion exists between management and team members.

Team members who become pregnant will be provided with an information pack containing information about each of the relevant diseases and disease-specific strategies to manage the risks.

Pregnant team members are encouraged to seek advice from a medical practitioner with regard to the risk of acquiring infectious diseases that may impact on pregnancy.

Bullying, Discrimination and Harassment

We will not tolerate bullying under any circumstances and will:

- Promote appropriate standards of behaviour at all times.
- Treat complaints of bullying in a sensitive, fair, timely and confidential manner.
- Implement training and awareness-raising strategies to ensure all employees know their rights and responsibilities.
- Provide an effective procedure for complaints of bullying to be addressed.
- Encourage the reporting of behaviour which breaches the bullying policy.
- Ensure protection from victimisation or reprisals for persons reporting bullying.

We are committed to ensuring that staffs work in an environment where people are treated with mutual respect. It is important for a productive and harmonious workplace that staff is aware of the impact of their behaviours on others. Bullying in the workplace is inappropriate and unacceptable behaviour.

All employees have a legal responsibility to care for their own health and safety and that of co-workers, and therefore must treat other staff with respect and courtesy and not engage in acts which constitute bullying behaviour. Staff found to have either committed or condoned such behaviour in the workplace will be subject to disciplinary action which may include the termination of employment.

Discrimination occurs when someone is treated less favourably than others because they have a particular characteristic or belong to a particular group of people, such as age, race or gender. **Harassment** involves unwelcome behaviour that intimidates, offends or humiliates a person because of a particular personal characteristic such as race, age, gender, disability, religion or sexuality. It is possible for a person to be bullied, harassed and discriminated against at the same time.

Various anti-discrimination, equal employment opportunity, workplace relations, and human rights laws make it illegal to discriminate or harass a person in the workplace. Work Health and Safety laws include protections against discriminatory conduct for workers raising health and safety concerns.

Bullying is repeated and unreasonable behaviour towards a worker or a group of workers. Our service will not tolerate bullying in any form because it may have a detrimental effect on the psychological,

emotional and/or physical wellbeing, health and safety of our educators and staff. Amendments to the Fair Work Act 2009 make it illegal to bully a person in the workplace from 1 January 2014.

Unreasonable behaviour includes actions that victimise, humiliate, intimidate or threaten and may be intentional or unintentional. It can occur directly and by using information technology such as email, texting and social media. While one incident of unreasonable behaviour is not considered to be workplace bullying, it may escalate and it will not be ignored. Examples include:

- abusive, insulting or offensive language or comments.
- unjustified criticism or complaints.
- continuously and deliberately excluding someone from workplace activities.
- withholding information that is vital for effective work performance.
- setting unreasonable timelines or constantly changing deadlines.
- setting tasks that are unreasonably below or beyond a person's skill level.
- denying access to information, supervision, consultation or resources that adversely affects a worker.
- spreading misinformation or malicious rumours.
- changing work arrangements, such as rosters and leave, to deliberately inconvenience a particular worker or workers.
- excessive scrutiny at work.

Reasonable actions taken by the Approved Provider or Nominated Supervisor to direct or control the way work is carried out is not bullying behaviour. Examples of reasonable behaviour include:

- setting reasonable performance goals, standards and deadlines.
- rostering and allocating working hours where the requirements are reasonable.
- transferring a worker for operational reasons.
- deciding not to select a worker for promotion where a reasonable process is followed and documented.
- informing a worker about unsatisfactory work performance when undertaken in accordance with any workplace policies or agreements such as performance management guidelines.
- informing a worker about inappropriate behaviour in an objective and confidential way.
- implementing organisational changes or restructuring.
- termination of employment.

The Approved Provider or Nominated Supervisor will:

- ensure all educators, staff, visitors and volunteers are aware of and comply with our Code of Conduct.
- investigate and manage incidents of workplace bullying, harassment and discrimination in accordance with our Grievance Guidelines.
- consult with educators, staff and volunteers during staff meetings when:
 - identifying the risk of workplace bullying, harassment and discrimination.
 - making decisions about procedures to monitor and address workplace bullying, harassment and discrimination.
 - making decisions about information and training on workplace bullying, harassment and discrimination.
 - proposing changes to the way work is performed or rosters managed as this may give rise to the risk of workplace bullying, harassment and discrimination.
- provide appropriate information, instruction, training or supervision to educators, staff, visitors and volunteers to minimise the risks to their health and safety from workplace bullying, harassment and discrimination.
- contact the Police if there are incidents of workplace bullying, harassment and discrimination that involve physical assault or the threat of physical assault, or a visitor engages in bullying behaviour, harassment and discrimination and refuses to leave the Service.

Educators, staff, visitors and volunteers will:

- consider whether something they do or don't do will adversely affect the health and safety of others
- comply with any reasonable instruction, policy and procedure given by the Approved Provider or Nominated Supervisor in relation to workplace bullying, harassment and discrimination.
- report all incidents of workplace bullying, harassment and discrimination using our Grievance Guidelines.
- talk to the Approved Provider or Nominated Supervisor if they have any questions about workplace bullying, harassment and discrimination.

Identifying Workplace Bullying, Harassment and Discrimination

The Approved Provider or Nominated Supervisor will minimise the risk of workplace bullying, harassment and discrimination occurring by:

1. Identifying the risk of workplace bullying, harassment and discrimination

- talking to educators, staff and volunteers (or conduct an anonymous survey) to find out if bullying is occurring or if there are unreasonable behaviours or situations likely to increase the risk of bullying, harassment and discrimination.
- monitoring patterns of absenteeism, sick leave, staff turnover, grievances, injury reports, workers compensation claims and other such records to establish any regular patterns or sudden unexplained changes.
- watching for any changes in workplace relationships between educators, staff, volunteers, visitors and/or managers
- seeking feedback on the professionalism of workplace behaviours in exit interviews and from supervisors and where relevant families.
- monitoring issues raised by our health and safety representatives and health and safety committee. See Work Health and Safety Policy for more information.

2. Implementing measures to prevent and respond to workplace bullying, harassment and discrimination

- implementing a Code of Conduct.
- providing educators, staff, volunteers and visitors with information about our bullying, harassment and discrimination policy and relevant procedures at staff meetings, via email and by displaying anti-bullying posters.
- implementing grievance procedures which deal with bullying complaints in a confidential, reliable and timely way (see Grievance Guidelines).
- implementing effective performance management processes.
- clearly defining jobs and seeking regular feedback from educators and staff about their role and responsibilities.
- reviewing and monitoring workloads and staffing levels.
- including educators and staff in decision making which affects their roles and responsibilities.
- consulting with educators and staff as early as possible about any changes that affect their roles and responsibilities.
- promoting and modelling positive leadership styles eg communicating effectively and providing constructive feedback both formally and informally.
- organising relevant leadership training for managers and supervisors eg on performance management.
- mentoring and supporting new and poor performing leaders, educators or staff.
- facilitating teamwork and cooperation.
- ensuring supervisors act in a timely manner on any unreasonable behaviour.

3. Reviewing measures to prevent and respond to workplace bullying, harassment and discrimination

The Approved Provider or Nominated Supervisor will implement a review of the bullying, harassment and discrimination policy and procedures if there is an incident of workplace bullying, at the request of

a health and safety representative or committee, when new or additional information about bullying becomes available or at the scheduled review date. Information will be obtained from confidential surveys, exit interviews and records of sick leave and workers compensation claims.

Training about Workplace Bullying, Harassment and Discrimination

The Approved Provider or Nominated Supervisor will organise face-to-face training, role plays and group work to ensure all educators, staff and volunteers can recognise workplace bullying, harassment and discrimination. Training will cover:

- our bullying, harassment and discrimination policy and procedures
- measures used to prevent bullying, harassment and discrimination from occurring
- how to report workplace bullying, harassment and discrimination
- how bullying, harassment and discrimination reports will be responded to
- where to go for more information and assistance.

The Approved Provider or Nominated Supervisor, and other educators and staff who may be involved in resolving workplace bullying, harassment and discrimination will be familiar with conflict resolution skills and undertake training in that area if required.

Bullying, Discrimination and Harassment Procedure

What can I do if I believe I am being bullied?

Below is a summary of the steps that can be taken to address individual concerns and who to contact:

- If you can, try to resolve the problem yourself with the person(s) involved as soon as possible. You may find that they didn't mean to do what they did.
- If you're unsure of how to handle the problem yourself, you can report your complaint verbally by talking to your Centre Director or Approved Provider or in writing by completing a **Staff Incident Report Form** and giving or emailing it to your Centre Director or Approved Provider.
- If your complaint is about your Centre Director you can report your complaint directly to the Approved Provider (or vice versa).

If you observe an incident in which another employee is being bullied, bring it to the attention of your Centre Director or Approved Provider.

If you do make a complaint about bullying, you are responsible for ensuring that you:

- Make the complaint honestly and in good faith.
- Provide all the facts relevant to the complaint.
- Co-operate with the investigation and resolution processes.

If a complaint of bullying has been made about you, you are responsible for ensuring that you:

- Cooperate with the investigation and resolution processes and maintain confidentiality.
- Provide a written or verbal response to the complaint which has been made.
- Provide all relevant facts to the person conducting the investigation.

Management responsibility

If an employee brings an allegation of bullying to your attention:

DO NOT

- Ignore the complaint.
- Tell the employee making the complaint to sort it out themselves.
- Make a judgement about whether the complaint is true or not.
- Say that the employee should put up with the bullying.
- Talk to anyone about the issue except those involved in the investigation and resolution of the complaint.

DO

- Behave consistently with the Bullying Policy.
- Resolve the complaint as quickly as possible.
- Be sympathetic, sensitive and serious; the complaint is obviously serious to the person making it.

- Enquire into the matter within 2 working days, and attempt to resolve it as soon as possible.

Manager's action steps

1. Notify the Approved Provider immediately.
2. Document the complaint using Staff Incident Form or the Peer Statement Form
3. In discussion with the Approved Provider or their designate, determine who should investigate, and if possible, mediate the complaint.

Investigation and mediation:

- Investigation of the complaint of bullying must commence within 2 days of receipt of the complaint.
- With the Approved Provider or their designate, determine who the appropriate person to investigate the complaint is.
- The investigator must be someone who is impartial, and who has been trained to conduct investigations. This person may need to be sourced from outside the organisation.
- Mediation may be a consideration in resolving a bullying complaint. This form of dispute resolution is appropriate only if the person making the complaint agrees.

Confidentiality

Anyone involved in a complaint of bullying, or its investigation, must ensure that the circumstances and facts of the complaint are disclosed only to those people who are directly involved in progressing its investigation and resolution, or have a 'need to know'. In particular, it is important that staff who either make a complaint, or may be witnesses to the circumstances giving rise to the complaint, do not discuss the matter outside the investigation and resolution processes.

Investigator's discussion with the complainant

1. Determine whether an interpreter is required.
2. The complainant may bring a support person with them (eg. family member, friend). The support person is not a spokesperson for the complainant but present to support.
3. Arrange an appropriate venue to meet with the complainant. The venue should be private and free of interruptions, and wherever possible put the complainant at ease. The venue must be conducive to maintaining confidentiality.
4. Allocate enough time to discuss the complaint in full.
5. Explain how the investigation process is conducted, and the possible outcomes.
6. Advise the complainant that the matters discussed are confidential, and that the complainant must only discuss the issue with those persons investigating the complaint, or who are formally supporting them through the process.
7. Using the Peer Statement Form or Grievance Register and Procedure Forms, obtain a broad outline of the complaint, followed by a detailed description of what is alleged to have happened, when it is alleged to have happened (including dates and times), where it is alleged to have happened and by whom. Obtain and record all relevant facts.
8. Ask the complainant whether there are any witnesses, or other evidence that supports their version of events. Record the names of any witnesses to the alleged bullying, and obtain copies of any documentation provided to support the allegations. Advise the complainant that witnesses will be interviewed, and that the complainant should not speak to the witnesses about the issues.
9. Explain any other action that will be taken, eg. speaking to the alleged bully.
10. Ask the complainant what they want done in regard to the alleged bullying. It is at this point that mediation should be offered as a means of resolving the complaint. If mediation is accepted, the mediation should be arranged to occur as soon as possible.
11. Counselling should be offered to the complainant. If accepted, this may be arranged through the local agency.
12. Advise the complainant when they can expect to be advised of an outcome to their complaint.

Investigator's discussion with the person about whom the complaint is made

1. Determine whether an interpreter is required.
2. The respondent may bring a support person with them (eg. Chaplin, family member, friend).
3. Arrange an appropriate venue to meet with the respondent. The venue should be private and free of interruptions, and wherever possible put the respondent at ease. The venue must be conducive to maintaining confidentiality.
4. Allocate enough time to discuss the allegation(s) in full.

5. Explain to the respondent your role in the investigation of the complaint against them, and that it is important that their version of events is obtained.
6. Advise the respondent that the matters discussed are confidential, and that the respondent must only discuss the issue with those persons investigating the complaint, or who are formally supporting them through the process.
7. Explain to the respondent exactly what it is that they have been accused of saying or doing, including names, dates, times and locations.
8. Provide the respondent with a written memo stating the allegations.
9. Ask the person to respond, either verbally or in writing, to the allegations.
10. Using the Peer Statement Form, take notes of the respondent's version of events. Obtain and record all relevant facts.
11. Ask the respondent whether there are any witnesses, or other evidence that supports their version of events. Record the names of any witnesses, and obtain copies of any documentation. Advise the respondent that witnesses will be interviewed, and that the respondent should not speak to the witnesses about the issues.
12. If the allegations are denied, ask the respondent if they can think of a reason why these allegations have been made.
13. Ask the respondent what they believe may resolve the matter.
14. Inform the respondent that they must not victimise or bother the complainant in any way. Doing so will result in immediate action against them, eg. being stood down pending the outcome of the investigation.

Outcome:

- Determine whether or not the allegations are substantiated, or whether there is insufficient evidence or information to make a conclusion. Record in writing your conclusions, and the reason for them.
- If the alleged bullying is substantiated, there may be a basis for disciplinary action against the person responsible. If termination of employment is appropriate, this must comply with unfair dismissal laws.
- If the allegations of bullying are unsubstantiated, ensure that there is no action taken against the person whom the complaint was made, and that no notes about the complaint are retained in their personnel file.
- If there is insufficient information to make a determination in regard to an allegation of bullying, the report notes should be kept in the Staff File, and no action is to be taken against either party.
- Advise the complainant and the person about whom the complaint was made of the investigation findings and actions. If the allegation is substantiated, action will include a written record of the investigation being placed in the perpetrators personnel file.
- Provide a written summary to the Approved Provider.
- The Centre Director will store all bullying complaints files securely and confidentially.

In situations where bullying complaints are substantiated, management may take the following actions:

- Require an apology from the respondent
- Make changes to work practices
- Undertake disciplinary action
- Require employees to participate in training
- Place notes in staff file
- Any other action deemed fair and reasonable.

Links to other Policies and Company Resources

In conjunction with this policy please read the Staff Handbook (POL002), the Workplace Health and Safety Handbook (POL003), the Policy Handbook (POL001) and Worksafe Victoria resources.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

References:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>
QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.1	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
	6.2.2	<i>Effective partnerships support children's access, inclusion and participation in the program.</i>
	6.2.3	<i>The service builds relationships and engages with their local community.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	168	<i>Policies and procedures are required in relation to health and safety</i>
	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>
	107	<i>Space requirements—indoor</i>
	108	<i>Space requirements—outdoor space</i>
	109	<i>Toilet and hygiene facilities</i>
	110	<i>Ventilation and natural light</i>
	111	<i>Administrative space</i>
	112	<i>Nappy change facilities</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>
	115	<i>Premises designed to facilitate supervision</i>
	116	<i>Assessments of family day care residences and approved family day care venues</i>
	117	<i>Glass (additional requirement for family day care</i>
	73	<i>Educational programs</i>
	74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
	75	<i>Information about the educational program to be kept available</i>
	76	<i>Information about educational program to be given to parents</i>
	80	<i>Weekly menu</i>

	86	<i>Notification to parents of incident, injury, trauma and illness</i>
	99	<i>Children leaving the education and care service premises</i>
	102	<i>Authorisation for excursions</i>
	111	<i>Administrative space (centre-based services)</i>
	168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
	171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i> <i>Children respond to diversity with respect</i> <i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i> <i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i> <i>Children transfer and adapt what they have learned from one context to another</i> <i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Other References:

Legislation:

Work Health and Safety Act 2011

Work Health and Safety Regulation 2011

Occupational Health and Safety Act 2004.

Workplace bullying – prevention and response – Guidance Material October 2012 (WorkSafe Victoria).

Occupational Health and Safety Act 2004.

Sex Discrimination Act.

Race Discrimination Act.

Disability Discrimination Act.

Equal Opportunity for Women in the Workforce Act.

Human Rights and Equal opportunity Commission Act.

Workplace Relations Act.

Crimes Act.

Federal and State worksafe management agencies

- Federal – Comcare www.comcare.gov.au
- New South Wales – WorkCover New South Wales www.workcover.nsw.gov.au/default.htm
- Queensland – WorkCover Qld www.workcoverqld.com.au
- South Australia – WorkCover SA www.workcover.com
- Victoria – WorkSafe Victoria www.workcover.vic.gov.au

Sources and further reading

- Stevenson, David (2001) *Health and Safety in Queensland Child care Centres: A blueprint for Success*, Scintel NSW
- Tansey, S. (2005) Supervision in children's services. *Putting Children First*, 15, 8-11.
- Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)
- National Childcare Accreditation Council
(http://www.ncac.gov.au/policy_development/policy_templates.asp) Retrieved 25 June, 2008
- Preventing and Responding to Workplace Bullying: Safe Work Australia Draft Code of Practice Anti-bullying jurisdiction: FairWork Commission
- Workplace bullying – prevention and response – Guidance Material October 2012 (WorkSafe Victoria).

11 - SUPERVISION POLICY

Position Statement

Children are required to be supervised at all times to ensure that they are moving and playing in the environment in a safe manner. Supervision of children is the cornerstone of duty of care for team members working with young children in an Early Childhood Setting.

Objective

To ensure that team members are aware of the expectations for supervision of children whilst on duty.

Implementation

- Team members will adhere to the supervision ratios as per all regulations pertaining to this centre, including but not limited to, State and Commonwealth legislation.
- As children are signed into the centre by their parents or guardians using the OWNA Kiosk System, they will be introduced to a team member so that they can be directly supervised. Direct supervision of children is the responsibility of the team members of each room for the allocated children within the room and globally, for the team who are in attendance at the Centre in any one day. Team members will know how many children they have in their group and be able to account for them at all times whilst working.
- Active supervision will be maintained during all transitional periods such as movements from inside to outside, meal times, family groupings and evacuations. It is expected that team members will follow procedures to ensure that supervision during these times is maintained and that duty of care is maintained.
- Team members will follow service supervision protocols and plans. Effective communication is essential for active supervision. Please refer to Appendix 2: Service Active & Engaging Supervision Strategies Protocols.
- Educators can achieve active supervision in 6 steps:

Active supervision in 6 steps



Developed from the Queensland Department of Education resource
Active supervision in 6 steps: Get your supervision active <https://www.doe.qld.gov.au>

**Use the QR code to explore each of
the 6 steps of active supervision.**

(For more information see ACECQA – Active Supervision in 6 Steps. See Appendix 1)

Brief Procedures

(Please refer to the Opening Procedure and Lock Up Procedure at your centre for more detailed directions)

Room OWNA Kiosk Rolls & Child Wellbeing Schedule

Team members will check room rolls (Using OWNA App Attendances) to ascertain which children are in attendance each day.

If Daily Information Schedule Used (Paper Version) at Service/Room: OWNA App Attendances should be cross checked to the Child Wellbeing / Daily Information Schedule (Paper schedule). Using the Child Wellbeing/ Daily Information Schedule Educator cross check to OWNA Attendance Schedule is required to be completed for each children as they arrive and depart from the service. Please note if child has been absent this will also be indicated on Child Wellbeing / Daily Information Schedule as “ABSENT” (did not attend the service today) and educator to initial that they have marked the OWNA App Attendances as ‘ABSENT’. The educator will write “Absent” on the Child Wellbeing Schedule and write their initial as indication of completion of this task and check performed.

Family Grouping Times

When children are grouped together at the beginning and end of the day team members will review the OWNA App Attendances to review that all children present. The OWNA App Attendances will be marked as children arrive and depart by parents using the OWNA App. Educators will check periodically (approximately every 30 minutes during peak children attendance and departure times. I.e. From 8.00am to 9.30am and 3.30pm to 5.30pm. Outside these times each hour) that the parent a parent has signed in and out their child/ren.

Team members will know how many children are present, their relevant age groups, the required staff – to – child ratios and consistently apply these ratios at regular intervals as child numbers change.

As the groups separate in the morning to go to their rooms, team members will check all children marked present in OWNA App Attendances. In the afternoon, late team members will review OWNA App Attendances for children who are present or absence. This includes completing a headcount on transition to rooms or different environments.

Child Electronic Sign In/Out System Sheets – OWNA App Attendances: Sign in and Out Kiosk

Sign in/out records are a legal requirement and will be maintained as per procedures and regulations. Early and late rostered team members will ensure that parents meet their obligations and sign children in and out using the OWNA App Attendances. If parent forgets to sign in or out their child/ren the Qualified Educator will be required to sign in or out the child/ren. Please note the parent will be required to confirm the sign in and out of their children next time they attend the service. Late team members will follow close / lock up procedure to ensure that all children have been picked up and duly signed out.

Environments

- **Bathrooms:** Team members will be aware of children’s movements to and from the bathroom area at all times. Team members will directly supervise the toileting of all children under the age of three years, and when required will directly supervise the toileting of older children.
- **Sleep times:** Children will be supervised at all times including sleep time and will not be left unattended. Should a child still be asleep when the group is transitioning to the outdoor area either they will be gently woken or a team member will stay behind to supervise any sleeping children.
- **Outdoor:** Children will not access the outdoor environment without the correct ratio of team members in attendance.

Preparation for closing

Team members will be aware of children remaining at the centre towards the end of the day. Children will be supervised and have engaging activities provided for them for the entire time that they are at the centre. Team members will maintain direct supervision of all children remaining as they work through closing procedures at the end of the day.

OWNA App Attendances: Closing educator (qualified) will perform the following duties:

- Review the OWNA App Attendances – Sign In and Sign Out Time for each child. Each child should have a sign in time and out time. Or Marked as Absence for the day. This is reviewed to ensure all children are accounted for.
- Before closing service, all children will be accounted using the OWNA App Attendances for all rooms.
- OWNA App Attendances and all physical environments are reviewed by two closing educators to ensure all children are accounted for. Follow services closing procedures.

If paper version of Daily Information Schedules used in service and room: educator initial (to indicate that the educator has checked child has left for the day and also child is signed out in OWNA App Attendances). Please note if child has been absent this will also be indicated on Child Wellbeing Schedule as "ABSENT" (did not attend the service today) and educator to initial that they have recorded the 'ABSENCE' in the OWNA App Attendances.

Communication

Effective supervision of children depends upon sound communication between team members. Team members will always communicate their movements and needs to others to ensure that supervision of the children is effectively maintained.

Team members will follow the Service supervision protocols and plans.

It is also expected that team members will share relevant information about children during the day. This may include children's movements, behaviour, general information, etc, and at the end of the day, parent messages.

Team member movements

Team members are sometimes required to leave the group of children they are supervising. If a team member needs to leave the group, they will communicate this to their colleagues and only leave once they are satisfied that adequate supervision can be maintained, and they have acknowledgment from the team members that they are leaving the group.

Supervision plans are displayed in all indoor and outdoor environment to assist educators to identify locations for effective supervision.

Students and Volunteers will not be left with sole responsibility of children or left out of sight of team members either in the inside OR outside environment. Students and Volunteers do NOT count towards the child/staff ratio. Please see Student and Volunteers Policy.

Children will not be left unattended on the change table area.

If more than one staff member within the room, staff are situated at different locations of the room to support adequate supervision. This also allows small group experiences as well as individual experiences to be undertaken.

Children will not be taken by any staff member where they become non-visible to others and will not be taken into any room which can be locked.

Staff will ensure they are constantly supervising children when they are playing, eating or drinking.

The safety of children is not compromised by the age ranges of children within the groups.

Appendix 1 – Active Supervision in 6 Steps

Active supervision in 6 steps



Developed from the Queensland Department of Education resource
Active supervision in 6 steps: Get your supervision active <https://www.doe.qld.gov.au/active-supervision>
Use the QR code to explore each of the 6 steps of active supervision.

(Source: <https://www.acecqa.gov.au/active-supervision-6-steps>)

1. Set up the environment

- Regularly review risk assessments to determine the level of supervision for specific situations and identify the best locations and approaches to supervision.
- Arrange the environment to ensure maximum visibility and accessibility, while maintaining good flow around furniture and opportunities for large and small group play.
- Using child and adult-sized furniture at waist height can allow for a clear view across learning spaces, from various angles and can be easily rearranged to accommodate different interests, needs and abilities of children.
- Be mindful of where children are and how they will use equipment and spaces. For instance, set up climbing equipment away from fences and arrange high-risk activities, such as carpentry, in areas where supervising educators can assist and engage with children while also adequately supervising the rest of the environment.

2. Position educator

- Careful planning of rosters can ensure educators are always available to respond to children.
- Be conscious of facing the majority of the group and consider a 'back to the wall' strategy where educators can see most of the children.
- Collaborate and communicate with other educators and decide on the best positions for supervision. This includes when educators are supporting children with care routines, starting a new activity or during arrival and departure times.
- Consider where other educators are located and ensure educators are distributed throughout the environment. Educators should position themselves near areas that have less visibility or that may be a hazard.
- Supervision arrangements should be flexible to allow for supervision of individual or small groups of children, such as sleeping children or indoor and outdoor experiences offered simultaneously.

3. Scan and count

- Effective supervision requires a combination of observation, scanning, counting and engagement.
- Continually scan and observe the environment to know where everyone is and what they are doing.
- Scan and check the environment for hazards such as open doors or gates, play equipment in unsafe positions, sleeping or resting children or children trying to enter out-of-bounds areas.
- Develop a system of regular head counts and communicate with other educators about children's whereabouts, activity and behaviour, it is important to know where each child is at all times.
- Make sure enrolment records are easily accessible and up to date with the names of people authorised or not authorised to collect each child.

4. Listen

- Identify signs of potential danger by paying attention to the sounds, or the absence of sounds in children's play, for example splashing water, crying, gasping, changes in language, or silence.
- Listen carefully to children and noting any changes of tone or volume in their voice.
- Listen to the language of other educators for cues (such as signs of fatigue or tiredness) that might mean support, engagement or redirection is needed.

5. Anticipate behaviour

- Observing children's play and anticipating what may occur next can allow educators to assist children as challenges arise and to intervene when there is potential danger to children.
- Educators should use what they know about each child's knowledge, culture, ideas, abilities and interests to anticipate what they may do.
- Create challenges that children are ready for and support them in succeeding.
- Utilise your service's available data to understand when (time of day) and where (locations) incidents are most likely to occur, to anticipate behaviour and supervision needs. You can also utilise ACECQA's [Occasional Paper 9](#), to consider national trends and data on serious incidents that resulted in injury, trauma or illness in Australian children's education and care services.

6. Engage and redirect

- Use existing knowledge of each child to plan learning experiences and routines that are engaging and child-centred.
- When appropriate take part in play and planned activities to support engagement and minimise the risk of harm and injury.
- Stay close by to children who may need more support or to children engaging in high risk activities, such as when they are using high climbing equipment or using scissors.
- Plan for times of the day when children may need additional support, such as at the end of the day and before sleep or rest time.
- Be alert and ready to redirect children to other play before a situation becomes risky or escalates.

Appendix 2 – Service Active & Engaging Supervision Strategies Protocols

Fundamental Strategies

- There is a responsible person in charge and management of the service at all times. This is indicated on rosters and also on the sign in the front foyer. This indicates to everyone who is responsible for the overall service.
- Parents are directed to the person in charge of room to answer important queries or discuss incidents.
- The Opening & Close Checklist, Playground Checklist and Room Checklist is used as a reminder and prompt for each team about the procedures and safety checks that need to be performed daily. Checklists are compiled as task is completed throughout the day, [not at end of day].
- Chemicals are to be stored out of reach in a secure location or locked cupboard.
- Room Leader and educators assess the size of toys or resources to prevent choking. Broken toys are removed immediately from room. Blind cords are checked regularly to ensure they are out of reach of children.
- Any electrical appliances have cords fixed to the wall.

Child to Educator Ratio Review: Periodic head counts occur for room and across service by Room Leader and Responsible Person of Service. Information is passed on to Responsible Person asap, i.e., on arrival for shift. The roster is adjusted to cater for child and educators' changes.

Change of shifts

Handover briefings are completed by all educators (on commencing shift and end of shift). Room Leaders /people in charge of room give advance warning before finishing their shift/leaving the room, to allow each person to exchange information).

Relief Educators

Room Leaders/person in charge of room need to consult Relief Educator/s entry and exit of room regarding the following:

- a) To ask what role and tasks they will be performing, (i.e., what to complete first, where to go (indoor or outdoor experiences), what rostered jobs were allocated to the person they are covering, etc).
- b) If covering breaks: to check the order of breaks and if all breaks have been covered.
- c) To advise Room Leader/person in charge of room when they are leaving the room to go home or attend another room.

Non-regular agency educators:

Room leaders consult agency educators and assess what role, tasks and responsibilities will be delegated for the shift. Agency educators are provided the roster times, so they know where to be at what times, etc.

Opening / Arrival and Departure/Closure Protocols

Hazard checks of room and outdoor environments are completed by staff. See Opening & Close Checklist and Playground Checklist.

Opening and Closing Checklists are completed. (The Checklists prompt and remind educators of the required tasks, including to identify any risks or hazards). (Without checklist educators would need to remember these tasks, then document the completion as evidence they are completed.)

Protocols and procedures for accounting for all children at all times:

- a) Regular head counts are completed by the Room Leader/person in charge of room and outdoor environment.
- b) Family Sign-in and out of children: Responsible person and room leader review periodically through day at approximately 7.45am, 8.30am, 9.30am, 3.30pm, 4.30pm, 5.30 and 6pm. (Narrate information: Children Numbers/Educator at bottom of Wellbeing Chart or Routine Checklist)
- c) Educators and Room Leader/person in charge of room know who is collecting child/ren from service. They also always follow authorised nominee procedures to check on enrolment form or OWNA contacts who is permitted to collect child. If not 100% sure, the parent is contacted before releasing child.
- d) Daily Routine/Wellbeing Schedule in all rooms is completed. (A cross-check is completed between the head count & OWNA Kiosk sign in and out by the Room Leader and a Responsible Person. The second educator completes the headcount and compares their count with the first educator's count to cross-check.)

On closure of the service: A first and second check of the service for children and staff is performed before closing for the day.

Opening / Arrival of Children Protocols:

- Parent/guardian required to sign-in child using OWNA kiosk. If parent/guardian misses sign regularly – discuss and resolve.
- Parent / guardian to perform handover to educator. Child's name added to Child Wellbeing Chart / Roll by educator.
- Sufficient supervision levels are maintained:
 - a) If for any reason (E.g. educator unplanned absence) there is insufficient educators to cover the children in attendance. The Responsible Person in Charge of Service will inform the parent / guardian that they will need to stay in the service until the sufficient number of educators are present. (For opening of service this requires at least two educators; at least one educator holding a diploma or bachelor's degree). As children attendance increases the number of educators to cover child ratio will increase accordingly. When reviewing ratio; counts are made progressively from service opening hours.
 - b) Responsible person would contact another educators or relief agency to cover educator absence.
 - c) Roster would be reviewed immediately by Centre Director or Responsible Person in Charge of Service (within 24 hours) to assess if roster needs to be altered. I.e. Roster times or people. All teams room will email to communicate change of roster (also published from Wageloch).

Outdoor Environment Protocols

Active supervision strategies are discussed and agreed with all team members in outdoor environments.

- a) The Outdoor Supervision plan is used to identify and discuss risky play areas, blind spots, gate and door supervision.
 - Room Leader/ Person in Charge is responsible for delegating positioning in room and outdoor environment.
 - If the space is shared, Room Leaders confirm with each other that the equipment is per the educational plan and is set-up safely (height/position).
 - Room Leader and team discuss and agree on the educator positions for effective and active supervision to be achieved outdoors.
 - Educators are spread evenly through the yard in areas of risk (including experiences that require high educator involvement).
- b) All educators scan the environment regularly; adjust their location in space to actively supervise.
- c) Equipment is set up safely according to guidelines displayed.
- d) Periodically throughout the day educators locate themselves to actively supervise children according to the equipment and experiences that have been set-up.
- e) When a transition to indoors has occurred for all children, two educators complete a check of the outdoor environment (with particular attention to risk areas) and confirm head count. The Routine Checklist/Wellbeing Schedule/OWNA Head Count Tool is used to document count performed.

Other

- All educators reset and modify experiences (in consultation with team) to enhance engagement of all children.
- Educators decide when to engage in play to scaffold children's learning and when to let children play by themselves.
- Educators seek guidance from Room Leader/Responsible Person on how to balance engagement, supervision and tasks.
- Incident forms are reviewed each week to identify any reoccurring incidents and reflect on what could be reduced or mitigated.

Mealtime Active Supervision Protocols

- The 'Room Flexible Routine' and 'Room Duty Roster' is used by the Room Leader/person in charge of the room to discuss and agree roles and responsibilities in the room, including preparation for mealtimes such as wiping tables, organising face washers and bibs, filling water jugs, etc.
- Having items needed prior to routine assists in it running smoothly and supports effective supervision. (This should be noted on routine)
- The room leader/person in charge of the room uses the supervision plan to guide educator positions in room or outdoors during mealtimes. - A progressive mealtime approach is used to support children's agency and to ensure a smooth-flowing mealtime.

Mealtime protocols:

- a) A minute meeting before lunch time routine starts is held so that the Room Leader/person in charge of the room can delegate tasks and raise awareness of some aspects (particularly to relief educators) including:
 - who has the primary responsibility for implementing the mealtime with children.

- which children have allergies. Ensure meals are reviewed. Use allergy card to label meals.
- when to commence inviting children to mealtime.
- who to invite first to lunch: based on individual children's needs, i.e., who needs to sleep first, which children have expressed hunger or who did not have much morning tea/breakfast.
- b) all educators ensure children wash their hands and follow hygiene practices. Person delegated to be leading the mealtime will ensure children's hands are clean/sanitised. (Washed with soap and water at sink or cleaned with a wipe and sanitised.) A second educator may assist child with effective hand washing procedures.
- c) An educator is always within close distance (preferably sitting with children) at mealtimes and able to see and hear each child. Educators engage with children, encourage conversations and support self-help skills.
- d) Positioning and roles of other team members while lunch is occurring is delegated and discussed.
- e) The room's flexible routine schedule is displayed in the room to guide all educators (particularly new and relief educators) and includes tips and reminders for the smooth running of the room.

Highchair Protocols.

- The Highchair Policy provides guidance.
- A Supervision Plan is used during mealtimes. An educator is in close proximity at all times to provide safe supervision while a child is eating in a highchair. Children are only situated in a highchair during mealtimes.
- Highchairs are situated in a safe position to avoid tipping. Children are securely seated in the highchair.

Toileting

- See 101 – Nappy Change and Toilet Support Policy.
- Nappy changes are required to be recorded at the end of each nappy change to ensure accurate information is recorded in the children wellbeing charts.
- Supervision plans and strategies are agreed with all educators each day including who will be supervising children who are using the bathroom areas.
- Procedures are followed in relation to supporting children who are toilet training to support their hygiene and cleaning of bottoms.
- All rooms (including Kindergarten and Prekindergarten room) follow the procedure if child has a toileting accident. (I.e., Educators are allocated to supervise children during toileting).
- Educators will use intercom to request support if needed from Centre Director or Responsible Person in Charge of Service.
- OWNA Wellbeing Schedule is used to flag children who need additional support for toileting and inspecting that their bottoms and clothing are clean. This information is used to plan any additional support if needed.
- Toilet areas are regularly checked so that they are hygienic, clean and that there is no water on floors.

Nappy Change Protocols

- See 101 – Nappy Change and Toilet Support Policy.
- The Nappy Change Procedure is displayed in all nappy change areas for guidance.
- The nappy change schedule in OWNA is used to record when changes have been performed or nappies checked. (Room Leader/person in charge of room regularly checks the OWNA nappy change schedule and Routine Checklist Schedule to support each child's wellbeing. E.g., whether a child's nappy is overdue for a change, or if a child has not eaten morning tea and lunch. Water intake.
- The nappy change procedure is followed at all times. Nappy change limits are implemented: 4-5 nappies at one time, (i.e., limit the number of nappy changes by one educator at one time to ensure a higher engagement level with each child and also to support educator wellbeing).
- The nappy change area is stocked each day with sufficient supplies before starting nappy changes. (This ensures that at no time is an educator distracted or compelled to move away from the nappy change area).
- The nappy change schedule is completed after each nappy change.

Indoor Environment Protocols

The Room Compliance and Risk Assessment Checklist is used as a reminder and prompt for each team about the procedures and safety checks that need to be performed daily. Checklists are compiled as task is completed throughout the day, [not at end of day] The person who completed the task initials the checklist.

- a) active supervision actions: the environment is scanned using eyes and ears. Educators locate themselves with back to walls not to children and are aware of blind spots.
- c) regular head counts, and assessment of ratio cover occur. (I.e., at transition times and other key times throughout the day)
- d) communication occurs between room leaders when sharing environments, (each team is responsible for the information that needs to be exchanged).
- e) delegation of roles is made by Room Leader/person in charge of the room. (Agreement is reached on who is engaging with children and who is doing tasks (i.e., nappy changes, daily journal) aiming for a model where more educators are engaging than standing doing tasks).
- f) supervision takes priority over tasks being completed.
- g) the Room Leader/person in charge of room establishes procedures for supervision when the door to the outdoor environment is open for free flow indoor/outdoor play.
- h) periodic checks are made by the Room Leader/person in charge of room and all educators that doors to the hallway are closed. (Rooms need to establish procedures for when door is open (indoor/outdoor program versus indoor program versus outdoor program)
- i) when a transition out of the room has occurred for all children, an educator completes a check of the indoor environment and confirms head count.
- j) environments checked for small items and other items that are unsafe for younger children (e.g. scissors). These can be dangerous to young babies and toddlers. (Especially before commencement of Family Group Session Times)

Other:

- a) Children are never left unsupervised. (Students are not counted in child/educator ratio.)
- b) The phone intercom system is used to transfer calls or phone messages are taken.
- c) Personal mobiles phones or smart watches are not used while working as this impacts the ability to supervise children and can cause distraction.
- d) Times and schedules are agreed by the room team for completion of documentation on iPads.
- e) If applicable, Room Leader/person in charge of room assesses when low windows need to be shut and locked, e.g., if everyone is indoors.
- f) All educators reset and modify experiences (in consultation with team) to enhance engagement of all children.

Sleep and Rest

- The Sleep and Rest Policy provides guidance for all educators.
- Knowledge and skills gained from Red Nose training is embedded in policies, procedures and practices.

Sleep and Rest Protocols:

- Clothing is loosened, any necklaces or dummy cords/ribbons are removed before children are put to sleep.
- No children are put to bed with bottles.
- Sleep checks are performed by educators every 10 minutes for sleeping children. Checks are documented on sleep check record and educators initial who performed the check each time.
- When inspecting children who are sleeping educators observe whether the child's chest rises to confirm breathing and that they are safe (i.e., bedding and clothing not restricted, etc).
- Educators are able to view and hear children wherever they are sleeping, whether indoors or outdoors.
- Top and tail configurations are used for positioning of beds.

Child Medication

- The Medication Policy provides guidance.
- All educators have first aid training and knowledge of policy and procedures associated with medical management plans.

- Medical Management Plans summarises are displayed in rooms (reviewed for expiry dates). Child Medical Management Plans are held in rooms and reviewed by Educators to know each children's medical conditions action plan, risk management and communication plan.

Medication Protocols:

- When giving medicine to children, the Room Leader/person in charge of the room and a second educator are required to review medication and dosage before giving the medication. Review medical action plan is in date and medication is in not expired.
- The directions on the pharmacy label are followed and in accordance with the child's Medical Management Plan or Medication Form.
- All medicines require a pharmacy label for that child and are stored securely in the medicine storage container within the room or medication bag in the fridge (room or kitchen).
- If the prescribed medication (that required a medical action plan. Ie Anaphylaxis, Allergy, Asthma, etc) has expired or is not available, the child is unable to attend the service until the medication is replaced and medical action plan is current.
- If prescribed medication (such as an antibiotic cream) is forgotten; parent and guardian to inform educator team that the medication is not needed for day. This protocol does not apply for other prescribed medication such allergies, anaphylaxis, asthma, etc.
- All staff members know the location of the first aid kits.

Appendix: 3 - OWNA Headcount Procedure

(For more information see: <https://ownaportal.tawk.help/article/evacuationheadcount-practice-real>)

Overview

- Headcount procedure during key transition times is crucial for ensuring the safety and well-being of all children.
- Performing a headcount procedure should verify that the OWNA rolls is current with the correct children and educators signed in and out.
- Use the OWNA Evacuation / Headcount Tool to submit headcounts throughout the day. If OWNA App is not functioning, headcounts are to be performed using paper schedules.
- The headcount list will display a list of Children only.
- The names of the staff conducting the headcount must be recorded in the “Details” section of the form. This includes the educator completing the headcount and the second educator verifying it. (Visitors do not need to be recorded, as they are not involved in the headcount.)
- If the child or staff member is not signed in or out when performing headcount this is required to be completed to accurately record the headcount.
- If a child is not signed IN by a parent or guardian upon drop-off and educator should sign in the child into the roll as soon as possible. Periodic checking of roll (15 minutes) should be performed by an educator during drop off and pickup time to check sign in and out is occurring by parent or guardian.
- If a child is still signed IN on the roll but not present in the room or service. Educator must follow procedures to verify the child has left the service with an authorised adult listed per child’s enrolment form. This includes confirming with educators they have witnessed the child leaving with parent/guardian. Once this is verified, the lead educator should sign-out the child on behalf of the parent/guardian.
- When a headcount is performed this information is discussed with other educators to ensure everyone is informed what children and educators are present and accounted for.

Headcount checks are required to be performed at the following times:

- At Family Grouping transition times – morning and afternoon.
 - a) Morning transition from ‘Family Grouping Room’ to Rooms: headcount is performed for each room who transition to another room – this is performed before children move to their room. Also a headcount is performed for the children who remain in the family grouping room.
 - b) Afternoon transition from Room to ‘Family Grouping Room’: headcount is performed for each room who transition to family grouping room – this is performed before children transition to the family grouping room. At the time of Family Grouping, the children who are originally present in the family grouping room need a headcount checked. All children are covered in the headcount at this time.
- Before child morning meeting/group time or before educator morning educator minute meeting. So all educator verify all children present and recorded on roll.
- Before or at child rest/sleep time to account for all children at this transition time.
- If a group of children transition from one room to another throughout the day.
- At the end of an incursion when children and transition back to play indoors or outdoors.
- See 48 - Excursion and Incursion Policy for procedures for when headcounts are required to be performed during excursions.
- See 22 - Emergency Evacuation Policy for procedures to perform headcounts for drills and real events.
- See Rest Time / Sleep Policy for sleep checks that are performed during this period of time. The sleep check procedure include a headcount check of individual children who are sleeping and resting.

Assigned roles for headcount:

- Two educators to perform the headcount. One educator performs the headcount, and the second educator complete a second headcount check before submitting headcount report in OWNA.

- Two educator cross-verify that both headcount match. If there's a discrepancy, repeat the counting process.

Conducting the Headcount:

- Depending on age of children the headcount will involve children or not. If children are engaging in an activity or experience it is not necessary to interrupt the child to perform the headcount.
- **First Count:** The leading educator calls out each child's name from the OWNA headcount roll visually confirm child is present.
- **Second Count:** After the first count is complete, the second educator reviews the OWNA Headcount roll and confirms children names and visually confirms child is present.

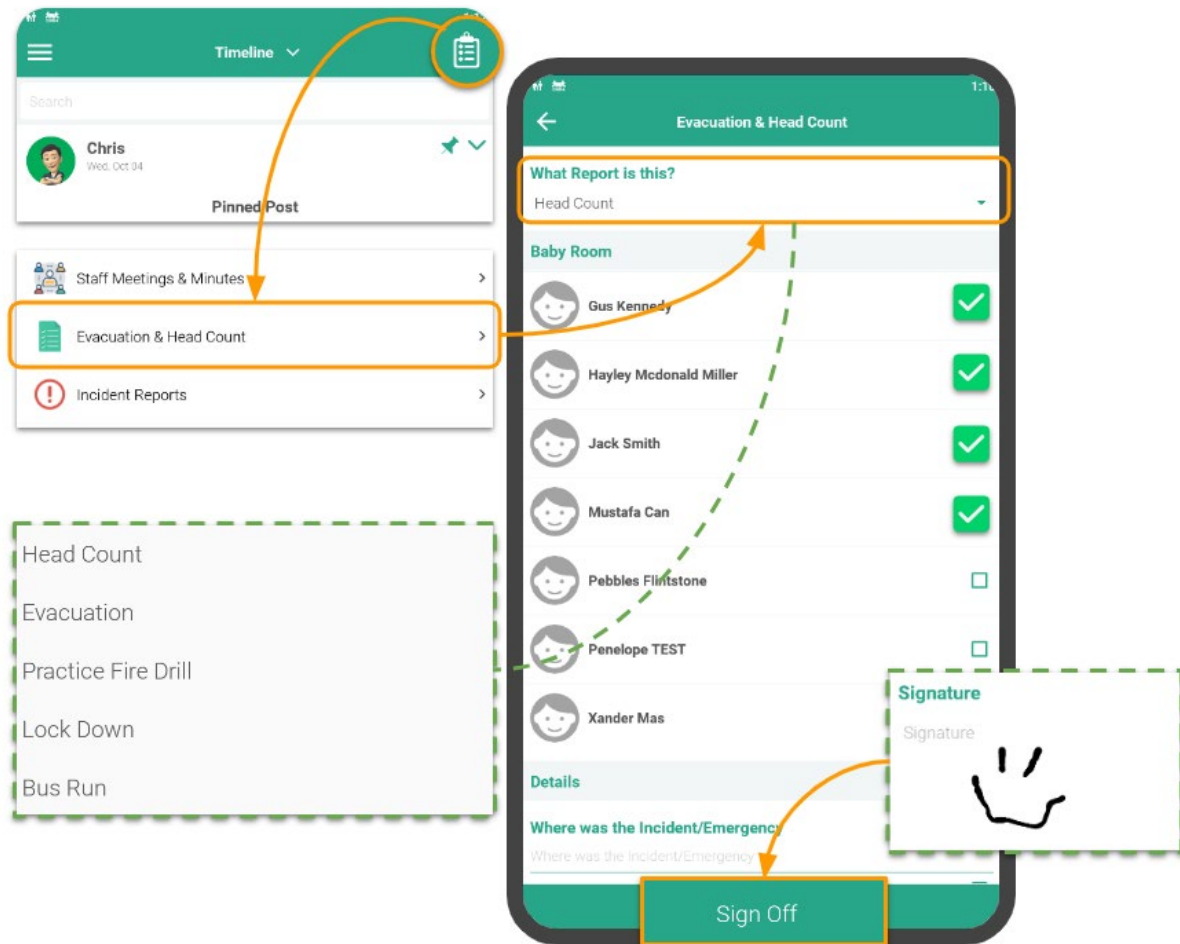
Transitioning Planning:

- **Communicate:** Provide children a warning of when a transition or change routine is going to be performed. Inform the children about the next activity and what is expected during the transition.
- **Lead the Group:** One educator leads the group, while the other remains at the back to ensure all children are moving safely. If one educator, the educator actively supervises children into room or outdoor environment to ensure all children are accounted for.
- **Final Count:** Once the group of children has settled into the new activity or location, lead educator performs a final headcount to ensure no child is missing.

Steps to Submit Headcount Check

Actions Menu > Evacuation & Head Count

1. Select the type of Report you want to complete
 - Head Count – use for headcount check performed throughout the day.
 - Evacuation – real event
 - Practice Fire Drill
 - Lock Down
 - Bus Run
1. Tick the checkbox next to the name of all children
2. Check off all staff and visitors (evacuation, drill, lockdown, bus runs)
3. Fill out additional fields (optional)
4. Draw your signature – lead educator
5. **Sign Off** the check



Reviewing Submissions of Headcounts

Please note only Admin users can access submissions.

Health & Safety > Evacuation/Head Count

- Click **Edit** to update and make changes to the record
- Click **View** to get a print/export-friendly format of the record

Children Participation (Last 5)

Lists children who have participated in the Last 5 checks carried out. Use the Filter to adjust the Type of event. You can toggle to include HeadCount as well.

Staff Participation (Last 5)

List Staff who have participated in the Last 5 checks carried out. Use the Filter to adjust the Type of event. You can toggle to include HeadCount as well.

Policy Reviewed: 19/3/2026

Reference:

NQS

QA4	4.1.1	The organisation of educators across the service supports children's learning and development.
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regulations numbered 240 and higher are state or transitional regulations

Reg	122	Educators must be working directly with children to be included in ratios
Reg	123	Educator to child ratios—centre-based services
Reg	126	Centre-based services—general educator qualifications
Reg	130	Requirement for early childhood teacher—centrebased services—fewer than 25 approved places
Reg	131	Requirement for early childhood teacher—centrebased services—25 or more approved places but fewer than 25 children
Reg	132	Requirement for early childhood teacher— centre-based services—25 to 59 children
Reg	133	Requirement for early childhood teacher—centre-based services—60 to 80 children
Reg	134	Requirement for early childhood teacher—centre-based services—more than 80 children
Reg	135	Early childhood teacher illness or absence
Reg	136	First aid qualifications
Reg	240	Qualifications for educators—centre-based service applies until 31.12.15 Applies to reg 126
Reg	241	Persons taken to hold an approved early childhood teaching qualification
Reg	242	Persons taken to be early childhood teachers applies from 1.1.14 to 1.1.16 Applies to regs 130-134
Reg	243	Persons taken to hold an approved diploma level education and care qualification
Reg	244	Persons taken to hold an approved certificate III level education and care Qualification
Reg	245	Person taken to hold approved first aid qualification. Applies until 31.12.12 or qualification expires.
Reg	246	Anaphylaxis training
Reg	247	Asthma management training
Reg	355	Educator to child ratio—children over preschool age Applies to reg 123(1)(d)
Reg	356	Qualifications for educators—children over preschool age Applies to reg 126(2)
Reg	357	Educator to child ratio—children aged over 24 months but less than 36 months Applies to reg 123(1)(b)
Reg	358	Working with children check to be read
Reg	360	Educator to child ratios—children aged 36 months to preschool age Applies to reg 123(1)(C)until 31.12.15
Reg	361	General qualifications—centre-based service Applies to reg 126(1) until 31.12.15
Reg	362	Early childhood teacher in attendance—fewer than 60 children Regs 130-132 do not apply until 1.1.14.
Reg	363	Early childhood teacher in attendance—60 or more children Reg 133(1)(a) and (b)does not apply until 1.1.14 Reg 134(1)(a) and (b) does not apply until 1.1.20
Reg	364	Educational qualifications—centre-based services— saving of existing experience and qualification Applies to reg 126(1)(b)

Other References:

Handbook on Child Care Licensing
QIAS Principle 5.2, 5.3

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

12A – CHILD SAFE STANDARDS POLICY

Position Statement

Our Statement of Commitment to Child Safety

The organisation has a strong commitment to child safety, wellbeing and protection, through implementing and adherence to the Child Safe Standards and the Information Sharing Scheme. This commitment extends to all children in our service and community with an aim for children to feel safe, secure and supported. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations. We have a zero-tolerance approach to child abuse and harm. We acknowledge the importance of our organisation and community to be committed to culturally safety of aboriginal children, the cultural safety for culturally and/or linguistically diverse children and safety of children with a disability.

Objective

To ensure the safety, health and wellbeing of children through organisational culture, values and practices that embed a culture of child safety and protection. All educators, staff, visitors and volunteers are committed to identifying possible risk and significant risk of harm to children and young people at the Service. We comprehend our duty of care responsibilities to protect children from all types of abuse and neglect and will adhere to our moral and legislative obligations at all times. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations, including financial interests or other obligations of management, and are embedded in our daily practices, policies and procedures.

We aim to implement effective strategies to assist in ensuring the safety and wellbeing of all children. Our Service will act in the best interest of each child, assisting them to develop to their full potential in a secure and child safe environment. We understand our statutory duty of care to comply with both the Child Safe Standards and Reportable Conduct Scheme to build our capacity as an organisation to prevent and respond to allegations of child abuse.

The centres' approach to child safety and protection is based on the following principles:

- ☐ Children have the right to feel safe, secure and under the protection of responsible, caring, trustworthy, adults at all times.
- ☐ Children will be respected and in no way degraded, endangered, exploited, intimidated or harmed psychologically or physically.

Definitions

See Child Safety, Protection and Wellbeing Policy for further definitions.

Child abuse means:

- a sexual offence committed against a child
- an offence committed against a child under section 49M(1) of the Crimes Act 1958 (Vic), such as grooming
- physical violence against a child
- causing serious emotional or psychological harm to a child
- serious neglect of a child.

Harm is damage to the health, safety or wellbeing of a child or young person, including as a result of child abuse by adults or the conduct of other children. It includes physical, emotional, sexual and psychological harm. Harm can arise from a single act or event and can also be cumulative, that is, arising as a result of a series of acts or events over a period of time.

Child Safe Standards: The 11 Child Safe Standards aim to protect children and young people, by requiring organisations to put policies and processes in place to prevent and respond to abuse. They aim to make keeping children and young people a key focus of organisations in Victoria.

Child/Children means a person who is under the age of 18 years.

Concerns and complaints

A concern refers to any potential issue that could impact negatively on the safety and wellbeing of children.

A complaint is an expression of dissatisfaction to the organisation related to one or more of the following:

- our services or dealings with individuals
- allegations of abuse or misconduct by a staff member, a volunteer or another individual associated with organisation.
- disclosures of abuse or harm made by a child or young person
- the conduct of a child or young person
- the inadequate handling of a prior concern
- general concerns about the safety of a group of children or activity.

Failure to disclose and failure to protect: Failure to disclose or take action in relation to suspected child sexual abuse can constitute a criminal offence. The law requires any adult who holds a reasonable belief that a sexual offence has been committed in Victoria, by an adult against a child (aged under 16) disclosed this information to police.

Forming a reasonable belief/reasonable grounds: A person forms a reasonable belief that a child is in need of protection, or their safety or wellbeing is at risk when they are more likely to accept rather than reject their suspicion and the belief is formed through disclosures, observations or other information. Proof is not required to support your claim.

Reasonable grounds for forming a belief may include where:

- a child states they have been abused
- a child states they know someone who has been, or is being, abused
- someone who knows the child states that the child has been abused, is being abused, or is at risk of abuse
- you observe a child's behaviour, actions or injuries that may place them at risk of harm or abuse
- you are aware of persistence violence, parental substance misuse, disability that is impacting on the child's safety, stability or development
- you observe signs or indicators of abuse.

Staff should make sufficient enquiries to form a belief; however, it is not the role of staff to conduct an investigation into child protection concerns or criminal offences. (DFFH) Child Protection or Victoria Police will determine what is to be investigated.

Inappropriate conduct to a child: any behaviour that a reasonable person would consider to be inappropriate in an education and care service, taking into account the child's age, development, safety, wellbeing and the context of the interaction.

Mandatory reporters: in Victoria, are people who deliver the following services, wholly or partly, to children as part of their paid or professional work:

- registered medical practitioners
- nurses, including midwives
- Victorian police officers
- registered teachers and school principals
- out of home care workers (excluding voluntary foster and kinship carers)
- early childhood workers
- youth justice workers
- registered psychologists

- school counsellors
- people in religious ministry.

Reportable conduct: The Reportable Conduct Scheme requires employees in approved education and care services (kindergartens, after school hours care services) and Children's services (occasional care providers) to notify the Social Services Regulator and the regulatory authority of any allegation of employee misconduct involving children. [Note: changes to the reporting regulator occur from 23 February 2026]

There are five types of reportable conduct:

- sexual offences committed against, with or in the presence of a child
- sexual misconduct committed against, with or in the presence of a child
- physical violence against, with or in the presence of a child
- any behaviour that causes significant emotional or psychological harm to a child
- significant neglect of a child

Types of abuse and neglect: The VIC Government identifies the following types of abuse and neglect:

- physical child abuse
- child sexual abuse
- grooming
- emotional child abuse
- neglect
- family violence
- children exhibiting inappropriate sexual behaviour

Staff within early childhood services play a vital role in protecting children from harm by responding to and reporting any incidents, disclosure or suspicions. Educators and staff are best placed to identify signs and behaviours that may indicate that a child has been subject to abuse or identify a community member, staff member, contractor or volunteer may be a perpetrator. Understanding the signs of child abuse is critical in supporting children's safety and wellbeing.

Victoria State Government-Child protection in early childhood PROTECT provide definitions and physical indicators: [Identify signs of child abuse](#)

Child Safe Standards: Victoria's mandatory Child Safe Standards (the Standards) have been in effect since 2016. Following the Royal Commission, the Victorian Government reviewed the Standards. The review found strong support for the Standards and recommended a number of changes to better align the Standards with the National Principles for Child Safe Organisations, and to strengthen administration of the Standards. In line with these recommendations, the new Standards were released by the Victorian Government in 2021. They consist of 11 updated Standards and apply from 1 July 2022.

Approved services - National Quality Framework: Approved early childhood services (that is services operating under the *Education and Care Services National Law Act 2010*) include any service providing or intending to provide education and care on a regular basis to children under the age of 13 years. Every approved service **must** meet the requirements of the National Quality Framework and associated regulatory system. Most notably, it is a legal requirement that **every reasonable precaution must be taken to protect children from harm and any hazard likely to cause injury** within approved early childhood services. Failure to meet this requirement can amount to a criminal offence.

Code of Conduct: Our service has a Child Safe Code of Conduct. Staff, volunteers and the Management team must comply with the Code of Conduct at all times. Breaches of the Code of Conduct may result in disciplinary action including termination of a person's involvement with the organisation. All third-party contractors are also expected to abide by the Child Safe Code of Conduct.

Please refer to Ethical Conduct and Code of Ethics Policy.

Inappropriate conduct to a child: any behaviour that a reasonable person would consider to be inappropriate in an education and care service, taking into account the child's age, development, safety, wellbeing and the context of the interaction.

It includes behaviour that is:

- inconsistent with professional standards
- causes or is likely to cause emotional, psychological, or physical harm
- or has violent or sexual connotations.

A child may be subjected to inappropriate conduct – either directly or indirectly as a witness – that occurs through:

- through verbal or electronic communication
- by electronic capture, including photographs, recordings or live video
- repeated actions or ongoing patterns of behaviour over time.

A child's consent – or an adult's belief that a child has consented to the behaviour or their intent – is irrelevant for the purposes of determining an offence under section 166A.

Examples of inappropriate conduct

- Inappropriate physical contact – touching a child in a sexualised or intrusive way, such as tickling, wrestling, prolonged or unnecessary hugging that is not warranted in the circumstances, kissing a child on the cheek, massaging, or physical closeness (such as encouraging a child sit on an educator's lap for an extended period) that is not age appropriate or required based on the needs or abilities of a child.
- Grooming behaviours – including but not limited to favouritism, offering gifts or special privileges, encouraging emotional dependency (attachment fostering), or initiating private or secret communication with a child.
- Ill treatment that is not disciplinary in nature – including physical or verbal abuse, threats, yelling, swearing, rough handling or other conduct likely to cause emotional or psychological harm.
- Unprofessional communication – such as in-person conversations, sending personal messages, capturing or sharing images of children via unauthorised devices or platforms, or engaging with children online through social media or apps unrelated to your role.

Investigation: A formal and systematic inquiry to establish facts about the complaint by collecting, documenting, examining and evaluating evidence.

Related Policies

Families and Visitor Code of Conduct

Inclusion Policy

Child Safety, Protection & Wellbeing Policy

Safeguarding Children: Look, Do & Tell Framework Policy

Education, Curriculum and Learning Policy

Ethical Conduct and Code of Ethics Policy

Governance of Service, Management and Educator Policy

Smoke, Alcohol & Drug Free Environment Policy

Excursion Policy

Family Violence Safety Policy

Complaints and Grievance Policy

Incident, Injury, Trauma and Illness Policy

Parent Involvement Policy

Policy and Procedure Review Policy

Privacy and Confidentiality Policy

Employment Policy

Interactions with Children Policy

Staffing Arrangements Policy

Technology & Digital Usage Policy

Internet, Email & Social Networking Policy

National Model Code Policy

Implementation

All managers, staff and volunteers are committed to implementing the Child Safe Standards. The safety, health and wellbeing of children is the number one priority of all staff and volunteers who understand children's safety is a shared responsibility. We have zero tolerance for any form of harm to children and are committed to acting in children's best interests. Our policies and procedures support and inform this commitment, and our leaders and managers regularly review staff and volunteer practices and understanding, prioritising training or taking other relevant action if required. Our policies and procedures also support and comply with the Education and Care National Law and Regulations, and the National Quality Standard (NQS), whose guiding objective and principles outlined in section 3 include ensuring "the safety, health and wellbeing of children attending education and care services", "the rights and best interest of children are paramount", the principles of equity, inclusion and diversity underlie this Law", "that Australia's Aboriginal and Torres Strait Islander cultures are valued", and "that the role of parents and families is respected and supported." This objectives and principles are embedded in many of the Laws and Regulations which we must comply with. The Laws, Regulations and NQS elements which are particularly relevant to child safety are listed above.

Below we discuss our Service's approach to the eleven Child Safe Standards, and outline Service policies or practices which support them (Standard 11 Policies and procedures document how the organisation is safe for children and young people.) Our contracts of employment require all employees to comply with service policies and procedures.

Standard 1: Organisations establish a culturally safe environment in which the diverse and unique identities and experiences of Aboriginal children and young people are respected and valued

Our service will:

- encourage and actively support a child's ability to express their culture and enjoy their cultural rights
- embed strategies that equip all staff, volunteers and children to acknowledge and appreciate the strengths of Aboriginal culture and understand its importance to the wellbeing and safety of Aboriginal children and young people
- not tolerate racism, adopt measures to identify and confront racism, and address instances of racism with appropriate consequences
- actively support and facilitate Aboriginal children and their families' participation and inclusion
- ensure our policies, procedures, systems, and processes together create a culturally safe and inclusive environment and meet the needs of Aboriginal children and their families.

It is the right of every Aboriginal child to be immersed in their culture in a way that allows them to feel their identity is valued and respected. Culture includes community and family connections, spiritual and material relationships with lands and waters that hold significance under traditional laws and customs, and languages, dance, ceremony and heritage.

Educators are required to:

- promote learning outcomes in the approved learning framework which include outcomes related to "Children have a Strong Sense of Identity," and Principles which include:
 - 'Respect for diversity'** which includes *"promoting greater understanding of Aboriginal and Torres Strait Islander ways of knowing and being."*
 - 'Ongoing Learning and Practice'** where educators *"become co-learners with children, families and community, and value the continuity and richness of local knowledge shared by community members, including Aboriginal and Torres Strait Islander Elders."*
 - 'Responsiveness to Children'** which includes educators *"respond(ing) to children's expertise, cultural traditions and ways of knowing, the multiple languages spoken by some children, particularly Aboriginal and Torres Strait Islander children"* (Belonging Being & Becoming The Early Years Learning Framework)

- meet the NQS Principle about valuing Australia's Aboriginal and Torres Strait Islander Cultures." Many of the indicators for meeting and exceeding the 40 NQS elements reference the need to consider and respond to First Nations' identities, culture, language and history in a local community and national context.

Specific actions we take to embed a culturally safe environment for First Nations children include: development of Reconciliation Action Plan - with support from Narragunnawali, Acknowledgement of Country, training, developing strong relationships with and encouraging visits from local Aboriginal organisations, Elders and families, employing Aboriginal staff, using teaching strategies that intentionally promote First Nations' culture, heritage and history, learning about key community events (including National Sorry Day, National Reconciliation Week and NAIDOC Week), physical environments and learning displays which include local First Nation symbols, images and objects.

Our service support and is committed to have a cultural safe environment. Cultural safety is described as 'the positive recognition and celebration of cultures. It is more than just the absence of racism or discrimination and more than 'cultural awareness' and 'cultural sensitivity'. It empowers people and enables them to contribute and feel safe to be themselves. Cultural safety for Aboriginal children has been defined as 'the child being provided with a safe, nurturing and positive environment where they are comfortable with being themselves, expressing their culture... their spiritual and belief systems, and they are supported by the carer... (who) respects their Aboriginality and therefore encourages their sense of self and identity.

For further information please see [Cultural safety for Aboriginal Children](#) and [A Guide for Creating a Child Safe Organisation](#)

Service policies which support this Standard include our:

- **12B - Child Safety, Protection & Wellbeing Policy**
- **12C – Safeguarding Children: Look, Do & Tell Framework**
- **Education, Curriculum and Learning Policy** which requires educators implement an educational program which includes each child's culture, language and everyday lives
- **Governance of Service, Management and Educator Policy** which makes it clear any form of racism will breach our Code of Conduct
- **Complaints and Grievance Policy** which highlights our Service takes instances or allegations of discrimination very seriously
- **Parental Involvement Policy** which encourages families to share their culture and experiences with educators and the children
- **Interaction with Children Policy** which requires educators, in line with the approved learning framework, support and implement an anti-bias, cross cultural program throughout the Service and intentionally teach children about avoiding stereotypes and the benefits of diversity
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 2: Child safety and wellbeing is embedded in organisational leadership, governance and culture

Our service will:

- make a public commitment to child safety
- champion and model a child-safe culture
- facilitate the implementation of our Child Safety Standards Policy and Child Safety, Protection & Wellbeing Policy
- reiterate our Code of Conduct, which provides guidelines for staff and volunteers on expected behavioural standards and responsibilities

- continue to implement risk-management strategies that focus on preventing, identifying, and mitigating risks to children
- ensure staff and volunteers understand their obligations on information-sharing and record keeping.

We strongly support a child safe organisational culture which drives the way things are done and how issues and risks are managed. Strong, transparent and accountable leadership can deter perpetrators of harm, and ensure employees understand and comply with their obligations to report suspicions or disclosures of harm. Our managers and leaders regularly review practices to ensure ongoing compliance with Service policies and procedures, and to promote a child safe culture.

All paid and unpaid staff (including volunteers, students on placement, trainees) and parents, family members and members of the service community of are responsible for the safety and wellbeing of children and young people who engage with the service. All people of the service community are expected to act in accordance with this Child Safe Code of Conduct in their physical and online interactions with children and young people under the age of 18 years.	
WILL:	• Act in accordance with the organisation's Child Safety and Protection Policies and procedures at all times. • Approved Provider will notify the Regulatory authority of any incidents or allegations of physical or sexual abuse to a child while being educated or cared for by the Service within 24 hours. • Behave respectfully, courteously and ethically towards children and their families and towards other staff. • Listen and respond to the views and concerns of children, particularly if they communicate (verbally or non-verbally) that they do not feel safe or well. • Promote the human rights, safety and wellbeing of all children in the organisation. • Demonstrate appropriate personal and professional boundaries. • Consider and respect the diverse backgrounds and needs of children. • Create an environment that promotes and enables children's participation and is welcoming, culturally safe and inclusive for all children and their families. • Involve children in making decisions about activities, policies and processes that concern them wherever possible. • Contribute, where appropriate, to the organisation's policies, discussions, learning and reviews about child safety and wellbeing. • Identify and mitigate risks to children's safety and wellbeing as required by the organisation's risk assessment and management policy or process. • Respond to any concerns or complaints of child harm or abuse promptly and in line with the organisation's policy and procedure for receiving and responding to complaints. • Report all suspected or disclosed child harm or abuse as required by the legislation and by policy and procedure on internal and external reporting. • Comply with the organisation's protocols on communicating with children. • Comply with regulation and the organisation's policies and procedures on record keeping and information sharing.
WILL NOT:	• Engage in any unlawful activity with or in relation to a child. • Engage in any activity that is likely to physically, sexually or emotionally harm a child. • Unlawfully discriminate against any child or their family members. • Be alone with a child unnecessarily. • Arrange personal contact, including online contact, with children I am working with for a purpose unrelated to the organisation's activities. • Disclose personal or sensitive information about a child, including images of a child, unless the child and their parent or legal guardian consent or unless I am required to do so by the organisation's policy and procedure on reporting. • Use inappropriate language in the presence of children, or show or provide children with access to inappropriate images or material. • Work with children while under the influence of alcohol or prohibited drugs. • Ignore or disregard any suspected or disclosed child harm or abuse.

If you believe the Child Safe Code of Conduct has been breached by another person in the organisation, you will:	• Act to prioritise the best interests of children. • Take actions promptly to ensure that children are safe. • Promptly report any concerns per the policy and procedure. • Follow the organisation's policies and procedures for receiving and responding to complaints and concerns. • Comply with legislative requirements on reporting, and with the organisation's policy and procedure on internal and external reporting.
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Service policies which support this Standard include our:

- Promotes a child safe environment while complying with the Australian Privacy Principles
- **12B - Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy** which clearly outlines the responsibilities and obligations of all employees and volunteers under State child protection legislation. It also outlines our Child Protection Risk Management Strategy, including our Code of Conduct, recruitment procedures which ensure potential employees fully understand and support the child safe Service culture, procedures for managing disclosures or suspicions of harm and breaches of the Strategy, the Reportable Conduct Scheme, and risk management for high risk activities
- **Child Safety Standards Policy** which, in addition to the Child Safe Standards, also includes our obligations under the Child Information Sharing Scheme legislated in the Child Wellbeing and Safety Act 2005 to share information with other entities to promote the safety or wellbeing of a child. These are discussed after Standard 11
- **Governance of Service, Management and Educator Policy** and **Ethical Conduct and Code of Ethics Policy** which includes our Code of Conduct and Management Responsibilities. *The Code* includes clear expectations of appropriate interactions with children, and examples of appropriate and inappropriate interactions, and requires compliance with all Service policies and procedures. The Code also guides the behaviour of families and visitors, indicating they will not have physical contact with other children at the service unless a staff member is present. *Management responsibilities* include ensuring staff meet qualification requirements, mandated educator to child ratios are maintained, all staff understand their responsibilities under the National Law, Regulations and NQS, all staff, visitors and volunteers comply with the Code of Conduct, organising or providing appropriate staff training and performance reviews, and regularly implementing documented staff meetings
- **Family Violence Safety Policy** which implements our obligations under the Family Violence Sharing Scheme legislated in the Family Violence Protection Act 2008 to share information with other entities if we reasonably believe this will help manage an established risk of a perpetrator committing family violence, or an established risk of a victim survivor being subjected to family violence
- **Governance Policy** which includes the Principle “recognise and manage risk to children.” This Principle is evident in the regular risk assessments completed by educators and staff as they assess and manage the potential risk of harm to children during activities and events, and in the regular formal and informal training educators and staff receive to refresh their skills and understanding of practices that ensure the safety of children including but not limited to child protection and child safe practices such as adequate supervision, managing incidents and complaints, privacy requirements, implementing risk assessments, and safe excursion and transport practices
- **Complaints and Grievance Policy** which includes our complaint handling procedures and guidelines, including potential outcomes
- **Incident, Injury, Trauma and Illness Policy** which requires staff maintain records that enable us to monitor, review and report incidents as required under the National Law (see also Standard 7)

- **Privacy and Confidentiality Policy** which outlines how our practices are consistent with the Australian Privacy Principles
- **Interactions with Children Policy** which covers positive interactions with children by educators, staff and other children (in cases of bullying for example)
- **Staffing Arrangements Policy** which includes procedures for supervising children to ensure their safety and remove any potential for harm to children, and requirements for clear Working With Children Checks.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Families and Visitors Code of Conduct**

Our Child Safety Standards Policy is available to all families in our policy folder, as part of our regular policy review program.

Standard 3: Children and young people are empowered about their rights, participate in decisions affecting them and are taken seriously

Our service will:

- inform children about their rights, including to safety, information, and participation
- deliver an educational program based on an approved learning framework and based on the developmental needs, interests and experiences of each child. This includes covering topics such as Child Safety.
- provide an environment for children that promotes their agency (element 1.2.1), maintains respectful and equitable relationships (regulation 156) with each child and allows children to feel secure and confident.
- we provide education and care in a way that maintains at all times the dignity and rights of each child and has regard to the family and cultural values, age and physical and intellectual development of each child (regulation 155).
- recognise the importance of friendships and encourage support from peers to help children feel safe and less isolated
- offer access to sexual abuse prevention programs and relevant related information in an age-appropriate way
- establish an environment of trust and inclusion that enables children to ask questions and speak up if they are worried or feeling unsafe. We understand that children often do not report abuse because they feel uncomfortable or they do not know how to raise their concerns or allegations of abuse.
- attune staff and volunteers to signs of harm and facilitate child-friendly ways for children to express their views, participate in decision-making, and raise their concerns
- put strategies in place to develop a culture that facilitates participation and is responsive to the input of children
- provide opportunities for children to participate
- respond to their contributions to strengthen confidence and engagement.

Educators promote a culture of participation and inclusion, viewing all children as active participants and decision makers as they seek to include the interests, ideas, strengths, culture, abilities etc of each child. They understand this is an essential part of effectively implementing the NQS and the Early Years Learning Framework (EYLF) which promote each child's agency, perspective and participation, and require that educators are responsive to each child's ideas, interests and needs. See for example NQS Elements 1.1.2 Child-centred, 1.2.3 Child directed learning and 5.2.1 Collaborative learning (where children are supported to learn from and help each other), EYLF Principle 'Secure, respectful and reciprocal relationships' and EYLF Practice 'Responsiveness to Children' which promote respectful and reciprocal relationships with children based on deep understanding of their interests, community and culture.

As part of this process, educators regularly empower children to take part in discussions about their safety and decisions that affect them, and value their ideas, suggestions and feedback. Educators are attuned to babies' and younger children's non-verbal cues to plan curriculum and respond in ways that empowers them. Our Educational Leader regularly reviews educators' practices and supports them to continually improve.

Educators will regularly include child protection issues in the curriculum. For example they will intentionally teach children:

- our service philosophy and educational program promotes and supports child safe environments through empowering children (promoting child agency), increasing children's awareness and providing them the strategies, tools and support in relation to Child Safety.
- age-appropriate resources for children and families on children's rights.
- child-friendly information and communications about personal safety, and how to voice concerns about personal safety.

- provide a safe environment for children to express concerns where they will be taken seriously and not judged. We have a Wellbeing Program that supports all children's wellbeing, health and safety.
- about acceptable/unacceptable behaviour, and appropriate/inappropriate contact in a manner suitable to their age and level of understanding
- that they have a right to feel safe at all times
- to say 'no' to anything that makes them feel unsafe
- the difference between 'fun' scared that is appropriate risk taking and dangerous scared that is not ok
- to use their own skills to feel safe
- to recognise signs that they do not feel safe and need to be alert and think clearly
- that there is no secret too awful, no story too terrible, that they can't share with someone they trust
- that educators are available for them if they have any concerns
- to tell educators of any suspicious activities or people
- to recognise and express their feelings verbally and non-verbally
- that they can choose to change the way they are feeling.

Educators believe that:

- children are capable of the same range of emotions as adults
- children's emotions are real and need to be accepted by adults
- an adult's response to a child during their early emotional development can be hugely positive or detrimental depending on the adult's reaction
- children are very in touch with their bodies' reactions to their emotions
- children who better understand their body's response to an emotion are more able to foresee the outcome of a situation and avoid them or ask for help.

Service policies which support this Standard include our:

- **Child Safety, Protection & Wellbeing Policy and 12C – Safeguarding Children: Look, Do & Tell Framework Policy** which requires educators, as part of the 'Strategies for Communication and Support' component of the Child Protection Risk Management Strategy, to regularly include age appropriate learning about the way adults should behave from a child safety perspective, including their right to make decisions about their body and privacy, that they have a right to feel safe at all times, how to keep themselves safe, and what to do if they feel unsafe
- **Education, Curriculum and Learning Policy** which outlines practices which implement the EYLF, including regularly involving children in decision making about their learning and environment as part of the assessment and planning cycle
- **Interactions with Children Policy** which contains detailed guidelines and examples about children's inclusion, agency and peer support. The Policy also contains information and example of appropriate Behaviour Guidance strategies which support children to express and meet their needs.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

We also display the United Nations Convention on the Rights of the Child.

Standard 4: Families and communities are informed, and involved in promoting child safety and wellbeing

Our service will ensure:

- families participate in decisions affecting their child
- we engage and openly communicate with families about our child safe approach and that relevant information is accessible
- families contribute to the development and review of our policies and practices
- we inform families and carers about our operations and governance.

Educators constantly build partnerships with families and community members, which provides many opportunities for discussion and feedback about safety practices. Educators and staff regularly encourage families to contribute to decisions about Service practices as well as their child's learning through questionnaires, information nights, face to face conversations and our 'Open Door Policy.' Families know they can locate Service policies and procedures, and asked for feedback when we review our policies. Providing information about service operations is also a requirement under NQS Element 6.1.3 'Families are supported.'

Child safe information is provided to families in various formats including displays and brochures, and is discussed in more detail with families the week our professional development covers element 2.2.3 Child Protection. Several Policies and our Code of Conduct are also provided to in our families upon enrolment and upon request.

We note this is consistent with our obligations under the guiding principles in the National Law section 3(3) to respect and support the role of parents and families.

Our response to child safety, including in cases of harm, suspected harm or complaints, is also culturally sensitive. For example we engage with families and/or community members to improve our understanding of cultural behaviours and inform our responses.

Service policies which support this Standard include our:

- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy** as discussed above
- **Ethical Conduct and Code of Ethics Policy** which includes our Code of Conduct as discussed above
- **Governance of Service, Management and Educator Policy** which includes our Service structure and identifies the people in leadership and management positions
- **Complaints and Grievance Policy** which includes our complaint guidelines and the name of our Complaints Officer. We include this Policy in our enrolment information.
- **Parental Involvement Policy** which highlights our 'Open Door' approach to facilitating transparent, two-way communication. We have an **Open Door Policy**.
- **Policy and Procedure Review Policy** which outlines how and when families will be encouraged to provide feedback on policies and procedures.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 5: Equity is upheld and diverse needs respected in policy and practice

Our service:

- understands children come from diverse circumstances, and supports each child and their family in meaningful and appropriate ways
- provides children with access to information, support and complaints processes in ways that are culturally safe, accessible and easy to understand
- pays particular attention to children with additional needs and children who are unable to live at home
- pays particular attention to the needs of Aboriginal children and provides/promotes a culturally safe environment for them.

Educators regularly plan and implement learning which promotes the benefits of cultural diversity, and the unique abilities and strengths each person has regardless of any additional needs, personal preferences or orientations, or family structure. Educators focus daily on ensuring *each child* can reach their potential in an inclusive environment which supports their participation, identity, connection to their world and sense of wellbeing. These practices underpin the NQS and the EYLF which promote equity, diversity and inclusion. See for example NQS Element 3.2.1 Inclusive Environment and Standard 5.1 Respectful and Equitable Relationships are maintained with each Child, as well as EYLF Principle High Expectations and Equity and EYLF Practice Cultural Competence. Educators also use the indicators under the EYLF Learning Outcomes to help identify children with vulnerabilities and additional needs.

In addition, where possible our staff profile reflects the diversity at our Service and in the local community, as this assists in building relationships with children and families, and in building the cultural competence of all educators and staff.

Service policies which support this Standard include our:

- **Inclusion Policy** which discusses how we design or adapt the environment to ensure each child can participate and achieve meaningful learning outcomes, and how we work with external professionals, families and children to help achieve this outcome
- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy** which includes the indicators of harm, the practices to effectively manage a suspicion or disclosure of harm, including where children may be unwilling to discuss this, and regular activities and experiences which teach children about their right to privacy, safety and who to approach for help if they don't feel safe
- **Education, Curriculum and Learning Policy** which outlines additional strategies educators use to promote each child's inclusion and participation, including establishing links with local cultural groups and using intentional teaching strategies to promote cultural customs, stories, traditions, history, inviting families and community members to visit and complete activities with children
- **Interactions with Children Policy** which covers inclusive practices as outlined in Standard 3 and also outlines practices to help support children through traumatic events
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 6: People working with children and young people are suitable and supported to reflect child safety and wellbeing values in practice

Our managers will ensure:

- recruitment, including advertising, referee checks and staff and volunteer pre-employment screening, emphasise child safety and wellbeing
- relevant staff and volunteers have current working with children checks or equivalent background checks
- all staff and volunteers receive an appropriate induction and are aware of their responsibilities to children, including record keeping, information sharing and reporting obligations
- we focus ongoing supervision and people management on child safety and wellbeing.

Our managers do not rely solely on clear Working with Children Checks, VIT registration Verification and Review of Register of Prohibited Persons & Suspended Educators. Our human resource practices support the recruitment and ongoing management of employees who implement strong child safe practices every day. For example during our recruitment we discuss our Statement of Commitment to Child Safety, interview questions establish the suitability of staff, referees are contacted, our Induction process requires all staff, including volunteers, to review and acknowledge in writing our Child Safety, Protection & Wellbeing Policy and Code of Conduct, job descriptions set clear expectations about child safety, and we implement probationary periods. The Nominated Supervisor also implements an ongoing training program tailored to each staff member's needs and goals which are identified through regular performance reviews.

Our employee and volunteer induction processes include reviewing child safe policies, procedures and expectations, and educators and staff review policies every month as part of their professional development. Volunteers are required to comply with all service policies and procedures and code of conduct.

Information on all employees in the Service who have direct contact with children is recorded in the **National Early Childhood Worker Register (Worker Register)** and the **Victorian Arrival Workforce Register** (Victorian register use ended 27 March 2026 on introduction of National Workforce Register). Please see 52 – Employment Policy for further details.

Service policies which support this Standard include our:

- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy** which includes Recruitment practices that clearly describe the expectations for employees to provide a child safe environment
- **Governance of Service, Management and Educator Policy** which, in line with NQS Element 7.2.3 Development of professionals, requires the performance of educators and staff to be regularly evaluated against their position description, and training plans devised and implemented where there is an identified need, including for example in child safe practices. Training may occur through a recognised face to face or on-line training course, mentoring and at staff meetings. The Nominated Supervisor must include training funds in the annual budget. The Policy also outlines orientation practices to ensure new or returning staff are aware of current service policies and procedures, including those ensuring they meet their child protection obligations
- **Employment Policy** which contains screening processes, use of position descriptions and interviews
- **Staffing Arrangements Policy** which contains qualification requirements, including those consistent with the Worker Screening Act 2020, and Supervision procedures with a child safety focus
- **Smoke, Alcohol & Drug Free Environment Policy** which clearly states educators and staff must not drink alcohol or take drugs at the Service, and must not attend if adversely affected by alcohol or drugs, including prescription medication.
- **National Model Code Policy**

- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 7: Processes for complaints and concerns are child-focused

Our service:

- has an accessible, child-focused complaint handling policy that clearly outlines the roles and responsibilities of leadership, staff and volunteers, approaches to dealing with different types of complaints, breaches of relevant policies or the Code of Conduct and obligations to act and report
- has effective complaint handling processes that are culturally safe, and that children, families, and staff understand
- takes complaints seriously and responds promptly and thoroughly
- has policies and procedures in place that address reporting of complaints and concerns to relevant authorities, whether the law requires reporting, and co-operates with law enforcement
- meets reporting, privacy, and employment law obligations.

Complaints are taken very seriously at our Service, and children's safety and wellbeing is always our number one priority. Our child safety practices emphasise a child focus rather than focusing on adult intent, and as mentioned under Standard 4, we engage with families and/or community members to respond in culturally sensitive ways where appropriate.

Approved Provider will notify the Regulatory authority of any incidents or allegations of physical or sexual abuse to a child while being educated or cared for by the Service within 24 hours.

With reference to Standard 7, the terms 'complaints' and 'concerns' should be interpreted broadly. A concern is any potential issue that could impact negatively on the safety and wellbeing of children. A complaint can include expressions of dissatisfaction about an organisation related to one or more of the following:

- the organisation's services or dealings with individuals
- allegations of abuse or misconduct by a staff member, a volunteer or another individual associated with the organisation
- disclosures of abuse or harm made by a child
- the conduct of a child at the organisation
- the inadequate handling of a prior concern
- general concerns about the safety of a group of children or activity.

All reports of child abuse and child safety concerns will be treated seriously, whether they are made by an adult or a child and whether they are about the conduct of an adult or a child. All complaints and child safety concerns will be responded to promptly and thoroughly. Approved Provider will notify the Regulatory authority of any incidents or allegations of physical or sexual abuse to a child while being educated or cared for by the Service within 24 hours.

As noted we have a complaint handling policy that includes information for families, staff and volunteers about how a complaint or child safety concern will be responded to. An easy-to-understand complaints information sheet will be provided for children, families and the community to know about the complaint process and the supports available to those making a complaint and those involved in the complaint process.

If a complaint includes an allegation or incident of child abuse or harm, then staff and volunteers must report it in accordance with the complaint and grievance policy and the "**Complaint Handling Guide: Upholding the rights of children and young people**". Staff and volunteers are required to prioritise children's safety in any response and to report all potentially criminal conduct to Victoria Police. Under

the complaint handling and disciplinary policies, staff and volunteers may be subject to actions to support child safety including:

- being stood down during an investigation or terminated following an investigation
- having their duties altered so they do not engage with children
- not allowing unsupervised contact with children
- removing their access to the IT system and facilities.

Complaints can be emailed to the service or Approved Provider or you can speak with Approved Provider Centre Director or Responsible Person in Charge.

If there is concern for the immediate safety of a child, immediately call 000.

Service policies which support this Standard include our:

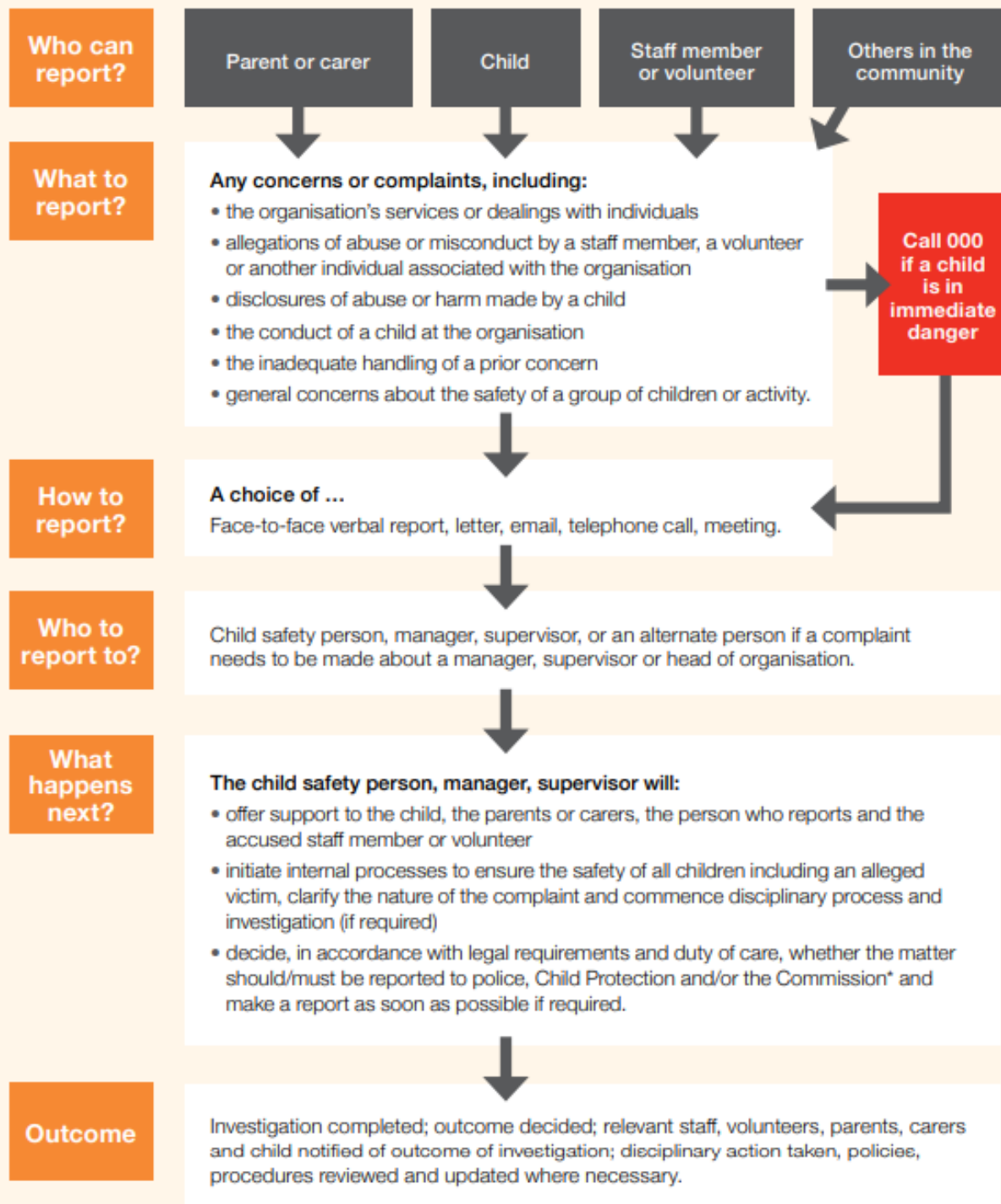
- **Child Safety, Protection & Wellbeing Policy and 12C – Safeguarding Children: Look, Do & Tell Framework Policy** which contains a clear step by step procedure for reporting harm or suspected harm, for example to Police, Child Protection, the ECEC Regulatory Authority and Social Services Regulator (SSR) (under the Reportable Conduct Scheme). It also includes information about referrals to family support services
Where staff are the subject of child safety complaints, the Approved Provider or Nominated Supervisor must review their duties, decide if it's safe for them to continue working with children while the complaint is investigated, and if so ensure they're appropriately supervised at all times
- **Compliant and Grievance Policy** which contains clear and transparent investigation guidelines including time frames, review processes, privacy and confidentiality requirements and potential outcomes of complaints including disciplinary action. The Policy clearly requires educators and staff support children to access the complaints process for all complaints alleging their safety or wellbeing is or could be harmed, if not already implementing the procedures in the Child Safety, Protection & Wellbeing Policy. In line with the Policy and guidelines we also maintain a Complaint Register and regularly review the types, content and outcome of complaints to identify any patterns and underlying issues.

Please refer to the “**Complaint Handling Guide: Upholding the rights of children and young people**” for guidance on how to handle complaints in relation to Child Safe related matters (<https://chidsafety.pmc.gov.au/resources/complaint-handling-guide-upholding-rights-children-and-young-people>)

- **Incident, Injury, Trauma and Illness Policy** which includes requirements under the National Law and Regulations to document and promptly notify the Regulatory Authority any ‘Serious Incidents,’ complaints alleging a Serious Incident has occurred, complaints that the National Law or Regulations have been breached, any circumstances arising at the Service that pose a risk to the health, safety and wellbeing of a child, and of any incident, complaint or allegation that physical or sexual abuse of a child has occurred.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Flowchart: child safety reporting process

This chart outlines a process for reporting to an organisation.



* Applies only to organisations covered by the Reportable Conduct Scheme. Refer to the Commission's website for details.

(Source: A guide for creating a Child Safe Organisation, Page116)

Standard 8: Staff and volunteers are equipped with the knowledge, skills and awareness to keep children and young people safe through ongoing education and training

We train and support our staff and volunteers to:

- effectively implement our Child Safety Standards Policy and Child Safety, Protection & Wellbeing Policy
- recognise indicators of child harm including harm caused by other children and young people
- respond effectively to issues of child safety and wellbeing and support colleagues who disclose harm
- build culturally safe environments for children.

Our Service is committed to a culture of ongoing learning and continuous improvement (as required under NQS element 7.2.1). This includes a commitment to ensuring all educators and staff know how to identify and respond to all forms of child harm, and are aware of current child safe knowledge and practices.

Service managers, for example, ensure they and all educators and staff participate in annual refresher training on their child safe obligations, the importance of taking a child-focused approach, the indicators of harm and patterns of behaviour that indicate a risk to children, record keeping and reporting procedures, and Service child safe practices, policies and resources. Training may be provided in-house, or by external providers where appropriate. Child safety is also regularly included on staff meeting agendas. Child safe training always includes case studies and examples to support the development of practical skills and 'real-life' child safe responses.

Recruitment, Selection and Training Procedures

The Approved Provider, Nominated Supervisor, Responsible Person and organisation will implement recruitment, professional development and training procedures for employees and where relevant volunteers to ensure no-one at the service poses a risk to children and everyone understands how to manage disclosures or suspicions of harm (Recruitment procedures at Appendix and training procedures in Educator and Management Policy). Requirements include:

- job advertisements which include qualifications and skills required, and culture of child safety and protection
- job descriptions which outline level of professional skills and responsibilities
- The organisation seeks to have a culturally diverse workplace; embracing different culture and backgrounds. This includes promoting applicants from Aboriginal and /or people from culturally and/or linguistically diverse backgrounds.
- processes (including job advertisements) which ensure employees and volunteers have clear Working With Children Checks or they are exempt (see <http://www.workingwithchildren.vic.gov.au/>)
- interview questions and referee checks which reference person's approach to child safety and protection
- documented induction/ orientation checklists which reference child safety and protection, supervision, compliance with National Law and Regulations, NQS, Code of Conduct, policies and procedures
- annual training and development to ensure individuals are clear about their roles and responsibilities to protect children from harm, are aware of their reporting obligations, can confidently recognise the indicators of harm (see Appendix) and understand documenting and reporting procedures.
- all educators, staff, volunteers and students have completed mandatory National Child Safety Training (from 27 February 2026)
- annual performance appraisals for employees
- regular inclusion of child protection and risk management strategy at least every 6 months in staff meetings and annual review of written training plans which must include Child Protection matters (eg disclosures and suspicions of harm)

- providing access to relevant legislation and other resources to help employees and volunteers meet their obligations

Service policies which support this Standard include, as previously discussed, our:

- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy** (see all Standards)
- **Child Safety Standards Policy** (see Standard 2)
- **Governance of Service, Management and Educator Policy** (see Standard 6)
- **Family Violence Safety Policy** (see Standard 2)
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 9: Physical and online environments promote safety and wellbeing while minimising the opportunity for children and young people to be harmed

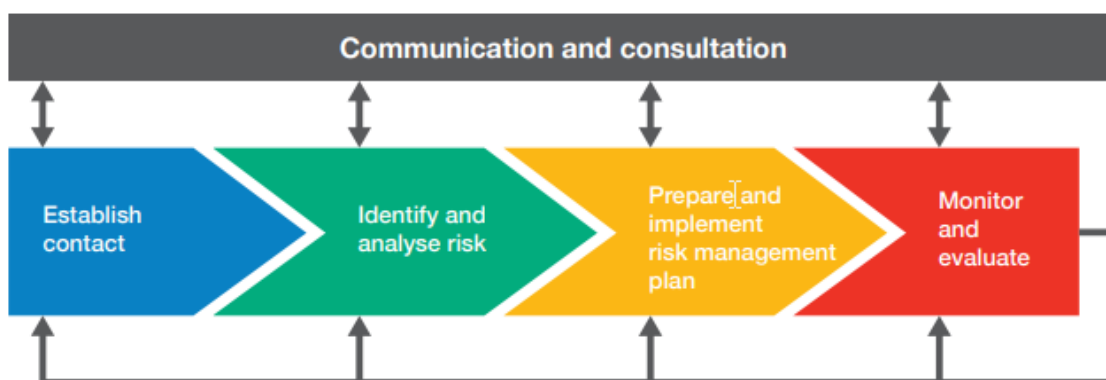
We will ensure:

- Staff and volunteers identify and mitigate risks in the online and physical environments without compromising a child's right to privacy, access to information, social connections and learning opportunities
- we use our online environment according to our Code of Conduct , Technology Usage Policy Child Safety Standards Policy and practices
- our risk management plans consider our settings, activities and the physical environment
- if we contract facilities and services from third parties, our procurement policies ensure the safety of children.

We comply with child safety requirements under the National Education and Care Law and Regulations (refer those listed at the front of this policy). For example, Law section 165 'Offence relating to protection of children from harm and hazards' requires all employees and volunteers to take a risk management approach to safety, and Regulation 168(2)(h) requires services to have Policies relating to providing a child safe environment. ACECQA has indicated in its draft Child Safe Policy Guideline this covers supervision, staffing, child protection and the physical environment. Employees and volunteers regularly complete risk assessments prior to implementing activities or events that could potentially expose children to harm from a child protection and environmental perspective. Risk assessments always involve identifying and assessing risks, implementing measures to eliminate risks or to reduce them to acceptable levels, and ongoing monitoring. There are also the notification requirements outlined in our Incident, Injury, Trauma and Illness Policy to promptly notify the Regulatory Authority of child safety issues which we discussed under Standard 7.

Managers, staff and educators understand the physical environment can provide opportunities for harm to occur and implement supervision practices that reduce this risk. Where possible, physical environments are altered to increase natural lines of sight while respecting a child's right to privacy, and their need for risky play which allows them to temporarily 'disappear' (refer Sandseter's categories of risky play.) There is an increased focus on higher-risk locations including nappy change areas, and during events, transitions and excursions. This is reflected in our risk assessments, including those required under the National Regulations prior to conducting excursions or transporting children.

We also consider and protect children's safety in the online environment in an age appropriate way. Educators may discuss with younger children, for example, how there's no way to be sure who they're interacting with online. With older school age children, educators may cover risks like sharing intimate images, cyberbullying, and meeting in person with online contacts. If needed, staff and volunteers are provided with training on online risks to children, and families are provided with relevant information to support their parenting and children's wellbeing in line with NQS Element 6.1.3.



(Source: A guide for creating a Child Safe Organisation: Risk Management Process, page 143)

Service policies which support this Standard include our:

- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy**
- **Governance of Service, Management and Educator Policy** and **Ethical Conduct and Code of Ethics Policy** which includes our Code of Conduct and the requirements when visitors attend the service. These include ensuring visitors are never left alone with children, and have a clear child protection clearance (unless exempt)
- **Incident, Injury, Trauma and Illness Policy** as discussed under Standard 7
- **Photography consent** (in enrolment form) which promotes children's privacy and safety by requiring parents to consent before photos/videos are taken of their child, and before these are posted on any social media platforms, websites or used in service publications
- **Technology & Digital Usage Policy** and **Internet and Internet, Email & Social Networking Policy** which also promotes children's privacy and safety by not permitting educators and staff from using personal cameras or phones to take photos/video at the service, and from posting children's private information, photos or videos to their personal social media accounts.
- **Staffing Arrangements Policy** which discusses issues affecting the adequacy of supervision (these include the visibility and accessibility of areas where children are playing), supervision practices including the need for educators to position themselves to see as much of the play area as possible and to follow any playground supervision plans, particular activities that require focused supervision (including in bathrooms/toilets and higher risk activities). Other practices which support Regulation requirements include ensuring educator to child ratios are always met and educators under 18 are always supervised
- **Technology & Digital Usage Policy** and **Internet, Email & Social Networking Policy** which requires devices only be used to support children's learning, only age appropriate websites are accessed, and that educators directly supervise children when they're on-line.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 10: Implementation of the Child Safe Standards is regularly reviewed and improved
Our Service:

- regularly reviews, evaluates and improves child safe practices
- analyses complaints, concern, and safety incidents to identify causes and systemic failures to inform continuous improvement
- reports on the findings of relevant reviews to staff, volunteers and families, and children where appropriate.

We are committed to a culture of continuous improvement, and we strive to maintain an Exceeding rating under the NQS. We continually review our child safe practices and policies, and review any incidents and complaints to identify systemic causes which we address where possible. Regular training to embed child safety practices, and identify emerging trends and knowledge is a priority (reflected in training plans), and where relevant we seek a specialist external provider to facilitate this.

To support our continuous improvement, all managers, educators and staff also regularly engage in documented critical reflection of Service practices and policies, interactions with children and families, and children's learning and development. This involves considering issues from multiple perspectives including those of children, families, colleagues and theorists, considering social justice and power imbalances, and often implementing changes in practice as a result. As part of our regular reviews, we may review the Child Safe resources available on Department websites.

Under the National Law and Regulations, our Service is required to regularly update our 'Quality Improvement Plan' and submit it to the Regulatory Authority when requested.

Service policies which support this Standard include our:

- **Child Safety, Protection & Wellbeing Policy** and **12C – Safeguarding Children: Look, Do & Tell Framework Policy**
- **Policy and Procedure Review Policy** which requires all Service policies and procedures to be regularly reviewed and updated in a scheduled, documented process. Regulatory authorities implementing the ***Education and Care Services National Law and Regulations require all policies be reviewed at least annually***
- **Interactions with Children Policy** which contains extensive guidelines and examples to support positive interactions with children and behaviour management guidance. The Policy also clearly states staff must not isolate or intimidate children, or use corporal punishment to guide behaviour.
- **National Model Code Policy**
- **Record Keeping and Retention Policy**
- **Technology and Digital Use Policy**
- **Internet, Email and Social Networking Policy**
- **Privacy & Confidentiality Policy**
- **Families and Visitors Code of Conduct**

Standard 11: Policies and procedures document how the organisation is safe for children and young people

Our Service:

- complies with policies and procedures which address all Child Safe Standards, are developed in line with best practice models, and are easy to understand
- consults with staff, families, volunteers and interested community members when developing and reviewing policies and procedures
- has managers and leaders who model compliance with our policies and procedures and take action if practices are inconsistent with these
- ensures staff and volunteers understand and implement policies and procedures.

As discussed above, we have many Policies and Procedures that support our child safe focus. These are accessible to employees and families in hard copy and electronic versions and where possible bilingual staff discuss relevant policies with families in their home language.

Managers implement a robust performance management procedure where non-compliance with the National Law, Regulations or Service policies and procedures is identified, for example through reporting and feedback arrangements between room/group leaders, the Educational Leader and the Nominated Supervisor, or through our complaint handling process.

See above the organisations recruitment, human resources and volunteering policies.

Child Information Sharing Scheme (CISS), Family Violence Information Sharing Scheme (FVISS) and the Family Violence Multi-agency Risk & Management Framework (MARAM)

Our Service will align with the MARAM Framework to assist in identifying, assessing and responding to family violence risk. This includes ensuring our educators and staff:

- have a shared understanding of and consistent response to family violence
- work collaboratively to provide coordinated and effective risk assessment and management responses to identify family violence
- share information relevant to family violence risk with other services
- promote continuous improvement in risk assessment and management practice, information sharing and enhanced collaboration with other services.

The [Child Information Sharing Scheme](#) (CISS) allows professionals working with children (from birth to 18 years of age) to gain a complete view of the children they work with, making it easier to identify wellbeing or safety needs earlier, and to act on them sooner. Victorian education and care services can share, request and use information about child wellbeing or safety through the scheme with other professionals provided certain requirements for sharing are met.

Educators within the Service need to have sufficient information in order to be able to support a child who has been impacted (or is suspected to have been impacted) by child abuse. Therefore, it is legally allowable for educators to share certain information about a child with other staff members of the Service without the consent of a parent/carer and without breaching privacy laws. Data security requirements that apply to the Service must be followed when sharing any information.

Under CISS, information sharing entities (ISEs) can share confidential information with other ISEs for the purpose of promoting the wellbeing or safety of a child or group of children or sharing information to inform action. This Scheme and the Family Violence Information Sharing Scheme complement each other.

Sharing of information may assist the ISE to

- access and engage with supportive services
- access resources for learning and development
- make a decision, assessment or plan
- start or conduct an investigation
- provide a service relating to a child/children
- manage any risk to a child/children

The Approved Provider and Nominated Supervisor are responsible for ensuring compliance with the Scheme. They are protected from liability if they share information in good faith and with reasonable care. If in doubt about their obligations or Scheme requirements, they will refer to the Child Information Sharing Scheme Guidelines. They will never use information obtained under the Scheme to prevent a child enrolling or continuing at the service.

As our Service is an 'Information Sharing Entity (ISE) under the Scheme, the Approved Provider or Nominated Supervisor will voluntarily share any personal information with other ISEs, or respond in a timely way to requests from ISEs to share information, if the information sharing meets the legal requirements of the Scheme. They may also request information from another ISE to promote the safety or wellbeing of a child.

The Approved Provider or Nominated Supervisor will use their professional judgement and work with other ISEs as required to determine whether the *threshold for sharing information* is met ie:

1. information is requested or shared for the purpose of promoting the safety or wellbeing of a child
2. if sharing information they reasonably believe this may help the receiving ISE to manage any risk to a child (eg through an assessment, plan, investigation or service provision to a child)
3. information is not excluded (includes information that might endanger a person's life or result in physical injury, prejudice legal proceedings or police investigation, contravene a court order, or is subject to legal professional privilege.)

If this threshold is met, consent is not required from any person to share information with other ISEs. However, the Approved Provider or Nominated Supervisor will inform the child and/or family members

of the information collected, who it will be shared with, how it might be used, how they might access it, and consider their views, *if it's safe, appropriate and reasonable to do so*. They will also advise the information sharing is allowed under the Scheme, and that complaints about the sharing of personal information may be made to the Victorian Information Commissioner. The Approved Provider or Nominated Supervisor may refuse to give a person access to their confidential information if they reasonably believe doing so would increase the risk to a child's safety.

The Approved Provider or Nominated Supervisor will confirm a person or organisation requesting information is an ISE before sharing, for example by asking for an official work email or calling an organisation's switchboard. They will also be aware of local service providers and professionals who can support children and their families.

The Approved Provider or Nominated Supervisor may share information with a child (where appropriate) or with their parent/guardian where this will help manage a risk to the child's safety. They will consider the nature and significance of the risk, whether the information will help manage the risk and whether the information is excluded. (Information will never be shared with a perpetrator or alleged perpetrator of family violence or other offences against a child.) However, where there is not an immediate threat to the child's safety, the Approved Provider or Nominated Supervisor may refer the child/family member to an ISE with expertise in this area, including expertise from a culturally sensitive perspective.

Under the Family Violence Information Sharing Scheme (FVISS) relevant information can be shared for two main purposes:

- Family violence assessment purpose-to establish or assess the risk of a person committing family violence or being the subject of family violence
- Family violence protection purpose- manage the risk to the victim survivor. This includes information sharing to support ongoing risk assessment.

Risk Assessment Entities (RAEs) can request and receive information for a family violence assessment. RAEs include DFFH Child Protection, Victoria Police, family violence services and some Orange Door services. Our Service will respond to all requests for information in a timely manner. The approved provider or Approved Provider should maintain contact with Victoria Police and DFFH Child Protection as necessary to protect the safety and wellbeing of the children involved. Disclosure of information to an RAE does not constitute unprofessional conduct or a breach of professional ethics.

Record Keeping

The Approved Provider or Nominated Supervisor will ensure appropriate records are kept securely and confidentially when information is requested and shared including where relevant:

- if the service received a request for information, who this was from, what was requested and the date of the request
- if the service disclosed information voluntarily, who the information was shared with, what was disclosed and the date disclosed
- if the service requested information, who this was made to and the date, what was requested
- how a disclosure of information met the threshold for sharing
- reasons any information sharing request is declined (these will also be provided to the ISE)
- whether the views of the child and/or family member were sought and obtained in relation to information sharing, what those views were, if they were advised the information would be shared or why their views were not considered
- copies of any family violence risk assessment and/or safety plan
- details of any complaint received including what it's about, date made, action taken to resolve the complaint and prevent similar complaints and time taken to resolve.

Any questions about child safety or this Policy may be directed to a Responsible Person at the service (the Approved Provider, Nominated Supervisor or Person in Day to Day Charge). If employees, volunteers and families have concerns about the Approved Provider or Nominated Supervisor's ability to provide or maintain a child safe environment, they should first discuss their concerns with another

Responsible Person who will investigate or contact directly the Child Protection Hotline on 131 278, VECRA, the Police, or the Social Services Regulator (SSR) for advice.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2027

Reference:

NQS

Element	1.1.2	Child-centred -Each child's current knowledge, strengths, ideas, culture, abilities and interests are the foundation of the program.
	1.2.3	Child directed learning - Each child's agency is promoted, enabling them to make choices and decisions that influence events and their world.
	2.2.3	Child Protection - Management, educators and staff are aware of their roles and responsibilities to identify and respond to every child at risk of abuse or neglect.
	3.2.1	Inclusive environment - Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments
	5.1.1	Positive educator to child interactions - Responsive and meaningful interactions build trusting relationships which engage and support each child to feel secure, confident and included
	5.1.2	Dignity and rights of the child - The dignity and rights of every child are maintained
	5.2.1	Collaborative learning - Children are supported to collaborate, learn from and help each other.
	6.1.3	Families are supported - Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.
	7.2.1	Continuous improvement - There is an effective self-assessment and quality improvement process in place.
	7.2.3	Development of professionals - Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development

National Law

Education and Care Services National Law Act 2010

Section	162A	Persons in day-to-day charge and nominated supervisors to have child protection training
	165	Offence to inadequately supervise children
	166	Offence to use inappropriate discipline
	167	Offence relating to protection of children from harm and hazards
	169	Offence relating to staffing arrangements
	170	Offence relating to unauthorised persons on education and care service premises
	173	Offence to fail to notify certain circumstances to Regulatory Authority
	174	Offence to fail to notify certain information to Regulatory Authority
	175	Offence relating to requirement to keep enrolment and other documents

National Regulations

Education and Care Services National Regulations

Reg	12	Meaning of serious incident
	82	Tobacco, drug and alcohol-free environment
	84	Awareness of child protection law
	87	Incident, injury, trauma and illness record

	100	Risk assessment must be conducted before excursion
	102B	Transport risk assessment must be conducted before service transports child
	120	Educators who are under 18 to be supervised
	123	Educator to child ratios—centre-based services
	145	Staff record
	146	Nominated Supervisor
	147	Staff members
	166	Children not to be alone with visitors
	168(2)(h)	Education and care services must have policies and procedures in relation to providing a child safe environment
	175	Prescribed information to be notified to Regulatory Authority
	177	Prescribed enrolment and other documents to be kept by approved provider

Source

Worker Screening Act 2020

A Guide for Creating a Child Safe Organisation

Child Information Sharing Scheme Ministerial Guidelines (includes Appendix 2 ISEs): VIC Govt

Child Wellbeing and Safety Act 2005 (includes Part 6 Child Safe Standards and Part 6A Child Information Sharing Scheme)

Child Wellbeing and Safety (Information Sharing) Regulations 2018

Early Years Learning Framework

Education and Care Services National Law and Regulations

National Quality Standard

UN Convention on the Rights of the Child

Child Wellbeing and Safety Act 2005 (Vic)

12B - CHILD SAFETY, PROTECTION & WELLBEING POLICY

Our Statement of Commitment to Child Safety

The organisation has a strong commitment to child safety, wellbeing and protection, through implementing and adherence to the Child Safe Standards and the Information Sharing Scheme. This commitment extends to all children in our service and community with an aim for children to feel safe, secure and supported. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations. We have a zero-tolerance approach to child abuse and harm. We acknowledge the importance of our organisation and community to be committed to cultural safety of aboriginal children, the cultural safety for culturally and/or linguistically diverse children and safety of children with a disability.

Team members may come in contact with children who are suffering from abuse (physical, emotional, or sexual) and/or neglect and it is essential that team members are aware of their responsibilities under child protection legislation and Child Safe Standards. Child Safety and Protection information is an integral part of our team member recruitment and induction process. All team members are required to obtain the necessary clearance to work with children as per state legislation, Children and Young Persons Act 2005, Child Safe Standards contain in the Child Wellbeing and Safety Amendment (Child Safety Standards) Act 2015. Knowing how to deal with alleged child abuse effectively is essential for the child's wellbeing.

Objective

To ensure all employees take their responsibility to protect children from any type of harm very seriously, understand their reporting obligations and are aware of our risk management strategy which includes practices designed to ensure the safety and wellbeing of children is paramount.

All educators, staff, visitors and volunteers are committed to identifying possible risk and significant risk of harm to children and young people at the Service. We comprehend our duty of care responsibilities to protect children from all types of abuse and neglect and will adhere to our moral and legislative obligations at all times. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations, including financial interests or other obligations of management, and are embedded in our daily practices, policies and procedures.

We aim to implement effective strategies to assist in ensuring the safety and wellbeing of all children. Our Service will act in the best interest of each child, assisting them to develop to their full potential in a secure and child safe environment. We understand our statutory duty of care to comply with both the Child Safe Standards and Reportable Conduct Scheme to build our capacity as an organisation to prevent and respond to allegations of child abuse.

The services' approach to child safety and protection is based on the following principles:

- ☐ Children have the right to feel safe, secure and under the protection of responsible, caring, trustworthy, adults at all times.
- ☐ Children will be respected and in no way degraded, endangered, exploited, intimidated or harmed psychologically or physically.

Definitions

Please refer to 12A - Child Safe Standards Policy.

Personal electronic devices: any device that can take images or videos, such as mobile phones, digital cameras, tablets, smart watches and other new and emerging technologies, where those technologies have image taking or video recording capability. The National Model Code and Guidelines also address use and possession of unapproved/restricted / personal storage media devices, for example, an SD card, USB drive, hard drive or use of a cloud-based application which can store images or video recordings.

Child Safe Standards

Victoria's mandatory Child Safe Standards (the Standards) have been in effect since 2016. Following the Royal Commission, the Victorian Government reviewed the Standards. The review found strong support for the Standards and recommended a number of changes to better align the Standards with the National Principles for Child Safe Organisations, and to strengthen administration of the Standards. In line with these recommendations, the new Standards were released by the Victorian Government in 2021. They consist of 11 updated Standards and apply from 1 July 2022.

Approved services - National Quality Framework

Approved early childhood services (that is services operating under the *Education and Care Services National Law Act 2010*) include any service providing or intending to provide education and care on a regular basis to children under the age of 13 years. Every approved service **must** meet the requirements of the National Quality Framework and associated regulatory system. Most notably, it is a legal requirement that **every reasonable precaution must be taken to protect children from harm and any hazard likely to cause injury** within approved early childhood services. Failure to meet this requirement can amount to a criminal offence.

Related Policies

Child Safety Standards Policy
Safeguarding Children: Look, Do & Tell Framework Policy
National Model Code Policy
Technology & Digital Usage Policy
Governance of Service, Management and Educator Policy
Family Violence Safety Policy
Privacy and Confidentiality Policy
Record Keeping and Retention Policy
Inclusion Policy
Education, Curriculum and Learning Policy
Ethical Conduct and Code of Ethics Policy
Smoke, Alcohol & Drug Free Environment Policy
Excursion Policy
Family Violence Safety Policy
Complaints and Grievance Policy
Incident, Injury, Trauma and Illness Policy
Parent Involvement Policy
Policy and Procedure Review Policy
Privacy and Confidentiality Policy
Employment Policy
Interactions with Children Policy
Staffing Arrangements Policy
Internet, Email & Social Networking Policy
Recording Keeping and Retention Policy
Protected Disclosures (Whistleblowers) Policy

Related Documentation

Incident Injury Trauma and Illness Records
Educator Induction Processes
Educator Appraisal Processes
Educator Recruitment Processes
Educator Professional Development Processes
Job Descriptions
Staff Records
Risk Management Plans

Summary of Organisation Roles

Social Services Regulator

The Social Services Regulator (SSR) is an independent statutory authority that exists to safeguard the rights of children and young people as well as people who use social services in Victoria.

The SSR regulates the [Social Services Standards](#) and the [Child Safe Standards](#) across a range of settings. It also administers the [Worker and Carer Exclusion Scheme \(WCES\)](#), [Reportable Conduct Scheme \(RCS\)](#), the [Working with Children Check \(WWCC\) scheme](#) and the [NDIS check](#).

The SSR takes an integrated and intelligence-driven approach to identifying, preventing and excluding unsafe people from working with children and young people, as well as ensuring organisations that work with children and young people keep children safe from abuse and harm by meeting the Child Safe Standards.

As a co-regulator of the Child Safe Standards, the SSR works with other co-regulators of the Child Safe Standards including the Department of Health, Victorian Registration and Qualifications Authority (VRQA), Victorian Early Childhood Regulation Authority (VECRA), and Workforce Inspectorate Victoria to keep children safe from harm and abuse.

Historical Role: Pre 23 February 2026)

Commission for Children and Young People

The role before 23 February 2026. An independent statutory body that promotes improvement in policies and practices affecting the safety and wellbeing of Victorian children and young people. We have a particular focus on vulnerable children and young people. Roles:

- provide independent scrutiny and oversight of services for children and young people, particularly those in the out-of-home-care, child protection and youth justice systems
- advocate for best practice policy, program and service responses to meet the needs of children and young people
- support and regulate organisations that work with children and young people to prevent abuse and make sure these organisations have child safe practices
- bring the views and experiences of children and young people to the attention of government and the community
- promote the rights, safety and wellbeing of children and young people.

Functions and powers are outlined in the Commission for Children and Young People Act 2012 and the Child Wellbeing and Safety Act 2005.

Staff Member: Employee, Student and Volunteer

As a staff member of an early childhood service, you have a moral obligation and a range of legal obligations to protect the children in your care from abuse.

Child Safety Officer role is held by the Nominated Supervisor as part of their role to promote and maintain safety and health of children. In the temporary absences of the Nominated Supervisor the Responsible Person – Person in Charge of Service will hold this role. The Child Safety Officer role is also supported by the Health, Wellbeing and Safety committee of the service.

The role of the Child Safety Officer is to lead and embed the organisations commitment to child safety and protection, through the implementation and adherence to the Child Safe Standards and Child Safety and Protection Policy. The Nominated Supervisor (or in their absence: the Responsible Person – in charge of the service) will be the Child Safety Officer in relation to Child Safety Standards and this policy to support children, their families and educators in implementing, monitoring, managing disclosures and suspicions of harm, making reports, completing internal investigations and maintaining confidentiality.

Regulator: Victorian Early Childhood Regulatory Authority (VECRA) - regulates education and care services

VECRA regulates education and care services and is responsible for ensuring the safety, health and wellbeing of children in Early Childhood Services.

VECRA is responsible for the regulation early childhood education and care services, as required by the Education and Care Services National Law Act 2010 and Children's Services Act 1996 and the respective regulations.

VECRA takes a risk-based approach to regulation to ensure most effort is directed toward those services with poor levels of compliance.

Where organisations that provide early childhood services operate in more than one sector, VECRA will collaborate with the Social Services Regulator(SSR) and the other sector regulators. SSR from February will to oversee the Reportable Conduct system.

VECRA is also responsible for the assessment of early childhood education and care services against the National Quality Standard and ensuring information is made available to families and communities about the quality of service provided. In its regulatory role, VECRA responds to notifications and complaints, monitors compliance, grants approvals, determines administrative and statutory sanctions and publishes appropriate public advice.

VECRA is comprised of staff in both central office and regional area locations.

Approved Provider will notify VECRA of any incidents or allegations of physical or sexual abuse to a child while being educated or cared for by the Service within 24 hours.

Victorian Police

Victoria Police (SOCIT) is responsible for criminal investigations into alleged child abuse.

Child Protection Services

DFFH Child Protection is the lead agency responsible for the care and protection of children and may bring cases before the Children's Court and support orders granted by the Children's Court.

Child Protection Services is a department within the State Government Body (E.g. Victoria: Department of Families, Fairness and Housing (DFFH)) with the objective specifically targeted to those children and young people at risk of harm or where families are unable or unwilling to protect them. The main functions of child protection are to:

- investigate matters where it is alleged that a child is at risk of harm
- refer children and families to services that assist in providing the ongoing safety and wellbeing of children
- take matters before the Children's Court if the child's safety cannot be ensured within the family
- supervise children on legal orders granted by the Children's Court
- provide and fund accommodation services, specialist support services, and adoption and permanent care to children and adolescents in need

Mandatory Reporting to Child Protection Services: People such as teachers, educators, doctors, nurses, police are legally obliged to report suspected child abuse. In addition, any person who believes on reasonable grounds that a child needs protection can make a report to the Child Protection Service. It is the Child Protection worker's job to assess and, where necessary, further investigate if a child or young person is at risk of harm.

Child First / The Orange Door

Child First / The Orange Door Referral and Support Teams are family services practitioners experienced in assessing the needs of vulnerable children and their families. Child First / The Orange Door teams work closely with community-based DHHS Child Protection workers.

Educators and teachers involved with vulnerable children, young people aged 0 to 17 years and their families, including families with an unborn child, may from time to time consider they should report or refer a concern to either child protection or the Child First / The Orange Door intake service.

A Referral to Child First / The Orange Door: A referral to Child First / The Orange Door may be the best way of connecting children, young people and their families to the services they need, where families exhibit any of the following factors that may impact upon a child's safety, stability or development:

- significant parenting problems that may be affecting the child's development

- family conflict, including family breakdown
- families under pressure due to a family member's physical or mental illness, substance abuse, disability or bereavement
- young, isolated and/or unsupported families
- significant social or economic disadvantage that may adversely impact on a child's care or development

Child First / The Orange Door: Factors for consideration; What specifically has happened to the child that has caused your concerns and what is the impact on their safety, stability, health, wellbeing and development?

- How vulnerable is the child?
- Is there a history or pattern of significant concerns with this child or other children in the family?
- Are the parents aware of the concerns, capable and willing to take action to ensure the child's safety and stability, and promote their health, wellbeing, and development?
- Are the parents able and willing to use support services to promote the child's safety, stability, wellbeing and development?

A referral to Child First / The Orange Door; should be considered if, after consideration of the available information you are, on balance, more inclined to form a view that the concerns currently have a low to moderate impact on the child, where the immediate safety of the child is not compromised.

On receiving a referral from a professional or community member the Child First / The Orange Door team will conduct further assessment of the family and may consult an experienced community-based child protection worker who is based in each Child First / The Orange Door team. This assessment may lead to the involvement of a local family services organisation. In most circumstances Child First / The Orange Door will inform you of the outcome of your referral.

Please quick reference guidance below to diagram “Four Critical Actions for Early Childhood Services – Responding to Incidents, Disclosures and Suspicions of Child Abuse” and Appendix 1 for A Step by Step Guide to Making a Report to Child Protection or Child First / The Orange Door.

Child Protection Reporting Overview

Early Years obligations and the Reportable Conduct Scheme

I am concerned about a child's safety. What should I do?

Follow the PROTECT guidelines: the Four Critical Actions for Early Childhood Services in responding to incidents, disclosures and suspicions of child abuse



Call Victoria Police on 000 if you have immediate concerns for a child's safety

Victoria Police

You must notify Victoria Police immediately if you have reasonable belief that a child has been abused or that criminal behaviour has occurred. Failure to notify Victoria Police that an adult may have committed a **sexual offence** against a child is a criminal offence.

DFFH Child Protection

You must report to DHHF Child Protection if you for a reasonable belief / reasonable grounds that a child is in need of protection, or their safety or wellbeing is at risk and the child's parent/carer has not protected or is unlikely to protect the child from harm.

(Victorian Early Childhood Regulatory Authority- VECRA) and Victorian Institute of Teaching

You must notify your regulatory body of any incidents, circumstances or complaints which raise concerns about the safety and/or wellbeing of children using your service. This includes notifying VIT if the source suspected harm is an early childhood teacher.

Reportable Conduct

The Reportable Conduct Scheme (the Scheme) operates alongside other reporting obligations. The scheme applies to all Victorian early childhood services from **1 January 2019**. All workers, volunteers and contractors are covered by the Scheme and the Scheme captures allegations about reportable conduct and misconduct that may include reportable conduct which occurs **both within and external** to your organisation.

There are five categories of Reportable Conduct:

Against, with, or in the presence of a child

Physical violence

Sexual offences

Sexual misconduct

Behaviour that causes significant emotional or psychological harm

Significant neglect

Report to: Social Services Regulator



SSR operates [an online form for heads of organisations to notify of a reportable allegation](#). (SSR requires that heads of organisations use the online form.)

Non - head of an organisation can use the online form or the following contacts:

Phone: 1300 310 778

Email: Contact@ssr.vic.gov.au

Additional Child Protection Contacts Business Hours	
Divisions	Telephone
East	1300 360 391
South	1300 655 795
North	1300 664 977
West Metro only	1300 664 977
West Rural and Regional	1800 075 599

SEE APPENDIX H FOR OTHER CONTACTS.

See ‘A step-by-step guide to making a report to Child Protection or The Orange Door’ on the Department website. (Link:

<https://www.education.vic.gov.au/childhood/professionals/health/childprotection/Pages/eccritmustact.aspx>)

Guide also includes contact numbers for Child Protection and The Orange Door. See Appendix.

(Link:

<https://www.education.vic.gov.au/Documents/childhood/parents/health/mandreportsep10.pdf>)

Implementation

Under the *Children Youth and Families Act 2005* a child is considered to be in need of protection if:

- the child has been abandoned by their parent(s) and no other suitable person is willing and able to care for the child.
- the child's parent(s) are dead or incapacitated and there is no other suitable person willing and able to care for them.
- the child has suffered, or is likely to suffer, significant harm as a result of physical injury, sexual abuse, emotional or psychological harm and the child's parent(s) have not protected, or are unlikely to protect, the child from that harm.
- the child's physical development or health has been, or is likely to be significantly harmed and the child's parent(s) have not provided or arranged, or are unlikely to provide or arrange, basic care or effective medical, surgical or other remedial care.

Mandatory reporters must make a report to Child Protection as soon as possible after forming a belief on reasonable grounds that a child is in need of protection from significant harm as a result of **physical or sexual abuse**, and the child's parents are unwilling or unable to protect the child.

Please see **Appendix B** for further information on the following:

- **B1: Indicators of Harm, Abuse and Neglect**
- **B2: Family Violence**
- **B3: Children Problem Sexual Behaviours**
- **B4: Recognising Grooming**

Mandatory reporters include the Approved Provider, Nominated Supervisor, Teachers registered under the Education and Training Reform Act 2006 and Qualified Educators.

Note all adults must report a reasonable belief that a sexual offence has been committed by an adult 18 and over against a child under 16 to Victoria Police unless they believe the information has already been disclosed to Police eg a report has been made to Child Protection who confirm they will pass information to Police.

It is a criminal offence not to report in these circumstances.

To ensure that you fulfil your duty of care obligations for all children who are involved in, or affected by, the suspected child abuse, you must follow: Four Critical Actions for Early Childhood Services. See Appendix 1.

A report to Child Protection will be made if:

- the harm or risk of harm has a serious impact on the child's immediate safety, stability or development
- the harm or risk of harm is persistent and entrenched and is likely to have a serious impact on the child's immediate safety, stability or development
- the child's parents cannot or will not protect the child from harm.

A report to The Orange Door will be made if concerns about the child have a low to moderate impact on the child and the immediate safety of the child is not compromised. Some of these concerns may include:

- family conflict or family breakdown
- young or isolated families
- significant parenting problems that may be affecting the child's development.

A step by step guide to making a report to Child Protection or Child FIRST/The Orange Door is available on the Department website. See Appendix 1.

A person may form a belief on **reasonable grounds** that a child is in need of protection after

becoming aware that a child's health, safety or wellbeing is at risk and the child's parents are unwilling or unable to protect the child. For example:

- a child states that they have been physically or sexually abused
- a child states that they know someone who has been physically or sexually abused (sometimes the child may be talking about themselves)
- someone who knows the child says they have been physically or sexually abused
- a child shows signs of being physically or sexually abused
- a staff member is aware of persistent family violence or parental substance misuse, psychiatric illness or intellectual disability that is impacting on the child's safety or development
- a staff member observes indicators of abuse, including non-accidental or unexplained injury, persistent neglect, poor care or lack of appropriate supervision
- a child's actions or behaviour place them at risk of significant harm and the child's parents are unwilling or unable to protect the child.

The reporter is not required to prove that harm has occurred.

Criminal Offences – All Adults

In response to the Betrayal of Trust Report, the Victorian Government has introduced criminal offences to protect children from sexual abuse. Under these reforms a failure to report, or take action in relation to suspected child sexual abuse can now constitute a criminal offence, including:

Failure to disclose

This offence applies to **all adults** (not just professionals who work with children) who form a reasonable belief that another adult may have committed a sexual offence against a child under 16 years of age and fail to report this information to Victoria Police.

Failing to disclose a sexual offence based on concerns for the interests of the perpetrator or organisation (e.g. concerns about reputation, legal liability or financial status) will not be regarded as a reasonable excuse.

Failure to protect

This offence applies to a person in a **position of authority** within an organisation who:

- knows of a substantial risk that a child who is under 16 years and in the care and supervision of the organisation may become the victim of a sexual offence committed by an adult associated with that organisation (e.g. an employee, contractor, volunteer or visitor); and
- fails to take reasonable steps to remove or reduce the risk.

Within an early childhood service a position of authority includes local service managers and staff in management positions within licensed or approved services.

Child Protection Risk Management Strategy

The Approved Provider, Nominated Supervisor, employees and volunteers will implement a Child Protection Risk Management Strategy to ensure the health, wellbeing and safety of all children at the service, protect children from harm and protect the integrity of employees and volunteers. The Strategy includes the following component:

1. Aim/objective and Statement of Commitment (see above)
2. Code of Conduct
3. Recruitment, Selection and Training Procedures for employees and volunteers which include child protection principles
4. Procedures for managing disclosures and suspicions of harm
5. Procedures for Managing Breaches
6. Risk Management for High Risk Activities and Special Events
7. Strategies for Communication and Support

2. Code of Conduct

The service upholds the Code of Conduct in our Educator and Management Policy for employers, educators, volunteers, students, families and children to ensure the safety and wellbeing of children.

Please see to Ethical Conduct and Code of Ethics Policy and Governance of Service, Management and Educator Policy.

Educators, Staff and Students will:

- adhere to the Service's policies and procedures
- promote the welfare, safety, and wellbeing of children at the Service by creating and maintaining child safe environment and adhere to the Child Safe Standards (CSS)
- foster a culture of openness, respect and cultural safety where children and young people feel safe to disclose risk of harm to children or report abuse
- participate in a comprehensive induction and orientation program, including an understanding of child protection law
- provide valid WWCC and VIT details during their employment and engagement at the Service
- advise the approved provider of any circumstances that may affect their WWCC, VIT or fit and proper status including changes to teacher accreditation or registration (if applicable), in writing, within 24 hours
- not use, or have access to, any personal electronic devices, including mobile phones or smart watches used to take images or video of children at the Service
- complete mandatory national child safety training (**effective from February 2026**)
- complete mandatory online training- Protecting Children: Mandatory Reporting and Other Obligations Training (PROTECT) - Victorian
- participate in training about the Child Information Sharing, Family Violence Reforms and Family Violence Multi-Agency Risk Assessment Management Framework (MARAM)
- implement consistent, appropriate interactions with children
- ensure no child is subjected to inappropriate conduct or discipline
- understand their legal responsibilities to report any allegations or disclosures that a staff member or volunteer is engaging or has engaged in inappropriate conduct to notify the VECRA, approved provider and nominated supervisor within 24 hours
- allow children to be part of decision-making processes where appropriate
- provide ongoing monitoring and follow-up for children's health and wellbeing.

3. Recruitment, Selection and Training Procedures

The Approved Provider or Nominated Supervisor will implement recruitment, professional development and training procedures for employees and where relevant volunteers to ensure no-one at the service poses a risk to children and everyone understands how to manage disclosures or suspicions of harm (Recruitment procedures at Appendix A and training procedures in Educator and Management Policy). Requirements include:

- job advertisements which include qualifications and skills required, and culture of child safety and protection
- job descriptions which outline level of professional skills and responsibilities
- processes (including job advertisements) which ensure employees and volunteers have clear Working With Children Checks or they are exempt (see <http://www.workingwithchildren.vic.gov.au/>) (Police Checks or VIT checks may be required for people who are exempt)
- interview questions and referee checks which reference person's approach to child safety and protection
- documented induction/ orientation checklists which reference child safety and protection, supervision, compliance with State legislation, National Law and Regulations, NQS, Code of Conduct, policies and procedures
- annual training and development to ensure individuals are clear about their roles and responsibilities to protect children from harm, are aware of their reporting obligations, can confidently recognise the indicators of harm (see Appendix B) and understand documenting and reporting procedures.
- annual performance appraisals for employees
- regular inclusion of child protection and risk management strategy at least every 6 months in staff meetings and annual review of written training plans which must include Child Protection matters (eg disclosures and suspicions of harm)

- providing access to relevant legislation and other resources to help employees and volunteers meet their obligations

4. Procedures for managing disclosures and suspicions of harm

What is a *disclosure* of harm?

A disclosure of harm occurs when someone, including a child, tells you about harm that has happened, is happening, or is likely to happen to a child. Disclosures of harm may start with:

- I think I saw...||
- Somebody told me that...||
- Just think you should know...||
- I'm not sure what I want you to do, but...||

What is a *suspicion* of harm?

A suspicion of harm is when someone has a reasonable suspicion that a child has suffered, is suffering, or is at an unacceptable risk of suffering significant harm. Note there may be circumstances where there is concern for a child's welfare but it does not reach the threshold to be considered a disclosure or suspicion of harm. In this case educators will connect families with a relevant family support service with the family's consent. See Appendix B for further guidance.

The Approved Provider, Nominated Supervisor, employees and volunteers may suspect harm if:

- a child says they have been harmed
- someone else, for example another child, a parent, or an employee, says harm has occurred or is likely to occur
- a child tells them they know someone who has been harmed (it is possible that they may be referring to themselves)
- they are concerned at significant changes in the behaviour of a child, or the presence of new unexplained and suspicious injuries
- they see the harm happening.

Helping the Child

When a child tells you that they have been abused, the child may be feeling scared, guilty, ashamed, angry and powerless. You, in turn, may feel a sense of outrage, disgust, sadness, anger and sometimes disbelief.

However, it is important for you to remain calm and in control of your feelings in order to reassure the child that something will be done to keep him or her safe.

Things you can do to show your care and concern for the child:	You will NOT be helping the child if you:
• listen carefully to what the child is saying • acknowledge that it is hard to talk about such things • tell the child that you believe them and taking them seriously • make it clear that whatever has happened or happens is not the child's fault and that the child is not bad • tell the child that you know that adults sometimes do the wrong thing and that this has also happened to others • believe, validate and support the child • control your expressions of panic or shock • reassure the child that they did the right thing telling you (many abusers	• avoid or reject them, you might be the only other significant adult in his or her life • investigate further and inquire into the details of the abuse. This should be left to an interviewer skilled in asking such questions • press for details beyond those that the child freely wants to tell you. Your role is to listen to what the child wants to tell you and not to conduct an investigation. • make promises to the child that you cannot keep, e.g. promising not to tell anyone • seek a medical examination or treatment unless it is an emergency or there are serious health risks to the child • pressure the child to show you injuries that are covered by clothing

threaten children to prevent disclosure) • tell the child honestly that you have a legal duty to tell someone so that they can help stop the abuse • record details of the conversation, any visible injuries and any other observations you have made.	• confront the parents or guardians or the alleged perpetrator. This may place the child at further risk e.g. parents may threaten the child not to disclose and/or remove the child from a place of safety.
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Managing and recording a disclosure of harm

If the Approved Provider, Nominated Supervisor, educators have concerns about the safety of a child they will:

- find a private place to talk
- remain calm and listen in an attentive, active and non-judgemental way
- encourage the person (including a child) to talk in their own words
- take anything a child says seriously
- allow children to be part of decision-making processes where appropriate
- ask just enough open ended questions to act protectively without asking any leading questions which suggest an answer and could compromise later investigations
- tell the person they have done the right thing in revealing the information and they'll need to tell someone who can help keep them safe
- not try to investigate or mediate the matter themselves
- record their own observations as well as accurate details of any conversation with a parent (who may for example explain a noticeable mark on a child)
- document as soon as possible so the details are accurately captured including:
 - time, date, location and who was present
 - full details of the (suspected) harm
 - exactly what the person said using "I said", "they said," statements
 - the questions educators asked
 - any comments educators made
 - educators' actions following the disclosure
- ensure the managements and storage of records complies with our Privacy and Confidentiality Policy.
- follow our reporting procedures

See template at Appendix C

Managing and recording a suspicion of harm

The Approved Provider, Nominated Supervisor, employees and volunteers will:

- remain alert to any warning signs or indicators
- pay close attention to changes in the child's behaviour, ideas, feelings and the words they use
- make written notes of observations in a non-judgemental and accurate manner, and manage in line with our Privacy and Confidentiality Policy
- assure a child that they can come to talk when they need to, and listen to them and believe them when they do
- follow our reporting procedures. See template at Appendix C

Making a Report / Notification

A report and notification will be made using the following procedure.

Approved Provider will notify the Regulatory authorities of any incidents or allegations of physical or sexual abuse to a child while being educated or cared for by the Service within 24 hours for VECRA and 3 Business Days for Social Service Regulator (SSR).

Reports will be kept confidential while the matter is investigated. Employees or volunteers must not discuss the Report with anyone who's not involved to ensure the matter can be thoroughly and fairly investigated and the person's reputation preserved in the event the allegation is not substantiated. As an early childhood staff member, you **must** keep clear and comprehensive notes relating to incidents, disclosures and allegations of child abuse. All staff are to use the **Responding to Suspected Child Abuse: Template (See appendix 1 or <https://www.education.vic.gov.au/childhood/professionals/health/childprotection/Pages/eccritmustact.aspx>)**

Making a Report / Notification - Educators and staff will:

- contact the police on **000** if there is an immediate danger to a child and intervene if it is safe to do so
- respect what a child discloses, taking it seriously and follow up on their concerns through the appropriate channels
- be aware of their mandatory reporting obligations and responsibilities to report that a child is at risk of abuse, harm, neglect or ill-treatment to the approved provider or nominated supervisor
- report all instances (alleged or witnessed) of child abuse, including assault or sexual abuse (including grooming) to relevant authorities including Victoria Police, DFFH Child Protection, Social Services Regulator and the VECRA in accordance with the Education and Care Services National Law
- follow the [Four Critical Actions](#) to take to respond to an incident, disclosure or suspicion of child abuse
 - responding to an emergency
 - reporting to authorities
 - contact parents or carers
 - provide ongoing support
- prepare accurate records recording exactly what happened, conversations that took place and what was observed to pass on to the relevant authorities to assist with any investigation
- share information with other professionals working with children as part of the information sharing and family violence reform schemes
- understand their legal obligations to report or take action in relation to suspected child abuse under the Reportable Conduct Scheme
- notify the approved provider if there are any allegations of reportable conduct involving a staff member, volunteer or contractor as soon as practicable
- identify and report any concerns and allegations of inappropriate conduct (observed or suspected or disclosures) by other staff members, volunteers or students to the approved provider and VECRA within 24 hours
- refer families to appropriate agencies where concerns of harm do not meet the threshold of significant harm. These services may be located through [Orange Door](#). Depending on circumstances, family consent may or may not be sought before making referrals.
- NOT investigate suspicion of abuse or neglect but collect only enough information to substantiate concerns and pass on to Child Protection or appropriate authority
- understand that allegations of abuse or suspected abuse against them are treated in the same way as allegations of abuse against other people.

Making a Report / Notification - Approved Provider and Nominated Supervisor will:

Follow the [Four Critical Actions](#) to take to respond to an incident, disclosure or suspicion of child abuse

- responding to an emergency
- reporting to authorities
- contact parents or carers
- provide ongoing support

Ensure documentation is completed to assist in making reports to relevant authorities including an incident, injury, trauma and illness record

1. Consider whether disclosure or suspicion needs to be reported to Police

- contact the police on 000 if there is an immediate danger to a child and intervene immediately if it is safe to do so.
- contact the police on 000 where the child has been or may be the victim of a criminal offence (**including sexual abuse** and where a child is at risk of significant harm outside the family)
- contact the Police immediately on 000 if the Approved Provider, or an employee, volunteer or visitor has abused or is alleged to have abused a child
- get clear guidance from Police about who will tell child's parents about the disclosure and who can give ongoing support

2. Consider whether the disclosure or suspicion must be reported to Children Protection

- make a report by phone to the Child Protection hotline on **131 278** (available 24 hours/7 days a week). You may also contact Regional offices (See page 2)
- make the report with the assistance or support of the Nominated Supervisor. If the Nominated Supervisor does not follow through and make the report, employees and volunteers will make the report
- get clear guidance from the person answering about who will tell child's parents of the disclosure and who can give ongoing support

3. Consider whether referral is needed to The Orange Door

- connect families with [The Orange Door](#) (available 24 hours/7 days a week) where concerns of harm do not require reports to Child Protection or the Police. Get family consent first

4. Consider obligations under Child and Family Violence Information Sharing Schemes

- share information with other Information Sharing Entities under the Child or Family Violence Information Sharing Schemes if considered appropriate, safe and within the legal requirements of the Schemes

5. Consider whether you must notify Regulatory Authorities

- notify **Victoria Police**
- notify **DFFH Child Protection**
- notify the **(Victorian Early Childhood Regulatory Authority- VECRA)** through the online NQA ITS portal about any incident/allegation, including any suspected or alleged incident of child abuse that has occurred at the service **within 24 hours of the incident/allegation**. This includes:
 - any incident or allegation where it is reasonably believed that physical and/or sexual abuse of a child has occurred or is occurring while the child is being educated and cared for by the Service
 - any complaints alleging that a serious incident has occurred or is occurring at the Service
 - a serious incident, which may include physical or sexual abuse where emergency services attended the Service
 - any reportable sexual conduct by a nominated supervisor, educator, staff member, student or volunteer against, with or in the presence of, a child
 - any allegation (observed or suspected or disclosures) that a staff member or volunteer is engaging or has engaged in appropriate conduct to a child
 - any change to a person's WWCC or any relevant risk information related to child safety
- notify the **Social Services Regulator(SSR)** using the online form (or other contact details) **within 3 business days** of becoming aware of reportable allegations/conduct (see next section), providing name of employee/volunteer, date of birth, whether police have been contacted and police report if relevant, service contact details, Approved Provider's name, nature of allegation

- get clearance from Police first if allegation is a criminal offence and start investigation using grievance procedure ie gather information to establish facts, decide whether it is more likely than not that reportable conduct occurred and make findings/recommendations
- consider when to tell alleged perpetrator eg could they destroy evidence like emails/texts/documents, what sort of evidence is there, will they have enough time to prepare response
- provide a progress report to the Social Services Regulator(SSR) within 30 calendar days of becoming aware of reportable allegations/conduct, providing name of person investigating allegation, details of allegation and your response including any disciplinary action taken or proposed, any reasons why you no action has been taken, any written response from employee/volunteer
- provide outcomes of investigation to the Social Services Regulator(SSR) (if these not included in progress report) including any disciplinary action taken or proposed and reasons for taking or not taking action
- help any employee/volunteer subject to allegations access appropriate support/counselling. (Also provide this to others involved in reportable incidents if appropriate)
- protect the identity of employees/volunteers where possible in relation to unsubstantiated complaints
- suspend volunteers pending outcome of investigation, and ensure employees subject to allegations are supervised at all times. Note it may be appropriate to place the person in a non-contact role while the matter is investigated, including where advised to do so by police or Child Protection
- seek legal advice about restricting that person's work activities if relevant.

Reportable Allegations/Conduct involving Employees and Volunteers

As head of the organisation, the Approved Provider must report allegations of reportable conduct to the Social Services Regulator(SSR) if they involve employees aged 18 and over, or people 18 and over who volunteer at the service with the verbal or written agreement of the Approved Provider or Nominated Supervisor, if they believe that the alleged conduct may have occurred. For example, they may have observed the conduct, or heard about it from a child or other person. Allegations must be reported even if a person does not have direct contact with children, the conduct occurred outside of their work or the person resigns.

Any employee or volunteer who becomes aware of conduct that is potentially reportable must inform the Approved Provider.

Reportable conduct includes sexual offences, sexual misconduct and physical violence all against, with or in the presence of a child, behaviour that causes significant emotional or psychological harm and significant neglect.

Confidentiality

It is important that any notification to Police, VECRA, DFFH Child Protection and SSR remains confidential, as it is vitally important to remember that no confirmation of any allegation can be made until the matter is investigated. The individual who makes the notification should not inform the suspected perpetrator (if known) nor attempt to investigate any allegation themselves. This ensures the matter can be investigated without contamination of evidence or pre-rehearsed statements by the appropriate authorities. It also minimises the risk of retaliation on the child for disclosing.

Protection for Reporters

All reporters are protected against retribution for making or proposing to make a report under amendments to the Children Youth and Families Act 2005 and Crimes Act 1958 effective April 2021. The identity of the reporter is protected by law from being disclosed, except in certain exceptional circumstances. Provided the report is made in good faith:

- does not breach standards of professional codes of conduct or ethics
- cannot lead to defamation and civil and criminal liability
- A report is also an exempt document under the Freedom of Information Act 1982.

Support after disclosure

The Approved Provider or Nominated Supervisor will provide assistance to access appropriate support and counselling services for all parties affected by a disclosure of harm. Please refer to the services EAP.

Key Differences Between Mandatory Reporting and Reportable Conduct

Feature	Mandatory Reporting	Reportable Conduct Scheme
Who Must Report?	Childcare educators, teachers, doctors, social workers, etc	Employers & heads of organisations working with children
What is Reported?	Suspected child abuse or neglect	Allegations of misconduct or abuse by employees
Who is Reported?	Parents, guardians, or caregivers suspected of harm	Employees, volunteers, or carers accused of misconduct
Where to Report?	Need to report to: • You must also report internally to management who will notify your Approved Provider/Service Owner/Head of Organisation. • Victoria Police • DFFH Child Protection • VECRA • VIT (if a registered Teacher) • The Orange Door You must make a new report in any circumstance where you become aware of any further reasonable grounds for the belief. Every report is critical to protecting a child by building evidence and enabling authorities to gain a clearer understanding of the risks. This means that you must make a report to protect a child even if you are aware that: • DFFH Child Protection or Victoria Police were previously involved or are already involved with the child and/or their family • another party, such as a family member, has already raised concerns with the relevant authorities.	Need to report to: • You must also report internally to management who will notify your approved provider. • Victoria Police • DFFH Child Protection • VECRA • VIT (if a registered Teacher) • Social Services Regulator • The Orange Door

	<i>If another person has already made a report:</i> Once you form a reasonable belief that a child has been, or is at risk of being abused, your obligation to report is separate from the obligations or actions of other people. You should still make a report.	
Legal Requirement?	Yes, failure to report is a legal offence	Yes, organisations must investigate allegations and report them

5. Procedures for Managing Breaches

All employees and volunteers working with children have a duty of care to support and protect children which is breached if a person:

- does something that a reasonable person wouldn't do in a particular situation
- fails to do something that a reasonable person would do in the circumstances
- acts or fails to act in a way that causes harm to someone owed a duty of care.

In relation to our Child Protection Risk Management Strategy, a breach of that duty of care includes any action or inaction by an employee, volunteer or child that fails to comply with any of the components of the Strategy.

Employees, volunteers or families should report the breaches to the Nominated Supervisor or Approved Provider who will manage an investigation into the breach in a fair, unbiased and supportive manner in line with our Grievance Policy and Procedure (see Report Breach template at Appendix D unless advised not to do so by the police or Child Protection because the breach is a child harm related matter). For example:

- those involved in the breach will be able to provide their version of events
- matters discussed in relation to the breach will be kept confidential
- an appropriate outcome will be decided
- everyone affected will receive a clear written statement (letter, email or SMS) of the outcome
- records will be kept about the details of the breach, including the versions of all parties and the outcome of the breach

Depending on the nature of the breach outcomes may include:

- emphasising the relevant component of the Child Protection Risk Management Strategy, for example, the Code of Conduct
- providing closer supervision
- professional development and training
- mediating between those involved in the incident (where appropriate)
- disciplinary procedures if necessary
- reviewing current policies and procedures and developing new policies and procedures if necessary
- termination of employment.

6. Risk Management Plan for High Risk Activities and Special Events

The Nominated Supervisor and educators will analyse the risk of 'harm' to children for all relevant events including purchase of new equipment as well as high risk activities where there is an increased risk of harm to children for example:

- water based activities
- special events like service concerts and family information days where there will be a large number of visitors or people present
- events or activities where visitors will be present
- excursions
- playground renovations
- activities using dangerous equipment

The Nominated Supervisor and educators will:

1. Identify all the elements of an activity (eg objectives, location, participants, transportation, toileting/change room procedures, appropriate supervision and adult to child ratios, photography policy, managing medications, managing illness and injury, procedure applying to visitors, physical environment)
2. Identify the risks
3. Analyse the likelihood and consequences of the risks
4. Evaluate the level of risk (eg low, moderate, high, extreme)
5. Implement strategies to eliminate or minimise the risk
6. Review the activity to determine how it could be improved

See Appendix E for a Risk Management template.

Where relevant (eg playground renovations) the Nominated Supervisor will encourage families to provide feedback on the risk of harm to children and strategies to minimise the risk. Feedback may be sought via newsletters or survey forms, or during parent information sessions.

7. Strategies for Communication and Support

The Nominated Supervisor will implement the following to ensure families, employees, volunteers and children are aware of our Child Risk Management Strategy:

- regularly advise families and prospective families (at least every 6 months) via service newsletters, emails and information evenings about the reasons for and components of our risk management strategy, where they can access our Child Safety, Protection & Wellbeing Policy and Risk Management Strategy, and that we welcome feedback about the Policy/Strategy. We may include what we aim to teach children about protective behaviour (see Attachment G)
- provide written information about our risk management strategy during enrolment and orientation and include in Parent Handbook
- regularly include (at least every 6 months) the reasons for and components of our risk management strategy in staff meetings and include in Staff Handbook
- ensure educators talk to children about the Strategy where appropriate and provide any feedback to the Nominated Supervisor
- display posters about child protection issues, including safe and supportive environments
- include child protection issues and our risk management strategy in employees' performance and training plans
- ensure educators regularly include learning about appropriate child protection issues in the Curriculum, including how to keep themselves safe, and what to do if they feel unsafe
- make available to employees and families relevant resources

Breaches of Child Safety and Protection Policies and Procedures

A breach is any action or inaction by any individual within the Service, including children and young people, that fails to comply with any part of the policy. All educators, students, volunteers and staff working with children are mandatory reporters under the *Crimes Act 1958 (Vic)* and have a duty of care to support and protect children. The rights and best interests of children are paramount, and breaches will be treated as serious.

Any allegations of criminal offences against children must be reported to the Police immediately. Failure to report child sexual abuse to the police is a criminal offence. Any person who reports a

breach or concern in good faith, will be protected against retaliation under the *Protected Disclosures (Whistle Blower) Policy*.

All breaches will be recorded, and outcomes documented, in accordance with record-keeping requirements. A breach may include (but is not limited to):

- failure to report suspected child abuse, neglect, or risk of harm
- engaging in or failing to prevent inappropriate conduct or inappropriate discipline to a child
- breaching confidentiality about a child protection matter
- failing to maintain a valid Working with Children Check (WWCC) and VIT Registration
- not following the Service's Code of Conduct, supervision or reporting procedures
- any action or inaction that places a child at risk of harm.

Managing Breach Child Safety and Protection Policies and Procedures

Management will investigate any breaches to this policy in a fair, unbiased and supportive manner by:

- Management may undertake an internal investigation. Factors to assess if internal investigation will be conducted; DFFH Child Protection or the Police are not conducting their own investigation or if their action has concluded
- liaising with the DFFH Child Protection, Social Services Regulator, Victoria Police and VECRA as required, for appropriate processes to ensure chain of evidence is not destroyed or compromised
- not undertaking and investigating the allegation whilst the Social Services Regulator or the Police are conducting an investigation
- following directions from the DFFH Child Protection, VECRA, Social Services Regulator and Victoria Police that may include removal of the educator or staff member (who is the subject of allegations) immediately from a role with contact with children or young people until authorities conclude their investigation
- displaying the Service's compliance history and any compliance actions that are the subject of review by the regulatory authority for families to view.

When performing an internal investigation, Management will:

- give the educator, staff member, student or volunteer the opportunity to provide their version of events
- document the details of the breach, including the versions of all parties
- record the outcome clearly and without bias
- ensure the matters in relation to the breach are kept confidential
- reach a decision based on discussion and consideration of all evidence.

Outcome of Breach Child Safety and Protection Policies and Procedures

Staff members or educators who fail to adhere to this policy may be in breach of their terms of employment. Visitors or volunteers who fail to comply to this policy may face termination of their engagement. Depending on the nature of the breach outcomes may include:

- disciplinary procedures if required including dismissal of employment
- emphasising the relevant element of the child protection policy and procedure not followed
- providing closer supervision
- providing further education and training
- providing mediation between those involved in the incident (where appropriate)
- reviewing current policies and procedures and developing new policies and procedures if necessary
- external regulatory action- including compliance history, fines, suspension of rating or Service closure.

Working With Children Checks and VIT Checks

All team members must possess the required abilities to care for children as per the company job description and employment policy. Team members must also be a fit and proper person to care for children.

In Victoria, the Working with Children Check (WWCC) is required per the Working With Children Act 2005 or VIT. All team members must provide an original and current (not more than 6 months old) Working With Child Check(WWCC) check before they can commence work at a children's service as

per the Regulations. Failure to meet this legal requirement for the eligibility for staff to work in the centre will mean that employment will not be commenced until the relevant documentation can be provided. It is the responsibility for employees to have a current WWCC. If an employee WWCC expires they are not allowed to work at the service per the Regulations.

Strategies for Educators and Child Protection

Staff in children's services can be open to allegations of reportable conduct unless care is taken to implement protective strategies for staff and children.

It is the responsibility of the Centre Director/Responsible Person/Room Leaders to carefully monitor the adequacy of the supervision of children throughout the day and ensure that the following strategies are implemented.

- ☐ Individuals or groups of children are not being permitted to be withdrawn into areas where they are not visible by other staff members, or taken into rooms that can be locked.
- ☐ Permanent and temporary child-care staff are to be directed to ensure that they at all times remain readily accessible and within the visibility of other staff while with the children.
- ☐ Students and volunteers are to be directed to ensure that they are never alone with children in a direct, un-supervised capacity.
- ☐ Casual staff are to be directed to ensure they are not alone with children in a direct, un-supervised capacity until they have received successful employment screening.
- ☐ Casual staff are to at all times remain readily accessible and within the visibility of other staff while with the children.
- ☐ While there is a child on the premises of a service, there is to be a minimum of 2 staff on the premises. It is the responsibility of the Director to ensure that rosters meet this requirement and it is the responsibility of **all** staff to ensure this requirement is upheld.
- ☐ Parents are to be informed of the procedures staff use to toilet children. Changing of babies should take place within view of other adults.
- ☐ Parents should be made aware that it may be necessary to physically restrain an out of control child who may be compromising the safety of him/herself, other children or staff, or to isolate the child from others for short periods of time.
- ☐ Up-to-date developmental records on all children are to be kept. Relevant conversations with parents that may relate to a child's behaviour change (e.g. parents separating, new baby in the family, moving house etc.) are to be noted in the developmental record.
- ☐ Staff should be aware of their own level of tolerance and stress factors and acknowledge that there may be some children that are found to be more difficult to relate to than others. Team members are to be requested to assist in the appropriate management of the children in these instances.
- ☐ Staff should ensure that they are aware of the individual child's stage of development and particular needs and plan the curriculum accordingly.
- ☐ Staff should work with parents to develop and record appropriate procedures for managing toilet practises and behaviour management.

Child Information Sharing Scheme

Please refer to Child Safety Standards Policy for information.

Record Keeping

Please refer to Record Keeping Policy and Child Safety Standards Policy for information.

Appendix- 1A: (The Orange Door Replaced Child First)

A step-by-step guide to making a report to Child Protection or The Orange Door

Protective concerns

You are concerned about a child because you have:

- received a disclosure from a child about abuse or neglect
- observed indicators of abuse or neglect
- been made aware of possible harm via your involvement in the community external to your professional role.

At all times remember to:

- record your observations
- follow appropriate protocols
- consult notes and records
- consult with appropriate colleagues if necessary
- consult with other support agencies if necessary

STEP 1	RESPONDING TO CONCERNS	STEP 2	FORMING A BELIEF ON REASONABLE GROUNDS	STEP 3	MAKING A REFERRAL TO Child FIRST	STEP 4	MAKE A REPORT TO CHILD PROTECTION
	1. If your concerns relate to a child in need of immediate protection; or you have formed a belief that a child is at significant risk of harm*. Go to Step 4 2. If you have significant concerns that a child and their family need a referral to Child FIRST for family services. Go to Step 3 3. In all other situations Go to Step 2. * Refer to Appendix 2: Definitions of child abuse and indicators of harm in the Protocol – <i>Protecting the safety and wellbeing of children and young people</i>		1. Consider the level of immediate danger to the child. Ask yourself: a) Have I formed a belief that the child has suffered or is at risk of suffering significant harm? YES / NO and b) Am I in doubt about the child's safety and the parent's ability to protect the child? YES / NO 2. If you answered yes to a) or b) Go to Step 4 3. If you have significant concerns that a child and their family need a referral to Child FIRST for family services. Go to Step 3		Child Wellbeing Referral 1. Contact your local Child FIRST provider. • See over for contact list for local Child FIRST phone numbers. 2. Have notes ready with your observations and child and family details.		Mandatory/Protective Report* 1. Contact your local Child Protection Intake provider immediately. • See over for contact list for local Child Protection phone numbers. • For After Hours Child Protection Emergency Services, call 131 278. 2. Have notes ready with your observations and child and family details. * <i>Non-mandated staff members who believe on reasonable grounds that a child is in need of protection are able to report their concerns to Child Protection</i>

For further information refer to *Protecting the safety and wellbeing of children and young people – A joint protocol of the Department of Human Services Child Protection, Department of Education and Early Childhood Development, Licensed Children's Services and Victorian Schools*

(Source:

<https://www.education.vic.gov.au/Documents/childhood/parents/health/mandreportsep10.pdf>

)

Early Years obligations and the Reportable Conduct Scheme

I am concerned about a child's safety. What should I do?

Follow the PROTECT guidelines: the Four Critical Actions for Early Childhood Services in responding to incidents, disclosures and suspicions of child abuse



Call Victoria Police on 000 if you have immediate concerns for a child's safety

Victoria Police

You must notify Victoria Police immediately if you have reasonable belief that a child has been abused or that criminal behaviour has occurred. Failure to notify Victoria Police that an adult may have committed a **sexual offence** against a child is a criminal offence.

DFFH Child Protection

You must report to DHHF Child Protection if you have a reasonable belief / reasonable grounds that a child is in need of protection, or their safety or wellbeing is at risk and the child's parent/carer has not protected or is unlikely to protect the child from harm.

(Victorian Early Childhood Regulatory Authority- VECRA) and Victorian Institute of Teaching

You must notify your regulatory body of any incidents, circumstances or complaints which raise concerns about the safety and/or wellbeing of children using your service. This includes notifying VIT if the source suspected

Reportable Conduct

The Reportable Conduct Scheme (the Scheme) operates alongside other reporting obligations. The scheme applies to all Victorian early childhood services from **1 January 2019**. All workers, volunteers and contractors are covered by the Scheme and the Scheme captures allegations about reportable conduct and misconduct that may include reportable conduct which occurs **both within and external** to your organisation.

There are five categories of Reportable Conduct:

Against, with, or in the presence of a child

Physical violence

Sexual offences

Sexual misconduct

Behaviour that causes significant emotional or psychological harm

Significant neglect

Report to: Social Services Regulator



**Social Services
Regulator**

SSR operates [an online form for heads of organisations to notify of a reportable allegation](#). (SSR requires that heads of organisations use the online form.)

Non - head of an organisation can use the online form or the following contacts:

Phone: 1300 310 778

Email: Contact@ssr.vic.gov.au

Early years workers and volunteers and the Reportable Conduct Scheme

The types of Reportable Conduct are:

Against, with, or in the presence of a child

Physical violence

This can include **hitting, punching, kicking, pushing or throwing something that strikes a child or another person**. It can also include the apprehension of violence – words, gestures or actions that cause a child to believe physical force is about to be used against them.

Sexual offences

Sexual offences are criminal and must be reported to Victoria Police. Sexual offences include sexual abuse, assault, indecent acts, possession of child abuse materials and 'grooming'.

Sexual misconduct

This is conduct of a sexual nature that is not necessarily criminal. It can be evidenced through physical action or communication, **including online**. Misconduct can include unwanted/inappropriate touching, inappropriate exposure or conversations of a sexual nature, overly personal/intimate conduct.

Behaviour that causes significant emotional or psychological harm

This can include severe or sustained verbal abuse, coercive or manipulative behaviour, hostility, rejection, humiliation, belittling and scapegoating children. There must be a **clear link** between the adult behaviour and the child's harm. The harm must be **significant**, and more than trivial or temporary.

Significant neglect

This can occur where there is **deliberate or reckless failure** to meet the basic needs of a child which has had, or could have, considerable effect on the child's safety or wellbeing. Neglect includes **physical, emotional, educational and supervisory neglect** – this can include inadequate supervision or failure to seek or comply with medical treatment.

What does it mean for me?

If you are **over the age of 18** and employed or engaged by an approved education and care or children's service (even if you do not have direct contact with children as part of your role) then you are covered by the Reportable Conduct Scheme.

If the organisation you are employed or engaged by is covered by the Scheme, your conduct **both within the organisation and external to it** is covered by the Scheme. This includes as a **volunteer or contractor**.

The Scheme applies to all employees and those engaged by an organisation regardless of the length of time they are employed or engaged – including part-time, full-time, casual or sessional roles.



What do I need to do?

If you think that a worker, volunteer or anybody else covered by the Scheme has committed reportable conduct, or misconduct that may involve reportable conduct, either within or outside of the organisation, then you should report this to the Head of Organisation.

The head of the organisation must report it to the Social Services Regulator (SSR) within 3 days. Additionally, worker, volunteers or parent can report it to the SSR.

Under the National Law and the Children's Services Act (CS Act), the approved provider of an education and care service must ensure that an incident, injury, trauma and illness record is kept. This template aligns with this requirement, and it's strongly recommended that all early childhood service staff utilise this template for incidents, disclosures and suspicions of child abuse.

As an early childhood staff member, you must keep clear and comprehensive notes relating to incidents, disclosures and allegations of child abuse.

It's strongly recommended that you use this template:

<https://www.education.vic.gov.au/childhood/professionals/health/childprotection/Pages/eccritmustact.aspx>

Your aim should be to provide as much information within the template as possible. These records will be helpful in making a report of the abuse to the relevant authorities. If you require support to complete the template, you should seek support from your manager/service provider.

This section outlines four critical actions to take when responding to an incident, disclosure or suspicion of child abuse.

These actions include:

- [One: Responding to an Emergency](#)
- [Two: Reporting to Authorities](#)
- [Three: Contacting Parents/Carers](#)
- [Four: Providing Ongoing Support](#)

Four critical actions: <https://www.vic.gov.au/child-protection-early-childhood-protect/report-child-abuse-early-childhood>

Appendix A:

Recruitment Process

- The Approved Provider/Nominated Supervisor will oversee and approve the recruitment process:
 - ensuring there is a documented position description for the vacant position that is accurate and current.
 - arranging for the position to be advertised
 - ensuring there is a standard list of interview questions for all applicants
 - reviewing the applications that have been received and making a short list of applicants
 - arranging suitable interview times with the shortlisted applicants
 - contacting referees for the most suitable candidate(s)
 - making an offer of employment in writing which the applicant must sign as an acceptance of the offer. The applicant must sign a contract of employment containing the specific terms and conditions of employment. A base Employment Contract is attached.
 - notifying unsuccessful applicants by letter, telephone or email.
- Recruitment and selection decisions will be made by the Approved Provider/Nominated Supervisor.

Job Description

Every position must have a position description which:

- summarises the job and describes the tasks,
- details the skills, qualifications and experience required to perform the job and whether these are essential or desirable criteria.
- Clearly describes the expectations for educators/staff members to provide a safe and supportive environment for children.

Advertising

- Positions may, at the discretion of the Approved Provider/Nominated Supervisor and where relevant, be initially advertised internally via email. This process gives current employees the chance to be considered for a transfer or nominate a suitable contact as a potential candidate.
- External advertising will occur when a suitable internal candidate (including employee contact) is unavailable, or may occur concurrently with the internal advertising where the Approved Provider/Nominated Supervisor believes it is in the service's best interests to source additional candidates.

The Job Advertisement

The job advertisement will be written in clear, concise and non-discriminatory language and will contain:

- the title of the position
- a summary of the role and conditions of employment
- the essential and desirable criteria for candidates
- information about what applicants should provide with their applications
- clear, concise details about our Service and our safe, supportive work practices
- advice that the successful applicant will need to undergo a successful Working With Children Check
- the name of a contact person
- the closing date for receipt of applications
- a statement that the Service is an Equal Opportunity Employer

Interviews

The Approved Provider/Nominated Supervisor will conduct the interview. The format of the interview will be:

- advise the applicant about the position and the Service
- discuss the applicant's skills and experience as they relate to the position
- discuss the applicant's understanding of child safety and child protection
- answer any questions the applicant may have
- advise the applicant about the next steps in the selection process
- obtain permission to contact the applicant's nominated referees.

Selection of Candidates and Offer of Employment

Following the interviews, we will check the work histories and references of the most suitable candidates(s) after obtaining their permission. If a decision is made to employ the most suitable candidate, we will make a written offer of employment.

The successful applicant must sign a contract of employment containing the specific terms and conditions of their employment.

If a decision is made to employ the most suitable candidate, we will make a written offer of employment.

The successful applicant must sign a contract of employment containing the specific terms and conditions of their employment.

Exit Interviews

If an employee resigns, management will undertake an exit interview with the person to:

- gather information about the effectiveness of the recruitment process.
- identify possible areas for improvement in organisational processes, management, job design, remuneration or career planning and development.
- receive positive feedback on what is working well.

Appendix B provides information on the following:

- **B1: Indicators of Harm, Abuse and Neglect**
- **B2: Family Violence**
- **B3: Children Problem Sexual Behaviours**
- **B4: Recognising Grooming**

B1: Indicators of Harm, Abuse and Neglect

There are many indicators of harm to children. Behavioural or physical signs which assist in recognising harm to children are known as indicators. The following is a guide only. One indicator on its own may not imply harm. However a single indicator can be as important as the presence of several indicators. Each indicator needs to be considered in the context of other indicators and the child's circumstances. A child's behaviour is likely to be affected if he/she is under stress. There can be many causes of stress and it is important to find out specifically what is causing the stress. Abuse and neglect can be single incidents or ongoing, and may be intentional or unintentional.

General indicators of harm, abuse and neglect

- marked delay between injury and seeking medical assistance
- history of injury
- the child gives some indication that the injury did not occur as stated
- the child tells you someone has hurt him/her
- the child tells you about someone he/she knows who has been hurt
- someone (relative, friend, acquaintance, sibling) tells you that the child may have been abused

Physical Abuse

Physical indicators include:

- Bruises, burns, sprains, dislocations, bites, cuts
- Fractured bones, especially in an infant where a fracture is unlikely to occur accidentally
- Poisoning
- Internal injuries
- Bald patches where hair has been pulled out

Possible behavioural indicators include:

- Showing wariness or distrust of adults
- Wearing long sleeved clothes on hot days (to hide bruising or other injury)
- Demonstrating fear of parents and of going home
- Becoming fearful when other children cry or shout
- Being excessively friendly to strangers
- Being very passive and compliant
- Not reacting or showing little emotion when hurt
- Showing little or no fear when threatened
- Often being absent
- Showing regressive behaviour such as bed-wetting
- Often feeling sad or crying

Sexual Abuse

A child is sexually abused when any person uses their authority or power over the child to engage in sexual activity. This can include exploitation through pornography or voyeurism. Sexual abuse is not usually identified through physical indicators. Often the first sign is when a child tells someone they trust that they have been sexually abused. However the presence of sexually transmitted diseases, pregnancy, or vaginal or anal bleeding or discharge may indicate sexual abuse.

Physical indicators include:

- Injury to the genital or rectal area
- Vaginal or anal bleeding or discharge
- Discomfort in toileting
- Inflammation and infection of genital area

- Bruising
- Frequent urinary tract infections

One or more of these behavioural indicators may be present:

- Child telling someone that sexual abuse has occurred
- Complaining of headaches or stomach pains
- Experiencing problems with schoolwork
- Displaying sexual behaviour or knowledge which is unusual for the child's age
- Showing behaviour such as frequent rocking, sucking and biting
- Experiencing difficulties in sleeping
- Having difficulties in relating to adults and peers
- Drawing or telling stories that are sexually explicit
- Showing regressive behaviour such as bed-wetting

Emotional Abuse

Emotional abuse happens when a child is repeatedly rejected, isolated or frightened by threats or by witnessing family violence. It also includes hostility, derogatory name-calling and putdowns or persistent coldness from a person to the extent the child's emotional development and behaviour is at serious risk of being impaired. There are few physical indicators, although emotional abuse may cause delays in emotional, mental, or even physical development.

Physical indicators include:

- Speech disorders
- Delays in physical development
- Failure to thrive

Possible behavioural indicators include:

- Displaying low self esteem
- Tending to be withdrawn, passive, tearful
- Displaying aggressive or demanding behaviour
- Being highly anxious
- Showing delayed speech
- Acting like a much younger child, eg. soiling, wetting pants
- Displaying difficulties in relating to adults and peers
- Showing mental or emotional displays
- Having overly high standards and a fear of failure

Neglect

Physical indicators include:

- Frequent hunger
- Malnutrition
- Poor hygiene
- Inappropriate clothing, eg. Summer clothes in winter
- Left unsupervised for long periods
- Medical needs not attended to
- Abandoned by parents

Possible behavioural indicators include:

- stealing food or gorging when food is available
- staying at school outside school hours
- often being tired, falling asleep in class
- abusing alcohol or drugs
- displaying aggressive behaviour
- not getting on well with peers
- poor socialising habits
- withdrawn, listless, pale and thin

The presence of indicators such as those described may alert us to the possibility that a child is being abused. It is important that anyone who has concerns that a child or young person is in need of protection contacts a local Child Protection Service for assistance and advice.

B2: Family Violence

Family violence, either threatened or actual, occurs within a family, including physical, verbal, emotional, psychological, sexual, financial and social abuse. Child Protection must be informed when there are strong indicators that family violence is placing a child at significant risk if danger.

What is domestic and family violence?

Domestic or family violence occurs when one person in a "relationship" uses violent or abusive behaviour to control another.

The behaviour which may be classified as domestic or family violence includes:

- Physical abuse
- Damage to property
- Sexual abuse
- Verbal abuse
- Harassment or intimidation
- Financial abuse, or
- *Threatening any of the above.*

Domestic violence does not just occur between spouses.

- Spousal — including defacto, biological parents of a child or same sex couples
- Intimate personal — two people in an established relationship (does not have to be sexual)
- Family — related by blood or marriage, or culturally related
- Informal care — unpaid carer who assists with day to day living.

B3: Children Problem Sexual Behaviours

Understanding "Normal" Versus "Problem Sexual Behaviour" (0-4 years old children)

Source: Child Wise.

Website References: www.childwise.net

Reference 1: <http://www.childwise.net/page/39/fact-sheets>

Reference: 2: http://childwise.blob.core.windows.net/assets/uploads/files/Fact%20Sheets/Sexual_behaviour_0to4yearsold.pdf

Reference 3: <http://www.nctsn.org/products/sexual-development-and-behavior-children-information-parents-and-caregivers-2009>

Extract from Fact Sheet: Sexual Behaviour of Children Aged 0 to 4 years Old

To know what sexual behaviour is problematic, we need to first understand what is normal for each age and stage of child development.

Considerations

- *Is the observed sexual behaviour age appropriate or concerning?*
- *What is the context of the sexual play?*
- *Age difference and relationship between children/young people*
- *Do not over or under react **Gil (1993)***

Age appropriate sexual behaviours

0 – 4 years; Pre-school

- *Touching/rubbing his/her own genitals*
- *Likes to be rude*
- *Showing others his/her genitals*
- *Playing doctors and nurses*
- *Playing house; Mummies and Daddies*
- *Touching or looking at private parts of familiar children or adults*
- *Using slang words/dirty language for bathroom and sexual functions*

Very Concerning Sexual Behaviours

- *Persistently touches or rubs self to the exclusion of normal childhood activities; hurts own genitals by rubbing or*
- *touching*
- *Simulating sex with other children with or without clothes on*
- *Oral sex*
- *Sexual play between children involving forceful anal or vaginal penetration with objects.*

Situations of sexualised behaviour and peer sexual play which causes concern (Tony Cavanagh Johnson, 1994)

- *The children engaged in the sexual play do not have an ongoing mutual play relationship*
- *The children engaged in the sexual play/behaviour are of different ages or developmental levels*
- *Any sexual play/behaviour which continues in spite of consistent and clear requests to stop*
- *Sexual behaviours which occur in public or other places where the child has been told they are not acceptable*
- *Sexual play/behaviour which is eliciting complaints from other children or adversely affecting them*
- *Sexual behaviour which progresses in frequency, intensity or intrusiveness over time*
- *Sexual behaviour that is associated with fear, anxiety, deep shame or intense guilt*
- *Children who manually stimulate or have oral or genital contact with an animal*
- *Sexual behaviours which cause physical or emotional pain to self or others*
- *Children who use sex to hurt others*
- *When verbal and/or physical expressions of anger precede, follow or accompany the sexual behaviour*
- *When coercion, force, bribery, manipulation or threats are associated with sexual behaviours*

End of Extract from Child Wise.

Other behavioural indicators can include:

- **Nervousness and anxiety**
- **Withdrawn behaviour**
- **Adjustment problems, few interests and poor school performance**
- **Enuresis (bed wetting)**
- **Psychosomatic illnesses (asthma, stomach aches, headaches etc.)**
- **Excessive cruelty to animals**
- **Aggressive behaviour and language**
- **Pseudo-maturity**
- **Boys imitating aggressive behaviours, bullying/ abusing their mothers.**

B4: Recognising Grooming

Recognising the signs of grooming can be difficult. Grooming behaviours can often look like normal caring behaviours. Anyone can groom a child, including people involved with your organisation. People engaging in grooming behaviour may use social media, the internet and mobile phones to interact with children and ask the child to keep the interaction secret. This could continue for months before the offender arranges a physical meeting. There may be no online element to the grooming.

Some signs of grooming include a child:

- having unexplained gifts or money and not wanting to talk about where they came from
- not wanting to talk about what they've been doing or lying about it
- getting lots of messages from someone they only know online
- spending less time with friends or changing friendship groups suddenly
- not wanting to talk about their day, thoughts or feelings anymore
- regularly missing school, work or other activities
- developing an unusually close connection with an older person
- not wanting others around when they're with particular friends or adults.

Groomers may also try to gain the trust of a child's family or carers including:

- offering to take the child to activities (such as sports) or babysit
- offering to mentor or coach the child individually
- buying gifts or doing things for the family (such as repairs)
- complimenting the family and parenting.

Appendix C

OWNA Incident Response

Overview

OWNA electronic form to respond to a complaint, concern, allegation or disclosure about the safety of a child, per the ACECQA NQF Child Safe Culture Guide. If the matter involves an incident, injury or trauma, also complete the [Incident, Injury, Trauma & Illness Record](#).

Record respond to a complaint, concern, allegation or disclosure about the safety of a child

 Health & Safety > Incident Response

1. Click on the plus **+** button in the top right corner
2. Fill in the relevant information
3. Click **Save Incident Response**

New Incident Response

Home / Incident Response / Add

Use this form to respond to a complaint, concern, allegation or disclosure about the safety of a child, per the ACECQA NQF Child Safe Culture Guide. If the matter involves an incident, injury or trauma, also complete the Incident, Injury, Trauma & Illness Record.

Log the complaint, concern or disclosure

Date received
2026-06-09

Type of matter

-select-

Received by

Subject

-select-

How received

-select-

Details of what occurred

Child concerned (if enrolled)

-- none / not enrolled --

Persons involved
Who was involved or present (children, staff, family, others).

What occurred
A clear, factual, objective account of the complaint, concern or disclosure.

When did it occur
Date / time if known

Location

Witnesses

Or subject name (if not an enrolled child)

Information gathered

All available information
Statements, observations, records reviewed, and any other relevant information gathered in response.

Advice from authorities & notification

Advice sought / authorities consulted
Tick each authority from whom advice was sought on whether a report or notification is required.

☐ Regulatory authority

☐ Child protection

☐ Police

☐ Reportable Conduct Scheme

Other advice / detail

Is a notification required?

-select-

Date notified (if applicable)

yyyy-mm-dd hh:mm

Incidents or allegations of physical or sexual abuse must be notified to the regulatory authority within 24 hours.

Notification notes

5 Initial risk assessment

☐ A risk assessment is required for this matter
 Complete to identify actions needed to mitigate ongoing risk to the child / children.

Hazard / risk identified

Likelihood

-select-

Consequence

-select-

Risk rating (before controls)

-select-

Controls / actions to mitigate risk

Residual rating (after controls)

-select-

6 Course of action

Actions decided, responsible person(s) and timeframes
 What will be done, by whom, and by when.

Communication with family

☐ Family / parent notified

Date family notified

yyyy-mm-dd

Notes

7 Review & continuous improvement

Outcome of the matter

Changes to practice / policy to reduce recurrence

Review date

yyyy-mm-dd

8 Attachments

Upload supporting documents (statements, correspondence, photos, notification confirmations). You can select multiple files. Accepted types: PDF, JPG, JPEG, PNG, GIF.

Choose Files

No file chosen

Current attachments
 No attachments uploaded yet.

☒ Sign-off & status

Completed by

Scott Zannes

Position

Reviewed by (Approved Provider / Nominated Supervisor)

Status

Open

Cancel

Save incident response

Disclosure of harm

A disclosure of harm occurs when someone, including a child, tells you about harm that has happened, is happening, or is likely to happen to a child. Disclosures of harm may start with:

- I think I saw...||
- Somebody told me that...||
- Just think you should know...||
- I'm not sure what I want you to do, but...||

Child's name _____

What is the name of the person who made the disclosure? _____

Are they related to the child? Yes ☐ No ☐

If yes, what is the relationship? _____

What did the person disclose? Try to use the exact words they used. Use "I said" "they said" statements, include any questions you asked and comments you made

What date did the person make the disclosure? _____ What time? _____AM/PM

Where did the disclosure occur? _____

Was anyone else present during the disclosure? Yes ☐ No ☐ If yes what is/are their name, role and employer?

Have you followed the procedure in the Child Safety, Protection & Wellbeing Policy for making a report?

Yes ☐ No ☐

Describe the actions you have taken following the disclosure

Name of person completing form

Date _____

Signature of person completing form

Time _____ AM/PM

Suspicion of harm

Educators may suspect harm if:

- a child says they have been harmed
- someone else, for example another child, a parent, or an employee, says harm has occurred or is likely to occur
- a child says they know someone who has been harmed (it is possible that they may be referring to themselves)
- they are concerned at significant changes in the behaviour of a child, or the presence of new unexplained and suspicious injuries
- they see the harm happening.

Child's name:

Why do you suspect harm? Try to use the exact words a child or someone else uses if relevant. Provide as much detail as possible

[illegible]

If relevant, what date did the person say something? _____ What time? _____ AM/PM

Have you followed the procedure in the Child Safety, Protection & Wellbeing Policy for making a report?

Yes ☐ No ☐

Describe the actions you have taken because of your suspicion

Name of person completing form

Signature of person completing form

Date _____ Time _____ AM/PM

Appendix D

CHILD PROTECTION RISK MANAGEMENT STRATEGY BREACH INCIDENT REPORT FORM

Date breach occurred _____ Time breach occurred _____

Location of breach _____

Name of person(s) involved in the breach

Description of the breach

Immediate action taken

If no action taken – reason

Name of person completing form

Signature of person completing form

Date _____

Time _____ AM/PM

Authority breach reported to (if relevant)

Name of person reported to

Appendix E

Child Protection Risk Management Strategy – Template for High Risk Activity	 Management
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STEP 1	STEP 2	STEP 3	STEP 4	STEP 5	STEP 6
Describe the activity <i>Identify all elements of the event from beginning to end eg activity, objectives, location, participants</i>	Identify Risks <i>Something that could happen that results in harm – also consider physical, emotional, sexual and cultural risks from children, adults, visitors, employees, volunteers</i>	Analyse the Risk <i>How likely is the risk, what would happen if the risk did occur?</i>	Evaluate the Risk <i>Likelihood/Consequences</i>	Manage the Risk <i>Assess the options to reduce the risk</i>	Review <i>Nominate who will review after the event/activity</i>

Determine likelihood of the risk by using the left hand column of the Risk Analysis Matrix (below). Use the impact information to determine the consequences level. Combine the Consequence and Likelihood ratings to arrive at the Risk Level (i.e. <i>Low, Medium, High</i> or <i>Critical</i>). CONSEQUENCES					
LIKELIHOOD	Insignificant	Minor	Moderate	Major	Extreme
Very likely Expected to occur in most circumstances	Medium	Medium	High	Critical	Critical
Likely Will probably occur in most circumstances	Low	Medium	High	High	Critical
Possible Might occur at some time	Low	Medium	Medium	High	High
Unlikely Not expected to occur	Low	Low	Medium	Medium	High
Rare Occurs in exceptional circumstances only	Low	Low	Low	Medium	Medium

Appendix F

Working with Children Check record-keeping sheet example (Also See Services Staff Record Schedule)

Club name/location:				Year:			
Clubs, associations or movements (sports, cultural, recreational, etc.) that provide services or activities for children, or whose membership is mainly comprised of children, must ensure that all paid and volunteer workers doing child-related work have a Check. Some clubs and associations have policies that exceed the minimum requirements of the <i>Working with Children Act 2005</i>. Check this with your club or association.							
Responsible person & contact details:							
Position requiring the Check	Current holder of position	Volunteer/paid	Does an exemption apply/why?*	Application receipt number	Check number	Card sighted?	Expiry date

*You should periodically re-assess who is exempt in your organisation. For example, a parent was exempt because their child is involved in an activity, but is their child still involved in the activity?

Appendix G

Educating Children about Protective Behaviour

Educators will regularly include child protection issues in the curriculum. For example they will intentionally teach children:

- about acceptable/unacceptable behaviour, and appropriate/inappropriate contact in a manner suitable to their age and level of understanding
- that they have a right to feel safe at all times
- to say 'no' to anything that makes them feel unsafe
- the difference between 'fun' scared that is appropriate risk taking and dangerous scared that is not ok
- to use their own skills to feel safe
- to recognise signs that they do not feel safe and need to be alert and think clearly
- that there is no secret too awful, no story too terrible, that they can't share with someone they trust
- that educators are available for them if they have any concerns
- to tell educators of any suspicious activities or people
- to recognise and express their feelings verbally and non-verbally
- that they can choose to change the way they are feeling.

Educators believe that:

- children are capable of the same range of emotions as adults
- children's emotions are real and need to be accepted by adults
- an adult's response to a child during their early emotional development can be hugely positive or detrimental depending on the adult's reaction
- children are very in touch with their bodies' reactions to their emotions
- children who better understand their body's response to an emotion are more able to foresee the outcome of a situation and avoid them or ask for help.

Appendix H: CONTACTS

SOCIAL SERVICES REGULATOR

Website: <https://www.vic.gov.au/about-social-services-regulator>

Notify or update about reportable allegations under the Reportable Conduct Scheme:
<https://rcs.ssr.vic.gov.au/>

Please note: If you have concerns about the immediate safety of any person, please call [000](tel:000).

If you'd like to ask us any questions before notifying SSR about a reportable allegation, please contact them via email reportable.conduct@ssr.vic.gov.au , or call 1300 310 778

If you need an interpreter, please call the Translating and Interpreting Service on [13 14 50](tel:131450) and ask them to contact the SSR on [1300 310 778](tel:1300310778)..

COMMISSION FOR CHILDREN AND YOUNG PEOPLE

Website: <https://ccyp.vic.gov.au/>

CHILD SAFETY OFFICER

Department of Justice and Community Safety
GPO Box 4356
MELBOURNE VIC 3000
Phone: (03) 8803 8571
Email: reportable.conduct@justice.vic.gov.au

DHHF CHILD PROTECTION

AREA
North Division 1300 598 521
South Division 1300 555 526
East Division 1300 360 452
West Division 1300 360 462
AFTER HOURS (24 hours)
After hours, weekends, public holidays 13 12 78

ORANGE DOOR

Please refer to the following website: <https://orangedoor.vic.gov.au/find-a-service-near-you>

THE ORANGE DOOR IN MALLEE

Ph: 1800 290 943
Email: mallee@orangedoor.vic.gov.au
113 Madden Avenue Mildura 3500
9am to 5pm Monday to Friday (closed public holidays)

Services offered:

- Women, children and young people's family violence services
- Child and family services
- Aboriginal services
- Men's family violence services

THE ORANGE DOOR IN BAYSIDE PENINSULA

Phone: 1800 319 353
Email: bpa@orangedoor.vic.gov.au
60-64 Wells Street, Frankston 3199
9am to 5pm Monday to Friday (closed public holidays)
Services offered:

- Women, children and young people's family violence services

- Child and family services
- Aboriginal services
- Men's family violence services

VICTORIA POLICE

000 or contact your local police station

VICTORIAN EARLY CHILDHOOD REGULATORY AUTHORITY (VECRA)

Enquiry line: 1300 307 415.

NORTH-WESTERN

Loddon Mallee (03) 4433 7502

Northern Metropolitan (03) 7005 1989

SOUTH-EASTERN

Gippsland Area (03) 8904 2500

Southern Metropolitan (03) 8904 2500

NORTH-EASTERN

Eastern Metropolitan 03 5771 4471

Hume (03) 5771 4471

SOUTH-WESTERN

Barwon South West (03) 5215 5136

Western Metropolitan (03) 7005 1801

Grampians (03) 4334 0589

Sexual Assault Crisis Line DHHS - Phone: 1800 806 292 (24 hour service)

Mallee Sexual Assault Unit: Mildura. Phone: 03 5025 5400 (24 hour service)

Safe Steps – Family Violence Response Centre (<http://www.safesteps.org.au/>): 1800 015 188 (24 hour service)

Evaluation and Review

The policy will be reviewed annually and will be conducted by:

- Management
- Employees
- Families
- Interested Parties

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2027

References:

NQS

QA2	2.2.3	<i>Child protection - Management, educators and staff are aware of their roles and responsibilities to identify and respond to every child at risk of abuse or neglect.</i>
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National Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	84	<i>Awareness of child protection law</i>
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Legislation References:

Child Wellbeing and Safety Act 2005 (includes Child Safe Standards and Child Information Sharing Scheme)

Child Wellbeing and Safety (Information Sharing) Regulations 2018

Child Information Sharing Scheme Ministerial Guidelines VIC Govt

Children, Youth and Families Act 2005

Crimes Act 1958

Education and Care Services National Law and Regulations

Education and Training Reform Act 2006

Family Violence Protection (Information Sharing and Risk Management) Regulations 2018

Family Violence Protection Act 2008

Family Violence Information Sharing Guidelines: VIC Govt

Licensed Children's services and Victorian Schools "Protecting the Safety and Wellbeing of Children and Young People"

Worker Screening Act 2020

Sources and Further Reading:

Additional information on Child Protection can be obtained by contacting your Support Manager. Other useful websites include;

- <http://www.education.vic.gov.au/about/programs/health/protect/Pages/ecguidance.aspx>
- **National Association for the Prevention of Child Abuse and Neglect - www.napcan.org.au**
- **Kids Helpline – www.kidshelp.com.au**
- **Dept of Child Safety (Qld) – www.childsafety.qld.gov.au**
- <http://www.aifs.gov.au/nch/> **National Child Protection Clearing House retrieved Jan 2009**
- <http://www.napcan.org.au/> **National Association for Prevention of Child Abuse and Neglect retrieved Jan 2009**
- <http://www.unisa.edu.au/childprotection/> **Australian Centre for Child Protection retrieved Jan 2009**
- <http://www.aihw.gov.au/> **Australian Institute of Health and Welfare retrieved Jan 2009**

12C – SAFEGUARDING CHILDREN: LOOK, DO & TELL FRAMEWORK POLICY

Position Statement

Our organisation is committed to safeguarding children from abuse, neglect, and harm. Ensuring the safety, health, and wellbeing of children, families, and our people is our first and most important obligation.

Objective

Our Service takes very seriously its moral, ethical, and legal obligations to safeguard children from harm, abuse, and neglect. Our Child Safe Framework together with the Look, Do, Tell Framework (for Safeguarding Children) underpin our approach to the prevention, identification and response to child harm, abuse and neglect and provides a common language about the protection of children.

All staff play a vital role in protecting children from harm, abuse, or neglect by responding to and reporting any disclosures, reasonable suspicions, incidents, or complaints. Seeing, understanding, and responding well to child harm, abuse and neglect can make the difference between a child feeling heard and understood and a child feeling silenced and unlikely to seek help again. A shared approach to child safety and wellbeing can improve safety and lead to support and healing for a child and their family.

Related Policies, Procedures and Documents

- 7 - Complaints and Grievance Policy
- 8 - Family and Visitor Code of Conduct Policy
- 11 - Supervision Policy
- 12A – Child Safe Standards Policy
- 12B – Child Safety, Protection & Wellbeing Policy
- 16 - Privacy and Confidentiality Policy
- 19 - Incident, Injury, Trauma and Illness Policy
- 20 - Emergency Management and Evacuation Plan Policy and Procedure
- 34 - First Aid Policy
- 55 - Student and Volunteer Policy
- 56 - Team Member Self Protective Behaviour Policy
- 70 - Ethical Conduct and Code of Ethics Policy
- 72 - Interaction with Children Policy\
- 78 - Governance of Service, Management and Educator Policy
- 81 - Hazard Risk Management Policy Procedure and Program
- 86 - Technology & Digital Usage Policy
- 99 - Family Violence Safety Policy
- 100 - National Model Code Policy
- 102 - Protected Disclosures (Whistleblowers) Policy

Responsibilities

Our people play a vital role in bringing our commitment to safety and wellbeing to life. Every member of our organisation is responsible for prioritising the safety, health, and wellbeing of children, families, colleagues, and themselves in everything they do.

Together, we foster a culture where children are safe, respected, and supported, and where everyone feels empowered to contribute to a positive and protective environment. Creating and maintaining a child-safe culture is a shared responsibility. We are committed to modelling safe practices at every level and encouraging open communication so that concerns can be raised without fear.

By following our Look, Do, Tell Framework, we all play an important role in preventing, recognising, and responding to child abuse, neglect, and harm, ensuring the safety and wellbeing of every child in our care

Our Look, Do, Tell Framework is to be implemented by all organisation workplace participants and operational environments including:

- all employees across education and care services, funded programs, inclusion services and all head office employees
- all volunteers, students, school-based trainees and apprentices and regular service visitors (e.g., allied health workers)
- contractors and consultants (and their personnel) who deliver services to children, or who are otherwise involved in organisation projects, or funded activities involving children must also comply with this framework in accordance with relevant contractual obligations.

Wellbeing and Child Safety Learning Plans and our resource library provide the knowledge, tools, and guidance needed to effectively implement this framework and uphold our commitment to child safety.

Implementation

It is our collective responsibility to safeguard children from harm, abuse, and neglect.

If you see or hear something, you must take action, and you must say something.

Please see Appendix 1 and Appendix 2 for Look, Do and Tell Framework posters to be displayed in the service.

	LOOK Be the protective 'eyes and ears'. Did you see or hear something? Be observant and curious. Spot the signs of child harm, abuse, and neglect. Stay tuned in to your environment. Are there spaces in your environment where it is difficult to see what is happening? Children might tell you directly, or indirectly through their behaviour about their experience of harm. A parent, carer, staff member or another child might be acting in a way that is harmful to children.
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	DO Take action immediately. Your role is to ensure the child's safety first and foremost. Once you identify risk to a child you need to act in the moment. Step in. Protect immediately. Reassure a child who has disclosed. Ask a parent/carer or staff member if you can help them - or tell a colleague you are going to help them. You might also need to act after the incident. Such as identify and link in supports for children and families, provide education and information.
	TELL Tell someone. Your observations help to keep children safe. You MUST tell someone about what you saw or heard. Do NOT stay silent. Report your concern immediately. Your actions can stop harm and make a difference in a child's life. By speaking up, you ensure the issue is addressed properly and that every child remains safe in our care. Who you tell varies depending on the type of incident. All allegations, reasonable suspicions and complaints alleging child harm, abuse or neglect must be reported to Centre Director /Nominated Supervisor, Responsible Person in Charge of Service or the Approved Provider/Service Owner within 2 hours and on the same day. If you are concerned about: • Physical Violence • Sexual Offences • Sexual Misconduct • Behaviour that causes significant emotional or physical harm • Significant neglect Follow the PROTECT guidelines for reporting child abuse in an early learning centre. Contact the following: • You must also report internally to management who will notify your Approved Provider/Service Owner. • Victoria Police • DFFH Child Protection

	• VECRA and/or VIT • Social Services Regulator <i>(Contact details can be found in policy 12B - Child Safety, Protection & Wellbeing Policy)</i> See website “Report child abuse in early childhood”: https://www.vic.gov.au/report-child-abuse-early-childhood If you are unsure about who to tell and what to do, tell your Centre Director/Nominated Supervisor or Responsible Person and report it. Your Centre Director and Approved Provider will support you through the matter. If you believe a child is at immediate risk or requires emergency medical treatment, call 000.
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Code of Conduct

Our Code of Conduct outlines the standards, values, and behaviours expected of everyone who represents our organisation. It applies to all employees, leaders, students, volunteers, contractors, trainees, and service providers.

The Code reflects our commitment to acting with integrity, professionalism, and respect in everything we do. It requires every person representing our organisation to consistently uphold our values and demonstrate appropriate behaviours in every action and decision.

Our Code of Conduct clearly defines our expectations and responsibilities, including our commitment to creating and maintaining a child-safe environment.

The Code and Policies sets out the standards for how all representatives of our organisation must behave around children at all times—whether on our premises, at work-related activities, in the community, or online.

These expectations apply both during and outside of work hours to ensure children are always safe, respected, and protected from harm.

See 70 - Ethical Conduct and Code of Ethics Policy for more information.

Know Children's Early Warning Signs of Feeling Uncomfortable, Scared or Unsafe



LET'S END CHILD SEXUAL ABUSE
ONE TALK AT A TIME

Australian Government
National Office for Child Safety

E2
EDUCATE
EMPOWER
PUBLISHING

www.ChildSafety.gov.au/one-talk-time

www.e2publishing.info

Content has been adapted from Educate2Empower Publishing with permission.

(Source: <https://www.childsafety.gov.au/resources/my-early-warning-signs>)

Child Safe Standards

Please refer to Policy 12A - Child Safe Standards Policy for how the service implements the 11 Victorian Child Safe Standards and National Child Safety Principles.

Definitions

Domestic or family violence: The use of violence or abuse to hurt, control or bully someone else in an intimate personal, family, or informal carer relationship. Children and young people can be adversely affected by violence whether they witness it, hear it, or are exposed to the consequences of emotions post the violence. They do not need to be directly involved to be impacted.

Emotional/psychological harm: The intentional use of inappropriate comments or behaviour towards a child which results in significant emotional harm or trauma to a child. This includes, verbal abuse, threatening a child or using manipulative behaviour, hostility towards or rejection of a child, deliberately making a child feel unimportant or embarrassed, blaming a child or exposure to domestic and family violence or stories and/or media about violence. Evidence of psychological harm could include the child displaying patterns of out of character behaviour, regression in behaviour, distress, anxiety, physical symptoms, or self-harm.

Ill treatment: Seriously inappropriate, cruel, or inhumane treatment of a child such as making excessive or degrading demands of a child; a pattern of hostile or degrading comments or behaviour towards a child; and using seriously inappropriate forms of behaviour management towards a child. This includes handling children aggressively such as pulling or grabbing children, particularly by their arms, legs, or neck, swinging or picking up children by their arms or repeatedly swearing, yelling, screaming, scolding, or intimidating children verbally or physically.

Inappropriate behaviour guidance: Inappropriate, unacceptable and damaging methods of attempting to control children's behaviour, such as yelling at a child, deliberately embarrassing a child, handling a child in an inappropriate way, withholding food or drink from a child, unreasonably restraining a child (this may include restraint in a high chair), excluding children from events, consistently moving children to the office or other space away from the play areas and/or moving children to another room as punishment.

Physical assault: The intentional use of physical force against a child that results in, or has a high likelihood of resulting in, a serious injury to the child. This includes hitting, kicking, shaking, biting, pinching, spitting, scratching, strangling, scalding, burning, poisoning, suffocating, physically restraining a child with unreasonable force (this may include forcefully holding or pinning a child, pinning a child's arms down/back, tying up or sitting on a child, physically forcing a child to sleep or rest) and making threats to physically harm a child through words or gestures regardless of whether the person actually intended to apply any force.

Restraint: The application of physical force for the purpose of restraining a child's body. Restraint does not include holding a child without force in order to comfort the child or holding a child's hand to safely escort them from one area to another. Occasionally, there may be circumstances where a child becomes an imminent risk to themselves and may need to be removed from the situation or physically restrained to prevent harm to themselves. Children should be physically restrained only in emergency situations when a child is in a clearly unsafe situation such as attempting to scale a fence or run onto a road, behaving in ways that are destructive to themselves. Any force used must be reasonable in all the circumstances and no more force should be used than is necessary. If the force used is unreasonable, or is more than is needed, the act could be physical assault.

Sexual misconduct: There are four categories of sexual misconduct:

- Crossing professional boundaries - behaviour that can be construed as involving an inappropriate and overly personal or intimate relationship with and/or conduct towards and/or focus on a child or group of children. This includes intimate relationships with children, staff members taking children to their house or in a personal vehicle, seeing children outside of work when no family connection exists (including foster care arrangements), engaging with children through personal social media accounts, phone, email, or other electronic communication. It also includes inappropriate kissing and being overly affectionate toward children.
- Sexually explicit comments and other overtly sexual behaviour - behaviour which includes, but is not limited to sexualised behaviour and sexual behaviour towards children, inappropriate conversations of a sexual nature, unwarranted and
- inappropriate touching involving a child, personal correspondence, and communications with a child in relation to the adult's romantic or sexual feelings for a child, watching children undress in circumstances where supervision is not required or is clearly inappropriate.
- Grooming behaviour - a behaviour or pattern of behaviour that demonstrate that the staff member is grooming the child for sexual activity and where there is no other reasonable explanation for the behaviour. Grooming behaviour can include but is not limited to persuading a child or group of children they have a 'special' relationship by spending inappropriate special time with the child, showing special favours, inappropriately giving gifts or asking the child to keep the relationship to themselves as well as grooming the adults that have contact with that child in order to avoid detection.
- Sexual offence: Criminal offences involving a sexual element that are 'committed against, with or in the presence of a child'. This includes, indecent assault, sexual assault, sexual intercourse and attempted sexual intercourse, possession/dissemination/production of a child pornography or child abuse material, using children to produce pornography, grooming, or procuring children under the age of 16 years for unlawful sexual activity and non-consensual sexual activity.

Significant neglect: An intentional or reckless failure to meet the basic needs of a child, where the adult understood the needs of the child, and had the opportunity to meet those needs but failed to do so. This includes, inadequate supervision, failure to keep a child safe, failure to meet a child's physical needs including the provision of adequate and appropriate food, clothing, shelter, medical needs or physical hygiene needs, failure to ensure that a child's formal education needs are being met and failure to provide adequate nurturing, affection encouragement and support to a child.

Supervisory neglect: an intentional or reckless failure to adequately supervise a child that results in the death or harm to a child or involves a gross breach of professional standards and has the potential to result in death or harm to a child.

Staff and Volunteers: A paid staff member or paid or unpaid volunteer or student, engaged at service or other work location, for the purpose of educating and caring for children or providing support or services.

LOOK DO TELL

**We all have a responsibility to keep children safe.
If you see something, if you hear something, you must say something.**



Be the protective eyes and ears.

- ✓ Did you see or hear something? Notice and be curious.
- ✓ Spot the signs of child harm, abuse and neglect.
- ✓ Stay tuned in to your environment. Are there spaces where it is difficult to see what is happening?
- ✓ Children might tell you directly, or indirectly through their behaviour about their experience of harm.
- ✓ A parent, carer, staff member or another child might be acting in a way that is harmful to children.



Take action immediately.

- ✓ Your role is to ensure the child's safety first and foremost.
- ✓ Once you identify risk to a child you need to act in the moment. Step in. Protection immediately.
- ✓ Reassure a child who has disclosed. Ask a parent/carer or staff member if you can help them – or tell a colleague you are going to help them.
- ✓ You might also need to act after the incident. Such as identify and link in supports for children and families, provide education and information.
- ✓ For more information on policies and procedures see, 12B – Child Safety, Protection & Wellbeing Policy and other specific policies.



Tell someone.

- ✓ You **MUST** tell someone about what you saw or heard.
- ✓ Your actions can stop harm and make a difference in a child's life. By speaking up, you ensure the issue is addressed properly and that every child remains safe in our care.
- ✓ Who you tell varies on the type of incident. You may need to report the matter to the Police, Child Protection, VECRA, VIT and the Social Services Regulator.
- ✓ All allegations, reasonable suspicions and complaints alleging child harm, abuse or neglect must be reported to Centre Director / Nominated Supervisor, Responsible Person in Charge of Service or the Approved Provider/Service Owner within 2 hours and ideally on the same day.



**If you believe a child is at immediate risk
or requires emergency medical treatment
call 000.**

CALL 000

LOOK DO TELL

All children have the right to feel safe, be safe and be heard.



If you **SEE** something or **HEAR** something or something happens





that makes you **FEEL** unsafe

your body lets you know



- Hair feels like it is standing on end
- Start to cry
- Goose bumps
- Sweaty hands
- Need to go to the toilet
- Sweaty brow
- Heart beats fast
- Shaky all over
- Feel sick or butterflies in the tummy
- Wobbly legs



Tell your safe people



Appendix 3: Responding to complaints, concerns, allegations and disclosures (Checklist)

(Source: Child Safe Culture Guide – Complaints and Disclosures)

https://www.acecqa.gov.au/sites/default/files/2025-06/CSCG_Incident%20Response%20%20Template_AttC.pdf

This checklist has been developed for the service to respond to a complaint/concern about the safety of a child or children, and/or when a disclosure is made by or about a child at the education and care service. (The checklist is from the “Child Safe Culture Guide – Complaints and Disclosures”)

If the complaint or disclosure relates to an incident, injury to, or trauma of a child, refer also to the Incident, Injury, Trauma and Illness Record.

For guidance on managing complaints and disclosures and more detail on the steps outlined in the template below, refer to the Child Safe Culture Guide – Complaints and Disclosures and:

- 7 - Complaints and Grievance Policy
- 12A – Child Safe Standards Policy
- 12B – Child Safety, Protection & Wellbeing Policy

Action	Details	Yes	No	Comments / Responsible Person
1. Record the incident/allegation	Ensure all details are accurately documented.	☐	☐	
2. Conduct an initial risk assessment	Identify immediate hazards or risks and determine how they will be managed.	☐	☐	
3. Notify the Regulatory Authority	Confirm whether notification is required and identify the purpose of the notification. E.g. VECRA	☐	☐	
4. Report to external authorities	Notify relevant agencies as required (Police, Child Protection Agency, Reportable Conduct Scheme, VIT, eSafety Commissioner, ACCCE, etc.).	☐	☐	
5. Notify parents and carers	Seek guidance from external authorities regarding who, when, and how to communicate with children, parents, and carers.	☐	☐	
6. Collect and secure evidence	Identify and secure relevant records, CCTV footage (if available), witness statements, and other evidence. Confirm secure storage arrangements.	☐	☐	
7. Liaise with NQF Regulatory Authority and external agencies	Maintain ongoing communication and advice. Identify who will manage these relationships.	☐	☐	
8. Conduct internal investigation	Determine whether an internal investigation is required and identify participants.	☐	☐	

Action	Details	Yes	No	Comments / Responsible Person
9. Support the child and family	Keep the family informed and provide appropriate support throughout the process. Identify responsible person.	☐	☐	
10. Consider other children and families	Determine who else may need to be informed and confirm with authorities what information can be shared, with whom, and when.	☐	☐	
11. Support staff	Provide support, wellbeing assistance, and/or referrals for staff members involved.	☐	☐	
12. After the investigation	Record the process and outcome, ensure records are securely stored, and review related policies, procedures, risk assessments, and self-assessment processes.	☐	☐	

Appendix 4: Disclosure / Written Statement for Child Safety Incident

Disclosure / Written Statement Objective

- The service is investigating a child safety matter and is seeking information relating to any concerns in relation to child safety.
- The statement is a chance for you to put forward your understanding of an incident, issue or concern.
- This statement may be provided to Police, VECRA, SSR, VIT, Child Protection and other relevant agencies.
- Please note that this statement will be used to identify the issue, make any reports to agencies and resolve the issue or incident in accordance with service policies and the regulations.

Name of Person Completing Disclosure/Statement:	
Date and Time Statement written:	

Are you familiar with the service policies on child safety and protection and your duties to report any incidents of harm or abuse of children? YES / NO

Do you have any concerns or suspicions in relation to the child safety, harm or abuse of any children in the service? YES / NO

If you answered "YES" to the above question, please complete the remainder of this form. If you require more space, please attach and number additional pages.

If you have answered "NO" to the above question, please put "N/A" in each question box below to advise this information is 'Not Applicable'.

- **Record the details of incident, issue or concern below (What? When? Who? Where? Anything Else?)**
- **What is the primary concern? Any secondary concerns? What outcome is sought?**
- **Are there any immediate safety concerns that need to be addressed?**
- **Is anyone else aware of the issue, or did anyone observe the incident?**

Peer: Date and time when your first became aware of alleged incident, issue or concern?

Date:

Time:

Describe where you were located and positioned. (Eg what part of room or yard. Proximity/distance/facing in relation to people. Task doing at the time? Use a room or yard diagram if easier to indicate location of people).

Where you were rostered at the time?:

Describe your role in the room at the time?:

(Eg Permanent educator in room, Floater, Relief, Break-cover)

Please advise of any policies, procedures, practices and protocols that relate to the incident, issue or concern?

Please advise of any other incidents, issues or concerns? *(That may or may not be related to this matter?)*

Any other comments or notes:

Please sign the statement to indicate that it is a true and accurate record.

Peer Signature: _____ **Date:**

Appendix 5: Responding to suspected child abuse: Template for All Victorian Early Childhood Services

(Source: <https://www.vic.gov.au/report-child-abuse-early-childhood>)

PROTECT



Education
and Training

When to use this template?

It is strongly recommended that **all*** early childhood service staff utilise this template to document any suspicion that a child has been, is being, or is at risk of being abused.

If needed, staff should be supported by management to complete this template, and to ensure that they meet their obligations.

This template should be used in conjunction with following the **Four Critical Actions For Early Childhood Services: Responding to Incidents, Disclosures and Suspicions of Child Abuse**.

Completing this template should not impact on reporting times. If a child is in immediate danger staff should immediately contact Victoria Police on 000.

Whilst you may need to gather the information to make a report, remember it is not the role of staff to investigate abuse, leave this to Victoria Police and/or DHHS Child Protection.

Why record this information?

When completing this template your aim should be to provide as much information as possible. This information will be critical to any reports and may be sought at a later date if the matter is the subject of Court proceedings. These notes may also later assist you if you are required to provide evidence to support any decisions.

Responding to an Incident, Disclosure or Suspicion of Child Abuse

PLEASE NOTE: IF YOU ARE MAKING A REPORT TO DHHF CHILD PROTECTION OR VICTORIA POLICE YOU MUST SEEK ADVICE BEFORE CONTACTING PARENTS/CARERS SO AS NOT TO COMPROMISE ANY INVESTIGATION OR PLACE A CHILD AT FURTHER RISK

STAFF MEMBER LEADING THE RESPONSE

NAME:

OCCUPATION:

LOCATION (SCHOOL ADDRESS):

RELATIONSHIP TO CHILD:

Critical Action 1: immediate response to an incident

If anyone is in immediate danger staff should report immediately to Victoria Police on 000.

See action 1 of Four Critical Actions For Early Childhood: Responding to Incidents, Disclosures and Suspicions of Child Abuse.

RESPONDING TO AN EMERGENCY

DID THE CHILD REQUIRE FIRST AID? PROVIDE DETAILS IF 'YES'.

WHO ADMINISTERED THIS? (NAME AND TITLE)

DID THE CHILD REQUIRE FURTHER IMMEDIATE MEDICAL ASSISTANCE?

CURRENT LOCATION AND SAFETY STATUS:

E.G. ARE ALL IMPACTED STUDENTS SAFE AND NOT IN ANY IMMEDIATE DANGER?

IF A CHILD IS IN IMMEDIATE DANGER SCHOOL STAFF SHOULD REPORT IMMEDIATELY TO VICTORIA POLICE ON 000

Information of the alleged victim

CHILD'S PERSONAL DETAILS	
NAME:	GENDER:
RELATIONSHIP TO SERVICE: (E.G. 2 DAY, 3 YEAR OLD KINDER)	DATE OF BIRTH:
RESIDENTIAL ADDRESS:	
PARENT/CARER NAME/S:	
PARENT/CARER CONTACT:	
LANGUAGE(S) SPOKEN BY CHILD:	
DISABILITIES, MENTAL OR PHYSICAL HEALTH ISSUES:	

CHILD'S BACKGROUND
CULTURAL STATUS AND RELIGIOUS BACKGROUND
PREVIOUS HISTORY OR INDICATORS OF SUSPECTED ABUSE

FAMILY BACKGROUND

FAMILY COMPOSITION (IF KNOWN):

LIST PARENTING OR CARE ARRANGEMENTS AND SIBLING NAMES AND AGES

ANY OTHER PEOPLE LIVING WITH THE CHILD (IF KNOWN):

FAMILY BACKGROUND

DISABILITY, MENTAL OR PHYSICAL HEALTH ISSUES IN FAMILY (IF KNOWN):

LIKELY REACTION TO A REPORT BEING MADE (IF KNOWN):

Details of the incident, disclosure or suspicion

Grounds for your belief that a child has been, or is at risk of abuse

Indicators or instances which led you to believe that a child/children are subject to child abuse, or at risk of abuse:

Detail any disclosures or incidents or suspicions (including names, times and dates documenting a child's exact words as far as possible). Include specific detail here on what led you to form a reasonable belief that a child has been, or is at risk of being abused.

Any physical indicators of abuse:

Any behavioural indicators of abuse:

Any patterns of behaviour or prior concerns leading up to an incident, disclosure or suspicion:

DETAILS OF PERSONS ALLEGED TO HAVE COMMITTED THE ABUSE (IF KNOWN)

NAME:

GENDER

DATE OF BIRTH:

RELATIONSHIP TO CHILD:

NOTHING IF THEY ARE WITHIN THE SCHOOL OR WITHIN THE FAMILY AND COMMUNITY (THIS WILL IMPACT ON WHO YOU REPORT TO)

ADDRESS:

CONTACT DETAILS:

Critical Action 2: Reporting

See Action 2 of Four Critical Actions for Early Childhood Services: Responding to Incidents, Disclosures and Suspicions of Child Abuse

REPORTING TO AUTHORITIES

TICK THE AUTHORITIES YOU HAVE REPORTED TO:

- ☐ VICTORIA POLICE
- ☐ DHHF CHILD PROTECTION
- ☐ VECRA
- ☐ VIT (if registered Teacher)
- ☐ SOCIAL SERVICES REGULATOR
- ☐ THE ORANGE DOOR
- ☐ DECISION NOT TO REPORT

IF YOU'VE DECIDED NOT TO REPORT, LIST YOUR REASONS HERE. ALSO INCLUDE ANY FOLLOW-UP ACTIONS UNDERTAKEN BY YOU BELOW:

PROVIDE OF YOUR REPORT

DATE:

TIME:

AUTHORITY:

NAME OF PERSON SPOKEN TO:

OUTCOMES FROM THE REPORT:

REPORTING INTERNALLY

PROVIDE DETAILS OF YOUR DISCUSSION WITH NOMINATED SUPERVISOR & APPROVED PROVIDER

TIME:

DATE:

NAMES:

DISCUSSION OUTCOMES:

NOTIFICATION TO THE REGULATOR (LICENSED AND APPROVED SERVICES):

ALL LICENSED AND APPROVED SERVICES MUST NOTIFY THE QUALITY ASSESSMENT AND REGULATION DIVISION IF THERE IS AN INCIDENT AT THE SERVICE AND/OR THE HEALTH, SAFETY OR WELLBEING OF A CHLD HAS BEEN COMPROMISED WHILST ATTENDING THE SERVICE.

TIME:

DATE:

NAMES:

DISCUSSION OUTCOMES:

Critical Action 3: Contacting Parents/Carers

See Action 3 of Four Critical Actions For Early Childhood Services: Responding to Incidents, Disclosures and Suspicions of Child Abuse

ACTIONS TAKEN (ALLEGED VICTIM)

PROVIDE DETAILS OF YOUR DISCUSSION WITH PARENTS/CARERS (IF APPROPRIATE):

YOU MUST CONSULT WITH VICTORIA POLICE AND/OR DHHS CHILD PROTECTION TO DETERMINE IF IT IS APPROPRIATE TO CONTACT PARENTS, IF IT IS DEEMED APPROPRIATE, PARENTS MUST BE CONTACTED AS SOON AS POSSIBLE (WITHIN 24 HOURS OF THE INCIDENT, DISCLOSURE OR SUSPICION).

HAVE YOU SOUGHT ADVICE FROM CHILD PROTECTION DEPT OR VICTORIA POLICE?

- ☐ NO
- ☐ YES

IS IT APPROPRIATE TO CONTACT PARENT/CARER

- ☐ NO
- ☐ YES

LIST REASONS IF IT IS NOT APPROPRIATE TO CONTACT PARENT/CARER:

IF CONTACTING PARENT/CARER, PROVIDE THE FOLLOWING DETAILS:

NAME OF STAFF MEMBER MAKING THE CALL:

NAME OF PARENT/CARER RECEIVING THE CALL:

DISCUSSION OUTCOMES:

Critical Action 4: Providing Ongoing Support

See Action 4 of Four Critical Actions For Early Childhood Services: Responding to Incidents, Disclosures and Suspicions of Child Abuse

PLANNED ACTIONS

INCLUDE DETAIL ON WHAT FOLLOW-UP ACTIONS HAVE OCCURRED TO SUPPORT THE STUDENT (FOR EXAMPLE, REFERRAL TO SPECIALISED SERVICES):

FOLLOW UP ACTIONS

SUPPORT:

REFERRALS(S):

PROCESS OF REVIEW

COMPLETE THIS SECTION BETWEEN 4-6 WEEKS AFTER AN INCIDENT, SUSPICION OR DISCLOSURE OF ABUSE IN CONJUNCTION WITH YOUR MANAGEMENT/APPROVED PROVIDER. THIS WILL SUPPORT YOU AND YOUR SERVICES TO CONTINUE TO PROTECT CHILDREN IN YOUR CARE AND TO REFLECT ON YOUR PROCESSES AND THE NEED FOR ANY FOLLOW-UP ACTION.

SAFETY AND WELLBEING

CURRENT SAFETY AND WELLBEING OF THE CHILD

IS THE CHILD SAFE FROM ABUSE AND HARM?

- ☐ NO
- ☐ YES

IF NOT CONSIDER THE NEED TO MAKE A FURTHER REPORT

DOES THE CHILD HAVE ANY WELLBEING ISSUES THAT ARE NOT CURRENTLY BEING ADDRESSED?

- ☐ NO
- ☐ YES

IF SO, CONSIDER HOW THESE CAN BE ADDRESSED AND CAPTURED WITHIN A STUDENT SUPPORT PLAN

CURRENT WELLBEING OF OTHER CHILDREN WHO MAY BE IMPACTED BY THE ABUSE

ARE THERE ANY OTHER CHILDREN WHO MAY BE IMPACTED BY THE ABUSE?

- ☐ NO
- ☐ YES

IF SO HAVE THEIR WELLBEING NEEDS BEEN MET

- ☐ NO
- ☐ YES

CURRENT WELLBEING OF IMPACTED STAFF MEMBERS

DOES THE STAFF MEMBER WHO MADE THE REPORT/ WITNESSED THE INCIDENT, FORMED A SUSPICION OR RECEIVED A DISCLOSURE REQUIRE ANY SUPPORT?

- ☐ NO
- ☐ YES

IF SO HAS THIS BEEN RECEIVED?

- ☐ NO
- ☐ YES

REVIEW OF ACTIONS TAKEN

HAVE SERVICE STAFF FOLLOWED THE FOUR CRITICAL ACTIONS FOR EARLY CHILDHOOD SERVICES: RESPONDING TO INCIDENTS, DISCLOSURES OR SUSPICIONS OF CHILD ABUSE?

WAS AN APPROPRIATE DECISION MADE IN RELATION TO WHEN TO ACT?

- ☐ NO
- ☐ YES

COULD THE SUSPECTED ABUSE HAVE BEEN DETECTED EARLIER?

- ☐ NO
- ☐ YES

ACTION 1

DID THE STAFF TAKE APPROPRIATE ACTION IN AN EMERGENCY?

- ☐ NO
- ☐ YES

ACTION 2

WAS A REPORT MADE TO THE APPROPRIATE AUTHORITIES AND INTERNALLY?

- ☐ NO
- ☐ YES

WERE SUBSEQUENT REPORTS MADE IF NECESSARY?

- ☐ NO
- ☐ YES

ACTION 3

DID THE SERVICE CONTACT THE PARENTS/CARERS ASAP?

- ☐ NO
- ☐ YES

HAVE THE PARENTS CONTINUED TO BE ENGAGED IF APPROPRIATE?

- ☐ NO
- ☐ YES

ACTION 4

HAS THE SERVICE PROVIDED ADEQUATE ON-GOING SUPPORT FOR THE CHILD?

- ☐ NO
- ☐ YES

HAVE ANY COMPLAINTS BEEN RECEIVED?

- ☐ NO
- ☐ YES

HAVE THE COMPLAINTS BEEN RESOLVED?

- ☐ NO
- ☐ YES

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 26/6/2026

Next Policy Review: 15/6/2027

References:

NQS

QA2	2.2.3	<i>Child safety and protection - Management, educators and staff are aware of their roles and responsibilities regarding child safety, including the need to identify and respond to every child at risk of abuse or neglect.</i>
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National Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	84	<i>Awareness of child protection law</i>
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Resources and legislation:

- *The United Nations Convention on the Rights of the Child. United Nations Convention on the Rights of the Child*
- *Relevant Commonwealth, state or territory legislation or regulations, for example legislative requirements for working with children checks and mandatory reporting.*
- *Victoria- Working with children check status checker*
- *Reporting requirements ACECQA- Reporting requirements about children | ACECQA*
- *National Principles for Child Safe Organisations, New South Wales Child Safe Standards.National Principles for Child Safe Organisations*
- *Victorian Child Safe Standards. CCYP | The 11 Child Safe Standards*
- *Victorian Reportable Conduct Scheme. CCYP | Reportable Conduct Scheme*
- *ACECQA National model code- National Model Code FAQs - Final.pdf*

13 - WATER SAFETY POLICY

Position Statement

Paddle pools, water troughs, buckets and other objects that can be filled with water can provide a drowning hazard for young children. Childcare team members need to be vigilant at all times that the environment remains safe and that every child is supervised in accordance with regulations. Paddle pools and large buckets used as wading pools are not to be used in the service or on excursion locations due to risk of children drowning.

Objective

To ensure that Educators are aware of their responsibilities to protect children from drowning and ensure that water in the childcare environment is used in a safe manner that promotes learning through play.

Implementation

- During water play events, pools, wading pools and water troughs and large buckets or tubs are NOT to be used or set-up in the service or on excursion locations due to risk of children drowning.
- Team members will ensure that parent permission has been provided for each child for water play. (Please note water play does not permit the use of pools, wading pools, tubs or large buckets on the ground (E.g. larger than 5 litres of water).
- Before a water play incursion takes place, the service must complete an internal risk assessment. Families must be informed of the upcoming water play experience through a post or email outlining the activities, purpose, date, time, and any items children need to bring (e.g., towel and bathers). Parent or guardian confirmation and consent must be obtained to ensure they are fully aware of, and agree to, their child's participation in the water play incursion.
- Please note sensory tub containers are permitted for use for sand and water sensory experiences. These containers are not to be used as pools or wading pools during water play. Team members will ensure that appropriate supervision ratios are met when preparing for water experiences.
- At all times children near water are closely supervised. A child will never be left unattended near any water, and two educators must supervise if children are participating in water play.
- Team members will ensure that a change of clothing is available for each child to wear should their clothing become wet after water play.
- Under no circumstances will a child be left unattended during water play.
- All water play equipment or containers are emptied immediately after use and will be stored to prevent the collection of water.
- All water containers that could constitute a drowning hazard will be safely covered and inaccessible for children except during appropriately supervised play.
- Team members will dry off each child after water play and ensure that they are appropriately dressed.
- Ensure water play area and equipment is hygienically cleaned, disinfected and chlorinated appropriately:
 - Before use of water play equipment remove leaves and debris, hose away surface dirt and scrub inside with disinfectant.
 - Wash away disinfectant before filling water tray/table stands.
 - Children with diarrhoea, upset stomach, open sores or nasal infections should not use (please see Illness Policy).
 - For water play all children should wear appropriate bathers, go to the toilet before entering water play area, and follow correct toileting hygiene practices while participating.
 - At end of water play experience water should be emptied onto grass or garden areas.

- If an accident occurs in the water play area, remove all children immediately, empty water and disinfect equipment and area.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/11/2025

Reference:

NQS

QA2	2.2.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>
	115	<i>Premises designed to facilitate supervision</i>
	73	<i>Educational programs</i>
	74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
	75	<i>Information about the educational program to be kept available</i>
	76	<i>Information about educational program to be given to parents</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 5.5, 6.2, 6.6

Standards Australia

Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)

Sources and Further Reading:

Kids Alive – Do the Five www.kidsalive.com.au

14 - SAFETY AND MAINTENANCE POLICY

Position Statement

An aspect of providing a safe environment for children relates to the ongoing vigilance and maintenance of equipment and the environment. Team members need to be aware of, and alert to, maintenance issues that could result in an unsafe environment for children.

Objective

To ensure that the building and all equipment in it are maintained in a safe and hygienic manner at all times.

Implementation

Team members are to check and maintain the safety of the centre and equipment at all times in accordance with Centre procedures, and to notify the Director of any issues immediately.

A maintenance form (*OPR003*) or OWNA Maintenance and Hazard Log is to be completed when reporting any breakages or maintenance required.

The service undertakes periodic risk assessments- see risk assessment and management register.

All playground areas and all equipment are to be checked by team members at the beginning of each day:

- Check for spiders, hairy caterpillars, bees, wasps, vandalism, glass, etc
- Rake sandpit and check for any dangerous items including syringes and cat excreta
- Disinfect sandpits on a weekly basis. It is recommended to complete this task each Friday (after final use or part of closing procedures tasks). Apply a weak solution of child-safe household disinfectant using watering can.
- Remove sand contaminated by food or other materials, human or animal faeces, blood or other bodily fluids. When this occurs wash remainder of sand using a neutral detergent and water distributed with a watering can.
- In accordance references, aeration from raking and sunshine is the most effective way of disinfecting sand.
- Check fences for breakages or potential escape routes
- Check grounds for dangerous items such as glass, razor blades, syringes
- Check yard for animal excreta
- Make sure all access gates are securely locked and child proof
- Ensure all plants and other vegetation are non-poisonous
- Wipe all spillages on floors or verandas immediately
- Store medication in locked container in the fridge
- Completion of quarterly OHS&W audits
- Review of maintenance requirements and completed of maintenance tasks on a periodic basis

In an Emergency:

1. Turn off circuit breakers to faulty electrical appliances
2. Remove damaged equipment if possible
3. Remove children from dangerous area and keep supervised
4. Report immediately to Director, Representative, Approved Provider, Police, SES (whichever applies)

Safety Guidelines in the Centre:

- Poisons, disinfectants, corrosive substances and other dangerous items will be marked and stored as per the Toxic and Potentially Dangerous Products Policy.
- Bulk cleaning supplies will be kept in a room/cupboard that is locked and not accessible by children.
- Electrical appliances, urns and hot water pipes will be inaccessible to children.
- All electrical lights and power fittings are at a minimum height of 1.5m from the floor or fitted with safety plugs when not in use.
- Hot water taps accessible to children are thermostatically controlled to temperature no more than 38 degrees.
- Children are not permitted in the kitchen without adult supervision.
- Plastic bags, cling wrap and polystyrene are not accessible to children.

The playground equipment will:

- ✓ Have a soft fall underneath and around it that meets the Australian standards.
 - ✓ Not be high enough to cause injuries from falls
 - ✓ Be free of sharp or rough edges or projections
 - ✓ Not cause a likelihood of pinching, trapping, crushing or striking a child.
- The playground and sandpit will be checked daily for the removal of foreign objects.
 - Sandpits not in use are covered to prevent access by vermin and animals.
 - Sand used in sandpits will be changed as required by regulations.
 - Paddling pools will be emptied after use and stored in such a way as to prevent the collection of water.
 - Animals will not be kept at the centre other than for educational purposes and then only under conditions approved by an authorised person.
 - Smoking is prohibited on premises.
 - Fences and gates around the centre will provide an effective barrier to children and meet regulatory requirements in each state of operation.

Appendix 1: Syringes and Needles Disposal Procedure

Needles and syringes in the workplace may be contaminated with human blood and body fluid or other infectious material. Exposure to these contaminants through the piercing of skin or contact with already broken skin may pose a health risk for transmission of certain infections.

Managers and Supervisory Staff need to ensure that all persons who use or may come in contact with needles and syringes are aware of the specific hazards associated with these and how to minimise their risk of exposure. This involves communicating requirements to parents, staff and students under their management and ensuring that appropriate training is provided.

The purpose of this information sheet is to provide guidance on the correct use and disposal of needles and syringes and action to be taken following a needlestick injury. This should be used as a starting point to assist with the implementation of appropriate risk control measures for the respective task(s).

Risk Management

Before you commence a task requiring the use or removal of a syringe, ensure that: A risk assessment has been completed for the task/activity and the required control measures have been implemented. Consideration of the following points may assist in this process:

- appropriate training in correct handling and disposal of needles and syringes must be provided and records kept

Activity	Recommended training
Staff member teaching involving the use of sterile needles	Supervised teaching of procedure Safety information to be included in handbooks
Collecting syringes/needles found in the workplace i.e Cleaning & Staff	Sharps information session (provided by OHS Nurse consultants)

- appropriate Personal Protective Equipment (PPE) for the designated task is readily available (Syringe Collection Kit)
- a sharps container is in close proximity enabling the immediate disposal of the connected syringe and needle
- the sharps container is not full and there is sufficient space to accommodate the additional needles.

What if someone finds a “sharp”?

- Students or children should never handle needles/syringes
- Supervisor on Duty needs to be advised immediately. The removal and disposal should be managed/co-ordinate by the supervisor on duty.
- Before staff handle sharps - move away any people (especially children) who are nearby
- Ensure there is space to move and to clearly observe the sharps and your hands
- Do not handle more than one item at a time. If there are multiple sharps, carefully separate them using a stick or implement – do not try to flick them or pick them up with a grabber/implement

How do you dispose of sharps appropriately?

See guidelines appendix 1 – “Guidelines for the disposal of needle/syringe into a sharps container”. The aim is to transfer the needle/syringe into an appropriate container safely to minimise the risk of needle-stick injury.

Finding syringes and needles while you are working

If, as part of your work, you find a needle or syringe in your work area, then:

- **do not** pick it up and **do not** take any action until you have informed your supervisor.
- **secure** the area
- **place bucket over the syringe or needle**

- **do not** try to recap the needle
- **do not** carry it from the area
- **Follow** Guidelines for the disposal of needle/syringe into a sharps container (Appendix 1)

If you find a needle or syringe in a public area, then:

- **do not** pick it up and **do not** pick it up and **do not** take any action until you have informed your supervisor.
- **secure** the area and **place bucket over the syringe or needle**
- **contact Local Police or Local Council** to arrange for collection and disposal.
- **Ensure person removing needle or syringe is following** Guidelines for the disposal of needle/syringe into a sharps container (Appendix 1).

If needle or syringe imposes a risk to families, staff or children due its location and the Local Police or Local Council are not available for immediate removal – assessment of risk needs to be undertaken. Procedures may be needed to be followed for needle/syringes safe removal.

Prohibited Activities when Disposing of Sharps

Services should dispose of containers that contain needles/syringes services recommended facility or a facility recommended by your local council.

- Do not dispose of sharps containers in the general waste.
- Do not throw needles/syringes down drains because they may then be washed out to other areas
- Do not throw needles/syringes down toilets

Disposing of sharps in these ways is unlawful.

Recognising a Sharps Container

A sharps container is a receptacle intended for the collection and disposal of sharps. It is:

- rigid-walled,
- puncture-proof and
- sealable.

Do not use glass jars or bottles, plastic drink containers or aluminium drink cans. These can break or may be recycled, potentially leading to injuries to other people such as waste collectors. To minimise the risk of the sharp puncturing the container it is best to use a sharps container that complies with AS/NZS 4261:1994 'Reusable containers for the collection of sharp items used in human and animal medical application'. Look for the following features:

- yellow in colour
- labelled as “sharps” or “infectious waste”
- carries the biohazard and AS/NZS symbols



(Photo: Example of sharps containers)

How do we prevent needle stick injuries?

- Services need to adopt practices that minimise the risk of students, staff or others coming into contact with sharps;
- Conduct regular inspection of the school grounds to ensure the early detection and disposal of discarded sharps

- Do not place your hands into areas or objects where you cannot see as sharps may be concealed there e.g. overgrown garden beds, rubbish bins. Use tongs or rubbish grabbers to pick up or move rubbish in these areas.
- Do not manually compress rubbish bags in case they contain needles/syringes.

Needlestick injury management

- Immediately wash the exposure area and follow first aid procedures.
- Report the incident to your supervisor and OHS&W officer as soon as possible.
- See a doctor as soon as possible.
- If the needle and syringe contained human or animal blood or bodily fluids, discuss with the doctor the arrangements for blood tests, immunisation where appropriate and a follow-up plan.
- If there is blood still in the syringe, and/or if the owner of the blood is known, advise your supervisor, OHS&W officer and your doctor.
- Complete an Incident Report and file with Centre Director and forward copy to management as soon as you are able.

APPENDIX 1: Guidelines for the disposal of needle/syringe into a sharps container
(STOP: IF YOU HAVE NOT BEEN TRAINED ON THIS PROCEDURE DO NOT PERFORM PROCEDURE.)

Equipment : thin, disposable latex, vinyl or nitrile gloves sharps container	
Procedure	Precautions
Step 1 Put on disposable latex or vinyl gloves (if available). Gloves will not prevent the wearer from being injured but will form a clean barrier between the hands and the syringe.	Do not attempt to recap the needle – this is how most accidental needle-stick injuries happen. The cap is usually bright orange and can be disposed of separately. Do not break, bend or otherwise try to render the syringe useless.
Step 2 Bring your rigid-walled, puncture-resistant, sealable, sharps container to the syringe.	Take the sharps container to the syringe, do not walk with the needle/syringe.
Step 3 Place the container on the ground or flat surface beside the syringe.	Do not hold the sharps container or ask another person to hold it as you are disposing of the syringe.
Step 4 Pick up the syringe by the middle of the barrel	The safest method of picking up a syringe is by hand. Staff can also chose to wear thin, disposable gloves that do not hinder dexterity. Do not crack the plastic barrel of the syringe or flick the syringe.
Note Do not use a dustpan & brush to “sweep up” the syringe as the sweeping movement can cause the syringe to flick into the air and cause further risk.	Plastic tweezers are not recommended as they may also cause the needle/syringe to flick (commonly bright coloured and found in many ‘sharps disposal kits’ sold at pharmacies etc).
Step 5 Place the syringe in the container sharp end first.	Keep the sharp end of the needle facing away from you at all times.
Step 6 Securely place the lid on the container and ensure it is sealed. Hold the container by the top when carrying.	Place the sealed container into your sharps disposal bin or contact your local council or health department regarding safe ways to dispose of your sharps container.
Step 7 Remove gloves carefully so any contaminated fluid on the glove does not come into contact with your hand. Wash your hands with running water and soap.	Other items that have come into contact with blood (i.e. gloves) should be disposed of in the same container as the used syringe or placed into double plastic bags and then into the rubbish.

What to do if a needle stick injury occurs

- Stay calm.
- Encourage the wound to bleed (gently squeeze).
- As soon as possible wash the area with running water and soap (if available).
- Apply an antiseptic and band-aid.
- As soon as possible contact your supervisor.
- It is important to be medically assessed as soon as possible. Visit your local doctor or hospital emergency department promptly; they will manage blood testing, counselling and possible hepatitis B and tetanus vaccination and/or medication.
- Staff can access the Employee Assistance Service (EAS) for free confidential counselling or seek the assistance of another counselling service.
- Dispose of the needle/syringe safely. Testing of syringes is usually not conducted so there is no need to keep the syringe.

Keep a copy of this page with each sharps kit in your service.
This guideline should also be displayed in relevant areas within your service e.g. Cleaner’s storeroom etc.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/3/2026

Policy Reviewed Next: 15/3/2027

Reference:

NQS

QA2	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>
QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing,</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
	6.2.2	<i>Effective partnerships support children's access, inclusion and participation in the program.</i>
	6.2.3	<i>The service builds relationships and engages with their local community.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>
	107	<i>Space requirements—indoor</i>
	108	<i>Space requirements—outdoor space</i>
	109	<i>Toilet and hygiene facilities</i>
	110	<i>Ventilation and natural light</i>
	111	<i>Administrative space</i>
	112	<i>Nappy change facilities</i>

113	<i>Outdoor space—natural environment</i>
114	<i>Outdoor space—shade</i>
115	<i>Premises designed to facilitate supervision</i>
116	<i>Assessments of family day care residences and approved family day care venues</i>
117	<i>Glass (additional requirement for family day care)</i>
73	<i>Educational programs</i>
74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
75	<i>Information about the educational program to be kept available</i>
76	<i>Information about educational program to be given to parents</i>
80	<i>Weekly menu</i>
86	<i>Notification to parents of incident, injury, trauma and illness</i>
99	<i>Children leaving the education and care service premises</i>
102	<i>Authorisation for excursions</i>
111	<i>Administrative space (centre-based services)</i>
168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i> <i>Children respond to diversity with respect</i> <i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i> <i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i> <i>Children transfer and adapt what they have learned from one context to another</i> <i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 5.1, 5.2, 5.3, 5.4, 5.5

Standards Australia

Health & Safety Fact Sheet – Safe Handling & Disposal of Needles and Syringes. Version: January 2012

Sources and Further Reading:

Kidsafe Qld (2006) Backyard Safety <http://www.kidsafeqld.com.au/>

Kidsafe NSW

Rsposa; Royal Society for the Prevention of Accidents

15 - SECURITY POLICY

Position Statement

Team members and children need to have a safe and secure environment in which to conduct their daily activities.

Objective

To ensure the security and safety of the building, grounds, and occupants at all times.

Procedure

- All team members/students/volunteers/visitors need to record their arrival, departure and breaks daily as required by their centre (time sheets or electronic pin).
- Team member records will verify current police check / blue card / Working With Children Check(WWCC) and VIT information.
- All ancillary providers where required, will produce a current Working With Children Check (Victorian Centres), police checks or blue cards for verification to the Centre Director.
- Any unauthorised person will be requested to leave the centre.
- The Director will ensure that banking is counted out of view of the foyer where possible, and is taken to the bank in an unmarked bag.
- The building will be securely locked at the end of each day. Please refer to the centre lock up procedure.
- **Team members are to check that all children have been collected before leaving the Centre.** For further information please refer to the Opening and Closing Policy.
- Team members will be responsible for externally securing their own room, before departure.
- The senior team member on duty will check the whole building before departure as per the closing procedure.
- The building will be locked and secured in the evenings.
- Please see **Appendix 1: Guidelines and Compliance with Victorian Protective Data Security Standards**. See our organisations policies and procedures for managing and protecting public sector information across all security areas including governance, information, personnel, Information Communications Technology (ICT) and physical security.

Appendix 1: Guidelines and Compliance with Victorian Protective Data Security Standards

The organisation's policies and procedures to cover the Victorian Protective Data Security Standards.

Governance: Information Security Management Framework (Standard 1)

- the organisation invests in security management based on a risk based approach. We ensure we have security policies and procedures, training, business continuity and disaster recovery, security incident management, external party engagement.

Security Governance measures:

- The organisation engages external Information Technology businesses to assist in its IT needs: Qikkids, OWNA, Wageloch, MYOB, Docusign, Microsoft, Dropbox, Etc. We use Cybernet Evolution to support and guide our IT requirements.
- organisation's information security management framework contains and references all legislative and regulatory drivers
- management commits to providing sufficient resources to support the organisation's ongoing information security program(s)
- The organisation monitors, reviews, validates and updates the information security. We use Avast IT Security Monitoring.

Information security:

- protection of information across the information life cycle from when it is created to when it is disposed or destroyed. Please see the Record Management Policy.

Information security measures include:

Information Security Value (Standard 2)

- The organisation's Information Management Framework incorporates all security areas.
- The organisation continually reviews the security
- The organisation manages the secure disposal (archiving/destruction)

Information Security Risk Management (Standard 3)

- The organisation considers information security risks in organisational planning
- The organisation records the results of information security risk assessments and treatment plans in its risk register
- The organisation communicates and consults with internal and external stakeholders during the information security risk management process.
- The organisation governs, monitors, reviews and reports on information security risk (e.g., operational, tactical and strategic).

Information Access (Standard 4)

- The organisation identifies and has an access management policy based on the principles of least-privilege and need-to-know.
- The organisation manages identities and issuing secure credentials (registration and de-registration) for physical and logical access.
- The organisation implements physical access controls (e.g., key management, key pad access) based on the principles of least-privilege and need-to-know
- The organisation implements logical access controls (e.g., network account, password, two-factor authentication) based on the principles of least-privilege and need-to-know
- The organisation manages the end-to-end lifecycle of access by following provisioning and de-provisioning processes
- The organisation limits the use of, and actively manages, privileged physical and logical access and separates these from normal access (e.g., office access, server room access, administrator access)
- The organisation regularly reviews and adjusts physical and logical access rights taking into account operational changes.

Information Security Obligations (Standard 5)

- The organisation documents its information security obligations and communicates these to all persons with access to public sector information (e.g., policies, position descriptions)
- The organisation's information security training and awareness content covers all security areas
- The organisation delivers information security training and awareness to all persons with access to public sector information, upon engagement and at regular intervals thereafter in accordance with its training
- All persons with access to public sector information acknowledge their information security obligations at least annually (e.g., during performance development discussions, attending security briefings, completing security training).

Information Security Incident Management (Standard 6)

- The organisation documents and communicates processes and plan(s) for information security incident management covering all security areas.
- The organisation articulates roles and responsibilities for information security incident management.
- The organisation's information security incident management processes and plan(s) contain the five phases of: Plan and prepare; Detect and report; Assess and decide; Respond (contain, eradicate, recover, notify); and Lessons learnt.

Information Security Aspects of Business Continuity and Disaster Recovery (Standard 7)

- The organisation identifies and assigns roles and responsibilities for information security in business continuity and disaster recovery processes and plans
- The organisation regularly tests (at least annually) its business continuity and disaster recovery plan(s).

Third Party Arrangements (Standard 8)

- The organisation's information security policies, procedures and controls cover the entire lifecycle of third party arrangements (e.g., contracts, and information sharing agreements).
- The organisation undertakes an information security risk assessment of the third party's service offering and addresses any residual risks prior to finalising the arrangement.
- The organisation identifies and assigns information security roles and responsibilities in third party arrangements.
- The organisation monitors, reviews, validates and updates the information security requirements of third party arrangements and activities
- The organisation manages the delivery of maintenance activities and repairs (on-site and off-site).

Information Security Reporting to Office of the Victorian Information Commissioner (OVIC) (Standard 9)

- The organisation notifies OVIC of incidents that have an adverse impact on the confidentiality, integrity or availability of public sector information

Personnel security (Standard 10)

- ongoing management and monitoring to ensure the continued eligibility and suitability of people accessing information. The organisation has robust employment procedures (including completing reference checks) and verification of working with children checks to ensure staff employed have adequate clearance.

Measures include:

- The organisation's personnel security policies and procedures address the personnel

lifecycle phases of: Pre-engagement (eligibility and suitability), Engagement (ongoing and re-engagement); an Separating (permanently or temporarily).

- The organisation verifies the identity of personnel, re-validates and manages any changes as required.
- The organisation manages personnel separating from the organisation commensurate with its security and probity obligations and risk profile.
- The organisation develops security clearance policies and procedures to support roles requiring high assurance and/or handling security classified information

Information Communications Technology (ICT) security (Standard 11)

-we have secure communications and technology systems processing or storing information. The organisation uses Microsoft Office and Sharepoint. We use Dropbox as our storage provider. The Dropbox storage is backup to an external hard drive off-site.

Information Access measures:

- The organisation manages security documentation for its ICT systems
- The organisation manages all ICT assets (e.g., on-site and off-site) throughout their lifecycle
- The organisation manages communications security controls (e.g., cabling, telephony, radio, wireless networks).
- The organisation verifies the vendors security claims before implementing security technologies
- The organisation manages security measures for email systems
- The organisation uses secure system administration practices
- The organisation designs and configures the ICT network in a secure manner (e.g., segmentation, segregation, traffic management, default accounts)
- The organisation manages malware prevention and detection software for ICT systems
- The organisation manages backup processes and procedures (e.g., schedule, isolation, storage, testing, retention).
- The organisation manages security measures for enterprise mobility (e.g., mobile device management, working from home).

Physical security (Standard 12)

secure physical environment including facilities, equipment and services and the application of physical security measures to protect information

Measures include:

- The organisation plans and documents physical security measures
- The organisation has scalable physical security measures ready for activation during increased threat situations.
- The organisation implements physical security measures when handling information out of the office.
- The organisation manages physical security measures throughout their lifecycle.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/8/2024

Reference:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	168(2)(e)	<i>Policies and procedures in relation to emergency and evacuation</i>
	97	<i>Emergency and evacuation procedures</i>
	98	<i>Telephone or other communication equipment</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 5.1, 5.2, 5.4

Standards Australia

Public Health and Wellbeing Act 2008

<https://ovic.vic.gov.au/data-protection/standards/>

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Privacy and Data Protection Act 2014 (Vic).

16 – PRIVACY & CONFIDENTIALITY POLICY

Position Statement

Management is committed to the responsible use of information and protects the privacy of children in our care, their parents and guardians. All matters pertaining to our Parent Handbook are governed and interpreted in accordance with relevant Privacy Legislation.

Objective

To assure all stakeholders that this information, which can be of a very delicate nature will be managed in accordance with the relevant legislation. Please see appendix 1: Australian Privacy Principles Compliance.

Implementation

- All enrolment info and personal family records will be stored in our online software system – OWNA, Office 365 and any additional child and family information stored in a lockable filing system.
- All child progress records are to be held within the centre and can only be accessed by team members in order to maintain confidentiality.
- No information regarding a parent or child will be given out without the parent's consent. With regard to seeking emergency medical care, or use of debt collectors for unpaid accounts, this permission is given on the enrolment form.
- Information given to team members in confidence will be kept confidential.
- Enquiries by unauthorised persons regarding the attendance or non-attendance of a child at this Centre will be neither confirmed nor denied.
- Team members can provide information such as developmental reports, newsletters, and attendance patterns to estranged parents ONLY where custody arrangements specifically detail that this information is to be shared.
- Where requests are made for information regarding a child's development, behaviour or general wellbeing from estranged parents where **no custody arrangement specifically details dissemination of information about the child**, team members will not hand over any Centre information. The Centre Director will inform the Support Manager/Approved Provider should such a request be made. The parent will be notified that any such request must be made via a solicitor before confidential information can be given out.
- Team members personal details will be kept confidential in accordance with relevant legislation
- Team member's personal details will be kept in a lockable filing cabinet and shall not be released without the team member's permission.
- Team members personal details will NOT be under any circumstances be given out to parents or over the telephone without the team member's written consent.
- Information given to team members in confidence will be kept confidential.
- If a student has a valid training requirement that involves the gathering of certain information pertaining to you or your family, the student MUST have written consent from you and the Centre Director.
- We will take reasonable steps to protect your personal information (and that of your family) from misuse, loss and from unauthorised access or disclosure. Under the Child Care Regulations (specific legislation is listed below) the centre is required to ensure that records relating to a child are made available to; the Approved Provider; the Nominee; Nominated Supervisor; team member authorised to access records by Approved Provider; the Director-General; a person otherwise authorised by law to inspect the records; a parent/guardian of the child (as per custody arrangements); any person authorised (in writing) by the parent of the child to inspect the records.
- The organisation complies with the Australian Privacy Principles (APPs) that form part of the privacy framework in the Privacy Act 1988. The policy covers how the organisation covers the Australian Privacy Principles.

Collection of Personal Information

There are occasions when we need to obtain information from centre stakeholders (including; children, families and team members, etc). When we collect information about you and/or your family this may be required to satisfy our legal obligations under child care legislation, and we may use it for research or to improve the services we offer. We may also use it to continue contact with you via email or other means, eg. to send you new information or special offers. If you do not wish to receive this information via a particular means, please let us know.

Where it is possible to do so, we collect information direct from parents and guardians of the children in our care.

Sometimes we may need to collect information from others such as medical professionals, who can assist us in providing the best care for your child. Except to the extent that we are required to collect information and maintain records by law, if you do not wish us to collect particular information about your child, please discuss this with our Centre Director.

Please note that our Centre retains paper based records that are relevant to the day to day care of the children enrolled as required by law.

What information do we collect?

We collect personal information directly from you through our enrolment and application processes and sometimes we collect or confirm this information from a third party such as funding agencies of health practitioners.

The private information we are required to collect includes but is not limited to:

- your name, address, date of birth and full name, date of birth and address of the child
- name, address and contact details for:
 - each known parent
 - any emergency contacts
 - any authorised nominee
 - any person who is authorised to consent to medical treatment or administration of medication
 - any person who is authorised to give permission to an educator to remove the child from the education and care service premises
 - details of any court orders, parenting orders or parenting plans
 - gender of the child
 - language used in the child's home
 - cultural background of the child and parents
 - any special considerations for the child, such as any cultural, religious or dietary requirements or additional needs
 - authorisations for:
 - the approved provider, nominated supervisor or an educator (including family day care educator) to seek medical treatment for the child and/or ambulance transportation
 - the service to take the child on regular outings
 - name, address and telephone number of the child's registered medical practitioner or medical service
 - child's Medicare number (if available)
 - details of any specific healthcare needs of the child, including any medical condition, allergies or a diagnosis that the child is at risk of anaphylaxis
 - any medical management plan, anaphylaxis management plan or risk minimisation plan for the child
 - any dietary requirements of the child
 - immunisation status of the child
 - if the approved provider or staff member has sighted a health record for that child, a notation of that fact
 - certificates of immunisation or exemption as required depending on the applicable state or territory jurisdiction.
 - Residential status and proof of identity

You need to also be aware that when you visit our website, apps or other web-based content and services ("Websites"), either we or our service provider may record information (such as your computer's IP address and top-level domain name, the type of browser you are using, the date, time and pages accessed) in relation to your visit.

Sharing Information

The personal information you provide to us will be retained only for as long as necessary to fulfil the purposes for which the information was collected or as required by law. We do not share personal information with third parties, except as necessary to carry out our business at your request, or as required by law, and we never sell your personal information.

Access to Personal Information

With reference to the protection of personal information guidelines, you may gain access to personal information that we hold about you or your child. You can request to correct any errors about you or your child in our records by contacting the Centre Director, Responsible Person in Charge of Service or access our online systems directly. To protect your privacy and the privacy of others, we may need to gain evidence of your identity and written consent before we can give you access to information or make changes.

Enrolment and other documents kept by the Service can be made available to a parent of the child on request (unless limited by a court order). These documents may include the personal information of a parent, including their address and contact number. There are cases where one parent may not want the other parent to access their personal information, including for security and safety reasons. Laws exist to protect personal information in these documents and privacy.

"Personal information" is defined in the Privacy Act 1988 (Cth) and includes any information about an identified individual such as their home address, email address, telephone number, date of birth, medical records, bank account details, and tax file number.

Protection of "personal information" contained in the following documents:

- The documentation of child assessments or evaluations for delivery of educational programs
- An incident, injury, trauma and illness record
- A medication record
- A children's attendance record, or
- A child's enrolment record.

Written consent must be first obtained before any "personal information" of one of the following people is disclosed or shared:

- a parent of a child enrolled at the service, other than the person requesting the documentation
- a person required to be notified of an emergency if a parent cannot be contacted
- an authorised nominee of a child
- a person authorised to consent to medical treatment or the administration of medication to a child
- a person authorised to authorise an educator to take a child outside the service premises
- a person authorised to authorise the service to transport a child or arrange transportation of a child.

Consent must be expressly given in writing. The service will check the consent before each disclosure is made. Consent may be withdrawn at any time after it has been given. Withdrawal of consent must be in writing.

With reference to the protection of personal information guidelines, you may ask to seek access to information held about you or your family, there may be a fee levied to cover the costs associated with processing this request.

Please contact the Centre Director if you have questions or comments about our Privacy and Confidentiality Policy or would like to correct or update information you have provided us.

Use and disclosure

We only collect personal information where it is reasonably necessary for one or more of our functions or activities, such as:

- the administering and management of early childhood education and care
- assessing your eligibility for funding support or other benefits.
- complying with any legal or regulatory obligations imposed on us
- performing our necessary business functions.

To do this, some of your private information will be shared with government agencies or funding organisations as required in order to entitle you to access various support if any.

We may also disclose your personal information to organisations that carry out functions on our behalf. This may include for example education software or information technology service providers, professional advisers, regulators and government authorities. Our agreements with these entities ensure this information is only used to carry out functions on our behalf and use your private information for the purpose it was disclosed.

We may also disclose your personal information to an individual or an organisation (a 'third party') if:

- You direct us to do so;
- You consent to the third party obtaining the information from us; or
- You consent to the third party accessing the information on our systems, and/or do anything which enables the third party to obtain access.

Your consent to a third party obtaining or accessing information may be implied from:

- Your use of any service or application which a third party provides to you, or makes available to you, which involves the third party obtaining or accessing personal information held by us or organisations like us; or
- You doing anything else which enables the third party to obtain access to the information.

Anonymity

There are limited circumstances where you may be able to deal with us anonymously or by using a pseudonym if you request to do so. Examples include where you are seeking only general information about our services or indicative pricing. In most cases it will not be possible for you to deal with us in this way, or to commence or complete an enrolment application due to the information and identifiers required by regulators and government agencies.

Access and correction

You may request access to your personal information that we hold at any time and request a correction of any errors in that information.

We will also take reasonable steps to amend or correct your personal information to keep it accurate and up-to-date.

Please contact us if you would like to access or request a correction of your personal information

Storage and security of your personal information

We will take reasonable steps to keep the personal information that we hold about you secure to ensure that it is protected from loss, unauthorised access, use, modification or disclosure.

Our employees and authorised agents are obliged to respect the confidentiality of any personal information held by us.

You can also help to keep the personal information that we hold about you secure by taking care before you authorise or otherwise assist any third party to obtain or gain access to that information.

Our websites and the use of cookies

We use our best efforts to ensure that information received via our Websites remains secured within our systems. We are regularly reviewing developments in online security; however, users should be aware that there are inherent risks in transmitting information across the internet.

We use cookies on our Websites. Cookies can make using our Websites easier by storing information about your preferences and enabling you to take full advantage of our services. Cookies are very small text files that a Website can transfer to your computer's hard drive or portable electronic device's memory for record keeping.

We may also use Cookies so that we can determine which parts of our Websites are visited most often, or whether you visited our site from a banner advertisement for one of our products or services on another party's website, and other sites you may visit from our Websites.

Sometimes Cookies are used by a third-party service provider with whom we have an agreement to monitor the success of our marketing campaigns. The third-party service provider uses the Cookies to collect information such as when you visited our site, your browser type and the server that your computer is logged in to.

The information is used in an aggregate form and generally no personal information is collected by the third-party service provider. Our agreements with these third parties ensure this information is only used to carry out functions on our behalf, and if any personal information is collected the confidentiality of that information is maintained.

We may also use Cookies so that we can see which parts of our Websites you visit when you access those Websites. We may use this information for marketing products and services to you. We keep this information confidential and we do not disclose it to third parties.

Most internet web browsers are pre-set to accept Cookies to enable full use of websites that employ them. However, if you do not wish to receive any Cookies on an internet web browser you may configure your browser to reject them or receive a warning when Cookies are being used. In some instances, this may mean that you will not be able to use some or all of the services provided on our websites. However, you may still be able to access information-only pages.

How we manage a data breach

A data breach occurs when personal information is lost or subjected to unauthorised access, modification, use or disclosure or other misuse.

Data breaches can be caused or exacerbated by a variety of factors and give rise to a range of actual or potential harms to individuals, agencies and organisations.

In the event of any suspected data breach, the matter will be investigated to determine:

- The nature of the breach
- The number of people impacted
- The nature of the breach and extent to which an individual or group may be harmed by the breach
- Remedial action to minimise or prevent impact
- Review of systems to minimise the possibility of future similar breach

Identification and Destruction of Records

When your child/children leave our service, the following documents and records must be retained by law for the periods of time listed. At the conclusion of that period, the documents and record will be de identified and destroyed. Please see our Record Keeping Policy for more information.

Type of record	Timeframe	Reference
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Child Assessments	Until 3 years after child's last attendance	Regulation 74, 183
Incident, injury, trauma and illness records	Until the child is 25 years old	Regulation 87, 183
Medication record	Until 3 years after child's last attendance	Regulation 92, 193
Child's Attendance	Until 3 years after child's last attendance	Regulation 158-159, 183
Child enrolment	Until 3 years after child's last attendance	Regulation 160, 183
Death of a child while being educated and cared for by the service	Until 7 years after child's last attendance	Regulation 12, 183
Staff record	Until 3 years after child's last attendance	Regulation 145
Record of access to early childhood teacher	Until 3 years after child's last attendance	Regulation 152
Record of educators working directly with children	Until 3 years after child's last attendance	Regulation 151
Record of volunteers and students	Until 3 years after child's last attendance	Regulation 149

All other documents and records that are not listed above will be DE identified and destroyed within 30 days of your departure from the service.

Complaints

If you believe we have breached Privacy Laws or our Privacy Policy may lodge a complaint with the Approved Provider by telephone, email or by mail. The Approved Provider or Nominated Supervisor will follow the Service's grievance procedure to investigate the complaint. Individuals who are unhappy with the outcome of the investigation may raise their complaint with the Office Australian Information Commissioner www.oaic.gov.au GPO Box 5218 Sydney NSW 2001 or GPO Box 2999 Canberra ACT 2601, phone 1300 363 992 or email enquiries@oaic.gov.au

Appendix 1: Australian Privacy Principles Compliance

The Australian Privacy Principles (or APPs) are the cornerstone of the privacy protection framework in the [Privacy Act 1988](#) (Privacy Act). They apply to any organisation or agency the Privacy Act [covers](#).

There are 13 [Australian Privacy Principles](#) and they govern standards, rights and obligations around:

- the [collection](#), [use and disclosure](#) of [personal information](#)
- an organisation or agency's governance and accountability
- integrity and [correction](#) of personal information
- the rights of individuals to [access](#) their personal information

The Australian Privacy Principles are principles-based law. This gives an organisation or agency flexibility to tailor their personal information handling practices to their business models and the diverse needs of individuals. They are also technology neutral, which allows them to adapt to changing technologies.

Who has rights under the Privacy Act?

The Privacy Act regulates the way individuals' [personal information](#) is handled.

As an individual, the Privacy Act gives you greater control over the way that your personal information is handled. The Privacy Act allows you to:

- know why your personal information is being [collected](#), [how it will be used and who it will be disclosed to](#)
- have the option of not identifying yourself, or of using a pseudonym in certain circumstances
- ask for [access](#) to your personal information (including your [health information](#))
- stop receiving unwanted [direct marketing](#)
- ask for your personal information that is [incorrect to be corrected](#)
- [make a complaint](#) about an organisation or agency the Privacy Act covers, if you think they've mishandled your personal information

Who has responsibilities under the Privacy Act?

Australian Government agencies (and the Norfolk Island administration) and organisations with an annual turnover more than \$3 million have responsibilities under the Privacy Act, subject to some exceptions.

What small businesses are covered?

The Privacy Act cover some small business operators (organisations with an annual turnover of \$3 million or less), including:

a private sector health service provider — an organisation that provides a [health service](#) includes:

- a child care centre, a private school and a private tertiary educational institution

Australia Privacy Principles – Quick Reference

Principle	Title	Purpose
APP 1	Open and transparent management of personal information	Ensures that APP entities manage personal information in an open and transparent way. This includes having a clearly expressed and up to date APP privacy policy .
APP 2	Anonymity and pseudonymity	Requires APP entities to give individuals the option of not identifying themselves, or of using a pseudonym. Limited exceptions apply.
APP 3	Collection of solicited personal information	Outlines when an APP entity can collect personal information that is solicited. It applies higher standards to the collection of sensitive information .
APP 4	Dealing with unsolicited personal information	Outlines how APP entities must deal with unsolicited personal information.
APP 5	Notification of the collection of personal information	Outlines when and in what circumstances an APP entity that collects personal information must tell an individual about certain matters.
APP 6	Use or disclosure of personal information	Outlines the circumstances in which an APP entity may use or disclose personal information that it holds.
APP 7	Direct marketing	An organisation may only use or disclose personal information for direct marketing purposes if certain conditions are met.
APP 8	Cross-border disclosure of personal information	Outlines the steps an APP entity must take to protect personal information before it is disclosed overseas.
APP 9	Adoption, use or disclosure of government related identifiers	Outlines the limited circumstances when an organisation may adopt a government related identifier of an individual as its own identifier, or use or disclose a government related identifier of an individual.
APP 10	Quality of personal information	An APP entity must take reasonable steps to ensure the personal information it collects is accurate, up to date and complete. An entity must also take reasonable steps to ensure the personal information it uses or discloses is accurate, up to date, complete and relevant, having regard to the purpose of the use or disclosure.
APP 11	Security of personal information	An APP entity must take reasonable steps to protect personal information it holds from misuse, interference and loss, and from unauthorised access, modification or disclosure. An entity has obligations to destroy or de-identify personal information in certain circumstances.
APP 12	Access to personal information	Outlines an APP entity's obligations when an individual requests to be given access to personal information held about them by the entity. This includes a requirement to provide access unless a specific exception applies.
APP 13	Correction of personal information	Outlines an APP entity's obligations in relation to correcting the personal information it holds about individuals.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 16/1/2026

Reference:

NQS

QA4	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
QA5	5.1.2	The dignity and the rights of every child are maintained at all times
QA6	6.1	Respectful relationships with families are developed and maintained and families are supported in their parenting role.
QA7	7.1	Governance supports the operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	181	Confidentiality of records kept by approved provider
	181-184	Confidentiality and storage of records

Other References:

Privacy Act 1998

Australian Privacy Principles (APP)

Handbook on Child Care Licensing

Privacy Amendment (Private Sector) Act 2000

Data Protection – Victorian Protective Data Security Stands – March 2018

17 - OPENING AND CLOSING POLICY

Position Statement

It is imperative for the safety of children and team members that opening and closing procedures which ensure centre and personnel safety are outlined.

Objective

To ensure optimum conditions for children and team members at the child care centre by working diligently through the correct procedures for opening and closing.

Procedure

- Team members who are rostered on an early shift are required to open windows and unlock doors throughout the building, and set up an arrivals room with appropriate activities. The outdoor yards need to be set up according to plans from the individual classes. The sandpits **MUST** be raked, and the yard checked for foreign objects and safety.
- Completion of the Opening and Closing Compliance Daily Checklist (in OWNA). Including Food Safety Schedules such as fridge temperatures records.
- When the group moves outdoors, the indoor classroom must be tidied, and equipment put away. All children must have sunscreen applied and a hat in accordance with the Sun Smart Policy. Regulation staff – to – child ratios must be observed at **ALL** times.
- Each room is responsible for their own cleaning and washing and this should be concluded before the last team member for each room finishes their shift. Check for washing in other classes before putting on a small load.
- Before closing the centre the yards must be packed up, equipment stored, sandpits raked and covered, paths/paving to be swept and shed locked.
- The late shift team members must ensure the kitchen, the workrooms, bathrooms and classroom last used are tidy before they leave.
- Lock up should be carried out twice, once by the Responsible Person in Charge, then again by the Second Educator In charge when the last child has departed. This double check is essential for the security of the Centre. The Closing Checklist (in OWNA) needs to be checked off against and signed by the team members conducting the close procedure. Team members are prohibited from leaving the Centre whilst any child remains on premises. Team members are to check the entire building and sign in/out sheets to ensure that all children have been duly collected before locking the Centre and leaving for the evening.
- Closing shift educators need to review the OWNA rolls and the Child Daily Information Schedule (Child sign in and out columns). All children need to be accounted for and both the OWNA rolls and Children Information Schedules or Child Wellbeing Books completed.
- Any children collected after closing time must be recorded on the late sheet filed in the late folder at the reception desk. Any children still at the centre at closing time, without a telephone call from parents will be treated in accordance to our Late Children Policy. The Director must be contacted and informed before action is taken.
- Ensure all devices and computer equipment is stored securely (in locked cupboard, room or office.)

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

Reference:

NQF

QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.3	<i>The services builds relationships and engages with their local community</i>

QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	157	<i>Access For Parents</i>
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18 - LATE CHILDREN POLICY

Position Statement

In the event a parent or responsible adult has not collected a child by closing time, it is the Service's team member's responsibility to ensure they are safely cared for.

Objective

To ensure that parents and team members are aware of the actions to be taken when a child has not been collected.

Implementation

- It is recommended to review the estimated pickup time and complete courtesy call to parent to clarify pickup time. If parent has advised an earlier time and this passes it is recommended to call parent before closure time to clarify pick-up time. Eg. Noted pick-up time was 4.30pm, recommend educator complete courtesy call 30 minutes before closure.
- Team member(Nominated Supervisor or Responsible Person) will attempt to contact a parent or authorised person to collect the child.
- The Centre Director/Nominated Supervisor/Responsible Person will be contacted if a child remains at the centre after the scheduled closing time without contact from parents, and when the parents, authorised persons, or emergency contacts cannot be reached.
- If the Director or Service Provider/Centre Owner is unable to contact an authorised persons (wait 30 minutes), the Police and the Licensing Department Emergency After Hours Help Line will be called to collect the child.
- A message will be left on the front door of the centre stating the police station where the child can be found.
- The parent/s will be charged as per the Centre Fee Schedule for the time team members must remain at the centre to attain handover of the child to the appropriately authorised person. A receipt will be provided by educators or by Centre Director for late fee paid by parent at the time or next day. Please note the receipt of late fees in diary and Centre Director to record late fee in revenue system and bank money received. Educators will be paid for their time or provided time in lieu in accordance with Award.
- Late Collection Form (OPR024) is to be completed by team members and parents.
- If repeat occurrence by parent/s Centre Director to issue a formal letter to parent advising of enrolment terms and conditions in relation to collection of child (OPR037).

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 21/7/2026

Reference:

NQS

QA2	2.2	<i>Each child is protected.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	85	<i>Incident, injury, trauma and illness policies and procedures</i>
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Other References:

Handbook on Child Care Licensing

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

19 – INCIDENT, INJURY, TRAUMA & ILLNESS POLICY

Position Statement

This Service is designed to be a safe, secure environment to foster children's learning and development. When incidents, injuries and trauma's occur team members need to act decisively according to the situation at hand. Team members will follow through by contacting parents, speaking to health care professionals and knowing how to record the incident, injury and trauma as required.

Definitions

Incident: an incident is a situation, event or occurrence that impacts the child/ren's, educators or staff members health, safety or wellbeing.

Related Policies

32- Infectious Diseases and Immunisation Policy
31 - Illness Policy
33 - Medication Policy
34- First Aid Policy
35 - Anaphylaxis Policy
36 - Asthma Policy
85 - Diabetes Policy
87 - Medical Conditions Policy
91 - Epilepsy Policy
92 - Health and Wellbeing Policy
94 - Cystic Fibrosis Policy
98 - Coronavirus Policy
1- Fee Policy

Objective

To ensure team members are aware of safety measures to minimize the risk of incidents, injury and trauma. Also, that the service is able to act appropriately in the event of an incident, injury or trauma.

Implementation

Identifying Signs and Symptoms of Illness, Trauma and Injury

Please refer to the above name related policies for specific guidance. Please refer to the 31 - Illness Policy and Staying Healthy In Childcare for information on management of Common Illnesses.

Early childhood educators and management are not doctors and are unable to diagnose an illness or infectious disease, however, as our educators are familiar with the children in their care, they will watch for symptoms of illness, trauma and injury. If a child becomes ill whilst at the Service, educators will respond to their individual symptoms of illness and provide comfort and care.

Educators will closely monitor the child focusing on the symptoms displayed and how the child behaves and be alert to the possibility of symptoms that may suggest the child is very sick and needing urgent medical assistance.

Educators will:

- understand the differences between *concerning and serious symptoms*
- if any *serious symptoms* are observed (breathing difficulties, drowsiness or unresponsiveness, looking pale or blue or feeling cold)
 - an ambulance will be called immediately
- if any *concerning symptoms* are observed (lethargy, fever, poor feeding, new rash, poor urine output, irritation or pain or sensitivity to light) educators will:
 - monitor the child carefully
 - call parents/carers
 - discuss symptoms with parents/carers and help them decide whether the child needs to see a doctor
- educators will monitor the child and will consider calling an ambulance if:
 - any concerning symptoms become severe
 - the child gets worse very quickly
 - there are multiple concerning symptoms.

(Staying healthy in Childcare)

DRSABCD Action Plan

DRSABCD is an acronym that helps you remember the basic first-aid steps. It stands for 'Danger, Response, Send (for help), Airway, Breathing, CPR, Defibrillation,' – seven actions to take when looking after someone during a medical incident.

Key actions include ensuring safety, checking responsiveness, opening the airway, verifying normal breathing, and initiating CPR if needed, particularly focusing on gentle handling for infants.

If it is a minor incident, you may only need to follow a few steps.

The DRSABCD steps are designed to help everyone, from babies to older adults.

DRSABCD action plan should be implemented by educators in all incidents.

Please refer to 34 - First Aid Policy for more information on DRSABCD.

In the event of any child requiring ambulance transportation and medical intervention, a serious incident will be reported to the regulatory authority (Reg. 12) by the service within 24 hours.

If the child has symptoms that suggest they are sick and they are not well enough to enjoy activities, they should go home and parents/caregivers will be contacted. The child will be cared for in an area that is away from other children in the Service to await pick up from their parent/guardian or emergency contact person. A child who is displaying symptoms of a contagious illness or virus (vomiting, diarrhoea, fever) will be moved away immediately from the rest of the group and supervised until he/she is collected by a parent or emergency contact person.

Symptoms Indicating Illness may Include (Refer to Illness Policy for further information):

- lethargy and decreased activity
- difficulty breathing
- fever (temperature more than 38°C)
- headaches
- poor feeding
- poor urine output/ dark urine
- a stiff neck, irritability or sensitivity to light
- new red or purple rash
- pain
- diarrhoea
- vomiting
- discharge from the eye or ear

- skin that displays rashes, blisters, spots, crusty or weeping sores
- loss of appetite
- difficulty in swallowing or complaining of a sore throat
- persistent, prolonged or severe coughing

(This is not an exhaustive list of indicators of illness)

When a Child is Unwell, Develops a High Temperature or Fever at the Service

- For infants under 3 months old, parents/guardians will be notified immediately for any fever over 38°C for immediate medical assistance. If a parent is uncontactable, emergency contacts will be contacted. If family members are unable to be contacted and emergency medical assistance is required, the Service will follow its first aid procedures and contact emergency services
- Educators will check a child's temperature if they think the child has a fever. If it is between 37.5°C and 37.9°C educators will retest within 30 minutes (records will be kept of time, date and temperature)
- Educators will notify parents when a child registers a temperature of 38°C or higher
- Educators will follow the *Illness Policy* for methods to reduce a child's temperature or fever
- The child will need to be collected from the Service as soon as possible (within 30 minutes)
- Educators will monitor the child carefully to ensure their condition does not get worse and call an ambulance immediately if required
- If needed the Parents/carers will be provided with a factsheet for further information
- Educators will complete an *Incident, Injury, Trauma and Illness* record and note down any other symptoms that may have developed along with the temperature (for example, a rash, vomiting, etc.).
- If the child has gone home from the Service with a unwell or a fever please refer to Illness Policy for exclusion periods.

Notifying Families and Emergency Contact – Sickness or Infectious Illness

- It is a requirement of the Service that all emergency contacts are able to pick up an ill child within a 30-minute timeframe.
- In the event that the ill child is not collected in a timely manner, or should parents refuse to collect the child, a warning letter will be sent to the families outlining Service policies and requirements. The letter of warning will specify that if there is a future breach of this nature, the child's enrolment position may be terminated.
- Parents or guardians are notified and receive a OWNA Incident Report or Incident, Injury, Trauma and Illness Record Form as soon as practicable and no later than 24 hours of the illness, accident, or trauma occurring.
- Families will be notified of any outbreak of an infectious illness (e.g.: Gastroenteritis, whooping cough) within the Service via our notice board, online app or email to assist in reducing the spread of the illness.
- When a child has been diagnosed with an illness or infectious disease, the Service will refer to information about recommended exclusion periods from the [Public Health Unit](#) (PHU) and *Staying healthy: Preventing infectious diseases in early childhood education and care services*.
- Exclusion periods for illness and infectious diseases are provided to families and included in our Family Handbook and *Dealing with Infectious Disease Policy*
- Families are provided with clear information about any illness or disease via Factsheets from *Staying healthy in childcare* when there is an outbreak of infectious disease or illness at the service. .

Reporting outbreaks to the Local Public Health Unit

Outbreaks require immediate attention, investigation and coordinated efforts to control the spread of the disease. To break the chain of infection during a disease outbreak, the nominated supervisor should [contact the relevant Local Public Health Unit \(LPHU\) for advice](#). Public health staff can provide valuable advice, support and resources that help manage cases or outbreaks of infectious diseases.

Local Public Health Units	Contact Information
Council's Environmental Health Services	
LPHU: Loddon Mallee (led by Bendigo Health) LGA: Loddon Mallee	Phone: 1800 959 400 Email: LMPHU@bendigohealth.org.au
Mildura Rural City Council: Environmental Health Services	Phone: 03 5018 8216 Email: health@mildura.vic.gov.au (As service holds a Class 1 Food Premises)
LPHU: South East Public Health Unit (led by Monash Health) SEPHU LGA: Stonnington	Phone: (03) 9594 0570 Email: • SEPHU.trace@monashhealth.org (Communicable Diseases Prevention and Control) • SEPHU.Partnership@monashhealth.org (Health Promotion and Partnerships) • communityengagement@monashhealth.org (Community Engagement)
Stonnington City Council	

Note that during outbreaks, more rigorous environmental cleaning practices are required. See the recommendations in the [Staying healthy guidelines](#).

When to Notify LPHU and Council Environmental Health Service Departments of Infectious Disease Outbreaks.

Nominated supervisors or Responsible Persons are required to notify the LPHU within the following timelines:

Infectious Diseases Outbreaks	Local Public Health Unit Notification	Council Environmental Health Services
Gastrointestinal	Yes, if outbreak occurs. An outbreak may be defined as two or more cases of vomiting and/or diarrhoea among children and/or staff within 48 hours of each other. If this occurs and the symptoms cannot be explained by medication or other medical conditions, you may have an outbreak.	Yes, if outbreak occurs.
Respiratory Infectious Disease - example influenza, RSV)	Yes, if outbreak occurs. An outbreak is declared when 2 or more children or staff members test positive to RSV within a 72-hour period.	No, does not need to be reported to Council.
COVID	Generally, No. Yes, if assistance from LPHU is required.	No, does not need to be reported to Council. During Council's management of a Gastro outbreak in a childcare

	Healthcare, workplaces and educational settings can also report outbreaks, <u>if assistance is required</u>. Outbreak is defined as 2 or more people in organisation testing positive within a 72-hour period. (Source: https://www.health.vic.gov.au/infectious-diseases/covid-19-coronavirus-disease-2019#notification-requirement-for-covid-19)	service, we also require notification if COVID-19 is present in the centre at the same time as the gastro outbreak. This is necessary from an Occupational Health and Safety perspective so that Council can implement appropriate protective measures, such as the use of suitable PPE, to ensure the safety of staff attending the site.
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Nominated supervisors must monitor health alerts and guidelines from LPHU and the Victorian Department of Health for any advice, and emergency health management in the event of a contagious illness outbreak.

The service should subscribe to the [Chief Health Officer alerts](#) to stay updated with current health issues facing the Victorian public and health sector.

Nominated supervisors must notify the Regulatory Authority of any incidence of an infectious disease that poses a risk to the health, safety or wellbeing of children attending the service.

IN THE EVENT OF A MILD INJURY (bumps, bruises) OR ILLNESS:

- Educator to apply general first aid according to the time, place and injury. Follow **DRSABCD Protocols and Action Plan**. If it is a minor incident, you may only need to follow a few steps.
- Keep the child comfortable and as relaxed as possible.
- If child is unwell, take child temperature. Record information in incident form or child temperature record form.
- Make sure team members are aware of and are caring for other children in the group.
- When an incident, injury or trauma has occurred children must be intentionally educated regarding the topic to reduce the likelihood of this occurring again. It is recommended that this is taught in several different methods and days of the week to ensure all children are covered. Further, the intentional teaching is recommended to occur over a number of weeks. The duration will depend on the nature of the incident.
- Educators need to communicate with parents and provided resources on incident topic (i.e. Biting, Behaviour Guidance Strategies, etc). The method of communication and materials provided to parents need to be documented on the OWNA Incident Report or Incident, Injury, Trauma and Illness Record Form (*HEI007*). Please note on the form: date and time provided resources materials, date and time of further discussion or meetings and the title and source of the resource materials.
- Centre Director or Educational Leader to make a courtesy call to parents to let them know if the minor injury has involved any form of head injury, bite or injury resulting in bleeding (other than grazes). Please notify the parent that part of our procedure there will be an "Incident, Injury or Trauma form" to sign when they arrive to pick up their child and the nature of the minor injury. (Please ensure the form is completed before making call).

IN EVENT OF A SERIOUS AND MAJOR INJURY

- Follow **DRSABCD Protocols and Action Plan**.
- Remove any ongoing danger or hazard without endangering yourself
- Ensure the child/adult is comfortable and follow first aid protocols

- One team member must stay with the child while the other is with the group
- First aid is to be administered as soon as possible after the injury
- Call ambulance if urgent medical help is required (see table below for guidance)
- Parents/emergency contacts are to be notified by phone to collect child/adult
- Please refer to First Aid Policy, Summary of Symptoms, Injury & Trauma table below and Appendix 1 for “Head Injury – General Advice” The Royal Children’s Hospital Melbourne
- Child/adult may be taken to the nearest hospital/doctor with a member of team via ambulance, if parent/authorised person cannot be contacted
- All faulty equipment is to be removed immediately and the Director notified. Note what is required to be fixed on the maintenance sheet
- Approved Provider are to be notified immediately if a serious incident (within 2 hours)
- The OWNA Incident Report (or an Incident, Injury, Trauma and Illness Record Form (HEI007)) is to be properly completed at this time and then must state only what was seen to occur. The form must be filled out as soon as is practical following the team members first aid response to the situation. Normal protocols for filling out the reports are to be followed.
- In addition to completing the internal OWNA Incident Report (Incident, Injury, Trauma and Illness Record Form) the Centre Director/Nominated Supervisor, Responsible Person must complete the ACECQA form “Notification of Serious Incident Form -SI01” via the NQS ITS portal and forward to the Approved Provider for review. The reviewed and completed Notification of Serious Incident Form is forwarded to ACECQA within 24 hours of the incident.
- If a serious injury, determine if a call to Department of Education is made after the matter is discussed with the Approved Provider, completion of the Incident, Injury, Trauma and Illness Record Form and Notification of Serious Incident Form – SI01.
- Authorisation is held for the approved provider, nominated supervisor or educator to seek medical treatment from a medical practitioner, hospital or ambulance service and if required, transportation by ambulance service. It is advised the cost of an ambulance, medical treatment, hospital costs, etc is NOT covered or paid for by the Service.
- Parents will be invoiced for all transportation costs/ambulance and medical treatment expenses.

SUMMARY OF SYMPTOMS, INJURY & TRAUMA

The following table provides guidelines of actions:

- When to call an ambulance if as urgent medical care is needed.
- When to notify a parent of incident or injury or trauma.
- When to notify the Regulatory Authority via the NQS ITS.

Please refer to related policies (listed above) for specific policies and procedures. Eg 31 - Illness Policy, Asthma Policy, Anaphylaxis Policy, etc.

Incident, Injury, Trauma or Illness Type	Guidelines for Communication
Mild Injury: Graze (surface bleeding only). Please note this does not apply to grazes on the head; this is considered a head injury. Follow DRSABCD Protocols and Action Plan Examples of mild graze injuries:	Parent/Guardian Courtesy Call: Not required Incident form prepared and reviewed. Sent to parent using OWNA App. Message to be message in Child Routine Schedule or Room Diary to let collecting parent/guardian know there is an incident form to be signed. (This is also allowing all educators to know of incident form).

 	
Mild Injury: Bite	Parent/Guardian Courtesy Call: Yes (To both parents: child been bitten and biter) Notify Centre Director or Responsible Person on Duty: Yes Incident form prepared and reviewed. Sent to parent using OWNA App. Message to be message in Child Routine Schedule or Room Diary to let collecting parent/guardian know there is an incident form to be signed. (This is also allowing all educators to know of incident form).
Mild Injury: (bump, cut, graze). Follow DRSABCD Protocols and Action Plan	Parent/Guardian Courtesy Call: Yes (Please refer below and appendix 1 for further guidance on medical treatment for serious injuries and head injuries.) Notify Centre Director or Responsible Person on Duty: Yes Incident form prepared and reviewed. Sent to parent using OWNA App. Message to be message in Child Routine Schedule or Room Diary to let collecting parent/guardian know there is an incident form to be signed. (This is also allowing all educators to know of incident form). You need to notify the Regulatory Authority if a child experienced serious injury and/or trauma while being educated and cared for by your service and needed urgent medical attention, attended a hospital, or should have attended a hospital. If

	so, Serious Incident Form Lodged via the NQS ITS within 24 hours.
Serious Symptoms, Injury, Illness or Trauma <i>Call Triple Zero (000) Ambulance Immediately if someone need urgent medical help.</i> Call for urgent medical help (Ambulance) if a person has the following Serious Symptoms: • Breathing difficulty – breathing very quickly or noisily, or look pale or blue around the mouth. The child may be working hard at breathing, with the muscles between the ribs or at the base of the neck being drawn in with each breath. • Drowsiness or unresponsiveness – is less alert, sleeper than normal or difficult to wake from sleep, or they are not responding as they usually do (for example, making less eye contact than usual, or less interested in their surroundings than usual). • Poor circulation – the child looks very pale, and their hands and feet feel cold or look blue. • Chest pain or tightness • Signs of a stroke • Sudden numbness or paralysis of the face, arm or leg • Uncontrolled or severe bleeding • Physical assault causing service injury • Severe or large burns • Extreme pain <i>Other Related Serious Symptoms and Injuries:</i> <i>(Also Please refer to appendix 1 for further guidance on medical treatment for head injuries.)</i> • Neck pain or tenderness • Visible deformity of the skull. • Weakness or numbness/tingling in more than one arm or leg • Loses consciousness, even if only for a few seconds. <i>Loss of consciousness – time when a person is unable to open their eyes, speak or follow commands. They have no awareness of stimulation from outside their body.</i> • Increased confusion or deteriorating conscious state (becoming less responsive, drowsy) • Has a seizure, fit, or convulsion. <i>Convulsion, seizure or fit – this can occur when there is a momentary imbalance within the electrical and chemical circuits in the brain. The imbalance may create a temporary disturbance in the way the brain controls awareness and responsiveness, and may cause unusual sensations and/or abnormal movements and postures.</i> • Is difficult to wake up or is unusually drowsy/unresponsive. • Vomits more than once (repeated vomiting).	Call 000 (Ambulance) immediately for urgent medical help. Follow DRSABCD Protocols and Action Plan Notify Centre Director or Responsible Person on Duty: Yes Notify Parent / Guardian / Next to Kin of injury and medical action taken: Yes Incident form prepared and reviewed. Sent to parent using OWNA App. ***** You need to notify the Regulatory Authority if a child experienced serious injury and/or trauma while being educated and cared for by your service and needed urgent medical attention, attended a hospital, or should have attended a hospital. If emergency services attended because a child experienced a serious injury and/or trauma, you need to notify the Regulatory Authority of the serious injury/trauma, not the attendance of the emergency services. Serious Incident Form Lodged via the NQS ITS within 24 hours. ***** Establish return to care plan with parent/guardian and educators.

• Has a severe or worsening headache • Shows signs of confusion, memory loss, or behaves unusually (e.g., agitation, combativeness). • Experience sensory or motor changes, such as blurred/double vision, slurred speech, or weakness/numbness in arms or legs. • Has clear fluid or blood leaking from the ears or nose. (Not linked to a known medical condition (such as ear grommets or an ear infection)) • Was involved in a high-impact accident, such as a fall from more than one metre (or greater than the child's height) or a high-speed collision. If there are no red flags but signs and symptoms suggest potential concussion. Make contact with parents or carers for collection. Parent or carer to seek a medical assessment, even if the symptoms resolve.	
Concerning Symptoms (Source: <i>Staying Healthy in Childcare</i>) Other symptoms may be concerning but do not necessarily mean that a child is severely sick. These symptoms may also occur in combination with the serious symptoms. The more of these concerning symptoms you see, the more likely it is that the child is severely ill. If you see any of the following concerning symptoms, contact the child's parent or carer and monitor the child carefully. Consider calling an ambulance Triple Zero (000) if: - any symptoms are severe - symptoms rapidly get worse - multiple symptoms develop Concerning Symptoms Include: • Lethargy and decreased activity – the child wants to lie down or be held rather than participate in any activity, even activities that would normally interest them. • Fever – fever by itself is not necessarily an indicator of severe sickness. However, a fever (temperature more than 38.0 °C) in a child is usually a sign of infection and may need to be investigated. Children less than 3 months of age with a fever of more than 38.0 °C should see a doctor. Ensure you measure temperature accurately. • Poor feeding – the child has reduced appetite and eats and drinks much less than usual. • Poor urine output – the child is going to the toilet less often or not at all; for babies, there are fewer wet nappies than usual. • Pain – a child may or may not tell you they are in pain. Facial expression is a good indicator of pain in small babies or children who do not talk. General irritability or reduced physical activity may also indicate pain in babies or infants. • A stiff neck, irritability (excessive or high-pitched crying) or sensitivity to light – this may indicate meningitis. • New red or purple rash – rashes may be caused by viral infections and other causes (for example, nappy rash). Rapidly developing rashes may be a symptom of more serious	Consider calling an ambulance (000) if: - any symptoms are severe - symptoms rapidly get worse - multiple symptoms develop Call 000 (Ambulance) immediately for urgent medical help. Follow DRSABCD Protocols and Action Plan Notify Centre Director or Responsible Person on Duty: Yes Notify Parent / Guardian / Next to Kin of injury and medical action taken: Yes Incident form prepared and reviewed. Sent to parent using OWNA App. ***** You need to notify the Regulatory Authority if a child experienced serious injury and/or trauma while being educated and cared for by your service and needed urgent medical attention, attended a hospital, or should have attended a hospital; Based on the above assessment Serious Incident Form Lodged via the NQS ITS within 24 hour *****

diseases. Monitor the child carefully if the rash develops rapidly or if it is combined with other concerning symptoms.	
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- OWNA Incident Report (or a Incident, Injury, Trauma and Illness Record Form (HEI007)) and a courtesy call must be completed for ALL children involved in the incident, injury, trauma or illness. (For example: both the child who has been bitten and the biter.)
- Formally notify parents by way of OWNA Incident Report (or an Incident, Injury, Trauma and Illness Record Form (HEI007)), please remember that the parent must sign to acknowledge that they have read the form.
- A child may only leave the Service in the care of a parent, an authorised nominee named in the child's enrolment record or a person authorised by a parent or authorised nominee or because the child requires medical, hospital or ambulance care or other emergency.

Incident Reports are to be at all times objective and must include only what was observed. No subjective statements based on assumptions as to what a team member believed or thought happened are to be included.

Serious Incident report and ACECQA form are to be signed by the Director and filed in the Incident File/Folder after notification (via OWNA App or a paper form if system is offline). Incident reports are to be signed by the Director and filed in the child file/folder or via OWNA.

It is recommended a courtesy call is to be made to parents for all incidents. A courtesy call to parent should be made involving head injury, bites or bleeding (other than grazes), to alert parents to the incident and to remind them that the associated incident report will need to be signed on picking up the child.

Photo and Video Policy & Guidelines relating to Incident, Injury, Trauma and Illnesses forms:

- Parents and guardians as a term and condition of enrolment, give permission for their child's photographs and videos to be taken, used, and stored for the purpose of documenting and completing the Incident, Injury, Trauma, and Illnesses form, both for the OWNA App and Portal, as well as for manual records.
- Incident and illness forms in OWNA allow for photos and videos of a child's injury, but certain rules apply:
 - Avoid taking photographs or videos in the moment when a child is upset/injured.
 - Do not capture images or recordings of a child's private body parts (E.g. genital/bottoms/breast) that are injured.
 - Do not take photos or videos of private body parts or in situations that may compromise a child's privacy (e.g., nappy rash, genitals, bottoms, chests).
 - Do not take photos of private parts, even if requested by a parent or guardian for external viewing.
 - Parental Guidelines: If parents or guardians need to see an injury of a private body part, they should come to the service to attend to their child directly.
 - These guidelines ensure the child safety, privacy, and dignity of the children in care.

Incident, Injury, Trauma and Illness Reporting Register

The Centre Director or Responsible person must check that the Incident, Injury, Trauma and Illness Record form (OWNA Incident Reports) and Staff Incident Report has been completed correctly and log the form (completed in OWNA when form is submitted) on the Incident Report Register at the time the incident report is written and reviewed. Types of incident, injury, trauma and illnesses required to be record on the Incident Report Register are:

- any serious incident, injury, trauma and illnesses

- an incident that is required to be reported to VECRA on the NQS ITS.
- a serious incident that required child to be collected from care or child need to sought medical assistance or support.
- Any incidents that related to concerns in relation to child safety and protection.
- Any minor incidents are to be recorded if there is a repeat or multiple reoccurrences of incidents of a similar nature.

In relation to staff incident, if a staff incident occurs the Staff Incident form is required to be prepared and logged on the Incident Report Register when a staff member has injured themselves or been involved in a non-health related incident (i.e. non-resolved grievance or complaint). Please file a copy of the original incident form in the Incident Report Register (Held in office) or online in OWNA. A copy of incident form is to be file in the individual child online or staff file. Please inform the Approved Provider of incidents logged in Incident Report Register.

The incident statistics we collect are an integral part of our continuous improvement system and used to identify and address incident trends and anomalies. The data is also collated and benchmarked within the organisation to enable identification of areas requiring improvement in the service, room, and support or training an educator/s may require.

The following provides guidance on the completion of incident, Injury and Trauma Form. This should be completed with guidance of services' policy and procedures.

Details of the location of the incident / injury / trauma:

Indoors:

- Bathroom/nappy change area
- Child toilet
- Eating area
- Entrance/corridor
- Kitchen
- Laundry
- Play space/classroom
- Sleeping/rest area
- Staff room/administrative area
- Storage area
- Indoor other

Outdoors/Away from the Service:

- Play space/classroom
- Storage shed/s
- Toilet block
- Walkways
- Outdoor
- Excursion/regular outing (location and place)
- Away from service (other)
- Sandpit
- Playground
- Path

General activity at the time of the incident / injury / trauma:

- Arrival at the service
- Departure at the service
- Leisure based program
- Meal times
- Organised sport
- Play based program
- Sleep and rest
- Transition
- Playing in (E.g. Sandpit)

Cause of the incident / injury / trauma:

Policy Handbook – P004. 7.2026

- Adult/adult interaction
- Animal
- Chemical Substance
- Child/adult (non-staff) interaction
- Child/child interaction
- Child/staff Interaction
- Electrical Wiring/Power point
- Equipment/furniture/Toys
- Glass
- Hot Water / Steam
- Nail/wire/exposed metal/building material
- Needle stick
- Pre-existing medical condition
- Self-inflicted
- Food
- Sun/heat exposure
- Traffic Car
- Trip/fall

Question: Circumstances if child appeared to be missing or otherwise accounted.

If Not applicable: Enter: "N/A"

Questions: Circumstances if child appeared to have been taken or removed from service or was locked in/out of service.

If Not applicable Enter: "N/A"

Notifications

Field "Parent/guardian"= parent/guardian who educators called regarding incident or parent who is collecting child.

Field "Director/Educator/coordinator" = Name of the responsible person in charge at the time of the incident:

Field "Other agency (If applicable)"= to discuss with Centre Director. It could be ambulance, police, etc. Depending on the nature of incident (i.e.. child protection matter) police may not be listed based on form. (Police and child protection will advise on this matter).

Field "Regulatory Authority (if applicable)" : DET if a serious incident (must be within 24 hours).

Serious Incident / Injury / Trauma Follow-up

To be completed by the Nominated Supervisor (or Responsible Person):

1. Wellbeing of the child and date of return (or expected return date)
2. Summary of all communication with family (if applicable)
3. Steps that were taken or will be taken to prevent or minimise this type of incident in the future
4. Detail of any outstanding actions for follow up (if any), expected timeframes and responsibilities
5. Documented conversations with the Regulatory Authority or other agencies (if applicable)

In the event of an Incident (inappropriate behavioural resulting in injury or child sexual behaviours):

- Apply general first aid according to the time, place and injury
- Keep the child comfortable and as relaxed as possible
- Make sure team members are aware of and are caring for other children in the group.
- When an incident, injury or trauma has occurred, children must be intentionally educated regarding the topic to reduce the likelihood of the incident, injury or trauma occurring again. It is recommended that this is taught using a number of different methods and days of the week to ensure all children are covered. Further, the intentional teaching is recommended to extend over a number of weeks. The duration will depend on the nature of the incident (i.e. sexual behaviours or inappropriate play).

- Educators need to communicate with parents and provide resources on incident topic (i.e. Sexual Behaviours, Behaviour Guidance Strategies, etc.). The method of communication and materials provided to parents need to be documented on the Incident, Injury, Trauma and Illness Record Form (HEI007).
- If the incident is related to child sexual behaviours, for guidance please refer to the Child Safety, Protection & Wellbeing Policy for what is 'normal' child/ren sexual behaviours. Further, refer to the services Behaviour Guidance Policy for guidance on managing children's behaviours.
- The Incident, Injury, Trauma and Illness Record Form (HEI007) is required to be completed and reviewed by the Centre Director (or responsible person on duty) prior to a telephone call being made.
- Incident, Injury, Trauma and Illness Record Form (HEI007) and a courtesy call must be completed for ALL children involved in the incident, injury, trauma or illness.
- Formally notify parents by way of Incident, Injury, Trauma and Illness Record Form (HEI007), please remember that the parent must sign to acknowledge that they have read the form.

SUGGESTED TELEPHONE CALL TO PARENT:

Telephone Calls for Inappropriate Behaviour Incidents:

- Centre Director or Educational Leader to make a courtesy call to parents to advise of incident. Notify the parent that you would like to discuss the incident, provide nature of and information about incident and that there will be an incident report to sign when they arrive at the service. The courtesy call should summarise the incident to provide advance notice of incident/s prior to pick-up. After providing information about the incident by way of the courtesy call, if the parent wishes to discuss the incident in detail they can do so on pick-up or in a pre-arranged meeting.
- Providing an option to the parent to attend the service to further discuss the incident now or before pick up may be recommended.
- Please note, if parent wishes to discuss the incident in more detail during courtesy call it may be more appropriate to arrange a meeting time to discuss the incident face to face. If this occurs, you may invite the meeting to take place in person rather than going into further detail over the telephone.
- For "not normal" sexual behaviours incidents (i.e. not "Normal" age appropriate behaviours: See extract below from Child Wise fact sheet "Understanding "Normal" Versus " Problem Sexual Behaviour" (0-4 years old children)") it may be established that detailed discussion on the sexual behaviour incident be held face to face with the parent instead of over the telephone. Please note: A courtesy telephone call is still required to arrange a meeting with the parent to discuss the incident face to face prior to pick-up of child on day of incident.

Telephone Calls for Sexual Behaviours Incidents:

- For child sexual behaviour incidents the Centre Director (or responsible person on duty) must make a courtesy call to parents to let them know of the incident.
- Suggested: Telephone conversations with Parent to inform them of Sexual Behaviours. Please use these words or words to the effect.

Please remember: you need to keep the other child/rens names confidential. If the parent asked who else was involved. You can use the words, "I am sorry, due to privacy, we are only allowed to discuss your child and not another child with you. However, we are having similar discussions with other children's parents involved in the incident" (If applicable).

- Good morning or good afternoon [Parent Name]
- It is [Centre Director Name] from [service name]
- I am just giving a courtesy call to you as part of our procedures to inform you that [child name] was involved in a behaviour incident of a sexual nature. We understand

that this can be a sensitive topic and thought it would be best to let you know and not wait until pick-up time today.

- (If applicable describe current child mood) Eg. I would like to let you know that [child name] is happy/settled [describe mood and what activity is involved in: eg. playing/sleeping] at the moment, etc.
- As it is a sensitive matter, would you prefer to discuss in person rather than over the telephone? (If parent prefers this option please set meeting time. Finish conversation with greeting. Plan for meeting, forms and resources.)

OR: (If parent does not wish to attend the service to meet and would like to discuss over the phone, continue with the following as a guide for conversation to discuss incident.)

- We understand that you would like discuss this over the telephone but our preference would be to discuss this face to face.

- This morning/afternoon [Parent child name] was involved in an incident of sexual nature: [Child name] was [describe observation of sexual behaviour]:
For example.

- *Touching/rubbing his/her own genitals (penis or vagina)*
- *Showing others his/her genitals (penis or vagina)*
- *Touching or looking at private parts of another child/ren*
- *Using slang words/dirty language for bathroom and sexual functions.*

(The above are age appropriate sexual behaviours. Source: Child Wise Fact Sheet. Please refer to below for Very Concerning Sexual Behaviours as per Child Wise Fact Sheet.)

- We understand that these behaviours can be confronting.
- *(Please refer to the extract below 'Child Wise Fact Sheet': if age appropriate sexual behaviour advise the following.)* In accordance with Child Wise organisation these are age appropriate sexual behaviours and some children are interested in [their body parts] /and [other children's body parts].
- The Educators/I have discussed the behaviour with the children/[child name] and have talked to them about inappropriate behaviour and private body parts. We will be incorporating learnings on private body parts and personal space into our curriculum over the coming weeks to discuss this with the children. We will continue to watch for any further behaviours of this nature.
- When you plan to come in today I am happy to discuss this further and provide you with some further information and reading materials.
- We understand that you will be concerned about the incident but we hope that through discussing and educating the children this will help.
- You may have further questions after we finish our conversation. Please do not hesitate to call. If you wish to discuss further or we can discuss at a meeting now or at pickup today.
- Greeting to end telephone conversation.

Extract: Understanding "Normal" Versus "Problem Sexual Behaviour" (0-4 years old children) Source: Child Wise.

Website References: www.childwise.net

Reference 1: <http://www.childwise.net/page/39/fact-sheets>

Reference 2:

http://childwise.blob.core.windows.net/assets/uploads/files/Fact%20Sheets/Sexual_behaviour_0to4yearold.pdf

Reference 3: <http://www.nctsn.org/products/sexual-development-and-behavior-children-information-parents-and-caregivers-2009>

Policy Handbook – P004. 7.2026

Extract from Fact Sheet: Sexual Behaviour of Children Aged 0 to 4 years Old

To know what sexual behaviour is problematic, we need to first understand what is normal for each age and stage of child development.

Considerations

- *Is the observed sexual behaviour age appropriate or concerning?*
- *What is the context of the sexual play?*
- *Age difference and relationship between children/young people*
- *Do not over or under react **Gil (1993)***

Age appropriate sexual behaviours

0 – 4 years; Pre-school

- *Touching/rubbing his/her own genitals*
- *Likes to be rude*
- *Showing others his/her genitals*
- *Playing doctors and nurses*
- *Playing house; Mummies and Daddies*
- *Touching or looking at private parts of familiar children or adults*
- *Using slang words/dirty language for bathroom and sexual functions*

Very Concerning Sexual Behaviours

- *Persistently touches or rubs self to the exclusion of normal childhood activities; hurts own genitals by rubbing or touching*
- *Simulating sex with other children with or without clothes on*
- *Oral sex*
- *Sexual play between children involving forceful anal or vaginal penetration with objects.*

Notification of serious incidents and complaints:

The Approved Provider will notify the regulatory authority within 24 hours of any serious incident at our service (s. 174). This includes any serious injury or trauma to, or illness of a child which a reasonable person would consider required urgent medical attention from a medical practitioner or for which the child attended, or ought reasonably to have attended, a hospital.

Serious injuries, traumas and illnesses include:

- | | |
|--|--|
| • head injuries | • epileptic seizures |
| • broken limbs | • bronchiolitis |
| • burns | • whooping cough |
| • removal of fingers | • measles |
| • meningococcal infection | • diarrhoea requiring urgent medical attention |
| • anaphylactic reaction requiring urgent medical attention | • asthma requiring urgent medical attention |
| • witnessing violence or a frightening event | • sexual assault |

A serious incident also includes:

- The death of a child at the service or following an incident at the Service
- An incident at the service where the emergency services attended or ought reasonably to have attended
- A child is missing
- A child has been taken from the service without the authorisations required under the regulations
- A child is mistakenly locked in or out of the service.

If our service only becomes aware that the incident was serious afterwards, we will notify the regulatory authority within 24 hours of becoming aware that the incident was serious.

We will notify the regulator using form SI01 Notification of Serious Incident .

The Approved Provider will also notify the regulatory authority in writing:

- within 24 hours of any complaints alleging that the safety, health or wellbeing of a child is being compromised at the service or
- within 7 days of any circumstances arising at the Service that pose a risk to the health, safety and wellbeing of a child.
- within 24 hours of the attendance of any children being educated and care for in an emergency. This includes where the child needs protection under a child protection order or the parent of the child needs urgent health care. The emergency care can be for no more than two consecutive days the service operates.

We will advise the regulatory authority what the emergency is and make a statement that the Approved Provider has taken into account the safety, health and wellbeing of all the children attending the service before deciding to accept the additional child/children.

In the Event of a Death:

Centre team members cannot make the assumption that a death has taken place. Team members should respond to a suspected death as they would to a serious incident and commence whatever first aid is required while other actions as detailed below are put in place. Cardio Pulmonary resuscitation (CPR) once commenced will be continued until ambulance personnel arrive and take over the situation.

In the event of a suspected death, the Director must notify the police and Approved Providers/management immediately. This action is to be followed in order by:

- Notifying the parents that they need to come to the Centre. Team members are not to notify parents of a suspected death. Police will fulfil this role in the event that the child has actually died.
- Remaining staff to remain calm and continue with the care of other children, removing them away from the incident.
- The Director/Supervisor must remain with the child at all times.
- The area (i.e. cot room) to remain untouched in order for information to be sighted and collected by necessary authorities.
- Enrolment Documents to be kept with the child and taken to the hospital.
- In addition to completing the internal Incident Form the Centre Director, Nominated Supervisor or the Approved Provider must complete a "Notification of Serious Incident Form -SI01" " form and forwarded by the Approved Provider to ACECQA within 24 hours of the incident.
- All written documentation is to be totally objective, accurate and cleared by an approved solicitor, before being shown to any other person.
- Under no circumstances are the children's files or other documentation to be released to any other person without first seeking authorisation from either the Approved Provider, or their solicitors.
- Detailed report of the Incident, Injury, Trauma and Illness Record Form (*HEI007*) and the Centre operations on the day including incident report, team member staffing levels, number and age of children on premises, action taken at the time, team member statements, follow through and any other relevant information to be compiled by both the Centre Director and management and forwarded to Management.
- Report and copy of the incident form to be forwarded by the Approved Provider to ACECQA.
- If the child is declared deceased, appropriate counselling should be offered to all involved

NB. Centre Director or Responsible to please contact Approved Provider and/or seek legal advice (Eg. From the service's Guild Insurance Advisor) before discussing any insurance related matters.

Work Health and Safety (OHS) requirements

Services should contact WorkCover to determine requirements under current legislation. Under the laws:

- the death of a person is a “notifiable incident”.
- The approved provider or nominated supervisor must notify WorkCover by telephone or in writing (including by facsimile or email) as soon as possible after the death.
- Records of the incident must be kept for at least 5 years from the date that the incident is notified.
- The approved provider/nominated supervisor must ensure the site where the death occurred is left undisturbed as much as possible until an inspector arrives or as directed by WorkCover.

Under the laws serious injury or illness is a “notifiable incident”. Serious injury or illness means a person requires:

- immediate treatment as an in-patient in a hospital, or
- immediate treatment for:
 - the amputation of any part of the body
 - a serious head injury
 - a serious eye injury
 - a serious burn
 - the separation of skin from an underlying tissue (such as degloving or scalping)
 - a spinal injury
 - the loss of a bodily function
 - serious lacerations or
- medical treatment within 48 hours of exposure to a substance.

A serious illness includes any infection to which the carrying out of work is a significant contributing factor, for example an infection that can be linked to providing treatment to a person or coming into contact with human blood or body substances.

A dangerous incident is also notifiable under the legislation. Dangerous incidents include:

- an uncontrolled escape, spillage or leakage of a substance
- an uncontrolled implosion, explosion or fire
- an uncontrolled escape of gas or steam
- an uncontrolled escape of a pressurised substance
- electric shock
- the fall or release from a height of any plant, substance or thing
- the collapse, overturning, failure or malfunction of, or damage to, any plant that is required to be authorised for use in accordance with the regulations
- the collapse or partial collapse of a structure

- the collapse or failure of an excavation or of any shoring supporting an excavation
- the inrush of water, mud or gas in workings, in an underground excavation or tunnel

The approved provider or nominated supervisor must notify WorkCover by telephone or in writing (including by facsimile or email) as soon as possible after the injury, illness or incident. Records of the incident must be kept for at least 5 years from the date that the incident is notified. The approved provider/nominated supervisor must ensure the site where the incident occurred is left undisturbed as much as possible until an inspector arrives or as directed by WorkCover.

Appendix 1: Head Injury – General Advice (Extract)

(Source: http://www.rch.org.au/kidsinfo/fact_sheets/Head_injury/) – February 2026

Key Points

- Head injuries can be mild, moderate or severe.
- **Call an ambulance if your child has had a head injury involving high speeds or height, or if after a knock to the head they lose consciousness or vomit more than once.**
- Your child may develop a number of different symptoms in the weeks after a head injury. If your child develops any of the red flag symptoms described in this fact sheet, you should seek immediate medical attention.
- Children with concussion should have a period of relative (not strict) rest for the first 24-48 hours. Your child should have sufficient sleep, limit screen time but continue with daily activities e.g. reading, walking and gradually increase these activities as long as it only results in brief (less than one hour), mild worsening of symptoms.
- Most children recover well after a mild head injury. If your child is still requiring support to return to everyday activities after two weeks following a mild head injury, they should be reviewed by their GP for a medical assessment. Children with symptoms lasting more than four weeks can be referred to the [RCH Victorian Paediatric Rehabilitation Service \(VPRS\)](#). GPs can make referrals to your local outpatient VPRS services via its website.
- If you have any questions regarding accessing outpatient VPRS, you can contact the RCH VPRS outpatient coordinator by calling 03 9345 9300 or emailing rehab.services@rch.org.au.

Children often bump or bang their heads, and it can be difficult to tell whether an injury is serious or not. Any knock to the head is considered a head injury.

Head injuries are classified as mild, moderate or severe. Many head injuries are mild, and simply result in a small lump or bruise. Mild head injuries can be managed at home, but if your child has concussion symptoms, they should see a doctor.

You should call an ambulance immediately if your child:

- has had a head injury involving high speed or height greater than a metre, for example, a car crash, a high-speed skateboard accident or falling from playground equipment.
- loses consciousness (passes out).
- has a seizure, convulsion or fit.
- is increasingly confused, has memory loss or is less responsive or drowsy.
- has loss of vision or double vision.
- has weakness or numbness/tingling in more than one arm or leg.
- has neck pain or tenderness.
- seems unwell and vomits more than once.
- has a severe or increasing headache.
- is increasingly restless, agitated, or combative.

Glossary of head injury terms

Concussion – a mild traumatic brain injury that alters the way the brain functions. Concussion symptoms are usually temporary, but can include a range of physical symptoms (headaches, nausea, dizziness, fatigue, visual problems, poor balance, sensitivity to noise), changes in emotion (anxiety, irritability, sadness), changes in thinking (fogginess, confusion, difficulty remembering, slower thinking), and sleep disturbances.

Loss of consciousness – when a person is unable to open their eyes, speak or follow commands. They have no awareness of stimulation from outside their body and may not remember the immediate periods before and after the injury.

Signs and symptoms of head injury

The symptoms experienced straight after a head injury are used to determine how serious the injury is. The information below is a guideline.

Moderate to severe head injury

If your child has a moderate or severe head injury, they may exhibit 'red flag' symptoms such as those listed above.

Mild head injury (no concussion)

A mild head injury is when your child:

- has had a bump to the head.
- is now alert and interacts with you.
- has not vomited.
- may have bruises or cuts on their head.
- is otherwise normal.

You should seek medical advice if your child develops new symptoms of head injury or you are worried about them. Otherwise, continue to observe your child for any of the signs and symptoms listed under care at home.

Mild head injury (possible concussion)

A concussion is when your child:

- may have displayed altered level of consciousness (e.g. passed out, confused or forgetful) at the time of the injury.
- may experience a range of physical symptoms (headaches, nausea, dizziness, fatigue, visual problems, poor balance, sensitivity to noise).
- may experience changes in emotion (anxiety, irritability, sadness).
- may have changes in thinking (fogginess, confusion, difficulty remembering, slower thinking) and sleep disturbances.
- these symptoms are usually temporary.

You should seek medical advice if your child has any of the above symptoms of concussion.

Care at home

Children and adolescents with concussion can take up to four weeks to recover, but most concussions will get better on their own over several days. Following a mild head injury, your child should have a period of relative (not strict) rest for the first 24 to 48 hours. They should return to activities of daily living and light physical activity (e.g. walking) but should minimise intense exercise, contact sports and screen time such as television, computers and smartphones during this time.

The **HeadCheck app** can help manage concussion recovery at home and is freely available on the App store and Google store. HeadCheck is an interactive tool; the main benefit is

Policy Handbook – P004. 7.2026

allowing families to monitor children's symptoms on a daily basis, and then providing examples of appropriate activities based on symptoms and age.

Your child may have a headache after a head injury. Give them paracetamol every six hours if needed to relieve pain.

There is no need to wake your child during the night unless you have been advised to do so by a doctor. **Call an ambulance immediately if you have any difficulty waking your child.**

Children who have had a head injury may develop symptoms at various times. Some of the symptoms may not be evident immediately after the initial injury, but may show up over a few days (e.g. fatigue, sleep problems, changes in mood).

If your child experiences any of the following 'red flag' symptoms, take them to the doctor or nearest hospital emergency department immediately:

- worsening symptoms of headache (severe, persistent, not relieved by paracetamol), confusion (unusual or confused behaviour), irritability or vomiting.
- excessive sleepiness or difficulty waking.
- bleeding or any discharge from the ear or nose.
- fits/seizures/twitching/convulsions.
- blurred or double vision.
- poor coordination or clumsiness.
- any new arm or leg weakness, or any existing weakness that gets worse or does not improve.
- difficulty swallowing or coughing when eating or drinking.
- sensitivity to noise.
- slurred or unclear speech.

If your child has had a head injury, they should return to school gradually.

If your child has had a concussion, they should not return to sport for 21 days. However, if they still show signs of concussion at 21 days, it is important they wait until all their symptoms have resolved.

Excessive fatigue

Fatigue is a common problem that can happen after a head injury. When a child has 'excessive' fatigue, it means their brain has to work harder on tasks that were usually easily done, for example doing school work, physical routines, watching TV, playing computer games, or having a long conversation.

Your child may experience some or all of the following symptoms after a concussion, and usually these will gradually decrease and resolve by four weeks post-injury:

- headaches.
- blurred vision.
- dizziness, poor balance, sensitivity to noise and light.
- slowness when thinking, understanding and responding to questions or commands.
- problems concentrating.
- difficulties with memory.
- difficulty thinking of the right words to say.
- being more demanding than usual, and become easily frustrated.
- being more fearful and anxious.
- changed sleep patterns.
- mood swings and irritability.

If your child's physical or cognitive performance or behaviour is very different to normal, or it is getting worse, take them back to the doctor or your nearest hospital emergency department.

While symptoms persist, children should have a period of relative (not strict) rest but should continue with activities of daily living and return to light physical activity e.g. walking. Children should have sufficient sleep, a healthy diet and limit watching of TV or playing on mobile electronic devices. Allow your child to gradually return to reading and other activities that require periods of greater concentration or thinking.

Returning to Care

Please also refer to the Kids Health Info factsheet: Head Injury - return to sport.

A minimum of one week away from school is recommended for children who have sustained a head injury that requires medical attention. It is important to let the school know about your child's head injury. If your child is tired, allow them to return to school and other activities gradually. An individual plan will be recommended for your child as every head injury is different. Discuss the details with your child's doctor.

Returning to sport

Your child should avoid sports and activities that involve height and speed, and any other activities where they are at risk of another head injury.

Your child may be able to participate in sports listed below after six weeks, however, this will be determined by the treating medical team when your child attends a clinic for a follow up appointment.

Following a head injury your child's reaction times and thinking may be slower, and this can put them at risk of further injury.

At risk sports and activities include:

- tackle football
- soccer and hockey
- basketball and netball
- horse riding
- riding motorbikes or BMX bikes
- skiing, snowboarding and surfing
- riding a bike, scooter, skateboard or skates
- playing on a trampoline
- climbing trees or other high structures.

Make sure your child always wears a helmet when bike riding or skateboarding.

It is important to encourage gradual return to physical activity. Encourage your child's participation in gentle physical activities such as the following:

- swimming
- walking
- using an exercise bike
- practicing ball skills (with a soft ball)
- tennis
- dancing.

If your child's behaviour is very different to their normal behaviour, or the pain does not go away, go back to the doctor or to your nearest hospital emergency department.

Disclaimer

This information is intended to support, not replace, discussion with your doctor or healthcare professionals. The authors of these consumer health information handouts have made a considerable effort to ensure the information is accurate, up to date and easy to understand. The Royal Children's Hospital, Melbourne accepts no responsibility for any inaccuracies, information perceived as misleading, or the success of any treatment regimen detailed in these handouts. Information contained in the handouts is updated regularly and therefore you should always check you are referring to the most recent version of the handout. The onus is on you, the user, to ensure that you have downloaded the most up-to-date version of a consumer health information handout.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/04/2026

Reference:

NQS

QA2	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	85	<i>Incident, injury, trauma and illness policies and procedures</i>
	86	<i>Notification to parents of incident, injury, trauma and illness</i>
	87	<i>Incident, injury, trauma and illness record</i>
	88	<i>Infectious diseases</i>
	89	<i>First aid kits</i>
	97	<i>Emergency and evacuation procedures</i>
	161	<i>Authorisations to be kept in enrolment record</i>
	162	<i>Health information to be kept in enrolment record</i>

Other References:

Staying Healthy in Childcare – 6th Edition

Handbook on Child Care Licensing

QIAS Principle 5.1, 5.2, 5.4, 5.5

Standards Australia – First Aid

Work Health and Safety Act

http://www.rch.org.au/kidsinfo/fact_sheets/Head_injury/

Department of Education - First Aid for Students and Staff <https://www2.education.vic.gov.au/pal/first-aid-students-and-staff/guidance/general-first-aid-procedures-staff-and-students>)

Legislation:

Work Health and Safety Act

Occupational Health and Safety Act 2004

Occupational Health and Safety Regulations 2007

20 - EMERGENCY MANAGEMENT AND EVACUATION PLAN POLICY & PROCEDURES

Position Statement

Emergencies can occur despite the best preventative planning. In the event of an emergency happening, it is vital that the team members and visitors are able to act effectively to maintain the safety of all those involved.

Objective

In the event of an emergency at the service, we aim to manage the situation in a rehearsed, timely, calm and safe manner to secure the safety of each person at the service. Emergencies may require us to evacuate, lock out or lock down the premises. The safety and wellbeing of each child and adult at the service is paramount above any other consideration in the time of an emergency or evacuation. Any other procedures will be carried out only if it is safe to do so.

Definitions

“Direct Egress” - means the ability to move and directly exit to an assembly area that is at the same level as the education and care service and is outside the service premises and away from the building. This does not include travelling through sets of stairs (including fire isolated stairwells), busy occupied areas, traffic or other hazards, or obstructions. Source: ACECQA Guide to the NQF

“Emergency” - an incident, situation or event where there is an imminent or severe risk to the health, safety or well-being of a person at the service (e.g., a flood, fire or a situation that requires the service premises to be locked down or other type of emergency response). Source: ACECQA Guide to the NQF

“Emergency drill/rehearsal” - a process to rehearse the possible emergency scenarios or events, designed to help clarify roles and responsibilities, provide training and test the adequacy of the emergency response. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Emergency Services” - includes ambulance, fire brigade, police and state emergency services. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Evacuation diagram/plan” - an evacuation plan is used where it is deemed necessary to evacuate the immediate area or building. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Evacuation route” - Continuous path of travel (including exits, public corridors) from any part of a building to a safe place. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Harm” - Physical or mental injury; hurt. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Hazard” - a danger or risk, even though often foreseeable. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Lock Down” - a security measure taken during an emergency to prevent people from leaving or entering a premises until the threat or risk has been resolved. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Lock Out” - a security measure taken during an emergency to prevent people from entering a premises until the threat or risk has been resolved. Source: ACECQA Policy Guidelines: Emergency and Evacuation

“Risk assessment” - assessing the risk means working out how likely it is that a hazard will harm someone and how serious the harm could be. Source: ACECQA Risk assessment template: Excursions

“Responsible Person / Responsible Person In Charge of the Service”

- is the individual physically present and in charge of daily operations, ensuring legal compliance, safety, and supervision.

Related policies

34- First Aid Policy
75 - Lockdown Policy
81 - Hazard Risk Management Policy Procedure and Program
21 - Severe Weather Policy
10 - Occupational Health, Safety & Wellbeing Policy
73 - Lost Child Policy
15 - Security Policy
87 - Medical Conditions Policy
33 - Medication Policy
32 - Infectious Diseases and Immunisation Policy
35 - Anaphylaxis Management Policy
36 - Asthma Policy
85 - Diabetes Policy
94 - Cystic Fibrosis Policy
91 - Epilepsy Policy

Implementation

Our service has procedures in place to ensure the safety, health and well-being of children, staff and visitors during emergencies and evacuations. Specifically, we:

- Conduct thorough risk assessments in line with the *National Regulations*
- Follow best practice emergency planning and management, as well as guidance from ACECQA and regulatory authorities
- Have an Incident Management Team (IMT)
- Have a comprehensive *Emergency Management Plan* that includes clear procedures for dealing with a range of emergencies
- Have an *Emergency Communication Procedure*
- Conduct quarterly emergency drills for evacuations and other emergencies
- Have defined roles and responsibilities for staff, volunteers, students and families
- Train staff, volunteers and students on our policies and procedures, both at induction and at regular intervals
- Communicate our emergency policies and procedures to the children and families
- Ensure that our emergency equipment – such as fire extinguishers, blankets, Emergency Kits – are maintained and tested according to instructions
- Monitor and audit compliance and strive for continual improvements to our practices.

Risk assessments

We will conduct a risk assessment of potential emergencies that are relevant to our service at least once every 12 months and as soon as practicable after becoming aware of any circumstances that may affect the safe evacuation of children from our service. A record of each risk assessment conducted will be kept, in line with our regulatory obligations.

We will ensure that children are protected from risks, and that any newly identified risks are communicated to the nominated supervisor and approved provider as soon as practicable.

Risk assessment areas to consider

Risk assessments must identify and assess the risks of potential emergencies and specify how the risks will be minimised and managed.

Possible emergencies including:

- Natural disasters such as earthquake, flood, bush fire, extreme weather and storms
- Personal injuries and threats, suspicious mail, siege, bomb scare

- Chemical spills, biohazards, fire, gas leak, explosion.

Potential hazard during emergencies including:

- Heavy items falling particularly if they are stored high
- Hazardous materials leaking
- Communication/power outages or live wires
- Debris blocking preventing safe evacuation
- Structural damage to buildings.

EMERGENCY MANAGEMENT PLAN

Our *Emergency Management Plan* is based on all identified risks and includes appropriate responses including evacuation, lockdown, lockout and shelter-in-place, and drill and training schedules. Where appropriate, we will consult with local emergency services (e.g., fire, police, ambulance), local government, community leaders and other relevant agencies for advice about issues like evacuation routes, assembly points and accessibility for adults or children with special needs.

Our Emergency Management Plan will include:

- Emergency contact details for people who have specific roles or responsibilities
- Contact details for local emergency services
- A description of how we will alert people to an emergency, e.g., siren/bell
- Emergency procedures (see **Appendix A**). We may discuss our procedure - for example, evacuation point/route and alternatives - with local emergency services, community leaders and/or other relevant authorities, or access their online resources to ensure our procedures are robust, and accessible for adults or children with special needs
- How we will assist any child or person with special needs
- An Evacuation Diagram (see below for details) based on service floor plans showing the location of fire equipment, emergency exits and assembly points
- Processes for advising neighbouring businesses/residences about emergencies
- Processes to ensure staff are trained in our emergency procedures
- Processes we will follow after an incident
- Procedures we will follow to test the Emergency Management Plan and familiarise children and staff with the Plan
- For services located in multi-storey buildings shared with other occupants and on a storey with no direct egress to an assembly area should consult the Australian Standard AS3745:2010 Planning for Emergencies in Facilities to determine the standards for emergency planning. The *National Regulations* also require these services to include in their plans and procedures the following:
 - All possible evacuation routes from each storey on which the premise is located
 - The evacuation routes that are proposed to be used in an evacuation
 - How all children will be safely evacuated from the premises, including non-ambulatory children
 - The stages in which an evacuation will be carried out
 - The identity of the person in charge of the evacuation
 - The roles and responsibilities of staff members during an evacuation

- The arrangements made with the other occupants of the multi-storey building in relation to the evacuation of the multi-storey building.

Evacuation Diagram

The Evacuation Diagram of the floor or area will include the following elements and will be at least A4 size:

- Title, e.g. Evacuation Diagram
- Designated exits in green
- Hose reels, hydrants, extinguishers, fire blankets in red
- “You are here” location
- Location of assembly area(s) including shelter in place location (stated pictorially or in words)
- Location of communication equipment in red
- Legend with the symbols used
- Date Diagram completed and when it will be reviewed
- If the Diagram includes additional elements, including those listed below, recommended to be at least A3 size:
 - Direction of North
 - Paths of travel in green
 - Location of First Aid Kits
 - Location of Hazards
 - Emergency information (e.g., phone numbers, procedures).

Displaying information from the Emergency Management Plan

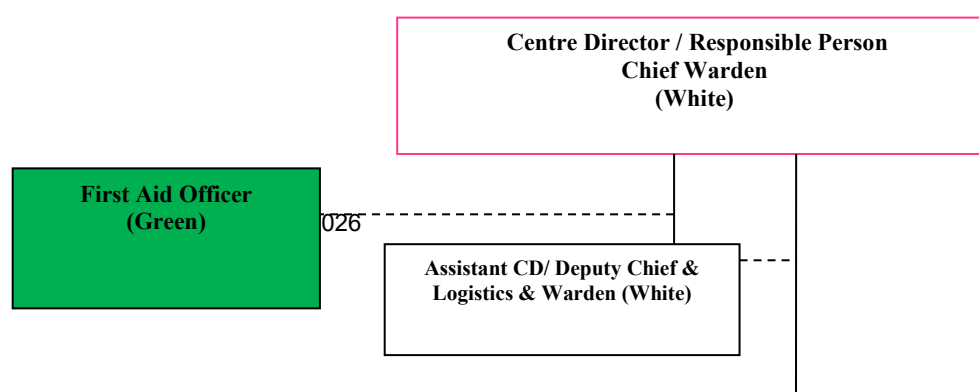
Relevant information from the Emergency Management Plan is displayed prominently at our service to ensure it can be easily identified and is accessible to all educators, staff, visitors, volunteers and families. Relevant information includes:

- Emergency service telephone numbers which will be displayed near telephones and service exits
- Evacuation procedures and diagrams which will be displayed near each exit. The Evacuation Diagram will be displayed at a height not less than 1200 mm and not more than 1600. It will also be oriented correctly in relation to the emergency exit, the ‘You are here’ point and the assembly area(s).

Incident Management Team (IMT)

Our service has established an Incident Management Team (IMT) to plan for and manage emergencies. The nominated supervisor or responsible person is the Chief Warden / team leader. The Incident Management Team are the Room Leaders or Lead Educators of the Room.

The Chief Warden at the time of the incident will allocate present service staff to the following “Logistic & Deputy Chief Warden” and also the “First Aid Officer” roles. The Room Warden Role by default will be the Educators performing the Room Leader or Lead Educator role at the time of the emergency.



IMT Member	Role	Nominated Staff Member Roles	Nominated 'Back up' Staff Member Roles
Chief Warden: Incident Controller	In charge of overall management of emergency situation	Centre Director/ Nominated Supervisor	Assistant Centre Director Or Responsible Person
Deputy Chief & Logistics Warden	Responsible for securing materials, resources, services, additional staff	Assistant Centre Director or Appointed Qualified Educator	Appointed Qualified Educator
First Aid Officer	Co-ordination and application of first aid	Appointed First Aid Officer 1	First Aid Officer (Backup)
Room Warden	Children care/ensuring adherence to service protocols, procedures	Room Leader	Lead Educator - Qualified Educator

Incident Management Team is responsible for: updating risk assessments; advising the approved provider and nominated supervisor of any necessary updates to our policy and procedures; scheduling rehearsals and training; sharing information with other staff, children and families; and maintaining and testing equipment.

IMT Responsibilities

Chief Warden – Nominated Supervisor or Responsible Person

Pre-emergency

- Maintain a current register of IMT members.
- Replace IMT members when a position becomes vacant.
 - Conduct regular exercises/drills.
- Ensure the emergency response procedures are kept up-to-date.
 - Coordinate meetings of the IMT as appropriate.
 - Plan for resources required.
- Maintain records and logbooks and make them available for emergency response.
 - Ensure emergency contact details are up-to-date.
 - Attend training and emergency exercises, as required.
- Ensure logistics officer and wardens have communicated the emergency response procedures to all occupants within their nominated areas.
- Coordinate Safety practices (e.g. clear egress paths, access to first attack equipment e.g. fire extinguishers and disposal of rubbish) by logistics officer (wardens) throughout their areas.

During emergency

- Attend the emergency control point.
 - Ascertain the nature and scope of the emergency.
 - Ascertain the nature and location of the emergency.
 - Ensure the appropriate response has been actioned.
 - Ensure that the emergency services have been notified.
- Establish communications with operations officer (area wardens).
 - Initiate evacuation of affected areas if necessary.
 - Transmit instructions and information.
- Brief the incoming emergency services and respond to their requests.

Post-emergency

- When the incident is rendered safe or the emergency services returns control, notify the IMT members to have staff and students return to the service.
- Organise a debrief with the IMT and, where appropriate, with any attending emergency service.
- Compile a report for the IMT using the OWNA Head Count and Emergency Evacuation Record form.
- Collate records of events during the emergency for the debrief and ensure they are secured for future reference. (OWNA App: Evacuation & Head Count tool. OWNA Forms: Emergency Evacuation Record)

Deputy Chief & Logistics Warden

Pre-emergency

- Ensure staff are aware of the emergency response procedures.
- Carry out safety practices (e.g. clear egress paths, access to first attack equipment e.g. fire extinguishers and disposal of rubbish).
- Attend training and emergency exercises.

Emergency

- Activities may include the following:
 - operate the communication system in place
 - check that any fire doors and smoke doors are properly closed
 - close or open other doors in accordance with the emergency response procedures
 - search the floor or area to ensure all people have evacuated, this function is of greater importance than a later physical count of those evacuated
 - ensure orderly flow of people into protected area
 - assist occupants with disabilities
 - act as lead of groups moving to nominated assembly areas
 - report status of required activities to the area warden upon completion.

Post-emergency

- Compile report of the actions taken during the emergency for the debrief.

Room Warden – Room Leader / Lead Educator

A room warden is a designated individual responsible for preventing fire hazards, implementing safety procedures, and leading emergency evacuations. They proactively monitor risks, ensure equipment maintenance, guide occupants to safety, and account for personnel during incidents, acting as a crucial link in the emergency management plan.

Emergency Communication Procedure

Our Emergency Communication Procedure is at **Appendix B**.

Ensure there is access to reliable communication channels in the event of an emergency by maintaining access to a telephone (such as fixed-line telephone or mobile phone) at all times.

The main telephone is located at Service Office. If there is a complete loss of electricity and the telephones are not available, a mobile phone will be used by the Centre Director or the Responsible Person and make emergency contact.

Emergency Kit and Supplies

The nominated supervisor/responsible person will ensure an Emergency Kit includes:

- Access to a Mobile phone. ie Nominated Supervisor or Responsible Person
- Current emergency contact details for each child
- Employee and next of kin contact details (Accessed via OWNA App)
- Emergency service telephone numbers
- Working torch and spare batteries
- Fully stocked first aid kit
- Administration of medication records and medical management plans
- Drinking water
- Woollen blankets/towels
- Gloves
- Portable radio and torches - battery-powered (Spare batteries)
- Whistle
- Pen and paper
- Alcohol based hand sanitiser
- Sunscreen
- Nappies/wipes/change mat

- Infant bottles and formula (to be collected on exit)

The nominated supervisor/responsible person will ensure the Kit contains a list of items that must be added at the last minute in an emergency. Items will include:

- Attendance registers for children, staff and volunteers
- Medications for staff and children

Emergency and evacuation rehearsals/drills

The service will add to each child's sense of security, predictability and safety, and ensure all educators and staff are familiar with our emergency procedures, by conducting evacuation drills at least **every three months**. Both an evacuation rehearsal/drill and a lockdown rehearsal/drill needs to be performed every 3 months.

If necessary, the nominated supervisor will prepare a risk assessment to identify and manage risks that may arise in emergency drills and evacuation points.

The nominated supervisor will develop a schedule for conducting drills for the different types of emergencies identified in the *Emergency Management Plan* and will diarise to ensure these are completed. All rehearsals and drills will be recorded and available for inspection.

The drills will:

- Take place at various times of the day and week (rather than always on a Tuesday at 10am, for example) to ensure all children and staff members get the opportunity to rehearse. Rehearsals may also be conducted when families are present to help them become familiar with the emergency procedures. All persons present at the service during the evacuation drill must participate
- Be documented and assessed against specific outcomes using the checklist at **Appendix C**. The service will appoint an observer to evaluate our drills using the checklist at Appendix C. The observer maybe the Nominated Supervisor or Responsible Person or a person appointed to complete this role.
- Be immediately followed by a debriefing session, if possible, to identify any improvements that may be made. Any training needs will be identified, and action will be taken to implement the relevant training

Dealing with Trauma

Emergencies and natural disasters are extremely stressful, and it is normal for children and adults to feel overwhelmed and distressed. People cope with trauma in many different ways. Children look to adults for reassurance, care and opportunities to share their feelings. It is important for educators to understand the impact of disasters and seek help when needed.

The approved provider/nominated supervisor will support educators to provide information to parents and families following any emergency or natural disaster including:

- will the service be open in the days and weeks ahead?
- how to find alternative care and education
- how to contact services for support with dealing with trauma

Several organisations offer support for educators in these situations:

[Emerging Minds](#)

BeYou- [Trauma informed practice](#)

Information sharing, training and monitoring

The nominated supervisor will include the *Emergency Management & Evacuation Policy and Procedures* in staff inductions and ensure staff, volunteers and students receive practical training in relation to the requirements, including how to identify and manage risks. The

nominated supervisor also implements an ongoing training program tailored to each staff member's needs and goals, which are identified through regular performance reviews.

At orientation, parents will be provided with the *Emergency Management and Evacuation Policy and Procedures*. Staff and family can access the risk assessments we conduct. Where appropriate, visitors will also be made aware of the emergency response procedures.

Our *Emergency Management Plan* will be tested every 3 months and reviewed at least annually and as soon as practicable after an emergency event. Other scheduled discussions and training for the IMT and all staff, students and volunteers will take place regularly, e.g., how to use fire extinguishers, fire blankets and other emergency equipment, communication arrangements, identifying assembly points and the location of emergency equipment, first aid arrangements and how to turn off the electricity and gas supplies. All new staff, students and volunteers will receive training during their induction and refresher training will take place at least annually. (Note – our *Incident, Injury, Trauma and Illness Policy* describes our service's training arrangements for First Aid).

We will use informal games and discussions to familiarise children with our evacuation and emergency procedures as well as regular rehearsals.

The approved provider and nominated supervisor will monitor staff to ensure they are following our policies and procedures for emergencies and evacuations. They will act quickly to fix any issues and will give staff any extra support or training they need to comply. Volunteers and students are also required to comply with all service policies and procedures.

We will keep a record of all actual and rehearsed emergencies, training and risk assessments, which can be accessed by staff, volunteers and families. These will be recorded in OWINA software or on paper forms.

Roles and responsibilities

Responsibilities	Role
Ensure our service meets its obligations under the <i>Education and Care Services National Law and Regulations</i> , including to take every reasonable precaution to protect children from harm and hazards likely to cause injury and ensure children are adequately supervised at all times.	Approved Provider Nominated Supervisor
- Ensure that our service has policies and procedures in place for emergencies and evacuations that address specific areas set out in the <i>National Regulations</i> - i.e., this <i>Emergency Management and Evacuation Policy and Procedures</i> will be in place. - Ensure our procedures set out instructions for what to do in an emergency and include an emergency and evacuation floor plan.	Approved Provider
Take reasonable steps to ensure that nominated supervisors, staff and volunteers follow, and can easily access, the <i>Emergency Management and Evacuation Policy and Procedures</i> , including by: - Providing information, training and other resources and support - Providing this <i>Policy</i> at induction - Clearly defining and communicating roles and responsibilities for implementing this <i>Policy</i> - Communicating changes to routines and policies - Monitoring and auditing of staff practices (including through spot checks) and addressing non-compliance quickly - Regularly reviewing this <i>Policy</i> The <i>Policy</i> must also be available for inspection.	Approved Provider (ultimate responsibility) Nominated Supervisor
Notify families at least 14 days before changing <i>Emergency Management and Evacuation Policy and Procedures</i> if the changes will: Affect the fees charged or the way they are collected; or	Approved Provider

- Significantly impact the service's education and care of children; or - Significantly impact the family's ability to utilise the service.	
- Ensure risk assessments are conducted to identify potential emergencies, in accordance with regulations and having regard to all the areas covered in Risk Assessment section of this <i>Policy</i>. - Ensure a risk assessment is conducted at least once every 12 months and as soon as practicable after becoming aware of any circumstance that may affect the safe evacuation of children from our service and update our policies and procedures accordingly. - Ensure staff are aware of and can access/use the risk assessment to manage risks and ensure the safety of children and each other. - Keep a record of all risk assessments conducted.	Approved Provider (ultimate responsibility) Nominated Supervisor
Ensure that procedures are appropriate in practice to our service, identify risks and hazards, and any potential improvements to make to the <i>Emergency Management and Evacuation Policy and Procedures</i> . Report any issues to the appropriate staff member (either approved provider, nominated supervisor, or educators).	Approved Provider Nominated Supervisor Educators and Families
Implement the <i>Emergency Management and Evacuation Policy and Procedures</i> .	Nominated Supervisor and Responsible Person
Be aware of legal obligations and understand and follow the <i>Emergency Management and Evacuation Policy and Procedures</i> .	Educators and other staff Volunteers
Be aware of <i>Emergency Management and Evacuation Policy and Procedures</i> and follow the directions of staff if present during a rehearsal or real emergency or evacuation	Families
Ensure that a copy of the Emergency Management Plan and Evacuation Diagram and instructions are displayed in a prominent position near each exit at our premises.	Approved Provider and Nominated Supervisor
Ensure emergency equipment is maintained and tested according to instructions and easily accessible	Approved Provider and Nominated Supervisor
Keep up-to-date emergency contact lists in each room in our service	Nominated Supervisor
Provide up-to-date contact details and emergency contact details	Families
- Ensure that the emergency and evacuation procedures are rehearsed every 3 months with staff, volunteers and children present, and under the guidance of the nominated supervisor. - Keep a record of all rehearsals carried out. - Ensure that all educators are trained in the procedures and know their roles and responsibilities and the evacuation points.	Approved Provider and Nominated Supervisor
Participate in emergency and evacuation procedures if present at the service when they are occurring.	Nominated supervisor Educators and other staff Families Visitors
Communicate the emergency procedures with children and families.	Nominated Supervisor Educators and other staff Volunteers
Complete attendance records when delivering or collecting child	Families

Preparing for an Emergency Links

- Australian Government Department of Education - [Help in an emergency:](https://www.education.gov.au/early-childhood/providers/extra-support/emergency) <https://www.education.gov.au/early-childhood/providers/extra-support/emergency>
- [Australian Government Bureau of Meteorology:](https://www.bom.gov.au/) <https://www.bom.gov.au/>

Jurisdiction Specific Information

VICTORIA (VIC)
- Country Fire Authority Victoria: www.cfa.vic.gov.au - Victoria Police: www.police.vic.gov.au - Victoria State Emergency Service: www.ses.vic.gov.au
NEW SOUTH WALES (NSW)
- NSW Police: www.police.nsw.gov.au - NSW Rural Fire Service: www.rfs.nsw.gov.au - NSW State Emergency Services: www.ses.nsw.gov.au

Implementation

- All exits to be clearly marked
- All fire alarm points and evacuation assembly zones to be known to team members
- All fire fighting equipment locations to be known by team members
- Emergency evacuation drills (both fire and lockdown) need to be carried out regularly (as specified in state regulations) are less than every three months/90 days for both fire and lockdown drills. Each drill is to be documented using the specific forms (see below). Drills should be held on rolling days to ensure that all children (regardless of their attendance pattern) are included in the drill practice.
- All rooms and foyer will display the evacuation procedure plus building plan with the closest exit highlighted
- All persons in the building during an evacuation practice must participate in the evacuation
- Every six months all fire extinguishers, fire hose reels and emergency exit lights are to be checked by a certified fire extinguisher company.
- External agencies will be employed to assist the service with this maintenance if no currently employed staff members are qualified to complete the maintenance checks (i.e. fire fighting equipment).
- Please refer to the Safe Child Environment Policy for fire safety review procedures.

Our service will conduct a risk assessment to identify potential emergencies that could affect our service and use this to prepare emergency and evacuation procedures. An evacuation may be necessary in the event of a fire, chemical spill, bomb scare, earthquake, siege, flood etc.

Head Count Procedure

Regardless of whether the evacuation is for a drill or real the following procedures need to be followed:

- Identify the immediate source of danger.
- Stay in place to safeguard those children who may be at risk of danger.
- Call for help from other team members and make sure that the Centre Director or a senior team member or management is notified immediately.
- Should emergency services be needed the Centre Director or Person in Charge will notify them.
- Please note: If the evacuation is a drill the Centre Director/Responsible Person must alert the security company to the fact that the Centre is holding a fire drill prior to the drill being carried out (if service as an alarm system and being used in the drill).

- On hearing the fire alarm raised, all contact team members will stop the children doing any activity and marshal them in the room or outdoor assembly point (depending on where hazard has been identified).
- Centre Director or responsible person on duty nominate emergency evacuation roles and wear warden vests. See below roles of wardens.
- Appoint an **Emergency Drill/Exercise 'Observer'**. The Emergency Drill / Exercise 'Observer' for Drills will be the Centre Director / Responsible Person or a person appointed to complete this role. Please see Emergency Drill/Exercise Record to be completed during and at the end of drill.
- Infants are to be placed into the fire cot and wheeled to the evacuation point.
- Room Leaders/Room Lead In charge will collect the room rolls/room routines and organise to lead the children calmly through the marked exit from the room.
- Educators will follow the children in the rear and check that all children have been collected with the group.
- Once the children, staff and visitors have exited the room/yard and are assembled at the evacuation assembly point each Room Leader/Room In-charge will perform headcounts and complete the following forms:

Task 1: Evacuation & Head Count (OWNA App> Evacuation & Head Count tool) – complete this form for each room to account for children, staff members and visitors in that room. The head count should be checked by a second educator to verify headcount. Select the relevant report type from the list.

Evacuation & Head Count		
 CHILDREN	 STAFF	 VISITORS
0/90	0/21	0/1
What Report is this? Head Count - Head Count - Excursion Head Count - Bus Run Head Count - Practice Lockdown - Practice Evacuation / Fire Drill - Practice Medical Emergency - Lockdown - Evacuation - Medical Emergency		

The Centre Director / Responsible Person in Charge of Service / Lead Fire Warden will ensure that headcounts are reconciled for all rooms and service (and double checking of headcount has occurred).

Task 2: Emergency Evacuation Record: for Fire or Lockdown (Practice Drill / Real Event) See OWNA App > Forms > Emergency Evacuation Record (custom form). This form is completed by each room to document the lockdown drill, fire drill or real event.

Emergency Evacuation Record

Points to note: • Fire drills must be done minimum of each 3 months to comply with Regulations. • Keep a record of each drill for a 2 year period. • Please ensure that drills take place in differing locations around your service, on different days of the week and different times of the day. • New staff members and relief staff need to be made aware of the centre's emergency evacuation procedures upon arrival.

Emergency Evacuation Record Type

- Fire ☐

- Lockdown ☐

Practice or Real Emergency

- Practice Drill ☐

- Actual Emergency ☐

- During the evacuation, the Centre Director or Responsible Person in charge of Service or Lead Fire Warden will go through the Centre areas to check that all children and adults have exited the service.
- As each room is checked the doors are to be closed preventing children or team members returning to the building. A sign should be placed at the service front door or gate to advise that a fire drill, lockdown drill or real event is taking place and not to enter the service.
- The Centre Director / Responsible Person in Charge of Service at the end of the fire or lockdown drill will review the Evacuation & Head Count form and Emergency Evacuation Record to review the contents and comments. If there are any reflections that require resolution, please document the follow-up and resolution as a reflection in OWNA. The service needs to document the resolutions of any procedure adjustments or changes.

The following will be collected from the office by the Director or Responsible Person as they leave and taken to the evacuation assembly point:

- Room Child Routine Schedules (i.e. showing children attendance information)
- First Aid box
- Emergency evacuation bags (containing gloves, spare nappies, etc)
- Parent details Centre Contact List (entire centre)
- Mobile phone (personal-to be used in the event of an emergency for phone calls only)

Educators should also gather their evacuation bags from their rooms or shed (as applicable).

- The time, date, time taken to evacuate and number of children and team members involved in the fire drill are to be recorded in the fire drill book. Any comments that may be helpful in future evacuation drills are to be added.
- Team members will ensure that all children remain in the evacuation assembly area until the “all clear” is given by the Director or senior team member in charge to return to the Centre.

- Should the evacuation be a real experience a full report will need to be written and sent to the Support Centre as soon as possible following the incident.
- Should the evacuation be real the parents of all the children will be called as soon as possible to ask that the children be collected, unless the usual day can be resumed in a short time frame with no danger or trauma to children or team members.
- Team members will follow the directions of all emergency personnel while the emergency is being dealt with.

Emergency Management Plans

- **FIRE (CODE RED):** Team members should follow the emergency evacuation plan forthwith. Please also refer to guidance on the safe operation of fire extinguisher. Please also watch instruction video on how to use a fire extinguisher.
- **LOCK DOWN / PERSONAL THREAT (CODE BLACK):**
 - a) UNAUTHORISED PERSON ENTRY INTO THE CENTRE: e.g. custody disputes, etc.
 - b) BURGLARY
 - c) POTENTIAL HOLD-UP OR ABDUCTION:
- **MEDICAL EMERGENCY (CODE BLUE)**
- **INTERNAL EMERGENCY (CODE YELLOW) - EMISSION / SPILLS (Eg Cleaner's storeroom)**
- **EXTERNAL EMERGENCY (CODE BROWN) – Natural Disaster, Severe Weather**
Severe weather, storms and heat. See 21 - Severe Weather Policy.
- **BOMB / CHEMICAL THREAT (CODE PURPLE):** If a bomb/chemical threat is received by telephone or electronically.
- **SERIOUS INCIDENTS:** Involving a child or other adult; team members should follow the 19 - Incident, Injury and Trauma Policy.

EVACUATION – CODE ORANGE

Emergency Evacuation Procedure

Use this procedure for on-site and off-site evacuations. On-site evacuations may occur when it is necessary to evacuate the building but not the entire service premises.

Nominated supervisor or Responsible Person should advise neighbouring businesses/homes of emergency ASAP if possible, e.g., by phone or visiting businesses/homes.

If service is located within a multi-storey building that is shared with other occupants and we have no direct egress to an assembly area, ensure additional procedures and communication arrangements are set out here also. These procedures will be specific to the building in which you are located (see *Emergency Management and Evacuation Policy* for more details).

1. If there's a fire, staff member who's first on scene immediately (ALSO FOLLOW FIRE EMERGENCY PROCEDURE – CODE RED)
 - sounds alarm, e.g., sounds bell/whistle **three short, sharp blows/sounds repeated three times**. Repeat as walking through the service.
 - and says an evacuation (not practice) is taking place
 - evacuate and extinguishes fire if safe/time to do so
 - calls 000
 - advises nominated supervisor / responsible person in charge of service (chief warden)
2. For other emergencies requiring evacuation, employees or volunteers advise nominated supervisor/responsible person who sounds alarm and says an evacuation (not practice) is taking place and calls 000
3. The nominated supervisor/responsible Person (chief warden) based on emergency circumstances will nominate the emergency assembly point (see options above)
4. For fire emergencies
 - nominated supervisor/responsible person delegates the task of turning off gas and electricity supplies if appropriate and safe
 - staff close all doors and windows
5. All educators on breaks return to their room to help evacuate children to assembly area
 - if children are outside, evacuation may occur from there if this is the safest option
 - if only some children outside, educators/room leader will immediately decide who is responsible for evacuating children who are inside and outside
6. Room leaders advise which educators in room will
 - evacuate mobile children
 - evacuate infants and toddlers in evacuation cots if necessary

- help children and adults who cannot walk by most appropriate method which has been previously discussed, e.g., evacuation cots, wheelchairs, physical assistance
7. Educators evacuate children to nominated assembly area
 - wherever possible use the stairs NOT the lifts in a multi-storey building unless the stairs are unsafe. Never use lifts if there's a fire
 - conduct head count so aware if all children accounted for (see head count procedure)
 - locate child if there is time to do so and this won't risk safety of other children/adults

Educators must acknowledge room leader's directions.
 8. Nominated supervisor / Responsible Person in Charge of Service (RP) advises which educators/staff will check indoor environments, toilet, kitchen, cot rooms and outside areas for children and adults and guide remaining children and visitors to the on-site/off-site assembly point.

Educators/staff must acknowledge nominated supervisor's directions.
 9. Designated educators/staff evacuate toilet, kitchen, playrooms, cot rooms and outside areas to nominated assembly area
 10. Nominated supervisor/RP collects children's and staff attendance sheets, visitor register and the Emergency Kit including medications before leaving centre (must include parent/emergency contact phone numbers)
 11. Nominated supervisor/RP locks door if there is immediate danger inside building. Places notification on entrance to building or gate.
 12. Nominated supervisor/RP advises neighbouring businesses/homes of emergency, e.g. by phone or visiting businesses/homes if this has not already occurred
 13. Educators check all children in their groups are present at assembly area using attendance sheets and report any absences to Nominated Supervisor/RP as soon as possible
 14. Nominated supervisor/RP checks all educators, staff and visitors are present at assembly area
 15. Nominated supervisor/RP advises emergency services immediately if any child or adult is missing and follows their advice
 16. Educators and staff supervise and reassure children
 17. Educators and staff support children, staff and visitors who are injured and apply first aid if required - first aid applied by employees with current first aid qualifications
 18. Educators and staff follow instructions from emergency services
 19. Nominated supervisor/RP and educators contact parents/emergency contacts to tell them what has and will happen by the most appropriate method in the situation, e.g., via service OWNA App post, email, answering machine, telephone calls, phone texts
 20. Nominated supervisor/RP ensures no-one leaves assembly point until emergency services give all clear

After Emergency Evacuation

21. Nominated supervisor/RP ensures children or adults who are injured receive medical attention if required

22. Nominated supervisor/RP and educators contact parents/authorised nominees to collect children if required by the most appropriate method in the situation, e.g., via OWNA App Post, email, answering machine, telephone calls, phone texts
- tell parents/authorised nominees any relevant information, e.g., building damaged and unsafe, evacuation point, areas to avoid, parking instructions
23. Nominated supervisor/RP ensures educators stay on duty to care for and supervise children (after rostered hours if necessary) until families or relief staff arrive
24. Nominated supervisor/RP implements following where parents/emergency contacts cannot be contacted, or are unable to get to the centre, to collect their child:
- contact parents/emergency contacts and authorised nominees every 15 minutes where previous attempts to make contact have been unsuccessful
 - ensure there are sufficient numbers of service staff available (including relief staff) to adequately care for and supervise each child
 - ensure child is never left alone with any adult unknown to staff, or not assisting in managing the emergency or child's care in a professional capacity
 - contact the police or Child Protection Services for advice if emergency is over and service staff are unable to stay with the child any longer
25. Complete *Incident, Injury, Trauma and Illness Record* for children who have suffered an injury or trauma
26. Get parent/authorised nominee to sign *Incident, Injury, Trauma and Illness Record* and give them a copy
27. File original *Record* in child's file and record summary details in the *Incident, Injury, Trauma and Illness Register (OWNA)*, including time notified to Education and Care Regulatory Authority if relevant
28. Service notifies Education and Care Regulatory Authority of serious incident within 24 hours through NQS ITS and records summary details in the *Serious Incident Register*, including time notified to Regulator if relevant
- File acknowledgement with *Incident Record* in child's file via OWNA.
29. If service closes, approved provider reports closure and reason within 24 hours to:
- CCMS
 - Education and Care Regulatory Authority
 - any third-party software provider
- Advise these agencies when service reopens.
30. Approved provider notifies WorkSafe Victoria as soon as possible about **work related incidents** including:
- death of a person
 - a person needing medical treatment within 48 hours of being exposed to a substance
 - a person needing immediate treatment as an in-patient in a hospital
 - a person needing immediate medical treatment for one of the following injuries: amputation, serious head injury or serious eye injury, removal of skin (example: de-gloving, scalping), electric shock, spinal injury, loss of a bodily function, serious lacerations (example: requiring stitching or other medical treatment)

See [WorkSafe Victoria](#) for more information.

31. File notification in WHS Register – Incident Report Register (OWNA)
32. Debrief after emergency, review emergency plan and procedures, and implement any improvements to policy and procedures
33. Record improvements in QIP.

FIRE & SMOKE EMERGENCY – CODE RED

Upon hearing the Emergency Alarm or discovering a FIRE take the following action:

- R Remove** people from the immediate vicinity of the Fire
- A Alert** the Fire Service by telephoning direct '000' or by operating the nearest Manual Call Point [break glass alarm]
- C Confine** the Fire and smoke by closing doors and windows in the affected area
- E Evacuate** to the designed assembly area
- Extinguish** or control the Fire if trained and if it is safe to do so

FIRE AND SMOKE EMERGENCY PROCEDURES

All staff will remain calm and report the outbreak of fire immediately to the nominated supervisor/responsible person who will:

- Follow R.A.C.E protocols
 - Activate the fire alarm. Alarm Procedure:
 - The Centre bell, whistle or horn will **sound three short, sharp blows/sounds repeated three times**. Repeat as walking through the service.
 - The Centre Director or responsible person in charge will sound the bell, whistle or horn and make the following announcement.
- **“There is a FIRE – CODE RED – Name the location of Fire”**
 - **“This is not a drill” OR “This is a drill/rehearsal”**
 - **“Everyone perform Service Evacuation Procedure – Code Orange”**
 - **“Go to Offsite Assembly Point” OR “Go to Onsite Assembly Point”**
 - **Repeat Message to communicate direction to all staff members.**
- Phone **000** to notify the fire brigade. Advise service address and nearest cross street. Exact location of fire (room/area). The number of children and educators on site. The evacuation status and plans.
 - Extinguish the fire (**if safe to do so**). See *Safe Operation of Fire Extinguisher procedure*.
 - Implement Evacuation Procedures - Code Orange, if threat exists and close all doors and windows
 - Check that all areas have been cleared.
 - Check children, staff and visitors are accounted for. Perform Headcount Procedure.

- Follow all evacuation procedures including contact parents and guardians of attending children (if need to be collected from service)
- Notify Education and Care Regulatory Authority of incident as set out in our *Incident, Injury, Trauma and Illness Policy*.

SAFE OPERATION OF FIRE EXTINGUISHERS

Fact

Poor maintenance and incorrect usage of fire extinguishers in the home are two key reasons small house fires can spread; endangering lives and causing considerable damage to property. Fires can be brought under control within the first few minutes of ignition if attended to correctly with an extinguisher that is well maintained, which can buy valuable time before the Fire Services arrive.

How to operate a fire extinguisher

There are a number of different types of portable fire extinguishers, each can be identified by the colour coding and labelling. Check that the extinguisher you intend to use is suitable for the type of fire encountered E.g. a water extinguisher must never be used on any fire involving electrical equipment.

There are four (4) basic steps for using modern portable fire extinguishers.

The acronym **PASS** is used to describe these four basic steps.

1. **Pull (Pin)**
Pull pin at the top of the extinguisher, breaking the seal. When in place, the pin keeps the handle from being pressed and accidentally operating the extinguisher. Immediately test the extinguisher. (Aiming away from the operator) This is to ensure the extinguisher works and shows the operator how far the stream travels
2. **Aim**
Approach the fire standing at a safe distance. Aim the nozzle or outlet towards the base of the fire.
3. **Squeeze**
Squeeze the handles together to discharge the extinguishing agent inside. To stop discharge, release the handles.
4. **Sweep**
Sweep the nozzle from side to side as you approach the fire, directing the extinguishing agent at the base of the flames. After an A Class fire is extinguished, probe for smouldering hot spots that could reignite the fuel.

Extinguisher tips

- The Australian Standard 2444 (AS 2444) Portable Fire Extinguishers and Fire Blankets selection and location will provide comprehensive information.
- Ensure everyone in the service knows the location of all extinguishers and how to use them.
- Only ever operate an extinguisher if safe to do so. If in doubt, get out.
- Remove the safety pin by pulling it sharply (this also breaks the plastic seal). Test to ensure that the extinguisher is operable immediately after removing from the mounting bracket.
- Always try to work in pairs for safety.
- Carry or drag extinguisher to the scene of the fire.

In an Emergency Call Triple Zero (000)

MEDICAL EMERGENCY: DRSABCD - CODE BLUE

Refer to 19 - Incident, Injury, Trauma Illness Policy for more detailed guidance.

Refer to Specific Policies for Guidance:

35 - Anaphylaxis Management Policy 36 - Asthma Policy 85 - Diabetes Policy 94 - Cystic Fibrosis Policy 91 - Epilepsy Policy	22- Emergency Management & Evacuation Plan Policy & Procedure 19 - Incident, Injury, Trauma and Illness Policy 87 - Medical Conditions Policy 33 - Medication Policy 32 - Infectious Diseases and Immunisation Policy
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MEDICAL EMERGENCY PROCEDURE:

- Implement **DRSABCD Action Plan**
- If urgent medical help is required DIAL: 000 (Ambulance)
- If child becomes severely injured or ill whilst at the service and requires hospital treatment, one staff member will apply appropriate first aid and **DRSABCD Action Plan** to the child while another staff member calls an ambulance.
- Parents/Guardians will be contacted as soon as possible.
- If the child needs to be transported by ambulance and a parent is not present, the child will be accompanied by a staff member, or by your nominated authorized emergency contact person.
- Appoint a staff member, if possible, to meet the ambulance at service entrance.
- Remain with the patient until medical help arrives - you are only responsible for providing assistance within your level of expertise.

LOCKDOWN / PERSONAL THREAT – CODE BLACK

(Refer to 75 - Lockdown Policy for further guidance)

Lockdown - a security measure taken during an emergency to prevent people from leaving or entering a premises until the threat or risk has been resolved.

Lockout - used when an internal immediate danger is identified and it is determined that children should be excluded from buildings for their safety.

Lockdown Examples:

• Death of a child on centre property. • Children/staff being taken hostage. • A siege of centre property.	• A disaster in the local community. • Unusual amounts of media attention. • Aggressive trespassers.
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LOCKDOWN/LOCKOUT - PERSON THREAT (CODE BLACK) PROCEDURES

Initial Notification

If an event takes place that requires a "Lockdown" or "Lockout", the following should occur:

- Any staff member who witnesses the event must try to raise an alarm.
- 000 must be called immediately.
- The Director or person in charge will determine the need for a "Lockdown" or "Lockout" and raise the appropriate alarm.

Alarm Procedure

- ***The Centre bell, whistle or horn will sound 5 long blows/sounds. Repeat as walking through the service.***
- The Director or person in charge will sound the bell, whistle or horn and make the following announcement.

LOCKDOWN PROCEDURE (GO INSIDE ROOMS) - Movement of Children and Staff inside rooms

- **"This is a LOCKDOWN – CODE BLACK" OR "This is a LOCKDOWN – CODE BLACK – this is a drill/rehearsal"**
 - **"This is not a fire drill"**
 - **"The threat is located at <name the location>"**
 - **LOCKDOWN - "Everyone is to stay in the room, remain seated and keep calm and quiet."**
- Students and Staff will remain in their rooms.
 - If possible, staff will make efforts to seal and lock classroom doors and windows.
 - Children should remain under the desks (babies placed in evacuation cots) or down low and out of sight during the lockdown period. Preferably away from windows and exit doors.
 - If children are outside staff should move children straight into their rooms.

LOCKOUT PROCEDURE (GO TO YARDS) - Movement of children to outdoor yards

- **"This is a LOCKOUT – CODE BLACK" OR "This is a LOCKOUT – CODE BLACK – this is a drill/rehearsal"**
 - **"This is not a fire drill"**
 - **"The threat is located at <name the location>"**
 - **LOCKOUT – "Everyone to move to the outdoor yards, remain seated and keep calm and quiet"**

The following *Lockout Procedure* will be used when an internal immediate danger is identified, and it is determined that children should be excluded from buildings for their safety. The nominated supervisor/responsible person will:

- Activates lockout procedures
- Announces lockout with instructions about what is required.
- Instructions may include nominating staff to:
 - Lock doors to prevent entry
 - Check the premises for anyone left inside
 - Obtain Emergency Kit.
 - Contacts emergency services on 000
 - Go to the designated assembly area
 - Checks that children, staff and visitors are all accounted for.

Staff Responsibilities:

- Person in charge to contact Police and follow their instructions.
- Staff not involved in the lockdown/lockout or without children to supervise are to go to the front of the Centre to liaise with emergency services if it is safe to do so.
- Educators to complete OWNA documentation records:
 1. Evacuation & Head Count (OWNA Tool)
 2. Emergency Evacuation Record Fire or Lockdown (Practice Drill / Real Event): (See OWNA Forms)
- Head Count: Staff must ensure all children are present and complete a head count using the OWNA Head Count Tool (and Wellbeing chart is used). Any absences must be reported to the Director as soon as it is safe.

- Staff are to close and lock all doors and windows, turn the lights off and ensure children are kept below the window level. (The Director/Person in Charge will need to move from room to room around the centre to lock the doors (that require keys) with the Centre Keys. Any Educational Leaders rostered on with a key must use their keys to lock as many doors as possible.)
- All staff, children and anyone else present will remain in the locked room until the All Clear is given by the Director or person in charge.

All Clear Signal

The All-Clear Signal is as follows:

- The Director or person in charge will sound the Centre's bell (if applicable) or blow a whistle for 5 seconds.
- The Director or person in charge will then say "The Lockdown (or Lockout) has now ended, everyone follow me and the staff in an orderly manner."
- Head Count: After the "All Clear Signal" Staff must ensure all children are present and complete a head count using the OWNA Head Count Tool (and Wellbeing chart is used). Any absences must be reported to the Director as soon as it is safe.
- Emergency Evacuation Record form (OWNA form) will be completed by each room leader to document the Lock down.
- Nominated supervisor or person in charge notifies Education and Care Regulatory Authority of incident.

SHELTER-IN-PLACE PROCEDURE

The following *Shelter-in-place Procedure* will be considered when an event takes place outside of the children's service and emergency services determine the safest course of action is to keep children and staff inside a designated building in the children's service until the external event is handled.

If a shelter-in-place action is determined, the nominated supervisor/responsible person:

- Activates shelter-in-place procedures
- Moves all children, staff and visitors to the pre-determined shelter-in-place area
- Obtains emergency kit
- Notifies parents/families if the shelter-in-place is going to extend beyond the services hours of operation
- Notifies Education and Care Regulatory Authority of incident as set out in our *Incident, Injury, Trauma and Illness Policy*.

INTERNAL EMERGENCY – CODE YELLOW

Examples Include:

Essential services failure	Chemical spill
Person trapped	Gas leak
Structural damage	Suspicious package
Power failure	Water leak / supply failure

FIRE - Refer to **CODE RED**

FOR EVACUATION - Refer to **CODE ORANGE**

INTERNAL EMERGENCY (CODE YELLOW) PROCEDURES:

- Inform the Centre Director / Responsible Person / Chief Warden
- Notify the Emergency Services. Call 000 if required.
- Remove children and people from immediate area
- Isolate hazard if safe to do so

- Restrict access to affected area
- Provide information in relation to:
 - Type of emergency
 - Location of emergency
 - Number of people injured
 - Nature of injuries
 - Remove people from immediate danger
 - Restrict access to the affected area
- Prepare for Onsite or Offsite Evacuation – Refer to Evacuation – Code Orange.
- Provide attending emergency service with relevant information regarding the location of on-site services. Eg Electrical Distribution Boards, Gas Isolation Points, Water Isolation Points
- Complete Incident, Injury, Trauma and Illness Record (if required).
- Notify parent/guardian as soon as practicable.
- Notify Regulatory Authority if required (serious incident).
- Notify WorkSafe Victoria if required.

EXTERNAL EMERGENCY – CODE BROWN

Examples Include:

Natural disasters	Flooding
Bushfires	Civil disturbance
Severe weather	Transport incidents

EXTERNAL EMERGENCY (CODE BROWN) PROCEDURE

Information pertaining to the actual or likelihood of an incident occurring shall be provided to the Nominated Supervisor / Responsible Person (Chief Warden) as soon as practicable.

Assess the situation. Call Approved Provider and/or SES.

If required, call for assistance. Dial 000 or call SES 132 500

Nominated Supervisor / Responsible Person / Chief Warden shall liaise with Emergency Services and assess the necessary action to be taken.

In the event the emergency has or is likely to occur in close proximity to the site, the Nominated Supervisor / Responsible Person/ Chief Warden shall provide the following information to the relevant Emergency Service:

- The type of actual or potential emergency
- Location of the actual or potential emergency
- The numbers of people actually or potentially involved

Inform team member of external emergency and emergency plans.

Where practical, information respecting any actual or impending incident which has the potential to affect the site, shall be relayed to Approved Provider as soon as possible. The following action shall also be considered:

- Restrict access to the affected area/s

- Emergency Closure of Service: early collection of children by parents and guardians.
- Prepare for evacuation of the site: See Evacuation Procedure – Code Orange
- Control mechanisms which may minimise the effect of the emergency

Contact relevant service providers (gas, water, electricity)

Follow advice from Emergency Services

NATURAL DISASTER, SEVERE WEATHER /STORMS AND FLOODING

(Refer to 21 – Severe Weather Policy for further guidance)

The nominated supervisor / responsible person will direct educators and staff to:

- Store or secure loose items external to the building, such as outdoor furniture
- Secure windows (close curtains & blinds) and external doors. If necessary, tape windows and glass entrances. Utilise boards and sandbags if required
- Protect valuables and disconnect electrical equipment – cover and/or move this equipment away from windows
- During a severe storm remain in the building and ensure they and children keep away from windows. Restrict the use of telephone landlines to emergency calls only
- Tune in to ABC radio, if possible, to follow any emergency instructions
- Report to the nominated supervisor/responsible person regarding the status of children, staff and visitor safety.

After the storm passes, the nominated supervisor/responsible person will evaluate the need to evacuate if uncontrolled fires, gas leaks, or structural damage has occurred as a result of the storm.

EARTHQUAKE

If outside:

The nominated supervisor/responsible person will instruct staff and children to:

- Stay outside and move away from buildings, streetlights and utility wires.
- DROP, COVER and HOLD
 - DROP to the ground
 - Take COVER by covering their head and neck with your arms and hands
 - HOLD on until the shaking stops.

If inside:

The nominated supervisor / responsible person will instruct staff and children to:

- Move away from windows, heavy objects, shelves etc.
- DROP, COVER and HOLD
 - DROP to the ground
 - Take COVER by getting under a sturdy table or other piece of furniture or go into the corner of the building covering their faces and head in their arms
 - HOLD on until the shaking stops.

After the earthquake the nominated supervisor will:

- Evaluate the need to evacuate if there are uncontrolled fires, gas leaks or structural damage to the building.
- Assess implementation of Evacuation Procedures – Code Orange or any other emergency procedure.
- Instruct educators, staff and children to watch out for fallen trees, power lines, and stay clear of any structures that may collapse
- Ask educators and staff about the status of staff, children and visitor safety.
- Arrange medical assistance where required
- Instruct educators and staff to help others if possible
- Tune in to ABC radio, if possible, to follow any emergency instructions.
- If there is damage to the facility and it is OK to do so, the nominated supervisor may take notes and photographs for insurance purposes.

BOMB THREAT – CODE PURPLE

BOMB/CHEMICAL THREAT PROCEDURE

IN THE EVENT A BOMB THREAT IS RECEIVED. TREAT ALL THREATS AS GENUINE

FIRE - Refer to **CODE RED**

FOR EVACUATION - Refer to **CODE ORANGE**

FOR LOCKDOWN Refer to **CODE BLACK**

The nominated supervisor/ responsible person will implement the following procedures:

BOMB THREAT RECEIVED BY PHONE		
1	If a direct or indirect threat or warning is received by phone, the receiver answering the phone will check the caller identification number and record the number	
2	The receiver of the phone call will alert the Nominated Supervisor/Responsible Person to call emergency services on another phone without alerting the caller (not on the office main phone).	
3	The receiver of the phone call will try to keep the person calm and talking. DO NOT hang up the phone	
4	The receiver of the phone call will refer to the <i>Bomb Threat Checklist (see below)</i> , asking each question and writing their exact response. Do not say anything that may encourage irrational behaviour.	
5	The Nominated Supervisor/Responsible Person is to follow emergency services instructions. Do not evacuate the service unless instructed to do so.	
6	If instructed to evacuate by emergency services, Responsible Person is to check the immediate area to see if there are any packages, bags or other form of suspicious items that appear to not belong. Do not attempt to inspect the suspicious item.	
7	If a suspicious object is found, clear everyone away from the immediate area	
8	Implement the Service's <i>Lockdown Procedure</i> or <i>Emergency Evacuation Procedure</i> if advised by emergency services	
9	Once instructed to evacuate, ensure doors and windows are left open	
10	Do not leave the evacuation assembly area until instructed by emergency services	

11	After event, debrief team, children and families. Provide contact information for EAP. Review security systems. ie door code.	
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SUSPICIOUS LETTER/ITEM/PACKAGE IDENTIFIED AT THE SERVICE		
1	The educator who identifies the letter/item/package is to avoid handling the item and REMAIN CALM	
2	The Nominated Supervisor/Responsible Person is to notify emergency services immediately and follow all instructions as authorised by emergency services	
3	Do not approach, touch, tilt or tamper with the package	
4	Educators are to move children away from the area and implement the <i>Emergency Evacuation Procedure</i> if advised by emergency services	
5	Check that all children, staff and visitors are accounted for using attendance records	
6	Follow directions from emergency services	
7	Following the resolution of the incident, the Nominated Supervisor / Responsible Person is to notify the Regulatory Authority of the serious incident within 24 hours	
7	If the bomb threat is received electronically- by email or text- DO NOT DELETE THE MESSAGE	
8	Follow the procedure above and alert emergency services immediately	
9	After event, debrief team, children and families. Provide contact information for EAP. Review security systems. ie door code.	

Bomb/substance Threat Checklist

This checklist should be held by persons who regularly accept incoming telephone calls.

KEEP CALM

CALL TAKER		CALL TAKEN	
Name		Date/Time:	
Telephone #		Duration of call	
Signature		Number of caller	

Complete the following for a BOMB THREAT

QUESTIONS	RESPONSES
When is the bomb going to explode?	
Where did you put the bomb?	
What does the bomb look like?	
What kind of bomb is it?	
What will make the bomb explode?	
Did you place the bomb?	
What is your name?	
Where are you going?	
What is your address?	

Complete the following for a SUBSTANCE THREAT

QUESTIONS	RESPONSES
When will the substance be released?	
Where is it?	
What does it look like?	
When did you put it there?	
How will the substance be released?	
Is the substance a liquid, powder or gas?	
Did you put it there?	

CHARACTERISTICS OF THE CALLER	
Sex of caller	
Estimated age	
Accent if any	
Speech impediments	
Voice (loud, soft, etc)	
Speech (fast, slow etc)	
Dictation (clear, muffled, etc)	
Manner (calm, emotional, etc)	
Did you recognise the voice?	
If so, who do you think it was?	
Was the caller familiar the area or service?	

LANGUAGE	
☐ Abusive	☐ Taped
☐ Well Spoken	☐ Irrational
☐ Incoherent	☐ Message read by caller
☐ Other (Specify)	

BACKGROUND NOISE	
☐ Music	☐ Local call
☐ Machinery	☐ Long Distance Call
☐ Aircraft	☐ Other (specify)

EXACT WORDING OF THREAT

ACTIONS	
REPORT CALL TO:	
ACTIONS:	

Appendix B

Emergency Communication Procedure

The approved provider will ensure there is access to reliable communication channels in the event of an emergency by maintaining access to a telephone (such as fixed-line telephone, mobile phone, satellite phone, 2-way radio, video conferencing equipment) at all times.

The main telephone is located in the Service Office. If there is a complete loss of electricity and the telephones are not available, a mobile phone should be used by to make emergency contact.

The nominated supervisor / responsible person will, *where possible*:

- Listen to local radio stations or website (e.g., local ABC station) during emergencies to access current information about the situation.
- Ensure that families are provided with current information about an emergency situation, the actions taken to protect their child, and what actions families should take, through an accessible communication channel such as social media, OWINA App, service website, SMS or email. Multiple channels may be used to assist the flow of information
- Ensure that current information about any service closure due to the emergency is provided through the same communication channels.
- Ensure current information about families' emergency contact details is maintained. This may include families' phone numbers, email addresses and social media accounts. The nominated supervisor will regularly remind families through service communications to update their contact details if required. This information is located in OWINA Portal and App.

Appendix C

Emergency Drill/Exercise 'Observer' Guiding Questions

Item	Yes ✓	No ✓
Were emergency services briefed on exercise prior to exercise being started?		
Did the person discovering the emergency alert the other occupants?		
Was the alarm activated?		
Was the emergency service notified promptly?		
Did staff direct persons from the building/site per the evacuation procedures?		
Were isolated areas searched?		
Was the evacuation logical and methodical?		
Did someone take charge? If yes, who?		
Did occupants act as per instructions?		
Was a roll call conducted for:		
Children		
Staff		
Visitors (including contractors and volunteers)		
Was someone appointed to liaise with the emergency service/s?		
Was someone appointed to liaise with the parents/community?		
Was the emergency service given the correct information?		
Did anyone re-enter the premises before the "all clear" was given?		
Did anyone refuse to leave the building/site?		
Area of Emergency plan tested by current exercise:		

Appendix D

See service's Emergency and Evacuation Risk Assessment and Management Plan in a separate document.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 21/7/2026

Reference:

NQS

Element	2.2.1	Supervision - At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.
	2.2.2	Incident and emergency management - Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.
	3.1.1	Fit for purpose - Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child
	6.2.3	Community engagement - The service builds relationships and engages with its local community.
	7.1.2	Management Systems - Systems are in place to manage risk and enable the effective management and operation of a quality service
	7.1.3	Roles and responsibilities - Roles and responsibilities are clearly defined, and understood, and support effective decision-making and operation of the service.
	7.2.3	Development of professionals - Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.

National Law

Section	165	Offence to inadequately supervise children
	167	Offence relating to protection of children from harm and hazards

National Regulations

Regs	97	Emergency and evacuation procedures
	98	Telephone or other communication equipment
	168(2)(e)	Policies and procedures in relation to emergency and evacuation including the matters set out in regulation 97
	170	Policies and procedures to be followed
	171	Policies and procedures to be kept available
	172	Notification of change to policies or procedures

EYLF

LO3	<i>Children become strong in their social and emotional wellbeing.</i>
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Sources

**Australian Standard 3745-2010 Planning for Emergencies in Facilities
Education and Care Services National Regulations**

***National Quality Standard
Work Health and Safety Act 2011
Work Health and Safety Regulations 2011
Fact Sheet Emergency Plans – Safe Work Australia
Guide to Developing an Emergency Management Plan for Early Childhood Services, Victorian Government
ACECQA - Emergency and Evacuation Policy Guidelines
ACECQA – Multi-Storey Buildings: Evacuations and Approvals***

21 - SEVERE WEATHER POLICY

Position Statement

Australia is known for severe storms and cyclonic conditions throughout the year. Preparation for a severe storm will lessen the impact of the storm. Suitable emergency equipment needs to be available at the Service.

Objective

To maintain the safety of children, team members and visitors during extreme weather conditions.

Related Policies:

- 42 - SUN PROTECTION AND SAFETY POLICY
- 88 – CHILD SAFE ENVIRONMENTS POLICY

Implementation

Orientation will include information about Workplace Health & Safety responsibilities and emergency procedures.

When a storm is approaching:

- Keep the radio on and listen for warnings.
- Turn off all unnecessary electrical appliances and take the plug from the socket.
- Ensure that emergency torches are working and spare batteries available.
- Use phones only for emergency calls in accordance with National Model Code Policy.
- Check the outside of the building and secure any loose items.
- Close windows throughout the centre.

When a cyclone is approaching:

- Listen to warnings and prepare.
- Wherever possible emergency care requirements should be in place well before there is imminent danger to the centre. The Approved Provider/ Centre Management should be notified when any severe weather warnings are in place. Management will instruct when non-essential care should be cancelled, and parents asked to collect children (when this is safe to do so).
- Follow directions for **a storm**, PLUS:
- Contact families to collect children before the cyclone comes if possible.
- Send visitors and some team members home if possible, keeping correct child/team member ratios.
- Tape all windows and doors with masking tape in a large cross, diagonally across windows.
- Tape the main doors with masking tape and place mattresses against the doors.
- Stay inside, away from windows and shelter in the safest, strongest part of the centre.
- Take first aid kit to the safe area.
- Place mattresses on the floor in the safe area to sit on.
- Take pillows, toys, games, nappy changing requirements, food, water, books, and radio with batteries.
- Ensure all electricity is turned off at main board.

When a storm arrives:

- Stay calm. Be responsible for your actions.
- Cover windows with the curtains.

- If windows are damaged, shelter in the corridors.
- If power fails, use emergency torches, keep fridges closed to maintain temperature.
- If the building is damaged, move children, visitors and team members to the safest area and administer first aid if necessary.

When a cyclone arrives:

- Follow directions for a *storm*, PLUS:
- Leave windows open on the opposite side of the building to the oncoming wind and rain. Be aware that in the eye of a cyclone, the wind direction can change, and you may need to change the open and closed windows.

After a storm or cyclone has passed:

- Administer any first aid required.
- Team members are to check rooms and surroundings before children are allowed out of the safe area.
- Move children into a safe play area if one is available while other team members check the playground and the exterior of building.
- Senior team members are to assess the situation on whether to remain inside or find a safer area outside the building.
- Notify management for assistance and advice. Liaise with the State Emergency Service (SES) for assistance (if required).
- If possible telephone parents to let them know the situation of the children.
- Once all children have left the building, team members will pack away safe areas and ensure all debris is taken care of.

Bush Fire Planning and Preparations

As per the Royal Bushfire Commission Report, the service will prepare an Action Plan should a bushfire effect the service's operations.

The aim of this Emergency Management Plan is to outline the activities that educators, children, families and visitors to be undertaken in the following circumstances:

- On days of Total Fire Ban
- When there is a fire in the local district.
- When a bushfire is threatening or impacting the site.
- During a period of recovery if a bushfire impacts the service.

During peak bush fire seasons, such as Spring and Summer, educators will monitor the Fire Danger Rating. Should the Rating be above High, educators will monitor the situation in line with the process in our region, such as via the internet or radio, to keep aware of the situation.

Trees will be trimmed to a distance of 2m from the buildings in risk areas. Educators will inspect the ground at the beginning of Spring and the owner/Nominated Supervisor will arrange for any lopping of branches if necessary. In line with this, gutters and roofs will regularly be clean and kept free of leaves. Also, time and paintwork on buildings will be kept well maintained. Please see Maintenance of Building and Equipment Policy for further details.

An Emergency Kit will be organised and stored somewhere that is easily accessible. Recommended kit items will include:

- A copy of the Emergency Management Plan
- Emergency Contact Details for each child.
- Child attendance registers.
- Emergency telephone numbers.
- Working torch and spare batteries
- First Aid Kit
- Educators/Children Medications and Medical Register
- Mobile Phone and Charger (Or staff mobile will be accessed in accordance with National Mode Code Policy)

- Drinking Water
- Nappies
- Blankets
- Towels
- Change Mat
- Gloves
- Nappy Wipes

Very High, Severe or Extreme Fire Danger Ratings

On days where the Fire Danger Rating is Very High, Severe or Extreme, the service will inform families by posting a warning in the entrance/noticeboard.

Children will be transitioned throughout the day as per our usual practice.

Any educators who are planned to attend off-site training will stay at the service, and have their training cancelled/rescheduled.

The OHS/WHs Officer or Fire Warden will ensure that all gates have access keys and ensure the locks are in working order.

All educators will monitor conditions when on duty outside. Educators will also ensure that no art and craft works, posters etc are hung outside and that garbage bins are emptied throughout the day.

Family members will be required to provide a reliable contact number for the day and families are required to provide the service with their child's asthma medication for the day.

The Nominated Supervisor/nominated educators will ensure that all outdoor taps are in working order with hoses attached and buckets are available to use.

The Nominated Supervisor/nominated educators will ensure that the outdoor industrial dumpster is closed at all times.

One nominated educators will ensure access to a mobile phone and is in full working order with a fully charged battery, and that the charger is available. The Emergency Contact register and Daily Roll will be added to the Kit. The medical register for the day will also be added to the kit.

The Fire Warden will ensure all hazards are removed from passages and walkways and nothing is blocking emergency exits.

Fire Reported in Local Area

The Nominated Supervisor will inform families and educators via a notice posted on the front door/foyer/noticeboard.

All nominated educators will be called together to discuss the situation and perform their duties.

The children will continue to be transitioned through their day with our usual transition method.

Any educators who are planned to attend off-site training will stay at the service, and have their training cancelled/rescheduled.

The OHS/WHs Officer or Fire Warden will ensure that all gates have access keys and ensure the locks are in working order.

All children's activities outside the building will be cancelled.

Educators, families and visitors to the service will be encouraged to reverse park their cars.
Policy Handbook – P004. 7.2026

Families will be required to provide a reliable contact number for the day and provide asthma medication if their child suffers from asthma.

The Nominated Supervisor/nominated educators will ensure that all outdoor taps are in working order with hoses attached and buckets are available to use.

The Nominated Supervisor/nominated educators will ensure that the outdoor industrial dumpster is closed at all times.

One nominated educator will ensure access to a mobile phone and is in full working order with a fully charged battery, and that the charger is available. The Emergency Contact register and Daily Roll will be added to the Kit. The medical register for the day will also be added to the kit.

The Fire Warden will ensure all hazards are removed from passages and walkways and nothing is blocking emergency exits.

The Nominated Supervisor will turn off any gas cylinders

Nominated educators will back up all computer files.

The Fire Warden will ensure that whistles are in place beside each portable fire extinguisher. These are to be used to notify everyone if a fire starts on site.

Educators will be diligent in ensuring children's personal items are placed in their bags when not in use.

Fire Reported in Immediate Vicinity or Directly Impacting the Service

The procedure above will be followed immediately.

Educators will move all hoses inside building

Educators will close all doors and windows

Educators will access the roof space every 10-20 minutes to check for spot fires.

The Nominated Supervisor will inform the owner/Approved Provider of the situation and regularly keep them updated of the situation.

The Approved Provider/Nominated Supervisor will inform the appropriate service about the situation, advising the Licensing officer of the number of children affected, the educators ratios in place and any issues or injuries that have arise.

Recovery after the Front has passed.

The Nominated Supervisor will ensure that no educators, family member, child or visitor associated with the service leaves the building until the situation is safe by members of the emergency services.

The Service Emergency Response Team (made up of members of educators) will assess the situation and if necessary make arrangements for the care of children for an extended period of time.

Educators at the service will stay on duty until all children have been collected or relief educators arrive.

Only a qualified educator will administer first aid should the situation arise.

The Nominated Supervisor will at all times work to keep the Approved Provider abreast of the situation. The Approved Provider will then advise the Licensing Unit/Officer of the number of children affected, emergency educators ratios in place, any issues that arose and if possible the projected impact to the service.

The Emergency Response Team of nominated educators will continue to check the building and surrounds for 2-4 hours after the front has passed.

Relevant educators will undertake a debrief of the fire emergency situation and the procedures undertaken. Educators will be requested to review their own roles, responsibilities and preparation before and during the crisis. The policy will be reviewed to ascertain its effectiveness.

Nominated educators will arrange to have fire fighting equipment, warning system and Emergency Kits checked and readied for use again.

If necessary, the owner/Nominated Supervisor will arrange for relevant authorities to check the safety of the site.

Fees and Charges

Attendance fees for the day will still be charged. Should family member be unable to collect, or arrange collection of their children within one hour of the usual operation hours of the service, fees will apply as per our Arrival and Departure Policy.

CCS payments are available for up to 42 days in this circumstance. For further information on this please read:

<http://www.deewr.gov.au/Earlychildhood/Resources/Documents/FactSheets/CCFactSheet.pdf>

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 2/04/2026

Reference:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	168(2)(e)	<i>Policies and procedures in relation to emergency and evacuation</i>
Reg	97	<i>Emergency and evacuation procedures</i>
Reg	98	<i>Telephone or other communication equipment</i>

EYLF

LO3	<i>Children become strong in their social and emotional wellbeing.</i>
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Other References:

Handbook on Child Care Licensing

QIAS Principle 5.1, 5.2, 5.3, 5.4

Standards Australia

Public Health and Wellbeing Act 2008

Workplace Health and Safety Regulation 1997

The Bushfire Royal Commission Report

23 - TOXIC & POTENTIALLY DANGEROUS PRODUCTS POLICY

Position Statement

Toxic and potentially dangerous products may be stored on premises for cleaning and/or the on-going maintenance of the centre. All toxic and potentially dangerous products will be stored in the locked chemical cupboard and clearly marked in their original packaging. Any team members handling potentially dangerous products must be informed of and adhere to all safety precautions.

Objective

To minimise the use of toxic products and other potentially dangerous products without jeopardising hygiene.

Implementation

- Bulk chemicals will be stored in the chemical cupboard, which is locked at all times.
- All cupboards will be marked with a warning "Dangerous Substances Stored here", and if possible, a symbol for "Danger" eg. ☠.
- Material Safety Data Sheets (MSDS) must be supplied with all chemicals used within the centre.
- If a chemical product used within the centre is purchased from a supermarket or other supplier, the manufacturer must be contacted and the MSDS obtained.
- Regular team member training will be conducted in relation to universal precautions and the importance of handling potentially toxic and dangerous products.
- All new team members will work through a thorough induction process in regard to toxic and potentially dangerous products.
- A plan of action will be displayed in the cleaner's cupboard for emergency situations.
- **Poison information number: phone 131 126, anywhere in Australia** will be displayed at front reception.
- Cleaning products including spray bottles will **NOT** be left where they can be accessed by the children.
- When using spray bottles; chemicals will be sprayed onto cleaning cloth and **NOT** directly onto surfaces. This reduce airborne chemicals from spray bottle.
- All chemicals and dangerous products are to be stored in a secured cupboard or on a nominated shelf inaccessible to children with a label displayed "Chemical and dangerous products stored here" (including those used in playrooms, kitchens and bathroom). The chemicals stored on shelf should be limited and only for spray bottles needed for frequent cleaning requirements.
- Children's activities are not to include the use of any substance that may contain a hazardous material. Expressly there will be no use of Polystyrene foam in the centre for art and craft activities.
- Pest Control, carpet cleaning and any other services requiring the use of chemicals **MUST** be conducted outside of centre operating times when NO children are present on premises.
- Should team members suspect that children have accessed any hazardous material they are to follow first aid procedures and take with them a copy of the MSDS to show health authorities.

Responsibilities of Centre Director/Authorised Supervisor and all team members

The Centre Director/Authorised Supervisor and all team members must ensure that the following items are kept in secure storage facilities which are inaccessible to children;

- Poisonous and dangerous cleaning materials and/or substances
- Disinfectants
- Dangerous tools and equipment
- Toiletries and medications
- First Aid equipment

- Team member handbags and personal items
- Sharp or jagged objects which pose a risk to children

Use of Hazardous Chemicals or Products

When using hazardous chemicals or products all team members must;

- Read the MSDS and follow the instructions accordingly
- Ensure all chemicals which are stored or decanted for use are labelled with name, description of content, strength of chemical, hazards, precautions and directions for use
- Wear approved personal attire such as apron and goggles where specified when handling dangerous chemicals
- Spray bottles are to be placed on the coarsest spray possible so as to prevent inhalation of fine mist into the lungs. Spray onto cloth and not directly onto surface.
- Never “top up” chemical bottles/soap dispensers, but let them empty over time, clean and refill with a fresh solution
- All chemicals and dangerous products are to be stored in a secured cupboard or on a nominated shelf inaccessible to children with a label displayed “Chemical and dangerous products stored here” (including those used in playrooms, kitchens and bathroom).
- Chemicals are never to be stored in inappropriate holding containers, such as soft drink bottles, empty formula cans or ice cream containers.
- Any substance (medication) referred to in the above list that requires refrigeration is to be kept in a child resistant container in the refrigerator.

FIRST AID ACTION PLAN FOR DANGEROUS PRODUCTS

In the event of an emergency, always remove (yourself or the product) to manage the danger first.

It is important to always protect yourself and others when dealing with an emergency.

If an emergency occurs that involves a dangerous product please follow the following steps:

- 1. Remove danger. Ensure all others (staff, children) are safe. Remember that chemicals that have contact with skin can cause burns.**
- 2. Contact “Poisons Hotline” on 131126 and follow advice given.**
- 3. Have MSDS sheets on hand to assist giving information as needed. If these are not available refer to manufacturers directions.**
- 4. Notify Management and relevant family members.**
- 5. Administer First Aid as directed and call an Ambulance if required.**
- 6. Follow all the required steps within the Accident & Emergency Policy, including the completion of all paperwork.**

The Centre Director/Authorised Supervisor will ensure centre procedures are evaluated and changes are made if necessary to ensure the safety of children at all times.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/12/2025

Reference:

NQS

QA2	2.2.2	Effective illness and injury management and hygiene practices are promoted and implemented.
QA3	3.1.1	Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.
	3.1.2	Premises, furniture and equipment are safe, clean and well maintained

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	103	Premises, furniture and equipment to be safe, clean and in good repair
	104	Fencing and security
	105	Furniture, materials and equipment
	113	Outdoor space—natural environment
	114	Outdoor space—shade

EYLF

	Children become socially responsible and show respect for the environment
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Other References:

Handbook on child Care Licensing

QIAS Principle 5.3

Standards Australia

Food Safety Legislation 2001

Public Health and Wellbeing Act 2008

Occupational Health and Safety Act 2004

Prior Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Sources and Further Reading:

- Managing Health in Childcare (2001)
- Workplace Health and Safety Regulations (1997), Hazardous Substances. <http://hsis.ascc.gov.au/> Retrieved 1 October 2008
- Storage and Handling of Workplace Dangerous Goods (2001), National Standards.
- NSW Fire Brigades (2007), Household Chemical Safety Fact Sheet.

24 - TOXIC PLANT POLICY

Position Statement

It is important to ensure that children and the families are safe at all times.

Objective

To ensure safety, toxic plants will not be planted, or brought onto the Child Care premises.

Implementation

- Provide parents with information about Toxic plants and poisoning. (This information will be available in the centre reception area).
- Provide parents with contact details for information on poisoning and local authorities in the area. Poisons Information number: phone 131126, anywhere in Australia will be displayed at front reception.
- Team members are to conduct daily indoor and outdoor checks to ensure that the environment is safe (*OPRCK008 Opening and Closing Checklist*).
- Should team members suspect that children have accessed poisonous plants they are to follow first aid procedures and notify the poisons information centre immediately and follow all First Aid and Incident procedures.
- Playground Upgrades: When planning playground upgrades team members and landscapers are to be mindful of the toxicity of all plants utilized within the children's play space.

The following fact sheet from Kids Safe (SA, September 2010) provides a list of poisonous plants.

Poisonous plants

These plants are considered harmful to children and should be AVOIDED if you have young children

Plant	Toxic part	Symptoms
African Milk Bush <i>Synadenium Grantii</i>	All parts	Irritation of the lining of the mouth (mucous surfaces), diarrhoea, vomiting.
Agapanthus <i>Agapanthus Orientalis</i>	Leaves	Acute skin irritation and ulceration of the mouth.
Angels Trumpet <i>Datura Aborea</i> (<i>D. Candida</i>)	Fruit, seeds and flowers	Dry throat, increasing thirst. Disturbance of vision, giddiness, flushed face, headache, restlessness and staggering, dilation of pupils, fast heart rate (tachycardia), coma and death.
Arum Lily <i>Santideschia Aethiopica</i>	All parts	Swelling of tongue and throat. Causes severe illness and acute gastric upset.
Autumn Crocus <i>Colchicum Autumnale</i>	All parts	Gastro-intestinal upset, respiratory paralysis, renal damage, exhaustion and shock.
Azalea or Rhododendron	All parts	Nausea, vomiting, depression, difficulty in breathing, coma.
Bird of Paradise <i>Poinciana Gilliesii</i>	Pods and seeds	Vomiting, diarrhoea, drowsiness and vertigo.
Black Locust <i>Robinia Psuedoacacia</i>	All parts	Gastro-intestinal upset, coldness of extremities, heart irregularities, pupil dilation.
Box <i>Buxus Sempervirens</i>	Leaves and twigs	Gastroenteritis, bloody diarrhoea, respiratory failure and convulsions.
Cape Lilac <i>Melia Azedarach</i>	Fruit	Gastro-intestinal distress, bloody stools, excitement or depression. 6-8 fruit can be fatal to a child.
Castor-oil Plant <i>Ricinus Communis</i>	Seeds	Restlessness, cold clammy skin, thirst, rapid pulse, cramps, drowsiness, stuporous, collapse. 2-8 seeds are lethal dose for adult. Can cause a severe allergic reaction.
Cotoneaster	Fruit (red berries)	Intense nausea and vomiting.

These plants are considered harmful to children and should be AVOIDED if you have young children

Plant	Toxic part	Symptoms
Daffodil and Jonquil <i>Narcissus Pseudonarcissus</i> and <i>Narcissus jonquilla</i>	Bulbs	Nausea, vomiting, diarrhoea, trembling and convulsions.
Dumb-Cane <i>Dieffenbachia spp.</i>	All parts	Intense burning and irritation of mouth and tongue. Swelling and accumulation of fluid causes difficulty in swallowing.
Duranta <i>Idumata Repens</i>	Fruit	Sleepiness, high temperature.
Foxglove <i>Digitalis Purpurea</i>	All parts	Irregular heart beat and pulse. Usually digestive upset and dizziness.
Hemlock <i>Conium Maculatum</i>	All parts	Violent, painful convulsions and a gradual loss of muscle power.
Jessamine <i>Cestrum spp.</i>	All parts	Nausea, pupil dilation, muscular weakness, hallucinations, tachycardia, weak pulse, coma.
Laburnum <i>Laburnum</i> <i>Anagyroides</i>	All parts, especially seeds	Vomiting, purging, contraction of the muscles (tetanic spasms).
Lantana <i>Lantana Camara</i>	Green berries	Not fully documented, but includes muscular weakness and circulatory collapse.
Naked Lady <i>Euphorbia Tirucalli</i>	All parts	Vomiting, purging, delirium, death. Latex sap is an irritant, causes temporary blindness on contact with eyes. Blistering to the skin.
Nightshades	All parts, especially berries	Intense digestive disturbance, affects nervous system.
Oleander <i>Nerium oleander</i> and <i>Thevetia peruviana</i>	All parts	Extremely poisonous! Affects the heart and pulse. Usually causes digestive upset and mental confusion. Ingestion of one leaf or flower can be fatal to a child.
Poinsettia <i>Euphorbia Pulcherrima</i>	Leaves, stalk, sap	Severe nausea if swallowed. Extreme irritation to skin and eyes from latex sap.

These plants are considered harmful to children and should be AVOIDED if you have young children

Plant	Toxic part	Symptoms
Rhubarb <i>Rheum Rhabonticum</i>	Leaf blade	Staggering, abdominal pain, vomiting, diarrhoea, impaired clotting of blood, coma.
Rhus <i>Toxicodendron</i> <i>Succedaneum</i>	All parts	Severe irritation and blistering of the skin. Intense swelling of the face that may spread to other parts.
Toadstool	All parts	Intoxication (in a few minutes to two hours), unusual secretion of saliva, sweating, diarrhoea, vomiting and circulatory failure. Mental disturbance, convulsions and coma.
Thorn Apple <i>Datura Stramonium</i> and <i>Jimson Weed</i>	All parts	Abnormal thirst, distorted sight, delirium, incoherence, coma.
Winter Sweet <i>Carissa Spectabilis</i>	All parts	Severe gastro-intestinal irritation, irregular heart beat and pulse.

This information has been adapted from the Home-Safe Play-Safe Healthy Cities Wanneroo project.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/12/2025

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>

	105	<i>Furniture, materials and equipment</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>

EYLF

	<i>Children become socially responsible and show respect for the environment</i>
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Other References:

Handbook on Child care Licensing
Public Health and Wellbeing Act 2008
Staying Healthy 5th Edition
QIAS 5.1, 5.2, 5.3, 5.5

Legislation

Child Care Act 2002 and Regulation (2003) Qld
Children's Services Act (1985) & the Children's
Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Regulations (2004) and
Children and Young Persons (Care and Protection)
Act 1998 (NSW)
Children's Services Act 1996 (Vic) & Children's
Services Regulations 2009(Vic)
Occupational Health & Safety Act 2004

Sources and Further Reading:

- Kidsafe Qld Inc (2006) **Poisonous Plants Fact Sheet** <http://www.kidsafeqld.com.au/>
- Health Insite – An Australian Government Initiative
http://www.healthinsite.gov.au/topics/Poisonous_Plants (29 September 2008)
- Best Practice Guidelines in Early Childhood Physical Environments, 1996, Walsh & DOCS. Plants for Play Spaces

25 - VALUABLES POLICY

Position Statement

Team members and children need to feel secure about bringing belongings necessary for their day to the child care centre.

Objective

To manage the security and safety of the personal belongings of families and team members attending the centre.

Procedure

- Every item being brought to childcare will be clearly labelled with the child's name.
- Parents are encouraged to supply inexpensive play clothes and shoes for children attending the centre.
- Wherever possible valuable items will be kept at home as this is the only way their safety can be guaranteed.
- If it should become necessary for a child to bring a valuable item to childcare, then the parent will show the item to the team member in charge upon arrival and explain why it is necessary for the item to be in the centre. The team member and/or the Centre Director will decide if it is appropriate for the item to be in the centre, without accepting any responsibility for the item.
- Management takes no responsibility for any child's or employee's personal belongings that may be lost, stolen or broken while in the Centre.
- Team members will do their utmost to ensure the safe return of any valuable and necessary item that has been left in the centre, however, management and the team members of our centre cannot accept responsibility for valuables brought to child care.
- We advise team members not to bring valuables and money to the service and if do so do at their own risk. Team members are responsible for their own personal belongings. Mobile phones are required to be turned off or kept on silent while the team member is working. Mobile phones are not to be used or kept on oneself while working.
- Parents, team members and visitors are encouraged to lock their car before entering the Centre. Management takes no responsibility for valuables left in cars.
- Parents who may be staying in the Centre as parent helpers etc are advised not to leave or bring valuables to the centre. Management takes no responsibility for any parent valuables lost while in the Centre.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

References:

NQS

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.

	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

26 - HIGH CHAIR POLICY

Position Statement

High chairs when not used correctly can be dangerous for young children. Mealtimes should provide an enjoyable and safe opportunity to eat in a relaxed small group environment.

Objective

To provide guidelines for team members to ensure the safety and comfort of children whilst enjoying meals in a small group environment.

Implementation

- All highchairs will meet the Australian standards and will be easy to clean, preferably with a removable tray. The back and seat areas will provide a comfortable seating position for children.
- Highchair covers are replaced as required.
- The chair will be stable and if on wheels, these will be locked.
- The room will be set up prior to meal times to provide a pleasant eating environment ensuring that chairs are at a safe distance from walls and that areas in direct sunlight are avoided.
- Highchairs will have a 5 way harness to ensure the safety of the child. All harnesses will be fastened correctly as soon as a child is placed in the chair.
- The meal should be unhurried, however the child will not be left in the chair longer than necessary.
- To benefit the wellbeing of children and educators, high chairs can be used in the outdoor environment for morning tea, lunch and afternoon tea to occur.
- Chairs will be washed after each use and at the end of the day.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

Reference: NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
	2.1.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>

National Act and Regulations

Policy Handbook – P004. 7.2026

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	105	<i>Furniture, materials and equipment</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 5.1, 5.2, 5.4

Child Accident Prevention Foundation of Aust

Kidsafe Qld (Inc)

Standards Australia

27 - HEALTH AND HYGIENE POLICY

Position Statement

Hygiene practices are essential for the effective management of children's health. Young children's immune systems are not fully developed. A group setting environment potentially places additional strain on contagious conditions.

Objective

To provide a framework that ensures health & hygiene in the Centre is appropriately managed by team members.

Key Related Policies: please read this policy in conjunction with other related policies.

- 31 - Illness Policy

Implementation

- All team members are expected to follow hygiene procedures as detailed in the procedural section of this policy.
- Team members are encouraged not to come to work if they are unwell, as their presence may contribute to the spread of infections to other team members and children.
- Children who are not deemed to be well enough to participate in the daily program will be required to stay away from the Centre until well. Centre team members are required to access *Staying Healthy: Preventing infectious diseases in early childhood education and care services* (6th edition) and refer to the section titled 'Responsibilities of Childcare Providers/Team members', to ascertain whether the illness needs to be reported to the state Health authorities. In such an instance the Centre will not receive children back into care without a Doctor's clearance (see Illness Policy for more information). All illnesses registered with the state health authorities will be dealt with according to the guidelines set out by state health departments. This process will be instigated and monitored by the Centre Director and management. All information should be recorded in the Infectious Disease Register (OWNA). In the event of an outbreak of an infectious disease the centre will post a copy of the relevant fact sheet from *Staying Healthy in childcare* at the front of the centre informing families that a reported case has been identified at the centre. There should be no other identifying details regarding the person afflicted displayed or provided to parents in order to ensure confidentiality.
- Parents will be called to collect children who have arrived at the Centre and become unwell during the course of the day if the child is not well enough to participate in the normal program and activities planned for the day.
- Expectations regarding the collection of sick children from the centre and when children should not attend the centre due to illness are also included in the Parent Handbook.
- Team members are expected to foster and support children to gain an understanding of basic hygiene practices in an age appropriate manner.

Procedure

- All team members will follow strict hygiene routines while in the centre.
- Hand washing with soap will be carried out by team members and children:-
 - ✓ Upon arrival and departure from the Centre
 - ✓ Before preparing, serving, or eating food
 - ✓ Before preparing a baby bottle and before feeding an infant a bottle
 - ✓ After each nappy change or toilet assistance
 - ✓ After wiping anyone's nose
 - ✓ After toileting themselves
 - ✓ After handling animals

Team members will disinfect nappy change area after each use.

- Team members will use disposable gloves for **all** nappy changes. Team members who have an identified latex allergy will be offered alternative disposable gloves.
- Team members will ensure that toileting and nappy change procedures are a positive experience for all children eg. Smiling, repeating rhymes, positive encouragement, etc.
- Team members will wear disposable gloves for handling of **ALL** potentially infectious or contaminated substances.
- Team members will ensure that children do not eat food which has been handled by another child.
- Team members will ensure that food that has been dropped on the floor is not eaten by anyone.
- Team members will ensure that eating and drinking utensils are not shared by children.
- Team members will use separate cloths or tissues to wipe each child's face and nose.
- Team members will dispose of tissues immediately after use.
- Team members will ensure that bedding is never shared by children and is washed at least once a week and immediately after it has been soiled
- Team members will actively promote children's washing of hands
- Team members will encourage children to flush the toilet after use.
- Team members will approach **ALL** situations involving bodily fluids employing universal precautions. i.e. handle **ALL** bodily fluids with the assumption that they are contaminated and contagious.
- Team members will plan and promote children's understanding of basic personal hygiene practices including dental hygiene through role modelling and active weekly planning.
- Play equipment and resources will be used hygienically and stored in a clean and hygienic manner.
- Team members will regularly disinfect hard surface equipment and wash soft coverings to ensure that bacterial loads on equipment and resources is minimised.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child.</i>
QA2	2.2	<i>Each child is protected.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	77	Health, hygiene and safe food practices
	78	Food and beverages
	79	Service providing food and beverages
	80	Weekly menu

EYLF

LO3	Actively support children to learn hygiene practices.
	Promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community.
	Discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.
	Model and reinforce health, nutrition and personal hygiene practices with children.

Other References:

Handbook on Child Care Licensing
QIAS Principle 6.2, 6.3, 6.4, 6.6
Standards Australia

CCB – Federal Funding Guidelines

Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)
Occupational Health & Safety Act 2004
Australian and New Zealand Food Standards Code
Public Health and Wellbeing Act 2008

28 – NUTRITION, FOOD, BEVERAGE, DIETARY AND ORAL HEALTH REQUIREMENTS POLICY

Position Statement

Healthy eating and good nutrition have a major influence on children's health and wellbeing and a direct impact on their growth and development. Our centres are committed to providing high quality care for the children and the families that access our centres. Having good nutrition at a young age is essential for healthy development of young children, as most children have formed lifelong habits by school age. Some children have severe allergies to a variety of substances including foods and many of these allergies can be so severe as to be life threatening. We are sensitive to children's medical needs, cultural and/or religious requirements and each individual situation will be discussed with the family and a solution will be implemented.

Objective

This policy confirms our commitment to:

- Promote nutritious and healthy eating, good drinking habits and oral health through providing a healthy menu, discussion, education and role modelling.
- Provide a climate for eating and oral health that encourages independence.
- Encourage children to be aware of the different food and beverage varieties and food and beverage requirements of various cultures.
- Provide opportunities for children to develop healthy eating, drinking and oral health habits through role modelling, positive food and eating experiences, meaningful communication and a happy environment.
- Promote food storage and handling procedures that reduce the cause of illness and meet state regulations for providing registered food premises where necessary.

Related Policies

35 - Anaphylaxis Policy
92 - Health and Wellbeing Policy
36 - Asthma Policy
85 - Diabetes Policy
88 - Child Safe Environment Policy 94 - Cystic Fibrosis Policy
30 - Dental Care Policy
91 - Epilepsy Policy
29 - Food Safety & Hygiene
27 - Health & Hygiene Policy
32 - Infectious Diseases and Immunisation Policy
87 - Medical Conditions Policy
33 - Medication Policy
93 - Physical Activity Promotion Policy
61 - Fundraising Policy
40 - Rest Time/Sleep Policy
31 - Illness Policy
41 - Smoke, Alcohol & Drug Free Environment Policy
42 - Sun Protection and Safety Policy

Implementation:

Nutrition in centres that provide meals:

The Centre will design a menu in accordance with best practice guidelines for meeting the nutritional needs and dietary requirements of young children. We will provide children with a variety of tastes, textures, and flavours to encourage their appreciation of food and allow them to experience the diversity of food and beverages.

Families and educators are provided access to resources from "Get Up and Grow. Healthy Eating and Physical Activity for Early Childhood" and the Australian Dietary Guidelines during orientation as guidance on healthy eating and physical activity.

Strategies:

- The service menu promotes the consumption of fruit and vegetables on a daily basis and healthy food options in line with Australian Guidelines (Get Up and Grow. Healthy Eating and Physical Activity for Early Childhood. Australian Government, 2009, Australian Dietary Guidelines and Infant Feeding Guidelines. National Health and Medical Research Council, 2013 <http://www.eatforhealth.gov.au>).
- The service menu is assessed by the Healthy Eating Advisory Service via Foodchecker and meets the Menu planning guidelines for long day care.
- Menus will be planned to meet the recommended minimum food and beverage requirements and will be reviewed each season to include seasonally appropriate food and ensure variety.
- Milk will be served with meals at least once per day. Calcium sources will be obtained from a variety of foods.
- Only calcium fortified soy and rice beverages will be used as a substitute for milk.
- Children will have access to breads/cereals or fruit/vegetable snacks if they are hungry between meals.
- Tap water will be the primary drink and will be available at all times.
- Parents of children on special diets will be asked to provide details of any special food needs compiled by a medically qualified doctor.
- As children develop, the texture of the food provided will change to meet their stage of development.
- It is recommended to review food size and textures. For example: Grapes; slice in half to reduce the risk of choking.
- Sometimes foods and sweetened drinks (juices, cordial and soft drinks) are not provided by the service and are discouraged from being brought from home.
- The service seeks to ensure any sponsorship, advertisements or marketing of food and drinks are consistent with the service's healthy eating policy. Fundraising activities reflect the healthy eating policy and promote healthy lifestyle messages and discourages discretionary foods.
- Diversity and culture are considered when menu planning and when implementing healthy eating and oral health practices.

In centres that do not provide meals:

- Clearly marked baskets will be provided for parents to unpack food ready to be stored in the fridge area.
- Food is to be placed into the fridges provided as soon as the lunch box arrives to ensure that food is safely handled.
- Food storage practices are to be well signed so parents understand the expectations for food to be placed in fridge baskets.
- Should team members recognise that food and beverages brought from home has not been refrigerated in accordance with food safety requirements; the child is not to be given the food to eat. Alternative food is to be offered to children in this circumstance. The parent will be notified of the incident and the requirement for appropriate food storage.

The following policy guidelines will apply to all centres:

Food and beverage allergies:

This centre adopts policies and procedures with a view to ensuring that we are a nut-free zone. Whilst we are unable to guarantee this, all reasonable and possible steps are taken to seek to comply with this mandate. If your child has food allergies, then please notify your Centre Director of their specific requirements. Additional information on Food Allergies can be sourced from Anaphylaxis Australia.

Environment and Culture:

Mealtime atmosphere as well as the attitude and behaviour of team members are important to the development of healthy eating practices. The service aims to provide an eating environment that promotes family and multicultural values.

Strategies:

- Each Centre will provide children with flexible eating times which are reflective of the individual developmental and nutritional needs of the children.
- Presentation of each meal to the children is important to support positive healthy eating habits.
- Team members will sit with children and share the same food as often as possible.
- Mealtimes will be pleasant routine times where children use appropriate utensils that will encourage the development of eating skills and independence in eating.
- Appropriate seating arrangements that will encourage children's self-help skills and conversation will be used.
- Food will never be used as a form of punishment either by its provision or denial.
- Special occasions, fundraisers and parent events will be celebrated with nutritional culturally appropriate foods and beverages e.g. Birthdays, cultural and religious days.
- Recipes and food awareness activities will be chosen from a variety of cultures.
- Parents will be advised in advance of these days and given the opportunity to withdraw their child as they deem necessary.

Hygiene:

Careful preparation, storage and reheating of food and drinks and correct food handling techniques are necessary to ensure food safety and the possibility of contamination.

Strategies:

- Food and beverages will be stored in an appropriate manner and all necessary documentation will be in place to validate correct food storage.
- In states where there is a requirement for a food safety supervisor the appropriate team members will receive training and ensure that all requirements associated with this responsibility are met.
- In states where it is necessary, all relevant team members involved in the preparation and delivery of food to children will hold a food handlers' certificate. In states where this is not a regulatory requirement, team members will receive in-house training from team members with relevant qualifications to ensure that best practice is understood for the handling of food.
- Team members will wash hands thoroughly before handling food (team members should gather all things necessary for food preparation before they begin). Touching the fridge or cupboards or other things in the kitchen in the middle of food handling will require them to re-wash their hands before recommencing food preparation.
- Children need to wash their hands before eating or preparing food; team members are expected to role model good hand washing techniques.
- Team members will discourage children from handling other children's food and utensils.
- Team members will encourage the correct sharing of food at tables (e.g. when fruit platters are shared).
- Waste bins need to be emptied soon after lunchtime or fruit time session has finished to minimise the risk of contamination.

Safety:

It is important that team members remain vigilant to children's safety when sharing and/or supervising mealtimes.

Strategies:

- Children will be quietly seated and will be supervised when eating. (Please also refer to the Highchair Policy).
- Food will be served in appropriate portion sizes to allow children to chew and swallow without fear of choking.
- Food should be served to children at a safe temperature to eat. I.e. below 65 degrees Celsius.
- Children will not be force-fed.
- Children will not be given food or drinks (including bottles) whilst they are on mattresses or cots.

Food Serving Temperatures (please refer to 29 – Food Safety and Hygiene Policy for detailed guidance)

- To make sure food is warm enough to enjoy yet not hot enough to be a danger of burning child's mouth or throat, use a food thermometer and make sure the food is within serving temperature range. Record the food serving temperature.
- It is recommended to serve **children (ages above 18 months) hot food below 60 degrees celsius** to reduce the risk burning their mouth and throat. Food can further cool before child eats.
- Babies/infants have more sensitive mouths and throats. It is recommended for **infants (ages below 18 months) to serve hot food below 45 degree celsius**. It is recommended to serve infant hot food between the temperature of 32(90 F) to 45(113F) degrees celsius to reduce risk of burning their mouth and throat (*Source: Safe Food for Babies and Children, NDSU, FN715 (Review December 2021)*).

Education:

The early learning centres play an important role in the educating children about food, beverage, nutrition and dietary requirements and oral health. 'Being healthy' lifestyle choices include promoting good food and beverage choices, an appreciation of a variety of food (cultural, textural, smell, taste, etc.), inclusion of an appropriate amount of exercise daily and good oral health habits and practices. Families will be offered information on a range of food, nutrition and oral health topics.

Strategies:

- Food, beverage and oral health awareness activities will be included in the program and children will be encouraged to be involved in "hands on" food preparation activities.
- Foods being served to children will be discussed with them.
- The service provides a positive eating environment with relaxed, social and enjoyable experiences by:
 - encouraging independence at meal and snack times
 - educators and staff sitting with the children at meal and snack times to role model healthy eating and for socialisation and learning.
 - educators and staff sharing food with the children, where appropriate
 - children being given plenty of time to eat and socialise.
- Food and drink are not used as an incentive, bribe or reward at any time.
- Educators and families recognise they are role models and are encouraged to bring foods and drinks in line with the service's healthy eating policy. Where possible, healthy- eating options are encouraged at events, professional development and meetings.
- Healthy body image and an enjoyment of eating are encouraged by the service.
- Children will be involved in activities that demonstrate physical growth e.g. Height charts and promote physical activity.

- Children will be involved in activities that demonstrate good oral health practices.
- Information about food, food safety, dietary requirements, nutrition and oral health will be provided to families and team members.
- Menus will be displayed at the service and recipes will be provided to families.
- Families will be encouraged to be a part of the centre's nutritional experience by providing recipes, sharing a meal with the children and providing multicultural experiences and values if they so desire.
- Communication with families about each child's daily intake and enjoyment of food will provide for long term healthy eating patterns.

Learning and skills

- Educators and staff involve children in healthy food experiences through growing, cooking and shopping.
- Opportunities to learn about food, healthy eating and oral health are embedded in the educational program.
- Educators and staff are supported to access a range of resources and professional development to increase their capacity to promote healthy eating and oral health initiatives for children.
- Healthy eating and oral health policy requirements are included in educator and staff orientations / inductions.

Engaging children, educators, staff and families

- Educators, staff, children and families are seen as key partners in promoting and supporting healthy eating and oral health initiatives in the service.
- Educators, staff and families are provided with information, ideas and practical strategies on a regular basis to support healthy eating and oral health in the service and at home.
- Families' experiences, expertise and interests are drawn upon to support healthy eating and oral health initiatives.
- Families and children from culturally diverse backgrounds are consulted to ensure cultural values and expectations about food and eating are respected.
- Educators are supported to consume healthy foods and drinks at work
- When food is provided to educators and staff for meetings, celebrations and events, healthy food options are included, and discretionary/sometimes foods options are discouraged.

Community partnerships

- The service works with local health professionals, services and other organisations to support educators and staff to deliver and promote healthy eating and oral health initiatives.

Formula, Milk, Breast Milk and Bottle Procedures

The following information needs to be displayed in the kitchen or room who serve bottles to children.

Milk Food Safety and Routines

- In consultation with parents and guardian Educators will follow each child's feeding routine. This information will be documented in OWNA App for each child and also the Child Wellbeing Chart.
- Families need to complete a 'Food from Home Form' via OWNA App when supplying breast milk or formula milk for their child.
- Educators will record the amount of milk each infant consumed including the time of feeds in the OWNA App (and if applicable Child Wellbeing Chart).

- Implement safe food handling practices. Including wash your hands thoroughly using the correct handwashing procedure. Ensure workbench is clean and sanitised. Store all bottles in an appropriate area for food preparation areas.
- Breast milk or formula in feeding bottles is not to be reheated. Families need to provide additional portions of formula or breast milk to cover the babies' feeding requirements.
- To confirm understanding of this policy the following example is provided. If formula or breast milk is made up and the child only drinks 40ml, this feeding bottle must be discarded. Therefore if the child needs another feeding bottle in 2 hours later, a fresh sterilised feeding bottle needs to be made.
- Healthy lifestyles and good nutrition for each child is paramount. As such, we encourage all families to continue breast feeding their child until at least 12 months in line with recommendations from recognised authorities.
- We aim to work with families with children who are still being breastfed and provide a supportive environment by feeding children breast milk supplied by their families. We will provide a supportive environment for mothers to breastfeed.
- Families will be regularly reminded by educators and the service to update the service in regard to their child's preferences, habits, likes, dislikes, dietary requirements and restrictions.

Formula Milk Preparation

- Do not compact the formula powder in the scoop, and always ensure the correct ratio of formula to water as specified on the tin: Too little formula powder may leave the infant hungry, whilst too much can cause constipation
1. Implement safe food handling practices.
 2. Wash your hands thoroughly using the correct handwashing procedure. Ensure workbench is clean and sanitised.
 3. Put on clean gloves when preparing bottles.
 4. **Families will provide bottles(labelled with child full name) with measure boiled water.** If additional bottle is needed to be made, boiled water that has cooled to lukewarm temperature will be measured accordingly.
 5. Heat the bottle in bottle warmer selecting desired volume in millilitres (mls), then remove it carefully.
 6. **Families will provide measured formula in singular container/dispenser with measure boiled water.** Add formula to heated water. Ensure the formula powder is added directly to the centre of the water.
 7. Screw the teats and cap onto the bottle, then shake and swirl to ensure the formula powder is completely mixed/dissolved and to remove any heat pockets formed during heating.
 8. Test the milk's temperature before giving to child, by placing a few drops on the inside of your wrist. It should feel lukewarm or body temperature (37 Degrees Celsius), but not too hot. If the bottle feels too hot, cool it quickly by placing it in a container of cold water for a few minutes before checking again.
 9. Always check the bottle's temperature before feeding the child.
 10. Document the amount of milk each infant consumed including the time of feeds in the OWNA App.

Milk (Cow's and Goat's) Preparation

1. Implement safe food handling practices.
2. Wash your hands thoroughly using the correct handwashing procedure. Ensure workbench is clean and sanitised.
3. Put on clean gloves when preparing bottles.
4. Families will need to provide ample clean and sterile bottles (labelled with child full name) as the service is unable to sanitise bottle at the service.
5. Measure the correct amount of cow's milk into the bottle per the individuals requirement.
6. Place the bottle with teat top on, in the bottle warmer and heat using set mls.

7. Remove the bottle from the bottle warmer and rinse teat under cool water as the silicone can get hot before then shaking bottle to ensure no hot spots.
8. Shake the bottle gently to remove any heat pockets that may have formed during microwaving.
9. Test the milk's temperature before giving to child, by placing a few drops on the inside of your wrist. It should feel lukewarm or body temperature (37 Degrees Celsius), but not too hot. If the bottle feels too hot, cool it quickly by placing it in a container of cold water for a few minutes before checking again.
10. Always check the bottle's temperature before feeding the child.
11. Document the amount of milk each infant consumed including the time of feeds in the OWNA App.

Breast Milk Procedure

- Breast milk that has been expressed should be brought to the service in a clean sterile container labelled with the date of expression and the child's name.
 - We encourage families to transport milk to the service in cooler bags and eskies; this should be immediately given to educators, who will put it in the refrigerator.
 - Service will refrigerate the milk between 3-5 days at 4 degrees Celsius (or lower) until it is required. (do not store in door of fridge).
 - To avoid any possible confusion, we will not store unused milk at the service.
 - Unused milk will be returned to families at the end of the day when they come to collect their child.
 - If the service does not have enough breast milk from the family to meet the child's needs that day, individual families will be consulted on what the service should do in these circumstances.
 - Do not refreeze thawed breast milk.
 - A quiet space with a comfortable chair will be provided for mothers/women to breastfeed or express milk.
1. Implement safe food handling practices.
 2. Wash your hands thoroughly using the correct handwashing procedure. Ensure workbench is clean and sanitised.
 3. Put on clean gloves when preparing bottles.
 4. Families will provide clean and sterile bottles (labelled with child full name). These may need to be sterilised for re-use.
 5. Breast milk will be warmed and/or thawed by standing the container/bottle in a container of warm water.
 6. Fill a clean container or jug with boiling water and place breast milk bag/bottle inside. Water should be no higher than the amount of milk – do not cover entire bottle/bag.
 7. Monitor breast milk temperature by massaging the breastmilk bag or shaking bottle gently
 8. Test the milk's temperature before giving to child, by placing a few drops on the inside of your wrist. It should feel lukewarm or body temperature (37 Degrees Celsius), but not too hot. If it's too hot, cool it quickly by placing it in a container of cold water for a few minutes.
 9. Always check the bottle's temperature before feeding the child.
 10. Document the amount of milk each infant consumed including the time of feeds in the OWNA App.

How to Operate the Bottle Warmers

(Please read and follow Bottle Warmer Appliance user manual instructions. Instructions may vary depending of manufacturer.)

1. Implement safe food handling practices.
2. Wash your hands thoroughly using the correct handwashing procedure.
3. Put on clean plastic gloves.

4. Ensure the small water tank at the back has water: Fill small water tank behind warmer using **BOILED** cold water only, and slide the tank into machine into the designated slot.
5. Ensure the 2 inserted baskets are inside the machine before operation.
6. Select desired mls on machine by matching the mls from the bottle
Press the bottle emoji matching the temperature of the bottle:
 - Fridge with icicle – bottle from fridge
 - House – room temperature bottle
7. Once selected then use the arrows up and down to select the mls closest to mls of the bottle: 60mls, 120mls, 150mls, 240mls, 270mls and 330mls.
8. Close lid of bottle warmer and press the play button on the far right of the machine. Machine will immediately light blue and begin to heat bottle.
9. Once a beep is heard bottle is finished, lift lid and remove bottle.
10. Bottle warmer will automatically turn off until needed again.
11. If bottle warmer beeps displaying H2O the water tank behind the warmer will need refilling and replaced as stated in 1.
12. Follow milk serving procedures.

BOTTLE WARMER CLEANING (Nominated Day E.g. Friday) – Bottle warmers are to be dismantled and placed over the sanitise dishrack or bench to dry. Remove 2 x baskets from inside warmer and water tank. Flip bottle warmer upside down to drain in sink and set on dish rack.

Heating of Milk Bottles Using Microwave

We recognise that microwaves are an efficient and safe way to heat infant formula and cow's milk. We will not heat breast milk in the microwave as it may destroy some of the breast milk's properties.

For infant formula or cow's milk, the service will:

- Use microwave safe bottles.
- Heat a minimum of sufficient levels of formula/cow's milk in the bottle to ensure it does not overheat.
- Only heat formula/cow's milk that has been adequately refrigerated.
- Stand the bottle up straight
- Always take off the teat/bottle top and leave outside the microwave.
- Not use microwave ovens with wattage over 700W (or if ovens with a higher wattage are used, heat for less than the following times and take extra care when checking temperature)
- Follow specific manufacturer instructions provided with the microwave for recommended heating requirements.
- Minimise the risk of uneven heating by adequately rotating and shaking the bottle directly after microwaving. After the teat/bottle top is replaced, Invert the bottle at least 10 times and let the bottle sit for 1-2 minutes before testing the temperature.
- Check the temperature of the formula/milk on the inside of the wrist before giving to the child to ensure contents are at a safe temperature. It should feel lukewarm or body temperature (37 Degrees Celsius), but not too hot.
- Follow milk serving procedures.

Appendices - Additional information

Appendix 1

Healthy eating and oral Health: For the purposes of this policy and for the Healthy Early Childhood Services Achievement Program, the 'Healthy Eating and Oral Health' health priority area focuses on supporting healthy eating, including nutrition, positive food habits and food literacy, and supporting oral health and hygiene practices.

Healthy eating: Eating a wide variety of foods from the five food groups each day. These are:

- Fruit
- vegetables and legumes/beans
- grain (cereal) foods, mostly wholegrain
- milk, yoghurt, cheese and alternatives
- lean meat, poultry, fish, eggs, tofu, nuts and seeds and legumes/beans.

Healthy eating also means eating in a way that is socially and culturally appropriate, having regular meals and snacks and eating food to satisfy hunger, appetite and energy needs.

Nutrition: The process of providing or obtaining the food necessary for health and growth.

Discretionary/sometimes food and drink: Discretionary/sometimes food and drink are high in fat, sugar and salt or a combination of these. They typically have very little nutritional value and are often processed and packaged.

Examples of 'sometimes foods' include:

- chocolate, confectionery, jelly
- sweet biscuits, high fat/salt savoury biscuits, chipshigh sugar/high fat cakes and slices
- cream, ice cream
- deep fried foods (e.g. hot chips) and pastry-based foods (pies, sausage rolls and pasties)
- most fast food and takeaway foods
- some processed meats (e.g. sausages, frankfurts/hot dogs, salami, strasbourg, devon, some commercial chicken nuggets and fish fingers)
- soft drinks, fruit juice and fruit drinks, cordial, sports drinks, energy drinks, flavoured milk and flavoured mineral water.

Sometimes foods can also referred to as "discretionary" or "extra" foods.

Appendix 2

Healthy Eating Guidelines

- Exclusive breastfeeding is recommended, with positive support, for babies up to six months. Continued breastfeeding is recommended for at least 12 months - and longer if the mother and baby wish.
- If an infant is not breastfed, is partially breastfed, or if breastfeeding is discontinued, use an infant formula until 12 months of age.
- Introduce suitable solids at around six months.
- Make sure that food offered to children is appropriate to the child's age and development, and includes a wide variety of nutritious foods consistent with the Australian Dietary Guidelines.
- Provide water in addition to age-appropriate milk drinks. Infants under the age of six months who are not exclusively breastfed can be offered cooled boiled water in addition to infant formula.
- Plan mealtimes to be positive, relaxed and social.
- Encourage children to try different food types and textures in a positive eating environment.
- Offer an appropriate amount of food, but allow children to decide themselves how much they will actually eat.
- Offer meals and snacks at regular and predictable intervals.
- Ensure that food is safely prepared for children to eat - from the preparation stages to consumption.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 25/03/2026.

Next review date: 15/08/2026

Endorsed by: Approved Provider, Centre Director and Educators

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations 2011

Regs	77	<i>Healthy, hygiene and safe food practices</i>
	78	<i>Food and beverages</i>
	79	<i>Service providing food and beverage</i>
	80	<i>Weekly menu</i>
	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	162	<i>Health information to be kept in enrolment record</i>
	168	<i>Education and care service must have policies and procedures</i>

EYLF

LO3	<i>Children recognize and communicate their bodily needs (for example, thirst, hunger, rest, comfort, physical activity).</i>
	<i>Children are happy, healthy, and safe and connected to others.</i>
	<i>Children show an increasing awareness of healthy lifestyles and good nutrition.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.</i>
	<i>Educators engage children in experiences, conversations and routines that promote healthy lifestyles and good nutrition.</i>
	<i>Educators model and reinforce health, nutrition and personal hygiene practice with children</i>

Other References:

- Handbook on Child Care Licensing
- QIAS Principle 6.1, 6.2, 6.3
- Food Safety Legislation 2001
- Local Council Regulations for Food Premises
- The Food Act 2003.
- Occupational Health & Safety Act 2004

Additional Sources and Further Reading:

- Caring for Children: Food, nutrition & fun activities (3rd ed.) Bunney & Williams (1998)
- Anaphylaxis Australia <http://www.allergyfacts.org.au/> (Sourced 28th July 2008)
- Department of South Australia - Department of Health (2006) Food Safety Resources CD
- Feeding and Nutrition of Infants and Young Children (World Health Organisation, 2000)
- Australian Breast Feeding Association Guidelines. www.breastfeeding.asn.au

- Infant Feeding Guidelines. National Health and Medical Research Council, 2012
- Get Up and Grow. Healthy Eating _and Physical Activity for Early Childhood. Australian Government, 2009
- Australian Dietary Guidelines. National Health and Medical Research Council, 2013
<http://www.eatforhealth.gov.au>
- Menu Planning Guidelines for Long Day Care, Healthy Eating Advisory Service, Department of Health and Human Services, State Government of Victoria, 2012
- [Belonging, Being and Becoming. The Early Years Learning Framework for Australia. Commonwealth of Australia, 2009](#)
- [ACECQA National Quality Standard 2018 – Quality Area 2](#)
- [Quality Area 6 – Collaborative Partnerships with families and communities](#)
- [Quality Area 7 – Governance and Leadership](#)

29 - FOOD SAFETY AND HYGIENE POLICY

Position Statement

Team members need to have a thorough understanding of the importance of food safety and their role in ensuring that the food the children consume at the centre is safe and hygienic. Our policy reflects the Food Safety Standards of Australia in relation to safe premises, practices and equipment standards.

Objective

To ensure all team members have a clear understanding of the expectations in regards to safe food handling and safe food storage practices. To share safe food handling and storage information with families to encourage safe practices at home.

Implementation

All childcare centres need to comply with the Food Safety Standards which were developed by Food Standards Australia New Zealand (FSANZ) to protect children from unsafe food. Food safety is achieved when hazards are identified and controlled from point of production through to handling, transporting and serving food and the use of leftovers. In addition to the Food Safety Standards some centres are required to adhere to state specific legislation.

Each centre that provides food at their centre will develop a food safety plan that complies with the Food Act 1984 including Australia and New Zealand Food Authority standards 3.1.1, 3.2.1, 3.2.2 and 3.2.3.

All cooks will be required to have completed an accredited food handling course as specified by their Support Manager in accordance with the state and local government regulations. Informal food handling training will be provided for all centre team members in regards to safe food handling practices upon induction and updated at team meetings in accordance with best practice guidelines.

Food Safety Practices

Whilst an individual food safety plan has been developed for the centre it is essential that plan is implemented consistently to ensure that the centre adheres to safe food practices at all times.

Please refer to the centre job descriptions and Food Safety Plans for roles and responsibilities within the organisation in relation to food safety.

All staff of the centre should be aware of, and sensitive to children with cultural diverse or indigenous backgrounds and culturally practices without deviating from the Hygiene and Food Safety Policy or Food Safety Plan of the centre.

All team members responsible for the handling of food will ensure that they are familiar with the requirements of the food safety plan.

The Cook is responsible for the completion of the schedules and log in the Food Safety Plan and Centre Checklists. The Food Safety Supervisor and Cook are responsible for the completion of the internal food safety audit.

The cook and the food safety supervisor will be point of contacts for hygiene and food safety. The cook and food safety supervisor will work with Management and authorities to resolve any issues associated with hygiene and food safety.

All staff members are involved for monitoring and assessing the risk of food and hygiene practices within the centre. The cook, food safety supervisor and OHS&W officer of the centre are responsible to manage, assess and monitor the risks raised.

Purchase of Food Products

- Food MUST be checked on arrival at the centre prior to storing. Each item is to be visually inspected for packaging integrity and for visible contaminants.
- Food products must meet temperature requirements as per standards. Cold food must be below 5°C and frozen food must be hard frozen. Thermometers are to be cleaned and wiped with an alcohol swab.
- Food is to be checked that it is adequately labelled with the Manufacturer's or Importer's details, a description of the ingredients/food and a use by date or batch code.
- Raw food is to be transported separately from other foods to prevent cross contamination.
- Potentially hazardous foods include; milk and milk products, egg products, meat, poultry, seafood, smallgoods, prepared foods such as pasta or rice dishes, salads, etc. These items must be moved into temperature controlled storage facilities within 15 minutes of arrival at the centre
- Any foods delivered in breach of these guidelines are to be returned to the supplier.

Safe Food Storage

- Food is to be stored in appropriate areas which have been specifically designated as food storage areas. No other items (ie chemicals, cleaning equipment, personal belongings, etc) will be stored in this area. Food is to be stored on shelving, no food is to be placed on the floor.
- Thermometers will be installed on fridges and freezers to ensure appropriate temperatures are maintained and thermometers will be calibrated in accordance with the centre food safety plan
- Stock of the same type is stored in rotation so that the oldest stock (nearest to the use by date) is used first. Different batches of food are not mixed together in storage containers.

Hand washing

Hand washing is a critical element in minimising the spread of food borne illnesses and the cross contamination of foods.

- As per the centre Hygiene policy, hand washing with soap will be carried out by team members and children:-
 - ✓ Upon arrival and departure from the Centre
 - ✓ Before preparing, serving, or eating food
 - ✓ Before preparing a baby bottle and before feeding an infant a bottle
 - ✓ After handling raw food
 - ✓ After handling garbage
 - ✓ After wiping anyone's nose
 - ✓ After touching ears, nose, hair, mouth or other body parts
 - ✓ After toileting themselves
 - ✓ After every break

Hands must be dried with a single use disposable paper towel or under an air drier. Dishwashing or food preparation sinks are NOT used for hand washing.

Food handling gloves, where used, are;

- Changed according to the hand washing requirements listed above
- Discarded and changed when damaged

- Not re-used
- Not worn outside of the food work area

Food Preparation

- Long hair is to be tied back or covered so that it cannot contaminate food. Jewellery is limited to plain banded rings and sleepers for pierced ears.
- Food handlers with open cuts or wounds on their hands or arms must not handle food or food equipment, unless the injury is completely protected by a brightly coloured waterproof bandage. Flesh coloured adhesive dressing or bandages are not used.
- Disposable gloves are worn over bandages on hands. Bandages and gloves are changed regularly.
- Ready to eat foods are not handled with bare hands. Utensils such as tongs, spoons, spatulas as well as clean, gloved hands are used.
- Separate utensils or equipment are used for the preparation of raw food and cooked or ready to eat foods.
- Utensils or equipment which is used for both raw and cooked or ready to eat foods, are cleaned and sanitised between uses.
- All utensils and equipment are maintained in a clean, sanitary condition and are maintained in a good state of repair.
- Raw fruits and vegetables are thoroughly rinsed in water to remove any contaminants.
- Potentially hazardous foods outside of temperature control during preparation are returned to refrigeration as soon as possible, or otherwise cooked or served immediately.
- Frozen food is thawed under refrigeration to maintain the temperature of the food below 5°C.
- Food which is thawing is stored in an area of the refrigerator so that it does not contaminate other food. Thawed food is not to be refrozen.
- All food is cooked to a core temperature of 75°C for 2 minutes.
- Cooking of food follows a standard recipe and procedure to achieve the correct temperature.
- Any tasting of food is done with a separate, clean and sanitised utensil each time. Fingers are not used for tasting food.

Food Safety at Mealtimes

- Team members will ensure that the eating tables and utensils have been cleaned and sanitised prior to use
- Team member will test the temperature of food before serving to children (to ensure not too hot). Please record the temperature of food served on the "Child Wellbeing Schedule or Book" after the menu item. For example Chicken Curry & Rice (55 degree Celsius). Please see below further guidance on Food Serving Temperatures to children.
- Team members will ensure that children wash their hands before meal times
- Team members will ensure that children will not eat food that has been handled by another child
- Team members will ensure that food that has been dropped on the floor is not eaten by anyone
- Team members will ensure that eating and drinking utensils are not shared
- Team members will encourage children's independence whilst still maintaining hygiene in the serving of food
- Team members will ensure that children are seated whilst eating (to prevent choking) and will reinforce this to children in a positive manner
- Team members will discuss food safety practices with the children

Food Serving Temperatures to Children

- It is acknowledged that some children may prefer eating foods at different temperatures. Please consider the allergies management plans, food safe zones and safe serving temperatures before providing food to children. The food serving temperature is different to the Food Safe Zone per the Food Safety Program. Food must be cooked and heated correctly per the Food Safety Program in the kitchen before serving. Per the Food Safe Zone there is a two-hour window. Please do not keep food in the temperature danger zone of 5 to 60 degrees celsius for more than 2 hours (food should be disposed).
- To make sure food is warm enough to enjoy yet not hot enough to be a danger of burning child's mouth or throat, use a food thermometer and make sure the food is within serving temperature range. Record the food serving temperature.
- It is recommended to serve **children (ages above 18 months) hot food below 60 degrees celsius** to reduce the risk burning their mouth and throat. Food can further cool before child eats.
- Babies/infants have more sensitive mouths and throats. It is recommended for **infants (ages below 18 months) to serve hot food below 45 degree celsius**. It is recommended to serve infant hot food between the temperature of 32(90 F) to 45(113F) degrees celsius to reduce risk of burning their mouth and throat (*Source: Safe Food for Babies and Children, NDSU, FN715 (Review December 2021)*).
- In addition to measuring temperature of food for babies, you can dab a tiny bit of food on the inside of your wrist or back of your hand to see if it's a comfortable temperature before giving it to child. (Please do not serve child food used to complete test on hand or wrist.).
- Food Safety Reminder: Reheating temperature - Once potentially hazardous foods (E.g. raw and cooked meat, dairy products, processed or cut fruits and vegetables, and cooked rice and pasta) have been cooked above 75°C and then cooled for later use, they can then be reheated once to a temperature above 60°C. After reheating, wait until the food has cooled down and check the temperature is safe before offering food to child.
- Allow food to cool down to a safe serving temperature before providing meal or self-serving container to children. Children can be encouraged to test their own food temperature in their bowls and cool it if they prefer a cooler temperature. Children can be taught to blow on their own food. Due to spread of germs, educators should not blow on children's food.
- Cooling food: To cool food quickly put it in an airtight container and run under cold water. Or place food in another bowl or serving tray to allow food to more surface area to cool quicker.
- References:
 - Keeping children's food safe, ACECQ. Link: <https://www.acecqa.gov.au/latest-news/blog/keeping-childrens-food-safe>
 - Safe Food for Babies and Children: Heating Solid Food Safely. NDSU, FN715, Dec 2021. Link: <https://www.ndsu.edu/agriculture/extension/publications/safe-food-babies-and-children-heating-solid-food-safely>
 - Preventing a scald injury from Hot Food and Drink. Kidsafe. March 2013

Food Safety for Pacifiers and Feeding Bottles

Formula or Breast Milk Food Safety

Potential hazard foods include dairy products such as milk. Potentially hazardous food that requires reheating must be reheated rapidly (in under 2 hours) to 60°C or hotter. Potentially hazardous foods should be discarded and never reheated a "second" time. For clarification cooking is the first stage of heating milk/food, once milk/food cools and is reheated (this is the first re-heat). Potential hazardous food should discarded and not reheated the second time.

Breast milk or formula in feeding bottles is not to be reheated the second time. It is recommended that families provide additional portions of formula or breast milk to cover the babies feeding requirements to avoid the need to reheat.

General guidance is it recommended that a fresh batch of breast milk or formula milk be used at each mealtime. If formula or breast milk is made up and the child only drinks 40ml, this feeding bottle should be discarded. If the child needs another feeding bottle in 2 hour later, a

fresh sterilised feeding bottle needs to be made. Please place a time label the bottle to track time bottle was made or use wellbeing chart to document time bottle was made.

For different milk types please follow the relevant procedure.

Breast Milk: Important food safety tips include:

- use fresh breastmilk whenever possible;
- if mother is expressing breastmilk, hand hygiene procedure needs to be followed. ie. wash and dry hands before handling equipment and milk;
- use sterile equipment and containers;
- store expressed breastmilk in a sterile, food-grade container and label the container with the date it was expressed;
- store expressed breastmilk in:
 - the fridge (in the back where it is coldest) for no more than 72 hours; or
 - the freezer compartment inside a fridge (-15°C) for up to 2 weeks; or
 - in the freezer section of a fridge with a separate door (-18°C) for 3 months;
- thaw frozen milk in the fridge. Thawed breastmilk should be used within 24 hours;
- rewarm expressed milk by standing the container in heated shallow water for no more than 15 minutes;
- Bottles (by themselves) in microwave ovens are not recommended to re-warm milk as they heat unevenly and overheated parts of milk can contain 'hot spots' that can burn a baby's mouth;
- Check temperature of milk before feeding your baby by placing a drop of liquid on the inside of the wrist. It should feel warm;
- do not re-freeze expressed breastmilk;
- discard expressed milk after 2 hours at room or warm temperatures;
- once feeding has begun, any unused breast milk should be discarded after the feed; and
- use extra care with hygiene if parent, guardian and caregivers are suffering illnesses (see Illness Policy).

Powdered formula: Important Safety Tips

Harmful microorganisms such as *Cronobacter sakazakii* (previously known as *Enterobacter sakazakii*) and *Salmonella* occur naturally in the environment and might be present in many foods, including powdered infant formula. While manufacturers have strict controls in place to minimise contamination, powdered infant formula is not sterile. Take care to prepare and store Formula powder safely, to help reduce any risk of illness.

Before preparing formula

- Wash thoroughly hands and food contact surfaces (e.g. benchtops) with soap and warmwater and dry thoroughly.
- Wash and sterilise equipment such as bottles, teats and utensils.
- Use sterile tongs to remove bottles and so on, to avoid re-contaminating the equipment

Preparing formula

- Wash hands thoroughly with soap and warm water and dry with a disposable paper towel.
- Whenever possible, make a fresh batch of infant formula before each feed.
- Prepare infant formula exactly according to manufacturer's instructions.
- Check temperature of infant formula before feeding your baby by placing a drop of liquid on the inside of your wrist. It should feel warm.
- Avoid preparing infant formula if caregivers are suffering illnesses with vomiting or diarrhoea, and take extra care with hygiene.

After preparation storage

- If not used immediately, when made up infant formula must be stored in a sterilised bottle or
- container at the back of the fridge, in the centre where it is coldest.

- Discard un-used infant formula after 24 hours if refrigerated.

Re-warming stored, made-up formula

- Remove the prepared formula just before it is needed.
- Re-warm by placing the container in heated, shallow water for no more than 15 minutes or use a commercial bottle warmer.
- Microwaves are not recommended to rewarm formula as they heat unevenly and overheated parts of formula can burn the baby's mouth. Discard un-used infant formula after two hours at room temperature.

Liquid infant formula: Important food safety tips include:

Liquid infant formula is sterile before the package is opened. It may be a good alternative when travelling, but consult a health professional to ensure it is appropriate for your baby.

Follow the instructions for the product.

Once opened, if the liquid infant formula is not to feed to your baby immediately:

- Pour into a sterile food-grade container.
- Store immediately in the coldest part of the fridge: at the back, in the centre
- Re-warm by placing the container in heated, shallow water for no more than 15 minutes or
- use a commercial bottle warmer. Eg. Bottle warmer or a microwave bottle warmer device.
- Bottles in microwaves (by themselves) are not recommended to rewarm formula as they heat unevenly and overheated parts of formula can burn the baby's mouth. ie placing bottle in microwave by itself.
- Discard un-used infant formula after two hours out of the fridge.
- Discard after 24 hours in the fridge.

Sterilisation of Dummies/Pacifiers

It is recommended that children have a spare dummy for use.

If a baby uses a dummy it is important to keep it hygienically clean by washing and sterilising it between uses. A dummy that has split or cracked cannot be kept clean because liquid gets trapped inside it and allows germs to breed. Always check the baby's dummy for cracks or splits before giving it to them to suck and throw the dummy away if it is damaged.

Your mouth contains germs which can harm your baby. Never put a dummy in your mouth to clean it. Clean and sterilise the dummy regularly by boiling, steaming or using a chemical sterilising agent.

Please refer to methods below sterilising bottles for dummies/pacifiers.

To clean and sterilise a dummy you should:

- Start by washing your hands with soap and water according to the correct hand washing technique.
- Wash your baby's dummy in soapy water and rinse well in plain water.
- Sterilise the dummy, either by boiling, steaming or using a chemical steriliser. You can sterilise several dummies at the same time, and if the baby feeds from a bottle you can sterilise bottles, their teats and dummies together.

To sterilise a dummy by boiling:

- Place the dummy in a saucepan of tap water and put the lid on.
- Bring to the boil and leave to boil for five minutes.
- Allow the water to cool before removing the dummy.
- Allow the dummy to air dry – do not wipe it dry.

To sterilise a dummy using chemical steriliser:

Policy Handbook – P004. 7.2026

- Follow the directions on the packaging regarding the quantities of steriliser and water to use.
- Mix the solution in a clean plastic container.
- Always make a fresh sterilising solution each day.
- Ensure that the dummy is completely covered by the sterilising solution. It won't clean properly if the dummy is floating on top.
- Never add metal utensils to the sterilising solutions, for example do not use a metal spoon to remove the dummy.
- Do not rinse the dummy after removing it from the sterilising solution

Where a baby does not have a clean spare dummy educators will sterilise a dirty dummy. Educators will advise parents to discard the dummy if the baby has an infection.

For children over 12 months old dummies can be cleaned by washing with warm, soapy water and rinsing well.

Sterilisation of Bottles

Educators will sterilise all bottle-feeding equipment for babies up to 12 months old. The bottles, teats etc. will be washed in warm, soapy water using a clean bottlebrush to thoroughly remove all traces of milk. The bottles will be rinsed and then sterilised by one of the following methods.

Method 1: Boiling

Educators will:

- Place all equipment in a large pot and cover with tap water (make sure your water meets the Australian Drinking Water Guidelines if you are not using town water).
- Make sure there are no air bubbles trapped inside the bottles.
- Put the saucepan lid on and bring to the boil.
- Allow five minutes of rapid boiling.
- Turn off heat and allow to cool.
- Make sure they wash their hands thoroughly with soap and water before handling the equipment.
- Store the sterilised equipment in a clean container in the fridge and re-boil after 24 hours if it has not been used before then.

Method 2: Microwave steam sterilisers

Educators will:

- Read the instructions carefully.
- Check the microwave power needed to sterilise the equipment properly because not all microwave ovens are the same.
- Do not use metal inside these sterilisers.

Method 3: Steam steriliser appliances

Educators will use a steam steriliser appliance to the manufacturer's directions and:

- Place the equipment in the steriliser and add the recommended amount of water.
- Put the lid on and steam the equipment for the recommended period of time.
- Keep the lid on until the bottles are needed.
- Wipe and dry the inside of the container once a day to keep it clean.

Method 4: Sterilising chemicals

Educators will carefully follow the manufacturer's instructions to ensure correct strength and:

- Mix the chemicals with water in a big plastic or glass bowl
- Place all equipment in the solution ensuring all surfaces are covered– for example, by squirting solution through teats and getting rid of any air bubbles.

- Soak the equipment for at least one hour (or follow directions of chemicals) and change the solution every 24 hours.
- Scrub the container with warm soapy water and rinse thoroughly before refilling with new solution.
- Remove the equipment and shake off excess solution, but do not rinse.
- Use only glass or plastic, as metal will rust when left in the solution.

Food Handlers Health Issues

- Should a food handler be unwell they will be excluded from all food preparation and food handling duties
- It is the food handler's responsibility to notify their supervisor if they are suffering from vomiting, diarrhoea, sore throat with fever, fever and/or jaundice. If the food handler is suffering from one or more of these symptoms they must not work with food. They will be required to obtain a medical clearance before returning to work

Additional information in regard to our centre practices in the provision of meals at the centre can be obtained from the;

- Nutrition, Food, Beverage & Dietary Requirements Policy
- Anaphylaxis policy
- Dental and Oral Health policy
- Health and Hygiene policy
- Food Safety Plan

Food Safety and Hygiene Documentation, Records, Forms and Checklists

All documentation associated with Food Safety and Hygiene is required to be completed and filed in the relevant files and folder detailed in the Food Safety Plan.

Please refer to the Food Safety Plan for the following schedules and checklists:

- Internal Audit Record
- Cleaning Procedure Schedule
- Cleaning Record
- Thermometers Calibration Log Schedule
- Customer Complaints Schedule
- Approved Supplier Register
- Product Receival Log
- Equipment Temperature Log
- Records of Staff Illness Which Prevents Them from Handling Food Log
- Cooking Temperature Verification Log
- Training Record
- Chemical Storage and Record of Chemicals Used in Food Preparation Area Log
- Corrective Action Log

Disease Management

All childcare services must notify the State Health Authority if an outbreak of an infectious disease that is preventable occurs. The centre will collect and keep up to date records detailed in the Food Safety Plan.

If this occurs, the following should happen:

1. The child's parent/guardian or staff member is to notify the centre of their diagnosed disease.
2. The Centre Director will call the State Health Authority to advise of any disease outbreaks.

Preventative Strategies

Please refer to the following following policies and references:

Policy Handbook – P004. 7.2026

- Immunisation Policy – 32
- Health & Hygiene Policy – 27
- Food & Nutrition Policy
- Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)
- Food Safety Plan

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2025

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	77	<i>Health, hygiene and safe food practices</i>
	78	<i>Food and beverages</i>
	79	<i>Service providing food and beverages</i>
	80	<i>Weekly menu</i>
	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	162	<i>Health information to be kept in enrolment record</i>
	168	<i>Education and care service must have policies and procedures</i>

EYLF

LO3	<i>Children recognise and communicate their bodily needs (for example, thirst, hunger, rest, comfort, physical activity).</i>
	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Children show an increasing awareness of healthy lifestyles and good nutrition.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community.</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.</i>

	<i>Educators engage children in experiences, conversations and routines that promote healthy lifestyles and good nutrition.</i>
	<i>Educators model and reinforce health, nutrition and personal hygiene practices with children.</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 6.1, 6.2, 6.3

Food Safety Legislation 2001

Local Council Regulations for Food Premises

The Food Act 2003.

Fact Sheet: Feeding Babies and Food Safety: Government of SA, Food Safety and Nutrition Branch, SA Health

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Occupational Health & Safety Act 2004

Additional Sources and Further Reading:

- *Food Hygiene Australia(2008) 'Food Safety News' Issue 2, May 2008*
- *Food Standards Australia New Zealand (FSANZ) www.foodstandards.gov.au (Sourced 13th October 2008)*
- *Tansey, S (2006) 'Food Safety' National Childcare Accreditation Council Fact Sheet*
- *Caring for Children: Food, nutrition & fun activities (3rd ed.) Bunney & Williams (1998)*
- *Anaphylaxis Australia <http://www.allergyfacts.org.au/> (Sourced 28th July 2008)*
- *Department of South Australia – Department of Health (2006) [Food Safety Resources CD](#)*
- *Meat and Livestock Australia – Food Safety www.safefood.net.au (Sourced 13th October 2008)*
- *NSW Food Authority www.foodauthority.nsw.gov.au (Sourced 13th October 2008) Food Standards Australia New Zealand. (2001). Retrieved 1st March 2010, from <http://www.foodstandards.gov.au/>*
- *Safe Food Australia, 2nd Edition. January 2001. Retrieved on the 1st March 2010 from <http://www.foodstandards.gov.au/scienceandeducation/publications/safefoodaustralia2nd519.cfm>*
- *NSW Health, retrieved 1 March 2010 from, www.mhcs.health.nsw.gov.au*
- *C Burney and L Williams published by the Department of Health entitled “ Caring for Children- Food, Nutrition and Fun Activities ”, 4th Edition 2006*
- *Australian Guide for Healthy Eating, retrieved on 1 march 2010 from [http://www.health.gov.au/internet/main/publishing.nsf/Content/E384CFA588B74377CA256F190004059B/\\$File/fd-cons.pdf](http://www.health.gov.au/internet/main/publishing.nsf/Content/E384CFA588B74377CA256F190004059B/$File/fd-cons.pdf)*
- *Food Act 1984*
- *Occupational Health and Safety Act 2004*
- *Dietary Guidelines for Children and Adolescents in Australia incorporating the Infant Feeding Guidelines for Health Workers Endorsed 10 April 2003*
- *Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)*
- *Food Safety Standards for Australia 2001*
- *Food Standards Australia and New Zealand Act 1991*
- *Food Standards Australia New Zealand Regulations 1994*

30 - DENTAL AND ORAL HEALTH POLICY

Position Statement

Oral health behaviours have a major influence on children's health and wellbeing and a direct impact on their growth and development. Early childhood education and care services play an important role in promoting young children's oral health. This is a time when lifelong oral health behaviours are being formed.

Oral health is essential for children's overall health and wellbeing. Oral diseases can negatively affect individuals through pain, discomfort, general health and quality of life. The main oral health condition experienced by children is tooth decay, affecting over half of all Australian children, making it five times more prevalent than asthma. Tooth decay is Australia's most prevalent health problem despite being preventable (*Rogers J, Prevention and Population Health Branch, Government of Victoria, Department of Health, 2011, Evidence based oral health promotion resource*).

Objective

The educators, staff and management acknowledge the importance of oral health behaviours that contribute to overall health and wellbeing.

This policy confirms our commitment to:

- create environments that support good oral health and general health
- encourage children and educators to make healthy food and drink choices.

As a health promoting service we will promote the oral health of children, educators, staff and families through learning, policies, creating a safe and healthy physical and social environment and developing community links and partnerships.

Implementation

- The Centre will promote healthy eating habits – encouraging fresh fruit, minimal sweet discretionary foods, and plenty of water to drink. This will be communicated to the children during group time sessions, throughout the curriculum (ie healthy cooking experiences) and also through general discussions at mealtimes.
- The service menu promotes the consumption of fruit and vegetables on a daily basis and healthy food options in line with Australian Dietary Guidelines.
- Sometimes foods and sweetened drinks (juices, cordial and soft drinks) are not provided by the service.
- Safe drinking water (preferably tap water) is available indoors and outdoors at all times and is accessible to children. Children are encouraged to drink water regularly. Only tap water or plain milk is provided.
- Where possible the centre will arrange visits to the centre by a dental hygienist or similar to promote the care of children's teeth.
- The Centre will promote the development of healthy eating habits to families. Information will be available for families through newsletters, centre displays and appropriate literature which supports healthy eating habits and the care of ones teeth. Where possible this information will be provided in the home languages of the centre.
- All children will be encouraged to have a drink of water after eating to rinse their mouth. Water will be swallowed.
- The Centre may choose to introduce a tooth brushing program for children over three years of age (or younger), in consideration with current recommendations in regards to how often each day young children should brush their teeth. Should tooth brushing be implemented at the centre, this would be with written consent from the families and under strict health and hygiene guidelines.
- The centre will liaise with families to establish dental care practices both in the home and centre environment.

Learning and skills

- Children have opportunities to learn about, and develop skills for oral health through the educational program.
- Staff talk to children about age appropriate tooth brushing and why this is important.
- Educators and staff are supported to access a range of resources and professional development opportunities to increase their capacity to promote oral health initiatives for children.

Dummies (Pacifiers) in our Centre

Some parents choose to send their child to the centre with a dummy for comfort. Should a child present at the centre with a dummy the following will be applied in order to promote oral care and sound hygiene practices.

- Dummies are NOT to be “dipped” into any sugary substances.
- Dummies should be checked on a daily basis to ensure that they are in good condition. Dummies that have holes or are showing signs of wear and tear pose safety risks from either choking or spread of infection.
- Dummies that have fallen to the floor must be picked up and sterilised before being used again by the child.
- Dummies are NOT to be shared (even with siblings).

Baby Bottles in our Centre

Every year a large number of Australian children are treated for tooth decay as a result of ‘bottle caries’. This type of decay tends to affect infants and young children whose teeth have been exposed to frequent and prolonged exposure of a decay-causing substance. This usually occurs when children are put to bed with a bottle (which may contain breast or dairy milk or similar, formula, fruit juice, cordial, etc) for comfort.

As per our Rest Time/Sleep policy and in accordance with safe sleeping practices no child is to be put to bed with a bottle. In addition to ensuring the child’s safety this also can limit the prevalence of bottle caries in young children.

Managing a Dental Emergency

- Should a dental emergency occur at the centre due to an accident, team members will follow the centre Incident Policy and procedure.

Engaging children, educators, staff and families

- Educators, staff, children and families are key partners in developing and supporting oral health initiatives.
- Educators, staff and families are provided with information, ideas and practical strategies on a regular basis to support oral health in the service and at home.
- The service will regularly provide families with information on oral hygiene and how and where to access public dental services.
- Families’ experiences, expertise and interests are drawn upon to support oral health initiatives.
- Families and children from culturally diverse backgrounds are consulted to ensure cultural values and expectations about oral health are respected.
- Oral health information is provided as part of orientation of new staff including provision of the oral health policy.

Community partnerships

- The service works with local health professionals, services and other organisations to support educators and staff to deliver and promote oral health initiatives.

Related Policies:

- 92 - Health and Wellbeing Policy
- 29 - Food Safety & Hygiene
- 27 - Health & Hygiene Policy
- 61 - Fundraising Policy
- 28 – Nutrition, Food, Beverage, Dietary and Oral Health Requirements Policy
- 40 - Rest Time/Sleep Policy

Appendix – Additional Information

Oral Health Messages for the Australian Public

- Breast milk is best for babies and is not associated with an increased risk of dental decay.
- A cup can be introduced at around six months, to teach infants the skill of sipping drinks from a cup.
- Put an infant to bed without a bottle or take the bottle away when the infant has finished feeding. Don't let the infant keep sucking on the bottle.
- Use age appropriate fluoride toothpaste (e.g. child's toothpaste) over the age of 18 months.
- Children should have an oral health assessment by the age of one.
- Brush teeth and along the gum line twice a day with a soft brush.
- Drink plenty of tap water (fluoridated if available).
- Limit sugary foods and drinks.
- Choose healthy snacks – fruits and vegetables.
- Pregnant women should have their oral health assessed and treatment needs addressed.

Definitions:

Oral health: A standard of health of the oral and related tissues that enables an individual to eat, speak and socialise without active disease, discomfort or embarrassment and that contributes to general wellbeing

Discretionary/sometimes food and drink: Discretionary/sometimes food and drink is high in fat, sugar and salt or a combination of these. They typically have very little nutritional value and are often processed and packaged. Examples of 'sometimes foods' include:

- chocolate, confectionery, jelly
- sweet biscuits, high fat/salt savoury biscuits, chips
- high sugar/high fat cakes and slices
- cream, ice cream
- deep fried foods (e.g. hot chips) and pastry-based foods (pies, sausage rolls and pasties)
- most fast food and takeaway foods
- some processed meats (e.g. sausages, frankfurts/hot dogs, salami, strasbourg, devon, some commercial chicken nuggets and fish fingers)
- soft drinks, fruit juice and fruit drinks, cordial, sports drinks, energy drinks, flavoured milk and flavoured mineral water.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/9/2025

Next review date: 15/9/2026

Endorsed by: Approved Provider, Centre Director and Educators

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child.</i>

QA2	2.2	<i>Each child is protected.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	77	<i>Health, hygiene and safe food practices</i>
	78	<i>Food and beverages</i>
	79	<i>Service providing food and beverages</i>
	80	<i>Weekly menu</i>

EYLF

LO3	<i>Actively support children to learn hygiene practices.</i>
	<i>Promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community.</i>
	<i>Discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.</i>
	<i>Model and reinforce health, nutrition and personal hygiene practices with children.</i>

Other References:

Handbook on Child Care Licensing
QIAS Principle 6.1, 6.2, 6.6

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Sources and Further Reading:

- **Australian Dental Association – www.ada.org.au (13 October 2008)**
- **Dental Health Services Victoria (2008) 'Dental care – dummies' www.betterhealth.vic.gov.au**
- **South Australian Dental Service – www.sadental.sa.gov.au (13 October 2008)**

- *Raphael, S. 'Bottle Caries and Dental Neglect' Westmead Centre for Oral Health CSIRO Vol.10 No. 4 pp31-32*
- *Australian Research Centre for Population Oral Health 2011. Dental caries trends in Australian school children. Australian Dental Journal, Vol 56, pp 227–30*
- *Rogers J, Prevention and Population Health Branch, Government of Victoria, Department of Health, 2011, Evidence based oral health promotion resource*
- *Australian Dietary Guidelines, National Health and Medical Research Council, 2013, <http://www.eatforhealth.gov.au>*
- *Get Up and Grow – Healthy Eating and Physical Activity for Early Childhood. Commonwealth of Australia 2009*

31 - ILLNESS POLICY

Position Statement

Maintaining a healthy community in early learning services is the shared responsibility of parents and team members. Quality standards require services to put specific strategies into action to minimise the spread of infectious illnesses. Early Learning Services are unable to provide care to contagious and ill children.

Related Policies

Health & Hygiene Policy
Health and Wellbeing Policy
Incident, Injury, Trauma and Illness Policy
Infectious Diseases and Immunisation Policy
Medical Conditions Policy
Interactions with Children Policy
Rest Time/Sleep Policy
Occupational Health & Safety Policy
Anaphylaxis Policy
Asthma Policy
Diabetes Policy
Coronavirus Policy
Cystic Fibrosis Policy
Epilepsy Policy
First Aid Policy
Medication Policy
Excursion Policy
Fees Policy
Nutrition, Food, Beverage & Dietary Requirements Policy

Reference Documents

Staying Healthy in Child Care – Preventing Infectious disease in childcare 6th Edition publication:

www.nhmrc.gov.au/sites/default/files/documents/reports/clinical%20guidelines/ch55-staying-healthy.pdf

Objective

We aim to maintain the health and wellbeing of all children, staff, and their families, ensuring a healthy environment and minimising cross contamination and the spread of illnesses. This policy sets guidelines for caring for and managing children's illnesses to maintain a safe and healthy enrolment.

Department of Health (Victoria) Infectious Disease Guidance

(Source: <https://www.health.vic.gov.au/infectious-diseases/exclusion-periods-for-primary-schools-and-childrens-services>):

- In Victoria, children's services such as childcare centres and kindergartens have a responsibility under the Public Health and Wellbeing Regulations 2019 to help manage infectious diseases in their facilities and exclude certain children for periods as specified in the regulations. There is also a responsibility to follow any direction to exclude a child that is made by the Chief Health Officer. If the service has a sick child, management and educator must:

- ensure that unwell children do not attend the service, as per national guidelines (Staying healthy: preventing infectious diseases in early childhood education and care services);
- isolate children who become unwell during the day from other children and send the unwell child home as soon as possible;
- exclude the unwell child from the service; and
- consider actions such as alerting parents or displaying signage.
- It is noted by the Department of Health that childcare services are at particular risk of transmitting infectious diseases because of:
 - close contact with other children and staff
 - lack of previous exposure to common infections
 - lack of toilet training
 - lack of control of other body secretions
 - mouthing behaviour

Implementation

- Our Service adopts the Staying Healthy in Child Care – Preventing Infectious disease in childcare publication, developed by the National Health and Medical Research Council to guide the health and wellbeing of children. We aim to provide families with up-to-date information regarding specific illnesses and ways to minimise the spread of infection within the Service.
- As part of illness prevention and communication protocols, the service will display notices and provide information to inform families of illness occurring at the service.
- We understand that it can be difficult for families to know when their child is sick. Families may experience problems taking time off work or study to care for their child at home. However, it is imperative that families preserve a focus not only on the well-being of their own child but also upon the well-being of other children and the Service team.
- Children are more susceptible to picking up illnesses when they first start childcare because they come into contact with more people than they do in their own homes and are exposed to a range of infections they may not yet be immune to.
- The need for exclusion and the length of time a person is excluded depend on how easily the infection can spread, how long the person is likely to be infectious and how severe the disease can be. To protect the health of children and staff within the Service, it is important that children and staff who are ill stay away from the Service for the recommended exclusion period.
- The Approved Provider, Nominated Supervisor, Centre Director and/or the Responsible Person/s in Charge of Service has the ultimate responsibility for deciding if a child is well enough to be at and/or return to the Centre.
- Our Educators and Staff are not medical practitioners and are not able to diagnose whether a child has an infectious illness. However, our team have experience in managing illness in an early childhood setting and if an infectious illness or sickness is suspected, we will ask the family to collect their child from care as soon as possible or to not bring the child to care. The service may request families to seek medical advice and provide a medical certificate or clearance stating the following information:
 - the nature of the illness;
 - that the child is not infectious or no longer infectious;
 - if any medical treatment is required or not;
 - the child is well and able to return to childcare; and
 - any other information of importance.
- On receipt of information, the service will determine the exclusion period applicable and return to care plan.
- A child who has been excluded with an infectious illness will require a doctor's clearance or a medical certificate stating length of time required for infectious illness to clear (before they return to childcare).
- A doctor clearance or medical certificate is not required after 5 days of non- at service (and child is illness symptoms free and well).
- Please see Staying Healthy in Childcare for specific guidance on infectious disease and recommended exclusion period. General guidance when Doctor prescribed child

antibiotics; child not to attend care if unwell. Child not to attend care until at least 24 to 48 hours after antibiotics are taken based on infectious disease treatment. (Child able to return when not infectious and child is well.)

- Please refer to the Common Illness Guidelines table below for the service policy on illness management and exclusion periods. The table should be read in conjunction with all policies and procedures of the service and the guide “Staying Healthy in Childcare (5th Edition)”.
- Sometimes children have ongoing medical needs that can be taken care of at the service such as asthma, anaphylaxis, etc. In these situations, services and family will document the child’s general health and behaviour status at enrolment through completion of Medical Management Plans. Please refer to the specific policy for these medical conditions.
- For children with a reportable illness, the Centre Director, Nominated Supervisor or Responsible Person in Charge of Service will notify the state Health Authorities as appropriate. To ascertain if an illness is reportable, please refer to Staying Healthy for exclusion periods and responsibilities of childcare providers and team members. <http://www.nhmrc.gov.au/publications>. Service team members are also required to maintain the Infectious Disease register at the centre (HEI005). See below further guidelines on notifications and reporting.

Symptoms indicating a suspected illness may include (but are not limited to):

• Behaviour that is unusual for the individual child, such as child who is normally active and who suddenly becomes lethargic or drowsy; • High temperature of above 38 Degrees Celsius • Poor circulation; • Loose bowels; • Discharge from the eye or ear; • Red or purple rash; • Poor urine output; • Headaches; • Sensitivity to light; • Difficulty in breathing; • Green mucus discharge from nose	• Difficulty in swallowing or complaining of a sore throat; • Persistent, prolonged or severe coughing; • Faeces which is grey, pale or contains blood; • Vomiting; • Loss of appetite / poor feeding; • Skin that displays rashes, blisters, spots, crusty or weeping sores; • Dark urine; • Stiff neck or other muscular and joint pain; • Continuous scratching of scalp or skin; • Drowsiness; • Pain.
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Common Illness Guidelines – Quick Reference

The following table should be read in conjunction with all service policies and procedures and the “Staying Healthy in Childcare (5th Edition)” guide.

Symptom	What to Consider	Summary of Illness Actions and Exclusions
COVID	COVID-19 symptoms range in severity. The following symptoms are considered mild symptoms in children: • mild upper respiratory tract symptoms such as congested or runny nose, sneezing, or a scratchy or sore throat • cough with no difficulty breathing • not drinking their usual amount of fluid (such as water) in the past 24 hours • mild vomiting and diarrhoea (fewer than 4 times in the past 24 hours) • mild headache or body aches	COVID is an infectious disease. If your child displays symptoms of COVID or is diagnosed with COVID stay home. Where children become symptomatic at the service they should: • be collected by their parents or carers. • undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test).

	• mild fever • mild fatigue It's also possible to have COVID-19 but have no symptoms at all.	• Advise service of COVID test result. Guidance: • Person who test positive to COVID-19 should stay home and isolate for 5 days • Person should not attend the service after 5 days if still symptomatic • Person who are symptomatic but have not tested positive should not attend ECEC services. (If child is negative and has other illness symptoms please refer to policy for guidance.) • Stay at home until well and symptoms have resolved. (Source: Department of Health, from 11.59pm 12/10 2022) Based this advice, parents and carers should advise the service of COVID-19 positive test result and not attend the service for 5 days. (Day zero is day of positive test result.) This aims to prevent the wide spread of COVID to other children, families and educators. <i>Symptomatic is defined as exhibiting or involving medical symptoms.</i>
Fever	Children and older infants with a temperature of above 38°C. No other symptoms but not normal wellbeing. Per guidelines of Staying Healthy in Childcare, key things to remember about fever: • Recognised authorities suggest a child's normal temperature will range between 36.5°C and 38.0°C. • Children get fevers or temperatures for all kinds of reasons. Most fevers and the illnesses that cause them last only a few days. • Fevers are common in children. • If the child seems well and is happy (no other symptoms), there is no need to treat a fever. Monitor and rest. • If the child is less than 3 months old and has a fever above 38 °C, contact the child's parent and recommend they take the child to a doctor. <u>Teething:</u> If guardian or parent believe that regular fever is caused by teething and not another illness, a doctors certificate needs to be obtained and documented on a Long Term Illness Form. If child is ill due to teething, the child is still unable to attend the service and exclusion periods still apply until child is well. First Aid will be applied, and Educators will attempt to lower the child's temperature by: ▪ Taking off their shoes and socks. ▪ Applying a cool washer behind their neck and on their forehead. ▪ Removing extra clothing layers (jumpers etc). ▪ Place the child in a quiet area where they can rest, whilst being supervised. ▪ Continue to document any progressing symptoms.	When undertaking temperature checks. If initial temperature check is high; please complete two temperature checks using two different thermometer devices each time. Undertake 2 temperature tests 10 minutes apart. Allow child body to stabilise, remove any excess jackets, etc). <i>If COVID symptoms, test for COVID:</i> undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test). Provide service test results. If positive, see COVID guidance below. <u>Initial fever / high temperature:</u> • Child's temperature is above 38°C <u>and unwell</u>. Child not to be taken to care or child needs to be collected from service by family. <i>12 hour exclusion period from last high temperature.</i> • Child able to return to care provided is well; temperature below 38°C and no illness symptoms. <i>Temperature should be taken without medication</i> such as Panadol / Nurofen as these medications will mask fever and illness symptoms. <u>Next day of care:</u> • <i>if child temperature is above 38°C and unwell. Suspected enduring illness. Child not to be taken to care or child need to be collected from service by family. 24 hour exclusion period from last high temperature. (Or obtain doctors clearance needed to return</i>

	Complete Illness Record, ensuring the form has been completed correctly and signed by the parent/guardian/emergency contact.	earlier than 24 hours if symptom free). Child able to return to care when well and not displaying illness symptoms. If child returns to care after 24 hour exclusion period and temperature is above 38°C and unwell again, a doctor clearance is required before returning to care. Child should not be provided medication such as Panadol / Nurofen before arrival if child has a fever or another illness. If child is provided medication as part of medical management plan before attending or at service by parent, this needs to be documented on a Medication Form and educator team advised upon drop-off.
Common Cold - various symptoms	Symptoms may include coughing, runny nose and a temperature. The common cold (Viral upper respiratory tract infections) are common in children during the first 2 years in childcare setting. Following of hygiene and hand washing procedures are essential to be followed to stop the spread of infection between people.	Child not to attend the Service. Minimum of 24 hour exclusion period or obtain doctors clearance needed to return earlier than 24 hours if symptom free. Child able to return to care when well and not displaying illness / cold symptoms. If COVID symptoms, test for COVID: undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test). Provide service test results. If positive, see COVID guidance below. The service team will determine if the child is well enough to continue at the service or if the child requires parental care. Our Service aims to support the family's need for childcare, however families should understand that a child who is unwell will need one-on-one attention and cannot attend the service. Also, staying home breaks the chain of infection. A child with a fever - All children will occasionally have an elevated temperature (fever). Having a fever is one of the most common reasons for children to see a doctor. Colds and flu symptoms are very similar to the symptoms of COVID-19. Even if your child's symptoms are mild, they should get tested for COVID-19 immediately and isolate at home until they have received a negative test result.
Diarrhoea	If child has 2 or more consecutive bowel motions that are loose or watery. They may also have stomach cramps. Per Staying Healthy in Childcare; in cases of gastroenteritis, children, educators and other staff who no longer have diarrhoea (loose stools) may still shed diarrhoea-causing germs in their faeces for some time. This means that they are still a potential source of the germ. For this reason, it is important that the infection control process is always followed by all people in the education and care service. This is why exclusion periods are in place.	Please do not attend the Service for at least 24 hours after loose bowel motion or diarrhoea has stopped and no other illness symptoms. (Staying Healthy in Childcare) Diagnosed as Gastroenteritis (gastro) by a doctor: To assist in the management and control of gastroenteritis (gastro), medically diagnosed people with this illness will be excluded from the service for 48 hours after symptoms have ceased/disappeared. If diagnosed with Gastroenteritis (gastro) a doctor

		clearance is needed to be provided before return to care. <i>If have loose bowel motion or diarrhoea and is vomiting do not attend the Service for at least 48 hours after symptoms stop. Child to return once well and after exclusion period.</i>
Vomiting	If your child has vomited more than twice in 24 hours. Or if vomit once and displays other illness symptoms (i.e. is unwell) Watch for signs of dehydration.	<i>Child not to attend the Service for at least 24 hours after vomiting has stopped and no other illness symptoms. (Staying Health in Childcare)</i> <i>If have loose bowel motion or diarrhoea and is vomiting do not attend the Service for at least 48 hours after symptoms stop. Child to return once well and after exclusion period. (see above)</i> <i>Parent or guardian to keep the child home until they are feeling well and they have not had any symptoms for at least 24 hours.</i>
Cough	This will depend on the severity of the cough. Trouble breathing, wheezing or a harsh cough can be the sign of something more serious such as bronchitis, pneumonia or whooping cough. When an infected person sneezes or coughs, tiny droplets are spread into the air and onto surrounding surfaces. A sneeze can spread droplets as far as 2 metres away. The droplets may be breathed in directly by another person, or another person may touch a surface contaminated with the droplets, then touch their mouth, eyes or nose. Cough and sneeze etiquette should be followed. This is sometimes difficult with young children and an ongoing and persistent cough may require child be sent home or stay at home until cough stops as a strategy to prevent the spread of infection.	<i>Persistent cough and unwell – Child not to attend the Service. Minimum of 24 hour exclusion period or obtain doctors clearance needed to return earlier than 24 hours if symptom free.</i> <i>Child able to return to care when well and not displaying illness symptoms.</i> <i>If COVID symptoms test for COVID:</i> undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test). Provide service test results. If positive, see COVID guidance below. If other illness symptoms occur, refer to guidance on these symptoms and the associated exclusion period.
Bronchiolitis	Bronchiolitis is a potentially serious chest infection caused by a virus. The virus infects the small breathing tubes (bronchioles) of the lungs, causing inflammation, mucus production and breathing difficulties. <i>Respiratory syncytial virus (RSV)</i> is usually responsible for bronchiolitis, although other viruses may cause outbreaks. Infections often occur in infants less than 1 year old, usually in winter. The incubation period for RSV is usually 5 days but can range from 2 to 8 days.	People are infectious just before symptoms begin and during the active stage of the disease—this is usually 1 week in total. <i>A child with bronchiolitis should stay at home until they are feeling well and display no symptoms. Doctor's clearance needs to be provided for return to care and child is well.</i> <i>If child diagnosed with bronchiolitis; not to attend care until for 24 hours after antibiotics are taken and present no illness symptoms. If child not taken antibiotics minimum 48 hours exclusion period and no illness symptoms.</i>
Rash	Rashes can be caused by the following (<i>Staying Health in Childcare</i>): • Allergic reactions to a variety of things (e.g. food, medication, soap, clothing material, grass or any	<i>A Doctor needs to evaluate your child before sending them to care or after being sent home from care. Please provide service with a medical</i>

	number of irritants). Examples of allergic reactions include hives or eczema. Watch the child for signs of more serious reactions, including swelling around the face, tightness in the throat, difficulty breathing or vomiting. • Sensitivity to something—examples include nappy rash or dribble rash. • Viruses—examples include varicella, human parvovirus and roseola. • Bacteria—examples include impetigo (school sores). • Fungi—examples include ringworm. • Insect bites—can often appear as several red bumps.	clearance before returning to the service. The exclusion period depends on the cause of the rash. Even if the rash is not infectious, the child should stay at home if they are not feeling well. Child can return to care if rash is not infectious and feeling well.
Hand, foot and mouth disease	Per Staying Healthy in Childcare, symptoms of hand, foot and mouth disease include tiny blisters on various parts of the body, including in the mouth, and on the fingers, palms of hands, buttocks, nappy area, soles of the feet, upper arms or upper legs. The blisters last a little longer than a week. Some children may also have a fever, sore throat, runny nose or cough. Vomiting or diarrhoea are uncommon.	A Doctor needs to evaluate your child before sending child to care or after being sent home from care. Please provide service with a medical clearance before returning to the service. People are infectious as long as the blisters contain fluid. Faeces can remain infectious for several weeks. Child will be unable to attend care until all blisters have dried. If a service outbreak is identified, children who have been diagnosed may be excluded longer to break the chain of infection.
Runny nose	A runny nose, no other symptoms. Child's wellbeing the same as normal. Nose mucus discharge colour is clear. Green mucus discharge indicator potential infection. If child has other symptoms, such as a high temperature, it is an indication of a greater illness.	Able to attend provided clear mucus only, no other symptoms and child's wellbeing is normal. Runny nose (green mucus), high temperature and child unwell: <i>Child not to attend the Service. Minimum of 24 hour exclusion period or obtain doctors clearance needed to return earlier than 24 hours if symptom free. Child able to return to care when well and not displaying illness symptoms. If ongoing illness symptoms see doctor and obtain clearance.</i> If COVID symptoms test for COVID: undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test). Provide service test results. If positive, see COVID guidance below. If other illness symptoms occur, refer to guidance on these symptoms and the associated exclusion period.
Red eyes / Conjunctivitis	Is the eye red and watery? The eyelids may also stick together on waking. This could be conjunctivitis which is highly contagious. Red eyes are a symptom of some infectious diseases.	Red eyes or conjunctivitis: a Doctor or Registered Pharmacist needs to evaluate your child before sending child to care or after being sent home from care to determine that it is an infectious disease such as conjunctivitis OR not. Children with infectious conjunctivitis will be excluded until the discharge from the eyes has stopped (<i>Staying Health in Childcare</i>). Unless your doctor has diagnosed a non-infectious cause, keep your child home while there is discharge from the eye and speak to your doctor about a

		suitable product or treatment plan. ie eye drops with pharmacy label is required. If it is not an infectious disease and there is discharge from the eye (eg. As result of blocked tear duct) a doctor letter will need to be provided to advise child is non-infectious and fit and well to attend care. If child is unwell, please keep child at home.
Stomach ache	This can often be hard for you to judge as it could be caused by several things including constipation and even anxiety. If there are other symptoms such as vomiting or diarrhoea, please refer to these guidance's.	If child is in pain or distress – child not to attend care until well.
Impetigo (school sores)	Per Staying Healthy in Childcare: Impetigo appears as flat, yellow, crusty or moist patches or blisters on the skin, usually in exposed areas such as the face, arms and legs. The sores can be more than 1 cm in diameter. The disease is very infectious. The sores are filled with bacteria, which spread by contact with the sores or infected fluid. Because the sores are usually itchy, people can scratch them and spread the infection, via their hands, to other parts of the body or to other people. The infection can also be spread by touching contaminated clothing or other items.	A Doctor needs to evaluate your child before sending child to care or after being sent home from care. If impetigo, exclusion period applies while infectious. People are infectious for as long as there is fluid weeping from the sores. They are no longer infectious 24 hours after starting antibiotic treatment, or when the sores have healed. If child need to take medication, please complete Medication Records form and discuss with educators.
Itchy scalp	Head lice can cause intense itching. They live and breed on the scalp and are easily passed from child to child.	In the case of head lice, provided that the child has been treated to remove the lice, they will be allowed to return to childcare. If child is distressed or in pain from itchy scalp – child should not attend care. Your local pharmacy will stock special combs as well as shampoos, cream and other products which contain a special insecticide.
Hay Fever	Some of the symptoms can include sneezing, a runny or stuffy nose, itchy ears, nose and throat, red, itchy or watery eyes and headaches.	Doctor to diagnose as Hay Fever and not an infectious disease or other illness. Medical Management Plan for Hay Fever to be prepared by parent and doctor; including any medication instructions (if applicable). If child is unwell, they will be excluded from service until well.
Earache and Ear Treatments	Evaluate along with other symptoms, such as a fever. Common conditions of the ear include infection, inflammation and wax build up and some of these can be quite painful and uncomfortable.	Doctor and/or pharmacist to diagnose. If child is unwell, they will be excluded from service until earache is not present and child's wellbeing is normal. If child is well and no other illness symptoms, able to return care. Children requiring ear drops should otherwise be well enough to attend the service and should not attend if they are unwell, in pain, or experiencing symptoms such as a sore ear, fever, or discomfort requiring ongoing medical attention.

		Ear Drops: The service may administer prescribed ear drops where: - the medication is provided in its original pharmacy packaging, - the child's name and dosage instructions are clearly labelled with pharmacy label, and - a current medication authorisation form has been completed by the parent/guardian. The administration of ear drops remains at the discretion of the service. If the procedure causes significant distress to the child, presents a safety concern, or cannot be safely and appropriately performed by educators, the service may decline to administer the medication and request that the child be collected or alternative arrangements be made.
Respiratory Symptoms and Illnesses	Respiratory symptoms include cough, sneezing, runny or blocked nose and sore throat. It is not unusual for children to have five or more colds a year, and children in education and care services may have as many as 8–12 colds a year. A runny or blocked nose is a common symptom for many respiratory conditions or diseases which may be infectious such as a cold, influenza or COVID. Some causes, however, are not infectious such as allergies (hay fever). As each child may have different symptoms of a respiratory illness, our Service will consider exclusion based on the severity of the symptoms and the child's behaviour. Children can become distressed and lethargic when unwell and should be at home with a parent or carer under close supervision.	A child will be excluded from the Service if: - the respiratory symptoms are severe or; - the symptoms become worse during the course of the day (more frequent or severe) or; - the child has other concerning symptoms (fever, tiredness, pain, poor feeding). (Staying healthy in Childcare).
Sore throat	Sore throats can be caused by either viruses or bacteria. Viral and bacterial throat infections are spread by airborne droplets (coughing and sneezing). They can also spread indirectly by contact with surfaces that have been contaminated by infectious airborne droplets.	Per Staying Healthy in Childcare: - People with a sore throat caused by a virus are infectious for as long as they are coughing or sneezing. This may last several days. - People with a bacterial sore throat can be infectious for 2–3 weeks after becoming ill if they are not treated. If they take antibiotics, they are infectious until 24 hours after starting treatment. A Doctor needs to evaluate your child before sending child to care or after being sent home from care. Doctor to diagnose and provide medical clearance advising not infectious. Exclusion Period: Children with sore throats should stay at home until they are feeling well and not infectious. Children with sore throats caused by bacteria should stay at home until they are feeling well and have had antibiotic treatment for at least 24 hours. If not taking antibiotics; exclusion required until sore throat is cleared and no other illness symptoms.

		If COVID symptoms test for COVID: undergo testing for COVID-19 using a Rapid Antigen Test(RAT) or PCR test). Provide service test results. If positive, see COVID guidance below.
Sore Eyes and Treatments	Due to infection control risks, the potential spread of contagious eye conditions such as conjunctivitis, and the difficulty in safely administering eye medications within the education and care environment.	The service does not administer eye drops unless they form part of an approved medical management plan or emergency treatment requirement. The administration of eye drops remains at the discretion of the service. If the procedure causes significant distress to the child, presents a safety concern, or cannot be safely and appropriately performed by educators, the service may decline to administer the medication and request that the child be collected or alternative arrangements be made.
Please refer to "Staying Healthy in Childcare" guide for other common infectious diseases information.		

Thermometer methods for taking a child's temperature:

Forehead Infrared Sensor Thermometer

- Make sure the child's forehead is clean and dry (no sweat or hair in the way)
- Position: Place the scanner flat in the centre of the forehead
- Follow instruction for device. Slowly slide the thermometer across the forehead to the hairline
- Check the temperature displayed. Record Temperature.
- Note: This method may be **slightly less accurate** than oral measurements.

Axillary – underarm

Best for: All ages (especially infants as a safe first check)

Procedure:

- Place the thermometer in the child's armpit
- Hold the child's arm firmly against their body
- Leave the thermometer in place for approximately 4–5 minutes
- Read and record the temperature.

Thermometer – Oral

- **Best for:** Older children (usually 5+ years)
- **Procedure:**
 - Place the thermometer under the tongue
 - Ask the child to close their lips gently (do not bite the thermometer)
 - Leave in place for approximately 3 minutes
 - Read and record the temperature

Management and Educators will ensure:

- The service is following Department of Health (VIC) guidelines, such as:
 - ensure that unwell children do not attend the service, as per national guidelines (Staying healthy: preventing infectious diseases in early childhood education and care services, 5th edition);
 - isolate children who became unwell during the day from other children and send the unwell child home as soon as possible;
 - exclude the unwell child; and
 - considering actions such as alerting parents or displaying signage.
- A healthy and safe environment is provided to all children attending the centre, excluding children who are sick is a requirement to achieve this outcome.
- Effective hygiene policies and procedures are adhered to.
- Effective environmental cleaning policies and procedures are adhered to.

- All families will have access to relevant policies upon enrolment.
- A child who has not been fully immunised will be excluded from the Service if; an infectious disease is reported within the Service community and that child is deemed to be in danger of contracting the illness. Please refer to our Infectious Diseases and Immunisation Policy – 32.

Parent and Guardian Responsibility

- To prevent the spread of disease, families are required to monitor and inform the service of their child's illness, medical conditions, any changes to health or wellbeing, in particular:
 - Runny, green nose
 - High temperature
 - Diarrhoea
 - Red, swollen or discharging eyes
 - Vomiting
 - Rashes
 - Irritability, unusually tired or lethargic, etc.
- Child should not be provided medication such as Panadol / Nurofen before arrival if child has a fever or another illness. If child is provided medication as part of medical management plan before attending or at service by parent, this needs to be documented on a Medication Form and advised educator team on drop-off.
- Keep their children at home if they are unwell or have an excludable infectious disease.
- Keep their child at home if there is an infectious disease at the service and their child is not fully immunised against it.
- Inform the service if their child has an infectious disease or has been in contact with a person who has an infectious disease.
- Seek advice from medical practitioners to support their children's health and wellbeing. Share this information with the service to assist in making informed decisions.
- Obtain medical clearance before returning to care if their child has an infectious disease and have been excluded from care.

Children arriving at the Service who are unwell

The service will not accept a child into care if they:

- Are unwell and unable to participate in normal activities or require additional attention due to illness.
- Are suspected of having or have an infectious disease or illness.

Children who become ill at the Service

- Children may become unwell throughout the day, in which Management and Educators will respond to children's individual symptoms of illness and enact appropriate first aid and illness and infectious disease management strategies and apply applicable exclusion periods.
- While in care, if child is suspected of being ill, Educators will monitor health and temperature and document the child's symptoms on the Health Record.
- If child is sent home due to illness a 'Incident, Injury, Trauma and Illness Record Form (HEI007)' will be completed by Educator, communicated with family and signed by parent/guardian upon collection.
- Families will be asked to collect their child within 30 minutes (or as soon as practically agreed) of the call or arrange for an authorised nominee to collect. If there is no one able to collect the child within this time the child's condition will be monitored and if worsens an ambulance will be called.
- As part of our Enrolment Terms and Conditions; all families need to have emergency contacts and authorised nominee listed on Enrolment Records to receive calls or collect children (if parent/guardians are unable to be reached or attend the service personally).

Incident, Injury, Trauma and Illness Record Form

- The Incident, Injury, Trauma and Illness Record Form (HEI007) *via the OWNA App* to be properly completed at this time and then must state only what was seen to occur. The form must be filled out as soon as is practicable following the team members first aid response to the situation. Normal protocols for filling out the reports are to be

followed. If the child's illness requires the tracking of symptoms and recording of body temperature please use the Health Record (HEI004).

- The Regulatory Authority must be notified when they believe an illness or emergency meets the definition of serious incident or it poses a significant risk to the health, safety and wellbeing of children. Before reporting, consideration is to be given to the severity and scope of the matter; impact and risk to the health, safety and wellbeing of the child or children and number of children involved. If a serious incident or injury occurs and child needs to seek urgent and immediate medical assistance (i.e. Ambulance or attend hospital), in addition to completing the internal Incident, Injury, Trauma and Illness Record Form, the Centre Director/Nominated Supervisor or Responsible Person in Charge of Service may be required to prepare the ACECQA form on the NQS ITS system "Notification of Serious Incident Form -SI01" and forward to the Approved Provider for review. The reviewed and completed Notification of Serious Incident Form is forwarded to ACECQA within 24 hours of the reportable illness or injury.

Managing Illness and Excluding children from the Service

- In Victoria children's services, such as childcare centres and kindergartens, have a responsibility under the Public Health and Wellbeing Regulations 2019 to help manage infectious diseases in their facilities and exclude certain children for periods as specified in the regulations.
- Services aim to prevent the spread of illness by minimising the transmission of infectious disease from one person to another.
- All children, staff and families at the service benefit from the practice of excluding ill children. Unwell children need additional individual comfort and attention that is difficult to provide in a childcare environment. Ill children can also recover more quickly if they can rest and be cared for at home, ensuring the infection is less likely to spread to other children, families, and staff.
- When a child has been diagnosed with an illness or infectious disease, the Service will refer to Staying Healthy in Childcare and Service policies to find the recommended action and exclusion period. We will request a medical clearance from a doctor stating that the child is cleared to return to the childcare setting.
- Children who are sent home sick are not allowed to return to the centre within time frame advised by National Health and Medical Research Council (NHMRC) or recommendations advised in Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition). The child can only return to the centre when they do not present signs of sickness and will not spread infectious disease to other children. Exclusion periods are made in accordance with NHMRC guidelines and other medical guidelines such as Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition) or specific guidelines issued by the Department of Health or any other regulatory authority.
- When an infectious disease has been diagnosed, the Service will display appropriate documentation and alerts for families including information on the illness/disease, symptoms, infectious period and the exclusion period. (This information can be obtained from Staying Healthy in Child Care 5th Edition)
- Washing hands with soap and water is one of the most effective ways to prevent the spread of infection.

Notifying families and Emergency Contact

- It is a requirement of the Service that all emergency contacts are able to pick up an ill child as quickly as possible.
- In the instance that the ill child is not collected in a timely manner or should parents refuse to collect the child, a warning letter will be sent to the family outlining Service policies and requirements. The letter of warning will specify that if there is a future breach of this nature, the child's position could be terminated.

Returning to care after surgery

- Children who have undergone any type of surgery will need to take advice from their doctor/surgeon as to when it is appropriate to return to care.
- Children will require a medical clearance stating the child is fit and able to return to the Service and participate in daily activities.

Reporting Outbreaks to the Local Public Health Unit

Outbreaks of communicable diseases represent a threat to public health. To prevent outbreaks, it is important to monitor the number of people who contract certain infectious diseases and their characteristics, and to work with patients and their doctors to help prevent spread to other people.

The VIC Public Health Act 2010 lawfully requires and authorises doctors, hospitals, laboratories, school principals and childcare centre to confidentially notify VIC Health of patients with certain conditions, and to provide the information delineated on the notification forms. Specialist trained public health staff review this information and if necessary contact the patient's doctor, and sometimes the patient, to provide advice about disease control and to complete the collection of information.

All information is held confidentially in order to protect the patient's privacy. Both the VIC and Commonwealth Privacy Acts contemplate the release/disclosure of patient information where it is lawfully required or authorised.

Management is required to notify the Local Public Health Units (LPHU):

- Mildura Services: Loddon Mallee Public Health Unit - Mildura Rural City: Phone: 03 5454 6060, Email: info@bendigohealth.org.au. Website: <https://bendigohealth.org.au/LMPHU/>
- East Malvern: Monash Health's South East Public Health Unit (SEPHU)South East Public Health Unit (led by Monash Health)) - (Stonnington City): Phone: 1300 331 981, Website: <https://sephu.org/>

By phone or email as soon as possible (within 24 hours) after they are made aware that a child enrolled at the service is suffering from one of the following vaccine preventable diseases:

- Diphtheria
- Mumps
- Poliomyelitis
- Haemophilus influenzae Type b (Hib)
- Meningococcal disease
- Rubella ("German measles")
- Measles
- Pertussis ("whooping cough")
- Tetanus
- An outbreak of gastrointestinal or respiratory illness

Gastroenteritis outbreaks

With reference to the "A guide for the management and control of gastroenteritis outbreaks in children's centres" (Source: <https://www.health.vic.gov.au/publications/a-guide-to-the-management-and-control-of-gastroenteritis-outbreaks-in-childrens>):

- Service are recommended to notify of an 'outbreak'. An 'outbreak' may be defined as two or more cases of vomiting and/or diarrhoea occurring among children and or/staff with 48 hours of each other. If this occurs and the symptoms cannot be explained by medication or other medical conditions, the centre may have an outbreak. Please note the gastroenteritis must be diagnosed by a medical practitioner.
- Medical practitioners are legally required to notify the Department of Health and Human Services (VIC) if they become aware of cases who may have "Food or water borne illness (two or more associated cases)" and particularly for children who may attend the same children's services centre. Currently there is no legal requirement for children's services centres to notify the Department of Health and Human Services of an outbreak of gastroenteritis.
- Notification of an outbreak of gastroenteritis by a children's services centre is **strongly recommended** as the department and local government can provide advice and support in managing outbreaks to minimise the severity and duration of illness, particularly in children.

- If you suspect you have a gastro outbreak, the first step is to notify the Department of Health and Human Services, Communicable Disease Prevention and Control on 1300 651 160 within 24 hours. The department officer will collect information on the number of cases, symptoms, duration of illness and other details, and can discuss any issues you may have and provide advice if necessary. Based on the information you provide, the officer will assess the probable cause of the outbreak and the way in which it is likely to spread.
- Once an outbreak of gastroenteritis has been identified, it is essential that cleaning and infection control measures are implemented immediately to reduce the risk of the infection spreading and the number of cases increasing as per the attached document. The centre will use 'A guide for the management and control of gastroenteritis outbreaks in children's centres' as reference and follow the steps detailed in the guide.
- Clean-up and control measures must be implemented for all gastroenteritis outbreaks as soon as possible after an outbreak is suspected and must continue until the outbreak has been confirmed as being over (48 hours after symptoms have ceased in the last case – no further cases of illness occurring).

Outbreak of an Infectious Disease

Services should notify the Local Public Health Unit if an outbreak of an infectious disease that is preventable by vaccine occurs. The service will collect and keep up to date records of every child's immunisation status. Records must also be kept of children who have not received any vaccinations. Family members have a responsibility to tell the centre if their child develops a vaccine preventable disease.

If this occurs, the following should happen:

1. The child's parent/guardian notifies the service that their child has been diagnosed with a disease that can be prevented by vaccine.
2. The Centre Director/Responsible Person in Charge of Service will call the Local Public Health Unit (see details above)

The Centre Director may need to provide the following information:

- Child's Full Name
- Date of Birth
- Address
- Contact Details
- A list of children who are unimmunised and their details.

3. The service will follow instructions of the Local Public Health Unit and display information in the service on the infectious disease.

A list of infectious diseases and exclusion periods is available online.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 15/5/2026

Policy Next Review: 15/5/2027

Reference:

NQS

QA2	Children's Health and Safety	
	2.1.1	Each child's health needs are supported.
	2.1.4	Steps are taken to control the spread of infectious diseases and to manage injuries and illness, in accordance with recognised guidelines.
	2.2.1	Healthy eating is promoted and food and drinks provided by the service are nutritious and appropriate for each child.
	2.3.1	Children are adequately supervised at all times.
	2.3	Each child is protected.
	2.3.2	Every reasonable precaution is taken to protect children from harm and any hazard likely to cause injury.
	2.3.3	Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	85	Incident, injury, trauma and illness policies and procedures
	86	Notification to parents of incident, injury, trauma and illness
	87	Incident, injury, trauma and illness record
	88	Infectious diseases
	89	First aid kits
	90	Medical conditions policy
	92	Medication record
	93	Administration of medication
	97	Emergency and evacuation procedures
	161	Authorisations to be kept in enrolment record
	162	Health information to be kept in enrolment record

Other References:

- National Health and Medical Research Council: *Staying Healthy: Preventing infectious diseases in early childhood education and care services* (5th edition)
- Australian Government National Health and Medical Research Council (2012)
- 'A guide for the management and control of gastroenteritis outbreaks in children's centres' – November 2017
- Starting Blocks.gov.au "Illness in Early Learning Services" <https://www.startingblocks.gov.au/other-resources/factsheets/illness-in-childrens-education-and-care-services>
- ACECQA Policy and Procedure Guidelines "Dealing with Infectious Diseases" August 2021
- "Too Sick for Daycare?" Think Pharmacy First - Ask Your Pharmacist Fact Sheet.
- Public Health and Wellbeing Regulations 2019 (Vic)
- Victorian Public Health and Wellbeing Act 2008
- Public Health and Wellbeing Regulations 2010, Schedule 7
- Handbook on Child Care Licensing
- Guide to the National Quality Framework
- Standards Australia
- Food Safety Legislation 2001
- Occupational Health and Safety Act 2004
- CCMS – Federal Funding Guidelines
- Occupational Health & Safety Act 2004

32 – INFECTIOUS DISEASES AND IMMUNISATION POLICY

Position Statement

The occurrence of many infectious diseases has been greatly reduced and general health within the community has been improved due to immunisation. When immunised, the body is more able to resist infection from the disease against which the person has been immunised.

Related Policies

Excursion Policy
Fees Policy
Nutrition, Food, Beverage & Dietary Requirements Policy
Health & Hygiene Policy
Health and Wellbeing Policy
Illness Policy
Incident, Injury, Trauma and Illness Policy
Infectious Diseases and Immunisation Policy
Medical Conditions Policy
Interactions with Children Policy
Rest Time/Sleep Policy
Occupational Health & Safety Policy

Objective

To provide information about infectious diseases and immunisation to families and team members and to encourage team members and children to follow recommended immunisation schedules. Effective infection control reduces the spread of illness through practices and also notifying of occurrences as soon as possible.

Implementation

The National Immunisation Program (NIP) Schedule can be accessed and downloaded from <http://www.immunise.health.gov.au/>

Department of Health, Victoria

Immunisation Program
<http://www.health.vic.gov.au/immunisation/>
Telephone - 1300 882 008

Immunisation Records

- Parents who wish to enrol their child are required to provide a copy of a current Australian Childhood Immunisation Register (ACIR) Immunisation History Statement which shows that the child is fully immunised for their age in line with the National Immunisation Schedule.
- Parents are required to present the child's immunisation record at the time of enrolment and also at each stage of the immunisation schedule. Please upload the ACIR Immunisation History Statements in OWINA and complete/update by ticking the Vaccination period in OWINA. Please re-submit the online enrolment form on completion.
- This information allows children at risk of catching a vaccine preventable disease to be identified if there is a case of that disease at the service. It is the responsibility of parents to consult their doctor and maintain vaccines for their children.
- Acceptable immunisation records are –
 - An Immunisation History Statement provided by the Australian Childhood Immunisation Register (ACIR) is a valid immunisation record.

- The Australian Childhood Immunisation Register (ACIR) maintains immunisation records for children up until their seventh birthday. You can obtain an ACIR Immunisation History Statement for your child by calling 1800 653 809 or email acir@medicareaustralia.gov.au or Medicare (including the App).
- The original immunisation record is usually kept in the personal health record book. These books are usually given to parents at the time of their child's birth.
- Each child's Immunisation Record should stay intact until your child reaches primary school. Do not remove any of the duplicate pages until this time.
- Parents need to provide the childcare service or preschool with a photocopy of their child's original immunisation history statement.
- The service must be able to quickly access immunisation records and determine who has not been immunised. If there is a case of a vaccine preventable disease, and your child has not been fully immunised for that disease, they may be excluded from school or childcare for a period of time.

Catering for Children with Overseas Immunisation Records

- Overseas immunisation schedules often differ from the schedule recommended in Australia and a child may require extra vaccinations to be up to date with the Australian schedule.
- Parents are responsible for having their child's overseas immunisation record transcribed onto the Australian Childhood Immunisation Register (ACIR), if your child is less than seven years of age.
- A medical practitioner, registered nurse, registered midwife, enrolled nurse, or an individual authorised by the state/territory Health Officer may transcribe overseas immunisation records.

The service may enrol the following children without proof they are fully immunised under a grace period. The families of these children must take action to fully vaccinate their children, and provide proof of immunisation status within 16 weeks of the child's first attendance at the service:

- children evacuated from their place of residence due to an emergency (eg bushfire)
- children in emergency care within meaning of section 3(1) of the Children, Youth and Families Act 2005
- children in the care of an adult who is not their parent due to exceptional circumstances
- Indigenous children
- children whose parents hold a health care card, pensioner concession card, Veteran's Affairs Gold or White card
- children from multiple births of triplets or more
- other circumstances specified in the guidelines made by the Secretary to the Department of Health and Human Services

If evidence of immunisation status is not provided within 16 weeks of the child's first attendance at the service, the child's enrolment at the service may be cancelled. We will assist families with information about accessing immunisations and obtain required documentation.

Exclusion Periods

- If a child's immunisation record is not provided upon enrolment and if it is not updated by the parents, or if the child has not been immunised against certain diseases, then the child will be recorded as being not fully immunised by the service.

- If there is a case of a vaccine preventable disease at the service, your child may be excluded from school or childcare for a period of time or until the evidence of immunisation in an approved record is provided.
- To be fully immunised your child needs to have received all vaccines recommended for their age as part of the National Immunisation Program (NIP).
- A medical doctor determines if a child has a medical reason for not receiving a specific vaccine. This is known as a medical contraindication.
- Homeopathic immunisation is not recognised.
- If a child is not fully immunised and has been in contact with someone with a vaccine preventable disease outside of the service, they may need to be excluded from the service for a period of time.
- Please note childcare fees are payable for this period of time their child is excluded.
- It is the responsibility of families to inform the service that their child has come into contact with someone with a vaccine preventable or infectious disease.

As outlined in the Public Health and Wellbeing Regulations 2009 (Vic):

- parents must advise family day care educators as soon as possible if a child has an infectious diseases or the child has been in contact with a person infected with an infectious disease. These diseases and the minimum periods of exclusion are listed in Schedule 7 and can be accessed on-line at www.legislation.vic.gov.au
- service to inform the parents and the Secretary of the Department of Health within 24 hours of forming the belief that a child is suffering from pertussis, poliomyelitis, measles, mumps, rubella or meningococcal C.
- Service not allow a child who has been infected or had contact with the diseases outlined in Schedule 7 to the Regulations to attend the service for the exclusion periods outlined in Schedule 7.

Infectious Diseases requiring Notification to the local Public Health Unit

Infectious Diseases only require notification from doctors and laboratories. A list of diseases can be found on following form –

[http://docs.health.vic.gov.au/docs/doc/58118E0B95087FE0CA2578BC0008CCA3/\\$FILE/notifiable_conditions_form.pdf](http://docs.health.vic.gov.au/docs/doc/58118E0B95087FE0CA2578BC0008CCA3/$FILE/notifiable_conditions_form.pdf)

Immunisation for Educators

- The pandemic orders require all staff of the service to receive a third dose of the COVID-9 vaccine in order to be onsite in an education setting. Getting a third dose will lower the risk of COVID-19 outbreaks and reduce the risk of exposure to COVID-19 in Early Childhood Education and Care (ECEC) services.
- It is important that educators remain up to date with their vaccinations in order to protect themselves as well as children in their care. The National Health and Medical Research Council (NHMRC) recommends that individuals who work with children, including child care and pre-school staff (including child care students) and outside school hours carers, should be vaccinated against pertussis (whooping cough), hepatitis A, measles, mumps and rubella (MMR), varicella (chickenpox), and influenza (required annually).
- The service will take all reasonable steps to strongly encourage non-immune workers to be vaccinated.

- Occupational recommendations apply for the immunisation of educators at the service. As there are no mandatory requirements under the law for educators to be immunised, the service must follow the following requirements that our service has developed:
 - The Exclusion Periods requirements above apply to all educators.
 - Educators who are not immunised maybe excluded from the service until the Nominated Supervisor or Owner provides instructions for the educator to return to education and care duties at the service (this will be based on advice from the educators medical practitioner).
 - Further, Educators who are not immunised may use their best judgement to decide whether they exclude themselves from the service during an outbreak of an infectious disease. This decision needs to be supported by a medical practitioner certificate advise the reasons and the risk associated.

The National Health and Medical Research Council (NHMRC) recommend that educators should be immunised against -

- Hepatitis A.
- Measles-Mumps-Rubella (MMR).

Educators born during or since 1966 who do not have vaccination records of two doses of MMR, or do not have antibodies for rubella, require vaccination.

- Varicella if they have not previously been infected with chickenpox.
- Pertussis (whooping cough). An adult booster dose is especially important for those educators caring for the youngest children who are not fully vaccinated.
- Influenza (annually)
- Hepatitis B if caring for unimmunised children with intellectual disabilities (although the risk is low).

Our service will:

- regularly provide educators and staff with information about diseases that can be prevented by immunisation through in-service training sessions, fact sheets and the Staying Healthy in Childcare publication.
- regularly communicate with educators and staff that some infectious diseases may injure an unborn child if the mother is infected while pregnant through in-service training sessions, fact sheets and the Staying Healthy in Childcare publication. These infections include chickenpox, cytomegalovirus and rubella (German measles).
- ask new employees to confirm in writing that we have provided this information during their induction.
- encourage all non-immune staff to be vaccinated (staff to consult their medical practitioner on personal medical treatment).
- encourage female educators / staff who are not fully immunised to consider doing so before getting pregnant.
- communicate with pregnant educators and staff to review the Staying Healthy in Childcare publication and consult their medical practitioner to consider the risks of continuing to work at the service.
- encourage pregnant educators and staff to follow good infection control and hygiene procedures. Educators and staff should consult their medical practitioner regarding this matter and the risk associated.
- allow educators who are not immunised to use their best judgement to decide whether they exclude themselves from the service during an outbreak of an infectious disease.

Immunisation Related Payments for Parents - Child Care Subsidy

The benefit applies to children who are fully immunised or have an approved exemption from immunisation. This initiative ensures parents are reminded of the importance of immunising their children at each of the milestones. For parents to receive benefits without their child being fully immunised your healthcare provider needs to certify that:

- is on a catch-up immunisation schedule or
- has an approved exemption from the immunisation requirements. Approved exemptions include medical contraindications and existing natural immunity but do not include conscientious objection.

Parents are responsible for payment of fees while their child is excluded under all circumstances.

Reasonable steps are to be taken to prevent the spread of infectious diseases at the service. Refer to policy 11 - Illness Policy, 43 - Exclusion Policy and Staying Healthy: Preventing infectious diseases in early childhood education and care services (6th edition) for guidance on notifications and recommendations dealing with infectious diseases. Please note the service may also take into consideration all children and employees health and wellbeing. The service aims to apply best practices and procedures for managing infectious diseases. The service may go beyond the recommendations of 'Staying Healthy' to manage the containment and spreading of the infectious diseases and take into consideration the services' environment, the age of children, nature of disease/s, etc to manage the infectious disease.

Current Immunisation Schedule will be displayed in the service - The National Immunisation Program (NIP) Schedule

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/05/2026

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	77	<i>Health, hygiene and safe food practices</i>
	88	<i>Infectious diseases</i>
	90	<i>Medical conditions policy</i>

	162	Health information to be kept in enrolment record
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Other References:

Public Health and Wellbeing Act 2008

Handbook on Child Care Licensing

Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)

Anti-Discrimination Act

Public Health and Wellbeing Act 2008

CCB – Federal Funding Guidelines

Services Public Health (Amendment) Act 1992

Sources and Further Reading:

- Australian Government – Department of Health and Ageing. Immunise Australia Program.
- Australian Childhood Immunisation Register. Australian Government, Medicare.
- Department of Health and Aging, National Immunisation Program Schedule NHMRC.
- Medicare Australia

33 - MEDICATION POLICY

Position Statement

Medicating young children is an enormous responsibility. To safeguard the child, the team members, and the centre, strict guidelines need to be set to preclude any confusion or dosage errors.

Objective

To have guidelines that will ensure uniform safe handling of the medication of children.

Definition

Medicine: under the Commonwealth Therapeutic Goods Act 1989, means therapeutic goods that will or are likely to achieve a result by pharmacological, chemical, immunological or metabolic means. Therapeutic goods include things that may prevent, cure or alleviate a disease, ailment or injury. Regulations apply to a service for the administration of medication. This includes all over the counter and prescription medications.

Implementation

- Unless emergency situations arise, children will not be medicated without written directions from parents or legal guardians, accompanied by a medical practitioner's instructions in writing.
- It is the parent's responsibility to keep the centre abridged of changes to medication plans in writing.
- Form HEI001 - Medical Management, Risk Assessment and Communication Plan forms are required to be completed for children with medical conditions (Asthma, Diabetes, Allergy, Anaphylaxis and other long term illnesses and diagnoses). Please refer to relevant specific service policy for procedures managing these medical conditions. Without a current Medical Management, Risk Assessment and Communication Plan for these conditions a child is not allowed to attend or commence at the service.
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.
- Medication Authority Form and Record will be completed for children with other long term medication regimes.
- The provision of Panadol and Nurofen medications will be treated as all other medicines. The purchase of over the counter medicines with a Pharmacist or Chemist label does not constitute a prescribed medication. It is only a prescribed medication when a Doctor or Medical Practitioner has prescribed the medicine and directed the Pharmacist to procure the medicine from a Doctors' prescription. Medicine will only be administered if written instructions are provided by a Doctor.
- Our service does not administer over the counter medication unless it has been prescribed by a registered medical practitioner and on the medication label (prescription or pharmacy) explaining the purpose of the medication or there is a letter from the doctor. Medication may mask the symptoms of other, more serious illnesses and our educators are not qualified medical professionals.
- All prescribed medications will be labelled with the name of child, the frequency and dosage of medicine to administer, the date, period of time the medicine is administered and the Doctor's name who prescribed the medicine.
- We will administer nappy cream, mosquito repellent and sunscreen creams without prescription or pharmacy label if a parent or authorised person authorises this using the OWNA App form "Ongoing/Non-prescribed Medication/Cream Form", (Paper form HEI009 – Authority to Administer of Nappy Cream, Sunscreen Cream and Mosquito Repellent (Medication Form)) or on the Enrolment Form. Please note this only applies

to the nappy creams products named on this form. Authorisation to apply sunscreen and mosquito repellent supplied by the service is completed via the enrolment form. Sunscreen and Mosquito Repellent supplied by parents is authorised using the OWNA App form: 'Ongoing/Non-prescribed Medication/Cream Form' or the 'HEI009 – Authority to Administer of Nappy Cream, Sunscreen Cream and Mosquito Repellent (Medication Form)'.

The 'Ongoing/Non-prescribed Medication/Cream Form' is to give permission for the administration of Nappy Creams, Sunscreen Creams and Mosquito Repellents by the service. This form is not for the use for Prescription Medications.

Only the following creams and repellents are allowed on this form. If you wish to use another cream or spray, please complete the form "Medication Authority Form and Record". See OWNA Parent App: Medication Record.

Approved Nappy Rash Creams: The following nappy rash creams are the only ones permitted for use at our service due to the risk of nut products, which may cause anaphylaxis and allergic reactions.

- Sudocrem Baby Cream
- Lucas Papaw Ointment Nappy Cream
- Destin Nappy Ointment
- Johnsons Baby Nappy Cream
- Moo Goo Nappy Balm
- Danktozin Ointment – Nappy Rash
- Vaseline Petroleum Jelly
- Skin Basics – Soothing & Protection: Zinc & Castor Oil Cream
- Nappy-Goo Cream
- Mustela Nappy Rash Cream
- QV Baby Moisturising Cream
- Mongdies Soothing Gel

Sunscreen Cream using this form: Any sun protection product with an approved label for use by children.

Mosquito Repellent Sprays and Creams using this form: Product has an approved label for use by children. For example: Aero Guard Odourless Protection – 12 months of age and over. Other products accepted.

(Parent needs to review recommended age of child for use of product and provide permission for service to use).

The cream, spray and repellent is required to be in its original container bearing the original label with your child's full name on it and within the expiry date. Please do not cover product instructions.

Medications

There are 2 criteria of medications within our service:

1. Nappy Cream, Mosquito Repellent & Sunscreen Cream applied to skin. We only allow the named nappy creams products listed above to be authorised by a Parent. Please note if the nappy cream product is not listed on the form then it is treated as a Medication Cream (please follow procedure below – item 2 or 3). When nappy cream is applied to a child's skin it will be recorded on the nappy change schedule or child daily routine schedule/book. Please note authorisation for sun screen and mosquito repellent supplied by the service is provided by the parent using the child enrolment form or the form. For sunscreen cream and mosquito repellent product supplied by parents for their children the form: HEI009 – Authority to Administer of Nappy Cream, Sunscreen Cream and Mosquito Repellent (Medication Form) must be completed by the parent.

2. All other medication is required to be labelled with a medication prescription or a medication pharmacy label issued by a registered medical practitioner (doctor or pharmacist). The label must have the child's name, name of medication, application method / dosage and the purpose of medication. Medication Authority Form & Record is required to be completed and signed by a parent. The administration of medication of this nature will be recorded on the Medication Authority Form & Record; record section.

The service may require a parent to obtain a letter from a registered medical practitioner providing further information on the medication before being administered to a child.

The above procedure is due to the regulation requirements and also risks associated with all medications. Further, it is widely understood that medications may mask the symptoms of other, more serious illnesses and our educators are not qualified medical professionals. The service policy supports our aim of providing a safe environment for all children, parents and educators.

Procedure

- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.
- A Medication Authority Form & Record and Authority to Administer of Nappy Cream, Sunscreen Cream and Mosquito Repellent (Medication Form) will be kept in each room of the centre or stored in OWNA, will be completed and signed by the parent.
- Medication will be handed to a team member by parent and guardian.
- All medication dosages are to be given by Room Leader, Qualified Educator or the Centre Director/Responsible Person who can identify the child with 100% certainty, and witnessed by another team member. Relief team members are NOT permitted to medicate children.
- Non-prescription teething gels, mosquito repellents, sunscreens cream and nappy rash creams can be applied by Educators (Certificate 3, Diploma, Bachelor).
- Before Room Leader or qualified team member provides medication it must be witnessed by another Educator (Qualified or Assistant Educator).

The team member (Administer and Witness) will check that:

- the medication to be given has been prescribed by a Medical Practitioner
 - the medication is labelled for the child it is intended for
 - the medication is in the original container in which it was dispensed
 - the expiry date on the medication is correct.
 - The dose recorded on the Medication Record, and the dosage indicated on the medication must match.
-
- At no point will medication be given in larger doses than that prescribed on the medication.
 - The service reserves the right to contact a health care professional if team members are unsure about administering medication to a child, even if the parent or legal guardian has requested the medication to be administered.
 - All medication will be locked away in a child-safe manner where it cannot be accessed by children.
 - The medication record will be referred to throughout the day to ensure that medication is given on time.
 - The medication record is to be completed by the team members administering the medication.
 - Room Leader, Qualified educators or Responsible Person will record on the Medication Authority Form & Record the date and time of dosage administered, etc each time medication is administered.
 - Medication will be returned to storage in a child-safe manner where it cannot be accessed by children after administration.

- Parents are expected to collect medication from the child-safe storage when they pick up their children each day.
- At the end of the day, educators are required to inform parents to read the medication form and sign it in acknowledgment of reading the form.

NB. Medication must not be left in children's bags or placed into bottles of milk.

➤ **Team members who become aware of parents doing this will advise the Director and the parents will be re-advised of our centre policy regarding this practice.**

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/10/2025

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement - anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

Handbook on Child Care Licensing
QIAS Principle 5.1, 5.2, 5.3
Standards Australia
Privacy Act 1988
Policy Handbook – P004. 7.2026

Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2004
CCB – Federal Funding Guidelines
Drug Regulations

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)
Poisons and Therapeutic Goods Act 1996
Public Health and Wellbeing Act 2008
Occupational Health & Safety Act 2004

34 - FIRST AID POLICY

Position Statement

Working with young children will present occasions when First Aid is required. Young children can have allergic reactions to topical applications such as creams or band aids.

Objective

To ensure that the service maintains the necessary resources and appropriately skilled team members to ensure that child and team member first aid needs are dealt with.

Related Policies

19 - Incident, Injury, Trauma and Illness Policy
32- Infectious Diseases and Immunisation Policy
31 - Illness Policy
33 - Medication Policy
34- First Aid Policy
35 - Anaphylaxis Policy
36 - Asthma Policy
85 - Diabetes Policy
87 - Medical Conditions Policy
91 - Epilepsy Policy
92 - Health and Wellbeing Policy
94 - Cystic Fibrosis Policy
98 - Coronavirus Policy
1- Fee Policy

Implementation

- The Service will ensure that the regulatory requirements for team members on premises with current first aid certificates are met. Please refer to state regulations for the centre you are working in to verify this requirement.
- The Service will maintain records of team member first aid certificates to ensure that should first aid be required, team members will have the current knowledge and skills to deal with the situation.
- It is recommended that all Educators refresh their first aid knowledge and devices (Eg. Epi-pens).
- The Service will maintain a working first aid kit in a clearly labelled child proof container to meet regulatory requirements and best practice.
- The first aid kit will contain sufficient first aid materials to cater for the likely first aid needs of the children or team members at the centre, taking into consideration the licensed capacity of the centre.
- All applications of any form of first aid will be fully documented via Incident, Injury and Trauma report procedures.
- First Aid Qualifications (Regulation 136) the service must at all times when children are being cared for by the service – (a) at least one educator who holds a current approved first aid qualification; (b) at least one educator who has undertaken current approved anaphylaxis management training; (c) at least one educator who has undertaken current approved emergency asthma management training.

Team members are expected to hold a current first aid certificate. Maintaining a current first aid certificate is the responsibility of the employee. Failure to meet this legal requirement for team members may make them ineligible to work within the Centre will mean that employment will be suspended until such time as the certificate can be produced.

Procedure

- The Nominated Supervisor or Responsible Person in Charge is the appointed First Aid Officer for the service and is responsible for coordinating the first aid response to serious incidents, ensuring appropriate medical assistance is obtained, notifying families where required, and completing all required reporting and regulatory notifications.
- Qualified Room Leader team members will assist in caring to children and team members as required. They will provide immediate assistance as required until the Nominated Supervisor, Responsible Person in Charge or emergency services assume responsibility. A Child/Team Member Incident report (or an Incident, Injury, Trauma and Illness Record Form (HEI007)), (HEI007 or via OWNA) must also be completed at this time.
- Staff members delivering first aid in all situations apply the **DRSABCD Action Plan**.
- First aid is the help you give to someone straight after they have a sudden injury or medical episode. It is best to give first aid until the person gets better or you pass their care to paramedics.
The main goals of first aid are to:
 - preserve life
 - stop an illness or injury from getting worse
 - relieve pain, if possible
 - help recovery
 - support the person until paramedics arrive.
- The type of first aid care needed depends on a person's condition. It may be as simple as getting water for someone who feels faint or as serious as giving cardiopulmonary resuscitation (CPR) to someone having a heart attack.
- The Director or other authorised person must inspect the first aid kit once a month and record the contents in order to place any necessary first aid orders.

What is DRSABCD?

(Source: https://www.rch.org.au/kidsinfo/fact_sheets/first_aid/)

Extract:

DRSABCD is an acronym that helps you remember the basic first-aid steps. It stands for 'Danger, Response, Send (for help), Airway, Breathing, CPR, Defibrillation,' – seven actions to take when looking after someone during a medical incident. If it is a minor incident, you may only need to follow a few steps. The DRSABCD steps are designed to help everyone, from babies to older adults.

Call an ambulance Triple Zero (000) immediately if someone is having a medical emergency.

- **D** stands for danger = Check your surroundings to ensure there are no dangers to you or the person who needs help. For example, remove any pets from the area.
- **R** stands for response = See if the person is conscious and responsive; do they reply if you speak to them or respond if you touch their shoulder? If it is a baby, try pinching their earlobe to see if they respond.
- **S** stands for send for help = Call an ambulance (000) or ask someone nearby to call. You should stay on the phone until help arrives. It is best to put your phone on speaker when you dial, so you can use your hands to give first aid.

- **A** stands for airway = Check if anything is blocking the person's mouth or throat, and whether they are breathing.
 - If they are conscious and their airway is clear, ask how you can help them.
 - If they are conscious but there is something blocking their airway, it is best for them to stay in the same position and try to clear the blockage. For example, it may help them to sit up and cough.
 - If they are unconscious and there is nothing blocking their airway, gently tilt their head back into a neutral position and lift their chin. Try to keep their head tilted back into a neutral position so their airway is as open as possible.
 - If they are unconscious and there is something blocking their airway, roll them onto their side and try to clear the blockage. If you clear the blockage, roll them onto their back, then gently tilt their head back and lift their chin to check their breathing. Try to keep their head tilted back into a neutral position so their airway is as open as possible.
- **B** stands for breathing = See if the person is breathing. You should watch, listen and feel for 10 seconds. If they are breathing, place them in the recovery position (details below) and keep watching them. If they are not breathing and unresponsive, start CPR immediately.
- **C** stands for CPR = To perform CPR, give 30 chest compressions followed by two breaths. [Watch the videos below](#) for a demonstration. If you do not want to give breaths, continue with chest compressions. Only stop CPR:
 - to swap with another person
 - to use a defibrillator
 - when paramedics take over
 - if the person starts breathing
 - if you are physically unable to continue.
- **D** stands for defibrillator = If the person is not breathing and is unconscious, start using an automated external defibrillator (AED) if available. The machine gives an electric shock to get the heart beating normally again. Defibrillators are very easy to use; most will give verbal instructions, so you just need to follow what they tell you to do.

Recovery position

The recovery position is a way to keep someone's airway clear and open after a medical incident, excluding injuries to their spine. It also helps to stop someone from choking on any fluid or vomit.



Figure one: Two demonstrations of recovery positions – left for babies under one year, right for adults. Images have been reproduced with permission from [St John Ambulance](#).

Babies under one year

There are two safe ways to put a baby into the recovery position

- Lie the baby face down on your forearm and support their head with your hand.
- Place the baby on their side, with their head angled so their mouth faces down. Try to keep them stable in this position; you can put a small pillow or rolled-up blanket behind their back, so they do not roll backwards.

Children over one year and adults

1. Kneel beside the person and make sure they are lying on their back. Then, fold the arm closest to you across their chest. Place the other arm directly out from their body.
2. Bend their leg closest to you so their foot is flat on the ground.
3. Gently support their neck and roll their body towards their outstretched arm so their bent knee rests on the ground and they are on their side. This position will stop them from rolling onto their face.

Do not put someone in the recovery position if you think they could have a spinal injury. Someone may have a spinal injury if they:

- fell from a height
- were in a car or bicycle crash or hit by a car
- were hit directly in the back, neck or head
- complain of severe pain in their neck or back
- are unable to move their neck
- feel weak, numb or unable to move
- have lost control of their limbs, bladder or bowels.

Choking, Suffocation and Strangulation Prevention and First Aid

(Source:

https://www.rch.org.au/kidsinfo/fact_sheets/Choking_suffocation_and_strangulation_prevention/)

Extract:

Young children are naturally curious and are always exploring their environment. From as early as two months of age, babies start putting things in their mouths. Touching, feeling and mouthing any small item within reach is how babies and young children learn about their surrounding environment.

Babies and toddlers have small airways that are easily blocked. In addition, their reflexes are not well developed and their bodies are not very strong, so they may have difficulties getting themselves out of trouble.

If a child is choking or having trouble breathing, call an ambulance immediately.

Common hazards

There are a number of items in and around the home that present a choking, suffocation or strangulation risk to infants and young children.

Food

Certain foods can be dangerous for young children because they are easily inhaled and block the breathing tubes. These include nuts (not permitted in childcare service), raw carrots and other hard vegetables, pieces of apple, popcorn, corn chips, lollies and grapes. Children under the age of three years may not have their full set of teeth and can't chew properly, so any food that is small and firm is a choking hazard.

- Ensure young children sit quietly while eating or drinking.
- Never force young children to eat, as this may cause them to choke.
- Never give whole nuts (not permitted in service) to children under five years of age (smooth nut butters are OK, if your child is not allergic to them).
- Carrots, apples and other hard fruit and vegetables should be cooked, mashed, peeled or grated.
- Grapes should be cut into half lengthways, then into half again.
- Meat should be minced or cut into small pieces and served without bones.
- Hold baby while they drink from a bottle – do not leave them alone with a bottle propped up.

Small objects

Any small object can choke children under three years. Review environment for these objects. Make sure that young children cannot reach or play with:

- needles, pins or safety pins.
- coins, small magnets and small batteries – make sure that any toys containing batteries have the battery compartment lid screwed on tightly.
- buttons, beads, marbles, the tops of ballpoint pens and polystyrene beads (found in stuffed toys and bean bags), which are all easily inhaled.

Toys and play equipment

Choose age-appropriate toys that are well made, as less sturdy toys can break easily into small parts. Avoid toys that have small parts, especially if they can be removed. Review environment for these objects.

- Don't allow your baby or young child to have access to toys smaller than a D-sized battery (e.g. marbles, small building blocks or small bouncy balls).
- When outdoors, make sure that young children are always supervised on rope swings, as these can present a strangulation hazard.
- Do not attach cords, string or ribbons to dummies and make sure hanging mobiles are out of reach.
- Do not allow young children to play with balloons unsupervised as popped or uninflated balloons can be inhaled into the lungs. Long strings on balloons are also dangerous.

Curtain and blind cords

Young children can get caught in dangling curtain cords, which can strangle them if curtain and blind are not installed correctly.

Plastics

Plastic bags, dry cleaning bags and plastic wrap are especially dangerous for young children and not permitted in child environments. A child can easily suffocate if these items are pulled over their head. Review environment for these objects.

Prams

Do not leave a sleeping baby unsupervised in a pram. Babies can become trapped and suffocate.

Clothing

Be aware that cords and drawstrings on clothing such as parkas and hooded windcheaters can catch on play equipment.

- When choosing clothing, avoid ties, ribbons or crocheted jackets that may pull tight.
- Always remove a baby's bib before putting them down to sleep.
- Avoid necklaces and other jewellery that can get caught.

Choking first aid

It is recommended anyone caring for children should do a first aid course and learn infant and child cardiopulmonary resuscitation (CPR) in the case of an emergency.

If a child is choking and still breathing

In many cases, coughing will help dislodge the object.

- Encourage the child to cough. Infants and young children may cough instinctively.
- Stay calm and reassure the child.
- Continue to monitor for breathing until they recover.

If a child is choking and not breathing

If coughing doesn't help dislodge the object, or the child stops breathing at any time, call an ambulance. Follow the first aid steps outlined below.

- Infants (less than one year old) should be placed face down, across your knees and held with their head lower than their chest.
- Older children can sit and lean forwards if they are able; otherwise lie them on the floor on their side (recovery position).

Once the child is positioned appropriately:

1. Give five sharp back blows with the heel of your hand between the shoulder blades. In between each back blow, check to see if the object has been dislodged and if the child is breathing.
2. If the back blows are unsuccessful, turn the infant onto their back. If an older child is lying down, also turn them onto their back.
3. Give five chest thrusts using two fingers (infant) or the heel of your hand (child), keeping your hand in contact with the chest at all times. For a chest thrust, position your hand or fingers over the same area you would use for CPR compressions (the lower half of the sternum or 'breast bone').
4. After each chest thrust, check to see if the object has been dislodged.
5. If the blockage has not cleared, continue to alternate between five back blows and five chest thrusts (steps 1 to 4 above) until the ambulance arrives.

Do not use the Heimlich manoeuvre (forceful squeezing of the abdomen) at any time, as this can cause serious damage to internal organs.

If at any time the child becomes unconscious, call an ambulance and start CPR.

Poisoning - First Aid

(Source: https://www.rch.org.au/kidsinfo/fact_sheets/Safety_Poisoning_prevention/)

Extract:

Poisoning can happen to anyone, anywhere, but children are particularly at risk as they enjoy exploring and putting things in their mouth as part of their normal development. Poisoning is most common in children aged five years and under, with those aged between one and three years at greatest risk.

What is a poison?

Any household product or medicine used incorrectly can be a poison. Poisons include:

- drugs or medicines
- cleaning products (detergents, bleaches, drain cleaners)
- cosmetics
- other chemicals such as glues
- poisonous plants.

Poisoning can occur if the substance is swallowed, inhaled, spilt on the skin, splashed into the eye or injected.

First aid for poisoning

Call an ambulance immediately if child collapses, stops breathing or has difficulty breathing, or is having a fit.

Do not ring the Poisons Information Centre in a medical emergency. Call Triple Zero (000)

If you suspect a child has been exposed to poison, do not try to induce vomiting. Do not wait for symptoms to occur. Take the child and the container with you to the phone immediately and **call the Poisons Information Centre on 13 11 26** (24 hours a day in Australia). They will be able to give you first aid advice. It is helpful to know what the substance was and how much of the substance was taken.

Burns and scalds – First Aid

(Source: https://www.rch.org.au/kidsinfo/fact_sheets/Burns_prevention_and_first_aid/)

Extract:

What are burns and scalds?

Burns and scalds are injuries to the skin and other tissues. They are a leading cause of injury to young children, with toddlers most at risk because of their increased mobility and natural curiosity.

The main causes of burns are:

1. Scalds caused by hot liquids, foods or steam
2. Contact with hot objects
3. Flames
4. Friction
5. Chemical, such as burns from household cleaning products
6. Electrical, such as mouth burns from chewing on power cords.

There are also burns caused by very cold objects, such as dry ice, and radiant burns, like [sunburn](#).

One of the most common ways children get burns is by pulling hot foods and drinks off tables. This can cause severe burns on their face, neck, chest and shoulders.

All burns can be serious. Even small burns can have [long-term effects](#) on a child, depending on where they are on the body.

Types of burns

Burns are grouped into four categories based on how deep they are.

Superficial burns (first-degree)

- Mild, shallow burns
- Only affects the top layer of the skin
- Often causes mild pain and redness
- Sunburn is a common superficial burn
- Usually heals in three to six days.

Partial-thickness burns (second-degree)

- More serious, deeper burns
- Affects the top layer of skin and part of the next layer
- Often cause pain, redness, blistered skin and swelling
- A hot drink splash is an example of a partial-thickness burn
- Healing time depends on severity, usually between one to three or more weeks.

Full-thickness burns (third-degree)

- Most severe, deep burns
- Affects all layers of skin and nerve endings, and sometimes fat and muscle tissue
- Skin may look brown, black or white; it often has a waxy sheen
- Burn may not be painful due to nerve-ending damage
- Examples of full-thickness burns include burns from fire or treadmill running belts
- Healing time depends on severity; these burns often require skin [grafts](#).

First aid for burns and scalds

Cool water is the best treatment for almost all burns. If child gets a burn, hold the burned area under cool, running water as soon as possible for 20 minutes. This technique can help for up to three hours after a burn. Remove clothing or jewellery, but be gentle to avoid skin damage. Sometimes, you will need to cut clothing to remove it. **Do not pull anything off the burn that has stuck to it.**

Do not put the following on fresh burns:

- Ice
- Lotions or creams
- Food items like egg whites, butter, or potato
- Toothpaste.

Call an ambulance (000) immediately if child has severe burns.

Call an ambulance (000) if:

- child has severe burns.
- child's burn is to their throat or airway, and they are struggling to breathe.
- child's burn is deep – even if they do not feel any pain.
- child's burn is on their face, hands or genitals.
- child's burn is from electricity.
- child is too distressed to keep their burn under cold water for 20 minutes.

Thermometer methods for taking a child's temperature:

Forehead Infrared Sensor Thermometer

- Make sure the child's forehead is clean and dry (no sweat or hair in the way)
- Position: Place the scanner flat in the centre of the forehead
- Follow instruction for device. Slowly slide the thermometer across the forehead to the hairline
- Check the temperature displayed. Record Temperature.
- Note: This method may be **slightly less accurate** than oral measurements.

Axillary – underarm

Best for: All ages (especially infants as a safe first check)

Procedure:

- a) Place the thermometer in the child's armpit
- b) Hold the child's arm firmly against their body
- c) Leave the thermometer in place for approximately 4–5 minutes
- d) Read and record the temperature.

Thermometer – Oral

- **Best for:** Older children (usually 5+ years)
- **Procedure:**
 - a) Place the thermometer under the tongue
 - b) Ask the child to close their lips gently (do not bite the thermometer)
 - c) Leave in place for approximately 3 minutes
 - d) Read and record the temperature

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Reference:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	85	<i>Incident, injury, trauma and illness policies and procedures</i>
	86	<i>Notification to parents of incident, injury, trauma and illness</i>
	87	<i>Incident, injury, trauma and illness record</i>
	88	<i>Infectious diseases</i>
	89	<i>First aid kits</i>
	97	<i>Emergency and evacuation procedures</i>
	161	<i>Authorisations to be kept in enrolment record</i>
	162	<i>Health information to be kept in enrolment record</i>

Other References:

https://www.rch.org.au/kidsinfo/fact_sheets/first_aid/

Handbook on Child Care Licensing

QIAS Principle 5.3, 5.5, 6.6

NOHSC – First Aid

Public Health and Wellbeing Act 2008

Occupational Health and Safety Act 2004

35 - ANAPHYLAXIS POLICY

Position Statement

Anaphylactic reactions can be life threatening. We are committed to providing, as far as practicable, a safe and healthy environment in which individuals at risk of anaphylaxis can participate equally in all aspects of the service program and experiences. We will aim to minimise the risk of an anaphylactic reaction for those attending our service by implementing the following policy and procedures.

Objective

To ensure that all team members respond appropriately to an anaphylactic reaction.

Definitions

Anaphylaxis is a severe, sudden and potentially fatal allergic reaction. This occurs when an individual has been exposed to an allergen (ie foods, insect bites, etc) and involves the major body systems, (including the breathing and circulatory systems). As Anaphylaxis is potentially life threatening an emergency response is required.

Anaphylaxis medical management action plan: a medical management plan prepared and signed by a Registered Medical Practitioner providing the child's name and allergies, a photograph of the child and clear instructions on treating an anaphylactic episode. An example of this is the Australian Society of Clinical Immunology and Allergy (ASCI) Action Plan.

ANAPEN® contains the active ingredient adrenaline(epinephrine). *ANAPEN®* is used for the immediate emergency treatment of a severe allergic reaction (also known as Anaphylaxis).

EpiPen® kit: An insulated container, for example an insulated lunch pack containing a current adrenaline auto-injection device, a copy of the child's anaphylaxis medical management action plan, and telephone contact details for the child's parents/guardians, the doctor/medical service and the person to be notified in the event of a reaction if the parent/guardian cannot be contacted. If prescribed an antihistamine may be included in the kit. Auto-injection devices (e.g. EpiPen®) are stored away from direct heat.

Communication plan: A plan that forms part of the policy outlining how the service will communicate with parents and staff in relation to the policy and how parents and staff will be informed about risk minimisation plans and emergency procedures when a child diagnosed at risk of anaphylaxis is enrolled in the service.

Risk minimisation: The implementation of a range of strategies to reduce the risk of an allergic reaction including removing, as far as is practicable, the major sources of the allergen from the service, educating parents and children about food allergies and washing hands after meals.

Risk minimisation plan: A plan specific to the service that specifies each child's allergies, the ways that each child at risk of anaphylaxis could be accidentally\ exposed to the allergen while in the care of the service, practical strategies to minimise those risks, and who is responsible for implementing the strategies. The risk minimisation plan should be developed by families of children at risk of anaphylaxis and staff at the service and should be reviewed at least annually, but always upon the enrolment or diagnosis of each child who is at risk of anaphylaxis.

A sample risk minimisation plan is outlined in Schedule 3 of this document.

Implementation

- All team members are expected to follow anaphylactic/first aid procedures as detailed in the procedural section of this policy.
- The centre will maintain accurate and current records for all children and team members relating to severe allergies which may result in an anaphylactic reaction. These records are to detail all possible triggers and an individual plan of action should a reaction occur.
- The centre will liaise with the individual and their primary caregiver in relation to the provision of appropriate emergency medical requirements (ie Epipen).
- It is the responsibility of the parent to ensure that the child has their Epipen or Anapen at the service during attendance.
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Communication and Risk Minimisation Plans

The centre will:

- Once made aware of an individual's severe allergy conduct an assessment of the potential for accidental exposure to allergens at the service. The centre will then develop a risk minimisation plan in consultation with the child's family and centre team members. Ensure that a notice is displayed prominently in the main entrance of the services stating that a child diagnosed at risk of anaphylaxis is being cared for or educated at the service r.90(1)(c)(iv).
- Ensure that primary contact team members for the individual/s at risk of anaphylaxis attend anaphylaxis management training and this is reviewed/updated annually.
- Ensure that all floaters and relief team members are made aware of the symptoms of anaphylaxis, the individual/s concerned and their allergies, their action plan and the location of the Epipen kit.
- Advise parents that no child who has been prescribed an Epipen or Anapen is permitted to attend the service without that Epipen or Anapen.
- Ensure parents/guardians of the child diagnosed at risk of anaphylaxis are provided with a copy of the policy r.90(1)(c)(iii), (iv).
- Encourage ongoing communication between the parents/guardians and team members regarding the current status of the child's allergies, this policy and its implementation.
- Medical Conditions Registers including the child's photo are displayed in each Children's room, in a central folder in the office and also on OWNA App. These registers are updated regularly and as required.
- Display the ASCIA generic poster "*Action Plan for Anaphylaxis*" in a key location in the service.
- Display emergency numbers for Ambulance, centre address and nearest cross roads next to all telephones.
- Ensure that a child's individual anaphylaxis medical management action plan signed by a registered medical practitioner is inserted in the enrolment records for each child r. 90(1)(c)(i). This will outline the allergies and describe the prescribed medication for that child and the circumstances in which it should be used.
- Ensure that all staff know the location of the anaphylaxis medical management plan and that a copy is kept with the auto-injection device (EpiPen®) kit (Schedule 3 of the Regulations).
- Ensure that the staff member accompanying children outside the service carries the anaphylaxis medication and a copy of the anaphylaxis medical management action plan in the auto-injection device (EpiPen®) kit r.90(1)(c).

The staff will:

- Ensure a copy of the child's anaphylaxis action plan is visible/available to all staff.
- In the case of a staff member with anaphylaxis, their Epi-Pen (or Anapen) and Anaphylaxis Action Plan need to be stored in the room they work in (with other Epi-Pens / medical management plans to allow staff members to assist with medical management plan and deliver first aid, if needed). If the staff member is a relief staff member (and not working in one room) the epi-pen and medical management plan to be store in office first aid box or cupboard.
- Follow the child's anaphylaxis action plan in the event of an allergic reaction, which may progress to anaphylaxis.
- In the situation where a child who has not been diagnosed as allergic, but who appears to be having an anaphylactic reaction:
 - Call an ambulance immediately by dialing 000.
 - Commence first aid measures.
 - Contact the parent/guardian.
 - Contact the person to be notified in the event of illness if the parent/guardian cannot be contacted.
- Practice EpiPen (and Anapen) administration procedures using an EpiPen trainer and "anaphylaxis scenarios" on a regular basis, preferably quarterly.
- Ask all parents/guardians as part of the enrolment procedure, prior to their child's attendance at the service, whether the child has allergies and document this information on the child's enrolment record. If the child has allergies, ask the parents/guardians to provide a medical management plan signed by a Doctor.
- Ensure that parents/guardians provide an anaphylaxis action plan signed by the child's Doctor and a complete EpiPen kit while the child is present at the service.
- Ensure that the EpiPen kit is stored in a location that is known to all staff, including relief staff; easily accessible to adults (not locked away); inaccessible to children; and away from direct sources of heat.
- Ensure that the auto-injection device (EpiPen®) kit containing a copy of the anaphylaxis medical management action plan for each child at risk of anaphylaxis is carried by a staff member accompanying the child when the child is removed from the service e.g. on excursions that this child attends (Schedule 3 of the Regulations).
- Regularly check the EpiPen expiry date. (The manufacturer will only guarantee the effectiveness of the EpiPen to the end of the nominated expiry month.)
- Provide information to the service community about resources and support for managing allergies and anaphylaxis.
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Parents will:

- Inform staff, either on enrolment or on diagnosis, of their child's allergies.
- Provide staff with an anaphylaxis action plan and written consent to use the EpiPen in line with this action plan.
- Provide staff with a complete EpiPen kit.
- Regularly check the EpiPen expiry date.
- Assist staff by offering information and answering any questions regarding their child's allergies.
- Notify the staff of any changes to their child's allergy status and provide a new anaphylaxis action plan in accordance with these changes.
- Communicate all relevant information and concerns to staff, for example, any matter relating to the health of the child.
- Comply with the service's policy that no child who has been prescribed an EpiPen is permitted to attend the service or its programs without that EpiPen

Related Documents to this Policy:

1. Risk Minimisation Plan (see schedule 1 of this policy)
2. Enrolment checklist for children at risk of anaphylaxis (see schedule 2 of this policy)
3. Sample Risk Minimisation Plan (see schedule 3 of this policy)

Risk Minimisation Plan Template (see schedule 3 of this policy)
4. Medical Action Plan for Anaphylaxis (Please refer to <http://www.allergy.org.au/> website for current form)
5. Possible Exposure Scenarios and Strategies (see schedule 5)
6. Brochure titled “Anaphylaxis – a life threatening reaction” available through the Royal Childrens Hospital, Department of Allergy

Schedule 1 - Risk Minimisation Plan and Procedures

The following plans procedures have been developed in consultation with parents and information sourced from authorities implemented to help protect the child at risk of anaphylaxis from accidental exposure to food allergens:

In relation to the child at risk:

- This child should only eat food that has been specifically prepared for him/her;
 - Where the service is preparing food for the child, ensure that it has been prepared according to the parent's instructions.
 - Some parents will choose to provide all food for their child.
- All food for this child should be checked and approved by the child's parent/guardian and be in accordance with the risk minimisation plan.
- Bottles, other drinks and lunch boxes, including any treats, provided by the parents/guardians for this child should be clearly labelled with the child's name.
- There should be no trading or sharing of food, food utensils and containers with this child.
- In some circumstances it may be appropriate that a highly allergic child does not sit at the same table when others consume food or drink containing or potentially containing the allergen. However, children with allergies should not be separated from all children and should be socially included in all activities.
- Parents/guardians should provide a safe treat box for this child.
- Where this child is very young, provide his/her own high chair to minimise the risk of cross-contamination.
- When the 'at risk' child is allergic to milk, ensure non-allergic babies are held when they drink formula/milk.
- Increase supervision of this child on special occasions such as excursions, incursions or family days.
- Assess whether possible to eliminate the allergen from the service menu.

In relation to other practices at the centre:

- Ensure tables and bench tops are washed down after eating.
- Ensure hand washing for all children upon arrival at the service, before and after eating.
- Restrict use of food and food containers, boxes and packaging in crafts, cooking and science experiments, depending on the allergies of particular children. Staff should discuss the use of foods in such activities with parents/guardians of this child and these foods should be consistent with the risk minimisation plan.
- All children need to be closely supervised at meal and snack times and consume food in specified areas. To minimise risk children should not 'wander around' the centre with food.
- Staff should use non-food rewards, for example stickers, for all children.
- The risk minimisation plan will inform the centre's food purchases and menu planning.
- Food preparation personnel (staff and volunteers) should be instructed about measures necessary to prevent cross contamination between foods during the handling, preparation and serving of food – such as careful cleaning of food preparation areas and utensils.
- Where food is brought from home to the centre, all parents/guardians will be asked not to send food containing specified allergens or ingredients as determined in the risk minimisation plan.

Schedule 2 – Enrolment Checklist for Children at Risk of Anaphylaxis

A risk minimisation plan is completed in consultation with parent/guardian, which includes strategies to address the particular needs of each child at risk of anaphylaxis, and this plan is implemented.

Parents/guardians of a child diagnosed at risk of anaphylaxis have been provided a copy of the service's Anaphylaxis management policy	Yes / No
All parents/guardians are made aware of the Anaphylaxis management policy	Yes / No
Anaphylaxis medical management action plan for the child is signed by the child's Registered Medical Practitioner and is visible to all staff. A copy of the anaphylaxis medical management action plan is included in the child's autoinjection device (EpiPen®) kit.	Yes / No
Adrenaline auto-injection device such as an EpiPen® (within expiry date) is available for use at any time the child is in the care of the service	Yes / No
Adrenaline auto-injection device is stored in an insulated container (e.g. EpiPen® Kit), in a location easily accessible to adults (not locked away), inaccessible to children and away from direct sources of heat	Yes / No
All staff, including relief staff, are aware of each EpiPen® kit location and the location of the anaphylaxis medical management action plan	Yes / No
Staff responsible for the child/ren diagnosed at risk of anaphylaxis undertake accredited anaphylaxis management training, which includes strategies for anaphylaxis management, risk minimisation, recognition of allergic reactions, emergency treatment and practise with an EpiPen® trainer, and is reinforced at quarterly intervals and recorded annually	Yes / No
The service's emergency action plan for the management of anaphylaxis is in place and all staff understand the plan	Yes / No
A treat box is available for special occasions (if relevant) and is clearly marked as belonging to the child at risk of anaphylaxis	Yes / No
Parent/guardian's current contact details are available	Yes / No
Information regarding any other medications or medical conditions (for example asthma) is available to staff	Yes / No
If food is prepared at the service, measures are in place to prevent contamination of the food given to the child at risk of anaphylaxis	Yes / No

Signing of this checklist is acknowledgement that the **Enrolment Checklist for Children at Risk of Anaphylaxis** has been completed.

Name of Parent: _____ Signature of Parent: _____ Date: / /

Name of Director: _____ Signature of Director: _____ Date: / /

Schedule 3 – Sample Risk Minimisation Plan for Anaphylaxis

The following suggestions may be considered when developing or reviewing a children's service risk minimisation plan.

How well has the children's service planned for meeting the needs of children with allergies who are at risk of anaphylaxis?	
1. Who are the children?	- List names and room locations of each of the at risk children
2. What are they allergic to?	-List all of the known allergens for each of the at risk children - List potential sources of exposure to each known allergen and strategies to minimise the risk of exposure. This will include requesting that certain foods/items not be brought to the service
3. Does everyone recognise the at risk children?	-List the strategies for ensuring that all staff, including relief staff and cooks, recognise each of the at risk children -Confirm where each child's Action Plan (including the child's photograph) will be displayed
Do families and staff know how the service manages the risk of anaphylaxis?	
• Record when each family of an at risk child is provided a copy of the service's Anaphylaxis management policy • Record when each family member provides a complete auto-injector (EpiPen®) kit • Test that all staff, including relief staff, know where the auto-injector (EpiPen®) kit is kept for each at risk child • Regular checks of the expiry date of each adrenaline auto-injection device are undertaken by a nominated staff member and the families of each at risk child • Service writes to all families requesting that specific procedures be followed to minimise the risk of exposure to a known allergen. This may include requesting the following are not sent to the service: - o Food containing the major sources of allergens, or foods where transfer from one child to another is likely, for example peanut, nut products, whole egg, chocolate - o Food packaging of risk foods (see known allergens at point 2), for example cereal boxes, egg cartons and so on • A new written request is sent to families if the food allergens change • Ensure all families are aware of the policy that no child who has been prescribed an adrenaline auto-injection device such as an EpiPen® is permitted to attend the service without that device. • The service displays the ASCIA generic poster, <i>Action plan for anaphylaxis</i>, in a key location and locates a completed ambulance card by the telephone/s • The auto-injector (EpiPen®) kit including a copy of the anaphylaxis medical management action plan is carried by a staff, member when a child is removed from the service eg excursions	
Do relevant people know what action to take if a child has an anaphylactic reaction?	
• Know what each child's anaphylaxis medical management action plan says and implement it • Know who will administer the adrenaline auto-injection device (EpiPen®)and stay with the child; who will telephone the ambulance and the parents; who will ensure the supervision of the other children; who will let the ambulance officers into the service and take them to the child • All staff with responsibilities for at risk children have undertaken anaphylaxis management training and regular practise sessions for the administration of the adrenaline auto-injection device (EpiPen®).	
How effective is the service's risk minimisation plan?	
Review the risk minimisation plan with families of at risk children at least annually, but always upon enrolment of each at risk child and after any incident or accidental exposure.	

Schedule 3 – Risk Minimisation Plan for Anaphylaxis Examples

Name of Child: _____ Date of Birth: / /

How well has the children's service planned for meeting the needs of children with allergies who are at risk of anaphylaxis?											
1. Who are the children?	List names and room locations of each of the at risk children: <table border="1"> <thead> <tr> <th>Name of Child</th> <th>Room Name</th> </tr> </thead> <tbody> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> </tbody> </table>	Name of Child	Room Name								
Name of Child	Room Name										
2. What are they allergic to?	-List all of the known allergens for each of the at risk children <table border="1"> <thead> <tr> <th>Name of Child</th> <th>Allergens of Child</th> </tr> </thead> <tbody> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> <tr><td> </td><td> </td></tr> </tbody> </table> Strategies to minimise the risk of exposure. - Please refer to Schedule 1 - Risk Minimisation Plan and Procedures - Please refer to Schedule 2 – Enrolment Checklist for Children at Risk of Anaphylaxis - Nut free centre - Certain foods/items not be brought to the service - All food must be registered and signed into centre	Name of Child	Allergens of Child								
Name of Child	Allergens of Child										
3. Does everyone recognise the at risk children?	-Please refer to the section of this policy "Communication and Risk Minimisation Plans" for action plan to ensure everyone recognises risk children -The strategies ensuring that all staff, including relief staff and cooks, recognise each of the at risk children - Child's Action Plan (including the child's photograph) will be displayed in a prominent location in the room for Educators to see.										
Do families and staff know how the service manages the risk of anaphylaxis?											
Recorded when each family of an at risk child is provided a copy of the service's Anaphylaxis management policy?	Yes / No										
Recorded when each family member provides a complete auto-injector (EpiPen®) kit?	Yes / No										
Tested that all staff, including relief staff, know where the auto-injector (EpiPen®) kit is kept for each at risk child?	Yes / No										
Regular checks of the expiry date of each adrenaline auto-injection device are undertaken by a nominated staff member and the families of each at risk child?	Yes / No										
Centre provide information to all families requesting that specific procedures be followed to minimise the risk of exposure to a known allergen? This may include requesting the following are not sent to the service: - o Food containing the major sources of allergens, or foods where transfer from one child to another is likely, for example peanut, nut products, whole egg, chocolate - o Food packaging of risk foods (see known allergens at point 2), for example cereal boxes, egg cartons and so on	Yes / No										
A new written request is sent to families if the food allergens change?	Yes / No										
Ensure all families are aware of the policy that no child who has been prescribed an adrenaline auto-injection device such as an EpiPen® is permitted to attend the service without that device?	Yes / No										

The service displays the ASCIA generic poster, <i>Action plan for anaphylaxis</i> , in a key location and locates a completed ambulance card by the telephone/s?	Yes / No
The auto-injector (EpiPen®) kit including a copy of the anaphylaxis medical management action plan is carried by a staff member when a child is removed from the service eg excursions?	Yes / No
Do relevant people know what action to take if a child has an anaphylactic reaction?	
They will know what each child's anaphylaxis medical management action plan says and implement it	Yes / No
Know who will administer the adrenaline auto-injection device (EpiPen®) and stay with the child; who will telephone the ambulance and the parents; who will ensure the supervision of the other children; who will let the ambulance officers into the service and take them to the child. This information will be recorded in a medication administration form.	Yes / No
All staff with responsibilities for at risk children have undertaken anaphylaxis management training and regular practise sessions for the administration of the adrenaline auto-injection device (EpiPen®).	Yes / No
How effective is the service's risk minimisation plan?	
Review the risk minimisation plan with families of at risk children at least annually, but always upon enrolment of each at risk child and after any incident or accidental exposure.	Yes / No Date: / /

Date Risk Minimisation Plan for Anaphylaxis Completed: / /

Name of Centre Director: _____

Signature of Centre Director: _____ **Date:** / /

Schedule 5 - Possible exposure scenarios and strategies

Scenario	Strategy	Who
Food is provided by the children's service and a food allergen is unable to be removed from the service's menu (for example milk)	Menus are planned in conjunction with parents of at risk child/children and food is prepared according to parents instructions Alternatively the parent provides all of the food for the at risk child	Cook, Primary Nominee, Parent
	Ensure separate storage of foods containing allergen Proprietor & Cook	Cook
	Cook and staff observe food handling, preparation and serving practices to minimise the risk of cross contamination. This includes hygiene of surfaces in kitchen and children's eating area, food utensils and containers	Cook & Staff
	There is a system in place to ensure the at risk child is served only the food prepared for him/her	Cook & Staff
	An 'at risk' child is served and consumes their food at a place considered to pose a low risk of contamination from allergens from another child's food. This place is not separate from all children and allows social inclusion at mealtimes	Staff
	Children are regularly reminded of the importance of no food sharing with the at risk child	Staff
	Children are supervised during eating	Staff
Party or celebration	Give plenty of notice to families about the event	Proprietor/ Primary Nominee/ Qualified Staff
	Ensure a safe treat box is provided for the at risk child	Parent/ Staff
	Ensure the at risk child only has the food approved by his/her	parent/guardian Staff
	Specify a range of foods that families may send for the party and note particular foods and ingredients that should not be sent	Proprietor/ Primary Nominee
Protection from insect sting allergies	Specify play areas that are lowest risk to the at risk child and encourage him/her and peers to play in the area	Staff
	Decrease the number of plants that attract bees	Proprietor
	Ensure the at risk child wears shoes at all times outdoors	Staff
	Quickly manage any instance of insect infestation. It may	Proprietor

	be appropriate to request exclusion of the 'at risk' child during the period required to eradicate the insects	
Latex allergies	Avoid the use of party balloons or contact with latex gloves	Staff
Cooking with children	Ensure parents/ guardians of the 'at risk' child are advised well in advance if ingredients need to be bought by the parents	Staff

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and staff verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.1.3	<i>Healthy eating and physical activity are promoted and appropriate for each child.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement—anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

Handbook on Child Care Licensing
Health Act

Policy Handbook – P004. 7.2026

*Health Records Act
Department of Human Services (Vic)
Public Health and Wellbeing Act 2008
Workplace Health and Safety Regulation*

Sources and Further Reading:

- *Anaphylaxis Australia – www.allergyfacts.org.au (3 May 2011)*
- *NSW Health – [Guidelines for Children's Services 2007 – Anaphylaxis](http://www.allergyfacts.org.au/PDF/0562_anaphylaxis.pdf)
http://www.allergyfacts.org.au/PDF/0562_anaphylaxis.pdf*
- *State of Victoria Dept of Education and Early Childhood (2011) [Anaphylaxis model policy](http://www.office-for-children.vic.gov.au/earlychildhood),
www.office-for-children.vic.gov.au/earlychildhood*

36 - ASTHMA POLICY

Position Statement

Australia has the highest incidence of asthma in the world. Asthma represents a significant health problem in the Australian community affecting 1 in 6 children. Asthma is one of the most common causes of hospital admissions and visits to the doctor by children under five years of age. There is also evidence that the prevalence and severity of asthma in children is increasing. Dealing with pediatric asthma is a common expectation of team members working in the early childhood setting. While this illness is common it is by no means an insignificant problem. It is a serious complaint and can be life threatening to the sufferer.

As children under the age of six years generally do not have the skills and ability to recognise and manage their own asthma effectively, we place importance on our role in ensuring that a child's asthma symptoms are managed appropriately whilst in our care.

Objective

To provide guidelines for the successful management of individual Asthmatics.

Implementation

- Parents are requested to complete medical details as part of the enrolment process
- If parents identify the child as suffering from Asthma, the parents is required to seek a professional medical practitioner to complete the Asthma Action Management Plan forms (this includes the asthma management plan, risk minimisation plan and communication plan). This form provides details as to the normal drug regime the child is on, as well as outlining the treatment that the child's doctor has detailed in the event of an attack occurring.
- A child is not allowed to attend the service if the child has not provided an Asthma Management Action Plan, Risk Minimisation Plan and Communication Plan. Please note this include the Management Plan being signed by a doctor.
- Asthma management action plans are to be held together in the office so that there is a centralised register for children who are Asthmatic. A copy is also stored in the classroom.
- To ensure that all educators are familiar with the template; it is recommended that all children are to have the Blue Asthma Care Plan for Education and Care Services (Asthma Australia) completed.
- Asthma Management Plans are required to be review and updated on 12 monthly basis or a review period advised by a medical practitioner (this may be greater or less than 12 months). The date is taken from the date signed by the doctor.
- Parents are required to keep these plans up to date. Should any part of the regime of treatment alter, the records must be adjusted similarly.
- Add the Asthma Management Action Plan, Risk Minimisation Plan and Communication Plan to Child profile documents in OWNA portal. On child enrolment form answer the Asthma Medical Conditional question as "Yes".
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Medication and First Aid

- When a child is suffering or recovering from an Asthma attack, the parents will be required to send along any reliever medication and equipment that may be needed should the child have a relapse.
- Any medication that the child is to have/or may need to have is to be stored as per the medication policy and recorded on OWNA Medication Record.

- Administration of emergency medication must be in accordance with the Medication Policy.
- Our centre will encourage team members to attend yearly education by the Asthma Foundation or First Aid Consultant as required by regulations.
- Service will display within the centre an Asthma First Aid Poster as provided by the Asthma Foundation (or have a copy with Asthma Management Plan).
- If a Parent wants asthma medication at a specific day/time, they must complete a medication record – Asthma Action Plans are for Emergency Administration of Asthma medication.
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Asthma Emergency Kits/Resources

Asthma Emergency Kits should contain:

- Reliever medication
- x 2 small volume spacer device
- x 2 compatible children's face mask
- Record form (Available in form folder)
- Asthma First Aid instruction card (on display in Asthma Management Plan folder)

Please note that it is a recommendation of The Asthma Foundation of Victoria that spacers and face masks are single-person use only (*National Health and Medical Research Council (NHMRC) (2010), "Australian Guidelines for the Prevention and Control of Infection in Healthcare", Commonwealth of Australia, Canberra, 2010.*) . As precaution the service has at least two spacers and two face masks contained in office asthma first aid kit and that spacers and face masks are replaced each time they are used. Face masks are to be used by children unable to coordinate correct breathing through the mouthpiece of the spacer.

Asthma Emergency Kits should only ever be used as a back-up when a child who is known to have asthma does not have their own spacer available, or in the event of a first asthma attack or if the cause of breathing difficulty is unknown. Parents are responsible for the cost of the spacer if one must be used from the Asthma Emergency Kit.

Children's Face Mask and Spacer – Parents are to supply appropriate medication, spacer and face mask.

The service provides families with Asthma Australia Brochures when first advised of Asthma as a medical conditions. 'Asthma and the Under 5's' Asthma Basic Facts and Asthma Medications and Devices

Asthma and Triggers

Team Members can try to reduce possible asthma triggers by:

- Minimising the use of chemicals when children are present.
- Regularly vacuuming and damp dusting.
- Ensure that the centre environment is maintained in a hygienic manner, being particularly vigilant to minimise mould growth.

Team members also need to be aware of food allergies and intolerances that may trigger an asthma attack and modify the food we provide accordingly. To achieve this, we shall ensure:

- Chefs/cooks are aware of foods and food additives which are potential triggers for asthma and encourage the use of alternative foods and
- Keep a list of those children who have known food allergies or whose asthma is triggered by food or additives and ensure these children do not consume these foods.

Staff will:

- Ensure that they maintain current Asthma First Aid Training
- Ensure that they are familiar with each child with asthma in the care
- In regular consultation families, optimise the health and safety of each child through supervised management of the child's asthma
- Only administer prescribed and approved medication.
- No medication prescribed for anyone other than a particular child will be given
- All medication must be clearly marked with the child's name and be stored appropriately.
- Ensure that all regular prescribed asthma medication is administered in accordance with the information on Child's Asthma Record.
- Medication is to be administered (on a non-emergency basis) and is to be recorded accurately by the parent/guardian, in relation to time and dosage, and will be signed by a staff member on its administration.
- Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.
- Communicate without undue delay to management and families, if they are concerned about a child's asthma limiting his/her ability to participate fully in all activities.
- Provides families with details of Asthma Foundation
 - <http://www.asthmaact.org.au/>
 - 1800 645 130

Families will:

- Formally inform staff members and the Centre, either upon enrolment of their child with Asthma or on initial diagnosis (without undue delay), that their child has a history of asthma.
- Provide all relevant information regarding the child's asthma via the Asthma Record as provided by the child's doctor. Complete and provide a Asthma Management Plan for child signed by a Doctor. Work with the service to completed a Risk Minimisation and Communication Plan for their child. Without these forms completed and signed the child will not be allowed to attend the service.
- Notify the Centre staff members, in writing, of any alterations to the Asthma Record.
- Ensure that their child maintains adequate supply of appropriate medication (reliever) and spacer device clearly labelled with the child's name including expiry dates.
- Communicate all relevant information and concerns to staff as the need arises.
- Parent/guardian must give written authority for medication to be dispensed by filling in a Medication Form. If the medication Form is not filled in, except in the case of an emergency, medication will not be administered on that day.
- Do not leave medications in your child's bag or locker. Give it directly to a staff member upon arrival.
- Where possible and depending on ability, encourage their child to learn about their asthma and communicate to service staff if they are unwell or experiencing asthma symptoms

Asthma First Aid

Follow the written first aid instructions on the child's Asthma Action/Care Plan. If no specific and signed instructions are available, the instructions are unclear, or the child does not have an Asthma Action/Care Plan, begin the first aid procedure immediately (as authorised by the Education and Care Services National Regulations 2011).

Call emergency assistance to attend (000) if:

- the child's asthma symptoms are severe
- the child suddenly stops breathing
- the child's asthma symptoms continue to worsen
- there is no Asthma Action/Care Plan for the child
- blue/grey reliever medication is not available
- you are unsure what is causing the breathing difficulty

Recognising an Asthma Attack

Mild	Moderate	Severe
Talk in sentences	Shortened sentences	Few words per breath
Cough	Persistent cough	Persistent cough
Soft wheeze	Loud wheeze	Wheeze may be absent
Minor difficulty breathing	Difficulty breathing	Gasping for breath/distress
Tightness in chest		Pale, sweaty, blue lips
Young children may complain of a "sore tummy"		Muscle exertion

ASTHMA FIRST AID

Blue/Grey Reliever

Airomir, Asmol, Ventolin or Zempreon and Bricanyl

Blue/grey reliever medication is unlikely to harm, even if the person does not have asthma



DIAL TRIPLE ZERO (000) FOR AN AMBULANCE IMMEDIATELY IF THE PERSON:

- is not breathing
- suddenly becomes worse or is not improving
- is having an asthma attack and a reliever is not available
- is unsure if it is asthma
- **has a known allergy to food, insects or medication and has SUDDEN BREATHING DIFFICULTY, GIVE ADRENALINE AUTOINJECTOR FIRST (if available), even if there are no skin changes, then use a reliever**

1



SIT THE PERSON UPRIGHT

- Be **calm** and reassuring
- **Do not leave** them alone

2



GIVE 4 SEPARATE PUFFS OF RELIEVER PUFFER

- **Shake** puffer
- Put **1 puff** into spacer
- Take **4 breaths** from spacer
 - Repeat until **4 puffs** have been taken



If using **Bricanyl**, give 2 separate inhalations (5 years or older)

If you don't have a spacer handy in an emergency, take **1 puff** as you take **1 slow, deep breath** and hold breath for as long as comfortable. **Repeat** until all puffs are given

3



WAIT 4 MINUTES

- If breathing does not return to normal, give **4 more separate puffs** of reliever as above



Bricanyl: Give 1 more inhalation

IF BREATHING DOES NOT RETURN TO NORMAL

4



DIAL TRIPLE ZERO (000)

- Say '**ambulance**' and that someone is having an asthma attack
- Keep giving **4 separate puffs every 4 minutes** until emergency assistance arrives



Bricanyl: Give 1 more inhalation **every 4 minutes** until emergency assistance arrives



ASTHMA AUSTRALIA

1800 ASTHMA
(1800 278 462)
asthma.org.au

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WHITE MAGIC
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(Source: <https://asthma.org.au/treatment-diagnosis/asthma-action-plan/> Date:26 04 23)

In the event of a child having an asthma attack whilst at the Centre:

An asthma attack can become life threatening if not treated properly. If a child is displaying asthma symptoms, our service will:

- ensure a first aid trained educator/educator with approved asthma management training immediately attends to the child. If the procedures outlined in the child's medical management plan do not alleviate the asthma symptoms, or the child does not have a medical management plan, the educator will provide appropriate first aid, which may include the steps outlined by Asthma Australia as follows:
 1. Sit the child upright
 - Stay with the child and be calm and reassuring
 2. Give 4 puffs of blue reliever puffer medication
 - Use a spacer if there is one
 - Shake puffer
 - Put 1 puff into spacer
 - Take 4 breaths from spacer
 - Repeat until 4 puffs have been takenShake, 1 puff, 4 breaths
 3. Wait 4 minutes
 - If there is no improvement, give 4 more puffs as above
 4. If there is still no improvement call emergency assistance 000
 - Keep giving 4 puffs every 4 minutes until emergency assistance arrives
- contact the child's parent or authorised contact where the parent cannot be reached

The service will ensure that an asthma First Aid Kit is stored in a location that is known to all staff, including relief staff, easily accessible to adults (not locked away), inaccessible to children, and at room temperature in dry areas. An Asthma First Aid kit should contain:

- Blue or grey reliever puffer
- A spacer device that is compatible with the puffer
- A face mask compatible with the spacer for use by children under 5
- 70% alcohol swabs for cleaning devices. Note puffers, spacers and face masks from the Kit must be thoroughly cleaned after each use to prevent cross contamination. To clean:
 - Remove canister from puffer and wash device (but not canister) in warm water with kitchen detergent
 - Do not rinse or rub dry, allow devices to air dry.
 - When dry, wipe the mouth piece inside and outside with a 70% alcohol swab
 - When completely dry, replace the canister and ensure puffer is working correctly.

Medical Conditions Risk Minimisation Plan: Asthma Management

Asthma is a chronic lung disease that inflames and narrows the airways. While developing the Medical Conditions Risk Minimisation Plan our service will implement procedures where possible to minimise the exposure of susceptible children to the common triggers which can cause an asthma attack. These triggers include:

- dust and pollution
- inhaled allergens, for example mould, pollen, pet hair

- changes in temperature and weather, heating and air conditioning
- emotional changes including laughing and stress
- activity and exercise

Risk minimisation practices will be carried out to ensure that the service is to the best of our ability providing an environment that will not trigger an asthmatic reaction. These practices will be documented and reflected upon, and potential risks reduced if possible.

The service will display an Asthma chart called First Aid for Asthma Chart or Asthma First Aid in a key location at the service, for example, in the children's room, the staff room or near the medication cabinet

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Next Review: 15/6/2027

Reference: NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement - anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>

	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>
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Other References:

Handbook on Child Care Licensing

QIAS Principle 5.3

Anti-discrimination Act

Food Safety Legislation 2001

Public Health and Wellbeing Act 2008

Workplace Health and Safety Regulation 1997

CCB – Federal Funding Guidelines

Sources and Further Reading:

- *Asthma Foundation of Qld (2006) A Guide to Becoming an Asthma Friendly Childcare Service, Fortitude Valley*
- *Asthma Foundations Australia www.asthmaaustralia.org.au*
- *Australian Guidelines for the Prevention and Control of Infection in Healthcare (2010), National Health and Medical Research Council, www.nhmrc.gov.au/_files_nhmrc/publications/attachments/cd33_infection_control_healthcare_0.pdf.*

37 - BITING POLICY

Position Statement

Biting incidents are of significant concern for all parties: the parents of the child who was bitten, the child who did the biting, and for team members in general.

Whilst the handling of these incidents will be dealt with in an immediate sense in the general behavioural management policy, it has been seen there is a need to formalise a specific policy relating to the incidence of a child biting another child.

All children should be able to attend the centre and come into a safe and secure environment. Our professional commitment to Duty of Care for all children and team members places an expectation on us to ensure that such an environment is provided.

Children who bite jeopardise the safety and security of this environment for other children. Our Duty of Care towards all enrolled children underpins this policy.

Objective

To set out guidelines to deal with the serious incidence of children who bite others. These guidelines will address the actions that may need to be taken by the Centre team members in consultation with the children and their families who are involved in a biting incident.

Implementation

In Relation to Team members:

- Team members will consider all aspects of the room's operation so as to minimise the possibility of children biting eg. due to frustration caused by having to share with others.
- Team members will support children to learn conflict resolution skills and assertive language skills in preference to physical actions and as a protection against domineering and aggressive children.
- Team members will employ age appropriate behavioural management strategies within the room at all times. This may include time away strategies for children who are unable to control their behaviour. These management strategies will be in line with our behavioural management policy.
- All biting incidents will be documented and signed by the director on an Incident report. An incident report will be completed for both the child who is bitten and the biter.
- Educators will notify the parents of child who is bitten and also the child who has bitten, by phone after the child has been attended to. The reason for making a brief call prior to pick up is to allow the educator to advise of the incident prior to pick-up. If parent would like to discuss in detail educator can arrange a meeting time with parent during call.
- The Centre Director, Assistant Centre Director and/or the Educational Leader will speak to the parents of the biter and work out a collaborative approach to managing the child's behaviour.

In Relation to the Parents:

- It is an expectation that the parents of the child who is the biter will support the behavioural management strategies that are in place within the centre at home. i.e. they will reinforce that biting someone else is not acceptable, "Use words, not your teeth, say "STOP!"
- The parents of the child who was bitten will have a thorough explanation of the circumstances of the incident including first aid measures implemented and strategies to prevent further incidents.

NB At no time will the parents of the child who was bitten be privy to the name of the child who is the biter.

In Relation to the Centre:

- Biting is a serious behavioural matter and will be treated accordingly.
- Any bite that draws blood is viewed as extremely serious. Consultation with parents involved in this situation will occur at the earliest convenient time.
- The placement of the child who bites, and therefore places the safety of the other children at risk will be under careful consideration. Aspects of this consideration taken into account will be: – the response of both the family and child to the behavioural management strategies that have been put in place by the team members; the number and severity of the bites that the child has inflicted; and the age of the child involved.
- If, after the above deliberation, it is felt that the situation has become unworkable management has the right to alter the days the child attends or cancel the child's enrolment.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/10/2025

Reference:

NQS

QA5	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.</i>

LO2	Children develop knowledgeable and confident self identities.
	Children learn to interact in relation to others with care, empathy and respect.
	Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.
	Children respond to diversity with respect.
	Children become aware of fairness.
	Children become socially responsible and show respect for the environment.

Other References:

Handbook on Child Care Licensing
QIAS Principle 1.2

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Sources and Further Reading:

- Kinnell, G. (2003) **No Biting: Policy and Practice for Toddler Programs**, Pademelon Press, Castle Hill, New South Wales. (Australian edition)
- Stonehouse, A. (2008) **Biting in Child Care – An NCAC Factsheet**, <http://www.ncac.gov.au/factsheets/biting.pdf> (20 October 2008)

38 - CLOTHING POLICY

Position Statement

It is important that children are clothed appropriately for their health and for play and activities at the service.

Objective

To provide families and team members with guidelines for appropriately clothing children during the day at the service.

Implementation

- Parents will be provided with an enrolment pack which must be completed before the child can commence childcare. The enrolment pack will include a list of suggested clothing inclusive of nappies and spare clothing for the day's activities. Parents are required to supply own nappies, sunscreen, etc if there are allergies to service supplies. There will be no variation in service fees if this occurs.
- Team members will monitor weather conditions during the day and ensure children are clothed suitably.
- Newsletters, verbal communication, emails and memos will be provided to parents as a reminder to changes in weather conditions and required clothing.
- At times the team members will program special events such as "water play day" and "dress up day". Notices for these days will be provided to the parents.
- Children that are toileting and in nappies require clothing that is easy for them to manage. Parents are encouraged to send their child in clothing that is easy for the child to manage and easy for the team members to access when changing nappies.
- Team members carefully plan and implement a variety of activities to enhance children's learning and development. At times these activities may be "messy" and parents are encouraged to dress their child in "play clothes" so that they can enjoy and participate in all activities.
- Children will be provided with aprons during messy play activities, however should be dressed in clothes that are conducive to play and that they can get "messy" in.
- Team members will encourage families to provide appropriate footwear for children to wear at the centre. Footwear should be suitable for climbing and running and protect the children's feet from the outdoor environment (thongs or ugg boots are not suitable). It is also helpful as the children's self help skills develop that the footwear supplied is easy for the child to remove and put back on with minimal assistance. Infants and babies may require socks – depending on the weather conditions.
If a child comes to the service in inappropriate footwear and does not have appropriate footwear in their bag to change into, the parents will be required to either take the child home and return with the appropriate footwear or bring in the appropriate footwear within a reasonable time frame (30 minutes) to the service for the child to wear.
- For children who are toilet training, it is important that they are wearing clothes which allow for easy access to the bathroom, overalls and clothing with difficult fasteners may not be appropriate.
- Clothing should be safe; jumpers with cords should be avoided as these can become entangled on climbing equipment.
- Light-Up Shoes: Light-up shoes usually contain hidden batteries that power the lights. It's important for the shoes to be well-maintained, and the batteries should be inaccessible to others, meaning they must be embedded within the shoe sole.
- Clothing which should not be worn to the centre includes; brief clothing which does not comply with sun safe guidelines (ie singlet tops, sleeveless tops), clothing bearing offensive slogans or logos which may offend a particular group or individual, superhero outfits which may encourage aggressive play. We also discourage children wearing commercialised outfits, hats and clothing (E.g. Disney themes) as these items tend to

cause behavioural issues amongst the children. If the children are dressing up for an event in the outfits, positive play will be encouraged.

- Team members will supervise children whilst dressing/undressing and offer support, plenty of time and encouragement (not applicable for school children). Children's privacy and dignity will be respected at all times.
- Team members will promote the clothing policy through discussion about clothing requirements for the various seasons of the year and by the use of appropriate information displays and newsletters.
- The service accepts no responsibility for lost or damaged possessions; including but not limited to clothing, hats, footwear, etc.

Supporting information may also be located in the Sun Protection Policy.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/10/2025

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted</i>
	2.1.1	<i>Each child's wellbeing and comfort is provided for, including appropriate opportunities to meet each child's need for sleep, rest and relaxation</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

EYLF

LO3	<i>Children recognise and communicate their bodily needs (for example, thirst, hunger, rest, comfort, physical activity).</i>
	<i>Educators engage children in experiences, conversations and routines that promote healthy lifestyles and good nutrition.</i>
	<i>Educators consider the pace of the day within the context of the community.</i>
	<i>Educators provide a range of active and restful experiences throughout the day and support children to make appropriate decisions regarding participation.</i>

Other References:

Handbook on Child Care Licensing
QIAS Principle 4.5 5.1 6.4 6.5
Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2004

39 - SOILED CLOTHING POLICY

Position statement

It is important to monitor and prevent cross infection within the centre and to ensure that appropriate hygiene is in place at the centre at all times.

Objective

To manage and deal with soiled clothing hygienically and in accordance with regulations.

Implementation

- Team members must wear gloves when they come in contact and handle any soiled items.
- Team members will attend to the child's needs in a positive way.
- Soiled items will be rinsed (if time permits) in the centre laundry or sluice and placed securely in a small plastic bag, zip lock bag or wet bags (wet bags provided parent/guardian). (ALL PLASTIC BAGS MUST BE INACCESSIBLE TO CHILDREN).
- The sealed plastic bag will then be placed in an air tight container with the child's name on it. Containers will be stored in the centre laundry or other suitable location.
- Parents will be notified that they have soiled clothing in the laundry to collect either via a tag placed on the child's bag, OWNA App message, verbal communication or written communication.
- After the soiled clothing has been collected by the parents, the containers will be disinfected.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/5/2026

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	77	<i>Health, hygiene and safe food practices</i>
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EYLF

LO3	<i>Actively support children to learn hygiene practices.</i>
	<i>Promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community.</i>
	<i>Discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.</i>
	<i>Model and reinforce health, nutrition and personal hygiene practices with children.</i>

Other References:

QIAS Principle 6.4,6.6,7.3
 Public Health and Wellbeing Act 2008
 Occupational Health and Safety Act 2004
 Staying Healthy (5th Edition)
 Handbook on Child Care Licensing

Legislation:

Child Care Act 2002 and Regulations (2003)Qld
 Children's Services Act (1985) & the Children's
 Services (Child Care Centre) Regulations
 1998 (SA)
 Children's Services Regulations (2004) and
 Children and Young Persons (Care and Protection)
 Act 1998(NSW)
 Children's Services Act 1996(Vic) & Children's
 Services Regulations 2009(Vic)

40 - REST & SLEEP POLICY

Position Statement

The purpose of the Rest Time and Sleep Policy is to ensure the safety, health and wellbeing of children attending our service and appropriate opportunities are provided to meet each child's need for sleep, rest and relaxation. Our policy is based on the recommendations from the recognised national authority Red Nose (formerly known as SIDS & Kids).

Background

The Education and Care Services National Regulations requires approved providers to ensure their services have policies and procedures in place for children's sleep and rest.

Our Sleep and rest for children policy provides our educators, management, coordinators, other staff, families and the community with the information they need to support children's needs for sleep and rest are met while attending the service.

Objective

To provide each child with the opportunity to rest, sleep and relax by creating an atmosphere that accommodates the individual needs of each child and their family.

Procedures and Implementation

- Our approach to supporting and promoting children's health and physical activity, including safe sleep and nutrition, is informed by current recognised guidelines and up-to-date information.
- Red Nose is considered the recognised national authority on safe sleeping practices for infants and children. For more information on safe sleeping practices please refer to the Red Nose website (<http://www.rednose>) or the Red Nose: Safe Sleeping in Childcare kit available at this centre or online.
- All staff and volunteers of the service will be trained about best practice for children's sleep and rest, and follow service policy and Red Nose practice guidance. The staff and volunteer training will be performed internally and externally.
- Staff and volunteers will know the risks relating to the sleep products used at the service including cots, bouncers, recliners and other products.
- A service sleep and risk assessment will be conducted at least every 12 months, and as soon as practicable after becoming aware of any circumstances that may affect the safety, health or wellbeing of children during sleep and rest.
 - The risk assessment must consider all the matters set out in regulation 84C (see appendix 2) of the National Regulations, or regulation 53C of the CS Regulations.
 - A copy of the risk assessments will be retained as a record.
- Educators will document children's sleep and rest needs and provide information to families about their child's sleep and rest patterns. (See below for further guidance)
- Educators will consult families to gather information about individual children's needs and preferences.
- In accordance with Red Nose recommendations, educators and parents/guardian will complete the *Infant Safe Sleep Individual Child Risk Factor Checklist and Action Plan* for all children aged 12 months and under, as this age group is at the highest risk of Sudden and Unexpected Infant Death (SUDI). This process should be undertaken in consultation with the child's parent or guardian, ideally during the enrolment or orientation period. Form needs to be complete before first day of attendance.
- Upon completion, the "Safe Sleep: Individual Child Risk Factor Checklist & Action Plan" must be uploaded to OWNA using the template "Upload - Safe Sleep: Individual

Child Risk Factor Checklist & Action Plan.” A hard copy is to be stored in the child’s documentation folder within the room. Once the child is over 12 months of age and the checklist is no longer required, the final version retained in OWNA will serve as the official record, and the hard copy may be securely disposed of.

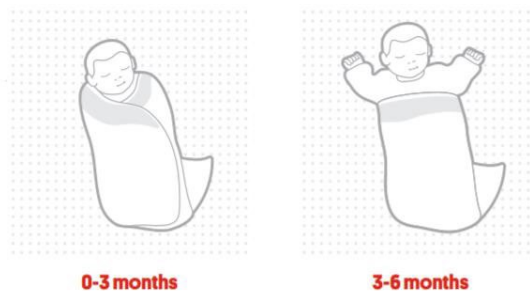
- Our team will create a relaxed environment by discussing with the children the routine for each day, outlining the rest time routine to suit the individual age groups and their level of understanding.
- Each room will have a daily routine outlining the times for rest /sleep times for the children. These routines will be flexible according to the ages of the children and their needs.
- No child will be forced to sleep. If a child chooses not to sleep they will be offered quiet activities or a cushion to lay down to relax/rest as an alternative.
- All children have rights and where appropriate, should have choice about sleeping and resting within the day.
- Opportunities will be provided to meet each child’s sleep, rest and relaxation needs and ensure children feel secure and safe.
- If a child requests a rest, or if they are showing clear signs of tiredness, regardless of the time of day, there should be a comfortable, safe area available for them to rest (if required). It is important that opportunities for rest and relaxation, as well as sleep, are provided.
- Educators need to plan for educational experiences and activities for children who do not sleep and also for children who have shorter rest/sleep routines. This allows children to engage in activities while other children are sleeping and resting.
- Taking into consideration the flexible routine rest/sleep times appropriate for the children’s age and their needs; educators need to ensure children’s needs are monitored and appropriate. It is recommended that children be given 5-10 minutes to stretch and gently wake up from a sleep. Once the child is fully awake, they need to be dressed and/or clothing tidied, hair tidied and face cleaned. Children should also visit the toilet or have their nappy changed if required. If children are going to the toilet shoes should be worn. Children are then encouraged to participate in an activity.
- Each room will provide an atmosphere that is quiet and peaceful, this will be achieved by playing quiet music (the radio is not appropriate), ensuring that ventilation, cooling and heating are monitored to suit the weather conditions.
- Children Entering Service:
 - If a parent is dropping off or collecting an older child with a child sleeping in a bassinet or pram, they can bring the bassinet, car capsule or pram into the centre as long as the child **is not enrolled** at the service and is being supervised by the parent/guardian at all times.
 - If a parent is dropping off a baby to a service that is enrolled in the service, parent/guardian cannot leave the child asleep in the pram, car capsule or bassinet. The child will need to be taken out of the pram/bassinet/capsule and placed in a service cot upon arrival.
 - To support safe sleeping practices and procedures educators will not accept sleeping children upon arrival and they will need to be woken up before being handed over to educator. The sleeping child’s wellbeing and health, needs to be assessed by educator on transfer to cot or bed. ie normal health (not ill or have an infectious disease) and breathing is normal. The educator sees the child awake and breathing normal at handover and then resettled to a cot or bed if needing a longer sleep.
- Room lighting: The room environment must maintain appropriate lighting levels to ensure children and educators can safely move around and comfortably participate in daily routines and activities. Lighting should be sufficient for clear visibility throughout the room, as environments that are too dark may impact supervision, safety, and children’s engagement in experiences. Adequate lighting can be supported through the use of natural light, including opening curtains or blinds, as well as artificial lighting where required. During sleep and rest periods, the environment should still maintain enough natural or ambient light to support a safe sleep environment and allow educators to effectively observe children’s breathing, positioning, and overall wellbeing while conducting regular sleep checks. Safe sleep practices rely on active supervision,

clear lines of sight, and effective monitoring to reduce the risk of injury or harm. The rooms should not remain in a full sleep-time set-up for a small number of children who are still resting, but should transition appropriately to support the needs of all children.

- With reference to age-appropriate sleep routines of the classroom; children may need to be encouraged to wake-up if continuing to sleep beyond recommend sleep time. To gently wake-up children, educators may gradually increase the level of light in the classroom and comfort the child to begin to wake-up.
- Rest/sleep routines need to be documented on the Daily Routine schedule and in OWNA for all children. The Daily Routine Schedule should be updated on completion of care routine to track tasks for educators and guardians to view.
- Each child will be provided with a Cot, Mattress (AS/NZ 2172) or Stretcher bed according to the child's age and ability.
- The team member's in each room will adequately and actively monitor and supervise resting children inline with the National Law, State Regulations and best practice standards. Physically checking/inspecting sleeping children at regular intervals (every 10 minutes for children who sleep or rest) by checking the rise and fall of the child's chest and the child's lip and skin colour from the side of the cot/bed. (source: <https://education.nsw.gov.au/early-childhood-education/eccec-resource-library/safe-sleep-and-rest>). When check/inspection is performed by an educator this information will be recorded on rooms Sleep Check Record. The form will be stored and filed by uploading to the OWNA Safe Sleep Check – Checklist.
- Parents will be asked to provide feedback and information about their child's sleeping patterns. Parents of children who are used to sleeping with a particular comfort object will be encouraged to bring the comforter to childcare to aid sleep in the new environment.
- Children **will not** be placed in cots, mattresses or beds with bottles as this may pose a safety risk to the child.
- Children are to rest with their face uncovered.
- All head coverings including hats, head bonnets and hooded clothing should be removed before sleep. Items such as necklaces, cultural neck attire, dummy chains, etc must be removed before sleep.
- Staff will report any potential hazards and discontinue use of the cot until hazard has been addressed.
- Supervision planning and the placement of educators across a service should ensure educators are able to adequately supervise sleeping and resting children.
- The care environment, including sleep and rest areas, should be well ventilated, either with fresh air or using mechanical ventilation.
- The sleep environment is free of potential hazards such as cords from hanging blinds, curtains, electrical appliances, and toy mobiles. Do not use hot water bottles, electric blankets or heated wheat bags.
- Children will not be left to sleep in the following: bonuncintetes, rockers, prams, pushers, sofas, bean-bags, hammocks, car-seat, etc.
- Equipment such as bumpers, infant positioners, inclined sleepers, or additional padding, mattresses, pillows and other soft items should not be used in the sleep and rest environment for children.
- Educators will assess whether older babies and toddlers have the ability to climb over the side of the cot. Where infants show they may be capable of climbing out of the cot, they should be transitioned to a safe bed or floor bed.
- Children sleeping and resting will always be adequately supervised so that educators can supervise children's safety and wellbeing.
- Educators will be able to visually check the child's sleeping position, breathing and the colour of the child's lips and skin, body temperature, head position, airway and the child's head and face, ensuring they remain uncovered.
- If a family's sleep/rest routine for their child conflicts with the recommendations from Red Nose - Safe Sleeping Practices the service is not to endorse this practice unless a written letter is provided by a medical practitioner.
- We will consult families about their child's routine for sleep and rest at home and carry this out at the service where possible and safe to do so, in line with the prevailing safe sleep practices recommended by Red Nose Australia.

Safe Resting Practices for Babies

- All children are to be placed on their back for all sleep and rest periods.
- Written direction from a medical practitioner (e.g. doctor) must be obtained by the parent/guardian before an educator will place a child in any other position (ie other than on their backs).
- If older babies turn over while sleeping, allow them to find their own sleeping position, but always lay them on their back when first placing them to rest.
- To prevent a baby sliding or wriggling under the sheets, place them with their feet closest to the end of the cot.
- Child should not be wrapped after 3 months of age or when child begins rolling. A parent may request for their older child to still be wrapped; the safety hazards should be discussed.
- Wrapped infants must have their arms free from 3 months of age and wrapping must be discontinued when infants shows readiness to roll. Do not introduce wrapping if the infant is not routinely wrapped at home.



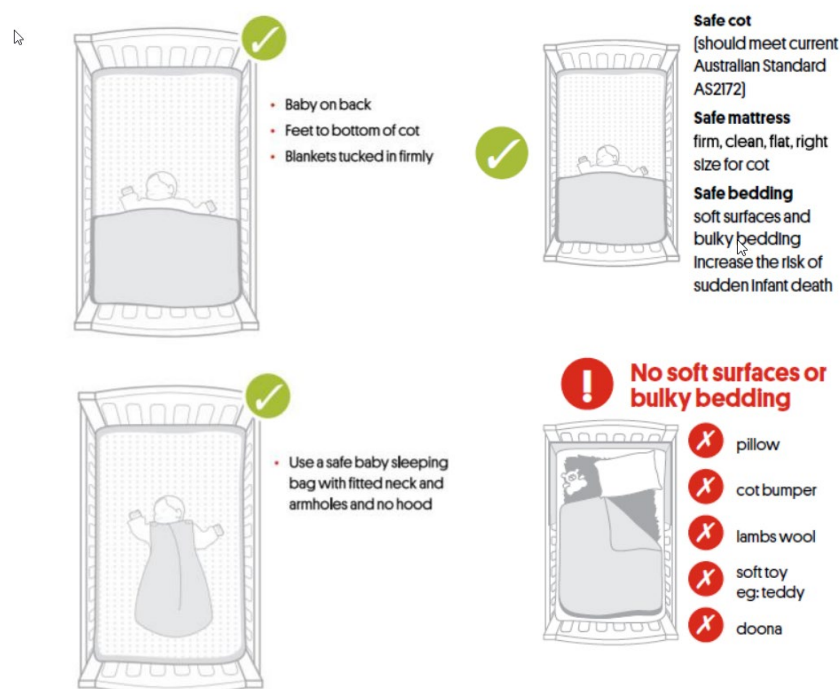
- Babies or young children should not be moved out of a cot into a bed too early; they should also not be kept in a cot for too long. When a young child is observed attempting to climb out of a cot, and looking like they might succeed, it is time to move them out of a cot.
- When a baby is placed to sleep, educators should check that any bedding is tucked in secure and is not loose. Babies of this age may be placed in a safe baby sleeping bag (i.e. with fitted neck and arm holes, but no hood). At no time should a baby's face or head be covered (i.e. with linen).
- Infants should be dressed with consideration to the room temperature. The infant should be warm but not hot to touch centrally (i.e. on the chest or abdomen). Bedding or clothing should be removed if the infant is hot to touch, or is flushed or sweating. Any clothing or jewellery that could pose a strangulation or choking hazard must be removed before the infant or child is placed in the sleep space.
- Pillows, soft toys, lambs wool, cot bumpers, quilts, etc are not to be placed in cots.
- Soft toys or comforters are not to be used in the cot before 7 months of age.

Dummy Use

- If a dummy is being used by an infant:
 - it may be offered for all sleep periods
 - it is recommended to be phased out between 6 -12 months of age
 - if a dummy falls out of a baby's mouth during sleep, it should not be re-inserted.
 - dummies should not be coated in anything sweet.
- Dummies and dummy containers should be cleaned regularly and returned to an individual, sealed and labelled container with the child's first and last names.
- Educators should ensure dummies are replaced if it becomes broken, pierced or unhygienic.

Protective Behaviours and Practices

- All children will be adequately and actively supervised by team members whilst resting and sleeping. Sleep check need to be performed and documented by educators.
- Children are to be monitored whilst sleeping and resting. Children should be checked every 10 minutes. The check will be documented on Sleep Check Record.
- Individual children sleep and rest information is required to be record in the OWNA App Child Daily Information (at the end of the sleep time)
- The Sleep Check Record (all children from) needs to be uploaded to the OWNA Sleep Record Check form each day via the OWNA Checklist tool.
- If the OWNA App is not working educator should use the paper version of sleep records and wellbeing schedules to perform checks and record this information.
- Students and volunteers are never to be left alone with children.
- Adults will not sleep or rest in the same environment as a child or group of children.
- When monitoring a child team members will observe the following;
 - the position of each child's body on the mattress/bed;
 - each child's breathing rate. If child is not breathing commence with First Aid;
 - the arrangement of bed linen. If a child's face is covered team members will immediately uncover the child's face;
 - the environment. Team members will be mindful of room temperature ensuring that it is comfortable for children. Check the cot sides are raised and in a locked position and check the safety of the rest of the environment.
 - Educators will check (visually) for signs of stress such as overheating or irregular breathing. If you can't see a child's chest rise or hear their breathing, you may need to gently put your hand on their chest to check.



Equipment for Rest Time

- All cots and other sleep equipment are in good condition, and meet the current Australian Safety Standard.
- Bassinets are prohibited to be used in all service premises. Regulation 84D requires approved providers and nominated supervisors to ensure that bassinets are not on

the education and care service premises at any time that children are being educated and cared for by the service.

- There is no definition of a bassinet under the National Quality Framework (NQF). The common design and characteristics of a bassinet may include, but are not limited to:
 - Size- smaller than a regular cot
 - Shape- may be basket- like, oval or shaped like a regular cot
 - Structure- may have handles, foldable or fixed legs or casters or ability to glide/rock
 - Portability- easy to carry or move and may be attached on a frame or a pram
 - Purpose- specifically intended for infants under 4-6 months old (before they show signs of rolling) and may be a stand-alone item, or an accessory (for example as a fitting on a pram).
- Many terms can be used to describe bassinets. This may include, but is not limited to:
 - Moses basket
 - co-sleeper
 - bassinette
 - cradle
 - bedside sleeper
 - carry cot
 - bassinet fitting (on a pram)
 - portable bassinet
- The following are examples of what a bassinet may look like:



- Prior to placing a child in a cot team members will check for any hazards ensuring that there are no loose or broken pieces or sharp edges paying particular attention to the safety latches. Team members will also check that mattresses are firm, in good condition(clean and flat, not elevated or tilted) and that it fits the base with no more a 20mm gap between the mattress and the side of the cot.
- The drop gate must be in the up and locked position whenever the cot is occupied.
- It is not recommended for stroller/prams/pushers be used as a sleep/rest aid for children in a room. When on an excursion and a stroller/pram/pusher is used, children need be restrained at all times when in a pram/stroller/pusher. Children are never to be left unattended in a stroller/pram/pusher. Team members are to be cognisant of not overloading pram handles which could cause prams/strollers/pushers to tip backwards.

Infant Incline Products & Equipment

- Incline products include rockers, bouncers, recliners, baby swings, car seats or capsules. Sleeping on an incline increases the risk of sudden death for babies from:
 - rolling into a position that blocks the airways; and
 - dropping their head onto their chest, which restricts their breathing.
- If babies are left unsupervised and fall asleep in these products, they risk serious injury or sudden death.
- Red Nose advises that infants spending long periods inclined in car restraints such as car seats, capsules or infant seats can also be at risk.
- Use of rockers and bouncers' guidance in early learning setting: children are only left in the rockers for short period of time and are actively supervised. Rockers are used to assist while attending to another and other children that maybe unsettled. Incline products (Eg. rockers or bouncers) are not to be used for sleeping. If an educator

notices that a child is drifting off to sleep in the rocker, the child is to be placed in a cots or bed to sleep.

- Sleep products – such as anti-roll pillows, sleep positioners, baby loungers, cot bumpers, pillows and soft toys. Soft and padded surfaces can stop babies from being able to self-correct if they roll over and babies may not have enough strength to lift their heads to breathe. Soft and padded surfaces also move to fit around the shape of a baby's head instead of remaining rigid. This increases the chance of suffocation and carbon dioxide rebreathing. Soft sleep related products can cover a baby's face and cause suffocation. For this reason, placing soft toys, pillows, or loose blankets in a baby's sleeping area is not to be used in cots or sleeping areas.
- Alternative sleeping locations – such as beanbags, sofas/couches, cushions, adult or bunk beds are not safe for infants. They may have dangerous gaps that may trap a child's head, neck or limbs. Read about what is required to meet the [mandatory standard for household cots](#).

Supervision

All educators continuously supervise sleeping children or infants during tummy time. This means:

- Sleeping children must be supervised by sight and sound and continuously monitored by educators every 10 minutes (who must have easy access to them).
- Cot rooms and rooms must always have enough light (natural and/or artificial) to allow for visual supervision of all children. Lights may be turned off provided there is enough light for educators to see all children's faces from the maximum distance in the room.
- Ratio's must be maintained at all times, including during rest and/or sleep times.
- Educators must monitor infants for signs of overheating (e.g. sweating and redness). If these signs are observed, the child should be awakened and dressed in lighter clothing.
- Infants who are engaging in 'tummy time' must do so on a comfortable and firm surface and must never be left unsupervised. If the infant falls asleep on their tummy, educators must place the infant into a cot on their back.

Safe Placement

- Ensure a safety check of sleep and rest environments is undertaken on a regular basis.
- If hazards are identified remove hazard immediately and follow our policies and procedures for the maintenance of a child safe environment.
- Ensure hanging cords or strings from blinds, curtains, mobiles, heaters and electrical devices are away from cots and mattresses.
- Do not use electric blankets, hot water bottles and wheat bags in cots.

Hygiene Practices

- Cot and stretcher beds are to be disinfected after each child's use or at the end of each day (whichever is more frequent).
- Each child is to have their own bed linen. Linen will be washed on a weekly basis. If linen is provided by families it is to be sent home at the end of each week for washing.
- Children's sleep and rest environments should be free from cigarette or tobacco smoke.
- To avoid third hand smoke risks, it should be recommended that any educators who are smokers:
 - Wear a smoking jacket.

- Practice respiratory etiquette (e.g., covering mouth while coughing).
- Practice oral hygiene (rinsing mouth)
- Practice hand hygiene (washing hands)
- Wear a hat or hair tied back while smoking.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 14/5/2026

Next Policy Review: 15/2/2027

Appendix 1: Service Sleep and Rest Risk Assessment

Plan prepared by: *Approved Provider and Nominated Supervisor in conjunction with Educator Team*

Plan prepared in consultation: *with Parents and Guardians of the Service*

Communicated with all relevant staff: *Yes (Reviewed annual as part of policy and procedure review)*

Activity	Hazard Identified	Risk Assessment	Elimination / Control Measure	Who?	When?
Safeguarding and Wellbeing – Selecting sleeping or resting equipment: bed or cot.	Children may climb out or fall out of cot resulting injury.	Moderate	-In consultation with family assess age and development stage of children being educated and cared to determine appropriate sleeping area; bed or cot. If child is able to stand in cot; move to bed. - educators understand the safety and suitability of cots, bedding and bedding equipment, having regard to the ages and developmental stages of children who will use them.	-Parent/Guardian -Room Leaders -Educators	Each sleep time.
Safeguarding and Wellbeing – culture and family requests that are outside Red Nose safe sleep and rest practices.	Potential unsafe cultural preferences and family requests for child's sleep and rest time.	Moderate	Service policy and procedures: - service will consult families about their child's routine for sleep and rest at home and carry this out at the service where possible and safe to do so, in line with the prevailing safe sleep practices recommended by Red Nose Australia. - educate and consult families in relation to safe sleep and rest practices. - having respectful conversations with families upon enrolment, during orientation and whilst they remain enrolled in our centre. These conversations build a shared understanding and commitment to safe sleep policies and practices. - communication of the sleep and rest policies and procedures to a parent	-Parent Guardian -Nominated Supervisor -Responsible Person -Room Leaders -Educators	Review daily
Safeguarding and Wellbeing – children's health and medical needs that may impact safe sleep or rest.	Ensure health care needs of each child is catered for and managed.	Moderate	- Parents required to provide medical action/management plans for asthma, anaphylaxis, epilepsy, and diabetes, if diagnosed. Parents also required to complete a Service 'Medical Risk Minimisation and Communication form' -Individual care needs of children are met.	-Parent Guardian -Nominated Supervisor -Responsible Person -Room Leader -Educators	Review daily
Safeguarding and Wellbeing – ensuring staffing arrangements to adequately supervise and monitor children during sleep and rest periods.	Insufficient supervision resulting in harm to a child.	Low	- Service follows required Regulations in relation to child to staff ratio. Correct staffing levels. - Supervision plans discussed and followed by educators to adequately supervise and monitor children. - Head counts to ensure all children are present, monitored and care for.	-Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily

			- sleep and rest practices are consistent with current health guidelines and best practices to ensure the safety of children during sleep and rest. -Service Policy and Procedure: Supervision planning and the placement of educators across a centre should ensure educators are able to adequately supervise sleeping and resting children.		
Safeguarding and Wellbeing - level of knowledge and training of the staff supervising children during sleep and rest periods	- Educator not having knowledge of safe sleep and rest practices. - New or temporary staff unaware of safe sleep and rest procedures.	Low	- Service has a detailed Safe Sleep and Rest Policy - Policy and procedures are reviewed annually as a team. - Educators undertaken training in relation to safe sleep and rest practices. - Educators knowledge of first aid. - the induction, training and knowledge of staff at the service in relation to best practice for children's sleep and rest.	-Approved Provider -Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Safeguarding and Wellbeing – Regular checking of children while resting and sleeping.	Due to lack of supervision potential case of SIDS or choking.	Low	-adequate supervision and monitoring during sleep and rest periods. Follow sleep safety guidance. Checks will be made every 10 minutes. -Documentation: Sleep check schedule is maintained by educators of sleeping periods and checks performed by educators. -following red nose safe sleep recommendations detailed in service policy and procedure: 1) Always place baby on their back to sleep 2) Keep babys face and head uncovered	-Nominated Supervisor - Responsible Person -Room Leaders -Educators	Per policy
Safeguarding and Wellbeing – assessing the cots and beds within the sleep and rest areas.	Sufficient spacing of cots. Issues with cots such as mattresses sag, elevated or tilted) and has 20mm gap or more between the mattress and the side of the cot.	Moderate	Service Policy: - baby to sleep is in their own safe space, with a safe mattress, and safe bedding. Baby placed on their back to sleep, with their feet at the bottom of the bassinet or cot. - Equipment: Safe beds, cot, mattress in accordance with Australian standards - Lightweight bedding, firmly tucked in and only pulled to the chest. - Service Policy and procedure: Prior to placing a child in a cot team members will check for any hazards ensuring that there are no loose or broken pieces or sharp edges paying particular attention to the safety latches. Team members will also check that mattresses are firm, in good condition(clean and flat, not elevated or tilted) and that it fits the base with no more a 20mm	-Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily

			gap between the mattress and the side of the cot.		
Safeguarding and Wellbeing – assessing the physical safety and suitability of sleep and rest environments.	- Potential high temperature in room - Insufficient lighting and ventilation	Low	- the management of physical safety and suitability of sleep and rest environments including temperature, lighting and ventilation. See service policy above on safe sleeping practices. Service Policy: Classroom lighting: The classroom needs to have sufficient light. The level of light present in the classroom is at a level to allow educators and children to see and orientate themselves throughout the room. If the room is too dark children and educators may not be able to see clearly. Further, sufficient light should be present for children engaged in activities. Service Policy: The care environment, including sleep and rest areas, should be well ventilated, either with fresh air or using mechanical ventilation. Service Policy: Sleep and rest environments and equipment should be safe and free from hazards (e.g. no power points and cords etc accessible to children, mobiles hanging over cots etc)	-Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Safeguarding and Wellbeing - potential hazards in sleep and rest areas or on a child during sleep and rest periods	- items hanging near cots (ie on walls) and able to be reached by children while in cots (remove). No loose item or potential loose item should be hung near cots. ie to reduce risk of item becoming unattached and falling in hands of children. - item hanging or draped over cots (remove). - Damaged or broken equipment: beds and cots. - Presence of pillows, soft toys, lambs wool, cot bumpers, quilts, etc in cots. - Necklaces - Tight clothing	Low	- See service policy and procedure for guidance on necklaces and clothing. (Removal) - Educators undertake visual checks on beds and cots before placing child for sleep and rest. - Service Policy: Presence of pillows, soft toys, cot bumpers, quilts or comforters are not to be used in the cot. - Safety checks of the equipment and areas are carried out to ensure that the equipment continues to be safe. -Periodic OH&S checklists completed by service OH&S officer.	-Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Safeguarding and Wellbeing - Lifting and carrying of children and babies	-manual handling causing injury -manual handling resulting in strain/back injury	Low	-Parent and staff to follow manual handling procedures.	- Approved Provider -Nominated Supervisor - Responsible Person	Review daily

				-Room Leaders -Educators	
Safeguarding and Wellbeing – fire while child is sleeping or resting.	-fire while child is sleeping or resting and needing to evacuate the service	Low	-Service team practices fire drills and have an emergency evacuation procedure in place. -Service has emergency cots to transport children during drills and emergencies. - emergency plans are located in a prominent location	- Approved Provider -Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Safeguarding and Wellbeing- Children being left unattended in/ on inclined surfaces	Positioning of child could cause/lead to suffocation	Low	-Children will not be left to sleep in inclined sleepers - Children will not be left in inclined sleepers for extended periods of time -Children who are in inclined sleepers will be within reach of an educator at all times. -An educator will be present at all times while a child is drinking a bottle	- Approved Provider -Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Health and Wellbeing – exposure to smoke after birth.	Exposure to smoke after birth can increase risk of SIDS and SUDI	Low	- Educating parents and educators about smoking and impacts on children they care for. - Smoking during pregnancy and around baby once they are born increases the risk of sudden infant death – this includes second-hand smoke. - Service policy and procedure: To avoid third hand smoke risks, it should be recommended that any educators who are smokers. See detail of policy above.	- Approved Provider -Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily
Health & Infection Control - Beds and cots bedding and surfaces.	Spread of infections and diseases.	Low	- Clean bedding will be provided for individual children. - Infection prevention/control policy in place - Exclusion policy for staff and children in place. Exclusion period dependent on illness/ disease - Posters displayed on correct hand washing techniques and children supervised and assisted to wash hands if necessary. - Service Policy and Procedure: 1. Cot and stretcher beds are to be disinfected after each child's use or at the end of each day (whichever is more frequent). 2. Each child is to have their own bed linen. Linen will be washed on a weekly basis. If linen is provided by families it is to be sent home at the end of each week for washing.	- Approved Provider -Nominated Supervisor - Responsible Person -Room Leaders -Educators	Review daily

Risks identified from this risk assessment have been addressed within policy and procedure (regulation 84C) as well as other matters required under regulation 84C, including:

- the number, ages and development stages of children being educated and cared for, including at each education and care service
- the sleep and rest needs of children at the service (including specific health care needs, cultural preferences, sleep and rest needs of individual children and requests from families about a child's sleep and rest)

- suitability of staffing arrangements required to adequately supervise and monitor children during sleep and rest periods
- the level of knowledge and training of the staff supervising children during sleep and rest periods
- the location of the sleep and rest areas, including the arrangement of cots and beds within the sleep and rest areas
- the safety and suitability of any cots, beds and bedding equipment and having regard to the ages and developmental stages of the children who will use them
- any potential hazards in sleep and rest areas or on a child during sleep and rest periods
- the physical safety and suitability of sleep and rest environments, including temperature, lighting and ventilation

Risk Matrix						
	Likelihood					
Consequences		Rare	Unlikely	Possible	Likely	Almost certain
	Major	Moderate	High	High	Critical	Critical
	Significant	Moderate	Moderate	High	High	Critical
	Moderate	Low	Moderate	Moderate	High	High
	Minor	Very low	Low	Moderate	Moderate	Moderate
	Insignificant	Very low	Very low	Low	Moderate	Moderate

Appendix 2: Extract of Education and Care National Regulations

84C Risk assessment for purposes of sleep and rest policies and procedures

- (1) The approved provider of an education and care service must ensure that a sleep and rest risk assessment is conducted in accordance with this regulation—
 - (a) at least once every 12 months; and
 - (b) as soon as practicable after becoming aware of any circumstance that may affect the safety, health or wellbeing of children during sleep and rest.
- (2) A risk assessment must consider the following—
 - (a) the number, ages and developmental stages of children being educated and cared for—
 - (i) by the education and care service; or
 - (ii) in the case of a family day care service, at each family day care residence or approved family day care venue of the service;
 - (b) the sleep and rest needs of children being educated and cared for (including health care needs, cultural preferences, sleep and rest needs of individual children and requests from families about a child's sleep and rest)—
 - (i) by the education and care service; or
 - (ii) in the case of a family day care service, at each family day care residence or approved family day care venue of the service;
 - (c) the suitability of staffing arrangements to adequately supervise and monitor children during sleep and rest periods;
 - (d) the level of knowledge and training of staff supervising children during sleep and rest periods;
 - (e) the location of sleep and rest areas, including the arrangement of cots and beds within the sleep and rest areas—
 - (i) at the education and care service; or
 - (ii) in the case of a family day care service, at each family day care residence or approved family day care venue of the service;
 - (f) the safety and suitability of any cots, beds and bedding equipment, having regard to the ages and developmental stages of the children who will use the cots, bed and bedding equipment;
 - (g) any potential hazards—
 - (i) in sleep and rest areas; or
 - (ii) on a child during sleep and rest periods;
 - (h) the physical safety and suitability of sleep and rest environments (including temperature, lighting and ventilation)—
 - (i) at the education and care service; or
 - (ii) in the case of a family day care service, at each family day care residence or approved family day care venue of the service;
 - (i) in the case of a family day care service that provides overnight care to a child, any risks that the overnight care provided at the family day care residence or approved family day care venue of the service may pose to the safety, health or wellbeing of the child.
- (3) As soon as practicable after conducting a risk assessment under this regulation, the approved provider of an education and care service must make any necessary updates to the sleep and rest policies and procedures.
- (4) The approved provider must keep a record of each risk assessment conducted under this regulation.

Appendix 2: Infant Safe Sleep - Individual Child Risk Factor Checklist and Action Plan

STEP 1: Per Red Nose Recommendations, the “Infant Safe Sleep Individual Child Risk Factor Checklist and Action Plan” will be completed for all children under 12 months of age. Red Nose advise this group is at the highest risk.

This Checklist Tool should be accessed in conjunction with the Red Nose Safe Sleep Individual Child Risk Assessment Guide. (see: <https://rednose.org.au/resources-category/early-childhood-educators/>)

This checklist tool is to be used by educators with Parents/Guardian to support the identification of key risk factors that may increase an infant's vulnerability to Sudden and Unexpected Infant Death (SUDI) during sleep. The checklist can be completed as part of the child's orientation.

Educators will use the checklist as an initial step to identify, assess, and manage potential risks of harm. It is important to recognise that additional risks may exist depending on the individual child and their sleep environment.

If further guidance or clarification is required, educators should contact Red Nose for advice, support, and up-to-date safe sleep information. A copy of the checklist and action plan can be found on Red Nose website: https://rednose.org.au/downloads/Infant_sleep_Risk-assessment-checklist.pdf



Education and care service name		Child's surname	
Educator's full name		Child's first name	
Service/venue/ residence address		Date of birth	
Assessment date			
Review date			

Safe Sleep

Individual Child Risk Factor Checklist and Action Plan

This Checklist Tool should be accessed in conjunction with the Safe Sleep Individual Child Risk Assessment Guide.

Note: This Checklist Tool is intended to assist educators with identifying key risk factors in individual children which may make them more vulnerable to Sudden and Unexpected Infant Death (SUDI) occurring during sleep. This checklist is focused on infants 12 months of age and below as this is the time of greatest risk. The risk factors in the below table are designed as a starting point for identifying, assessing and managing risk of harm.

There may be other potential risks to the individual child or in the sleep environment. Please contact Red Nose, if you have any difficulty with this tool or if you require any further clarification or information and support for safe sleep.

Also access:
[Safe Sleep Individual Child Risk Assessment Guide](#)

Considerations		Yes	No	Notes/Comments
Age	Is the child aged 0-6 months?			
Smoking exposure	Is there parental/ caregiver smoke exposure?			
Was the child born prematurely?	Was the child born before 37 weeks gestation?			
Social circumstances	Does the child regularly see a healthcare professional such as a MCH Nurse, GP, Paediatrician?			



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Individual Child Risk Factor Checklist and Action Plan
Aug 2023 - Page 1

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In relation to Question “Feeding = is the child formula fed?” Based on information from Red Nose it advised that breastfeeding is associated with a lower risk of SIDS. Services often record feeding type as part of overall safety safe sleep practices risk assessment.

The service contacted Red Nose on the 24 March 2026 to asks what action plans the service implement if the answer to the year was “Yes”. Red Nose informed the service that we do not need to use the question in the risk assessment, they are reviewing this question in the template. The Service will add the following note in the Risk Assessment for the Risk Identified and the Intervention Strategies; “No alteration to procedures required. Educators will implement Safe Rest and Sleep policy and procedures”.

Considerations		Yes	No	Notes/Comments
Medical history	Does the child have a known medical condition and/ or disability such as: Asthma, repeated respiratory infections, chronic lung disease or other (please state)?			
Cultural practice	Are there any cultural practices that can compromise an airway during sleep such as: religious jewellery, head coverings etc?			
Sleep aids	Does the child use a sleep aid such as: soft toys, comforter, special blanket, bottle in bed, sleep in a pram or swing?			
Feeding	Is the child formula fed?			

Ticked 'YES' to any of the risk factors, complete ACTION PLAN over page.

Ticked 'YES' to any of the risk factors, complete ACTION PLAN over page.



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Individual Child Risk Factor Checklist and Action Plan
Aug 2023 - Page 2

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Ticked 'YES' to any of the risk factors, complete ACTION PLAN

Action Plan

(For risk factors identified, list strategies below to manage the risk)

Risk Identified	Intervention Strategies	Date Actioned
Example 1: Child spent time in a Special Care Nursery/ NICU Example 2: Parents request child wears a religious necklace at all times	<i>I understand this may increase the risk to the child and I will increase the frequency and duration of checks of this child for all sleeps and naps</i> <i>I will discuss with the family the importance of sleep safety and the risks of sleeping with necklaces. I will find a mutually agreeable solution in which the child does not sleep with the necklace on.</i>	
Feeding: Child is Formula Fed? Yes	No alteration to procedures required. Educators will implement Safe Rest and Sleep policy and procedures.	<Date of first day of enrolment>
List other action plans here		

Review

(Reviews of this risk assessment should occur at intervals set by the service and/or if there are changes to the child's circumstances i.e. age and developmental changes in the child, health status, etc.)

Review Date	Risk Factors Ongoing/Resolved (List)	Revised Action Plan (Y/N)	Signed
Any changes – list here			

The above risk assessment for _____ has been completed in consultation with the child's parent/guardian and service management.

Parent full name:		Approved by:	
Parent Signature:		Name:	
Date:		Role:	
		Signature:	
		Date:	

Signed by Director and Room Leader



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Individual Child Risk Factor Checklist and Action Plan
Aug 2023 - Page 3

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STEP 2: Upload completed form to OWNA

Once the Infant Safe Sleep Individual Child Risk Factor Checklist and Action Plan is completed upload using the OWNA App: see template “Upload - Safe Sleep: Individual Child Risk Factor Checklist & Action Plan”

File a hard copy of the completed form in the Child Documentation Folder in the Room. A final version of form is uploaded to OWNA and can be disposed once stored electronically and child is greater than 12 months old and the Sleep Individual Child Risk Factor Checklist and Action Plan form is no longer required.

Learning Documents

Home / View All / Build your Learning Document

Custom Form has been successfully updated...

Title*

Upload - Safe Sleep: Individual Child Risk Factor Checklist & Action Plan

Description

Upload document storage tool - Safe Sleep: Individual Child Risk Factor Checklist & Action Plan (Red Nose)

☒ Share with Group ☐ Share on OWNA Template Marketplace

Build Form Below

Safe Sleep: Individual Child Risk Factor Checklist & Action Plan (Form Upload)

This Checklist Tool should be accessed in conjunction with the Safe Sleep Individual Child Risk Assessment Guide. This Checklist Tool is intended to assist educators with identifying key risk factors in individual children which may make them more vulnerable to Sudden and Unexpected Infant Death (SUID) occurring during sleep. This checklist is focused on infants 12 months of age and below as this is the time of greatest risk. The risk factors in the below table are designed as a starting point for identifying, assessing and managing risk of harm.

Child *

Please select

Room *

Please select

Date Completed *

e.g. 2016-10-31

Reference:

NQS

QA2	2.1.1	<i>Each child's wellbeing and comfort is provided for, including appropriate opportunities to meet each child's need for sleep, rest and relaxation.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Section 165	<i>Offence to inadequately supervise children</i>
Section 167	<i>Offence related to protection of children from harm and hazards</i>
Regulation 81	<i>Sleep and Rest</i>
Regulation 82	<i>Tobacco, drug and alcohol-free environment</i>

Regulation 84A	<i>Sleep and rest</i>
Regulation 84B	<i>Sleep and rest policies and procedures</i>
Regulation 84C	<i>Risk assessment for purposes of sleep and rest policies and procedures</i>
Regulation 87	<i>Incident, injury, trauma and illness record</i>
Regulation 103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
Regulation 105	<i>Furniture, materials and equipment</i>
Regulation 106	<i>Laundry and hygiene facilities</i>
Regulation 107	<i>Space requirements – indoor space</i>
Regulation 110	<i>Ventilation and natural light</i>
Regulation 115	<i>Premises designed to facilitate supervision</i>
Regulation 116 (FDC only)	<i>Assessments of family day care residences and approved family day care venues</i>
Regulation 168	<i>Education and care service must have policies and procedures</i>
Regulation 169	<i>Additional policies and procedures – family day care</i>
Regulation 170	<i>Policies and procedures to be followed</i>
Regulation 171	<i>Policies and procedures to be kept available</i>
Regulation 172	<i>Notification of change to policies or procedures</i>

EYLF

LO3	<i>Children recognise and communicate their bodily needs (for example, thirst, hunger, rest, comfort, physical activity).</i>
	<i>Educators engage children in experiences, conversations and routines that promote healthy lifestyles and good nutrition.</i>
	<i>Educators consider the pace of the day within the context of the community.</i>
	<i>Educators provide a range of active and restful experiences throughout the day and support children to make appropriate decisions regarding participation.</i>

Other References:

Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2007
Australian Standards for Household Cots AS/NZ 2172

Sources and Further Reading:

- www.rednose.com.au
- National Childcare Accreditation Council
(http://www.ncac.gov.au/policy_development/policy_templates.asp) Retrieved 25 June, 2008
- Red Nose: Safe sleeping in child care kit.
- Australian Competition and Consumer Commission (ACCC) Safety Alert: Cots Retrieved 28 May 2008 from
<http://www.accc.gov.au/content/item.phtml?itemId=646262&nodeId=4958ea860a95650a51f16e5fc99c980d&fn=Cot%20safety.pdf>
- Australian Competition and Consumer Commission (ACCC) (2008) Keeping baby safe: A guide to nursery furniture. Retrieved 10 June 2008 from
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- Kidsafe Child Accident Prevention Foundation of Australia Kidsafe QLD (Inc) (2006) Nursery Equipment Fact Sheet. Retrieved 28 May 2008 from
<http://www.gtp.com.au/kidsafeqld/inewsfiles/inews.15184.1.pdf>
- ACCC: Infant inclined products and sudden death risks: Website page:
<https://www.productsafety.gov.au/products/babies-kids/kids-equipment/infant-inclined-products-and-sudden-death-risks>

41 - SMOKE, VAPE, ALCOHOL & DRUG FREE ENVIRONMENT POLICY

Position Statement

The key to tobacco, e-cigarettes, alcohol and other drugs in the early years is prevention. Research has shown that any form of environmental tobacco smoking, and e-cigarettes and vaping is a health risk. There is now sufficient evidence to conclude that passive smoking and vaping increases the risk of heart disease and lung cancer as well as being responsible for triggering asthma attacks, causing watery eyes, headaches and sore throats (Cancer Council, 1992). The childcare environment will also be free of recreational (illicit) drugs and alcohol.

We recognise that every member of the service impacts the children's health and can contribute to creating a healthy environment. All members of our service will be supported to meet this policy.

Objective

As a health promoting service, we will provide a smoke-, vape- and drug-free environment for children, parents and team members, and promote low-risk alcohol consumption through learning, policies, creating a safe and healthy physical and social environment and developing community links and partnerships.

This policy confirms our commitment to:

- Ensures a smoke-, vape- and drug-free environment for children, families, educators, staff, volunteers and visitors.
- Promote low-risk alcohol consumption to our service community.
- Encourage educators and staff to build on opportunistic learning moments with children.
- Provide information to educators, staff and families about the health benefits of not smoking, vaping or taking drugs, and low-risk alcohol consumption.

Definitions:

Smoking: Refers to tobacco, e-cigarette and vaping smoking.

Tobacco, vape, alcohol and other drugs: Focusses on creating smoke-, vape-, and drug-free environments and low-risk alcohol consumption. Smoke- and vape-free refers to both traditional and electronic cigarettes (i.e. e-cigarettes)

Tobacco product: Any device or product that contains organic matter that is heated or burned to create aerosol or smoke that is inhaled by the user

E-cigarette product: Any electronic device that is intended to be used to create an aerosol for inhalation and any liquid, vape or product containing liquid, intended to be used in or with an electronic device to create an aerosol for inhalation. Also known as vaping substances and vaping devices.

Drug: Any substance taken to change the way the body and/or mind functions³

Alcohol: A depressant drug that slows down activity in the central nervous system⁴

³ Australian Drug Information Network, 2012, Glossary <https://adf.org.au/resources/did-glossary/>

⁴ Australian Drug Information Network, 2012, Glossary <https://adf.org.au/resources/did-glossary/>

Related Policies

- 41- Education, Curriculum and Learning Policy
- 92 - Health and Wellbeing Policy
- 10 - Occupational Health & Safety Policy

Implementation

- The childcare centre and grounds are a smoke, e-cigarette (vaping substances and devices), alcohol and illicit drug free environment.
- Cigarette, vape substances or devices, e-cigarettes and unlawful substances are not permitted on staff member person or pockets while on duties educating and caring for children.
- Smoke-free (tobacco, e-cigarette, vaping) signage is clearly displayed and can be understood by the whole community.
- It is prohibited to smoke or vape within 10 metres (front, rear and side) of the children services premises boundary. Per the Government Regulation, the public are not allowed to smoke or vape near premises within 5 metres.
- Staff are not to congregate near children services premises to smoke or vape.
- Smoking, vape, e-cigarettes and unlawful substances will not be permitted in the building or on the grounds of the centre when the service is being provided to children.
- First, second and third hand smoke and e-cigarette aerosol must not be present on staff while caring and educating for children and working with other educators. It is the responsibility of all people to ensure they do not expose people to first, second and third hand smoke or e-cigarette aerosol. Staff exposed to smoke and e-cigarette aerosol (first, second and third hand) are to implement strategies to ensure that uniform clothing and skin do not retain residual smoke, smoke chemicals or e-cigarette aerosol. These strategies includes but not limited to; washing of hands and wearing non-uniform clothing while smoking to reduce the absorption of smoke and e-cigarette aerosol into uniform clothing and skin.
- The policy applies at all times even when the centre is "child free".
- Team members who wish to smoke during working hours must leave the premises during their rostered breaks.
- Alcohol will not be permitted in the building or on the grounds of the centre when the service is being provided to children.
- Any persons smoking on any part of the premises when the service is being provided to children will be immediately removed from the centre.
- Any persons under the influence of drugs or alcohol when the service is being provided to children will be immediately removed from the centre.
- Parents will be advised of this policy on enrolment and will be expected to comply.
- Tobacco, e-cigarette, alcohol and other drugs policy requirements are included in educator and staff induction/orientation.

In order to keep children and staff free from the dangers of tobacco, e-cigarette and vaping smoke, the following rules apply.

Smoking and vaping is prohibited in all areas of the centre including:

- Inside
- Outside in the playground.
- Outside in the car-park.

Smoking and vaping is also prohibited:

- On excursions.
- While travelling with a Child.
- At staff meetings.
- At parent meetings.

Healthy Policies

- Educators, staff, families and children are active participants in the development and implementation of the whole service tobacco policy.
- Educators, staff and families are provided with information about policy requirements.

Healthy Physical Environment

- The service provides a totally smoke-, vape- and drug-free environment.
- All service sanctioned events (both on and off premises), including excursions, fetes and celebrations are totally free from tobacco smoke, e-cigarettes and vapes and drugs.
- If there is alcohol at a staff or family event, management ensures legal requirements related to the sale and supply of alcohol are met. Responsible consumption of alcohol is promoted.
- If external organisations use the premises, a formal agreement ensures the service premises are smoke-, vape-, and drug-free and promotes low-risk alcohol consumption.

Healthy Social Environment

- Staff who want to quit smoking or vaping, drinking or using other drugs are supported and referred to appropriate agencies
- As role models, educators, staff and families present smoke-, vape- and drug-free behaviour in line with this policy.
- Service contractors and visitors model smoke-, vape- and drug-free behaviour and refrain from smoking or vaping whilst in view of children.
- The service avoids encouraging the consumption of alcohol as part of awards, gifts and fundraising.
- Staff, educators and families are encouraged to engage in low-risk alcohol behaviours and abstain from taking drugs, particularly around children.

Learning and Skills

- Tobacco, e-cigarettes, vaping, alcohol and drug education and related health messages are reinforced in opportunistic learning moments.
- Educators are supported to access resources, tools and professional learning to enhance their knowledge and capacity to provide opportunistic learning about health and wellbeing initiatives.

Engaging Children, Educators, Staff and Families

- Families, children, educators and staff are key partners in developing and supporting initiatives.
- Information, ideas and practical strategies are provided to families, educators and staff on a regular basis to promote a good wellbeing at the service and home.
- Resources and information about the health risks related to smoking, vaping, high-risk drinking and taking other drugs are available to educators, staff and families, including information about accessing support services.
- The service engages families in health and wellbeing initiatives.
- The service engages with families and children from culturally diverse backgrounds to ensure cultural values and expectations about tobacco, e-cigarette, alcohol and other drugs are considered.

Community and Partnerships

- The service will engage local health professionals, services and other organisations to increase educator's capacity to deliver and promote tobacco, alcohol and drug prevention, and seek support in harm reduction initiatives where appropriate .
- The service has no partnerships with organisations that market or supply alcohol or tobacco.
- Information is provided to educators, staff and families about local support, as required.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/5/2026

Next Policy Reviewed: 15/5/2027

Reference:

NQS

QA2	2.3.2	Every reasonable precaution is taken to protect children from harm and any hazard likely to cause injury.
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National

Regulations

Reg	82	Tobacco, drug and alcohol free environment
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Other References:

Handbook on Child Care Licensing

Healthy Achievement Program – Tobacco Control is one of the Achievement Areas

QIAS Principle 7.2

Anti-discrimination Act

Privacy Act 1988

Cancer Council 1992

<http://www.melbourne.vic.gov.au/COMMUNITYSERVICES/HEALTH/Pages/Smokingtobacco.aspx>

Legislation:

Occupational Health & Safety Act 2004

Tobacco Act 1987

Child Wellbeing and Safety Act 2005 (VIC) (Part 2: Principles for Children)

42 - SUN PROTECTION AND SAFETY POLICY

Position Statement

Too much of the sun's UV can cause sunburn, skin and eye damage and skin cancer. Exposure to the sun's UV during childhood and adolescence is associated with an increased risk of skin cancer in later life. Vitamin D is essential for healthy bones and muscles, and for general health.

Centre team members will support the "Hat's on, Fun's on" policy. The service holds a SunSmart Membership. Educators and children are encouraged to access the daily local sun protection times at sunsmart.com.au or on the free SunSmart app to assist with the implementation of this policy. A combination of sun protection measures are used for all outdoor activities from mid-August to the end of April and whenever UV levels reach 3 and above which may extend beyond these months.

Where possible, active, outdoor sun safe play is encouraged throughout the day. SunSmart practices consider the special needs of infants. All babies under 12 months are kept out of direct sun when UV levels are three and above.

Objective

To protect children and team members from the damaging effects of overexposure to UV rays.

Implementation

All staff should follow the Sun Smart principles; including applying sunscreen, wearing suitable clothing and hat per the Sun Smart and Cancer Council recommendations. Sun Protection and Safety forms part of staff member induction and annually the policy will be reviewed for ongoing education. Educators need to role model sun protection and safety.

When enrolling their child, families are informed of the service's SunSmart policy.

Shade

- Provision of shady areas will be a priority for the centre, and will at least meet the minimum regulatory requirements. Management makes sure there is a sufficient number of shelters and trees providing shade in the outdoor area.
- Team members will initiate discussions with children about sun safety and encourage independent efforts to engage in sun safe behaviour.
- Team members when planning their programmes will give thoughtful consideration to the location of outdoor activities, planning (where possible) for these experiences to occur in shaded areas.
- Children are encouraged to choose and use available areas of shade when outside.
- Children who do not have appropriate hats or outdoor clothing are asked to choose a shady play space or a suitable area protected from the sun.

Suitable clothing

Parents will be encouraged to provide clothing that will protect children from the sun. Loose fitting clothing that protects as much of the child's skin as possible during outdoor play. Shirts that cover the shoulders and have collars and sleeves that are at least elbow length and longer style shorts or skirts are most suitable. If a child is not wearing sun-safe clothing (e.g. a singlet top or shoestring dress) they will be required to choose a t-shirt/shirt to wear over the top before going outdoors.

If a child comes to the service in inappropriate clothing and does not have appropriate clothing their bag for parent to change child into before commencing, the parents will be required to either take the child home and return with the appropriate clothing or bring in the appropriate clothing within a reasonable time frame (30 minutes) to the service for the child to wear.

The use of Sunscreen

- SPF 50 or higher Broad-Spectrum Water-Resistant Sunscreen will be supplied for the use of all children by the Centre.
- Team members, with parental permission, will thoroughly apply SPF 30 or higher Broad-spectrum water-resistant sunscreen to children 20 minutes before outdoor activity time and reapply every two hours.
- To help develop independent skills ready for school, children from three years of age are given opportunities to apply their own sunscreen under supervision of staff, and are encouraged to do so.
- Sunscreen will be reapplied every two hours when outdoors.
- Parents are requested to apply SPF 50 or higher broad-spectrum water-resistant sunscreen to children before coming to the Centre for the day, or to apply sunscreen to children upon arrival.
- Parents who wish to supply their own sunscreen should leave personal sunscreens with a team member, in order to limit access by other children. Please complete the HEI009_Authority to Administer Nappy Cream Sun Cream Mosquito Repellent(Medication Form)
- Sunglasses - The use of wrap-around sunglasses that meet Australian Standard 1067 (sunglasses – category 2, 3 or 4) is encouraged.
- Written consent is required to be provided by parents to instruct the staff to follow their instructions if do not wish for sun screen to be applied to their child's skin.

Hats on, Fun's On

- Children will be encouraged to wear hats when playing outside – “Hat's On – Fun's On”. Hats should be worn by children and educators when playing outdoors to reduce exposure to UV.
- Team members will at all times endeavour to be role models by wearing appropriate hats, sun protective clothing and sunscreen and seek shade when outdoors.
- Hats for children should not fall off easily, offer protection to face, ears and neck, and be acceptable to the child. All children and team members wear a broad brimmed, legionnaire or bucket hat. Please note that bucket hats must have a deep crown and a minimum 5cm brim for children under 5 years of age and 6cms for older children and adults. Baseball caps are not recommended for protection against the sun and children wearing caps will need to play in shaded areas.
- Parents will be encouraged to follow procedures (e.g. wearing broad brimmed hats and or the legionnaire style hat) particularly when involved in centre activities.
- Staff and families are provided with educational materials on sun protection, vitamin D, the centres Sun Smart policy and ways they can support it.
- The Sun Protection and Safety policy is reinforced by educators and through children's activities and displays

Sun Protection and Infants

Babies under 12 months of age have particularly sensitive skin and children of this age will be protected from direct sunlight and are to remain in dense shade when outside. If babies are kept out of the sun or well protected from UV radiation by clothing, hats and shade, then sunscreen need only be used occasionally on very small areas of a baby's skin. The widespread use of sunscreen on babies under 6 months old is not recommended. SunSmart does not ask that you keep children inside during the middle of the day but advised to make sure children are well protected for all outdoor play during the sun protection times.

Hot Weather Guidance

A 'heatwave' is generally defined as a period of abnormally and uncomfortably hot weather that could impact on human health, community infrastructure and services. Once forecast average temperatures are predicted to reach or exceed the heat health temperature threshold for a specific weather forecast district, the VECRA (or another Government agency) will issue a heat health alert for that district.

The following guidance will be followed during a heatwave and primarily during Terms 1 and 4 as temperatures can reach above 35C.

1. When the ambient temperature at the service is over 28C children will be encouraged to play in cooler and shaded areas (educators will move play spaces and equipment into cooler and shaded area) as the concrete, artificial turf and surfaces of equipment i.e. metal A-frames / outdoor furniture may be hotter to the touch when in direct sunlight. Also children playing in full sun are more likely to be effected by the heat.
2. On days where the forecast temperature exceeds 28C, before children play in areas with rubber play safe, artificial turf and metal climbing equipment staff will conduct a touch test to ensure surfaces are safe for children to use. (If an adult can hold their hand on a surface for a slow count to 10 the surface is safe for use.
3. When temperatures exceed 38C at site location children will remain inside. If outdoor cooling equipment is available at service (Eg misting systems) to reduce outside temperature to below 38C. Service playground temperature will be conducted by a temperature device. Children will only be able to play near misting system areas where space is cooled. Experiences and activities will be restricted so children are in cooler and shaded areas and do not become too hot.
4. Educator will assess routine and type of outside activities being offered (in particular during the heat of the day: generally between 11 am and 3 pm) and also when temperature is above 35C. Eg. Activities conducted in periods of hot weather are to be undertaken in the shaded areas. Running games and bikes are restricted. Cool down-time activities are provided to allow children to cool down after play.
5. On days where temperatures exceed 28C children and educators will be encouraged to drink water at regular intervals.
6. Educators will discuss physical signs with children to support children's understanding of their own body's responses to heat, for example "Your cheeks look red", "You're hot, it's time to go inside and get a drink of water, slowdown and rest".
7. Humidity levels will influence the period of time children spend outside during times of high humidity.
8. Families may exercise their right to pick their children up early in periods of extreme weather conditions such as heat waves. This is not mandated and will purely be based on parent preference. The service operates regardless of the temperature.

Avoiding Heat-Related Stress

Young children and babies are more vulnerable to heat-related stress than adults because their bodies do not adapt as well to changes in temperature. Very young children are at particular risk as they may not be able to communicate their needs. The following are simple precautions parents and childcare workers can take to avoid heat-related stress in young children:

Parent and Guardians:

- Make sure your child has a full bottle of water at the service.
- Make sure your child is wearing appropriate clothing for hot weather (e.g. lightweight, light-coloured, loose-fitting clothing).
- Ensure your child has appropriate sun protective equipment (e.g. wide brimmed or bucket hats, UV-protection rated clothing, sunscreen). Remove potentially hazardous cords from hats for very young children.
- Parents may choose to keep children home or pick them up early on excessively hot days. Fees are charged normally.
- If you have any concerns, discuss your childcare centre's hot-weather policy with Responsible Person in Charge of Service.

Educators:

- Monitor children for signs of heat-related stress (tiredness, irritability, fewer wet nappies, nausea, headaches, vomiting, dizziness or faintness).
- If a child does present with signs of heat-related stress, give them water and allow them to rest in a cool, shaded area. If their condition does not improve, or they show any signs of worsening, apply first aid and seek medical help. The child could have heat-stroke, which is a very serious condition.

- Offer children a drink of water frequently – before, during and after play. Babies can be offered cooled boiled water in addition to normal feeds.
- Check water bottles and re-fill as needed.
- Assess routine and type of outside activities being offered based on the weather conditions.
- Check on babies regularly and monitor nappy changes (well hydrated babies should regularly wet their nappies). Babies can be offered cooled boiled water in addition to normal feeds.

Evaluation and Review

This policy will be reviewed at least annually. Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered. Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 16/10/2025

Next Policy Review: 15/10/2026

References:

NQS

QA 2.1.3 Healthy eating and physical activity are promoted and appropriate for each child.
 QA 3 3.1.1: Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.
 QA 3.1.2: Premises, furniture and equipment are safe, clean and well maintained
 QA 3.2.1: Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.
 QA 6.1.3: Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.
 QA 6.2.1: Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.

National Act and Regulations

Education and Care Services National Law Act 2010
 Education and Care Services National Regulations

Regulations

103 Premises, furniture and equipment to be safe, clean and in good repair
 105 Furniture, materials and equipment
 110 Ventilation and natural light
 113 Outdoor space—natural environment
 114 Outdoor space—shade

EYLF

LO4

- Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity
- Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating
- Children transfer and adapt what they have learned from one context to another
- Children resource their own learning through connecting with people, place, technologies and natural and processed materials

Relevant documents / links

- Heat Health (DET Website): <https://www2.education.vic.gov.au/pal/heat-health/policy>
- Hot weather and child safety: <https://www.betterhealth.vic.gov.au/health/healthyliving/hot-weather-and-child-safety>
- Belonging, Being and Becoming – The Early Years Learning Framework (July 2009)
- Victorian Early Years Learning and Development Framework (VEYLDF) (May 2016)
- DET Building Quality Standards Handbook (BQSH): Section 8.5.5 Shade Areas
- Get Up & Grow: Healthy eating and physical activity for early childhood (Section 2) 2009
- Victorian Institute of Teaching (VIT) The Victorian Teaching Profession Code of Conduct - Principle 3.2
- Australian Professional Standards for Teachers (APST) – Standard 4.4 and 7.2
- ARPANSA Radiation Protection Standard for Occupational Exposure to Ultraviolet Radiation (2006)
- Safe Work Australia: Guidance Note – Sun protection for outdoor workers (2016)
- AS/NZS 1067.1:2016, Eye and face protection - Sunglasses and fashion spectacles
- AS/NZS 4399:2017, Sun protective clothing - Evaluation and classification

- *AS/NZS 2604:2012 Sunscreen products - Evaluation and classification*
- *Australian Government Therapeutics Goods Administration (TGA) – Australian regulatory guidelines for sunscreens: 4. Labelling and advertising – directions for use of the product*
- *AS/NZS 4685.0:2017, Playground equipment and surfacing - Development, installation, inspection, maintenance and operation.6.2.1 General considerations, 6.3.9 Shade and sun protection, Appendix A Shade and sun protection*
- *Public Health and Wellbeing Act 2008 Occupational Health and Safety Act 2004*
- *SunSmart, Cancer Council Victoria: SunSmart*
- *Qld Cancer Council (2006) Early Childhood Settings: Sun Smart Policy Guidelines*
- *The Cancer Council NSW (2008) SunSmart Childcare: A guide for service providers*
www.cancercouncil.com.au/sunsmart

43 - EXCLUSION POLICY

Position Statement

There are some circumstances that require children and families to be excluded from the Centre. This is not a position taken lightly and careful consideration of all the factors involved is required.

Objective

To produce guidelines for team members to work through to determine whether excluding the child and/or family from the service is warranted.

Related Policies:

Non-Compliance Policy - 95
Family and Visitor Code of Conduct Policy – 8
Behaviour Guidance Policy – 9
Child Safety, Protection & Wellbeing Policy – 12
Child Safe Environment Policy – 88
Complaints and Grievance Policy – 7
Ethical Conduct and Code of Ethics Policy – 70
Fee Policy – 66
Health & Hygiene Policy – 27
Inclusion Policy - 4
Incident, Injury and Trauma Policy – 19
Infectious Diseases and Immunisation Policy - 32
Medical Conditions Policy – 87
Occupational Health & Safety Policy – 10
Parent Involvement Policy – 6
Parent/Team Member Communication Policy – 5
Team Member Grievances – 54
Team Member Self Protective Behaviour Policy – 56
Internet, Email and Social Networking Policy - 62

Implementation

A child/children and/or parents will be excluded from the centre under these conditions:

- The child has an infectious illness, injury, or illness. The child can return with a clearance from a doctor.
- The health departments have enforced regulations that improve the timeliness of vaccine coverage in children and prevent outbreaks of serious vaccine-preventable diseases. Services are prevented from enrolling the child or the child attending unless immunisation certificates proving that they are fully immunised as provided to the service, or have a specified medical exemption. Parents are required to provide this documented evidence that children are up to date with their vaccinations or they are on a recognised catch schedule, or they have a medical contraindication to vaccinations. With this information the child will be excluded from the service until documentation evidence is provided or immunisation is completed.
- By Law (Public Health and Wellbeing Act 2008), to finalise enrolment for your child you must provide the service with an immunisation status certificate that shows your child is:
 - Up to date with vaccinations for their age; or
 - On a vaccine catch-up schedule; or
 - Has a medical condition preventing them from being fully vaccinated.
- Unvaccinated children will be excluded from the service in an event of an outbreak of a vaccine preventable disease for their own protection.
- Unacceptable Challenging behaviour by the child and/or a parent which endangers or negatively impacts the health, well-being and safety of other children and/or team members and/or themselves. Please refer to Behavioural Guidance Policy for guidance and procedures.

- The centre reserves the right to limit the number of children attending the centre requiring additional support and resources. This is due to the financial hardship and stress placed upon the centre to provide the additional support and resources required to care for children with high support needs. We are also required to limit the number of children with high support needs at the centre at one time to ensure we are providing quality care to all children.
- Failure to pay outstanding fees after three weeks or outside our fee policy and fee schedule terms.

Please note childcare fees will be charged for exclusion periods.

Procedure

- Conduct open discussions with the parents as to the Centre's concerns (fee payment, behaviour, illness etc). Please refer to relevant policy for specific procedure.
- Refer parents to the Centre policy in relation to the area of concern.
- Please refer to the Non-compliance Policy for policy and procedure for managing non-compliance of policy and procedures.
- Devise an action plan with the parent if applicable (fee payment plan, behaviour management plan etc). The plan must identify the expectations that the Centre has for remedying the situation, and the consequences for non-compliance to the plan. A specific time line is to be stipulated eg. the fees are to be brought up to date by 10/11/XXXX. Failure to do so will mean that your child's enrolment will be cancelled as of 11/11/XXXX.
- Document or file note the plan and any progress made during the short term period. Please refer to the Child Individual Learning Plan Documentation Kit for guidance on documenting Behaviour Guidance Management Plan.
- Ensure that the Support Manager and Approved Provider has a copy of the plan or file notes and is aware of any progress or lack of progress being made.
- Notify parent of the exclusion or termination of the child's enrolment as of the termination date. If the child is being temporarily excluded the letter of exclusion must stipulate the terms under which the child will be allowed to return and if applicable on what date.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 16/1/2026

Reference:

NQS

QA6	6.2.1	Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.
QA4	4.1.2	Every effort is made for children to experience continuity of educators at the service

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

44 - CURRICULUM & PLANNING POLICY

Position Statement

Curriculum plan needs to promote the physical, social, emotional, language, and cognitive development of all children in a safe, nurturing environment that meets the individual needs of the children. The curriculum achieves the following outcomes:

- Outcome 1: Children have a strong sense of identity
- Outcome 2: Children are connected with and contribute to their world
- Outcome 3: Children have a strong sense of wellbeing
- Outcome 4: Children are confident and involved learners
- Outcome 5: Children are effective communicators

Objective

To encourage children to be actively involved in the learning process, to experience and participate in a variety of developmentally appropriate activities that are aligned to the National (EYLF) and Victorian Early Years Learning and Development Frameworks (EYLF).

Implementation

The curriculum plan is to provide:

- Take into account individuals, interests, age and development stages to provide appropriate curriculum for individuals and groups; to celebrate children's achievements and achieved learning outcomes
- Consideration of the needs and interests of individuals and groups through Child, Children, Family, and Educators perspectives
- A variety of developmentally appropriate materials, resources and equipment
- For gender equity – an inclusive, unbiased, non-stereotypical approach
- A multicultural perspective which embraces and respects all cultures, including an understanding of Aboriginal and Torres Strait Islander culture within the program
- A balance of daily activities, indoor/outdoor, quiet/active, small and large groups, individual and child/team member initiated activity
- Large blocks of time for the completion of projects, particularly in over three age groups
- Flexible transitions and routines to accommodate changing needs and interests of children and spontaneous occurrences
- Flexible transitions and routines that provide learning experiences
- Activities and learning experiences aimed at fostering and challenging children's development and skills across all developmental and curriculum areas
- Our curriculum will showcase the children's learning on a daily basis through daily reflections/news, learning stories and emails to families.

The curriculum planning is to:

- Be prepared in line with the National Early Years Learning Frameworks (EYLF) and Victorian Early Years Learning and Development Framework (VEYLF).
- Take into consideration the use of Child, Children, Family, Educators perspectives, Planned and Intentional Teaching Experiences, and Spontaneous Teaching experiences to provide a learning environment for children to achieve their learning outcomes. The curriculum is to be child-centred, however, the inclusion of Intentional Teaching experiences will be incorporated into the plan to ensure children's core skills and knowledge is developed. Please refer to the service Curriculum Summary Guide.
- Promotion sustainability and community learning outcomes
- Encourage the extension of children's learning by building on experiences to enrich the children's knowledge and skills
- Be based on observations of individuals and groups plus long-term group learning outcomes

- Use a variety of observation types
- Foster a positive self concept
- Develop social skills
- Encourage children to think, reason, question and experiment
- Encourage language development; both expressive and receptive
- Enhance gross motor, fine motor and perceptual motor skills
- Encourage sound health, safety and nutritional practices
- Encourage creative expression
- Embrace cultural diversity of team members and children and use this diversity to enhance the program
- Acknowledge Australian Culture; including Aboriginal and Torres Strait Island culture and its place as an integral element of Australian culture.
- Develop self help skills
- Show linkages back to planned goals and outcomes as well as individual child planning objectives
- Respect the child's right to choose his/her own activity or to choose not to participate as they wish
- Be evaluated and the performance of daily, weekly, monthly and yearly reflections. Parents may participate in the evaluation process
- Be sensitive and responsive to individual children's needs
- Be displayed for parents to view and pass comment on
- Show team members as positive role models
- Be available for parents to participate in and add to as necessary

Children with Additional Needs:

- Team members are to be familiar with all community resources in order to use them as a referral for parents.
- Team members are to liaise with other organisations involved in the caring and programming for the child.
- Minor modifications may need to be made to the environment to accommodate special needs children.
- Curriculum planning is based on children's individual strengths, needs and interests and as such the inclusion of special needs children fits well with the ethos of early childhood program planning.
- Children with Additional Needs should have an Individual Learning Plan.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

NQS

QA1	1.1.1	<i>Curriculum decision making contributes to each child's learning and development outcomes in relation to their identity, connection with community, wellbeing, confidence as learners and effectiveness as communicators.</i>
	1.1.2	<i>Each child's current knowledge, strengths, ideas, culture, abilities and interests are the foundation of the program.</i>

	1.1.3	All aspects of the program, including routines, are organised in ways that maximise opportunities for each child's learning.
	1.3.3	Families are informed about the program and their child's progress.
	1.1.5	Every child is supported to participate in the program.
	1.2.3	Each child's agency is promoted, enabling them to make choices and decisions that influence events and their world.
	1.3.1	Each child's learning and development is assessed or evaluated as part of an ongoing cycle of observation, analysing learning, documentation, planning, implementation and reflection.
	1.2.2	Educators respond to children's ideas and play and extend children's learning through open-ended questions, interactions and feedback
	1.3.2	Critical reflection on children's learning and development, both as individuals and in groups, drives program planning and implementation.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	73	Educational programs
	74	Information about the educational program to be kept available
	75	Information about educational program to be given to parents
	76	Documenting of child assessments or evaluations for delivery of educational program

EYLF

LO1 – LO5	All Learning Outcomes under the Early Years Learning Framework will be addressed through our Policy and practices.
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Other References:

National Early Years Learning Framework

Victorian Early Years Learning Framework

Centre Support EYLF Guide

Handbook on Child Care Licensing

QIAS Principle 3.1, 3.2, 3.3

CCB – Federal Funding Guidelines

Occupational Health and Safety Act 2004

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Sources and Further Reading:

- Greenman, J. (1998) Caring Space, Learning Places: Children's Environments that Work, Exchange Press, Redmond, WA.
- Pitamic, M. (2004) Teach me to do it myself Hardie Grant Books, South Yarra, Vic.

45 - TRANSITIONS AND ROUTINES POLICY

Position Statement

All children need predictability in their day, while the program needs to have the flexibility to accommodate each individual's needs.

Objective

To ensure we are able to balance children's needs and provide genuine learning experiences through transitions and routines.

Implementation

- The daily routine will be displayed for parents and visitors and discussed in relation to each child upon enrolment.
- Daily routines will be displayed pictorially for children so that the routine has meaning for the children involved. This strategy links with the behaviour management model in use in the Centre and is an important aspect of the day's routine. (Nurse's room is not required to have a pictorially daily routine as the display will not be able to be interpreted by children of this age. A written routine will be displayed for educator and parents to view and follow.)
- The routine will be flexible to allow for individual needs yet give security and predictability to children as they learn to know what will occur next in the day.
- Within the daily routine there are many subsets of routines. In line with the behaviour management model, routines will have with them expectations of behaviour and skills. These aspects of the routine are to be actively taught to children to ensure that there is a shared understanding between children and team members as to the order of the day as it unfolds, as such routines should be treated as learning experiences to foster children's self help and independence.
- Transitions will be planned on a daily basis, and children should be moved in small groups. Transitions will form an integral aspect of the program and link to goals and objectives for children.
- Educators will complete head counts via OWNA (or paper rolls when OWNA is not available) during key transition times.
- Educators will verbally communicate in small groups giving children warning when moving through routine or experiences.
- Each room will have a written and visual routine. Educators will also have a collaborative written routine to guide smooth routines and transitions.
- The pictorial daily routine aims to assist children with building an understanding of time progression for the day and a sense of being able to predict what is coming next in their day.
- Waiting time will be kept to a minimum and should be filled with simple finger plays, stories, songs, etc.
- Written and verbal communication needs to be made with parents regarding child transition to other rooms; including transition date, planned orientation and any other information regarding transition.
- Routines will be critical reflected each term; gathering educator, family and children perspectives and also how to maximise the learning opportunities and also wellbeing of all children.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 19/11/2025

Reference:

NQS

QA6	6.2.1	Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.
QA4	4.1.2	Every effort is made for children to experience continuity of educators at the service..

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Handbook on Child Care Licensing
QIAS Principle 1.2, 1.3, 3.3, 4.1
Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2004

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

46 - FAMILY GROUPING POLICY

Position Statement

Children are to have the opportunity to spend time with their siblings during their day at childcare. Family grouping times during the day offer children the opportunity to mix with siblings.

Objective

Family grouping brings with it special considerations as to supervision, appropriateness of activities, and duty of care and regulatory requirements and therefore it is important to clearly outline expectations.

Procedure

- During family grouping times the team members at the centre are to observe all regulatory requirements.
- Team members will manage the environment so that team member – to – child ratios are observed.
- Head Count Procedure at family grouping times: a head count needs to occur as family grouping separates in the morning and then again as rooms join together in the afternoon.
- Whilst family grouping times are generally held during times of the day that other routine duties are conducted such as set up in the morning and cleaning in the afternoon, these duties are not to impede core responsibilities of supervision and interactions with children.
- A suitable curriculum needs to be planned to the family grouping times. The rooms will be set with activities that are engaging and age appropriate for all the children. All small items will be removed prior to the commencement of family grouping.
- Behaviour guidance practices are to continue as with all other times of the day.
- The room must be suitable to care for young infants with access to nappy changing facilities and a phone as well as providing for older children's requirements.
- Food and drinks are to be available to children as required.
- Team member – to – child ratios and interactions with the children need to be maintained throughout this time.
- The licensed room capacity must be adhered to as with all other times of the day.
- Where practicable, team members will move the children's bags from their allocated room to the family grouping room when the children commence family grouping times.
- OWNA Kiosk will be located in the foyer to ensure all parents are signing as required and to assist in an emergency evacuation should that become necessary. In addition, child wellbeing schedules, incident & medication forms and parent correspondence is to be available during this time.
- Family grouping rooms will have up to date allergy listings for all children.
- Team members rostered on at these times need to recognise the authorised person collecting the child on sight, or take the appropriate actions to identify them, to ensure that all children are collected by the legal guardians.
- Relief team members are not to be placed in charge during family grouping times. The roster is to be changed to facilitate for regular team members to be on hand for opening and closing times of the day.
- The nominated "in charge" team member is to make themselves aware of any incident reports or medication requirements for the children so as to be in an informed position to relay information to parents in the afternoon, and to rostered room team members in the morning.
- A Certified Supervisor must be present and on duty for opening and closing shifts and all operating hours of the service.

- Children can only be released into the care of persons over 18 years unless that person is the child's parent. ***Team members who fail to comply with this aspect of duty of care may be subject to instant dismissal.***
- Authorised persons collecting children who are not known to team members will need to provide photo identification before a child is released. The Driver's License No. of this person should be recorded in OWNA. Team members who are unsure as to the validity of the authorised person picking up a child are expected to make a phone call to the guardian or parent to confirm the person's identity to be authorised to pick up a child.
- A senior team member must be nominated as 'In charge – Responsible Person' and noted on the roster and display notice and team members are to follow their directives. The nominated "In charge – Responsible Person" team member is expected to monitor all team members interactions and other duties during family groupings to ensure that all expectations and regulatory requirements are met at all times.
- Should there be any incident or issue that arises during family grouping times, the team member deemed to be "In charge – Responsible Person" is to contact the Centre Director and/or the Approved Provider before leaving the centre.
- The nominated person in charge during family groupings is to monitor the group sizing and age break downs of the children on premises. The decision to combine children or to keep them separated must take into account the team members – to – child ratio requirements, rostering, and the suitability of combining children on the day.
- At no time during family grouping time is child supervision or duty of care to be compromised.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/12/2025

Reference:

NQS

QA3	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>
QA6	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	156	<i>Relationships in groups</i>
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EYLF

LO2	
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	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
	<i>Children respond to diversity with respect</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

47 - INDIGENOUS & MULTICULTURAL POLICY

Position Statement

Australia is a multicultural society. Children and adults need to be conscious of the variety and diversity of ethnic backgrounds and learn to understand, accept and respect the richness of many cultures including Aboriginal and Torres Strait Islander culture.

Objective

To recognise the diverse cultures that make up Australia and provide opportunities for all children and adults to learn about other cultures including Aboriginal and Torres Strait Island cultures and to give and receive respect.

Implementation

- Service will implement a Reconciliation Action Plan (RAP). <https://www.reconciliation.org.au/reconciliation-action-plans/>
- Cultural awareness is to be an integral element of the centre's program and philosophy.
- Multicultural resources will be provided. Books, music, instruments, crafts, clothing, food, posters and other resources are required.
- All parents will be encouraged to contribute information about their own culture to the program.
- Parent information will be translated into other languages and available for parents when necessary.
- Multicultural workers and/or interpreters will be used, where there is a need, to communicate with parents and to help settle new children.
- Where possible, team members will broadly reflect Australian society.
- All team members will be encouraged to attend in service training on multicultural programming and inclusion.
- All children and adults are to be treated respectfully, without bias in regard to cultural background.
- Adult interactions that do not reflect respect for others' culture will not be tolerated.
- Adults engaged in racial or cultural vilification will be dealt with as per the Family and Visitor Code of Conduct Policy. Team members found to be engaged in racial and/or cultural vilification will be disciplined under industrial protocols.

Staff

- Staff – the most critical element in cross cultural education.
 - The staff will be involved at Centre meetings and will attend in-services as his or her attitudes will impact on the service, policy and implementation.
- Staff will:
- become aware of their own beliefs, attitudes, cultural backgrounds, their relationship with the larger society and their attitudes to people;
 - acknowledge that they too have been influenced by their own background prejudices and their points of view;
 - accept that all children can learn and that differences in lifestyles and languages does not mean ignorance;
 - broaden their own cultural and ethnic group awareness and help children to understand themselves in relationship to their family, community and other cultures;
 - be actively involved in the development of appropriate resources, support and implement an anti bias, cross cultural program throughout the Centre environment which is reflective of all families/children and the diversity present in Australian society and network with community agencies involved with cross cultural issues wherever possible;

- be actively involved with children, showing respect, sharing ideas, experiences and ask questions.

Curriculum – Children

- The Centre will provide a cross-cultural perspective in all areas of the program.
- The children will:
 - listen to music and practice singing songs in different languages;
 - learn words and phrases in a language not native to children in their group;
 - talk to other children using the words from their culture;
 - be encouraged to become independent wherever possible and be actively involved with their peers.
- Children's self concepts grow when they feel an important part of the Centre and therefore:
 - children will explore with foods from other cultures (eg. have different home cultures come in and cook, to have "food tasting" parties);
 - we will have children bring in real objects and artefacts used by their families that may be historical or typical of that child's/family's cultural group including food;
 - collect ways the families and children recognise special days, events, holidays or rituals and have parents decorate the entry with displays of cultural days, their heritage etc;
 - through discussion and displays, food, books, interactions with families and the individual children, families and staff will bring with them specific knowledge, skills, attitudes, values and language specific to that cultural group;
- be provided with opportunities for families to be involved in the curriculum process (not only to the above) but also:
 - sharing of cultures with others
 - sharing expectations of the service
 - provision of feedback/evaluation as to how the service implements and meets the needs of all concerned ie. children, parents, staff and the Centre.

Additional information can be sourced in the Inclusion policy.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 17/12/2024

Reference:

NQS

QA5	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.</i>
	<i>Children develop knowledgeable and confident self identities.</i>
	<i>Children learn to interact in relation to others with care, empathy and respect.</i>
LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.</i>
	<i>Children respond to diversity with respect.</i>
	<i>Children become aware of fairness.</i>
	<i>Children become socially responsible and show respect for the environment.</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 1.4, 1.5, 3.3

CCB – Federal Funding Guidelines

Workplace Health and Safety Regulation 1997

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Racial and Religious Tolerance Act 2001

48 - EXCURSION & INCURSION POLICY

Position Statement

Excursions can be an important adjunct to learning in an early childhood setting. When organising an excursion, parent/friend involvement can be essential to supplement child/adult ratios. For this reason, clear guidelines need to be set in place so that uniformity of procedures and actions applies to all adults and children involved in the excursion or incursion.

Objective

To provide a clear set of guidelines for team members and adults to follow during the planning and implementation of an excursion or an incursion.

Related Policies

Acceptance and Refusal of Authorisations Policy
Physical Environment Policy
Staffing Arrangements Policy
Centre Vehicle & Transport Policy

Implementation

Our educators recognise that excursions and incursions offer are a fun way of connecting children with their community and provide endless opportunities to extend children's learning. Information gained during excursions and incursions can be used to plan ongoing activities and experiences that may last days or weeks as learning about one thing leads to new and exciting discoveries about related or different topics.

Excursions and incursions also allow educators to demonstrate how their practice is shaped by meaningful engagement with the community which is one of the themes which must be demonstrated to achieve an Exceeding NQS Rating.

Risk Assessments

Safety during excursions and incursions is a priority. The Nominated Supervisor or educators will always complete a risk assessment to identify, assess and remove or reduce risks of activities may pose to the safety, health and wellbeing of and each child before children are transported unless the arrangement is a 'regular outing (ie a walk, drive or trip to and from a destination that the service visits regularly as part of its educational program and where the circumstances and risks are substantially same on each outing) and a risk assessment has been completed within the last 12 months. The risk assessment will cover:

- Any risk that the excursion may pose to the safety, health and wellbeing of any child and identify how these risks will be managed and minimised
- Proposed route and destination
- Any water hazards and associated risks
- Means of transport and child restraint/seat belt requirements
- The process for entering and exiting the service premises or destination, and procedures for embarking and disembarking transport, including how each child will be accounted for
- Number of educators and children (and ratio)

- Whether extra adults are required for supervision/safety -educator to child ratios are minimum requirements. You may discuss supervision strategies at a staff meeting eg sourcing high viz vests and ropes which children can hold on to
- Any special skills required
- Proposed activities
- Proposed duration
- Any specific health care needs or medical conditions that need to be managed
- Items that should be taken

The Nominated Supervisor will update risk assessments for regular outings and obtain new authorisations from parents/guardians when circumstances that may affect the arrangements change, including for example:

- weather conditions (summer versus winter, extreme weather events like heatwaves, floods and bushfires)
- changes in routes for example because of road works
- the numbers and vulnerabilities of children.

EXCURSIONS

See Appendix 1: Coordination and Completion of Excursion & Risk Assessment forms. This appendix as the steps needed to be completed for organising, communicating and documenting an excursion.

Authorisations for Excursions

Authorisation for a child to be taken on an excursion must be given by a parent or other person named in the child's enrolment record as having authority to authorise the excursion unless the arrangement is a 'regular outing and there's an authorisation which is less than 12 months old. The authorisation will include:

- Child's name
- If it's a regular outing, a description of when the child is to be taken on the regular outings
- If it's not a regular outing, the date of the excursion
- Destination and proposed activities
- if transport involved, the means of transport, and any requirements for seatbelts or safety restraints under the relevant state/territory law
- How long the child will be away from the centre
- Expected number of children attending
- Expected ratio of educators to children
- Expected number of additional adults who will be attending
- Items child required to bring from home for excursion
- Advice risk assessment available at service.

Excursions: Other practices and procedures:

- See Appendix 2: Internal Audit - Excursion and Regular Outings tool to assess the requirements needed to be completed for an excursion.
- Head counts while on excursions can be completed using a paper roll printed from OWNA to identify attending children and perform headcounts. The lead educator can use a pen to complete headcount of children and staff members. The completed form can be uploaded to OWNA future reference. (This supports the no use of personal devices while on excursions)
- Person devices (such as a mobile phone) are not to be used while on excursions, except in emergencies circumstances.

- Authorisations for the possession and potential use of personal devices for 'essential purposes' will be documented in the excursion risk assessment and management plan as follows. "Name of person authorised to possess and use personal device in emergency situation while on excursion: <Insert Name>."
- Advance written authorisation needs to be sought for use of Personal Device for an 'essential purpose' from Approved Provider or Nominated Supervisor. This will be given only in accordance with the National Model Code guidelines and the service policies.
- The service must ensure a risk assessment is carried out in accordance with regulation 101 before authorisation is sought under regulation 102 for an excursion.
- The approved provider and nominated supervisor must review the risk assessment carried out in accordance with regulation 101 before authorisation is sought for the excursion.
- A risk assessment is not required under this regulation if (a) the excursion is a regular outing and (b) a risk assessment has been conducted for the excursion.
- A risk assessment for an excursion must (101 (1)(a) identify and assess risks that the excursion may pose to the safety, health or wellbeing of any child being taken on the excursion and (b) specify how the identified risks will be managed and minimised.
- Without limiting sub regulation (1) a risk assessment must consider
 - The proposed route and destination for the excursion; and
 - Any water hazards; and
 - Any associated risks with water-based activities; and
 - the transport to and from the proposed destination for the excursion; and
 - the number of adults and children involved in the excursion; and
 - given the risks posed by the excursion, the number of educators or the other responsible adults that is appropriate to provide supervision and whether any adults with specialised skills are required; and
 - the proposed activities; and
 - the proposed duration of the excursion; and
 - the item that should be taken on the excursion.
- All plans and risk assessments for excursions need to be authorised by the Centre Director & Support Manager. Consideration needs to be given to group management, location of excursion, method of transport, staff member holding current first aid qualifications, criteria for selection of volunteers, management of nappy change and toileting whilst away from the centre, etc.
- Planning for excursions needs to include all costs (transport, admission, refreshments, etc).
- Families will be given sufficient detail about the excursion and reasonable prior notice, in order to consider whether their child will participate. This information is to include; the date of the excursion, the proposed destination, method of transport or walking itinerary, activities to be carried out during the excursion, the number of adults to accompany and supervise the children, the name of the person with first aid qualifications who will accompany the children on the excursion, the proposed time during which the excursion is to take place, and if practicable an emergency contact number for the excursion.
- All permission forms will be kept together and handed to the Director before the excursion. Permission form may be signed using an electronic signature software.
- All excursions will be undertaken subject to having sufficient adult help to adhere to regulatory requirements regarding adult/child ratios. Should insufficient adults be available the excursion will be cancelled.
- All excursions are to meet state requirements for child/adult ratios. No excursion is to leave the Centre if team member – to – child ratios are compromised.
- All families need to authorise their child's participation in the excursion in writing (per the Acceptance and Refusal Authorisation Policy).
- Comprehensive rolls will be compiled of the lists of adults and the children that will be assigned to each adult during the excursion.
- Each adult will have their own roll, and the children will be marked off against this roll at specified times during the excursion.

- The Educational Leader in charge of the excursion will carry a mobile phone with them during the time they are away from the centre.
- The Educational Leader in charge of the excursion will carry a First Aid kit at all times when away from the centre.
- During the excursion the group will stay together. Educational Leaders are to take the overall responsibility for ensuring that no one is left behind.
- A total head count is to be carried out by the Educational Leader in charge of the excursion at departure times and specified times throughout the day.
- Thank you notes will be written out for all parent helpers and given to them the following day.
- The location of an excursion should be considered carefully. It is not appropriate for an excursion to be held in a busy area (ie theme park) which makes it difficult to manage and monitor the group of children.
- Excursions should not be held in high-risk areas particularly not near water. Additional assistance and further clarification on suitable locations can be sought from the Approved Provider / Approved Provider.

Transport

The refer to the Centre Vehicle & Transport Policy for policy and procedure associated with use of transportation while excursions, including what to include in risk assessment.

The means of transport must be stated on the permission note.

Buses – ensure that the seating capacity as displayed on the compliance plate is not exceeded. All children must sit on seats, preferably with, or close to, an adult. Seat belt guidelines must be followed depending on the bus. If the bus has seat belts, they must be worn at all times.

Trains – contact the station prior to the excursion to inform them of the time you will be travelling, the destination and the number of children and adults who will be travelling. Arrangements should be made to arrive at the station an adequate amount of time to allow for safe boarding. This will allow the station to inform the train guard so that he / she can hold the train for the period of time for safe boarding and alighting. All children should be seated at all times, with an adult close by. All children should be seated in the one carriage, if possible.

Cars – Any motor vehicle that is used to transport children on excursion (other than a motor vehicle with seating more than nine persons) is fitted child restraints and/or seatbelts that are appropriate for the age and weight of each child, that conform to the Australian Standards, and are professionally installed or checked by an authorised restraint fitter. Permission must be maintained from Approved Provider before this mode of transport is selected.

Insurance

Any excursion planned must be consistent with the requirements / exclusions of the Public Liability Cover held by the service.

INCURSIONS & EVENTS HELD AT SERVICE

Incursions are events, activities or experiences that are held at the service on enrolled day of the child. The documentation completed is different to an excursion.

Depending on the type and nature of the incursion will determine the forms to be completed:

Type of Incursion or Event	Documents completed
TYPE 1: Incursion requiring permission to participate from parent or guardian and need authorisation to charge costs to Parent Account in OWNA. For example: animal farm, etc) See Appendix 3: TYPE 1: Incursion requiring permission from parent / guardian to participate	1. OWNA Excursion / Incursion Form and Parent Consent Form. This includes information about the incursion, costs, authorisation to charge family account, consent form for parent to sign using the OWNA App. (See OWNA Excursion / Incursion Tool) 2. OWNA Risk Assessment (with OWNA Excursion / Incursion Tool) See Appendix 3 for instructions for completion of OWNA Incursion Forms and Risk Assessment.
TYPE 2: Incursion that forms part of room curriculum and educational program. This could be a regular incursion such as a sport program. Alternatively, the incursion could be an extra curricula incursion that is provided to compliment the education program. (no extra charge for incursion, apart of normal fees) See Appendix 4: Type 2 Incursion Risk Assessment and Management.	1. Complete Incursion Risk Assessment and Management Form – Delivered by Incursion Provider. (MS Word Template) 2. Prepare and send an email and post to parents and guardian to advise of details of incursion: - reason for incursion - The incursion will be held at the service as part of the curriculum on the child's booked/enrolled day. Educators will be actively supervising children when participating in the incursion. - who is performing the incursion. Name of person and organisation. - who is invited (what children, rooms, etc) - nature of the event and what activities / experiences will occur - advise a risk assessment has been performed and is available for viewing. - Inform parents and guardians that child safety review has been performed as part of risk assessment. The incursion provider holds a Working With Children Check (WWCC). Online verification of WWCC completed by Centre Director / Responsible Person before incursion occurs. WWCC information recorded on the incursion form. 3. Copy of public liability insurance obtained from Incursion Provide and information about policy recorded on incursion form.
TYPE 3: Event - Invitation for special person event held at service premises. Eg Father's Day, Mother's Day, Grandparent'ss Day, etc. (No incursion provider) See Appendix 5: Type 3 – Event Risk Assessment and Management	1. Completion of Event Risk Assessment and Management (see template) 2. Prepare and send an Email and Post to parents and guardian to advise of details of event: -reason for event

	- who is invited (what children, rooms, etc) - nature of the event and what activities / experiences will occur - reminder about signing children in and out of OWNA Kiosk. Also ensuring person picking up child at end of day has authorisation on enrolment form. - due date for RSVP - restrictions during event. Reminder for food, personal mobile phone and photos. 3. Service team to maintain a Visitors Register to record people attending the event. This can be recording using a paper schedule and complete by each person on entry to the service (Can have multiple sheets for ease of signing on entry by multiple people at one time). 4. Use of OWNA Event invitation tool to notify parents and guardians of events and use of RSVP tool to collate who is attending event.
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APPENDIX 1: COORDINATION AND COMPLETION OF EXCURSION & RISK ASSESSMENT FORMS

The following needs to be prepared for each excursion:

Task 1: Excursion Checklist: This is completed via the OWNA App. See Checklists > Excursion Checklist

Task 2: Excursion Form and Parent Consent Form

Task 3: Risk Assessment

Overview:

- See OWNA tutorial for use of OWNA tools in portal:
<https://ownaportal.tawk.help/article/excursion-forms>
- Commence first with the Excursion Checklist (this includes instructions to complete the excursion forms)
- For a new excursion copy from the excursion form template. Custom all information for the excursion.
- The excursion coordinators need to follow the instructions in the Excursion Checklist. Have the Centre Director review and sign off they have reviewed checklist, Excursion Form and Excursion Risk Assessment. The Centre Director needs to go into the Excursion Checklist and sign-off and date they have reviewed all forms and are happy for the excursion to occur.
- Final draft signoff by Approved Provider. Email Approved Provider to inform him that checklist (advise date created), parent consent form and risk assessment are ready for review.
- Approved Provider will advise of any changes to forms. And provide an email to advise the forms have been reviewed and approved.
- Once final approval has been received the Excursion forms can be issued to parents for signing via the OWNA app.

STEP 1: EXCURSION CHECKLIST

Access via OWNA App: Checklists > Excursion Checklist

Before commencing the Excursion Checklist, please discuss the following with the Nominated Supervisor / Centre Director:

- excursion idea and plan. The benefits and outcomes of the excursion
- the location and site for excursion. Initial risk assessment. Are the risks acceptable for the service?
- Do the benefits and outcomes outweigh risk. Can the risk be mitigated to an acceptable and manageable level?
- Budget: costs to service and costs to parents?
- potential date of excursion. Is there sufficient time to plan and coordinate excursion. i.e. children, team and parents' permission forms.
- team discussions regarding excursion. Are they supporting the excursion.

Purpose: the Excursion Checklist provides the Excursion Coordinator a list of tasks to complete before, during and after the excursion. The Excursion Checklist is created on a specific date. This date needs to be remembered as you need to return to this date to continue with the Excursion Checklist tasks.

(If there are two excursions being planned: the checklist can be commenced on two different dates so the checklist can be commenced by the excursion coordinator. This is due to only one excursion checklist being allowed to be created per day in using the OWNA app.)

Excursion Checklist - MELC - 2024-04-22

NAME OF EXCURSION

Brief Description and Purpose of Excursion

Rooms Name

Planned Date & Time (Start & End Time) of Excursion

Name of Excursion Coordinator

EXCURSION PLANNING

Checklist for Excursion Coordinator

Discussed excursion idea, budget and plan for outing with Nominated Supervisor / Responsible Person in Charge of Service (Insert Date)

After discussed excursion idea with Nominated Supervisor / Centre Director enter OWNA portal and create Excursion Form and Risk Assessment from template. (Enter OWNA Portal > Children > Manage Excursions/Transport Risk Mgt Plan > Press '+' > Select excursion template > Complete form sections and fields.

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Sign Off

STEP 2: EXCURSION FORM & PARENT CONSENT

Access via OWNA Portal: Children > Excursion/Incursion

Manage Excursions

Home / Children / Excursion/Incursion

Current Excursions						
Get Excursions						
#	Excursion Date	Excursion Title/Name	# Children Attending	Forms Signed By	Risk Assessment	Risk Assessment Reviewed
1	Thu - Sep 11, 2025	Excursion to Mildura Airport	4	24 - View Signed Permissions	View	11 Aug. 2025

Template for Excursion Form: this is access via the following section of the OWNA Portal. See GSELC Excursion Template version 29 07 25.

Manage Excursions

Home / Children / **Excursion/Incursion**

Template Excursions

Get Excursions

Creating a new Excursion form to use:

After discussed excursion idea with Nominated Supervisor / Centre Director enter OWNA portal and create Excursion Form and Risk Assessment from template. (Enter OWNA Portal > Children > Manage Excursions/Transport Risk Mgt Plan > Press '+' > Select excursion template > Complete form sections and fields.

Select > GSELC Excursion Template

Add Excursion

Home / Manage Excursions / Add Excursion

Excursion Details

Use an Existing Template

Excursion Title/Name*

Cost (if any)

1. Create the Excursion Form below.
2. A Risk Assessment form will also be generated for you to fill in (access via Manage Excursions page).
3. The Excursion Form is available for Parents on the App to sign. You can also print it out if needed.

Backup Copy GSELC Excursion Template_v07.25 (do not change)

GSELC Excursion Template_v29 07.25

Parents will also be able to pay via the App later on (if you allow it)

The following is copy of the GSELC Excursion Form template to parents and guardians. The form is needed to be customised for each excursion by the excursion coordinator and reviewed by the Centre Director. This is the letter that the parent receives and advises them of the excursion information.

Dear Families,

EXCURSION CONSENT & AUTHORISATION

The <insert name of room> are planning an excursion on the date/s noted below to <name of location of excursion>. We have summarised the excursion plan as follows and provide an excursion risk management plan (the excursion risk assessment is also available at the service for your viewing). The excursion plan should be read in conjunction with the the service policies and risk assessments. The risk assessment is available for viewing at the service.

The signing of the excursion form gives authorisation for your child to attend the excursion or regular outing on the dates detailed in this form.

If you have any questions regarding the excursion feel free to discuss with the Room Educators or Centre Director.

Kind regards,

<Insert Name of Excursion Coordinator>

<Insert Room Name>

EXCURSION INFORMATION:

EXCURSION LOCATION INFORMATION	
Name of excursion site or area:	<Brief name Eg. Kindergarten Room: Bus to the Zoo Excursion>
Address of excursion site:	<Insert address>
Name of Contact Person from excursion site (or N/a):	<insert name> or N/a

Details for excursion confirmed in writing from organisation:	Yes or N/a
Date confirmed received:	<insert date or if N/a>
Telephone number for excursion site:	<insert telephone number>
Reason for N/a above :	

Date/s of Excursion	Time of Excursion
<Insert Date Eg. Friday 20 May 2022>	<Insert time: Eg From: 9:45am To: 10:15am>
<Insert Date Eg. Friday 27 May 2022>	<Insert time: Eg From: 9:45am To: 10:15am>

Cost for Excursion per child (this cost will be added to family account. Signing of excursion form give authorisation for charging onto Service invoice / statement.)	<Insert \$>
Name of classroom/s attending excursion:	<Insert Room Name>
Age group of children attending excursion:	<X years to X years>
Maximum Number of children attending excursion:	<Insert Number>
Name of excursion coordinator:	<Insert Name>
Pick up location and destination(s): List each location travelled to and from as part of the excursion. E.g. the museum address, park address for lunch and service.	Pickup Address: <Insert address> Destination Address: <Insert address>
Method of Transport to be used during excursion: E.g. public bus, private bus, coach, tram, walking, etc	<Insert Method>

Proposed activities and reason for excursion (reason why child is to be taken outside the premises): When planning an excursion educators must consider how the excursion

supports the educational program and contributes to the outcomes outlined for each child (regulation 73(2)).

Transport will be by (company name):	<Name of transport company or N/a if walking>
Requirements for seatbelts or safety restraints in your state or territory have been met.	Yes / No (Requirement for bus service) Comment:
Reason for excursion: • <List here> • <List here> List all activities that will take place during the excursion: • <List here> • <List here>	
Learning Outcomes: <List LOs reference here. Eg 1.1, 2.1, 3.1>	

OTHER TERMS AND CONDITIONS OF EXCURSION – ACKNOWLEDGEMENTS BY PARENT

Child Behaviour

'I understand that in the event of my child's misbehaviour or behaviour that poses a danger to himself/herself or others before or during the excursion, he/she may be required to stay at the service, be sent home or required to be collected from the excursion location by the parent. I further understand that in such circumstances I will be informed and that any costs associated with his/her return will be my responsibility.'

Child illness

'I understand that in the event excursion educators determine it is necessary for my child to be sent home early due to illness or collected by parent from the excursion location, any cost associated with his/her return will be my responsibility.'

Cancellations or Alterations

'I understand that the Service may need to cancel or alter excursion arrangements at short notice, due to circumstances beyond the control of the Service, and while the Service will try to minimise inconvenience or financial losses to parents, these may be unavoidable.'

EDUCATOR/STAFFING INFORMATION

Number of Educators Attending: 3

Ratio Calculation: Number of Educators / Number of Children Attending Excursion: <Insert Number of Educators / Insert Number of Children = Example of Ratio (1:4)>

Educator Name	Qualification
<Insert Name>	<ECT, Diploma, Certificate 4, Working Toward Diploma, Working Toward Certificate 3>
<Insert Name>	<ECT, Diploma, Certificate 4, Working Toward Diploma, Working Toward Certificate 3>
<Insert Name>	<ECT, Diploma, Certificate 4, Working Toward Diploma, Working Toward Certificate 3>

Parent and guardian helpers attending:
Parent & Guardian Names:
<Insert Name>

EXCURSION RISK MANAGEMENT ANALYSIS	
Overall Excursion Risk Assessment is completed and there is an acceptable level of risk and sufficient strategies are documented (see risk management assessment attached):	
YES / NO (Needs to be "Yes" for excursion to be held)	
Excursion coordinator must visit the site to conduct a risk assessment of the area to identify potential risks and/or hazards and discuss how you will manage these prior to receiving approval for excursion. The excursion coordinator must also conduct another risk assessment upon arriving at the site on the day of the excursion to ensure that the area is still suitable.	
Use the risk assessment table to identify and assess risks to the safety, health or wellbeing of children and adults attending the excursion, and specify how these risks will be managed and minimised [regulation 101(1)]. This must include any risks associated with water-based activities. We have identified possible hazards and issues while attending the excursion and have rated the risk assessment using the matrix. If hazards and issues are rated as having a high level of risk due to the consequences, but the likelihood to occur is low. Educators have included strategies and information on how we will manage to minimise the risks to an acceptable level (Medium or Low) for a successful excursion to occur. Our risk assessment considers: - proposed route and destination for the excursion - any water hazards - any risks associated with water based activities - method of transport to and from the proposed destination	

- *number of adults and children involved in the excursion*
- *given the risks posed, the number of educators or other responsible adults that is appropriate to provide supervision and whether any adults with specialised skills are required to ensure children's safety*
- *proposed activities*
- *proposed duration of the excursion*
- *items that should be taken on the excursion.*
- *strategies for accounting for all children on the excursion, particularly during transition times, such as ensuring all children have been transported to the destination and have been returned to the service at the conclusion of the excursion.*

Note: A risk assessment is only required to be completed at least once for a 12 month period if the excursion is a 'regular outing'. v07 25*

STEP 3: RISK ASSESSMENT FORM

Access via OWNA Portal: Children > Excursion/Incursion

As mentioned above in Step 2: After discussed excursion idea with Nominated Supervisor / Centre Director enter OWNA portal and create Excursion Form and Risk Assessment from template. (Enter OWNA Portal > Children > Manage Excursions/Transport Risk Mgt Plan > Press '+' > Select excursion template > Complete form sections and fields.

Tasks:

- All fields and questions in the risk assessment form must be completed. No blank fields are allowed. If the field is not relevant to the excursion; put N/a.
- Excursion coordinator must attend the excursion site and take photo of the excursion site. Added photos to risk assessment
- Google maps is used to show a map of the proposed route and destination.
- Educator to Child Ratio: this may be assessed based on excursion risk assessment elements. As a general rule, this ratio needs to be a higher level of supervision than the Regulatory ratio requirements or other risk mitigation strategies are implemented.
- As a general rule, water hazards are a high-risk level for an excursion and will not be performed by the service unless all risk can be mitigated to a low level.
- The Risk Assessment needs to be discussed with team and the Centre Director
- The Risk Assessment needs to be reviewed and signed off by the Excursion Coordinator and Centre Director.
- The Risk Assessment need to be approved by the Approved Provider before the excursion forms issued to parents and guardians.

Example of Risk Assessment Form:

Excursion risk management plan

Excursion Details for: Starfish Room_Library Excursion_22.08.25

Excursion Location:	Mildura Library - 180 - 190 Deakin Avenue		
Date(s) of Excursion*:	2025-08-22	End Date:	
Activity/Transport:	2025-08-22	End Date:	
Estimated departure and arrival times:	9:45am until 11:45am	Duration of the excursion:	2 hours
Pick up location and destination (s):	180-190 Deakin Avenue, Mildura, VIC, 3500.		
Proposed Activities:	Reasons for excursion: - Connecting with our surrounding community. - Visiting our local library as part of Book Week. List all activities that will take place during the excursion: - Exploring our surrounding community. - Furthering the children's knowledge of road safety. - Exploring a range of books, songs and resources at the library.		
Any water hazards during the excursion, including any risks associated with water-based activities? (if yes, detail in the risk assessment table below):	No	Comments (if any):	
Is this a Water-Based Activity? (If yes, swimming ability forms must be completed)	No		
Proposed Route/Destination:	Please see attached map.		
Means of transport:	Walking - children holding onto excursion rope.		

STEP 4: FORM REVIEW & SIGNOFF

- The excursion coordinators need to follow the instructions in the Excursion Checklist. Have the Centre Director review and sign off they have reviewed checklist, Excursion Form and Excursion Risk Assessment.
- The Centre Director needs to go into the Excursion Checklist and sign-off and date they have reviewed all forms and are happy for the excursion to occur.
- Final draft signoff by Approved Provider. Email Approved Provider to inform him that checklist (advise date created), parent consent form and risk assessment are ready for review.
- Approved Provider will advise of any changes to forms. And provide an email to advise the forms have been reviewed and approved.
- Once final approval has been received the Excursion forms can be issued to parents for signing via the OWNA app. See step 5.

STEP 5: ISSUE EXCURSION FORM TO PARENT & GUARDIAN FOR REVIEW AND SIGNING

Excursion Coordinator and Centres Director Tasks:

Complete an EMAIL and OWNA Post to room parents and guardians advising them of the Excursion and the need to review and sign the permission form in OWNA App. (see example below)

Example of OWNA Post and Email

Subject/Title of Email/Post: <Room Name> Excursion: Name of Excursion eg Fire Station Excursion

Good morning <Room Name> Families,

We are excited to let you know that we have organised excursion for <room name> children to <Excursion Name>. The purpose of the excursion is to

Summary of the excursion information is as follow (please refer to the excursion form and risk assessment for more details regarding the excursion):

Date: Date, Date Month Year

Cost of excursion: <\$X.XX insert amount>

Address of excursion site: <insert address>

<Optional if bus being used> Bus Departs: X:XXam

<Optional> Arrival back to Service: XX.XXam

<Optional> Please ensure that your child/ren are at the service by X.XXam. We will be having <morning tea or lunch> at X.XXam.

We have conducted an excursion risk assessment to ensure the event runs smoothly. Please do not hesitate to discuss with us any of your concerns.

Just a reminder that the excursion consent form are available to be signed via the OWNA. Can you please sign the form by <insert date> , so we have permission for you child/ren to attend the excursion.

Steps as follows:

1 Open main menu (top left)

2 Go to forms

3 Select the excursion form

4 review and sign.

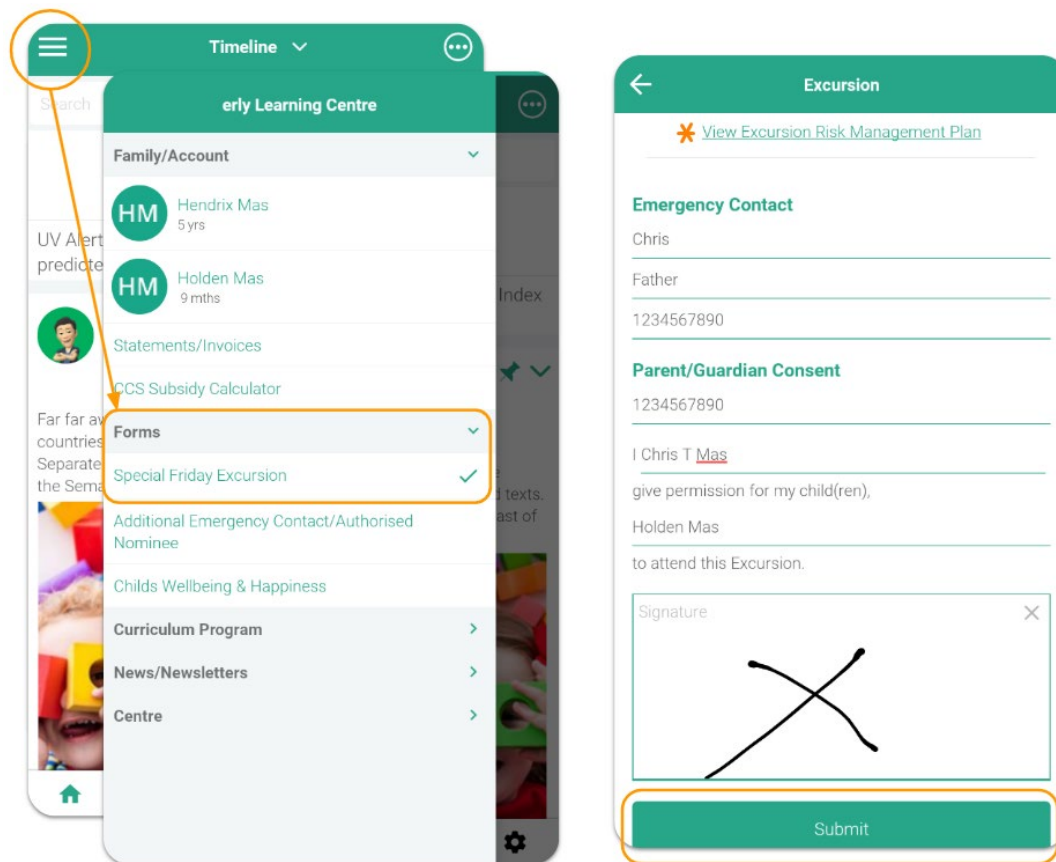
(please see image of instructions)

If you have any questions, please do not hesitate to discuss with me.

Kind regards,

<Excursion Coordinator and Centre Director Name>

Instruction on how to access Excursion Form and complete excursion permission/authorisation. Add Image to OWNA Post and Email



STEP 6: EXCURSION PROCEDURES: BEFORE, DURING & POST EXCURSION TASKS

Please continue follow the Excursion Policy & Procedures and the Excursion Checklist to ensure the excursion runs smoothly. Key tasks include:

- Track and monitor the completion of excursion permission forms by all parents and guardians. Collate signed consent forms and record receipt on printed roll.
- Send a reminder email and post 48 hours before the excursion. Upon drop-off and pick, educators to remind parent and guardian of excursion and the need to sign consent form.
- Centre Director is to review and ensure all signed consent forms are held for all children before excursion date.
- Educators to practice excursion routines with children. I.e walking together and following instructions. Discuss excursion with children: routine, safety and expectations.
- Understand and follow headcount procedures in accordance with Excursion Policy and Risk Assessment. Completed using children roll (paper). On exit of service and boarding of bus, arrival at excursion site, during excursion, return to serviced & boarding bus, when returned to room. The headcount needs to be performed by two educators each time. The headcount needs to be recorded on a paper roll.

APPENDIX 2: EXCURSION FORMS: INTERNAL COMPLIANCE CHECKLIST

Internal Audit - Excursions and Regular Outings

The Education and Care Services National Law Act 2010 (National Law) and the Education and Care Services National Regulations 2011 (National Regulations) prescribe a number of requirements for education and care services and family day care educators when they take children outside the education and care service, family day care residence or approved family day care venue as part of an excursion.

Excursions, including regular outings, provide valuable opportunities for children to explore the wider community and to extend the educational program. Approved providers, however, must be mindful of additional risks that may be present. Excursions must be conducted in a way that:

- ensures the safety, health and wellbeing of the children being educated and cared for by the service (section 167)
- meets the educational and developmental needs of the children being educated and cared for by the service (section 51).

For information and definitions of excursion, see regulation 4 of The Education and Care Services National Law Act 2010 (National Law).

Procedure for Internal Audit:

- *Print an Excursion Forms, Parent Consent and Risk Assessment*
- *Print Internal Audit – Excursions and Regular Outings form*
- *On excursion forms reference the internal audit reference number Eg <1A> on the documents to show how and where this requirement is covered.*
- *Create PDFs of completed documents.*

Internal Audit Review Task	Status
1 - Considerations for excursions	
In addition to the specific requirements of the National Regulations for taking children on excursions, important requirements of the National Law must be met at all times. The National Law requires the approved provider of an education and care service to ensure that: • 1A - all children being educated and cared for by the service are adequately supervised at all times that the children are in the care of the service (section 165) • 1B - every reasonable precaution is taken to protect children being educated and cared for by the service from any harm or hazard likely to cause injury (section 167) • 1C - relevant number of educators educating and caring for the children is no less than the number prescribed for this purpose (section 169). • 1D - maximum number of approved places for children who can be educated and cared for by the service at any one time must not be exceeded (section 52(g)). This means that the service will need to consider all the children being educated and cared for by the service at any one time, including the children at the education and care premises and those on the excursion.	Yes / No
2- Policies and procedures	
2A - A service must have in place policies and procedures in relation to excursions (regulation 168(2)(g)), including procedures complying with the requirements for conducting risk assessments and seeking authorisation for excursions set out in regulations 100 to 102. 2B- The policies and procedures must be followed, be readily accessible to all educators, staff members, and volunteers and be available for inspection at the service upon request (regulation 171).	Yes / No

2C - Services must ensure these policies and procedures include strategies for monitoring and accounting for children on excursions, that include regularly monitoring children in attendance to ensure they are accounted for at all times. Services should ensure children's presence is always checked against an accurate attendance record.	
3 - Authorisation for excursions	
A child must not be taken outside an education and care service, family day care residence or family day care venue on an excursion unless written authorisation is given by the child's parent or other person named in the child's enrolment record as having authority to authorise the taking of the child outside the education and care premises. The authorisation must state: • 3A - child's name • 3B - reason the child is to be taken outside the premises • 3C - date the child is to be taken on the excursion • 3D - description of the proposed destination for the excursion • 3E - method of transport to be used for the excursion • 3F - proposed activities to be undertaken by the child during the excursion • 3G - period the child will be away from the premises • 3H - anticipated number of children likely to be attending the excursion • 3I - anticipated ratio of educators to the anticipated number of children attending the excursion • 3J - anticipated number of staff members and any other adults who will accompany and supervise the children on the excursion • 3K - that a risk assessment has been prepared and is available at the service (regulation 102(4)).	Yes / No
4 - Authorisation for an excursion that is a regular outing If an excursion is a regular outing, the authorisation is required to be obtained once in every 12 month period (regulation 102(5)). However obtaining authorisation in these circumstances more regularly would be advisable. Authorisations for excursions must be kept in the enrolment record for each child enrolled at an education and care service (regulation 161(1)(b)) and in the enrolment record for each child educated and cared for by a family day care educator (regulation 161(2)(b)).	Yes / No
5- Risk assessments	
5A - The National Regulations require a risk assessment to be completed before an authorisation is sought for an excursion (regulation 100). A child being educated and cared for by the service must not be taken outside the service or the family day care educator's residence or approved family day care venue on an excursion unless written authorisation has been provided (regulation 102). 5B - The purpose of a risk assessment is to identify possible risks of harm to children prior to an excursion, to ensure risks to children's health, safety and wellbeing are minimised or avoided when taking children outside an education and care service , family day care residence or family day care venue. A risk assessment must include strategies for minimising and managing the identified risks (regulation 101). 5- When taking children outside an education and care service, a family day care residence or family day care venue, every aspect of the environment must be considered at each stage of the excursion to make sure that risks have been identified and addressed. A risk assessment must consider: • 5C - proposed route and destination for the excursion • 5D - any water hazards • 5E - any risks associated with water based activities	Yes / No

• 5F - method of transport to and from the proposed destination • 5G - number of adults and children involved in the excursion • 5H - given the risks posed, the number of educators or other responsible adults that is appropriate to provide supervision and whether any adults with specialised skills are required to ensure children's safety • 5I - proposed activities • 5J - proposed duration of the excursion • 5K - items that should be taken on the excursion. 5L - The risk assessment should also include strategies for accounting for all children on the excursion, particularly during transition times, such as ensuring all children have been transported to the destination and have been returned to the service at the conclusion of the excursion.	
Conducting a risk assessment for an excursion that is a regular outing A risk assessment is not required for a regular outing if a risk assessment has been conducted not more than 12 months before the excursion is to occur. Consideration should also be given to whether circumstances of the regular outing have changed since this risk assessment was completed (regulation 100(4)).	
Evaluating risks Once risks have been identified, the approved provider should categorise the risks as high, medium or low and then decide how the identified risks should be managed. Some risks may be able to be removed or strategies put in place to reduce children's exposure to the risk. Many potential risks can be managed by increasing the number of educators and/or adults accompanying and supervising children during the excursion. It is important to make a final check for risks before departing on a regular outing or excursion. Extreme weather or other unexpected events may increase the level of risk and may make it necessary to reconsider whether to proceed with the excursion.	Yes / No
Adequate supervision All excursions must be conducted in a way that ensures the health, safety and wellbeing of children. This means that an adequate number of educators and where necessary, other responsible adults, accompany the children. Generally, educator to child ratios will need to be higher than the prescribed ratios to ensure adequate supervision during an excursion. Depending on the supervision requirements for the excursion, educators will usually have other staff members and/or other responsible adults with them to ensure that adequate supervision of children is maintained at all times. Supervision is not confined to children. It also includes the supervision of responsible adults that assist with the excursion. The number of educators must be adequate to supervise any responsible adults who volunteer to assist with the excursion.	Yes / No
Staffing arrangements Educator to child ratios must be met whenever children are being educated and cared for by the service (section 169). This means maintaining educator to child ratios for the total number of children being educated and cared for by the service. To be included in the educator to child ratios, educators must be working directly with children (regulation 122). In addition, the qualification requirements for educators must be met (regulation 126-7).	Yes / No

A family day care educator must not educate and care for more than seven children at a family day care residence or approved family day care venue at any one time (regulation 124). For further details please see: Family Day Care Consideration must be given to the number of educators who have the required first aid qualifications and to the number of educators required to educate and care for children who remain at the service. Where a group of children are taken on an excursion while a number of children remain at the service, at least one educator who holds the following qualifications must be in attendance at both the excursion and at the service where children are being educated and cared for, and must be immediately available in an emergency. At least: • staff member or one nominated supervisor of the service who holds a current approved first aid qualification • staff member or one nominated supervisor of the service who has undertaken current approved anaphylaxis management training • staff member or one nominated supervisor of the service who has undertaken approved emergency asthma management training (regulation 136(1)). It is also important that any educators or responsible adults attending the excursion are aware of the service's policy and procedures that set out the instructions for what must be done in the event of an emergency (regulation 97(a)) and are aware of the risk assessment for the excursion. Family day care services should also be mindful of their obligations in relation to the number of children that can be educated and cared for at any one time (regulation 124).	
6- Planning excursions	
6A - Factors to be taken into account when planning an excursion include the: educational program: • the children's behaviour and any individual needs • the age of the children • the number of children • the proposed duration of the excursion • the abilities of children • the method of transport • any contact with water • accidents • access to toileting and nappy changing facilities. Most important is the requirement to ensure adequate supervision is provided at all times. The degree of planning required is influenced by the nature of the excursion, the level of risk and the children and adults who are participating. For excursions including regular outings that have previously been planned and conducted, previous risk assessments may need to be reviewed and revised. Consideration should be given to visiting the proposed excursion destination, where information can be gathered about the availability of toilets, hand washing, drinking and shade facilities including adequate mobile phone coverage and access for emergency services.	Yes / No
7- Educational program 7A - When planning an excursion educators and family day care educators must consider how the excursion supports the educational program and contributes to the outcomes outlined for each child (regulation 73(2)).	Yes / No
8 - Toilet, washing facilities, food and beverage requirements 8A- When planning for an excursion, consideration must be given to how long the children will be away from the service, family day care residence or family day care venue. If toileting facilities will be required, educators	Yes / No

must ensure that adequate toilet, washing and drying facilities are provided for safe use by children (regulation 109). 8B - Educators must ensure that children have access to safe drinking water and are offered appropriate food and beverages during the excursion (regulation 78). Adequate health and hygiene practices when handling, preparing and storing food must be in place to minimise risks to children (regulation 77).	
9 - Items to take on an excursion When children are taken from the service, family day care residence or family day care venue on an excursion, the following items should be taken with educators: • 9A - appropriate number of suitably equipped first aid kits (regulation 89) • 9B- operating mobile telephone with an appropriate telephone network (regulation 98) (In accordance with National Model Code Policy – use of personal mobile phone in emergency situations). • 9C - contact information of any person who is to be notified of any incident, injury, trauma or illness involving the child and the child's registered medical practitioner or medical service (regulation 162) • 9D - items specific to the excursion circumstances, such as sunscreen and hats.	Yes / No
10 - Meeting children's medical needs Children's medical needs must be addressed during an excursion. This may include the requirements for the administration of medication. Where a child attending the excursion has a specific health care need, allergy or relevant medical condition, the risk assessment should consider the management of the child's medical needs (regulation 160(3)).	Yes / No

APPENDIX 3: TYPE 1: Incursion requiring permission from parent / guardian to participate

The following needs to be prepared for each incursion requiring permission from a parent / guardian to participate in the incursion and/or authority to charge parent account for incursion fee:

Task 1: Incursion Checklist: This is completed via the OWNA App. See Checklists > Incursion Checklist

Task 2: Incursion Form and Parent Consent Form

Task 3: Incursion Risk Assessment

Overview:

- See OWNA tutorial for use of OWNA tools in portal:
<https://ownaportal.tawk.help/article/excursion-forms>
- Commence first with the Incursion Checklist (this includes instructions to complete the incursion forms and planning needed to be undertaken.)
- For a new incursion copy from the OWNA incursion form template. Custom all information for the incursion.
- The incursion coordinators need to follow the instructions in the incursion Checklist. Have the Centre Director review and sign off they have reviewed checklist, Incursion Form and Incursion Risk Assessment. The Centre Director needs to go into the Incursion Checklist and sign-off and date they have reviewed all forms and are happy for the incursion to occur.
- Final draft signoff by Approved Provider. Email Approved Provider to inform him that checklist (advise date created), parent consent form and risk assessment are ready for review.
- Approved Provider will advise of any changes to forms. And provide an email to advise the forms have been reviewed and approved.
- Once final approval has been received the incursion forms can be issued to parents for signing via the OWNA app.

STEP 1: INCURSION CHECKLIST

Access via OWNA App: Checklists > Incursion Checklist

Before commencing the Incursion Checklist, please discuss the following with the Nominated Supervisor / Centre Director:

- incursion idea and plan. The benefits and outcomes of the incursion
- the incursion plan. Initial risk assessment. Are the risks acceptable for the service?
- Do the benefits and outcomes outweigh risk. Can the risk be mitigated to an acceptable and manageable level?
- Budget: costs to service and costs to parents?
- potential date of incursion. Is there sufficient time to plan and coordinate incursion. i.e. children, team and parents permission forms.
- team discussions regarding incursion. Are they supporting the incursion.

Purpose: the Incursion Checklist provides the Incursion Coordinator a list of tasks to complete before, during and after the Incursion. The Incursion Checklist is created on a specific date. This date needs to be remembered as you need to return to this date to continue with the Incursion Checklist tasks.

(If there are two incursions being planned: the checklist can be commenced on two different dates so the checklist can be commenced by the incursion coordinator. This is due to only one incursion checklist being allowed to be created per day in using the OWNA app.)

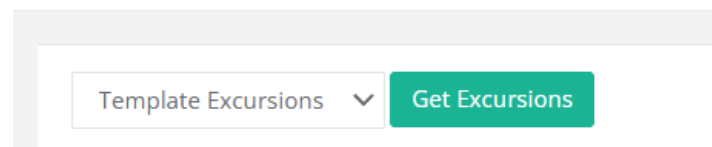
STEP 2: INCURSION FORM & PARENT CONSENT

Access via OWINA Portal: Children > Excursion/Incursion

Template for Incursion Form: this is access via the following section of the OWINA Portal. See GSELC Incursion Template in the Templates folder.

Manage Excursions

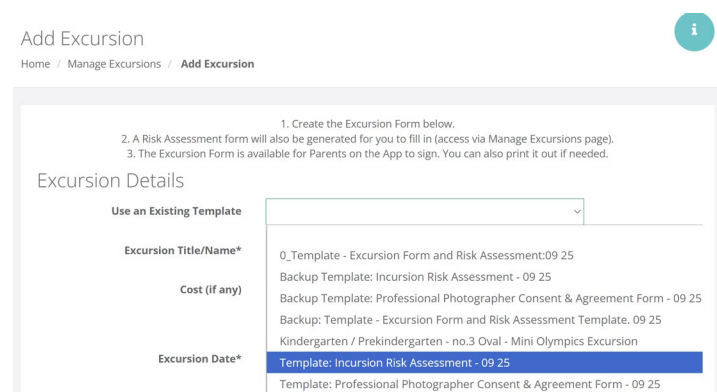
Home / Children / **Excursion/Incursion**



Template Excursions ▼ Get Excursions

Creating a new Incursion form to use:

After discussed incursion idea with Nominated Supervisor / Centre Director enter OWINA portal and create Incursion Form and Risk Assessment from template. (Enter OWINA Portal > Children > Manage Excursions/Transport Risk Mgt Plan > Press '+' > Select excursion template > Complete form sections and fields.



Add Excursion

Home / Manage Excursions / Add Excursion

1. Create the Excursion Form below.
2. A Risk Assessment form will also be generated for you to fill in (access via Manage Excursions page).
3. The Excursion Form is available for Parents on the App to sign. You can also print it out if needed.

Excursion Details

Use an Existing Template

Excursion Title/Name*

Cost (if any)

Excursion Date*

0_Template - Excursion Form and Risk Assessment:09 25
Backup Template: Incursion Risk Assessment - 09 25
Backup Template: Professional Photographer Consent & Agreement Form - 09 25
Backup: Template - Excursion Form and Risk Assessment Template. 09 25
Kindergarten / Prekindergarten - no.3 Oval - Mini Olympics Excursion
Template: Incursion Risk Assessment - 09 25
Template: Professional Photographer Consent & Agreement Form - 09 25

The following is copy of the GSELC Incursion Form template to parents and guardians. The form is needed to be customised for each incursion by the incursion coordinator and reviewed by the Centre Director. This is the letter that the parent receives and advises them of the excursion information.

Dear Families,

INCURSION CONSENT & AUTHORISATION

The <insert name of room> are planning an <name of incursion> incursion on <insert>. The incursion will provide<add in reason for incursion being held>.

With an incursion children do not leave the service and participate as part of their enrolled day. We have summarised the incursion plan as follows and completed an incursion risk management plan. The incursion plan should be read in conjunction with the service policies, procedures and risk assessments. The risk assessment is available for viewing at the service.

The signing of the incursion consent form gives authorisation for your child to attend the incursion detailed in this form.

Policy Handbook – P004. 7.2026

If you have any questions regarding the incursion, feel free to discuss with the Room Educators or Centre Director.

Kind regards,

<Insert Name of Incursion Coordinator>

<Insert Room Name>

INCURSION INFORMATION	
Name of Incursion:	<Brief name Eg. Kindergarten Room: Animal Farm>
Address Incursion:	<Service Name and address>
Date and Time of Incursion:	<Insert Date Eg. Friday 20 September 2025> <Insert time: Eg From: 9:45am To: 10:15am>
Incursion Provider's Name:	
Incursion Provider's Website:	
Cost for incursion per child (this cost will be added to family account. Signing of incursion form gives authorisation for charging onto Service invoice / statement.)	<Insert \$>
Name of incursion coordinator:	<Insert Name>
Name of classroom/s attending incursion:	<Insert Room Name>
Age group of children attending incursion:	<X years to X years>
Maximum Number of children attending incursion:	<Insert Number>
Reason for incursion: • <List here> • <List here> List all activities that will take place during the incursion: • <List here> • <List here> Learning Outcomes:	

OTHER TERMS AND CONDITIONS OF INCURSION – ACKNOWLEDGEMENTS BY PARENT

Child Behaviour: 'I understand that in the event of my child's misbehaviour or behaviour that poses a danger to himself/herself or others before or during the incursion, he/she may be sent home or required to be collected from the incursion by the parent. I further understand that in such circumstances any costs associated with the incursion will be my responsibility.'

Child illness: 'I understand that educators determine if it is necessary for my child to be sent home early due to illness, any cost associated with the incursion will be my responsibility.'

Cancellations or Alterations: 'I understand that the Service may need to cancel or alter incursion arrangements at short notice, due to circumstances beyond the control of the Service, and while the Service will try to minimise inconvenience or financial losses to parents, these may be unavoidable.'

INCURSION RISK MANAGEMENT ANALYSIS	
Overall Incursion Risk Assessment is completed and there is an acceptable level of risk and sufficient strategies are documented (see risk management assessment attached):	YES / NO (Needs to be "Yes" for incursion to be held)
Incursion coordinator conducts a risk assessment of the area to identify potential risks and/or hazards and discuss how you will manage these prior to receiving approval for incursion. The incursion coordinator must also conduct another risk assessment on the day of the incursion to ensure that the area is still suitable.	
Use the risk assessment table to identify and assess risks to the safety, health or wellbeing of children and adults attending the incursion, and specify how these risks will be managed and minimised. This must include any risks associated with water-based activities. We have identified possible hazards and issues while attending the incursion and have rated the risk assessment using the matrix. If hazards and issues are rated as having a high level of risk due to the consequences, but the likelihood to occur is low. Educators have included strategies and information on how we will manage to minimise the risks to an acceptable level (Medium or Low) for a successful incursion to occur. Our risk assessment considers: - any water hazards - any risks associated with water-based activities - number of adults and children involved in the incursion - given the risks posed, the number of educators or other responsible adults that is appropriate to provide supervision and whether any adults with specialised skills are required to ensure children's safety - proposed activities - proposed duration of the incursion - strategies for accounting for all children on the incursion, particularly during transition times.	
v09 25	

STEP 3: RISK ASSESSMENT FORM FOR INCURSION

Access via OWNA Portal: Children > Excursion/Incursion

As mentioned above in Step 2: After discussed incursion idea with Nominated Supervisor / Centre Director enter OWNA portal and create Incursion Form and Risk Assessment from template. (Enter OWNA Portal > Children > Manage Excursions/Transport Risk Mgt Plan > Press '+' > Select excursion template > Complete form sections and fields.

Tasks:

- All fields and questions in the risk assessment form must be completed. No blank fields are allowed. If the field is not relevant to the excursion; put N/a.
- Incursion coordinator must assess the service for the incursion.
- Educator to Child Ratio: this may be assessed based on incursion risk assessment elements.
- As a general rule; water hazards are a high-risk level for an incursion and mitigated to a low level.
- The Risk Assessment needs to be discussed with team and the Centre Director
- The Risk Assessment needs to be reviewed and signed off by the Incursion Coordinator and Centre Director.
- The Risk Assessment need to be approved by the Approved Provider before the incursion forms issued to parents and guardians.

Example of Risk Assessment Form Extract:

Excursion risk management plan

Excursion Details for: Template: Incursion Risk Assessment - 09 25

Excursion Location:
Insert Service Name & Insert Full Address of Service

Date(s) of Excursion*:
2025-09-20

End Date:

Activity/Transport:

End Date:

Estimated departure and arrival times:
Insert time

Duration of the excursion:
insert duration

Risk assessment

Use the table below to identify and assess risks to the safety, health or wellbeing of children attending the excursion, and specify how these risks will be managed and minimised [regulation 101(1)]. This must include any risks associated with water-based activities

#	Activity	Hazard identified	Risk assessment	Elimination/control measures	Who	When
1	Supervision of Adults	■ Insufficient supervision leading to safety incidents	High	■ Assign specific roles to adult volunteers roles and location with an educator. ■ Conduct briefing for all adults on safety protocols and child supervision. ■ Residual Risk: Low	Centre Director/RP, Incursion coordinator and educators	At all times
2	Hazards from the natural environment.	■ Insects, animal waste, hazardous plants.	Moderate	■ Conduct a inspection prior to the event. ■ Ensure cleaning supplies are available. ■ Mark hazardous areas ■ Residual Risk: Low	Centre Director/RP, Incursion coordinator and educators	At all times
3	Reaction to Allergies	■ Allergic reactions to food, natural environment, animal fur, etc	High	■ Review staff and child medical management plans. ■ Assess risk assessments for medical condition management	Centre Director/RP, Incursion coordinator and educators	At all times

STEP 4: INCURSION FORM REVIEW & SIGNOFF

- The incursion coordinators need to follows the instructions in the Incursion Checklist. Have the Centre Director review and sign off they have review checklist, Incursion Form and Incursion Risk Assessment.
- The Centre Director needs to go into the Incursion Checklist and sign-off and date they have reviewed all forms and are happy for the incursion to occur.
- Final draft signoff by Approved Provider. Email Approved Provider to inform him that checklist (advise date created), parent consent form and risk assessment are ready for review.

- Approved Provider will advise of any changes to forms. And provide an email to advise the forms has been reviewed and approved.
- Once final approval has been received the Incursion forms can be issued to parents for signing via the OWNA app. See step 5.

STEP 5: ISSUE INCURSION FORM TO PARENT & GUARDIAN FOR REVIEW AND SIGNING

Excursion Coordinator and Centres Director Tasks:

Complete an EMAIL and OWNA Post to room parents and guardians advising them of the Incursion and the need to review and sign the permission form in OWNA App. (see example below)

Example of OWNA Post and Email

Subject/Title of Email/Post: <Room Name> Incursion: Name of incursion eg 'Kindergarten: Animal Farm'

Good morning <Room Name> Families,

We are excited to let you know that we have organised incursion for <room name> children to <incursion Name>. The incursion is held at the service on the children's enrolled day. The purpose of the incursion is to

Summary of the incursion information is as follow (please refer to the incursion form and risk assessment for more details regarding the incursion):

Date: Date, Date Month Year

Cost of incursion: <\$X.XX insert amount or \$> (this will be charged to your account).

<Optional> Please ensure that your child/ren are at the service by X.XXam. We will be having <morning tea or lunch> at X.XXam.

We have conducted an incursion risk assessment to ensure the event runs smoothly.

Please do not hesitate to discuss with us any of your concerns.

Just a reminder that the incursion consent form are available to be signed via the OWNA. Can you please sign the form by <insert date> , so we have permission for you child/ren to attend the incursion.

Steps as follows:

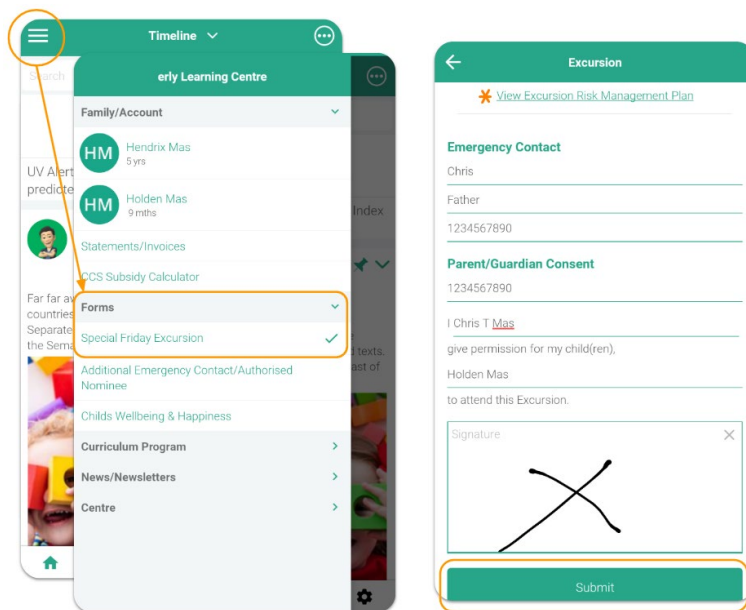
- 1 Open OWNA App main menu (top left)
 - 2 Go to forms
 - 3 Select the incursion form named above
 - 4 review and sign.
- (please see image of instructions)

If you have any questions, please do not hesitate to discuss with me.

Kind regards,

<Incursion Coordinator and Centre Director Name>

Instruction on how to access incursion Form and complete incursion permission/authorisation. Add Image to OWNA Post and Email



APPENDIX 4: TYPE 2 - INCURSION RISK ASSESSMENT & MANAGEMENT

Incursion Risk Assessment and Management – Delivered by Incursion Provider

(Incursions occurs at service. If children are leaving service, you are required to complete excursion form and risk assessment in OWNA.)

Service name:		
Name of Service incursion coordinator:		
INCURSION DETAILS		
Date(s) and Time(s) of incursion:		
Proposed activities: List all activities that will take place during the incursion		
Any water hazards during the incursion, including any risks associated with water-based activities?	Yes / No Comment: If yes, detail in the risk assessment table below	
Incursion Provider Details:	Name:	
	Organisation Name:	

	Organisation Address:	
	Email Address:	
	Contact Number:	
Sighted visitor/performer risk assessment and public liability insurance?	☐ Yes - Public Liability Insurance ☐ Copy held Date received: ☐ Yes – Incursion Provider Risk Assessment ☐ Copy held. ☐ No risk assessment. The following risk assessment has been prepared with Incursion Provider. Date provided risk assessment:	
Check Working With Children Check or VIT registration of visitor or volunteers?	WWCC or VIT Number: Verified: Yes / No Date Verified: ____ / ____ / ____	
Number and full names of each adult involved in the incursion: E.g. service staff, family members, volunteers	Educator names: Other adults names:	
The number of educators / responsible adults, appropriate to provide supervision, and whether any adults with specialised skills are required: E.g. for children's individual needs.		

The number of children involved in the incursion:		
Is room space within service requirements? <i>Guidance for space:</i> <i>3.25 square meters per child inside</i> <i>7 square meters per child outside</i>	Location of Incursion: Number of children normally in this space:	
Educator to child ratio, including whether this incursion warrants a higher ratio: Provide details in the risk assessment table below	☐ 1:11 (Over 3-year-old children) ☐ 1:4 (Under 3-years-old children) <i>Note if higher ratio needed</i>	
Incursion checklist – items to be readily available during the incursion (please tick)		
☐ List of educators and adults participating in the incursion	☐ Sighted visitor/performer risk assessment and public liability insurance?	
☐ Check Working With Children Check or VIT registration of visitor/performer or volunteers?	☐ Discussed with Incursion Provider, Service's Child Protection and Wellbeing Policy, National Model Code Policy, Digital & Technology Use Policy and Visitor Code of Conduct Policy.	
☐ Parents / guardians informed of incursion via email post. Date, time and if any costs. Permission is obtained from parent for costs. Comment:	☐ Reviewed medical and health information for each child. Assess any risks associated with incursion resources, animal or changes to environment.	
☐ Other items, please list:		

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Risk Assessment - Examples of Possible Activities (to be customised for each incursion)						
Activity	Hazard identified	Risk assessment (use matrix)	Elimination/control measures	Target Risk Assessment After Elimination / Control Measures	Who	When
Change of regular incursion provider.	Incursion provider lack of familiar with service policy and procedures	High	- Review and assess the new provider's safety protocols and qualifications. - Conduct a pre-event meeting with the new provider to discuss expectations and safety measures.	Low	Centre Director and incursion coordinator	Before incursion
Special events where families and visitor are invited to attend service.	Photos and video being taken by families and visitors with permission and against service policy	Medium	- Inform parents and visitors of service policy in relation to personal devices (phones); photos and video records. No photos or video allowed.	Low	Incursion coordinator and Centre Director	Before and during incursion
Add in other risk activities related to incursion.			-			

(Press tab to add more rows)

Please list how the team will plan for and address the below Child Safe Standards

Culturally Safe Environments for Aboriginal Children: *Establish a culturally safe environment in which the diverse and unique identities and Experiences of Aboriginal children are respected and valued.*

We believe a culturally safe environment can lead to better engagement and participation in learning activities. When children feel valued, they are more likely to take an active role in their education, leading to improved academic performance.

Promoting a culturally safe environment teaches all children about the importance of diversity and cultural respect. This cultivates understanding and acceptance among peers, reducing prejudice and fostering inclusivity.

Examples:

- *Creating a culturally safe environment for Aboriginal children is essential for their well-being, development, and overall success. It not only benefits the individual children but also enriches the entire learning community, fostering a culture of respect, inclusivity, and understanding.*
- *Valuing and respecting Aboriginal cultures helps preserve cultural identities and traditions. Children learn to appreciate their heritage, which is crucial for cultural continuity and community strength.*
- *Involving Aboriginal families and community members in the childcare environment fosters stronger connections between the service and the broader community. This collaboration enhances support networks for children and families.*
- *Recognizing and integrating Aboriginal cultural practices into the learning environment supports the holistic development of children, encompassing physical, emotional, social, and spiritual growth.*
- *Establishing a culturally safe environment helps combat racism and discrimination. By promoting awareness and understanding of Aboriginal cultures, children learn to challenge stereotypes and biases.*
- **Collaboration:** *Reach out to local Aboriginal community leaders, elders, and cultural educators to collaborate on the planning of the incursion. Their input is vital to ensure cultural appropriateness and relevance.*
- **Cultural Protocols:** *Discuss and establish cultural protocols and expectations that should be followed during the incursion.*
- **Culturally Relevant Activities:** *Plan activities that reflect the cultural practices and traditions of the Aboriginal community. This could include art workshops, traditional games, storytelling, and music.*
- **Food Considerations:** *If food is provided, ensure it respects dietary restrictions and cultural preferences. Engage community members to prepare traditional foods if appropriate.*

Child and Student Empowerment: *Children are empowered about their rights, participate in decisions affecting them and are taken seriously*

We empower children by recognising their rights, involving them in decision-making, and validating their experiences. This approach fosters a positive sense of self, promotes confidence, and encourages active participation in their learning environment.

Examples:

- **Collaborative Planning:** *Before the incursion, hold a meeting with children to discuss what activities they would like to include. Use drawings or brainstorming sessions to visualize their ideas, ensuring that their voices are heard in the planning process.*
- **Establish Ground Rules:** *Collaboratively create a set of ground rules for the incursion that everyone agrees upon, ensuring that children feel safe and respected. Allow them to contribute ideas about what makes a safe and inclusive environment.*
- **Assign Roles:** *Involve children in various roles during the incursion, such as helpers or leaders. For example, they could help set up materials, lead a group activity, or welcome guests. This fosters a sense of responsibility and ownership.*
- **Group Discussions:** *After the incursion, hold a group discussion where children can share their thoughts and feelings about the activities. Encourage them to express what they enjoyed and what they would like to do differently next time.*
- **Cultural Sharing:** *Allow children to share their own cultural stories, traditions, or artifacts during the incursion. This gives them a voice in representing their identities and promotes respect for diversity among peers.*

Risk Matrix:

Likelihood	Consequence				
	Negligible	Minor	Moderate	Major	Catastrophic
Almost Certain	Medium	High	High	Very High	Very High
Likely	Medium	Medium	High	Very High	Very High
Possible	Low	Medium	Medium	High	Very High
Unlikely	Low	Low	Medium	High	High
Rare	Low	Low	Low	Medium	High

Actions required based on Ratings:

Risk Level	Action	Details
Very High	Act immediately to mitigate the risk. Eliminate, substitute, or implement control measures.	Remove the hazard at the source. An identified very high risk does not allow scope for the use of administrative controls, even in the short term.
High	Act immediately to mitigate the risk. Eliminate, substitute, or implement control measures.	An achievable timeframe must be established to ensure that elimination, substitution or controls are implemented.
Medium	Take reasonable steps to mitigate the risk. Until elimination, substitution, or controls can be implemented, institute administrative or personal protective equipment controls. These "lower-level" controls must not be considered permanent solutions.	Interim measures until permanent solutions can be implemented: - develop administrative controls to limit the use or access - provide supervision & specific training related to the issue of concern
Low	Take reasonable steps to mitigate and monitor the risk — Institute permanent controls in the long term. Permanent controls may be administrative if the hazard has a low frequency, rare likelihood and insignificant consequence.	Operational procedures, regular risk awareness, risk reporting as part of department procedures

Plan and review		
Plan prepared by:	Full name: Role/position:	Date: Click or tap to enter a date.
Prepared in consultation Incursion Provider:	Full name: Role/position:	
Communicated to all relevant staff:	☐ Yes Comment if needed:	
Communicated to all relevant parent and guardians:	☐ Yes Date: Comment if needed:	
Centre Director/Nominated Supervisor/ Responsible Person in Charge approved:	☐ Yes ☐ No Comments if needed:	Date: Click or tap to enter a date.
Approved Provider approved:	☐ Yes ☐ No Comments if needed:	Date: Click or tap to enter a date.

APPENDIX 5: TYPE 3 – Event Risk Assessment and Management

Event Risk Assessment and Management

(No incursion provider. Event occurs at service. If children are leaving service, you are required to complete excursion form and risk assessment in OWNA.)

Service name:	
Name of Service event coordinator:	
EVENT / INCURSION DETAILS	
Date(s) and Time(s) of event:	
Proposed activities: List all activities that will take place during the event	
Any water hazards during the event, including any risks associated with water-based activities?	Yes / No Comment: If yes, detail in the risk assessment table below
Number and full names of each adult involved in the incursion: E.g. service staff, family members, volunteers	Educator names: Other adults names:

The number of educators / responsible adults, appropriate to provide supervision, and whether any adults with specialised skills are required: E.g. for children's individual needs.	
The number of children involved in the event:	
Is room space within service requirements? <i>Guidance for space:</i> 3.25 square meters per child inside 7 square meters per child outside	Location of Event: Number of children normally in this space:
Educator to child ratio, including whether this incursion warrants a higher ratio: Provide details in the risk assessment table below	☐ 1:11 (Over 3-year-old children) ☐ 1:4 (Under 3-years-old children) <i>Note if higher ratio needed</i>
Event checklist – planning and items to be readily for the Event (please add specific items needed for event. Tick on completion)	
☐ Team meeting to discuss the planning and key tasks for a successful event. Complete risk assessment. Establish to-do list.	☐ Check Working With Children Check or VIT registration of volunteers for event? Online verification of completed and copies of checks kept on file.

☐ List of adults participating in event. Visitors register schedule to be maintained for event. Visitors register scanned and saved to SharePoint or Teams folder.	☐ Review and discuss Service's Child Protection and Wellbeing Policy, National Model Code Policy, Digital & Technology Use Policy and Visitor Code of Conduct Policy.
☐ Parents / guardians informed of event via emails and OWNA post. Date, time, restrictions (food, personal devices, taking of photos). Comment:	☐ Reviewed medical and health information for each child. Assess any risks associated with event: resources, animal or changes to environment.
☐ Use of OWNA Event tool to send our diary reminder, invitation and RSVP tool.	☐ Other items, please list:

Risk Assessment for Event						
Activity	Hazard identified	Risk assessment (use matrix)	Elimination/control measures	Target Risk Assessment After Elimination / Control Measures	Who	When
Supervision of Adults	Insufficient supervision leading to safety incidents	High	- Assign specific roles to adult volunteers roles and location with an educator. - Conduct briefing for all adults on safety protocols and child supervision.	Low	Centre Director, incursion coordinator, educators	During incursion
Special events where families and visitor are invited to attend service.	Photos and video being taken by families and visitors with permission and against service policy	Medium	- Inform parents and visitors of service policy in relation to personal devices (phones); photos and video records. No photos or video allowed.	Low	Incursion coordinator and Centre Director	Before and during incursion
Add other activities relevant to event			-			

(Press tab to add more rows)

Please list how the team will plan for and address the below Child Safe Standards

Culturally Safe Environments for Aboriginal Children: *Establish a culturally safe environment in which the diverse and unique identities and Experiences of Aboriginal children are respected and valued.*

We believe a culturally safe environment can lead to better engagement and participation in learning activities. When children feel valued, they are more likely to take an active role in their education, leading to improved academic performance.

Promoting a culturally safe environment teaches all children about the importance of diversity and cultural respect. This cultivates understanding and acceptance among peers, reducing prejudice and fostering inclusivity.

Examples:

- *Creating a culturally safe environment for Aboriginal children is essential for their well-being, development, and overall success. It not only benefits the individual children but also enriches the entire learning community, fostering a culture of respect, inclusivity, and understanding.*
- *Valuing and respecting Aboriginal cultures helps preserve cultural identities and traditions. Children learn to appreciate their heritage, which is crucial for cultural continuity and community strength.*
- *Involving Aboriginal families and community members in the childcare environment fosters stronger connections between the service and the broader community. This collaboration enhances support networks for children and families.*
- *Recognizing and integrating Aboriginal cultural practices into the learning environment supports the holistic development of children, encompassing physical, emotional, social, and spiritual growth.*
- *Establishing a culturally safe environment helps combat racism and discrimination. By promoting awareness and understanding of Aboriginal cultures, children learn to challenge stereotypes and biases.*
- **Collaboration:** *Reach out to local Aboriginal community leaders, elders, and cultural educators to collaborate on the planning of the incursion. Their input is vital to ensure cultural appropriateness and relevance.*
- **Cultural Protocols:** *Discuss and establish cultural protocols and expectations that should be followed during the incursion.*
- **Culturally Relevant Activities:** *Plan activities that reflect the cultural practices and traditions of the Aboriginal community. This could include art workshops, traditional games, storytelling, and music.*
- **Food Considerations:** *If food is provided, ensure it respects dietary restrictions and cultural preferences. Engage community members to prepare traditional foods if appropriate.*

Child and Student Empowerment: *Children are empowered about their rights, participate in decisions affecting them and are taken seriously*

We empower children by recognising their rights, involving them in decision-making, and validating their experiences. This approach fosters a positive sense of self, promotes confidence, and encourages active participation in their learning environment.

Examples:

- **Collaborative Planning:** *Before the incursion, hold a meeting with children to discuss what activities they would like to include. Use drawings or brainstorming sessions to visualize their ideas, ensuring that their voices are heard in the planning process.*
- **Establish Ground Rules:** *Collaboratively create a set of ground rules for the incursion that everyone agrees upon, ensuring that children feel safe and respected. Allow them to contribute ideas about what makes a safe and inclusive environment.*
- **Assign Roles:** *Involve children in various roles during the incursion, such as helpers or leaders. For example, they could help set up materials, lead a group activity, or welcome guests. This fosters a sense of responsibility and ownership.*
- **Group Discussions:** *After the incursion, hold a group discussion where children can share their thoughts and feelings about the activities. Encourage them to express what they enjoyed and what they would like to do differently next time.*
- **Cultural Sharing:** *Allow children to share their own cultural stories, traditions, or artifacts during the incursion. This gives them a voice in representing their identities and promotes respect for diversity among peers.*

Risk Matrix:

Likelihood	Consequence				
	Negligible	Minor	Moderate	Major	Catastrophic
Almost Certain	Medium	High	High	Very High	Very High
Likely	Medium	Medium	High	Very High	Very High
Possible	Low	Medium	Medium	High	Very High
Unlikely	Low	Low	Medium	High	High
Rare	Low	Low	Low	Medium	High

Actions required based on Ratings:

Risk Level	Action	Details
Very High	Act immediately to mitigate the risk. Eliminate, substitute, or implement control measures.	Remove the hazard at the source. An identified very high risk does not allow scope for the use of administrative controls, even in the short term.
High	Act immediately to mitigate the risk. Eliminate, substitute, or implement control measures.	An achievable timeframe must be established to ensure that elimination, substitution or controls are implemented.
Medium	Take reasonable steps to mitigate the risk. Until elimination, substitution, or controls can be implemented, institute administrative or personal protective equipment controls. These "lower-level" controls must not be considered permanent solutions.	Interim measures until permanent solutions can be implemented: - develop administrative controls to limit the use or access - provide supervision & specific training related to the issue of concern
Low	Take reasonable steps to mitigate and monitor the risk — Institute permanent controls in the long term. Permanent controls may be administrative if the hazard has a low frequency, rare likelihood and insignificant consequence.	Operational procedures, regular risk awareness, risk reporting as part of department procedures

Plan and review		
Plan prepared by:	Full name: Role/position:	Date: Click or tap to enter a date.
Prepared in consultation Incursion Provider:	Full name: Role/position:	
Communicated to all relevant staff:	☐ Yes Comment if needed:	
Communicated to all relevant parent and guardians:	☐ Yes Date: Comment if needed:	
Centre Director/Nominated Supervisor/ Responsible Person in Charge approved:	☐ Yes ☐ No Comments if needed:	Date: Click or tap to enter a date.
Approved Provider approved:	☐ Yes ☐ No Comments if needed:	Date: Click or tap to enter a date.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 29/9/2025

Reference:

NQS

QA2	2.2.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	100	<i>Risk assessment must be conducted before excursion.</i>
	101	<i>Conduct of risk assessment for excursion.</i>
	102	<i>Authorisation for excursion.</i>

Other References:

Handbook on Child Care Licensing

QIAS Principle 4.5, 5.2

CCB – Federal Funding Guidelines

Public Health and Wellbeing Act 2008

Roads and Traffic Authority

Australian Standards

Legislation:

Bus Safety Act 2009

Transport Act 1983

49 - TRANSITION TO SCHOOL AND OTHER ORGANISATIONS POLICY

Position Statement

As each child moves closer to the time they leave the childcare centre, they can become apprehensive regarding what lays ahead of them. This policy has been developed to ensure that the transition from childcare to the next organisation is a smooth transition for each child and their family. School readiness will be a focus of programs in older groups, especially from September to December each year.

Objective

To prepare each child and their family for the transition from child care into school or other organisations.

Implementation

- Team members will make adjustments to the daily program and activities to expose each child to similar experiences as they will experience in their next environment.
- Children may be encouraged to participate in quiet activities during rest time where it is evident that they need less sleep in preparation for the longer days at school.
- Children may be asked to bring a lunch box on certain days so that their meals will resemble the expectation in the coming year.
- Team members will liaise with parents and devise a list of schools or organisations that their children will attend.
- Team members will make contact with the schools or organisations mentioned above to discuss the preparation required for each child and the information that can be provided by the childcare centre to that organisation to assist with transition. School also contact service to provided school enrolment information.
- Team members will consult with parents about their child's needs and areas of development to determine their readiness for the transition.
- Team members will be aware of individual children's development and ensure that these children are given opportunities to participate in experiences that prepare them for the new organisation.
- Parents information sessions are held for Pre-Kindergarten children families for their kindergarten year.
- Request families to advise when they expect to send their child to school to help the service with planning.
- Wherever possible, team members will organise visits to local schools or organisations to assist to familiarise children with their new environment.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 19/11/2025

Reference:

NQS

QA6	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
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QA7	4.1.2	Every effort is made for children to experience continuity of educators at the service..
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Handbook on Child Care Licensing
QIAS Principle 1.2, 1.3, 3.3, 4.1
Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2004

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

50 - AFTER SCHOOL CARE COLLECTION POLICY

Position Statement

The collection of school age children from schools and delivery of them to their enrolled centre is a responsibility that is taken very seriously. For this reason clear guidelines need to be set in place so that uniformity of procedures and actions ensures children arrive safely.

Objective

To provide a clear set of guidelines for team members to follow during the planning and pick up of school age children from school and their delivery to their respective After School Care service.

Preparation

- Permission forms specifying the mode of travel will be signed by parents or guardians for collection of children from school (OPR006).
- The Responsible Person in Charge for After School Care will ensure that all collections comply with requirements for child-to-team member ratios in relation to the mode of travel.
- The person in charge of collections will carry an approved mobile phone with them during the time they are away from the centre.
- It will be the responsibility of the Responsible Person in Charge for After School Care to ensure that the mobile phone has been powered and contains credit for phone calls.
- It is preferred practice that team members who are driving pull over to make/receive phone calls as soon as safely possible, however in emergency, it is legal to drive whilst using a Responsible Person in Charge for After School Care will ensure attendance rolls are correct at the beginning of each week and report any discrepancies to the Centre Director immediately.
- The Responsible Person in Charge for After School Care will familiarise themselves with the name, age and classroom of the children to be collected as well as any meeting points.
- To promote safe travel, the Responsible Person in Charge for After School Care will ensure that all children are aware of the expected behaviours for the trip. The centre's travel rules will be displayed in the After School Care area at the service, and where applicable, in the Centre Bus.
- The Responsible Person in Charge for After School Care will ensure that all children are aware of what to do in the case of not being collected ie. they will be instructed to go straight to school administration to advise teachers of non collection. This will form part of weekly discussions in order to reinforce the procedure.
- In the case of one of our neighbouring centres collecting children that are enrolled in your service, the following procedures will apply:-
 - Liaise with Educational Leader who is collecting child/children no later than 1.00pm daily by phone to confirm name of child/children or any cancellations.
 - The collecting Educational Leader will write the names of children and their days of collection on their own weekly attendance roll at the beginning of each week.

Implementation

- The Responsible Person in Charge for After School Care will check that each child on the roll has joined them. The Educational Leader will count the number of children picked up.
- If a child has been confirmed to be collected and is not at the designated meeting point, the Responsible Person in Charge for After School Care will immediately phone the

relevant Centre Director to reconfirm that collection was required. The Centre Director will phone the family contact to ascertain if collection was required. The Educational Leader will also confirm with the school's administration of the child's attendance that day.

- **Under NO circumstances** will an Responsible Person in Charge for After School Care leave the school until confirmation has been received, or the Centre Director directs the Responsible Person in Charge for After School Care to do otherwise.
- The Responsible Person in Charge for After School Care will ensure that all children travelling in a vehicle are safely secured with seatbelts or harnesses by checking each child once seated. When walking to and from school or to the transporting vehicle, the Responsible Person in Charge for After School Care will ensure that children are partnered/grouped appropriately.
- In the case of a neighbouring centre collecting children that are enrolled in your service, the following procedures will apply:-
 - The Responsible Person in Charge for After School Care will meet at a pre-designated safe place, preferably one of the centres. Children will be reallocated to the Responsible Person in Charge for After School Care for their enrolled centre at which time they will be checked on the roll. Procedures for safe travel will be rechecked before the groups disperse.
 - Any children on the enrolled service roll who have not arrived will be queried by the Responsible Person in Charge for After School Care for their enrolled centre to ensure that no children have been left uncollected.

Arrival at Centre

- The Responsible Person in Charge for After School Care will sign in each child at the service upon arrival, checking that they have signed in the same number of children they picked up.
- All children will be offered afternoon tea.

Evaluation and Review:

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	99	Children leaving the education and care service premises
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Other References:

OSHCQA Quality Areas 1-8
Handbook on Child Care Licensing

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

51 - VACATION CARE POLICY

Position Statement

Vacation Care allows young school age children to spend time with friends and to make new friends in a safe, supervised environment. Simple, clear guidelines that can be easily understood by all participants will ensure the safety and success of the Vacation Care program.

Objective

To provide a clear set of guidelines for team members planning vacation care programs.
To detail expectations of team members, parents and children during participation in the Vacation Care Program.

Implementation

- All plans for excursions need to be authorised by the Centre Director, Support Manager or Approved Provider (OPR008). Planning for excursions need to include all costs (transport, admission, refreshments, etc).
- No more than five excursions per fortnight will be planned except in exceptional circumstances with authorisation from the Support Manager.
- A Vacation Care Program will be available for parents to view at least 2 weeks prior to every Vacation outlining excursion details and costs.
- All Vacation Care will be pre-booked and paid for in full prior to commencement of the Vacation.
- No refunds will be made for absences in accordance with our centre fee schedule. All activities are staffed and paid for in advance according to the number of children booked and therefore refunds will not apply.
- All permission forms will be kept together and handed to the Director before the excursion.
- All excursions will be undertaken subject to having sufficient children booked. Where an excursion does not go ahead due to insufficient numbers, the excursion cost will be refunded to parents in full.
- All excursions will be subject to appropriate weather conditions. Where poor weather is responsible for cancellation of an excursion, and the excursion costs can be recovered from the venue, these costs will be refunded to families.
- All excursions will meet state requirements for child/adult ratios. No excursion is to leave the Centre if team member-to-child ratios are compromised.
- All parents need to authorise their child's participation in the excursion in writing.
- Each team member will have a roll with details for the children they are responsible for on the day and the children will be marked off against this roll at specified times during the excursion.
- The Educational Leader (person responsible for the excursion) in charge of the excursion will carry a mobile phone with them during the time they are away from the centre. The mobile phone must be charged and call access available. Also the location of an alternate telephone should be known. Eg. at excursion premises.
- The Educational Leader in charge of the excursion will carry a First Aid kit with them during the time they are away from the centre.
- Educational Leaders are to take the overall responsibility for ensuring that no one is left behind.
- A total head count is to be carried out by the Educational Leader in charge of the excursion at departure times and specified times throughout the day.
- The location of an excursion should be considered carefully. It is not appropriate for an excursion to be held in a busy area (ie theme park) which makes it difficult to manage and monitor the group of children.
- Excursions should not be held in high risk areas particularly not near water. Additional assistance and further clarification on suitable locations can be sought from the Support Manager.

Behavioural expectations:

Simple, clear guidelines for behaviour will be written for and discussed with children participating in the Vacation Care program. We encourage self discipline and respect for others. It is expected that all participants will be aware of the expectations and will respect the rules and each other. Vacation Care is intended to be a fun environment and to achieve this; guidelines for behaviour will be explained to all program participants.

- It is expected that all children will comply with the guidelines for excursions and that parents will support this policy.
- Consequences will apply for unacceptable behaviour while on excursions. Consequences may include being left in care at the child care service, or possible exclusion from the Vacation care program.
- Where consequences for unacceptable behaviour need to be implemented, the child will have the reasons explained to them, and the parents will be advised in writing of the reasons for exclusion.

Evaluation and Review:

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/04/2026

Reference:

NQS

QA1	1.1.1	Curriculum decision making contributes to each child's learning and development outcomes in relation to their identity, connection with community, wellbeing, confidence as learners and effectiveness as communicators.
	1.1.2	Each child's current knowledge, strengths, ideas, culture, abilities and interests are the foundation of the program.
	1.1.3	All aspects of the program, including routines, are organised in ways that maximise opportunities for each child's learning.
	1.3.3	Families are informed about the program and their child's progress.
	1.2.3	Each child's agency is promoted, enabling them to make choices and decisions that influence events and their world.
	1.3.1	Each child's learning and development is assessed or evaluated as part of an ongoing cycle of observation, analysing learning, documentation, planning, implementation and reflection.
	1.2.2	Educators respond to children's ideas and play and extend children's learning through open-ended questions, interactions and feedback.
	1.3.2	Critical reflection on children's learning and development, both as individuals and in groups, drives program planning and implementation.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	73	Educational programs
	74	Information about the educational program to be kept available

	75	<i>Information about educational program to be given to parents</i>
	76	<i>Documenting of child assessments or evaluations for delivery of educational program</i>

EYLF

LO1 – LO5	<i>All Learning Outcomes under the Early Years Learning Framework will be addressed through our Policy and practices.</i>
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Other References:

***Handbook on Child Care Licensing
OSHCQA Quality Areas 1-8***

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

52 - EMPLOYMENT POLICY

Position Statement

All team members need to know what the company's expectations of them are and where they can find the relevant information relating to their employment.

Objective

To set down clear guidelines so that prospective and existing team members know what is expected of all parties in regard to employment.

Implementation

- Employment of all team members will meet regulatory and industrial requirements for their role.
- Team members will work according to all conditions as set out in the Staff Handbook. Rostered hours will be in accordance with the relevant award. Should overtime be required by any team member or the Director, management must approve the overtime prior to it being worked.
- All new employees have a Probationary period of six months. During or on completion of the probation period the team member's performance will be evaluated and a decision made with regard to any further requirements for additional training and/or their ongoing employment with the company.
- All employees will read and familiarise themselves with the Staff Handbook, centre policies and procedures and orientation email.
- The Educator and Staff Orientation Checklist will be completed with the new employee and signed by both the new employee and Centre Director. All new employee's will be allocated an orientation buddy. The buddy is a room leader or experience educators of the service to assist in training and orientating the new team member. The completion of the Buddy Checklist is required to be completed by the new team member and buddy. Signed off by Centre Director to ensure new staff member has the knowledge and skills to perform role and duties.
- The Educator/ Staff Members Orientation Checklist and Buddy Checklist are to be scan and stored in OWNA > Qualifications and Certification > Type = "Other" > Select "Buddy Checklist" or "Orientation Checklist". Add PDF of checklist.

Manage Staff's Documents/Training

Home / Staff / Manage Staff's Documents/Training

Note: These documents are private to the Staff Member and Admins unless specified below.

Staff*	
Title*	
Type	Other v
Training/Courses/Program (if any)	-select- -select- 0. Buddy Checklist 0. Orientation Checklist 1. Staff Record
Expiry Date (if any)	

- Team members employed under casual conditions will be offered shifts according to the requirements of their specific area. Should the situation arise whereby insufficient work is available to sustain the employment of a casual team member, said team member's employment will be terminated as per award requirements.
- As per award conditions permanent employment status can be altered should management decide on restructuring of individual rooms, to cater for fluctuating attendance patterns; or centre wide, to cater for economic and/or child attendance trends; or for administrative team members, where changing workloads or infrastructural changes dictate. Should this happen all permanent entitlements will be

paid out up to the date of the change, with restructured employment conditions taking effect from this date forward. Any such changes will be done in accordance with the relevant awards.

- Leave requests will be accommodated wherever possible, however final approval will be dependent upon the operational requirements of the company. Staff will plan to take Annual leave during school term holiday periods and low occupancy periods. (See employment conditions in Staff Handbook). Staff leave requests forms should be submitted to the Centre Director as early as possible and greater than 2 weeks prior. Please note submitting a staff leave form does not result in automatic approval and is subject to service requirements and needs. Further, leave requests are **NOT** approved on a first applied basis, other factors are taken into consideration for the smooth operation of the service.
- Team members are expected to present a positive reflection of the centre and the company to the community as a whole. Any concerns that team members have regarding their area is to be dealt with according to the grievance procedure included in the staff handbook.
- Team members are expected to handle any sensitive information regarding families, children, colleagues and the organisation confidentiality. Any breaches of confidentiality will result in mediation and/or disciplinary processes being instigated.
- Formal team member evaluations will take place at least every 12 monthly or as required by management. Regular check-ins meetings will be held throughout the year with employee.
- If whenever returning late from leave, absent or unable to fulfil rostered hours; staff member is required to contact the Centre Director and/or Responsible Person in charge of service at the service to advise that they are unable to attend work and provide reason for non-attendance. Advance notice needs to be provided to ensure the Centre can accommodate the children education and care requirements. The absent employee must contact Centre Director or person on duty via telephone and not by email or text. Failure to do the above will be a serious breach of employment terms by the employee.
- Nominated supervisor is required to complete register of prohibited & suspected educators before employment. Use the National Quality Agenda IT System (NQA ITS) to perform a name and date of birth search for all potential and current employees. This search and result is documented on the Staff Record form.
- **National Early Childhood Worker Register (Worker Register)**
 - As part of national reforms to strengthen child safety across early childhood education and care services, a new National Early Childhood Worker Register (Worker Register) will commence on 27 February 2026.
 - The Worker Register has been established under the Education and Care Services National Law (National Law) and Education and Care Services National Regulations (National Regulations). It provides Regulatory Authorities with clearer visibility of who is working in services and where, strengthening child safety oversight nationally.
 - From 27 February 2026, services must:
 - enter information for all their current workers into the Worker Register
 - keep information accurate and up to date
 - record all data through the National Quality Agenda IT System (NQA ITS).
 - The requirements apply to all staff working in approved services under the National Quality Framework (NQF). This includes:
 - educators
 - volunteers
 - students
 - non-educator staff such as cooks, drivers and administration staff
 - teachers (regardless of whether they are registered or accredited with a teacher registration body)
 - nominated supervisors, coordinators, and family day care educator assistants

- temporary and agency or labour hire staff.
 - Service must:
 - enter all current staffing information within one month (following commencement of the Worker Register on 27 February 2026)
 - update worker details within 14 days when someone starts, leaves, or their details change
 - ensure that worker records meet the requirements set out in the National Law and Regulations.
 - No longer required – requirement ended 27 March 2026. *New employees are added to the **Early Childhood Workforce Register** in Victoria via the "Arrival" system, the Nominated Supervisor or Approved Provider is required to ensure the accuracy of staff records, including conducting necessary checks against prohibited persons. The employee is required to be added to Arrival when employment is terminated or volunteer period ends.* Please note on the introduction of the “**National Early Childhood Worker Register**” **National Early Childhood Worker Register (Worker Register)** the **Victorian Early Childhood Workforce Register** is no longer needed to be updated. Per the <https://www.vic.gov.au/early-childhood-workforce-register>
 - **Mandatory National Child Safety Training and Protecting Children – Mandatory Reporting and Other Obligations Early Childhood eLearn module**
 - Mandatory National Child Safety Training fits with the existing Protecting Children – Mandatory Reporting and Other Obligations Early Childhood eLearn module.
 - Both trainings cover important child safety content, but they serve slightly different purposes.
 - The **Protecting Children – Mandatory Reporting and Other Obligations Early Childhood eLearn** module (via the Vic Gov website) focuses on Victorian-specific requirements, including:
 - ❖ mandatory reporting obligations
 - ❖ early childhood–specific scenarios
 - ❖ state-based legislative responsibilities
 - ❖ This training remains really important and it's great to hear your team completes it annually and receives certificates
(Completed by all staff members of the service.)
 - The **Mandatory National Child Safety Training (via Geccko)** has been introduced to meet new national child safety requirements. It focuses more broadly on:
 - ❖ national child safety principles
 - ❖ child-safe practices across services
 - ❖ workforce responsibilities under the National Quality Framework
- The following people involved in ECEC must complete mandatory national child safety training:
- persons with management or control
 - nominated supervisors
 - persons in day-to-day charge
 - other staff, volunteers and students.
- The Victorian eLearning module does not replace the new national training. Even if staff complete the Vic Gov module each year, they will still need to complete the **Mandatory National Child Safety Training via Geccko** to meet the new requirements.
 - There will be Foundation and Advanced training modules. The Foundation training will be available from 27 February 2026. Advanced modules will be available in July 2026.

- **Personal Devices, Mobile Phones and Smart Watches Restrictions. Please refer to the following policies and procedures for guidance on this topic:**
 - 12A - Child Safe Standards Policy
 - 12B - Child Safety, Protection & Wellbeing Policy
 - 48 – Excursion Policy
 - 62 - Internet, Email and Social Networking Policy
 - 84 – Record Keeping and Retention Policy
 - 100 – National Model Code Policy
 - 101 – Nappy Change and Toilet Policy

Restricted Personal Electric Devices under the National Model Code are:

- personal devices that can take images or videos; and
- personal storage and file transfer media.

Examples of restricted Personal Devices:

- phones
- tablets
- digital cameras
- smart watches
- wearables, such as camera glasses
- SD cards
- USB drive
- hard drives

In summary:

- Only service-issued electronic devices are to be used when taking images or videos of children while providing education and care.
- Restricted Personal Devices such as phones, tablets, digital cameras, smart watches, wearables (such as camera glasses), SD cards, USB drive, hard drives should not be in the possession of any person or used while they are providing education and care and working directly with children.
- These personal devices are able to take images, videos, recordings, transfer information and data and therefore are required not to be used or in possession while providing education and care and working directly with children. Further, smart watches are an unnecessary distraction for active supervision and engagement.

Restrictions on use of personal devices apply to any person who is providing education and care and working directly with children. For example:

- teachers and educators, including casual and agency staff
- students attending the service as part of a practicum and representatives of tertiary providers who attend the service to assess students
- volunteers, including parent volunteers
- any third parties delivering programs or excursion activities to children in a service, whether paid or unpaid
- allied health and inclusion professionals attending a service to observe, assess or work with a child at the service
- mentors or coaches attending the service to support teachers or educators working with children or providing education and care
- preschool field officers
- primary school teachers attending a service as part of a school transition program.
- If a third-party professional attending the service and working directly with children (such as an allied health or inclusion professional) needs to use a device (for example, to undertake an assessment or take notes) they can use a device that is:
 - issued by their business or institution; and
 - used only for work purposes (and not personal use).
 (please see Technology & Digital Use Policy for detailed guidance)

Employees using personal devices to Access Arrival or other Service Software

Employee cannot use personal devices for multi-factor authentication to access and use Arrival while providing education and care and working directly with children. If not working directly with children, devices can be used. In non-child environments such as the service office.

Mobile phones are required to be turned off or kept on silent while the team member is working to prevent unnecessary noise.

Centre Directors and/or Responsible Person in Charge of Service are permitted to use mobile phones to perform their management and responsible person in charge duties in the Service's office (and non-child environments) and while not providing education and care or working directly with children. Calls and use of mobile phones by Centre Director / Responsible Person are restricted to office and non-child environments to perform service work-related tasks.

Permission for use of mobile phone will be obtained from Centre Director or Responsible Person in Charge of Service to use phone for excursion or special event or purpose (see excursion policy for guidance). Personal Devices are only permitted to be used in emergency situations and should not be kept in person hand (i.e. store in bag).

There is no objection to team members using of mobile phones or smart watch while on a lunch break and before or after work while not providing education and care and working directly with children. These must be used in non-child environments areas (Eg. staff room). Your personal device, mobile phone and smart watch should be left in your bag / locker / storage cupboard during work shifts times and when you are educating and caring for children.

If you need to receive an urgent call that cannot be attended during your break or lunch; please provide the services telephone number and the service team will do their best to accommodate by taking a message or providing you time to take the call in private. If you are expecting this is needed please discuss with the Centre Director / responsible person in charge of service.

Failure to do so may mean that children are not adequately supervised and breach of service policies which may result in disciplinary action as per the Grounds for Instant Dismissal.

Exceptions to Restrictions

The National Model Code lists the following essential purposes for which the use or possession of a personal devices may be authorised where access does not impede the active supervision of children and Service phones not available:

- communication in an emergency situation to ensure safety (lost child, serious injury or accident)
- emergency communication during excursions and regular outings (for example, when groups of children and educators get split up)
- emergency communication when children are transported or travel on transport arranged by the service.
- in the case of a lockdown or evacuation of the service premises
- technology failure; temporary out of service-issued electronic devices.

The Victorian Regulatory Authority does not consider routine communication during bush, beach or other nature programs to be an essential purpose for which approved providers can authorise the possession and use of personal devices.

Stand Down

1.1 Ongoing work at our service is determined by our operational needs and like all businesses, we are influenced by many known, seasonal and operational factors including public holidays, competition and the reputation of our service however, our service could at any time, be impacted by unforeseen circumstances, outside our control, that result in a significant impact to our financial viability and/or our capacity to provide approved education and care. If such a situation occurs were, we are unable to:

- (a) continue to usefully employ you in your usual classification of work, or
- (b) redeploy you to some other position or activities within our service for which you have the necessary skills and training, or
- (c) redeploy you to another service or location.

we reserve the right to stand you down from your employment without pay until further notice.

1.2 During any stand down period:

- (a) You will be treated for all purposes (other than payment of wages) as having continuity of service and will continue to accumulate entitlements based on the hours you worked before your stand down came into effect.
- (b) Subject to your available credit and approval, you may take paid annual leave during your stand down period.
- (c) Any national or regionally applicable public holiday that falls on a day that you would usually have worked but for the stand down will be paid.
- (d) You may take on other temporary work if you wish or resign from your employment.
- (e) Subject to operational needs, we may recall you to some or all of the hours of work you performed prior to the stand down. Refusal of any such shifts without a valid reason may lead to the termination of your employment.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies. Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

Policy Handbook – P004. 7.2026

Reference:**NQS**

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.

QA7	7.2.3	Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.
	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Handbook on Child Care Licensing
QIAS Principle 7.2, 7.4
Anti-discrimination Act
Equal Employment Opportunity Act
Immigration Law
Privacy Act 1988
Sexual Harassment Act
Workplace Relations Act
Industrial Relations Regulation 1990

Other Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)
Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)
Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)
Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

53 - TEAM MEMBER MATERNITY POLICY

Position Statement

In an industry employing a majority of female employees, it is not unusual to have team members become pregnant during the course of their employment. Pregnancy may place additional strain on the team member's energy reserves as they continue to perform duties as per their job description. Having a clear written policy that sets down management expectations of pregnant team members is important to ensure that no confusion exists between management and team members. A Maternity Information Pack will be provided to staff members who advise they are pregnant.

Reference Guide

Please refer to the Service Pregnancy and Parental Information Pack for further information to support this policy.

Objective

To put in place guidelines for management and team members to manage pregnancy and employment conditions during pregnancy and following confinement.

Eligibility

Employees who have had at least 12 months' continuous service with the Company prior to the date upon which they proceed upon Maternity leave are entitled to up to 52 weeks unpaid maternity leave upon production of certification as specified in the Children Services Award. Maternity leave of up to 52 weeks may be taken provided that such leave shall not extend beyond the child's 1st birthday.

As specified in the Award, this entitlement shall be reduced by any period of parental leave taken by the employee's spouse, apart from parental leave of up to one week at the time of confinement.

An employee may, in conjunction with maternity leave, take any annual leave or long service leave to which she is entitled and must not exceed 52 weeks in aggregate.

Requirements for leave

The employee shall produce the certification not less than 10 weeks prior to the presumed date of confinement.

The employee shall give not less than 4 weeks' notice in writing of the date upon which she proposes to commence maternity leave and state the period of leave to be taken.

The employee is also required to produce a statutory declaration stating particulars of any parental leave sought by her spouse as per the Award, and that for the period of maternity leave, she will not engage in any conduct inconsistent with her contract of employment.

Should the employee wish to continue to work within the 6 week period immediately prior to the expected date of confinement, she will be required to provide a medical certificate stating she is fit to work her normal duties on a weekly basis.

Requirements for return to work

The employee shall confirm her intention to return to work in writing not less than 4 weeks prior to the expiration of her maternity leave.

Should an employee wish to return to work after the birth of the child prior to the period stipulated in the relevant Award, she will be required to provide a medical certificate stating she is fit to work her normal duties. Due to rostering and backfilling employee plans the service requires notice before early return to work can be reviewed and we may reserve the right to maintain original date to return to work.

Should the position the employee held before the leave no longer exist, she shall be entitled to a position as nearly comparable in status and pay to that of her former position, as per the Award.

Infectious Disease Transmission

Whilst working at a childcare facility pregnant team members are at risk of acquiring infectious diseases that may cause foetal infection.

Pregnant team members will be provided with an information pack containing information about each of the relevant diseases and disease-specific strategies to manage the risks. Team members will be required to sign a declaration that they have been advised of these risks and strategies.

Pregnant team members are encouraged to seek advice from a medical practitioner with regard to the risk of acquiring infectious diseases that may impact on pregnancy.

In the case of a risk to the health or safety of the employee or her unborn child, where possible, the employee's working conditions or hours of work will be adjusted to avoid exposure to the risk, or the employee may be transferred to a 'safe' job. If this is not practicable, the employee may be required to take leave for such period as is certified necessary by a registered medical practitioner.

Such leave shall be treated as maternity leave or available paid sick leave, for as long as a doctor certifies it is necessary to avoid exposure to risk.

Conclusion

This policy by no means fully encompasses all the Awards. Employees should read the relevant Award for further clarification of situations, terms and conditions not mentioned in this policy.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 17/12/2025

Reference:

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Other References:

Handbook on Child Care Licensing

QIAS Principle 5.5, 6.6, 7.2

Anti-discrimination Act

Equal Employment Opportunity Act

Industrial Relations Regulation 1990

Sexual Harassment Act

Workplace Relations Act

Public Health and Wellbeing Act 2008

NSW Industrial Relations Act 1996

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

54 - TEAM MEMBER GRIEVANCES

Position Statement

A team member Grievance Policy helps to ensure that all team members are treated fairly and equally. The policy below will be used in an effort to settle disputes, before formal procedures for resolving industrial dispute are implemented. In the event of a workplace dispute arising, the procedures detailed below will be followed.

Objective

To enhance communication and cooperation between parties with compromise and mutually satisfactory solutions the desired outcome.

Procedure

Where a dispute arises with respect to work related matters there is an expectation that the team members involved will work towards a mutual resolution. It is expected that the parties involved, at a minimum, follow communication protocols, attempt to resolve the issue with the team member/s directly involved, and/or feedback from their supervisor.

Team members should follow communication protocols and review periodically to ensure everyone has common agreement on how to have professional and effective communication with one another.

Educational leaders / team leaders must give educators consistent clear feedback on a regular basis to minimise grievances arising. Educators need to acknowledge issues raised and address the issue in a professional and timely manner.

Grievance Process: The Two Working Day Rule

The process is based on the values of trust, responsibility and respect. The idea is that whenever someone has an issue with another's behaviour, the **TWO WORKING DAY RULE** applies. This rule requires every member of staff to either confront the behaviour or make mention of an intention to discuss the issue within two working days. If this does not occur, the understanding is that the individual has chosen not to address the issue and **MUST** let it go. This does not imply that the matter will be fully resolved within two days, only that the matter has been raised and all parties are aware of the requirement to further address a issue.

Feedback to another may occur with the support of a Director who may be used as a coach, support person and witness, but who must not speak for the person with the issue. This person may provide documentation for the meetings if this is deemed necessary.

This strategy would be used when a person felt tentative about raising the issue. In healthy cultures it would rarely be necessary.

Should a behaviour affect more than one, those affected should take responsibility for addressing the issue.

It is recommended that feedback be planned and a script developed to prevent criticism, blame or judgement in the feedback process. Professional development may be required to support this.

Implementation of this process requires a careful introduction and small group conversation. The key to success is that people understand they have permission from one another to take such action because it is in the interest of workplace harmony and sound relationships.

* * *

The employee and his/her supervisor will meet and attempt to resolve the grievance using non adversarial techniques in the following order:-

- Mediation
- Negotiated agreements
- Partnering with stakeholders

At the first meeting the parties will sit down and state their concerns in an attempt at resolution. The issues need to be recorded and worked through to ensure each party is clear. If possible a plan of action will be mapped out and a review date set down at the time.

Should the matter not be resolved through the initial process, the parties shall arrange discussions between the employee and a trained member of the management team in an attempt to mediate the issue. This is not a legal proceeding but a structured discussion to help both parties explain, clarify and narrow their differences through informal, confidential conference in the hope that a resolution will be reached. The mediation will be documented to protect the rights of all parties.

If the matter is still not resolved, a discussion will be held between employer representatives and the employee representative in a continued effort to reach a mutually satisfying resolution.

Educators understanding our communication plans and strategies will proactively assist in managing team member grievances.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 19/3/2026

Reference:

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

QA7	7.2.3	<i>Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

CCB – Federal Funding Guidelines

Handbook on Child Care Licensing

Centre Support Policy Reference Guide

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

55 - STUDENT AND VOLUNTEER POLICY

Position Statement

We recognize that students and volunteers are our future childcare professionals. As part of our commitment to the development of qualified team members in the childcare sector, Management welcomes the involvement of students and volunteers in our services. We believe in the importance of supporting and mentoring students and volunteers to develop the high standards we expect of our centre team.

Objective

To support students and volunteers as they develop appropriate practices and gain the necessary experience required to work within the childcare industry.

Related Policies

8 – Families and Visitors Code of Conduct
12A - Child Safe Standards Policy
12B - Child Safety, Protection & Wellbeing Policy
62 - Internet, Email and Social Networking Policy
96 - Technology and Digital Use Policy
100 – National Model Code Policy

Implementation

- Students and volunteers are to be organised with and approved by the Centre Director.
- Each Student and Volunteer will complete a “Student & Volunteer Orientation Checklist”.
- Students and volunteers are not team members under the service approval (‘licence’) requirements and as such are not permitted to supervise children alone.
- Students may be required, as part of their study, to observe and plan activities for children during their time at our centre. Written parental permission is required for any such involvement. No personal or identifying information regarding a child or their family is to be recorded by a student or volunteer.
- Students and Volunteers will be accountable to the Centre Director and/or Responsible Person. Should the performance of any student or volunteer prove to be unsatisfactory, the Director will contact the student’s supervisor or speak to the volunteer personally, to discuss whether the placement should continue.
- Students are required to provide written notice from their RTO or university if they require planning time as part of their placement time at the service.
- As a condition of their participation at the service, students and volunteers agree to abide by centre policies and procedures and by the directions of their supervising Room Leader.
- It is a requirement for placement in our service that all volunteers and students meet the necessary eligibility requirements as per state regulations to work within a child care environment (ie; Qld – current suitability card, NSW & VIC – working with children check, SA, & WA – current police check).
- Volunteers and students are required to abide by the Child Safe Standards and the service policies and procedures; 12A – Child Safe Standards Policy and 12B – Child Safety, Protection & Wellbeing Policy.
- Students should be encouraged to share their ideas and any new developments within the early childhood industry with centre team members.
- Students have a thorough orientation to ensure understanding of the services policies and procedures, supervision and their roles within our service. The completion of the orientation checklist should be completed and signed by student and Centre Director.

- It is recommended that students and volunteers not be provided security codes to buildings and gates as the codes can not be easily changed upon finishing at the service.
- Students are to complete their welcome sign in accordance with centre guidelines. The welcome sign is to be placed at the entrance of the door (not on windows). Also it is recommended that an email be sent by Educational Leader or Centre Director to parents to introduce student to the room.
- **Mobile Phones and Smart Watches Used by Educators, Staff, Student and Volunteers**
Only service-issued electronic devices are to be used when taking images or videos of children while providing education and care.

Restricted Personal Devices such as phones, tablets, digital cameras, smart watches, wearables (such as camera glasses), SD cards, USB drive, hard drives should not be in the possession of any person or used while they are providing education and care and working directly with children.

These personal devices are able to take images, videos, recordings, transfer information and data and therefore are required not to be used or in possession while providing education and care and working directly with children. Further, smart watches are an unnecessary distraction for active supervision and engagement.

Mobile phones are required to be turned off or kept on silent while the team member is working to prevent unnecessary noise.

If you need to receive an urgent call that cannot be attended during your break or lunch; please provide the services telephone number and the service team will do their best to accommodate by taking a message or providing you time to take the call in private. If you are expecting this is needed please discuss with you room leader, Centre Director / responsible person in charge of service.

There is no objection to team members use of mobile phones or smart watch while on a lunch break and before or after work in non-child environments. ie Staff room.

Failure to do so may mean that children are not adequately supervised which may result in disciplinary action as per the Grounds for Instant Dismissal from placement or employment.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 19/11/2025

Next Policy Review: 15/11/2026

Reference:

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

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QA7	7.2.3	<i>Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Other References:

Public Health and Wellbeing Act 2008

Occupational Health and Safety Act 2004

2003 Volunteering Australia

Legislation (Prior):

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

56 - TEAM MEMBER SELF PROTECTIVE BEHAVIOUR POLICY

Position Statement

During the normal working day, situations can arise where a team member's actions towards children or other team members may be misconstrued as inappropriate. It is important that team members have an understanding of self protective behaviours and company expectations for behaviour that will reduce the possibility of any opportunity for misinterpretation of team member interactions between colleagues and/or children in their care.

Related Policies:

- 9 – Behaviour Guidance Policy
- 12A- Child Safe Standards Policy
- 12B- Child Safety, Protection & Wellbeing Policy
- 72 - Interaction with Children Policy

Objective

This policy intends to step out to team members appropriate self-protective behaviours and company expectations of interactions and has been developed to assist team members to avoid wherever possible, allegations of inappropriate behaviour which could lead to litigation, suspension, or dismissal.

Implementation

Team members will, wherever practicable, not place themselves into a situation where they are alone with a child, especially where other team members are not able to verify the details of any interaction.

Team members should be aware of their colleague's movements and bring to the notice of the Centre Director or Management any team members who either show a lack of understanding or a wilful disregard of this policy.

Team members working alone with a group of children should do so within sight of team members, wherever practicable, and where not possible, team members should not be alone with only one child at any time.

Should team members be required to take a child on their own they must inform other team members of what they are doing. This practice is strongly discouraged unless there is an urgent requirement for the child to be taken to another part of the centre on their own. Children should be moved in small groups wherever practicable.

Specific Procedures

- Team members will not take any photos of children without express written permission of parents or guardians. Your Centre Director will advise if there is no permission.
- Team members will not take photos of other team members without the permission of the team members. Team member are to advise if permission is not granted for photos to be taken of them.
- Team members will not undress or touch any genital areas of children in isolation from other team members. It is satisfactory to conduct nappy changes and toileting of children where bathroom areas are viewable by other team members.
- Team members will not bath children in isolation from team members. It is satisfactory to conduct bathing where bathroom areas are viewable by other team members.
- Team members will not change any underwear of children without gloves.
- Team members will notify other team members should they be required to move a child to another part of the centre on their own as the situation permits. This practice is only to be used when other strategies are not appropriate. It is expected that, wherever

possible, team members will take a small group of children with them in an instance such as this.

- Team members are expressly forbidden to interact with children behind closed doors away from other team members.
- Team members will be mindful of appropriate touch with children. This includes not touching or fondling children on their chests, bottoms, genitals, or any other culturally inappropriate body parts.
- Team members will not kiss children, however, should a child wish to kiss a team member the team member is encouraged to use their own judgment in receiving a kiss. Any child giving a team member a kiss should be directed to kissing on the cheeks not mouth.
- Team members should be mindful that children sitting on adult laps can create a situation that is easily misinterpreted. It is preferable to have children stand beside or sit next to the team member when comforting or closeness is required.
- Team members may be required to defend themselves against aggressive behaviour from children. In this situation, Educators are encouraged to move themselves and other children away. If this is not practical, Educators may need to restrain a child from harming others or self. Educators should call for assistance during incident. Refer to Behaviour Guidance Policy and Exclusion Policy for further direction.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/8/2025

Reference:

NQS

QA5	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
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LO2	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.</i>
	<i>Children develop knowledgeable and confident self identities.</i>
	<i>Children learn to interact in relation to others with care, empathy and respect.</i>
	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.</i>
	<i>Children respond to diversity with respect.</i>
	<i>Children become aware of fairness.</i>
	<i>Children become socially responsible and show respect for the environment.</i>

57 - EXPENSE AND TRAVEL POLICY

Position Statement

The company employs a cluster strategy to minimise overhead costs. The broad geographic coverage of centres and logistical isolation from the Support team means that prudent management of these facilities requires team member travel.

Objective

To ensure efficient business practices, this policy seeks to provide criteria for the appropriate use of expenditure and travel. It also provides approval mechanisms to authorise such expenditure and travel, in addition to outlining guidelines to minimise expenses.

Implementation

All expenditure claims must be accompanied by a receipt/tax invoice. Failure to present appropriate documentation may result in non-reimbursement.

Operational Expenditure

Operational expenditure refers to the day-to-day expenses on items like consumables, which are essential for the ongoing operation of the centres.

All consumables in excess of the operational budget are to be approved by Management.

Capital Expenditure

All capital expenditure will require initial approval from Management and will be assessed with regards to legislative constraints, best practice guidelines and priority of need. Management will provide final approval for any such expenses.

Detailed guidelines are as follows:

- Centre Directors will be required to complete the Capital Expenditure Request Form or email.
- The form will be forwarded to Management for approval.
- Once final approval has been received, Management will determine timelines for implementation in consultation with Centre Directors.

Employment of Team members

Prior to the employment of any team member, written approval from Management must be obtained. Casual team members employed at short notice on a casual basis to meet regulatory requirements at a childcare centre in the absence of a suitably qualified person will be the only exception.

It is recognised that team member's wages are the single biggest and most critical expense within the child-care sector. Prior to the employment of any centre based or administrative team members, the following procedure will be followed:

Detailed guidelines are as follows:

- Prior to any action, Management must approve the need for additional team members.
- All positions must be advertised in an appropriate publication and approved by Management.
- All suitable applicants are to be interviewed; original qualifications, police checks, WWCC, first aid certificates, etc. to be sighted, and referee checks to be completed.

- Upon selection of a suitable applicant, the Employee Information Form is to be completed and forwarded to Management for approval.
- Once approval has been received, Management will arrange a letter of offer for the applicant.

Travel

It is recognised that due to the geographical location of childcare centres, essential travel will be required for the successful operation of the company.

All travel and accommodation expenses must be pre-approved in advance by Management.

Air Travel

- Air travel should only be used where it makes economic sense.
- The lowest cost airline should be used in all cases.
- Air travel is by economy class for all domestic flights.
- If a lower cost flight is available within one hour of the preferred departure time, this flight should be taken.
- Frequent flyer points gained from work travel may be used for personal use, however the cost of membership of such programs is at the individual's cost.
- As most flight changes incur additional charges and sometimes total loss of fares, all efforts should be made to adhere to the original booking. Should a new fare be required, a new Travel Booking Request is required to be approved.
- If air travel is required, a Travel Booking Request Form should be completed and forwarded to Management.

Accommodation

- Where possible, travel should be organised to minimise the need for overnight and weekend accommodation.
- Incidental "living" costs such as meals should be maintained within reasonable limits (\$50 per night) whilst ancillary expenses (such as in-house movies, mini bars, etc) are at your own expense.
- If accommodation is required, a Travel Booking Request Form should be completed and forwarded to Management for approval.

Daily Travel Allowance Claims

- The company will only reimburse incidental expenses that are incurred in the course of business travel requiring you to be away overnight (such as meals, telephone, internet, etc) in accordance with the other terms of this policy.

Taxi and Car Hire

- It is expected that Taxis or Hire cars will be required only when travelling outside the usual work environment.
- Rental cars will only be used where it is more economical than using taxis.
- A small to medium car should be hired where possible.
- In consideration of the significant premium charged for petrol by rental companies, the car should be re-filled before returning it to the depot.
- Travel between an employee's normal place of work and home will not be reimbursed.
- All travel bookings must be accompanied by a Travel Booking Request Form and forwarded to Management for approval.

Club and Professional Memberships

- Club and professional memberships are a personal expense and are not reimbursed by the company.

Phones

- Home telephones, fax and internet are a personal expense. Rental and personal call costs are an individual responsibility, however business calls (with appropriate documentation) will be reimbursed.
- Any mobile phones issued to company team members are for business use only.

Team Member Entertainment

- It is recognised that certain team social functions, to recognise achievements or milestones, are important to create a positive environment, and these will be reimbursed, provided they have been pre-approved and are reasonable and not excessive.
- All events will be assessed on an individual basis and will require specific justification.
- Casual lunches and dinners between members of team members will not be reimbursed.
- All team member entertainment requests must be sent to Management for approval.

Marketing and Promotional Events

Marketing forms a crucial element to the success of our company and should form part of a comprehensive marketing plan either for development centres or ongoing for operational centres.

Prior to undertaking any marketing initiative, a detailed marketing plan and budget should be prepared and submitted to Management for approval.

All marketing programs and promotional events must be:

- reviewed to ensure that costs are minimised whilst outcomes are maximised and all programs should be comprehensive and clearly identify the rationale for the program, budgets and anticipated outcomes.
- should be developed in consultation with the Support Manager.
- must be pre-approved by Management.

General Expenditure

- All expenditure must be forwarded to Management for approval.
- Specialist publications, which are fundamental to industry analysis, may be exempted however generalist publications (eg The Age, Herald Sun, Financial Review, Business Review Weekly) are considered personal expenditure.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy reviewed: 16/6/2026

Reference:

NQS

QA4	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
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	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

58 - TEAM MEMBER TRAINING POLICY

Position Statement

The company has a commitment to life-long learning and professionalism within the early learning and education field. All team members are expected as part of their employment to complete ongoing professional development, including on the job training and personal development as well as attendance at professional development seminars, courses and team member meetings etc outside of work hours. The company promotes both on the job and external training, however, believes it is the responsibility of the team member and the company to maintain professional competency.

Objective

To set guidelines for the delivery of team member training.

Implementation

Training for new team members commences immediately upon being offered a position within our centre. Team members **MUST** participate in the centre induction process which details their responsibilities with regards to the state Regulations and Act, Child Protection, ACECQ requirements, food safety requirements as well as company policies and procedures. Team members are required to be familiar with the abovementioned legislation and company policies, procedures and job descriptions. Team members will be assigned a mentor to assist them through the induction process.

The company will fund nominated and specified training opportunities for permanent and permanent part time team members, who have completed their 6 month probation period and have signed their full employment letter of appointment. This condition excludes team members who have tendered their resignation and are working out any termination periods.

Training Costs Agreement and Authorisation for Wages Deduction

Completion of training courses is an enduring benefit of life-long learning provided to employees. Training Costs Agreement and Authorisation for Wages Deduction Form will be required to be signed by employee undertaking of professional development course stipulated in the training cost agreement. This agreement is between the employee and employer to establish a contractual commitment, define the clawback provisions and provide employer authorisation to deduct training costs incurred. The repayment agreement is necessary as it is a pre-estimate of loss due to the costs incurred for fees, resources, travel expenses and employee wages to attend the training course. Further, employee attendance to a training course is an employer investment to benefit the organisation; if the employee leaves before the clawback period a financial loss as been incurred and also the lost opportunity to other employees as also been incurred as another person was not able to attend.

Various training models will be available:

- Group training, where the entire team is encouraged to attend. This will be held after opening hours at an agreed upon location and may cater for team members from more than one centre.
- Planning days specifically for identified team members, eg. Directors or Team Leaders.
- Individual outsourced training eg. Gowrie training sessions. This may involve 1 specified team member/s who will attend the training and then be required to share their knowledge with the rest of the team at a team members meeting.
Funding for outsourced training organisations will require authorisation from management before booking. Please note training is very expensive and costs need to be managed in this regard.

All team members are reminded that the information, handouts and any other professional training resources that are given to team members during training remains the intellectual property of the company.

Training opportunities will be offered in 2 categories. Team members will be notified under which category the training will be held.

- Compulsory
- Voluntary

External Study through a Registered Training Organisation

Team members who are enrolled with an RTO in further study to advance their qualifications will provide to their centre Director a copy of their confirmation of enrolment which will be maintained within their personnel file at the centre.

At the beginning of each month the centre Director/Support Manager will hold discussions with team members as to their progress with their studies and offer support as required. At this time team members will provide to their centre Director confirmation via a Statement of Attainment as each unit of study is completed. *{In Qld this will be recorded on the Progression of Study Record (SP012) to meet with state regulatory requirements. Any team member who doesn't continue to progress through their studies as required for the position for which they are currently employed may place their continued employment in said position in jeopardy.}*

Once team members have attained a qualification level it is their responsibility to provide confirmation via two (2) certified copies of their qualification to the centre Director. One copy will be maintained on file in the team member's personnel file and the second copy is to be forwarded to the Payroll department.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy reviewed: 16/4/2026

Reference:

NQS

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.

QA7	7.2.3	Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.
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	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

59 - BABYSITTING POLICY

Position Statement

In a profession where the public perception of educators is that they are experts in the field of childcare, it is necessary to safeguard the health and reputation of our team members and the organisation by making clear to team members and parents the risks associated with employment of childcare centre personnel for babysitting duties.

Objective

To make clear to our centre team members and parents the company's position of strongly discouraging babysitting by our team for our parents and the reasons for that position.

Implementation

As an organisation we strongly discourage our team members from Babysitting for parents for a number of reasons which include:-

- Most team members are already working up to 38 hours at the centre with children and many team members are also studying part-time. For the sake of physical and mental health it is recommended that our team have time for relaxation
- Centre team members may be seen as experts in their field, but are trained to work in the child care environment where there are always two team members on duty. Team members who choose to babysit do not have the luxury of always having someone to assist if things go wrong. Furthermore, in the case of any allegations which may be made against the babysitter, there are no adult witnesses.
- The familiarity of a babysitting arrangement can lead to conflict of interest in relation to centre confidentiality.

For these reasons, our organisation will not sanction or support babysitting and do not recommend any of our centre team as babysitters.

If team members choose to work in their own time outside centre hours, our organisation cannot stop them, however this will be a personal arrangement between parents and team member.

No arrangements will be made with team members during centre hours. Should a parent require a team member to babysit, they will be required to obtain the relevant details directly from the team member and contact them outside centre hours to make any arrangements.

We regret that messages relating to babysitting cannot be taken or passed on through your centre.

Evaluation and Review:

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy reviewed: 16/9/2025

Reference:

NQS

QA4	4.2.1	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>
QA5	5.1.2	<i>The dignity and the rights of every child are maintained at all times</i>
QA6	6.1	<i>Respectful relationships with families are developed and maintained and families are supported in their parenting role.</i>
QA7	7.1	<i>Governance supports the operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

60 - MEDIA/PRESS POLICY

Position Statement

Increasingly, private and public companies attract scrutiny. Team members need to seek authorisation to discuss Centre or organisation matters.

Objective

To prevent unknowing leaks being made by centre team members in regard to Company business, and/or the possibility of inflammatory, or potentially litigious comments in regard to the Centre or families or team members involved.

Implementation

- When team members talk about their work to friends and relatives in relation to the centre/management, they need to keep confidentiality in mind.
- Team members will seek authorisation from Management, before discussing any matters with the press or media in relation to the Centre or their employment with the Approved Provider.
- Should a member of the press contact team members at our centre for comment, the standard response is to be “no comment” and notify the Support team of the call and the caller’s identity and contact details as soon as practically possible.
- If team members are unsure of anything in relation to this policy, they should contact their Support Manager.
- Any press releases must be vetted and approved in writing by Management.

Copies of any print media articles are to be kept on file at Head Office.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters).

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2028

Reference:

NQS

QA4	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
QA5	5.1.2	The dignity and the rights of every child are maintained at all times
QA6	6.1	Respectful relationships with families are developed and maintained and families are supported in their parenting role.
QA7	7.1	Governance supports the operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Policy Handbook – P004. 7.2026

Reg	181	<i>Confidentiality of records kept by approved provider</i>
	181-184	<i>Confidentiality and storage of records</i>

61 - FUNDRAISING POLICY

Position Statement

Should a Parent Committee wish to engage in Fundraising activities for the centre there should be guidelines in place to facilitate the appropriateness of these activities.

Objective

To ensure that any fundraising activities conducted in relation to the centre be in keeping with Government legislation and guidelines.

Implementation

- Incentive and promotional funds is not classified as fundraising unless the incentive and/or promotional activity performed is to raise funds for a charitable purpose.
- Should any parent and services committee at the centre wish to conduct fundraising activities they must follow these guidelines.
- Any money raised by a parent and service committee cannot be used to enhance the financial profitability of the company. All money raised must be used for a charitable purpose, advancement of education for the children attending the service or for the benefit of the community. Some ideas for which fundraising money could be used include: gift funds to a registered charity, shows, incursions or excursions for the children, in-house educational programs such as an egg hatching program, disco or BBQ which all the children and families could attend, sponsorship of a child through a recognized charity which the children in the centre could correspond with to learn about another culture, etc.
- Dependant upon the regulations in each state in some instances a parent committee may be required to obtain an 'Authority to Fundraise' from the relevant government department. Further information on the requirements can be obtained by contacting the relevant government department as listed below.
- Detailed records must be kept for all fundraising activities including the amount of money raised, the date it was raised, in what form the money is held and how and when it was used.
- Prior to commencing fundraising the Parent & Service Committee should decide on what they are fundraising for and make the purpose known whilst conducting the fundraising activity.
- If service does not have a Parent Committee service bank account cash should not be kept at the service due to the security risk. The cash should be banked in to the service bank account and advise Account Department of the nature of money. The Account Department will account for this funds as separate Fundraising moneys. When the Parent and Service Committee wishes to spend funds in accordance with the policy guidelines, request for transfer of funds needs to be made with associated tax invoice.
- No cash should be left at the service or keep by parent or staff member due to security risks associated with holding cash. Please bank funds immediately.
- Any money raised by a parent and service committee needs to be banked into a separate parent committee account with a minimum of two signatories. Should this not be practical due to parents not wishing to accept this responsibility, then it is acceptable for the money to be transferred to Australia Post money orders in the name of the centre who will in turn cash the money orders as requested.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy reviewed: 16/06/2026

Policy Handbook – P004. 7.2026

Reference:

NQF

QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Dept of Equity and Fair Trading (Qld)
Office of Fair Trading (Qld)
Dept of Justice (Vic)

Consumer & Business Affairs Victoria
Ministry of Fair Trading (WA)
Office of Fair Trading (NSW)

62 – INTERNET, EMAIL & SOCIAL NETWORKING POLICY

Position Statement

Internet, email, and social Networking correspondence is a permanent written record which may be read by persons other than the addressee and which could result in personal or Management liability. This policy sets out the security, administration and rules which will be observed when communicating electronically or using IT facilities.

Objective

To minimise potential damage to you, your colleagues, families or the company which may arise as a result of the misuse of Information Technology. Information Technology includes but limited to: phones, smartphones, computers, laptops, tablets, handheld devices, etc.

Who is affected by this policy?

Child

Staff

Educators

Family Members, Family Members Associates, Parents and Guardians

Responsible Persons, Authorised Nominees

Management

Related Policies

- 12A - Child Safe Standards Policy
- 12B - Child Safety, Protection & Wellbeing Policy
- 12C- Child Safeguarding: Look, Do & Tell Framework Policy
- 52 - Employment Policy
- 84 - Record Keeping and Retention Policy
- 96 -Technology & Digital Usage Policy
- 100 - National Model Code Policy

Implementation

The company holding the centre license is the owner of copyright in all email and social network messages created by authorised users of the IT facilities.

Security:

- From time to time, the contents and usage of email may be examined by management or by a third party.

Monitoring:

- Our computer network is a business and educational tool to be used primarily for business or educational purposes. You therefore have a responsibility to use these resources in an appropriate, professional and lawful manner.
- All messages on the centre's system will be treated as business or educational messages, which may be monitored. Accordingly, you should not expect that any information or document transmitted or stored on the centre computer network will be private.
- You should also be aware that management may monitor your use of the Internet both during work hours and outside of those hours. This includes the sites and content that you visit and the length of time you spend using the Internet.
- Emails may be archived by management as it considers appropriate.

Personal Use:

- Outside work hours, you are permitted to use the Internet to send and receive personal messages, provided that such use is kept to a minimum and does not interfere with centre business. Any such use of the internet or email for personal purposes outside work times is still subject to the same terms and conditions as otherwise described in this Policy.
- In the case of shared IT facilities, you are expected to respect the needs of your colleagues and use the Internet and email in a timely and efficient manner.
- Excessive or inappropriate use of IT facilities for personal reasons during work hours may lead to restrictions of usage rights being placed upon you.
- In addition, use of the technology system; and any information documents, files or other intellectual property on the technology system; for personal commercial or personal business is prohibited unless the written consent of the Managing Director (or equivalent) is first obtained for that specific purpose and use.

Content:

- Staff, Educators, Families, Associates of Families and/or the Approved Provider may be liable for what you say in an email or social network message. Email and social network messages is neither private nor secret. It may be easily copied, forwarded, saved, intercepted, archived and may be subject to discovery in litigation. The audience of an inappropriate comment in an email or message may be unexpected and extremely widespread.
- Employees, Families, Associates of Families must not use the information technology devices for the following purposes:
 - post materials that is offensive, defamatory, threatening, harassing, bullying, discriminatory or otherwise unlawful;
 - hold out to be a representative of the organisation or express views on behalf of the organisation;
 - use the organisation email or logo without permission;
 - list the organisation names (including abbreviation) on internet websites without permission from Approved Provider;
 - post employee names or families (parent or children) names to discuss private and confidential matters, abuse, vilify, harass, or discriminate;
 - disclose confidential, private or sensitive information;
 - publicise workplace disputes, conflicts, grievances;
 - damage the employment or enrolment relationship, the organisation reputation or commercial interests, or brings the organisation into disrepute;
 - to abuse, vilify, defame, harass or discriminate (by virtue of sex, race, religion, national origin or other);
 - to send or receive obscene or pornographic material;
 - to injure the reputation of the centre or in a manner that may cause embarrassment to the organisation;
 - to spam or mass mail or to send or receive a chain email;
 - to infringe on the copyright or other intellectual property rights of another person; or
 - to perform any unlawful or inappropriate act.

The above actions are considered to be serious breaches of the organisations policies.

Social Networking and Websites:

Social media activity include:

- Interacting with social networking sites, e. g. OWNA, Facebook, Twitter, LinkedIn, Yammer
- Interacting with video and photo sharing websites, e. g. Flickr, You Tube, Instagram, Pinterest
- Interacting with blogs, including corporate blogs and personal blogs
- Interacting with blogs hosted by media outlets, e. g. “comments” or “your say” feature on the age.com.au
- Micro-blogging, e.g. Twitter
- Interacting with wikis and online collaborations, e. g. Wikipedia
- Interacting with forums, discussion boards and groups, e.g. Facebook, Google groups, Whirlpool

Social media activity has risks for the organisation include:

- Posting or sharing information on social media and online generally is not like having a verbal conversation with a person or group of people. This equally applies to any posts you make regarding your work.
- “Conversations” or posts online are in electronic form and have potentially wider circulation than a personal discussion. The nature of social media platforms means that comments might easily be forwarded on to others, widening the audience for their publication. Even if you limit the privacy settings on your social media platform to your “friends” or “contacts”, your “friends” or “contacts” might include individual stakeholders.
- Further, social media platforms leave an often-permanent written record of statements and comments made by people. These can be read at any time in the future until they are taken down and because of the nature of the Internet, it can be difficult (if not impossible) to remove information.
- Considerable care should be used in using social networking sites and an awareness that making comments or conducting conversations that relate to the organisation can affect the reputation of the business.

A social networking website can be defined as a website used to socialise or communicate. Our stance on social networking policies is that they are for personal use only and should not be accessed while the staff member is at work.

Unacceptable use of social media

When using social media, you must not:

- Disclose or use information that is confidential to the organisation
- Engage in non-work-related use of social media during work hours
- Post anything in which this organisation or any other person has intellectual property rights
- Convey information that would allow a reasonable person to ascertain the work being performed for this organisation or the identity of a stakeholder and their relationship with the organisation.
- When using IT systems, use any other person's ID or logon details or otherwise impersonate any other person
- Disparage, criticise or show disrespect for any stakeholder or person (Stakeholder examples: Organisation, parent, children, suppliers, agencies, departments, etc)
- Discriminate, harass, bully or victimise any stakeholder or person

Other unacceptable use of social media:

- No communication with anyone (I.e. parents, educators, other people, etc) via messenger or private chat groups in relation to children enrolled in the service or the centre (ie use of their name, experiences, or any other general information). All communication between educator and family member should be undertaken via services email or curriculum system or the services social media accounts (ie not the personal accounts).
- It is recommended that friend requests not be issued or accepted by parents or family member of children without discussion and consent with each other. If a friend request is accepted by parent or family member (or vice versa) the employee (or ex-employee) should not mention the children who is enrolled in the service in anyway. This aims to protect the parent and child's privacy and also assist in maintaining professional relationships between employee and parents.
- In relation to the Service Social Media Pages (E.g. Facebook): Only authorised personnel of the service can share information about what happens at the centre, share photos taken at the centre, or on an excursion. Permission from children's parent/guardian must be sought before posting on social media. Content of the Facebook page will only include – children's learning, centre events, professional development, VDELYF, community events, reminders, government information.
- Parents and families should not share any photos or videos that include identifiable characteristics of other children, staff, visitors or families without getting the relevant consent first.

If you become aware of unacceptable use of social media as described above, you must notify the Centre Director or Approved Provider.

No information about what happens at the centre, should be posted on a social networking website, nor should any photos taken at the centre, or on an excursion, be put on a social networking website. Posting to the official organisation social networking websites will be completed by authorised personnel only. If a staff member, family or parent does put photos of a child or children enrolled at the centre on a social networking website, families will immediately be contacted. If possible, the social networking website will be contacted to delete the photos. ***The person who placed information will face an inquiry into their actions and possibly face termination of employment or termination of enrolment and/or face legal action.***

Please be aware that social networking websites are not a private means of communication but can be accessed by the public, therefore, it is important not to share private information about centre families or other staff members on social networking websites. Should you do so, the person will face an inquiry into the situation by the Director/Authorised Supervisor and any involved party and depending on the severity of the situation face possible termination of employment or termination of child enrolment.

Should harassment of any kind take place on a social networking site, such as, but not limited to, sexual or verbal harassment, the person will face an inquiry into their actions and depending on the severity of the situation face possible termination of employment or termination of enrolment.

Should a family member related to the centre harass a staff member via a social networking website, the Director/Authorised Supervisor will conduct an inquiry into their actions and depending on the severity of the situation face possible termination of enrolment.

Family grievance policy, team member grievance policy and Family and Visitor Code of Conduct Policy should be followed at all times.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2027

Reference:

NQS

QA4	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.1.2	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>
QA5	5.1.2	<i>The dignity and the rights of every child are maintained at all times</i>
QA6	6.1	<i>Respectful relationships with families are developed and maintained and families are supported in their parenting role.</i>
QA7	7.1	<i>Governance supports the operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	181	<i>Confidentiality of records kept by approved provider</i>
	181-184	<i>Confidentiality and storage of records</i>

63 - CENTRE VEHICLE & TRANSPORTATION POLICY

Position Statement

Procedures and policies for transporting children and adults are essential to keep everyone safe.

Objective

To ensure the safety of all children and adults while using public or private transport, including any vehicles owned by the service.

Implementation

The safety of each child and all employees is paramount. Transportation can pose additional risks for children depending on the mode of transport involved, how it's used and the way children move between the transport and service or other environment. The service we provide includes times when we transport children, or arrange transportation, between the service and another location, including their home, school or excursion destination. The Nominated Supervisor and educators will always follow service procedures to minimise this risk including those which follow.

The Nominated Supervisor will include the Transport Policy and Procedures in staff inductions and ensure all relevant staff receive practical training in relation to the requirements. Where children are regularly transported they will also diarise to rehearse the procedures at least once every 3 months, and maintain written records of the rehearsals and staff training.

Risk Assessments

Please refer to the Excursion Policy for further policies and procedures on completing risk assessment involving transportation of children.

The Nominated Supervisor or educators will always complete a risk assessment to identify, assess and remove or reduce risks the transport may pose to the safety, health and wellbeing of and each child before children are transported unless the arrangement is 'regular transportation' (ie transportation where the circumstances are substantially the same on each occasion) and a risk assessment has been completed within the last 12 months. The risk assessment will cover:

- the proposed route and duration of the transportation
- the proposed pick-up location and destination
- the means of transport
- any requirements for seatbelts or safety restraints under the relevant state/territory law (for regular transportation consider whether this needs to be assessed more often than every 12 months if child grows etc)
- any water hazards
- the number of adults and children involved in the transportation
- the number of educators or other responsible adults that is appropriate to provide supervision and whether any adults with specialised skills are required, given the risks involved
 - consider children's ages, whether or not they are mobile, and whether any have additional needs
- whether any items should be readily available during transportation (eg a mobile phone and list of emergency contact numbers for the children being transported)
- the process for entering and exiting the education and care service premises and the pick-up location or destination
- procedures for embarking and disembarking the transport, including how each child will be accounted for and also the conducting the vehicle check after children have disembarked.

The Nominated Supervisor will nominate, as part of the risk assessment control measures:

- the driver (if using vehicle owned or operated by service)
- the lead educator/supervisor responsible for ensuring an excursion runs smoothly and children are adequately supervised, or for supervising children during trips that are not excursions
- the checker responsible for checking vehicle at end of trip (may be same as lead educator/supervisor – a second person needs to perform this duty).

The Nominated Supervisor will update risk assessments for regular transportation obtain new authorisations from parents/guardians when circumstances that may affect transport arrangements change, including for example:

- weather conditions (summer versus winter, extreme weather events like heatwaves, floods and bushfires)
- changes in routes for example because of road works
- the numbers and vulnerabilities of children.

Authorisations for Transportation

This section of the Policy does not include transport of children on excursions as authorisation requirements for excursions are covered in the Excursion Policy.

Authorisation for a child to be transported must be given by a parent or other person named in the child's enrolment record as having authority to authorise transportation of a child unless the arrangement is 'regular transportation' and there's an authorisation which is less than 12 months old. The authorisation will include:

- the child's name
- the reason the child is to be transported
- if it's regular transportation, a description of when the child is to be transported
- if it's not regular transportation, the date the child will be transported
- a description of the proposed pick-up location and destination
- the means of transport
- the period of time during which the child is to be transported
- the anticipated number of children likely to be transported
- the anticipated number of staff members and any other adults who will accompany and supervise the children
- any requirements for seatbelts or safety restraints under the relevant state/territory law
- advice a risk assessment has been prepared and is available at the service
- advice written policies and procedures for transporting children are available at the service.

Transport Guidelines

This section and the rest of the Policy covers all situations where we transport or arrange transportation of children, including excursions. Where relevant, the guidelines and procedures in this Policy will also be used to educate children, families and the community about safely transporting children and related issues like road and pedestrian safety.

Drivers

The Nominated Supervisor will ensure persons are safe, responsible drivers before allowing them to transport children. The Nominated Supervisor will nominate the driver (if not using a commercial transport company) and ensure they are not included in ratios or responsible for supervising children

as they cannot provide adequate supervision while operating the vehicle. In addition (if not using a commercial transport company) they will:

- make sure there are relief drivers available to fill in for any regular drivers
- keep copies of licenses and driving records which are less than 12 months old for drivers they approve to transport children and make sure drivers:
 - are at least 18 years old and fully licensed (no L or P plate drivers)
 - have a suitable driving history eg statement of demerit points
 - have proof of valid insurance and registration
 - are familiar with the first aid kit contents
 - can operate the fire extinguisher if required
 - have a clear working with children check if required
 - understand they must always comply with the road rules (eg no speeding or touching mobile phones)
 - understand they are responsible for paying any fines they incur
 - understand they must have a zero blood alcohol level when driving children
 - understand they must report any driving convictions incurred since the Nominated Supervisor reviewed their driving record
 - understand in relation to a vehicle operated by the service they must:
 - report any damage or maintenance needs to the Nominated Supervisor
 - ensure there's enough fuel to complete the trip (eg refuel when the tank moves below half full) and never refuel when children are in the vehicle.

Where bus companies are hired to transport children the Nominated Supervisor will discuss the company's policy on driver qualifications, driving history and maintenance procedures etc. to make sure they have reliable and consistent safety processes in place.

Safety Restraints

Seatbelts and restraints systems will be used as outlined in this policy which is based on the National Road Rules. We note the Australian Design Rules set out vehicle safety requirements including requirements for seatbelts and child restraint anchorage points.

Educators will assist motor vehicle drivers and bus drivers to ensure each child is transported safely at all times. The service understands that drivers of small vehicles carrying up to 12 people are responsible for road safety and ensuring each child under 16 is properly seated and restrained. All educators, however, have an equal responsibility to assist the driver and check that each child is seated and restrained appropriately using the following guidelines.

Vehicles built to carry up to 12 people including the driver

Generally educators and volunteers will follow the restraint arrangements which follow to ensure the safety of children. However, if a child is too small for a restraint specified for their age, they will be kept in their current restraint for as long as necessary. Likewise, if a child is too large for a restraint specified for their age, they may move to the next level of restraint.

Educators and volunteers will also comply with any current medical plans or certificates signed by a registered medical practitioner that states the child should not be restrained as outlined below while travelling in a vehicle (or bus) for medical reasons. Educators and volunteers will comply with the conditions in the medical certificate, and where possible, ensure the child travels in a rear seat. Medical certificates must have an expiry date.

The Approved Provider or Nominated Supervisor will ensure all safety restraints are safe, labelled with Australian Standard (AS/NZS1754) (restraints purchased overseas do not comply with Australian Standards and are not compatible with Australian vehicles), and have been professionally installed or checked by authorised fitter. They will also ensure there are sufficient restraints to meet the safety restraints requirements under the national Road Rules as detailed below.

Educators and volunteers will ensure:

- Each child under seven is secured in a child restraint or booster seat with seat belt or safety harness when travelling
- Babies up to six months of age are restrained in a rearward facing restraint with a built in 5 or 6 point harness
- Children from six months to under four years are restrained in a rearward restraint, or forward facing restraint with a built-in 6 point harness. Rear facing restraints offer better protection as long as the child fits in it
- Children from four years to under seven use a forward facing restraint or booster seat. A forward-facing child restraint offers better protection as long as the child fits in it
- Children under four years are not in the front row of a vehicle with two or more rows
- Children from four to under seven years only sit in the front row of a vehicle with two or more rows if all other seats are occupied by children the same age or younger in an approved restraint
- The number of children transported does not exceed vehicle rated seating capacities
- Children do not share a seat belt or child restraint
- All adults use available safety belts.

Buses

Buses are defined as vehicles built to carry over 12 people including the driver. The Nominated Supervisor, educators and volunteers will:

- ensure seatbelts/restraints are used if they're required to be fitted
- ensure seating capacity displayed on the compliance plate is not exceeded
- consider whether an alternative mode of transport should be used to ensure children's safety where restraints are not available.

Transport Procedures

To ensure children's safety all employees and volunteers will implement the **Transport Procedure** or **Transport Procedure Excursions** when transporting children to and from destinations. Under no circumstances will any child be transported if all of the guidelines and procedures in this Policy are not met.

From 1 March 2023, staff responsibilities for regular transportation conducted by centre-based services include:

- A nominated supervisor or a staff member (other than the driver) must:

- be present when children embark and disembark a vehicle at the service premises
- account for each child when they embark and disembark a vehicle at the service premises
- complete a check of the interior of the vehicle after all children have disembarked at the service premises to ensure there are no children left on the vehicle.

Updated record keeping and accounting for children during regular transportation includes:

a) The approved provider and nominated supervisor must ensure that records are kept for regular transportation that:

- confirm each child was accounted for when embarking and disembarking the vehicle at the service premises
- state how each child was accounted for at the service premises
- state that the interior of vehicle was checked after all children have disembarked at the service premises.

b) The records must be completed at the time of entry (ie not after or before). On the record that documents the child entering and exiting the bus; include the time, date, full name and signature of the person/s responsible for:

- accounting for the children during embarking and disembarking of the vehicle
- conducting the vehicle check after children have disembarked.

Maintenance

To ensure vehicles owned or operated by the service are safe and hygienic, where relevant the Nominated Supervisor will:

- follow the recognised service schedule and organise an annual mechanical inspection, or sight evidence vehicle has had mechanical inspection within the last 12 months
- look for obvious maintenance issues eg bald tyres
- pay insurance, registration etc or sight evidence vehicle is registered and insured
- ensure check oil, water and tyres every month
- ensure vehicle is regularly cleaned
- ensure children can't access vehicles when they're not being used.

Car Park and Driveway Safety

Young children are at risk from moving vehicles in low speed 'off road' locations such as driveways and car parks. Studies have shown for example, there are large 'blind zones' behind many vehicles where drivers cannot see what's happening. The Approved Provider, Nominated Supervisor and staff will implement measures to remove or control the risks posed by any car park on the premises. They will complete a risk assessment to identify and control risks and may implement measures such as:

- speed signs with maximum car park speed limits
- parking signs advising parking limited to eg 5 minutes or reverse parking required
- one way signs so all vehicles drive through car park in the same direction
- wickets to control/block access in particular areas
- supervising area during drop offs and pick ups
- encouraging people collecting children to walk around vehicle before they leave.

Road Safety

Educators understand that children are vulnerable road users. They may think they can handle crossing a road by themselves but:

- are easily distracted and focus on only one aspect of what is happening
- are smaller and harder for drivers to see
- are less predictable than other pedestrians
- cannot accurately judge the speed and distance of moving vehicles
- cannot accurately predict the direction sounds are coming from
- are unable to cope with sudden changes in traffic conditions
- do not understand abstract ideas like road safety
- are unable to identify safe places to cross the road
- tend to act inconsistently in and around traffic.

Educators will closely supervise all children when outside the service near roads. They will hold children's hands, or if not practical to do this for all children, implement measures which keep children safe eg ensure children hold on to a rope at all times and wear high visibility vests.

Educators will regularly integrate learning about road safety into the curriculum. They will also provide information to families about children and road safety including:

- the key role families have in educating their children about road safety and the close supervision children require in and around traffic to keep them safe
- opportunities in day-to-day routines to discuss road safety with children eg on the way to the shops, service or school, while crossing roads (when and why it is safe to cross)
- danger areas like car-parks, traffic lights, pedestrian crossings and driveways. In relation to driveways, it's vital to:
 - *always supervise children* whenever moving a vehicle ie hold their hands. *Put children securely in the vehicle with you* if you're the only adult around, even if moving just a small distance
 - *Encourage children to play in safer areas away from the driveway & cars.* The driveway is like a small road and should not be used as a play area
 - *Make child access to the driveway difficult* eg use security doors, fencing or gates.

Related Policies

Excursion Policy

Appendix: Transport Procedure

See Transport Procedure Excursions if transport involved in excursion or regular transportation of children

Staff must not consume tobacco, drugs or alcohol during trip.

Before transport starts and at the end of use of transportation vehicle

The Nominated Supervisor will:

- nominate the driver (if using vehicle owned or operated by service)
- nominate the supervisor responsible for supervising children on vehicle
- nominate the checker responsible for checking vehicle at end of trip (may be same person as supervisor)
- ensure transportation meets the requirements/exclusions in service public liability insurance.
- must ensure a staff member or nominated supervisor (other than the driver) accounts for children as they get in or out of a vehicle at an education and care service.
- must ensure records are made showing children are accounted for as they get in or out of a vehicle at the education and care service.
- must ensure records are made confirming a check of the inside of a vehicle was completed at the service after all children have left to ensure no children remain.
- Approved provider must notify the regulatory authority when they start providing or arranging regular transport, and again when they finish. This can be done through the NQA ITS

The Nominated Supervisor and staff will also make sure:

- a risk assessment has been completed. Complete before each transport event, or if regular transportation, make sure there's a risk assessment less than 12 months old
- In risk assessment (see also policy above):
 - a) confirm how each child is planned to be accounted for when embarking and disembarking the vehicle at the service premises (see procedure below)
 - b) state how each child was accounted for at the service premises
 - c) state that the interior of vehicle will be checked after all children have disembarked at the service premises.
- they have written authorisation for every child to leave service. Obtain before each transport event, or for regular transportation, obtain annually
- required educator/child ratios will be maintained and there'll be adequate supervision (driver cannot supervise children). Include educator/volunteer familiar with children's special needs if relevant
- seat belts/booster seat arrangements meet legal requirements and the number of occupants will not exceed the legal capacity of the vehicle (eg displayed on bus compliance plate)
- at least one educator/supervisor attending has current first aid/asthma/anaphylaxis qualifications
- they have first aid kit and charged mobile phone
- they have current list of children travelling and their current emergency contacts
- there are no loose or heavy objects where passengers sit which could dislodge during sudden braking.

Drivers must

- park vehicle at the curb or in a protected parking area/driveway and turn off engine at each stop
- only start driving when told it's safe to leave
- always follow road rules
- not talk on mobile phone, including hands free
- not play loud, distracting music
- not be impaired by alcohol or legal/illegal drugs.

As each child enters transport (eg bus)

- escort children to transport or ensure children escorted by eg educator/teacher etc
- as children embark look at each child, check name off roll (educator is to tick next to each child names on entry to bus as indication they have sighted child on entry of bus) and do a head count at the end (as a check). A second educator to complete the check also. On the record that documents the child entering and exiting the bus (ie the roll); include the time, date, full name and signature of the person/s responsible for the account for children
- (see below more requirements for records)
 - get parent/guardian to sign child onto bus if collecting from home
 - if a child is present but not named on the roll or vice versa, confirm arrangements for the day with service/parent before leaving and update roll
- fasten children's seat belt/safety restraint. Make sure car seats, booster seats and seat belts are properly secured, including those where parents have secured their child and for older children who have fastened their own. Do not let children share restraints.
- If extra adults are attending (eg parent helpers)
 - give adult list of any specific children they are supervising
 - clearly outline their responsibilities

During Trip

Supervise children

- ensure they're comfortable at all times eg adjust ventilation and heating/cooling before it becomes stuffy or too hot/cold.
- never leave children unattended
- stop children doing anything dangerous during the trip eg drinking, eating, standing and any other dangerous activities while in the vehicle
- ensure at least one person (eg supervisor) sits at back of vehicle for optimal vision
- if child gets sick or hurts themselves during trip
 - if medical emergency ring ambulance on 000, follow all instructions and park vehicle in safe location until ambulance arrives, otherwise apply appropriate first aid
 - ring parents/guardians and advise they will need to collect child from destination (eg service) or they may need to take child to the doctor when they collect them. Provide as much information about illness/injury as possible
 - complete incident, injury form
 - notify regulatory authority if required.
- complete medication record if medication administered to child during trip.

As each child leaves transport (eg bus)

- **as children exit transport** look at each child, check name off roll (educator is to tick next to each child names on exit of bus as indication they have sighted child on exit of bus) and do a head count at the end. A second educator to complete the check also. On the record that documents the child entering and exiting the bus (ie the roll); include the time, date, full name and signature of the person/s responsible for the account for children
- The above provide should ensure an educator accounts for each child when they embark and disembark a vehicle at the service premises
- get parent/guardian to sign child off transport if dropping at home
- escort children into service/destination or hand them into the care and supervision of a staff member, parent/guardian or person nominated by parent/guardian
- if destination is the service, ensure children are signed in. Do not prefill sign in records

- never leave child alone on transport for any reason eg they have fallen asleep
- complete a check of the interior of the vehicle after all children have disembarked at the service premises to ensure there are no children left on the vehicle. This must be signed off by person responsible for excursion and checked by a second person (ie educator or teacher).
- make sure no child is left on transport eg look carefully on and under seats and in storage areas
 - make a note in writing that transport clear (or ensure checker does this)
 - ask driver to also check bus is empty before leaving vehicle and to make a note in writing
- if there is no authorised person present to collect child (eg parent/guardian), ring parent/guardian. Consider appropriate options:
 - can you wait (if they're running late)?
 - can parents/guardian authorise someone else to collect child? Note you must be able to adequately identify this person by appearance or photo identification
 - will you keep child on bus and continue trip?

Never leave child if there is no authorised person present to collect them.

if a child is missing immediately conduct search for child on transport, at destination and any places visited or stopped at during the trip. If child can't be located immediately call parents/guardians, and police on 000 and follow any instructions.

Ways to Minimise the Risks of Children being Left Behind in Vehicles

(Source ACECQ Fact Sheet: Minimising the Risk of Children Being Left Behind in Vehicles. January 2023)

WAYS TO MINIMISE THE RISK OF CHILDREN BEING LEFT BEHIND IN VEHICLES:



- ensuring the number of educators or other responsible adults is appropriate to provide supervision ensuring active and effective supervision at all times – with a particular focus on the seating of children and positioning of educator/s.



- actively engaging with the children over the course of the journey.



- establishing effective signing in and out procedures which may include having a second person to check the record.



- conducting regular headcounts and attendance checks of children including but not limited to before each departure, after each child embarks and disembarks the vehicle and after all children have disembarked.



- ensuring procedures for managing unexpected risks, such as if a child is absent from a collection point, are established and understood by all staff.



- ensuring procedures for embarking and disembarking the vehicle and processes for entering and exiting each location are documented and understood by all staff.



- embedding safety procedures for every trip including trips as part of an excursion or single trips. This may include having two people to conduct physical inspections of the vehicle at all destinations to ensure no children remain on the vehicle every time children disembark.



- ensuring relevant policies and procedures are up-to-date, provided to staff and families and that all staff are trained so that they understand their roles and responsibilities.



- embedding clear and effective procedures to ensure each educator/staff member is aware of their responsibilities for each trip and so do not assume someone else has completed a check.



- staff training on safe transportation of children is included in induction processes and refresher training is held annually.



- embedding risk assessment and management procedures such as the use of safety checklists for each trip. See ACECQA's [Safe transportation of children safety checklist template](#).



- Embedding effective communication plans/procedures with families to ensure that parents/carers inform the service that children will not be attending the service and do not need to be collected from another location.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy reviewed: 29/10/2024

Reference:**NQS**

QA2	2.1.1	<i>Supervision - Supervision At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>
QA6	6.2.1	<i>Transitions - Continuity of learning and transitions for each child are supported by sharing relevant information and clarifying responsibilities</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	4	<i>Definitions</i>
	99	<i>Children leaving the education and care premises</i>
	102B	<i>Transport risk assessment must be conducted before service transports child</i>
	102C	<i>Conduct of risk assessment for transporting of children by the education and care service</i>
	102D	<i>Authorisation for service to transport children</i>
	168	<i>Education and care service must have policies and procedures (transportation)</i>

Other References:

National Road Rules Model

National Guidelines for Safe Restraint of Children Travelling in Motor Vehicles – Kidsafe

Motor Vehicle Standards Act 1989 and Australian Design Rules Cwth

Road Safety Road Rules 2017

64 - COMPANY VEHICLE POLICY

Position Statement

Company vehicles are a valuable resource and must be kept in good working order at all times. People form opinions about a Company based on appearances, and therefore the company vehicle should be well presented at all times.

Objective

To ensure that team members understand and are able to fulfil the responsibilities that derive from having the use of a company vehicle.

Implementation

- Team members issued with a vehicle will familiarise themselves with the policy and agree to abide by it at all times.
- Team members who are issued with an infringement notice whilst using the vehicle agree that their license details, if requested, will be supplied to the Transport Department.
- Team members who do not comply with the terms and conditions of this policy shall accept full responsibility for any fines or penalties incurred.

Use of the vehicle:

- Company vehicles are to be driven by company employees only. In a case of emergency, authorisation for anyone else to drive the car must be obtained from the Operations Manager.
- Company vehicles are a 'SMOKE FREE ZONE'. No smoking will be permitted in vehicles.
- Team members will use the vehicle in accordance with all relevant legislation and the local road rules, including, but not limited to, speed limits, traffic lights, road signs.
- Team members will only use 'hands free' phones when making phone calls in the vehicle, and will always comply with the local legislation in regard to phone calls.
- Team members will not risk their safety or that of other people by making or receiving phone calls. If there are any doubts in this regard, the team member should safely pull off the road first before using the phone.
- Team members **will not drive the vehicle whilst holding a phone**, this includes the **prohibition of texting** whilst driving the vehicle.
- Team members will ensure that E-tags are appropriately placed in the vehicle, and will not use toll roads without ensuring this. Drivers will advise the Support team when E-tolls need topping up ie. Audible beeps are heard when passing the toll booth, indicating less than \$10 credit.
- Team members will ensure before leaving a vehicle that it is parked in accordance with the local traffic rules, and that it is locked, with no valuables in view of the passing public.
- Should a team member be required to transport an enrolled child, they will ensure that the child's seat belts are fastened, their hands and heads are inside the vehicle at all times, and that all loose objects are adequately secured, before departing, and during their journey.
- Vehicles must be driven safely and in accordance with the traffic act. All parking fines and infringement notices are the responsibility of the driver.

Care of the vehicle:

Team members provided with the use of a company vehicle will agree to take responsibility for the vehicle as follows:-

- Ensuring the vehicle is garaged in a safe place.
- Recording when registration is due and ensuring the correct sticker is applied to the vehicle when received.
- Recording when the vehicle inspections are due and keeping the appropriate records.
- Keeping an appropriate level of fuel in the vehicle at all times, in case of an emergency trip required.
- Cleaning the vehicle inside and out as needed, but at least once per week at the team member's expense.
- Check tyre pressure, engine oil level, and radiator water level weekly.

- A monthly vehicle condition report will be completed and sent to the Support team. Any urgent action required to maintain the vehicle should be forwarded as required after phoning the Support team to advise.

In the event of an accident:

Team members agree to follow all procedures as outlined in the Vehicle Incident Pack in the event of an accident, or incident such as vandalism, etc. This is absolutely essential as negligence may negate any insurance claim. Should it prove to be the case that team members did not follow the procedure outlined in the Vehicle Incident Pack, the team member involved shall accept responsibility for any losses incurred by the Company. Team members are encouraged to contact the Support team to seek clarification of any matters.

In the event that an accident is due to a team member's non-compliance with the Company's policy, the team member will be expected to pay the excess up to a maximum of \$500.

Evaluation and Review

This policy will be reviewed at least annually.

Team member feedback will be considered in the review process. Any changes to this policy will be communicated to team members verbally and in writing. Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy reviewed: 29/10/2024

Reference:

NQS

QA2	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained.</i>

QA6	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	100	<i>Risk assessment must be conducted before excursion.</i>
	101	<i>Conduct of risk assessment for excursion.</i>
	102	<i>Authorisation for excursion.</i>
	99	<i>Children leaving the education and care premises</i>

65- DELIVERY AND COLLECTION OF CHILDREN POLICY

Position Statement

It is the responsibility of the Centre staff to ensure the following guidelines are followed during the delivery and collection of children from the Centre.

Objective

To ensure and encourage the safety of children, families, visitors and staff members during delivery and collection experiences at the Centre and to educate such persons of appropriate procedures.

Related Policies

Enrolment Policy
Acceptance and R
Excursion Policy

Implementation

The following guidelines must be adhered to at all times to ensure the safety of the children

Arrival:

- Wash or sanitize hands on entry to service.
- Using the OWNA App, all children must be signed **IN** by their parent or by a responsible adult above the age of eighteen in order for parents to be eligible for Childcare Subsidy. This also assists staff in the event of evacuation of the Centre. **This is the parent/caregivers responsibility.**
- To ensure each child is cared for at all times, a staff member will greet and receive the child at all times.
- To support safe sleeping practices and procedures, educators will not accept sleeping children upon arrival and they will need to be woken up before being handed over to educator.
- A locker, cubby space, hook or other such storage device should be made available to children and their families. The child's name maybe labelled on or near the storage device. Or the child will select a locker or storage device.

Departure:

- Wash or sanitise hands on entry to service.
- Parents are to ensure that the authorised pick-up list for each child is kept up to date. It is responsibility of the parent to advise and update the authorised people on the enrolment form who are able to collect the child. (See below procedure for recording ID check to enrolment form for authorised nominee who have permission to collect children from service).
- No child will be released into the care of any persons not known to staff. If staff do not know the person by appearance, the person must be able to produce some form of photo identification to prove that they are a person authorised to collect the child on the child's enrolment form / OWNA authorised nominee – pickup list.
- Parents must give prior notice where the person collecting the child is someone other than those mentioned on the enrolment form, e.g. in an emergency situation. The person nominated by the parent must be able to produce some form of photo identification.
- If there is a variation of collection person from what the parent has indicated on sign in, the parent must contact the centre to advise of the changed person even if this is an authorised person.
- Children are not to be released into the care of persons not authorised to collect the child, e.g. court orders concerning custody and access.

- Parents must give prior notice of any variation in the persons picking up the child. If notice is not given, and staff cannot contact the parent, the child must not be released into the care of that person.
- If the person collecting the child appears to be intoxicated, or under the influence of drugs, and staff feel that the person is unfit to take responsibility for the child, the staff members are to bring the matter to the person's attention before releasing the child into their care. Wherever possible, such discussion is to take place without the child being present. Staff are to suggest that they contact the other parent or emergency numbers from the enrolment form, inform them of the situation and request they collect the child as soon as possible. If the person refuses to allow the child to be collected by another authorised person, staff members are to inform the police of the circumstances, the person's name and vehicle registration number. Staff cannot prevent a parent from collecting a child, but do have a moral obligation to persuade a parent to seek alternative arrangements if they feel the parent is in an unfit state to accept responsibility for the child.
- Using the OWNA app, all children must be signed **OUT** by a parent or responsible adult in order for parents to be eligible for Childcare Subsidy. This also assists staff in knowing who has left the centre
- At the end of each day 2 staff members check all beds and the premises including outdoors and indoors to ensure that no child remains on the premises after the centre closes.

Delivery and Collection of Children

Children remain under the care and supervision of their parent or authorised nominee until they have been signed into the service, directly greeted by an educator, and formally handed over into the care of the service to an educator. A child must not be left unattended or permitted to enter the service independently. Any person under 18 years of age accompanying a parent or authorised nominee must remain under the supervision of that parent or authorised nominee at all times and must not be responsible for the care, supervision or delivery of another child to the service.

In the event a parent/responsible person has not signed a child in or out, a staff member will do so and the parent will be required to confirm the information on their next OWNA Kiosk Access.

Children are to be collected before the closing time to allow time to sign out and communicate with staff in the centre. Staff on duty must, under their Award, finish at the closing time of the centre. Parents should ensure that they arrive in time to communicate with staff before closing time, as staff will not be available after this time.

Staff who are kept waiting after closing time should consult the Late Children Policy for the appropriate procedures.

OWNA ID Checks – Collected of Children by Unknown Parent or Guardian

The parent or guardian should inform the service if an emergency contact or authorised nominee will be collecting a child. If there is uncertainty if authority has been given, a call needs to be made to parent to confirm collection plans. Verify the name of the person and time the child is being collected.

On enrolment form, there are 3 authorised nominee types (all of these people have authority to collect children from care):

- 1. Parents 1 and 2
- 2. Emergency Contact
- 3. Authorised Nominee – Permission to Only Collect Children (no other authority)

(These people will shown on the parent / guardian contact screen in app – see example below)

Any changes to enrolment forms need to be manually changes in OWNA database. Ie if a parent removes an emergency contact or person who can collect. The Centre Director needs to remove this person from the "Relationship/Permission" tab for children – OWNA Contacts. Enrolment form should match this record. Check enrolment form if need to confirm.

The screenshot shows a mobile app interface titled 'Child Test'. At the top, there's a date 'Nov 29, 2023'. Below it is a section titled 'Parents/Guardians Contacts'. This section contains four entries, each with a circular icon and text:

- PL** (Parent Test): Mother, Phone: 041234565
- PL** (Parent 2 First Last Name): Father, Phone: 0412345671
- EL** (EC1 First Last 291123MC): Uncle/Aunt, Phone: 0412341234
- AL** (AN1MCELC First Last): Grandparents, Phone: 1234567891

At the bottom of the contacts list is an 'OK' button. Below the contacts list is a 'UV Alert' section with the text: 'Sun protection 9:20am to 5:40pm, UV Index predicted to reach 11 [Extreme]'.

Procedure: OWNA ID Check Form – Filed in Learning Documents for Child

The OWNA ID Check – Authorised Nominee Collecting Child, is required to be completed by an educator to verify that a person is an authorised nominee to collect a child from the service.

“ID Check - Record of ID for Authorised Nominee Collecting Child” form allows educators and responsible persons to make a record of the Photo ID of an unknown person who has authority to collect children. Authorised nominees are listed on child’s enrolment form and in OWNA Child Contacts. On submission of the OWNA learning document form, this will save a copy of the ID check and photo of ID in child folder and send a copy to parent via the OWNA timeline.

The screenshot shows a mobile app interface titled 'Create Learning Document'. The main heading is 'ID Check - Authorised Nominee Collecting Child'. Below this is a description: 'MCELC: Record of ID for Authorised Person Collecting Children. The form allows educators to make a record or verifying authorised person to collect children. Review enrolment if authorised, review ID, take photo of ID for record.' Below the description is a dropdown menu with the text 'ID Check - Record of ID for Authorised Person Collecting Children' and a downward arrow. Below the dropdown are several form fields:

- Record Date:** A text field with the placeholder 'e.g 2016-10-31' and a calendar icon.
- Rooms Children are in:** A dropdown menu with the text 'Please select'.
- Children collected from service:** A dropdown menu with the text 'Please select' and a blue information icon.
- Name of Authorised Person Collecting Children (First & Last Name):** A text field.
- Upload Photo of ID (Eg Drivers Licence):** A dashed box indicating where to upload a photo.

Safe Arrival of Children – Travelling between the Service and other external OSHC, school external organisation. (Not applicable to our service)

From the 1 October 2023, legislation was introduced relating to safe arrival of children for centre-based and family day care services. The requirements are:

- ensure the service has policies and procedures in place for safe arrival of children who travel between an education and care service and any other education or early childhood service.
- Conduct a risk assessment at least once every 12 months, and as soon as practicable after becoming aware of any circumstance that may affect the safe arrival of children travelling between an education and care service and any other education or early childhood service.

This requirement is not relevant to our services as we do not offer transport, transfer or supervision of children travelling between outside school hours care (OSHC), school, external kindergarten/preschool or any other external organisation. Parents and guardians or authorised nominee – collection person drop-off and collect children from the service. If children are attending an excursion, the excursion policy will be followed.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 21/7/2026

Reference:

NQS

QA2	2.2.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	115	<i>Premises designed to facilitate supervision</i>
	99	<i>Children leaving the education and care service premises</i>
	102	<i>Authorisation for excursions</i>
	168	
	169	

Other References:

CCB – Federal Funding Guidelines

Policy Handbook – P004. 7.2026

***Handbook on Child Care Licensing
Centre Support Policy Reference Guide***

Legislation:

Child Care Act 2002 and Regulations 2003 (Qld)

Children's Services Regulation 2004 and Children and Young Persons (Care and Protection) Act 1998 (NSW)

Children's Services Act 1985 & Children's Services (Child Care Centre) Regulations 1998 (SA)

Children's Services Act 1996 (Vic) & Children's Services Regulations 2009 (Vic)

Occupational Health & Safety Act 2004

66 - FEES POLICY

Position Statement

To outline the requirements for the payment of fees for children attending the service.

Objective

For parents to pay their childcare fees on time and in accordance with enrolment terms and conditions.

Implementation

Please refer to the services Fee Schedule for current fees and fee information and also the Great Start Early Learning Centres Terms and Conditions.

The following policy outlines how fees can be paid. Fees must be paid prior to the first morning your child attends the centre.

- An initial booking fee is required to be paid when lodging an enrolment form (this fee is calculated at the **greater of** two weeks attendance fees or \$100). **This is non-refundable if your child does not take a place or attend our service.**
- On commencement of your child attending, the booking fee payments will be applied against fees due and outstanding.
- Booking fees are calculated using the attendance fee calculation tool prior to commencement.
- Unless booking fee and advance attendance fees are paid, enrolments will not be held.
- Booking fees will be lost if enrolment offer is not accepted or child does not commence as planned or agreed.
- Upon enrolment, families must pay two weeks fees (current week fees plus two weeks fees in advance).
- Fees must be paid in advance (current week plus two weeks in advance) and in accordance with fee schedule.
- If the policies and procedures of the service are not adhered to, the service reserves the right to suspend the attendance of the enrolled child/ren and/or cancel enrolment/s. During the period of suspension of care, fees will be charged as normal equivalent to the daily fee even if the child does not attend or care is suspended. For example, if account is not paid per fee policy (current week plus 2 weeks in advance) the service reserves the right to suspend attendance until fees are paid. Notice is required to be provided by enrolling party if wishing to end care arrangements.
- Fees must be paid weekly in advance (current week plus 2 weeks in advance) using the service's nominated payment method system; Fat Zebra Merchant. For additional payments other manual payments method is EFTPOS or Once-off Payment (Fat Zebra) using OWINA Parent App. Please note manual payments will only be used for additional payment under instructions of service and are not allowed to be used as a regular or routine payment method. Please note per the Regulations the service does not accept cash.
- It is a requirement to use the service's nominated direct debit system (ie. Fat Zebra Merchant) for payment of all fees, not using the service may terminate your child's enrolment.
- Direct Debit Authority forms may be signed using an electronic signature software via the Enrolment Form and OWINA Parent App. The service has a right to use the bank account and credit card information provided to collect payment of fees and outstanding account balances.
- The enrolling parent must complete and sign a new Direct Debit Authority form when their details change. This is completed via the OWINA App.

- The service does not accept any responsibility for errors or issues with Centrelink system or families personal Child Care Subsidy (CCS) entitlement issues of any nature. Fees will be charged on a weekly basis (current week plus 2 weeks in advance) using the information our CCS system has been provided by Centrelink at the time of processing this information. The set day is Wednesday each week.
- Fees are payable in advance for every day that your child is enrolled at the centre. This includes public holidays, sick days and family holidays.
- Child Care Subsidies (CCS) are available to all families who are Australian Residents. To find out their eligibility, families must contact Centrelink or use MyGov.
- Under Family Assistance Law, families are entitled to Child Care Subsidy (CCS) to the last attended day of their child's attendance at a service, not the last booked date. If your child does not attend the last booked date the government requires full fees without CCS to be charged between your child's last attended date and the last booked date. We recommend attending the centre as planned to avoid any additional out of pocket costs resulting from this government regulation.
- *Kindergarten Per Capita State Government Funding:* By way of background information our Service receives Per Capita Kindergarten State Government Funding. The kindergarten per capita funding is paid directly to the service and our fees take into account receiving this funding. Kindergarten per capita funding is not received by parents. The per capita funding helps the service deliver a comprehensive kindergarten program. (Please note our service does not receive kindergarten program subsidy funding.)
- ***Victorian Government Free Kinder Offset (Introduced by Victorian Government 2023)***

Who does the "Free Kinder" funding apply to?

Families with a child enrolled in a funded kindergarten program are eligible for additional Kinder funding – 'Free' Kinder Offset. This includes all children attending funded kindergarten programs in the year before school (Four-Year-Old Kindergarten) and Three-Year-Old Kindergarten program. Please refer to 1 – Enrolment Policy for further information.

- If your child attends our **4-year-old funded kindergarten program** for 2026, a \$2,101 'Free' Kinder Offset (paid direct to the service) for a 15-hour program will offset your out-of-pocket fees.
- If your child attends our **3-year-old funded kindergarten program** for 2026, a 'Free' Kinder Offset is pro-rata based on the child's kindergarten program attendance hours/days. I.e. between 5 and 15 hours per week. Please see Government website for thresholds and payments.
- Most families will still pay gap fees for their child's attendance across the week, depending on the number of days attending and their personal Commonwealth Childcare Subsidy(CCS) entitlement. The gap fees payment will be continued to be made via weekly direct debit.
- The service receives the "Free Kinder" subsidy directly from the Victorian Government and the service on receipt of funds will apply the fee offset on a monthly basis in arrear to the family account once payment and approval has been received for the child via the KIM system. The paperwork you are required to complete is the kindergarten program declaration form. You are not required to do any additional paperwork to receive the 'Free' Kinder Offset. Please refer to the information on the government website for your eligibility.

How much and how do you receive the credit?

- The \$2101 (2026 amount) (or prorata amount for 3 year old kinder) is designed to contribute towards the 600 hours/15 hours per week for 40 weeks of a funded kindergarten program.
- The credit per month will be applied to your account on a monthly basis (Starting February). These dates are subject to finalising policy and receiving funds from Government.

- If your gap / out of pocket is less than Free Kinder Subsidy after CCS is applied, the credit will only be applied to the outstanding out of pocket for that month. The Free Kinder Subsidy is non-refundable.
- We will narrate the credit on your Account Statement as “Victorian Government Free Kinder Offset”.

Key Criteria Points:

- Our service is classified as a “Long Day Care Service with a funded kindergarten program”. To be an approved kindergarten program and receive government funding
 - a) the service must employ an Early Childhood Teacher to deliver the kindergarten program hours
 - b) deliver the kindergarten program in accordance with the National Quality Standards
 - c) child must attend the days and hours of the funded kindergarten program
- Your child can only be funded for a kindergarten place at one service at any one time (for one year in a 3-Year-Old Kindergarten program and one year in a 4-Year-Old Kindergarten program). Unless your child is assessed as being eligible for a second year of Four-Year-Old Kindergarten by your child’s kindergarten teacher. The child can attend two kindergarten services but only one service is eligible to receive the ‘free’ kinder subsidy and per capita funding support. We wish for you to provide this nomination to our service to assist us in delivering a high-quality program.
- We offer enrolments and bookings on a priority access basis and priority will be given to children who nominate the service as their 3 and 4-year-old Kindergarten Program to ensure we can offer all families kindergarten enrolments. Children who are not eligible or parents do not nominate the service for their kindergarten program are still able to attend Kindergarten and Prekindergarten rooms if we offer a position in these rooms. As per prior years, part of the kindergarten program the family needs to nominate in writing the service to be the child funded kindergarten program – this form has already been provided to all families.
- It is our understanding that Free Kinder Offset is on top of all normal kindergarten funding streams **excluding the Kindergarten Fee Subsidy (KFS), the KFS Ratio Supplement (these 2 not relevant to our service) and the Early Start Kindergarten (ESK) Extension Grant, as these rates have been incorporated into the Free Kinder Offset.** ie If child receives ESK grant they will not receive the additional \$2,101 kinder funding. Families are still are required to nominate children who are eligible for ESK.
- Per the funding policy any surplus of any additional kindergarten funding will used to support the broader service and the additional funding is not refunded to parent.

The fee policy should be read in conjunction with Fee Schedule and Great Start Early Learning Centres Enrolment Terms and Conditions.

Please refer to the OWNA Parent App for invoices and account transaction reports/statements. This will include the child/children’s full name/s, date of care, date of payment, amount, etc. If the incorrect amount is paid, a refund will not be given but will be credited/adjusted to the families account.

Our **terminating policy** requires that two weeks written notice must be given otherwise **2 WEEKS FEES** will be billed to you. Please note (per the Government CCS rules)– CCS is not claimable if there is no attendance during 2 week notice period and therefore full fees will be charged for this period. To receive your CCS entitlement please attend up to and including your child/ren’s last day.

Direct Debit Merchants

From the 29 January 2024, our Service uses Fat Zebra for merchant services. The transactional fees charged to parents and guardians are as follows: (1) Direct Debit: 75 cents (2) Credit Card: 1.75% (3) AMEX: 2.65% (4) Dishonour Fees: \$2.50. These fees are subject to change. For further information please see: <https://www.fatzebra.com/>

If a direct debit is returned by customer Financial Institution as unpaid, the Dishonour/Default Fee may apply the next direct debit payment, in addition to any Financial Institution charges.

Dishonoured Payments

Fail Payments and Dishonours; if this happens, we regret to inform you a charge of **\$2.50** (*this amount is changed periodically by our Fat Zebra Direct Debit Merchant*) will be billed to your account and require to be paid. This charge will vary depending on the charge incurred by the Direct Debit Merchant and is subject to change without notice. Please note the customers bank may also charge a dishonour fee / fail payment fee. In all circumstances these charges are payable by the enrolling parent, guardian, organisation or agency (not the service).

- Before 29 January 2024, our Direct Debit Merchant was Debit Success Pty Ltd. Default / Dishonour fees are charged by Debit Success Pty Ltd. The default charge as at 28 March 2022 is \$19.95 and is subject to change. This default charge is paid to Debit Success as a dishonour fee and not to the Centre. Please see www.debitsuccess.com.au for further information.

Overdue Fees

Any family who is one or more weeks late with their fees will received an Overdue Fee Reminder; overdue letter/email and/or receive telephone call to verbally advised. Families can make appointments to speak with the Director regarding payments if there is a need to do so. Continually not paying fees will put your child/ren's place/s in the centre in jeopardy.

If an account continues to be overdue, the matter will be referred to Management and debt recovery procedures will commence. A debt recovery cost of 25% of the outstanding account balance will be added to your account if the account is sent to debt collection.

Medical, Dental, Hospital and Ambulance Treatment Costs

Parents and guardians authorise the service to obtain medical, dental, hospital and ambulance treatment for their child if required. In accordance with the enrolment terms and conditions, the service is not responsible for paying or covering any costs associated with medical, dental, ambulance, hospital or other treatment. Parents and guardians agree to reimburse Great Start Early Learning Centres and Service for any charges, costs or expenses incurred in arranging or obtaining such treatment.

Important Note:

Please refer to the services Fee Schedule for current fees and charges.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

NQF

QA7	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Handbook on Child Care Licensing
Centre Support Policy Reference Guide

67 - BACK CARE AND MANUAL HANDLING POLICY

Position Statement

To have a healthy and safe work environment for staff and children.

Objective

To ensure a healthy and safe work environment and prevent or minimize manual handling injuries to staff by using mechanical aids, team lifts and safer handling methods.

Implementation

Remember:

- Manual handling means any activity requiring the use of force exerted by the person to lift, lower, push, pull, carry or otherwise move, hold or restrain any person or object.
- Manual handling injuries may be caused by the above activities. Injuries can include back strains and similar strains and sprains in parts of the body such as the neck, arm, shoulder and knee.
- Manual handling injuries also encompass overuse injuries or, as a result of falling during manual handling, bruising or laceration.

Guidance:

IMPORTANCE OF KEEPING FIT

As working with children is physically demanding, it is wise to do warm-up exercises for three to five minutes before starting work. Muscles and tendons are more likely to be damaged when cold.

Simple exercises to warm and stretch all the major muscle groups will help prevent injury. In addition, regular exercise such as walking, tennis, golf or aerobics will help build up muscles but anyone with neck, back or muscular problems should see a doctor before exercising.

PRINCIPLES OF PREVENTING MANUAL HANDLING INJURIES:

1. Eliminate or reduce the amount of manual handling.
2. Reduce the amount of bending, forward reaching, and twisting, in all tasks.
3. Reduce worker fatigue.
4. Keep all equipment in good working order.
5. Keep the workplace environment safe.

To help prevent manual handling injuries:

- **Kneel down rather than bend down.**

For example, helping a child to do up a shoelace.

- **Sit down rather than bend.**

For example, if a child needs comforting sit on a chair and encourage child to sit or stand next to you.

- **Sit in an appropriate sized chair or on the floor.**

A suitable chair allows an adult to sit with upper legs horizontal to hips and feet flat on the floor.

- **Carry children only when necessary.**

The correct way to carry a child is with one arm under the child's buttocks and the other arm supporting the child's back. At the same time, hold the child facing you, as close to your body as possible.

Adults should try to avoid carrying a child on their hip because this can strain the back.

- When lifting awkward loads, be careful to **lift with a balanced and comfortable posture.**
- **Minimise the need to reach above shoulder level.**
If necessary use a step ladder.
- **Avoid extended reaching forward,**
For example, leaning into low equipment boxes.
- **Share the load** if the equipment is heavy, long or awkward. Using of trolleys or baskets to carry items or rubbish bags do not lift heavy or awkward items.
- To lift a child out of a cot, **lean against the cot and raise the child** as close as possible to your body. Do not stretch over and lift.
- When sliding, pulling or pushing equipment that is not easy to move, e.g. trestles or gym mats, **ask for help and organise a team lift.**
- Where possible, **rearrange surroundings to meet the needs of both children and adults.** Remember these needs when buying furniture and equipment or upgrading facilities.
- **Use equipment and furniture that can be moved around as safely and easily as possible.**
- To complete lengthy writing tasks, e.g. program planning, **sit at an appropriate adult sized chair at an adult sized table.**
- **Larger children to climb up steps/ladder provided to change table.**

AVOID TWISTING WHILE LIFTING

Many injuries result from twisting while lifting. To avoid this:

- Move equipment when children are not around.
- Rearrange storage so that it is easier and safer to replace and remove items.
- Lift only within the limits of your strength.
- Use beds and equipment that are easy to move.
- Make sure you can see where you are going when carrying equipment or children.
- Be especially careful when lifting a child with special needs.

AVOID ACCIDENTS WITH CAREFUL HOUSEKEEPING

Good housekeeping means fewer accidents. Check that:

- The floors and other walking surfaces are uncluttered, even and non-slippery.
- The workplace is tidy.
- There is adequate space to perform each task.
- Equipment is maintained regularly.
- Lighting is adequate.

HOW TO LIFT SAFELY

1. Place your feet in a stride position.
2. Keep your breastbone as elevated as possible.

3. Bend your knees.
4. Brace your stomach muscles.
5. Hold the object close to your centre of gravity, i.e. around your navel.
6. Move your feet not your spine.
7. Prepare to move in a forward-facing direction
8. Ask for help when it is not possible to lift on your own.

CONTACT WITH A CHILD: MOVING, LIFTING OR HANDLING

Never do the following when moving, lifting or handling a child as it can cause physical injury to the child and educator:

1. **Pulling a child up by the hands.** Pulling on hands or forearms can put stress on the elbows. Never pick up a child by the hands or wrists. Lifting under the armpits is the safest way to lift a child.
2. **Swinging a child by the arms.** Any type of swinging that involves holding the hands or wrists can put stress on the elbow joint and should be avoided.
3. **Jerking a child's arms.** Pulling a child along while walking or quickly grabbing his or her hand can jerk the arm, causing the annular ligament to slip. Reminder to be gentle when taking a child by the hand. The recommended practice is that an educator has a loose grasp of child's hand or the child holds the educator's hand.

HOW TO ORGANISE A TEAM LIFT

1. Ask a colleague who is willing and able to help.
Ideally the colleague should be fairly well matched with you in size and strength.
2. Agree on a plan of action.
A coordinated movement during a lift is important.
3. Timing is important for co-ordination.
One person should act as a team leader and 'call' the lift.

Please refer to Appendix 1 and 2 for more information on safe lifting techniques and procedures.

HOW TO ASSESS THE CORRECT STORAGE AND SHELVING HEIGHT

Correct storage and shelving height is important to prevent slips, falls and strains:

To check the height at which it is safest for each individual to work, stand with feet together and hands by sides:

- **the best height** range for handling loads is around waist level
- **the acceptable height** for lifting is any point between the individual's knuckle and shoulder.

Seldom-used objects can be stored at the shoulder-to-raised arm height (use ladders to avoid stretching).

Avoid storing objects at a level between an individual's knuckles and the floor.

Mechanical aids such as ladders and trolleys should be used where possible to avoid lifting.

A Health and Safety Solution



Lifting children in/out of cots and highchairs or on/off change tables

August 2011

What is the problem?

Workers experience musculoskeletal injuries (eg sprains, strains, fractures and soft tissue injuries) from the design, placement and characteristics of equipment children are lifted onto/into (eg cots, highchairs and change tables).

The implementation of risk controls must not introduce any new hazards or risks to workers or children.

What are the risks?

Workers are at risk of injuries from bending, twisting and reaching due to:

- poorly designed work areas
- insufficient space
- equipment that is poorly positioned
- equipment with a low working height (eg cots with bases below mid thigh height)
- equipment that is not adjustable (eg cots with fixed sides)
- equipment that does not have removable parts (eg fixed highchair trays)
- mismatch between equipment adjustability for children and desired adjustability for workers
- effort involved in lifting and lowering children
- unexpected or unplanned movement by children

What are solutions to the problems?

Organisations are encouraged to discuss how and when children are lifted onto equipment so workers' and children's needs are addressed.

Cots

- Reduce the distance the worker needs to bend and reach to lift the child by using cots:
 - that are designed for day nurseries with a higher working height
 - in the higher base position if the cot is designed with two base positions. Ensure this meets the children's safety requirements (eg child cannot climb out)
 - with drop-down sides.
- Prepare cots before children are lifted so sides are dropped down.
- Provide sufficient access around cots so workers can lift or lower children without reaching over the length of the cot.

- Maintain wheels, wheel-locks and drop-down sides on cots so they work effectively.
- If cots are wheeled:
 - provide appropriately sized wheels
 - assess floor surfaces for minimal friction for cots to be wheeled with minimal resistance. Modify existing floor surfaces if necessary.

Highchairs

- Use highchairs that have removable or adjustable trays so the tray can be put in place after children are seated.
- Use height adjustable highchairs for children to safely seat themselves.
- Maintain wheels and wheel-locks on highchairs.
- If highchairs are wheeled:
 - they should have appropriately sized wheels with lockable castors
 - assess floor surfaces for minimal friction so they can be wheeled with minimal resistance. Modify existing floor surfaces if necessary.

Change tables

- Use change tables that provide a space for workers feet underneath the table so they can stand close to the child being changed.
- Use change tables that are height adjustable to waist height for the worker.
- Use change tables with steps that have:
 - a handrail so children who can walk access the change table themselves
 - removable, retractable and lockable steps so workers do not have to reach over the steps when changing children
 - easy access so the worker does not have to repeatedly bend to access or put away the steps.
- Ensure steps are well maintained and easy for the worker to move.
- Provide sufficient access around the change table so workers can change the child from a front position (child's feet facing worker's stomach).
- Position changing supplies within easy reach to reduce the need for the worker to repeatedly twist, bend or use an extended reach.

A Health and Safety Solution Lifting children in/out of cots and highchairs or on/off change tables

Systems

- Assess if, how and when each child is lifted onto, into or from equipment.
- Provide reporting processes so safety issues can be identified and fixed as soon as possible.
- Provide workers with training, information, instruction and supervision on documented work procedures and use of equipment.
- Schedule and record regular inspections, and maintenance of all areas of the workplace and equipment.
- Use a system to rotate tasks among workers to reduce their overall exposure to the risks associated with these tasks.

The problem



Cot base is non-adjustable and close to the ground, requiring workers to reach and bend to lift children into/from the cot.

The problem



A change table without steps requires workers to bend, reach and twist to lift children onto/from the change table.

The change table is designed so the child is side on to the worker when being changed, requiring reaching and twisting.

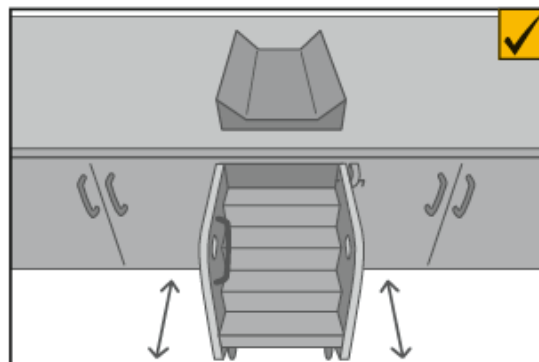
A solution



Cot is designed with a higher working height, reducing the workers need to reach and bend to lift children into/from the cot.

Cot has appropriately sized wheels and wheel-locks and worker can stand with their feet under the cot to reduce reaching distances.

A solution



A change table with stairs and handrails enables children to be assisted onto the change table without being lifted.

Retractable, lockable, and easily accessible steps reduce the need for the worker to bend.

The change table is designed so the child's feet are facing the worker, reducing the need for the worker to twist. There is space under the change table so the worker can easily reach the child.

Appendix 2: Correct Lifting Posture

(Source: [Correct Lifting Posture \(lifting position\) - Musculoskeletal Physiotherapy \(mphysio.au\)](http://mphysio.au))

Maintaining a correct lifting posture is vital in order to prevent injury and for your recovery to be enhanced along with your physiotherapy treatment.

Having a correct lifting posture will prevent unwanted strain, pressure and accumulative trauma to affected regions of your muscular, skeletal and neural systems of your body. The most crucial aspect of having a good lifting posture is to maintain a neutral lumbar spinal position of the lower back. It is integral to avoid arching or hunching the lower back.

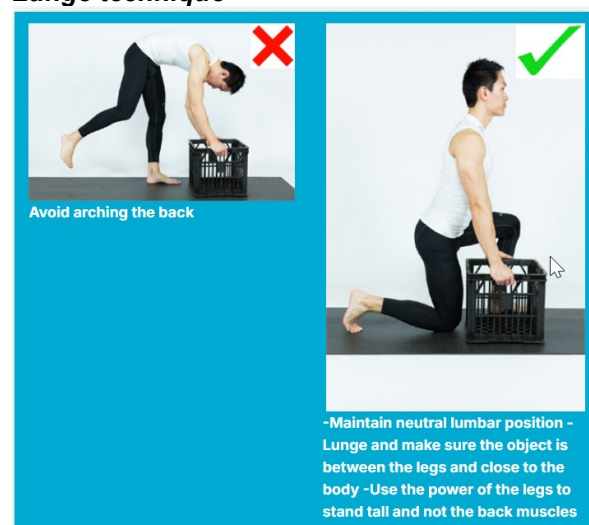
For medium to heavy objects use a squat technique For light objects use a lunge technique As the saying goes, 'Practice makes Perfect!' Practice these correct techniques and you will be amazed at how your body will respond! Once you read this page answer the questions on the sheet given by your physiotherapist at Musculoskeletal Physiotherapy Australia and bring that to your next treatment session.

Please view the following lifting posture examples to ensure you are lifting correctly.

Squat technique



Lunge technique



Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/8/2025

References: NQS

QA3	3.1.1	Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.
	3.1.2	Premises, furniture and equipment are safe, clean and well maintained
	3.2.1	Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.
	3.2.1	

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

	103	Premises, furniture and equipment to be safe, clean and in good repair
	105	Furniture, materials and equipment

Other Reference:

Baudet, Catherine (2003) 'Good Design Prevents Staff Injury', Gowrie RAP Reflections, no.11, May, pp11-12.

Maher, Lea (2001) 'You and Your Workplace', Gowrie RAP Reflections, no.3, May, pp16-17.
Australian Safety and Compensation Council. (2007). Index of national standards codes of practice and related guidance notes. Retrieved April 28, 2010, from
<http://www.ascc.gov.au/ascc/AboutUs/Publications/NationalStandards/IndexofNationalStandardsCodesofPracticeandrelatedGuidanceNotes.htm>

Tansey, S. (2005). Supervision in children's services. Putting Children First, 15, 8-11.

Tarrant, S. (2002). Managing OHS in children's services: A model for implementing an Occupational Health and Safety (OHS) management system in your children's service. NSW: Lady Gowrie Child Centre.

Federal

National Childcare Accreditation Council
Australian Workplace Safety Standards Act 2005
Australian Workplace Safety Standards Regulations 2005

State- Vic

Children's Services Regulations 2009
Occupational Health and Safety Act 2004

68 – BULLYING POLICY

Position Statement

Young children are learning to become active members of society. As a natural part of their learning; children will explore the limits placed upon them. Guiding children's behaviour is an essential part of early childhood education. Consistent approaches by team members in relation to management strategies are paramount to the overall success of a positive behaviour guidance policy; including bullying.

Objective

To provide an environment where the safety and wellbeing of everyone on the premises is ensured. Bullying can be on many levels such as verbal, physical, exclusion, online, extortion and through gestures. It is crucial that staff respond to bullying either amongst children or other staff members as quickly and effectively as possibly.

Implementation

Some characteristics of a child that bullies include:

- Preconceived notions of why children bully should be avoided.
- Children from all cultural backgrounds can bully.
- Many children who bully may be the victim of bullying.
- Research shows that bullying tendencies can begin as young as 2 years of age. This shows how important the role of child-care professionals in effectively dealing with bullying.
- The child who bullies will often think they are innocent of any bad behaviour, and that the child being bullied is deserving of this treatment.

Some characteristics of children who have been bullied include:

- Children of all cultural backgrounds can be victims of bullying.
- Preconceived notions of children who are bullying victims should be avoided.
- Victims may suffer from low self-esteem, low-confidence, lacked social views and be seen to be unpopular.
- Bully victims are often sensitive and easily hurt. They often feel incapable of preventing or stopping the negative experience.

Strategies to overcome bullying include:

- The centre will practice socially-inclusive care.
- Daily programs will recognise and value Australia's multicultural community.
- Staff will model and encourage appropriate behaviours.
- Staff will form close relationships with centre families in order to work together to overcome any bullying problems if they arise.
- Empowering children by giving them responsibilities so they will feel valued in the centre.
- Helping children to deal with their anger. This includes offering conflict-resolution methods.
- Seeking support of professional children's services if necessary.

Related Policies

Please refer to Policy 10 - Occupational Health & Safety Policy for employee Bully Policy and Procedures.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/11/2025

Reference:

NQS

QA5	2.1.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.</i>
	<i>Children develop knowledgeable and confident self identities.</i>
	<i>Children learn to interact in relation to others with care, empathy and respect.</i>
LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.</i>
	<i>Children respond to diversity with respect.</i>
	<i>Children become aware of fairness.</i>
	<i>Children become socially responsible and show respect for the environment.</i>

Other References:

National Childcare Accreditation Council
Children and Young People (ACT Childcare Services) Standards 2009 (No 1).

Sources

Managing a Child Care Service: A hands on guide for service providers
Slee, J. (2003). Managing difficult behaviour in young children. NSW: ECA.

69 – CHILD SELF RELIANCE AND SELF ESTEEM IN CHILDREN POLICY

Position Statement

To have a healthy and safe work environment for staff and children that supports positive self-esteem and reliance.

Objective

To develop children's enthusiasm for learning, impart self-management skills, facilitate the development of high order thinking and problem solving skills, help children establish satisfying and successful relationships and develop in children a healthy self-esteem and feeling of self-worth.

Implementation

When children have a healthy self-esteem, they:

- Make transitions easily
- Approach new and challenging tasks easily
- Set goals independently
- Have a strong sense of self-control
- Assert their own point of view when opposed
- Trust their own ideas
- Initiate activities confidently
- Show pride in their work and accomplishments
- Cope with (occasional) criticism and teasing
- Describe themselves positively
- Make friends easily
- Lead others spontaneously
- Accept the opinions of others
- Cooperate and follow rules, remaining largely in control of their own behaviour
- Make good eye contact (although this will vary across cultures).

(Sources: Adler et al 2001; Clark 1997 and Curry & Johnson 1990)

"Resilience does not come about by avoiding adversity but by receiving the necessary support to overcome it in a way that enhances children's self-esteem, confidence and faith in their own ability to master challenges". Therefore, the Centre has strategies in place for developing acceptance of and support for all children.

Educators will:

- Share in children's positive emotions and appreciate the pleasure the child derives from independence and competence.
- Actively encourage all children to show empathy and self-confidence irrespective of gender and other differences.
- Encourage healthy food choices.
- Monitor children's play and learning to prevent the development of inequity in all areas of children's development.
- Ensure that all children at the centre have equal opportunities to participate in all activities.
- Make sure the environment actively encourages all children to have a positive attitude to diversity including different abilities and backgrounds of others.
- Use effective individualised teaching and wellbeing strategies to foster self reliance and self-esteem of all children.
- Communicate with all children in ways that convey patience, genuine interest and respect.
- Show respect for all home and cultural customs.
- Interact in such a way that the child feels like an individual.
- Show interest in what the child is saying and is doing.
- Acknowledge children's efforts to solve problems and behave positively.
- Involve the children in setting the room limits.

- Recognise and respond to children's feelings e.g. Signs of sadness, boredom, interest, curiosity, pleasure, wariness, and frustration and help them to recognise, label and manage these feelings. (Tool: H.A.L.T)
- Respond to children's attempts at independence with encouragement and offer children choices when appropriate and assist when necessary to avoid frustration and a feeling of inadequacy, ie. given children sufficient confidence and resilience to persevere in the face of obstacles and not be devastated by the lack of success.

Children with other children will:

- Be encouraged to empathise with and support each other, especially children with special needs.
- Listen to each other's point of view.
- Be encouraged to develop problem solving skills and resolve conflicts.
- Be encouraged to think about how their peers may be feeling.
- Listen to one another and show interest, and ask questions in positive ways.
- Become involved in small interest groups which allow them to cooperate, collaborate, feel a worthwhile contributor to an end project/interest.

The centres uses programs such as Kimochi's, PALS and Box full of feelings to encourage children's development in this area.

To further support child reliance and self esteem the service will:

- Engage with a number of community agencies and health professional to support mental health and wellbeing initiatives, provide information and support referrals.
- Encourage educators and families to role model respectful relationships and a positive approach to mental health and wellbeing.
- acknowledge that families are key partners in developing and supporting mental health and wellbeing initiatives and promote their involvement.
- Encourage educators to access resources and professional development to enhance their knowledge and capacity to promote mental health and wellbeing.
- Build relationships and have positive transitions between early childhood services and local schools.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/8/2025

Reference:

NQS

QA5	2.1.1	<i>Each child's health and physical activity is supported and promoted.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
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	<i>Children develop knowledgeable and confident self identities.</i>
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LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.</i>
	<i>Children respond to diversity with respect.</i>
	<i>Children become aware of fairness.</i>
	<i>Children become socially responsible and show respect for the environment.</i>

Other References:

Children's Services Regulations 2009.

Young Children's Behaviour. Practical Approaches for Caregivers and Teachers, Louise Porter 2003

Challenging Behaviour in Young Children, Barbara Kaiser & Judy Raminsky 2003

70 – ETHICAL CONDUCT AND CODE OF ETHICS POLICY

Position Statement

The Centre, parents and employee to acts ethical and with a level of professionalism. We support and uphold the Early Childhood Australia Code of Ethics while also having a service Code of Conduct.

Objective

Our Centre has adopted the Moral Code of Conduct which establishes the expected behavioural requirements for staff members and parents, guardian and visitors whilst at the Centre. The Code guides everyone to act appropriately and assist them in fulfilling their statutory duty and social obligations to act honestly and professional in their interactions with other staff members, children, families and visitors.

The code of ethics provides a framework for reflection about ethical responsibilities.

Related Policies

Child Safe Standards Policy
Child Safety, Protection & Wellbeing Policy
Safeguarding Children: Look, Do & Tell Framework Policy
Families and Visitor Code of Conduct
Inclusion Policy
Child Safety, Protection & Wellbeing Policy
Education, Curriculum and Learning Policy
Governance of Service, Management and Educator Policy
Smoke, Alcohol & Drug Free Environment Policy
Excursion Policy
Family Violence Safety Policy
Complaints and Grievance Policy
Incident, Injury, Trauma and Illness Policy
Parent Involvement Policy
Policy and Procedure Review Policy
Privacy and Confidentiality Policy
Employment Policy
Interactions with Children Policy
Staffing Arrangements Policy
Technology & Digital Usage Policy
Internet, Email & Social Networking Policy
National Model Code Policy
Recording Keeping and Retention Policy

Early Childhood Australia – Code of Ethics

Please refer to the Early Childhood Australia Code of Ethic displayed with the centre notice boards. Also can be found at the website.

VISION

Professionals who adhere to this Code of Ethics act in the best interests of all children and work collectively to ensure that every child is thriving and learning.



DEFINITIONS OF TERMS IN ECA'S CODE OF ETHICS

A CODE OF ETHICS—defines the core aspirational values of the profession and provides guidance for professional decision making especially when there are conflicting obligations or responsibilities.

CORE PRINCIPLES—fundamental and prized values of the profession.

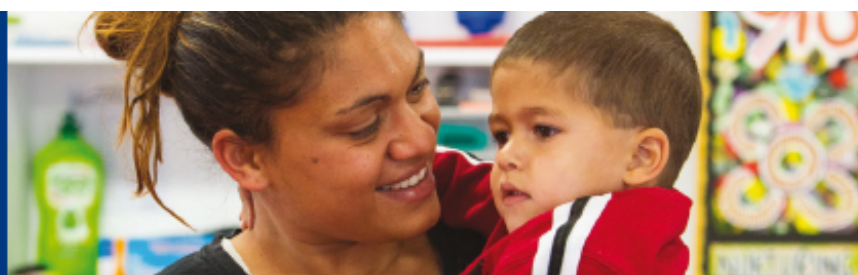
FAMILIES—the people who have significant care responsibilities for and/or kinship relationships with a child.

CHILDHOOD PROFESSIONAL—a person who works with or on behalf of children and families in education and care settings.

COMMUNITIES—a group of people living in the same place or having a particular characteristic in common.

COLLEAGUES—includes employers and those with whom you work directly or more broadly.

STUDENT—a person undertaking study at a secondary or tertiary institution.



PREAMBLE

Early Childhood Australia recognises that Aboriginal and Torres Strait Islander people have been nurturing and teaching children on this land for thousands of years. The Code of Ethics acknowledges Aboriginal and Torres Strait Islander traditional ways of being and caring for children.

This Code of Ethics is informed by the principles in the United Nations Convention on the Rights of the Child (1991) and the Declaration on the Rights of Indigenous Peoples (2007). A Code of Ethics is an aspirational framework for reflection about the ethical responsibilities of childhood professionals who work with, or on behalf, of children and their families. In this Code of Ethics the protection and wellbeing of children is paramount and therefore speaking out or taking action in the presence of unethical practice is an essential professional responsibility.

Being ethical involves thinking about everyday actions and decision making, either individually or collectively, and responding with respect to all concerned. The Code of Ethics recognises that childhood professionals are in a unique position of trust and influence in their relationships with children, families, colleagues and the community, therefore professional accountability is vital.

CORE PRINCIPLES

The core principles in this Code of Ethics are based on the fundamental and prized values of the profession. They act to guide decision making in relation to ethical responsibilities. These core principles require a commitment to respect and maintain the rights and dignity of children, families, colleagues and communities.

- Each child has unique interests and strengths and the capacity to contribute to their communities.
- Children are citizens from birth with civil, cultural, linguistic, social and economic rights.
- Effective learning and teaching is characterised by professional decisions that draw on specialised knowledge and multiple perspectives.
- Partnerships with families and communities support shared responsibility for children's learning, development and wellbeing.
- Democratic, fair and inclusive practices promote equity and a strong sense of belonging.
- Respectful, responsive and reciprocal relationships are central to children's education and care.
- Play and leisure are essential for children's learning, development and wellbeing.
- Research, inquiry and practice-based evidence inform quality education and care.

ACKNOWLEDGEMENT

The first Code of Ethics for the Australian early childhood profession was developed in 1988; it was widely cited and used for 19 years. The first review of the Code of Ethics began in 2003 with the second version launched in 2007. The second review of the Code of Ethics began in 2014 with the third (current) version approved by the ECA National Board in February 2016.

ECA is grateful to everyone who has contributed to the development and ongoing review of the Code of Ethics including those who have participated in consultation surveys, forums and workshops over the years as well as those who have provided advice and oversight through their service on reference and advisory groups. We would particularly like to acknowledge those engaged in writing each version of the Code.



Code of Ethics



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Early Childhood Australia
A voice for young children



Early Childhood Australia
A voice for young children

COMMITMENTS TO ACTION

IN RELATION TO CHILDREN, I WILL:

- act in the best interests of all children
- create and maintain safe, healthy, inclusive environments that support children's agency and enhance their learning
- provide a meaningful curriculum to enrich children's learning, balancing child and educator initiated experiences
- understand and be able to explain to others how play and leisure enhance children's learning, development and wellbeing
- ensure childhood is a time for being in the here and now and not solely about preparation for the future
- collaborate with children as global citizens in learning about our shared responsibilities to the environment and humanity
- value the relationship between children and their families and enhance these relationships through my practice
- ensure that children are not discriminated against on the basis of gender, sexuality, age, ability, economic status, family structure, lifestyle, ethnicity, religion, language, culture, or national origin
- negotiate children's participation in research, by taking into account their safety, privacy, levels of fatigue and interest
- respect children as capable learners by including their perspectives in teaching, learning and assessment
- safeguard the security of information and documentation about children, particularly when shared on digital platforms.



IN RELATION TO FAMILIES, I WILL:

- support families as children's first and most important teacher and respect their right to make decisions about their children
- listen to and learn with families and engage in shared decision making, planning and assessment practices in relation to children's learning, development and wellbeing
- develop respectful relationships based on open communication with the aim of encouraging families' engagement and to build a strong sense of belonging
- learn about, respect and respond to the uniqueness of each family, their circumstances, culture, family structure, customs, language, beliefs and kinship systems
- respect families' right to privacy and maintain confidentiality.

IN RELATION TO COLLEAGUES, I WILL:

- encourage others to adopt and act in accordance with this Code, and take action in the presence of unethical behaviours
- build a spirit of collegiality and professionalism through collaborative relationships based on trust, respect and honesty
- acknowledge and support the diverse strengths and experiences of colleagues in order to build shared professional knowledge, understanding and skills
- use constructive processes to address differences of opinion in order to negotiate shared perspectives and actions
- participate in a 'lively culture of professional inquiry' to support continuous improvement
- implement strategies that support and mentor colleagues to make positive contributions to the profession
- maintain ethical relationships in my online interactions.



IN RELATION TO THE PROFESSION, I WILL:

- base my work on research, theories, content knowledge, practice evidence and my understanding of the children and families with whom I work
- take responsibility for articulating my professional values, knowledge and practice and the positive contribution our profession makes to society
- engage in critical reflection, ongoing professional learning and support research that builds my knowledge and that of the profession
- work within the scope of my professional role and avoid misrepresentation of my professional competence and qualifications
- encourage qualities and practices of ethical leadership within the profession
- model quality practice and provide constructive feedback and assessment for students as aspiring professionals
- mentor new graduates by supporting their induction into the profession
- advocate for my profession and the provision of quality education and care.



IN RELATION TO COMMUNITY AND SOCIETY, I WILL:

- learn about local community contexts and aspirations in order to create responsive programs to enhance children's learning, development and wellbeing
- collaborate with people, services and agencies to develop shared understandings and actions that support children and families
- use research and practice-based evidence to advocate for a society where all children have access to quality education and care
- promote the value of children's contribution as citizens to the development of strong communities
- work to promote increased appreciation of the importance of childhood including how children learn and develop, in order to inform programs and systems of assessment that benefit children
- advocate for the development and implementation of laws and policies that promote the rights and best interests of children and families.

www.earlychildhoodaustralia.org.au/coe



Services Codes of Conduct Key Principles:

The Model Code of Conduct for our service is based on the following key principles:

Integrity
Selflessness
Respect
Honesty
Leadership
Accountability
Objectivity
Openness

Implementation

GENERAL OBLIGATIONS – See below our Code of Conduct

You must avoid conduct that:

- Is detrimental to the pursuit of the charter of the centre.
- Is improper or unethical.
- Is an abuse of power.
- Causes or involves intimidation, harassment or verbal abuse.
- Causes or involves discrimination, disadvantage or adverse treatment in relation to employment.

You must act lawfully, honestly and exercise diligence.

You must treat others with respect at all times.

Fairness and Equity

You have an obligation to consider issues fairly and consistently. That being, you must take all relevant facts into consideration and you must not take irrelevant matters into consideration when making decisions.

Harassment and Discrimination

You must not harass or discriminate against others, or support those who do the same.

Development Decisions

It is your duty to ensure that decisions are properly made and that parties involved are dealt with fairly.

If there is any uncertainty about the ethical issues around an action or decision you are about to take, you should consider these five points:

- Is the decision or conduct lawful?
- Is the decision or conduct consistent with centre policy and objectives?
- What will the outcome be for management, work colleagues, parents, children and any other parties?
- Do these outcomes raise a conflict of interest?

You have the right to question any instruction or direction given to you which you consider to be unethical. If you are uncertain you can seek advice from your manager or Authorised Supervisor or from the following organisations:

Independent *Commission Against Corruption* 02 8281 5999

Vic Ombudsman 1300 655 082

Gifts or benefits

You must not:

- Never accept an offer of money, regardless of the amount
- Seek or accept a bribe
- By virtue of your position acquire personal profit
- You may accept gifts or benefits of a nominal or token value that do not create a sense of obligation on your part.
- If you receive a gift of more than token value in circumstances where it cannot reasonably be refused or returned, you should accept the gift and disclose this promptly to your supervisor.

RELATIONSHIPS

Obligations of staff

The Manager is responsible for the efficient and effective operation of the centre
Employees have an obligation to

- Give their attention to business of the centre while on duty.
- Ensure that their work is carried out efficiently, economically and effectively.
- Carry out lawful directions given by any person having authority.

Obligations during meetings

You must respect management, other staff, parents or visitors present during meetings.

Inappropriate interactions

The following interactions are inappropriate:

- Employees approaching other employees directly on individual staff matters that doesn't concern them.
- Use centre information for personal purpose or benefit.
- Disclose any information discussed during a confidential meeting
- Use confidential information with the intention to improperly *cause* harm to another person.
- Convert any property of the centre to your own use unless properly authorised
- Use the centre's computer resources to search for, download, access or communicate any material of an offensive, obscene, pornographic, threatening or abusive nature.

You must:

- Protect confidential information,
- Only access information needed for centre business.
- Not use confidential information for any non-official purpose
- Only release confidential information if you have authority to do so.
- Only use confidential information for the purpose it is intended.
- Only release other information in accordance with established Centre policies and procedures and in compliance with relevant legislation.
- Be scrupulous in your use of Centre property and should not permit misuse by any other person or body.

REPORTING BREACHES, COMPLAINT HANDLING PROCEDURES:

Corrupt conduct, maladministration and waste of centre resources

You should report any instances of the above in accordance with Centre reporting policy

Reporting breaches of the code of conduct

You should report suspected breaches of the code of conduct to the Manager, preferably in writing. Where you believe that the Manager has failed to comply with this code, you should report the matter to the Authorised Supervisor or next in charge, preferably in writing. We ask for the breach to be in writing to allow us to clarify the facts of the breach.

Complaint handling procedures- staff conduct (excluding the Manager)

Where appropriate the Manager will make enquiries into breaches of the code of conduct regarding members of staff. Where the Manager has determined not to enquire into the matter, the Manager will give the complainant the reason/s in writing.

Enquiries made into all staff conduct which might give rise to disciplinary action must occur in accordance with the relevant local government award and make provision for procedural fairness.

OUR CODE OF CONDUCT

Rationale

The purpose of the Code of Conduct is to guide and enhance the conduct of our team in performing their duties. The Code applies to all team members, whether full time, part time, casual, students or volunteers. Members having concerns regarding the application or contravention of the Code in a particular situation should raise their concerns with the Approved Provider.

There are fundamental ethical principles from which the ethical obligations contained in the Code of Conduct are derived. The ethical obligations are summarised in this section.

Respect for the law

Team members should observe the laws of the State and the Commonwealth. This obligation is not intended to detract from the concept and practice of academic freedom, which is essential to the proper conduct of teaching.

Our Statement of Commitment to Child Safety and Wellbeing

Our organisation has a strong commitment to child safety and protection, through implementing and adherence to the Child Safe Standards, Information Sharing Scheme and the National Model Code. This commitment extends to all children in our service and community with an aim for children to feel safe, secure and supported. The welfare of the children in our care will always be our first priority and we have a zero-tolerance approach to child abuse and harm. We acknowledge the importance of our organisation and community to be committed to cultural safety of aboriginal children, the cultural safety for culturally and/or linguistically diverse children and safety of children with a disability.

UN Convention on the Rights of the Child

As a service and team of educator we support and abide by the UN Convention on the Rights of the Child. The Convention recognises that children have the same basic human rights as adults, while also needing special protection due to their vulnerability as children.

In 1989, the United Nations established the first legally binding document to protect the rights of children. The United Nations Convention on the Rights of the Child is a collectively agreed set of nonnegotiable standards and obligations to protect children's rights, to help meet their basic needs, and to expand their opportunities to reach their full potential. So far, 194 countries have ratified the convention and brought their legislation, policy and practice to reflect the universally recognised standards. Having ratified the Convention, the Australian Government has committed to making sure every child in Australia has every right under each of the 54 Articles in the Convention.

The advocacy of children's dignity and rights is the foundation from which we operate as early years practitioners. It forms the foundation of both the Early Childhood Australia (ECA) Code of Ethics and the Early Years Learning Framework (EYLF) which guide both best practice and understanding of quality education and care for children.

Respect for persons

Team members should treat students, colleagues and members of the community equitably and with respect. This involves:

- Courtesy and responsiveness in dealing with others
- A responsibility of fairness in supervising other team members
- Making decisions that are procedurally fair to people
- Avoiding discrimination, for example, on grounds such as gender, race, religion
- Avoiding behaviour which might reasonably be perceived as harassing, bullying or intimidating
- A responsibility for fairness and the encouragement of independent scholarly learning in the teaching role

We seek to create an environment where all persons are treated equitably and with respect, where persons' rights are respected and where staff and child, and student efforts are encouraged and their achievements given due recognition.

Integrity

Team members should be honest in carrying out their duties, and avoid conflicts between their private interests and their responsibilities with respect to:

- Personal relationships
- Sexual relationships
- Financial relationships
- Receipt of gifts
- Outside work
- Use of confidential information obtained in the course of their duties
- External activities and public comment

Diligence

Team members should carry out their duties in a professional and conscientious manner. This involves:

- Carrying out official decisions and policies faithfully and impartially
- Seeking to attain the highest possible standards of performance
- Exercising care for others in employment related activities
- Ensuring that personal use of alcohol and other drugs does not interfere with performance in the workplace, or work related activities
- Adhering to professional codes of conduct where applicable

We aim to achieve the highest standards of excellence in education and administration. All team members contribute to the achievement of this aim when they carry out their duties honestly and to the best of their ability.

All team members are expected to carry out their duties in a professional, responsible and conscientious manner, and to be accountable for their official conduct and decisions.

Economy and Efficiency

Team members should use centre/company material and resources only for legitimate educational purposes:

- Team members should avoid waste
- Team members should maintain adequate security over our property, facilities and resources

Team members should familiarise themselves with the Code of Conduct and endeavour to ensure that its principles are observed at all times. Significant departures from the standards of conduct outlined in the Code of Conduct may amount to misconduct on the part of the individual team member.

While the Code of Conduct expresses the professional commitment of staff in their service to our centre, our organisation encourages our team to aspire to the highest standards of conduct. The focus is therefore supportive and educative.

However, the Code of Conduct does regulate staff behaviour and where conduct falls below the standards outlined in the Code of Conduct, team members may be counselled in accordance with normal performance management strategies. Significant departures from outlined behavioural expectations may amount to misconduct on the part of the individual team member and may result in dismissal.

Making Fair Decisions

When making a decision, taking action of a discretionary nature or resolving a grievance which may adversely affect a person's rights, liberties, interests or legitimate expectations, the principles of procedural fairness should be applied. This means that persons affected should have the opportunity to respond to allegations or assertions made and to have a decision made without bias.

Decisions should be based on considerations relevant to the matter at hand. Decisions may be accepted more readily if reasons are given. For example, where an adverse report is received concerning a team member's performance or conduct, that team member should be given the opportunity to respond.

Team members must not discriminate

Team members should treat other persons equitably, irrespective of gender, sexual orientation, race, disability or medical condition, cultural background, religion, marital status, age or political conviction.

Respect for the Opinions of Others

Rational debate presupposes open communication and the freedom to voice alternative points of view. Team members should not cut off rational debate by verbal or physical violence or intimidation. For example, criticism of ideas should be distinguished from personal criticism; and team members should not verbally abuse, vilify or belittle children or other staff (including their supervisors) either personally or to others. Defamatory or inflammatory comments against the Company will result in disciplinary action.

Avoiding Conflicts of Interest

The trust that is placed in our team requires that you conduct yourself with honesty, fairness and propriety. This means that team members should avoid conflicts between their private interests and work responsibilities and should avoid situations where there is a reasonable basis for the perception of such a conflict. The obligations outlined in the following paragraphs detail specific occasions when conflicts of interest might occur.

Personal Relationships

A conflict of interest can arise where a team member makes or participates in decisions affecting another person with whom the team member has a personal relationship (such as near relative, spouse, close friend or personal associate). There are a number of situations in the course of a team member's duties in which a conflict may arise.

Where such a conflict arises in a clear case, a team member should withdraw himself or herself from the situation. Where the relationship is not so close, the team member should discuss the issue with someone in authority.

Sexual Relationships

While it is recognised that team members may sometimes form consensual sexual relationships with other staff, such relationships should not intrude or be seen to intrude on the workplace generally, since workplace practices may be compromised. Such relationships may also create difficulties in maintaining the boundaries between professional and personal life and may compromise the professional or personal standing of either one or both of the persons involved. Difficulties may also arise from the unequal power of the persons concerned. Staff in supervisory positions, in particular, must be alert to the conflicts which such a relationship may create in the workplace, and to the need to handle such situations in a sensitive manner.

Financial Relationships

Financial relationships (for instance, where a team member has a pecuniary interest in a company from which equipment is purchased or services obtained) can also give rise to a conflict of interest. Such a relationship must be disclosed before any question of purchasing or other financial or contractual decisions or dealings may arise. In general, team members should avoid involvement in such transactions.

Acceptance of Outside Work

Our centre permits team members to undertake paid outside work, but it must be recognised that they have a primary commitment to the duties of their appointment and position with the centre. Any use of centre/organisation resources in connection with outside work may only occur where prior approval has been obtained.

In the case of academic work, prior approval must be sought and obtained from the Operations Manager if outside work is undertaken. It is recognised that such work can be beneficial in forging links between our centre and the community.

Appropriate use of company and centre information

Our team members will gain access to information while engaged in caring, teaching, administrative, and technical and support activities. Some information may be unavailable to the community generally or the public. Team members should respect the privacy of others and ensure that information of a personal nature,

for instance, is accessed and used only for the intended purposes and is not disclosed except where company policy expressly permits this.

Information of a commercial or strategic nature should also be handled with the utmost sensitivity, and should not be used for personal gain. For example, team members having computerised access to personal information about others should not access or use that information for private purposes, and where a student, child, or parent confides in a team member, the information should not be discussed with or revealed to others, except where this is necessary to take action or decisions concerning the person who has provided the information.

Public Comment

Public comment by team members in their capacity as private citizens is certainly permitted. In making private comment (including via electronic means such as electronic mail and the Internet), every effort must be made to ensure that the opinions expressed are presented as a private view of an individual and not represented as an official view of the centre/company. *Comment made by a team member in a private capacity should not compromise or be perceived to compromise the team member's ability to carry out his or her duties or implement duly approved Company policy.*

Public comment by a team member representing the company and pertaining to the operations of our centre/company will only be made when approval has been given by the Operations Manager.

Public Comment – internet & social networking sites

As more people are going “online” there has been an increase in the use of social networking sites. Please be aware of the content of your communications on these social networking sites (ie facebook, twitter, etc.).

Remember that these sites can be accessed by numerous people and are therefore NOT confidential. Photos from your workplace and comments relating to specific individuals (colleagues, children and/or families) within your workplace are a breach of confidentiality.

Please also consider the consequences of asking junior team members or team members of the opposite sex to become an online “friend” – could they feel obligated to accept? Could this be construed as harassment, depending upon the information shared on your profile?

All internet usage at the centre may be monitored.

Duty to carry out official policies and decisions

Generally, team members have an obligation to carry out official decisions and policies faithfully and impartially. This obligation applies even though team members have the right to institute grievance procedures where they believe a decision is improper or they seek to have decisions or policy changed through appropriate channels.

Standards of Performance

Team members should endeavour to maintain and enhance their skills and expertise and keep up to date the knowledge associated with their particular field or area of work. High standards of performance and a focus on client service (whether clients are children, parents, students, other staff, and members of the academic or wider community) are expected. Team members should not allow outside work to interfere with the performance of their normal centre/company duties.

Duty to take Care

Team members should exercise due care in undertaking their activities, particularly where others will rely on advice or information offered. Team members have a duty to take reasonable care to avoid causing harm (including physical harm) to anyone. Thus, team members should actively promote safe working practices and environments for everyone using centre/company facilities.

Conflict of Interest Disclosure Form

Please complete this form if you believe that you may be involved in a conflict of interest situation or if you are unsure and seek to disclose a potential or perceived conflict of interest. Please read the *Ethical Conduct and Code to Ethics Policy*.

SECTION 1: PERSONAL DETAILS

NAME: [Click here to enter text.](#)

JOB TITLE: [Click here to enter text.](#)

PHONE: [Click here to enter text.](#) EMAIL: [Click here to enter text.](#)

SECTION 2: DISCLOSURE DETAILS

The actual, potential or perceived conflict of interest relates to: *(tick all appropriate box/s)*

- | | |
|--|---|
| ☐ Relationship with family or friends | ☐ Staff recruitment |
| ☐ Outside work activities (paid/unpaid) | ☐ Relationship with external parties |
| ☐ Financial interest | ☐ Disposal of assets |
| ☐ Gifts/benefits | ☐ Provision of external consultancy services |
| ☐ Procurement of goods and services | ☐ Other (if you selected other please provide details) |

The following actual, potential or perceived conflict of interest has been identified. *(please insert all relevant details)*

[Click here to enter text.](#)

The (actual, potential or perceived) conflict is expected to last: *(tick appropriate box)*

- | | |
|--------------------------------------|--|
| ☐ 0–12 months | ☐ >12 months or ongoing |
|--------------------------------------|--|

SECTION 3: TO BE COMPLETED BY THE EMPLOYER

In my opinion the details provided: *(tick appropriate box)*

- | |
|--|
| ☐ do not constitute a conflict of interest, and I authorise the employee to continue the activity (go to Section 4). |
| ☐ do constitute an actual, potential or perceived conflict of interest (please provide a detailed action plan below). |

If the situation does constitute a conflict of interest, please ensure that the following actions have been considered:

- ensure all information surrounding the conflict has been disclosed and documented

- inform likely affected persons of the conflict, seeking their views where relevant as to whether they object
- reformulate the scope of work or restricting access to certain information
- recruit a third party to oversee part or all of the process
- recommend to relinquish the interest that is causing the conflict
- temporarily remove the person from the process or responsibilities
- monitor the person's activities closely in relation to the conflict of interest
- take no further action because the conflict is minimal.

I have reviewed the above considerations and request that the Employee takes the following action to eliminate/manage the conflict:

[Click here to enter text.](#)

I will ensure this action plan is reviewed:

- | | | |
|---|--|---|
| ☐ Within 1 month | ☐ Within 3 months | ☐ Within 6 months |
| ☐ Within 12 months | ☐ Other – specify | ☐ N/A: the conflict is one-off or short duration |

SECTION 4: EMPLOYEE'S DECLARATION

To the best of my knowledge and belief any actual, perceived or potential conflicts between my duties as an employee and my private and/or business interests have been fully disclosed in this form in accordance with the requirements of the Conflict of Interest Policy.

I acknowledge, and agree to comply with, any approach identified in this form for removing or managing an actual, perceived or potential conflict of interest.

SIGNATURE:

DATE:

SECTION 5: EMPLOYER

The actions described in the approach outlined in Section 3 have been put in place to effectively manage any actual, potential or perceived conflict of interest disclosed in Section 2. The approach outlined in Section 3 ensures that the organisation's public interests and reputation is adequately protected.

NAME: [Click here to enter text.](#)

SIGNATURE:

DATE:

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

Next Policy Review: 15/2/2027

Reference:

NQS

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.

QA7	7.2.3	Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.
	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Early Childhood Australia Code of Ethics
Children, Youth and Families Act 2005
Managing a Child Care Service – a hands-on guide for managers
Handle with Care – A guide to Early Childhood Administration
Children's Services Regulations 2009

71 – FAMILY LAW AND ACCESS POLICY

Position Statement

To assist with the compliance with family law and access advised on the enrolment form.

Objective

To be knowledgeable about parent and guardian legal guardianship and custody rights and obligations effecting the enrolled child. To conduct a comprehensive orientation for new families and children and collate of information.

Definitions

Parental Responsibility – means that each parent/guardian has equal responsibility for their children's welfare, either in the long-term or on a day to day basis and includes matters such as where the children will live and with whom they will have contact. It is not affected by any change in the parents' relationship, for example if they separate or remarry.

Parenting Orders – are orders that the court will make when parents cannot decide on matters themselves. They change parenting responsibilities and stipulate which parent has what responsibilities. There are 4 types of parenting orders:

- Residence – an order to say with whom the child lives, including any shared arrangements
- Contact – an order to say the times that a child may have contact with a parent with whom they are not living, or anyone else who plays an important part in their life, such as a grandparent (contact can either be face to face, or by phone, letters)
- Child Maintenance – an order that provides for financial support of a child
- Specific Issues – an order about any other aspect of parental responsibility (this may include the day-to-day care, welfare and development of a child, issues relating to religion, education, sport, or other specific issue)

Residency

The parent with whom the child lives is responsible for day-to-day decisions like:

- Discipline
- Going out
- Clothes
- Accommodation
- Pocket money

Residency can be a shared arrangement.

Implementation

Parents/guardians, regardless of their marital status, have joint and equal legal responsibilities for their children unless there is a Court Order determining otherwise. Centre staff members need to be knowledgeable of which parent/guardian has specific legal rights and responsibilities. Thus, the Centre will need to access any relevant Court Orders issued. Services are not legally able to allow children to leave the Centre without permission of the custodial parent/guardian.

In the case where guardianship and custody is legally defined, Centre policy must be followed as stated on the enrolment form. When situations change a copy of the Custody Order must be provided to the Centre. Where confrontation situations arise over custody the child will be kept at the Centre, the custodial parent must be contacted without undue delay and if necessary the Police and/or the Regulatory Department (DHS Child Protection).

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/06/2026

Reference:

NQF

QA2	2.3.4	Educators, co-ordinators and staff members are aware of their roles and responsibilities to respond to every child at risk of abuse or neglect
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	84	Awareness of child protection law
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Other References:

Family Law (Shared Parental Responsibility) Act 2006
National Childcare Accreditation Council
Children's Services Regulations 2009.

The above information has been adapted from the Family Court of Australia website 2002 www.familycourt.gov.au

72 – INTERACTIONS WITH CHILDREN POLICY

Position Statement

Children are required to be supervised and interacted with to ensure that they are stimulated, moving and playing in the environment in a safe manner. Interaction of children is a cornerstone of educating young children in an Early Childhood Setting.

Objective

Our centre aims to ensure that all educators interact with the children in a positive way that makes them feel safe and supported in the centre. Educators will encourage children to interact with their peers in a positive way including interaction amongst the age groups.

Implementation

In order to provide an environment where children feel safe and supported and families are happy to leave their children we will:

- Frequently have educators talk with families to get an idea of the non-verbal forms of communication used by their children in order to convey messages such as hunger, needing the toilet, tiredness and emotions.
- Work with a child when he/she displays big emotions. Big emotions can occur as a way of releasing frustration. When this happens educators will work with the child to calm him/her down and then reassure the child. The educator will then if possible offer the child a solution to his/her problem.
- Reassure a child that he/she is safe in the centre. Most toddlers suffer a form of separation anxiety when away from their families. The educator need to reassure the toddler and work with the toddler's family in order to make the child feel safe and happy at the centre.
- Our routines allow for regular opportunities for children to engage in social play and collaborative experiences independently and safely.
- Collaborative conversations with children, that encourage children to have their own opinions, ideas and comments. The educator should support each child's agency and let them know that their ideas are valued and plan further collaborative learning opportunities.
- Build a rapport with the children and their families. This can be achieved with the children by offering them information from your own life that they can relate to. For example, if a child tells you he got a new dog on the weekend, tell them a little about your own pet.
- Allow time to talk to parents about their children. This allows the educator to gain insight into their home life.
- Use positive guidance. When a child is doing the wrong thing, let them know that what they are doing is wrong by telling them the right thing to do "We walk on the pavement" rather than "No running on the pavement."
- Give the children something to look forward to. This enables the children to feel proud of their learning.
- Be attentive to children at all times. In an attempt to prevent attention seeking behaviour the educator will at all times acknowledge each child to the best of their ability.
- In regards to children interacting with each other the educator will encourage children to:
 - Verbalise their feelings and ideas.
 - Listen to each other.
 - Respect each other.
 - Develop strong social skills.
 - Be accepting of other children's race, religion, values and culture.
 - Co-operate with educators and other children.
 - Have good relationships.
 - Share their experiences.
 - Accept routine.
 - Seek help from other children or educators when they're frustrated.
 - Be relaxed and happy.
 - Contribute to group activities.

- Respond to positive discipline.
- Have self confidence.
- Respect other children's space and privacy.

Refer to Supervision Policy for further reference and guidance.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/3/2026

Reference:

NQS

QA5	2.1.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	5.2.1	<i>Children are supported to collaborate, learn from and help each other.</i>
	5.2.2	<i>Each child is supported to regulate their own behaviour, respond appropriately to the behaviour of others and communicate effectively to resolve conflicts.</i>
	5.1.2	<i>The dignity and the rights of every child are maintained.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regs	155	<i>Interactions with children</i>
	156	<i>Relationships in groups</i>

EYLF

LO1	<i>Children feel safe, secure, and supported.</i>
	<i>Children develop their emerging autonomy, inter-dependence, resilience and sense of agency.</i>
	<i>Children develop knowledgeable and confident self identities.</i>
	<i>Children learn to interact in relation to others with care, empathy and respect.</i>
LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation.</i>
	<i>Children respond to diversity with respect.</i>
	<i>Children become aware of fairness.</i>
	<i>Children become socially responsible and show respect for the environment.</i>

Other References:

Children's Services Regulations 2009 <http://www.community.nsw.gov.au/>

73 – LOST CHILD POLICY

Position Statement

Children are required to be actively supervised at all times to ensure that they are moving and playing in the environment in a safe manner. Active supervision of children is the cornerstone of duty of care for team members working with young children in an Early Childhood Setting.

Objective

In the unlikely event that a child becomes lost while at the centre or on an excursion, we endeavour to find the child as quickly as possible as well as follow the procedure as outlined below.

Implementation

To the best of our abilities, staff will act to prevent children becoming lost or going missing. On an excursion, staff will:

- Frequently do head-counts (on arrival, transitions, before & after activities, during breaks, frequent intervals, etc)
- Use a buddy system. Each child will be assigned a buddy and will pair off at frequent times on the excursion. This will involve the children in keeping track of each other.
- Look around any area before moving on to make sure all children are present.
- Instruct older children about what to do if separated from the group.
- Not make the child's name visible to a stranger.
- Take note of any identifying factors of each child. For example, a brightly coloured t-shirt, hair colour etc.

If a child becomes lost while at the centre staff will:

- Conduct a search of the premises including all areas a child could fit into eg. Cupboards etc.
- Check to see if gates were secure.
- Ask children if they have seen the missing child.

If in either situation a child is determined to be lost the following will occur:

- Staff will immediately notify the Director of the Centre and the missing child's family.
- Staff will immediately notify the local police
- If on an excursion, staff will notify the facilities management to assist with locating the child.
- If lost child incident occurs this will be a reportable incident to Regulatory Authority – complete form on the NQS ITS within 24 hours.

Refer to Supervision Policy for further reference and guidance.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 21/7/2026

Reference:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
	2.2.1	<i>At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	168(2)(e)	<i>Policies and procedures in relation to emergency and evacuation</i>
	97	<i>Emergency and evacuation procedures</i>
	98	<i>Telephone or other communication equipment</i>

Other References:

Children's Services Regulations 2009 <http://www.community.nsw.gov.au/>

Childcare and Children's Health Vol 9 – March 2006, sourced 19/05/2009

74 – STAFF TO STAFF INTERACTION POLICY

Position Statement

Staff to Staff Interaction Policy helps to ensure that all team members are interacting effectively. The policy below will be used in an effort to provide strategies for creating a positive environment for all staff.

Objective

A major influence of the atmosphere in a centre is created by the way which staff relate to each other. A major factor in the centre environment is that both children and parents/guardians have a feeling of security and trust coming not only from good Centre communication but most importantly from positive and communicative staff.

Implementation

- All staff will maintain positive, trustworthy and co-operative relationships with their co-workers
- All team members should know, use and refer to service protocols for guidance on communication, shared environments, active supervision, and educators who are also parents. The protocols are displayed at the service and also are provided and discussed in the staff member orientation. The protocols guide daily practices including staff to staff interactions.
- Show respect for all staff and it will be returned.
- Accept differences in staff members, we are a diverse country.
- Show appreciation of each staff's member's background, any unusual skills and unique contributions to the centre will promote the Centre in a positive manner.
- Always look for and support the strengths in staff members and try not to focus on weakness.
- Resources and information will be shared amongst staff members.
- Staff members will offer each other support in meeting their professional development and needs.
- Co-workers will be given due recognition for professional achievements by all staff.
- Policies and working conditions will encourage competence, well-being and self-esteem for all staff.
- Where possible all staff will provide professional support, thoughtful input and resources for other staff members as they may be required through critical reflection and mentoring.
- Respect will be shown for all members of the team and consideration given to their feelings: values and opinions no matter how they may differ from your own.
- Information and observations regarding all of the Centres' children will be shared.
- Information relative to the families of the Centres' children which affects individual children will be shared.

- Staff will treat each other with empathy, respect and courtesy
- Staff will endeavour to develop positive working relationships, which will provide a positive role model of social skills to the children.
- Regular staff meetings and room meetings will provide a forum to collaborate and have group discussions on all matters. Any staff member unable to attend will be updated upon their return and be able to read the meeting minutes and notes.
- Room teams will use short meetings throughout the day to communicate important messages, plan tasks, routines and experiences. (the team refers these meetings as 'minute meetings'.)
- The Staff Diary and Room Diary will be used to communicate messages where shifts make it difficult to convey information face-to-face. These means will ensure all staff are informed on important matters. It is the staff member's responsibility to check the Diaries.
- Staff will consider each other and work as a team in order to share the workload. Decisions concerning children and programs shall utilise the appropriate training, experiences and expertise of each other.
- Staff will be honest and open when resolving differences of opinion or personal conflicts amongst themselves. These will be resolved quickly and always away from the children.
- Staff will approach the staff member directly involved about any grievances you may have with them. Don't complain or gossip to other staff members or parents/guardians – see Grievance Policy.
- Staff are encouraged to give positive feedback and praise to other educator's to acknowledge their performance in their role and duties, completion of curriculum documentation, quality interaction with a child, dealing with a parent issue, discussions with parents, etc.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/9/2025

Reference:

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

QA7	7.2.3	<i>Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

Children's Services Regulations 2009<http://www.community.nsw.gov.au/>
Handle with Care: A Guide to Early Childhood Administration

75 – LOCKDOWN POLICY

Position Statement

In the event of a lockdown event happening, it is vital that the team members involved are able to act effectively to maintain the safety of all those involved.

Objective

The Lockdown Policy aims to ensure the safety of all staff, children, families and other visitors to the Centre in the event of a threat. Also, the Centre aims to minimise the risk of harm or the exposure to danger to anyone on the premises through the implementation of this procedure.

Implementation

Examples of such critical incidents are:

- Death of a child on centre property.
- Children/staff being taken hostage.
- A siege of centre property.
- A disaster in the local community.
- Unusual amounts of media attention.
- Aggressive trespassers.

Initial Notification

If an event takes place that requires a "Lock Down", The following should occur:

- Any staff member who witnesses the event must try to raise an alarm.
- 000 must be called immediately.
- The Director or person in charge will determine the need for a "Lock Down" and raise the appropriate alarm.

Alarm Procedure

- The Centre bell, whistle or horn will sound 5 long blows/sounds. Repeat as walking through the service.
- The Director or person in charge will sound the whistle and make the following announcement.

- | |
|--|
| <ul style="list-style-type: none">• "This is a LOCK DOWN"• "This is not a fire drill"• "Everyone is to stay in the room, remain seated and to keep calm and quiet." |
|--|

Movement of Children and Staff

- Children and Staff will remain in their rooms.
- If possible, staff will make efforts to seal and lock classroom doors and windows.
- Children should remain on area away from windows (babies placed in evacuation cots) or down low and out of sight during the lockdown period. Preferably away from windows and exit doors.
- If children are outside staff should move children straight into their rooms.

Staff Responsibilities:

- Person in charge to contact Police and follow their instructions.
- Staff not involved in the lockdown or without children to supervise are to go to the front of the Centre to liaise with emergency services if it is safe to do so.
- Educators to complete OWNA documentation records:
 1. Evacuation & Head Count (OWNA Tool)
 2. Emergency Evacuation Record Fire or Lockdown (Practice Drill / Real Event): (See OWNA Forms)

- Head Count: Staff must ensure all children are present and complete a head count using the OWNA Head Count Tool (and Wellbeing chart is used). Any absences must be reported to the Director as soon as it is safe.
- Staff are to close and lock all doors and windows, turn the lights off and ensure children are kept below the window level. (The Director/Person in Charge will need to move from room to room around the centre to lock the doors (that require keys) with the Centre Keys. Any Educational Leaders rostered on with a key must use their keys to lock as many doors as possible.)
- All staff, children and anyone else present will remain in the locked room until the All Clear is given by the Director or person in charge.

All Clear Signal

The All-Clear Signal is as follows:

- The nominate supervisor or responsible person in charge will sound the Centre's bell (if applicable) or blow a whistle for 5 seconds.
- The Director or person in charge will then say "The Lock Down has now ended, everyone follow me and the staff in an orderly manner."
- Head Count: After the "All Clear Signal" Staff must ensure all children are present and complete a head count using the OWNA Head Count Tool (and Wellbeing chart is used). Any absences must be reported to the Director as soon as it is safe.
- Emergency Evacuation Record form (OWNA form) will be completed by each room leader to document the Lock down.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 21/7/2026

Reference:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	168(2)(e)	<i>Policies and procedures in relation to emergency and evacuation</i>
	97	<i>Emergency and evacuation procedures</i>
	98	<i>Telephone or other communication equipment</i>

<http://www.community.nsw.gov.au/>

76 – OPEN DOOR POLICY

Position Statement

To enable the Centre to provide the highest quality of care, it is imperative that parents and team members are able to communicate openly and with respect. Parents should feel comfortable in discussing any concerns they may have.

Objective

To provide open communication links between team members and parents, and to encourage parents to actively use these links.

Implementation

O

Our Centre can be accessed at any time for parental inspection.

P

Please come and see how we help your child develop and grow.

E

Entry by you any time shows that we are happy for you to see our practices at any time of the day.

N

Never leave your child in a centre unless you feel 100% competent in our ability to provide for your child.

D

Don't hesitate to ask us any questions about your child, their development or our centre philosophy.

O

Our Centre is proud of the quality of care we provide.

O

Our staff are qualified, trained, experienced and talented.

R

Rather than take our word for this

S

See for yourselves; feel free to call or attend the centre anytime

Families are welcome any time to attend the service. However, if parents wish to have an indepth discussion or meeting with educators to discuss a matter, a meeting time needs to be scheduled. Providing advanced notice for the meeting allows the Centre Director and educator to coordinate a meeting time and may allow the educator to be absent from the classroom.

Please refer to Parent involvement Policy for further guidance.

Related Policies

- 6 – Parent Involvement Policy

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

Reference:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

77 - BUSINESS DEBIT CARDS AND BANKING POLICY

Position Statement

To ensure efficient business practices, this policy provides criteria for the appropriate use of Business Debit Cards and banking practices. It also provides guidelines and approval mechanisms for the use of the debit cards and authorisation requirements.

Objective

The objective of this policy is to:

1. ensure that organisational transactions are carried out as efficiently as possible through the use of debit transaction cards as appropriate.
2. provide guidelines, approval mechanisms and authorisation requirements for the purchase of goods and services.
3. guard against any possible abuse of organisational cards.
4. Provide direction of banking of cash, limits of cash held at service.

Implementation: Debit MasterCard:

1. Card Issue

The Organisational Debit Card may only be issued by a staff member where their functions and duties would be enhanced by the use of an Organisational Debit Cards ('Cards'). Eg MasterCard, Weel, Etc. Cards are only to be issued and used by people who are approved Organisational Cardholders.

Each Organisational Debit Card will be issued to a specific person, who will remain personally accountable for the use of the card.

Each Debit Cardholder has a unique identification number for when purchasing goods and services on the debit card. Cardholders need to sign the attached declaration (Appendix A) before using the Card.

Only the authorised signatory may use the card. No more than one card shall be issued per cardholder. Debit cash limits as appropriate shall be set for each account by the Managing Director. The Debit cash limit will be provided each month.

2. Cardholder's Responsibilities

The Cardholder shall:

- **For MasterCards:** Enter and reconcile the expenses on a weekly basis. Ensuring that all transactions have tax invoices and are reconciled to statemeny on the last business day of the week.

For Weel Debit Cards: In the Weel App, enter tax invoice information and upload a copy of tax invoice after you purchase an item. Reconcile the expenses using the Weel App as you incur the expense and at the latest at the end of each week. Ensuring that all transactions are recorded in the Weel App.

Instructions on how to use Weel Debit Cards

Access the Weel Help Centre on their website: <https://help.letsweel.com/hc/en-us>

- [How to add your card to Apple Wallet](#)
 - [How to add your card to Apple Wallet and Google Pay](#)
 - [How to download the Weel App from the Apple App Store](#)
 - [How do I edit my expense report in the Weel app?](#)
 - [How to complete an expense report](#)
-
- In all cases obtain and retain sufficient supporting documentation to validate the expense (e.g. tax invoice). All expenses must have an original tax invoice to support the purchase.
 - Attach tax invoices to the statement for Mastercard
 - Review the statement for inaccuracies
 - Verify that the goods and services listed were received
 - Sign the statement/Petty Cash Report to verify that transactions have been made for official purposes
 - Forward the papers to the Support Office. Scan both the petty cash report and the corresponding receipts and email to accounts@mychildeic.com.au.
 - Notify the bank and the Managing Director immediately if:
 - The card is lost or stolen.
 - Any unauthorised transaction is detected or suspected
 - Notify the Managing Director of any change in name or contact details
 - Take adequate measures to ensure the security of the card.
 - Return the card to the Managing Director if:
 - the Cardholder resigns
 - the Managing Director determines that there is no longer a need for the cardholder to retain his or her card
 - the Organisational Debit Card has been cancelled by the bank.
 - Be personally liable for any unauthorised transaction unless the card is lost, stolen or subject to fraud on some part of a third party.

The Cardholder shall not:

- exceed any maximum limits set for the Organisational Debit Card
- obtain unapproved cash advances through the Organisational Debit Card
- use the card for any proscribed purchases
- authorise their own expenditure. Authorisation for use of card should be obtained from the Managing Director.
- claim double allowances or double pay invoices (i.e. request reimbursement for an expense already paid by the card or issue invoice to Support Office for payment when already paid).

3. Debit Card Expenditure

The Card will only be used for those activities that are a direct consequence of the cardholders' function within the organisation. The purpose of the Card is to purchase minor and adhoc items of a Petty Cash nature. The Cardholders monthly debit limit is set by the Managing Director and this amount equals the monthly Petty Cash amount for the centre. Confirmation will be provided by the Support Office when the monthly Petty Cash amount has been transferred into the Centre Bank Account (it is expected that this will be transferred during week 1 of each month).

At times, the Managing Director may authorise the Cardholder to use the card to purchase authorised good or services in addition to the monthly debit limit. The Managing Director will confirm when additional funds have been transferred into the account.

Where coincident and/or private expenditure occurs on the same transaction (where, for example, a person incurs a debt for personal telephone calls during a hotel stay) the cardholder must settle the private expense prior to charging the balance on the Organisational Debit Card.

Where doubt exists as to whether or not an item is function-related, prior authorisation should be obtained from the Managing Director.

The use of the Organisational Debit Card for "services of a dubious nature" is expressly prohibited. "Services of a dubious nature" are defined as any goods or services that might bring the name of the organisation into disrepute.

Implementation: Banking and Cash Receipts

- Examples of cash receipts sources:
 - a) Childcare fees
 - b) Promotional Incentives: When the service receives incentive money for selling photos, books, cakes, etc. Please deposit into centre bank account.
 - c) Excursions: If collecting money from parents for excursion, please also deposit funds into the centre bank account. You need to track who has paid and what you have deposited into bank account.
 - d) Uniforms and Badges: We should not be collecting cash for Uniform and badges as we take payment from payroll deduction authority. If you do for some reason.see procedure below.
 - e) Other: All other money received of any nature please deposited into centre bank account.
- All cash needs to be banked into centre bank account when greater than \$100 and if less than \$100 banked by the end of the week. Cash received for a centre related activities should only be handed by Centre Director or Assistant Centre Director.
- Service cash receipts should be issued for all cash received (using service receipt book).
- In accordance with our centre when the centre is performing fund raising for charities; it needs to be disclosed to everyone that the service is raising funds for charity and who the money is going to be donated to. It is preferred that before funds are raised that the charity is selected and advised to everyone where the donation is going. Please note we are not allowed to provide individual donation receipts. The fund raised will be donated by the centres. See centre policy for further information.

Banking Procedures:

- Per our policy we are not to keep any cash on premises. All cash needs to be banked when greater than \$100 and if less than \$100 bank by the end of the week.
- Suppliers should only make cheques or make direct deposits to the centres bank account and name only. Not other bank account or name.
- When banking money into centre bank account email Accounts Department at accounts@mychildelc.com.au the bank deposit slip. On receipt of this information, reason for deposit and deposit of funds into centre bank account; Accounts Department will allocate funds in the MYOB financial account for the relevant centre ie Centre fund raising account. This allows Accounts Department to keep track of the money

deposited from fund raising for each centre. The organisation does not classified this money as the business money but for tax purposes we need to account for all money in and out of the centres.

- Please deposit parent fees separate from fund raising money, promotional incentives or excursions money collected.
- If you need to do the following:
- Paid for excursion: Send account department a reconciliation to quote of funds deposited for what excursion (ie if a number of separate deposits have been made). Accounts Dept will make payment directly via bank deposit or cheque to supplier (for example: bus company, shows, etc.). A receipt will be obtain by Accounts Department for payment made to supplier.
 - a) **Payments to Charity:** Send Accounts Department a reconciliation of funds deposited from charity fund raising. Advise Accounts Department what charity the service is wishing to donate to. Accounts Department will make payment directly via bank and cheque for donation to register charity. A receipt will be obtained for donation made to charity.
 - b) **Promotional incentative payments from suppliers:** Send Accounts Department an email advising of funds deposited for promotional incentive payments. When the service decides what they want to spent money received from supplier for promotional incentive payment. Generally, money received for this nature should be applied ot benefit the service as a whole or promote learning, wellbeing or care. Or it can be donated to a charity. Obtain a tax invoice for purchase or quote, reconcile to money collected, provide to Accounts Department and he/she can make payment. Please note you do not need to spend all at once, if you spend part the balance will remain.
 - c) **Uniforms & Badges:** Provide uniform and badge order form and indicate on the form payment is made in cash. Attached bank depsit and scan/email to accounts@mychildelc.com.au . Advise Accounts Department of payment and nature (breakdown).

Debit Card and Banking Misconduct

Wherever a breach in this policy occurs, the Managing Director must assess the nature of the breach and if significant report the breach to the police for criminal investigation or if lesser in nature, institute an appropriate disciplinary process. Misconduct associated with the inappropriate use of Organisation Debit MasterCard is considered to be serious in nature.

Related Policies:

- 61 – Fundraising Policy

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Reference:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Bizmanualz – Human Resources: Policies, Procedures & Forms

APPENDIX A

DEBIT MASTERCARD ISSUE FORM

Organisation Name Great Start Early Learning Centres Pty Ltd

Centre Name

Cardholder Name

Position

I understand and agree that the Organisational Debit Card is issued to me on the express understanding that I will, at all times, comply with the following conditions.

USE

1. The Organisational Debit Card ('the Card') is the property of Great Start Early Learning Centres and is in my possession and under my strict control.
2. I will not permit the Card to be used by any person other than myself.
3. I will only use the Card for official and approved purposes in accordance with the organisations' policies.
4. I will immediately report any unauthorised use of the Card to the bank and the Managing Director.
5. I will not use the Card to pay for expenses that have already been claimed (or will be claimed or pay for on account).

TRANSACTION RECONCILIATION

I will be issued with a periodic statement by the Support Office. I will:

- Reconcile the Debit MasterCard using Petty Cash Schedule; entering reimbursements and receipts.
- Cash withdrawn from Debit MasterCard will appear Reimbursement in Petty Cash. Named: "Reimbursement – MasterCard". This is different from Reimbursements entered from Parent Fees Cash Payments. Named: "Reimburse". This needs to be entered on the day the withdrawal or reimbursement occurred. Receipts from purchases need to be entered on the day and date you incur the expense.
- Reimbursement from Parent Fees Cash Payments should NOT be completed without prior authorisation from the Managing Director. Our reimbursement preference is the use of the Debit MasterCard.
- All transaction (Reimbursements and Receipts) need to be reconciled by the last business day of each week.
- Ensure that all transactions that appear on the Petty Cash Reports and bank statement are verified by me.
- Ensure that sufficient supporting documentation is attached to the petty cash report and statement when it is submitted for approval.
- Certify that goods or services paid for using the Card have been actually delivered or provided.
- Sign the Petty Cash Report and statement provided to indicate that the transactions appearing on the statement have been made only for official purposes.

CHANGE IN CARDHOLDER DETAILS

1. I will immediately advise the Managing Director of any change in my name or contact details.
2. I will not request any account changes with the Commonwealth Bank of Australia without prior authorisation from the Managing Director.

UPON RESIGNATION OR TRANSFER

I will immediately return my Card to the Managing Director if I resign or retire, or if my services as an employee of the organisation are otherwise terminated, or if I am instructed to do so by the Managing Director.

LOST OR STOLEN CARDS

I will immediately report the loss or theft of my Card to the bank and the Managing Director.

DECLARATION

I confirm that I have access to only one Card.

I acknowledge that I have been briefed on all aspects of the operation and use of the Card. Please refer to the instruction manual and letter provided by CBA.

Cardholder Signature

Signature of Authorised Card Issuer

Date / /

78 – GOVERNANCE OF SERVICE, MANAGEMENT AND EDUCATOR POLICY

Position Statement

The safety, rights and best interests of all children is our paramount consideration in all decisions and actions taken when providing care and education for children (the paramount consideration). The rights and best interests of each child prevail over all our other interests or obligations.

To enable the Centre to provide the highest quality of care, it is imperative that team member are able to communicate openly and with respect. All team members need to know what the company's expectations of them are and where they can find the relevant information relating to their employment.

We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations, including financial interests or other obligations of management, and are embedded in our daily practices, policies and procedures.

Objective

Our Service aims to ensure that positive working relationships are formed between all educators and management. Educators and management will at all times conduct themselves in an ethical manner and strive to make all interactions positive and compliant with the Service's philosophy.

Related Policies

Service Philosophy

Statement of Commitment to Child Safety

7 -Complaints and Grievance Policy

8 – Families and Visitors Code of Conduct

10 - Occupational Health, Safety & Wellbeing Policy

12A - Child Safe Standards Policy:

12B - Child Safety, Protection & Wellbeing Policy

16 - Privacy and Confidentiality Policy

Physical Environment (Workplace Safety, Learning and Administration) Policy

55 - Student and Volunteers Policy

70 - Ethical Conduct and Code of Ethics Policy

79 - Staffing Arrangements Policy

82 - National Quality Framework Policy

83 - Policy and Procedure Review Policy

84 - Record Keeping and Retention Policy

88 - Child Safe Environment Policy

100 – National Model Code Policy

Implementation

ORGANISATION STRUCTURE & STAKEHOLDERS

This Service is privately owned and caters for children 0-6 years of age and is governed by the Australian Children's Education & Care Quality Authority (ACECQA).

Please refer Service Approval notice issued by the Early Childhood Regulatory Authority to authorise and advise of the service information including; the name of the education and care services, the location, the service approval number, name of the approved provider the maximum number of children the service is approved to provide education and care to, and service waivers. We are open 52 weeks per year and are only closed on gazetted public holidays.

Please refer to the foyer notice providing organisation structure information for the service. The following provide a summary of the organisational stakeholders for governance framework:

Approved Provider - Great Start Early Learning Centres Pty Ltd ATF Great Start Unit Trust

The approved provider has a range of responsibilities prescribed in the Education and Care Services National Law and Regulations, including keeping accurate records and retaining them for specified timeframes.

Our approved provider is also responsible for:

- ensuring the financial viability of the service;
- overseeing control and accountability systems; and
- supporting the Nominated Supervisor / responsible person in their role and providing resources as appropriate for the effective running of the service.

Responsible Persons include:

- The Approved Provider or a person with management or control.
- A nominated supervisors
- A person in day-to-day charge of the service.

Scott Zannes - Managing Director and Approved Provider Director

As Managing Director of the Approved Provider company he is responsible for supporting the service team in order to provide exceptional care in our Service, set strategic direction and ensure efficiency within our Service operations. The managing director is responsible for the duties of the Approved Provider including the duties of compliance and governance of the organisation. The Managing Director also liaises with parents and the community as a representative of our business.

The Approved Provider must ensure that a responsible person is present at all times the service is educating and caring for children.

Centre Director – Nominated Supervisor (and Responsible Person)

As Service Director she is to work directly with the staff and families ensuring quality care is being provided by the staff as well as supporting staff in their daily operations. The Director is also available at all times to the families in our service for both formal and informal discussions on the Service's operations and their children. The nominated supervisor will undertake child protection training and hold a current WWCC at all times. The name of each nominated supervisor must be displayed so that it is visible from the service's main entrance.

A nominated supervisor must:

- Be 18 years or older.
- Comply with all the Responsible Person requirements.
- Nominated Supervisors and people in day-to-day charge have successfully completed child protection training.
- Have adequate knowledge and understanding of the provisions of education and care to children.
- Have an ability to effectively supervise and manage an education and care service.
- Assist in the appointment a Responsible Person – Person in Day-to-Day Charge in their absence. At all times a Responsible Person needs to be assigned to this role.

Nominated Supervisor Responsibilities

As the persons responsible for the day-to-day management of an approved service, nominated supervisors have a range of responsibilities under the National Law and National Regulations including:

Educational programs

- ensuring educational programs are:
- based on and delivered in accordance with an approved learning framework • based on the developmental needs, interests and experiences of each child
- designed to take into account the individual differences of each child (section 168)

Supervision and safety of children

- ensuring children are adequately supervised, are not subject to inappropriate discipline, and are protected from harms and hazards (sections 165-167)

Entry to and exit from the premises

- ensuring children do not leave the education and care service premises except in accordance with the National Regulations (for example, with a parent, on an authorised excursion, or for emergency medical treatment)
- ensuring that a parent of a child being educated and cared for by the service may enter the service premises at any time when the child is being educated and cared for by the service—except when:
 - permitting entry would pose a risk to the safety of the children and staff or conflict with the duty of the supervisor under the National Regulations, or
 - the supervisor is aware the parent is prohibited by a court order from having contact with the child (regulation 99)
- ensuring an unauthorised person (as defined in the National Law) is not at the service while children are present unless the person is under direct supervision (section 170)

Food and beverages

- ensuring adequate health and hygiene practices and safe practices for handling, preparing and storing food are implemented at the service to minimise risks to children (regulation 77)
- ensuring children being cared for by the service have access to safe drinking water at all times and are offered food and beverages on a regular basis throughout the day (regulation 78)
- ensuring that, where food and beverages are supplied by the service, they are:
 - nutritious and adequate in quantity
 - chosen with regard to the dietary requirements of individual children (regulation 79)
- ensuring that, where food and beverages are provided by the service, a weekly menu that accurately describes the food and beverages to be provided is displayed at the premises in a location accessible to parents (regulation 80)

Administration of medication

- ensuring that medication is not administered to a child being cared for by the service unless the administration is authorised (except in the case of anaphylaxis or asthma emergency) and is administered in accordance with the National Regulations (regulations 93-96)
- where medication is administered to a child without authorisation in a case of an anaphylaxis or asthma emergency, ensuring that a parent of the child and emergency services are notified as soon as practicable (regulation 94) Prescription and non-prescription drugs and alcohol
- that while educating and caring for children at the service, all staff must not consume alcohol or be affected by alcohol or drugs (including prescription medication) so as to impair their capacity to supervise or provide education and care to children (regulation 83) Sleep and rest
- taking reasonable steps to ensure that the needs for sleep and rest of children are met, having regard to the ages, development stages and individual needs of children (regulation 81)

Excursions

- ensuring that a risk assessment is conducted before an excursion in accordance with the National Regulations (regulations 100-101), and specifically that the risk assessment is conducted before authorisation is sought to take a child on the excursion (regulation 102)

Staffing

- ensuring the prescribed educator to child ratios are met and each educator at the service meets the qualification requirements relevant to the educator's role (regulations 123 - 128)

Responsible Persons – Person in Day-to-Day Charge

The Approved Provider and Nominated Supervisor are able to appoint a responsible person - person in day to day charge of the service. The person holding this role must provide written consent. The nominated supervision and approved provider will assess the person for the role and have the Responsible Person complete the declaration using the Staff Records Appendix - *Documentation for the appointment of Nominated Supervision, Responsible Person or Education Leader. This form*

needs to be read in conjunction with 'Staff Record'. This is filed in the Staff Record file and also uploaded in the OWNA Staff Records.

Educators appointed to this role and are fit and proper to hold a position of responsible person in the absence of the nominated supervisor. The person in day-to-day charge is the point of contact for parents and staff in the absence of the nominated supervisor. If the Nominated Supervisor is absent a Responsible Person will be appointed for the "Responsible Person – Person in Day-to-Day Charge". This will be represented on the staffing roster and also displayed in the main entrance of the service.

A "Responsible Person – Person in Day-to-Day Charge" register needs to be maintained to record the person appointed to this role through the daily opening hours of the service. This schedule should be archived and scanned and emailed to compliance@mychildelc.com.au.

The Responsible Person – Person in Day-to-Day Charge will also be selected in the OWNA Staff Profile. Tick the "Responsible Person" field. Then select the Responsible Person Order (1 – High-est)

Setting priority in OWNA:

Centre Director / Nominated Supervisor – priority 1

Assistant Centre Director – priority 2

Opening and closing – priority 3

The screenshot shows a portion of the OWNA Staff Profile form. On the left, there are labels for 'Responsible Person', 'Responsible Person Order (1 - highest)', 'Medical Qualification(s)', 'Profile', and 'Profile Image'. The 'Responsible Person' dropdown menu is open, showing 'Director' and 'Assistant Director'. Below it, the 'Responsible Person Order' dropdown is also open, showing a list of priorities from 1 to 6. 'Priority 3' is selected and highlighted in blue. The 'Medical Qualification(s)' field is empty. The 'Profile' field is empty. The 'Profile Image' field is empty.

Minimum Requirement of a person in day-to-day charge:

- Must be 18 years of older.
- Nominated Supervisors and people in day-to-day charge have successfully completed child protection training.
- Provide written consent of accepting the role of responsible person – person in day-to-day charge.
- As an Approved Provider or the as Nominated Supervisor reasonable steps must be undertaken to ensure the person:
 - a) Has adequate knowledge and understanding of the provision of education and care to children
 - b) Has an ability to effectively supervise and manage an education and care service.

Approved Provider and Nominated Supervisor in relation to the Responsible Person must have regard to:

- the person's history of compliance with the National Law and other relevant laws
- any decision under the Law to refuse, suspend, refuse to renew, or cancel a licence, approval, registration, certification or other authorisation granted to the person under the National Law and other relevant laws
- Approved providers can ask the nominated person to complete and sign a Compliance History Statement template and then keep it on file.

Designed Educational Leader

The role of the educational leader is primarily to:

- collaborate with educators and provide curriculum direction and guidance
- support educators to effectively implement the cycle of planning to enhance programs and practices
- lead the development and implementation of an effective educational program in the service
- ensure that children's learning and development are guided by the learning outcomes of the approved learning frameworks.

The educational leader also has a significant role in:

- guiding and developing educators and families' understandings about play and leisure-based learning, and the significance of the early years in the education continuum for children
- building the knowledge, skills and professionalism of educators
- building a culture of professional inquiry with educators, coordinators and staff members to develop professional knowledge, reflect on practice and generate new ideas.

Educational Leader Strategies

Pedagogical leadership – leading the development and implementation of the educational program and assessment, and the planning cycle – requires a unique set of professional skills. The most effective educational leader views their role as a collegial, joint endeavour, enacting their role through mentoring, guiding and supporting educators.

Strategies might include:

- mentoring and supporting educators' understanding of educational program and practice, e.g. building relationships and interactions with children to assist their learning through play and leisure-based programs; developing documentation that is meaningful, relevant and promotes reflection on educators' pedagogy and practice
- drawing on educators' strengths to develop professional skills and confidence
- mentoring and supporting educators to assess learning
- ensuring the assessment and planning cycle is applied to each child and the program as a whole (Note: Some jurisdictions have different requirements for the program in school age services)
- assisting educators to understand and implement reflective practice and leading critical reflection discussions to examine policies, programs and practice
- talking to families about the educational program.

See Job Description for more details of role and responsibilities.

Legislative Requirements

Education and Care Services National Regulations Regulation 118: requires the approved provider to designate, in writing, a suitably qualified and experienced educator, coordinator or other individual as educational leader at the service to lead the development and implementation of educational programs in the service.

Regulation 148: requires the staff record includes the name of the person designated as the educational leader in accordance with Regulation 118.

Educational Room Leader & Educational Room Leader Committee

Within the service we have a committee of Educational Room Leaders. The Educational Room Leaders are room leaders within the service. Our Educational Leaders mentor, supervise and direct and engage both team members and children in relation the educational programs and care routines.

See Job Description for more details of role and responsibilities.

Team Members

Other team members include Assistant Centre Director, Kindergarten Teachers, Qualified Educators, Assistant Educators, Cooks, Cleaners and Maintenance staff.

See Job Description for more details of role and responsibilities.

Parent & Community Consultative Group

The parent and community consultative group are a volunteers parents who's children attend the service. The parent consultative group provide the service access to a group of key stakeholders to seek their opinion in relation to specific operational matters.

Responsible Person Register

Nominated Supervisors and Responsible Persons staff members are required to record service attendance times and listed prescribed staff information on the Responsible Person Register Schedule. The completion of the schedule is required to be completed upon commencement and end of shifts, breaks and all absences from the service. On absence from the service the leaving Responsible notifies the Responsible Person taking over duties. The Responsible Person Register Schedule is required to be completed to record the change of responsible person at the time of change occurred. The responsible person register can be found in OWNA

External Regulatory Authorities are:

Victorian Early Childhood Regulatory Authority (VECRA) - regulates education and care services

VECRA regulates education and care services and is responsible for ensuring the safety, health and wellbeing of children in Early Childhood Services. VECRA has transitioned from the Department of Education to become an independent statutory authority.

VECRA is responsible for the regulation early childhood education and care services, as required by the Education and Care Services National Law Act 2010 and Children's Services Act 1996 and the respective regulations.

VECRA is also responsible for the assessment of early childhood education and care services against the National Quality Standard and ensuring information is made available to families and communities about the quality of service provided. In its regulatory role, VECRA responds to notifications and complaints, monitors compliance, grants approvals, determines administrative and statutory sanctions and publishes appropriate public advice.

VECRA is comprised of staff in both central office and regional area locations.

Social Services Regulator

The Social Services Regulator (SSR) is an independent statutory authority that exists to safeguard the rights of children and young people as well as people who use social services in Victoria.

The SSR regulates the [Social Services Standards](#) and the [Child Safe Standards](#) across a range of settings. It also administers the [Worker and Carer Exclusion Scheme \(WCES\)](#), [Reportable Conduct Scheme \(RCS\)](#), the [Working with Children Check \(WWCC\) scheme](#) and the [NDIS check](#).

The SSR takes an integrated and intelligence-driven approach to identifying, preventing and excluding unsafe people from working with children and young people, as well as ensuring organisations that work with children and young people keep children safe from abuse and harm by meeting the Child Safe Standards.

As a co-regulator of the Child Safe Standards, the SSR works with other co-regulators of the Child Safe Standards including the Department of Health, Victorian Registration and Qualifications Authority (VRQA), Victorian Early Childhood Regulation Authority (VECRA), and Workforce Inspectorate Victoria to keep children safe from harm and abuse.

Australian Children's Education & Care Quality Authority (ACECQA)

Their primary role is to guide the administration of the National Quality Framework to ensure consistent implementation across all states and territories. The National Quality Framework creates a jointly governed uniform national system aimed at improving quality and reducing administrative burden for education and care services.

Child Protection Services

DFFH Child Protection is the lead agency responsible for the care and protection of children and may bring cases before the Children's Court and support orders granted by the Children's Court.

Child Protection Services is a department within the State Government Body (E.g. Victoria: Department of Families, Fairness and Housing (DFFH)) with the objective specifically targeted to those children and young people at risk of harm or where families are unable or unwilling to protect them. The main functions of child protection are to:

- investigate matters where it is alleged that a child is at risk of harm
- refer children and families to services that assist in providing the ongoing safety and wellbeing of children
- take matters before the Children's Court if the child's safety cannot be ensured within the family
- supervise children on legal orders granted by the Children's Court
- provide and fund accommodation services, specialist support services, and adoption and permanent care to children and adolescents in need

Mandatory Reporting to Child Protection Services: People such as teachers, educators, doctors, nurses, police are legally obliged to report suspected child abuse. In addition, any person who believes on reasonable grounds that a child needs protection can make a report to the Child Protection Service. It is the Child Protection worker's job to assess and, where necessary, further investigate if a child or young person is at risk of harm.

COMMITMENT TO GOOD GOVERNANCE

Our service has adopted the following eight Governance Principles and Recommendations, which we recognise as suitable for our business -

1. Lay solid foundations for management and oversight.

Management Principles

To ensure our working relationships are characterised by open and respectful communication, accountability and trust our service adheres to the following management principles.

A. Management by Agreement

Nominated Supervisors and educators agree to produce outcomes together. Educators agree on their accountabilities and to work according to existing procedures and policies. Nominated Supervisors agree to provide educators with training, resources and support.

B. Management by Exception

Once a system is in place or the Nominated Supervisor and educators have agreed upon a course of action, the educator is accountable for identifying and reporting whenever something significant occurs that isn't part of the plan.

C. Clearly Defined Reporting Relationships

Everyone in the Service has only one primary manager. This reduces confusion and increase accountability and transparency.

Information, requests, or delegations that would cause our educators/staff to take action or change the course of their actions will only come from the person to whom they report.

Our reporting relationships are:

- Owners who work in the Service will act according to the reporting relationships applicable to those positions.
- The Nominated Supervisor reports to the Approved Provider.
- The responsible person in day to day charge of the service reports to the Nominated Supervisor.
- Each Educational Room Leader reports to the Nominated Supervisor, Assistant Centre Director and the Designed Educational Leader.

- The Nominated Supervisor has the authority to communicate information about the work and to direct the activities of the Room Leader.
- Educators in the room(s) report to the Educational Room Leader.
- The Parent Consultative Group will have direct relationship with the Nominated Supervisor.

D. Guidelines for Effective Delegation

Our service will:

- identify the work/result to delegate and to whom
Educators/staff will not delegate responsibilities for which they are accountable or work/results that have been delegated to them with their agreement or work/results attached to someone else's position (unless that person has agreed).
- put the delegation in writing with a clear due date
- discuss the delegation with the educator/staff member whenever possible
- get the educator/staff member's agreement (for example through signed job descriptions)

The person who delegates remains accountable for making sure the right result is achieved.

E. Guidelines for Effective Regulation

Regulating work means monitoring, reviewing, and adjusting it to get the right result.

Our service will:

- regularly review the work process
- give quick, clear, and direct feedback and instruction that is timely and specific
- communicate in writing
- avoid under-regulating, over-regulating and unnecessary meetings.

2. Structure the management team to add value

To comply with these principles to the best of our ability and to ensure we can discuss issues and (potential) changes to policies, procedures or the regulatory environment, we will schedule regular communication between all members of our management team through meetings, phone communication including a communication book, written communication such as letters, notices, and electronic communication including email.

3. Promote ethical and responsible decision-making

Our service will make decisions which are consistent with our policies, our obligations and requirements under the national education and care law and regulations, our approved learning framework (EYLF) and the ethical standards in our code of conduct.

4. Safeguard integrity in financial reporting

Our financial records will be reviewed by an independent accountant.

5. Make timely and balanced disclosure

Unless there is a risk to the health, safety or wellbeing of a child enrolled at the service, our service will provide at least 14 days notice before making any change to a policy/procedure that may have a significant impact on our provision of education and care or a family's ability to utilise our service, including making any change that will affect the fees charged or the way fees are collected.

Our service will also:

- advise the regulatory authority of any required notifications including any change to the person designated as the Nominated Supervisor no later than 14 days after the change

- develop a Quality Improvement Plan that is completed regularly, available on request and ready for submission to the Regulatory Authority when requested.

6. Respect the rights of shareholders, parents, children

Our service will support and encourage the involvement of parents and families by:

- developing and implementing plans to ensure regular communication with families including advice about events, activities and policy updates
- enabling them to have access and provide input to reviews of policies and procedures
- providing space for private consultations
- providing and displaying a range of information about relevant issues
- ensuring we follow all policies and procedures including the Parental interaction and Involvement Policy and Privacy and Confidentiality Policy.

Our service will respect the rights of children by ensuring:

- the Nominated Supervisor complies with their responsibilities under the national law and regulations
- we follow our policies and procedures including the Relationships with Children Policy, Child Safety, Protection & Wellbeing Policy and Privacy and Confidentiality Policy.
- our children are provided with the experiences and learning which allows them to develop their identities, wellbeing and social connection.

7. Recognise and manage risk

Our service will take every reasonable precaution to protect children from harm and any hazard likely to cause injury. We will follow service policies including those covering Workplace Health and Safety, Child Protection, Excursions and the Delivery and Collection of Children and complete regular risk assessments and safety checks.

8. Remunerate fairly and responsibly

ETHICAL CODE OF CONDUCT

Each educator, committee member, educator, volunteer and student at the service will uphold the following core ethical conduct principles to positively promote interactions within the service and the local community. The service upholds and supports the following:

- Early Childhood Australia – Code of Ethics (2016)
- Our Service's Code of Conduct - Statement of Values and Beliefs

Please refer to policy “Ethical Conduct and Code of Ethics Policy – 70”.

Our service has a Families and Visitor Code of Conduct – 8.

CHILD SAFETY IS EMBEDDED IN OUR GOVERNANCE, LEADERSHIP AND CULTURE

- Child safety is at the core of our governance, leadership and culture.
- We will promote our Statement of Commitment to Child Safety, and champion and model a child safe culture at all levels in our service.
- Staff must follow our Child Safe Standard Policy (12A) and Child Protection and Wellbeing Policy (12B) and uphold our Child Safe Code of Conduct at all times.
- The approved provider and nominated supervisor oversees the policies, procedures, risk management, supervision, HR processes, training, reporting, communication and monitoring systems we have in place to keep children safe from harm or the risk of harm.
- The approved provider and nominated supervisor will regularly review and report on our performance in child safety matters as part of our continuous improvement practices. Child

safety is a regular agenda item at staff meetings to support the embedment of child safety in our organisation.

EDUCATOR INTERACTIONS

- All educators will maintain positive, trustworthy and co-operative relationships with their co-workers
- Show respect for all educators and it will be returned.
- Accept differences in educators, we are a diverse country.
- Show appreciation of each educator's background, any unusual skills and unique contributions to the service will promote the service in a positive manner.
- Always look for and support the strengths in educators and try not to focus on weakness.
- Resources and information will be shared amongst educators.
- Educators will offer each other support in meeting their professional development and needs.
- Co-workers will be given due recognition for professional achievements by all educators.
- Policies and working conditions will encourage competence, well being and self esteem for all educators.
- Where possible all educators will provide professional support, thoughtful input and resources for other educators as they may be required.
- Respect will be shown for all members of the team and consideration given to their feelings, values and opinions no matter how they may differ from your own.
- Information and observations regarding all of the Services' children will be shared.
- Information relative to the families of the Services' children which affects individual children will be shared.
- Educators will treat each other with empathy, respect and courtesy
- Educators will endeavour to develop positive working relationships, which will provide a positive role model of social skills to the children.
- Regular educators meetings will provide a forum for group discussions on all matters relating to educators problems. Any educator unable to attend will be updated upon their return.
- The Educators Diary and Room Diary will be used to communicate messages where shifts make it difficult to convey information face-to-face. These means will ensure all educators are informed on important matters. It is the educator's responsibility to check the Diaries.
- Educators will consider each other and work as a team in order to share the workload. Decisions concerning children and programs shall utilise the appropriate training, experiences and expertise of each other.
- Educators will be honest and open when resolving differences of opinion or personal conflicts amongst themselves. These will be resolved quickly and always away from the children.
- Educators will approach the educator directly involved about any grievances you may have with them. Don't complain or gossip to other educators or parents/guardians – see Grievance Policy.

MANAGEMENT INTERACTIONS AND RESPONSIBILITIES

In our service, Management holds responsibility for:

- Ensuring the service runs smoothly.
- Supporting the Nominated Supervisor in their role.
- Keeping all service families up to date with issues in the service.
- Selecting new educators and assisting all educators.
- Following policies and making sure all educators are following policies and procedures.
- Developing the service policies with the Nominated Supervisor.
- Ensuring educator ratio and qualification requirements are met.
- Financially running the service and reporting any financial problems to the Nominated Supervisor.

To allow effective communication to take place between educators and the management, different methods of communication must be made available. These include:

- Verbal communication in person.
- Phone Communication including SMS messaging if appropriate.
- Via a communication book.
- Educator meetings.
- Via other forms of written word such as letters, notices, emails etc.
- Educator appraisals and reviews.
- Online communication: Email, zoom, messenger.

PROFESSIONAL DEVELOPMENT GUIDELINES

We endeavour to employ caring, loyal and capable educators who bring a high skill level, appropriate qualifications and a wide and varying amount of experiences to help implement our service's philosophy.

Management and the Nominated Supervisor will ensure that all educators have:

- The correct qualifications to care for children as per the service's Educator Arrangements Policy.
- An understanding of their responsibilities under the law.
- The appropriate personality to care for children.

New educators will undergo an appraisal process, as will all educators. They will be introduced to this process during orientation. After 3 months at the service, the educator will undergo appraisal to ensure they are aware of their duties and responsibilities and from then on every 12 months a formal review and mid year appraisals.

The appraisal meeting will be linked to the educator's job description and will include the following:

- Appraisal for the employee's job description.
- Clarification of the educator member's job role and its expectations.
- Self assessment.
- Two way feedback.
- A discussion of future opportunities within the position.
- A discussion on an action plan for further training.
- Feedback about how the appraisal process could be improved.

The appraisal process will be used as a tool for identifying any need for further professional development and training. Also, the appraisal process is the best way to show evidence of continued poor work performance and allows formal written warning to occur if necessary.

Training will be provided in one of the following formats:

- Shared experiences by all educators.
- An outside presenter runs a workshop.
- Educators attend external workshops, seminars etc.
- Educators complete short TAFE, college or University courses.
- Educators learn through changes in their position at the service.
- Educator and management exchanges between services.
- Appropriate resources (books, movies, documentaries etc).

Educators need to respect the knowledge, experiences and skills of all educators. It is important not to criticise each other but rather work together so the services runs to the best of each educators' combined abilities.

Specific Course Requirements

- All educators must hold a current first aid certificate which is renewed when it becomes necessary.

- Educators must attend Child Protection Training and Development.

Recognising the Diverse Skills of Educators

- The service will actively celebrate the diverse skills and achievements of our educators using the following methods
 - The Nominated Supervisor will use Educator Meetings as an opportunity to provide verbal feedback to all educators.
 - Regular verbal communication and appraisal will occur from the Nominated Supervisor and Room Leaders with all educators.
 - Positive reinforcement of the achievements of educators will be shared with families and the service community through notices and newsletters.
 - The service will inform the service community about the professional development, training and qualifications of educators throughout their time at the service.

EDUCATOR STRESS MANAGEMENT GUIDELINES

If an educator feels stressed in any way they should firstly -

- Approach the Nominated Supervisor and talk together to see if the situation can be remedied in any way.
- If the educator feels they are unable to approach the Nominated Supervisor then the educator can approach their team leader or the owner.
- Accept opportunities to have stress alleviated.
- Accept opportunities for counselling if recommended.

In relation to educator stress management, management should -

- Monitor and review the effectiveness of educator stress management policies.
- Monitor workloads to ensure educator are not overloaded or overwhelmed.
- Monitor overtime hours and regular working hours to ensure educator are not overworked.
- Monitor holidays to ensure educators are taking, or at least aware of, their entitlements.
- Ensure that bullying and harassment is not taking place.
- Be vigilant for educators suffering personal stress, e.g. a death in the family or separation and offer additional support.
- Raise any issues in a sensitive manner if management suspects a educator is suffering stress.
- Refer educator to counselling as required. Refer employee to Employee Assistant Program(EAP).

In relation to stress leave management the following should occur -

- When a member of educator takes stress leave, management will identify the cause of this stress and discuss viable options with the educator.
- Management will work with the educator to set up a return to work plan.
- After the educator returns to work, management will continue to monitor and discuss with the educator their recovery and happiness in the workplace.

EDUCATOR MEETINGS

The service will hold periodic educator meetings. This will take place after hours so all educators can attend and that adult/child care ratio needed during hours is not jeopardised. We appreciate educators and staff giving their time to attend meeting outside work hours. There are number of Educator Meetings agendas; Centre Educator Meeting, Team Leader Educator Meetings and Classroom Educator Meetings. Each of the meetings have different agenda to communicate and discuss items relating to the operations of the Service.

Meetings will follow this structure:

- They will run for the agenda items to be discussed.

- The Nominated Supervisor (or other person) will chair the meeting and give a report. At times the Nominated Supervisor may delegate the chair to another member.
- The format of the meeting will be made available in the educator diary and any educator who wishes to speak can add their name.
- In the meeting, educators are able to:
 - Raise concerns
 - Negotiate solutions for any grievances.
 - Receive, share and discuss new information.
 (Please note Grievance Policy should be followed)
- In regards to the decision making process the following will occur:
 - If a decision cannot be reached about an issue the Nominated Supervisor will step in and make an informed decision.
 - If there is not sufficient information a educator will be nominated to do some research by the Nominated Supervisor.
 - At times, the discussion may need to be “taken off line” to be discussed at a different time. This may occur if there is insufficient information or the issue only relate to a number of individuals and can be resolved directly.
 - All decisions made will be made on a trial basis and their effectiveness will be discussed at the next meeting.
- Minutes are to be taken of all educator meetings.

EDUCATION ORIENTATION

Before a new educator commences their job, management will:

- Completion of the Employee Orientation Checklist (including the Buddy Checklist).
- Show the new educator the service and introduce them to other educators, children and families.
- Give the new educator a copy of the Educators Handbook.
- Highlight relevant policies including the philosophy.
- Highlight relevant legislation including OHS, Equal Opportunity, Confidentiality, Records, and Anti-Discrimination.
- Show new educators where the Policy and Procedures Manual is along with a copy of the Award/Industrial agreement and advise that they are available at all times.
- Induct and show the new educator techniques and relevant legislation in regards to Occupational Health and Safety.
- Provide new educator with necessary forms in regards to taxation, superannuation and payment of salary.
- Advise the new educator about the Service's management structure.
- Provide the new educator with a copy of their Job Description and go through it with them.
- Clarify any questions the new educator has.
- Allow the educator to spend some time in their designated room so they can be introduced to other educators, children and families.
- At the end of the new educator's first week the Nominated Supervisor will meet with the new educator to further clarify any questions or the job role.
- Complete the New Employee Orientation Checklist.

EDUCATORS RETURNING FROM EXTENDED LEAVE

- Our Service will work with both the educator who has been on leave and educators at the Service to ensure a smooth return to work. This will be carried out in the following way:
- The returning educator will if possible come in a few days beforehand to reacquaint themselves with the environment and take in any changes.
- The returning educator will be notified of any policy changes. It is the returning educators responsibility to understanding all new policies and procedures.
- Parents will be notified of the educators return.
- If necessary, educators training and development will be offered.
- If the period is due to an illness the educator must produce medical certificates stating they are ok to return to work.

- If special conditions or considerations are needed these will be discussed with management and appropriate plans commenced.

WORK EXPERIENCE STUDENTS AND VOLUNTEERS

Work Experience Students and Volunteers MUST follow all policies and procedures at the service.

The Service endeavours to support Work Experience Students and Volunteers in their efforts to become Early Childhood Professionals. They will be encouraged to complete their training enabling them to gain the knowledge and skills to provide children with care, support and developmentally appropriate programs.

Existing Educators Will -

- Maintain open communication with Work Experience Students and Volunteers along with their practicum teachers.
- Support all students, volunteers and individuals undertaking work experience needs during their placement.
- Pass relevant skills and knowledge onto each student, volunteers and work experience people.
- Ensure all educators are provided with relevant feedback about tasks that the student is required to complete in the service as part of their practicum.
- Be aware of the expectations perceived by the student, volunteers and work experience people.
- Have the time and capabilities to support each student, volunteers and work experience people in their placement.

Work Experience Students and Volunteers Will -

- Enjoy being with the children
- Learn about the children through observation and practical experience
- Develop skills and abilities needed to care for and educate children
- Learn about working as part of a team in the Early Childhood Profession
- Learn strategies employed when working in a team environment
- Learn skills already acquired by qualified educators in the Service
- Become familiar with families and children in the Service
- Keep educators aware of all written work required of the student, volunteers and work experience people

Families will:

- Be informed of when Work Experience Students and Volunteers are present at the service and their role and the time frame they will be spending at the Service.

METHOD:

An educator will be appointed 'Student Supervisor & Mentor' and this person will be required to:

- Arrange time for the student to visit the Service for a pre placement visit, during this visit the following will be arranged.
- Give the student times/hours and dates of the placement
- Given the student a student package
- Take the student on a tour of the Service
- Introduce the student to educators
- Student Supervisor will inform educators and Directors of student's, volunteers and work experience people's visit

- On the students, volunteers and work experience person's first day, they will be introduced to the Educational Leader. During this meeting the Student will inform the educators of all written work they are required to do and provide them with the time sheets and evaluation forms.

Work Experience Students and Volunteers Will -

- Inform the Student Supervisor, in writing of what will be expected of them by their training body, University or School, or any other training organisation.
- Be required to do different shifts during their time, so as to gain knowledge of each different aspect of the Service's day.
- Bring in a photo and a short statement addressing the following:
 - Name
 - Time they will be at the Service
 - What it is they are studying
- Discuss any problems they may be experiencing with the Student Supervisor.
- Complete all service student documentation induction and orientation procedures. Including provision of Working With Children Check documentation.

The Educational Room Leader Will -

- Communicate with the Student, volunteers and work experience people and discuss progress of their written work and performance.
- Discuss any issues raised by the student with the Student Supervisor.

Fail Procedure -

If educators feel that the student is at risk of failing their practicum, the following steps are to be taken:

- Room Leader is to alert the Student Supervisor of any concerns with the student.
- Both the Student Supervisor and the Room Leader are to discuss these issues with the Student.
- The Student Supervisor is to arrange with the students teacher/supervisor to visit the Service and discuss those issues that have arisen.
- The education institution of the student will ultimately determine the outcome of the practical.

Termination of Practicum:

Termination of student's placement will occur if:

- The student harms a child in the care of this child care service
- The student is under the influence of drugs or alcohol
- The student has disregard for the Service and fails to notify if not able to attend the Service
- The student is observed using repeated inappropriate behaviour at the Service.
- The student does not comply with all policies and procedures addressed in the student package.
- The student does not provide the photo with an introduction on commencement.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 25/3/2026

Next Policy Review Due: 15/9/2026

Reference:**NQS**

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
	4.2.2	Professional standards guide practice, interactions and relationships.
	(Pre 1 February 2018: 4.2.1)	
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
	(Pre 1 February 2018:4.2.2)	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
	4.2.1	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
	(Pre 1 February 2018: 4.2.3)	

QA7	7.2.3	Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.
	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations*Education and Care Services National Law Act 2010**Education and Care Services National Regulations*

Reg	168(2)(i)	<i>Policies and procedures in relation to governance and management of ther service</i>
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Other Reference (Prior and Other):***Education and Care Services National Regulations 2011******National Quality Standard******Early Years Learning Framework******Bryant, L. (2009). Managing a Child Care Service: A Hands-On Guide for Service Providers. Sydney, Community Child Care Co-Operative.******Corporate Governance Principles and Recommendations ASX Corporate Governance Council***

79 – STAFFING ARRANGEMENTS POLICY

Position Statement

The safety, rights and best interests of all children is our paramount consideration in all decisions and actions taken when providing care and education for children (the paramount consideration). The rights and best interests of each child must prevail over all our other interests or obligations.

We are committed to meeting our regulatory requirements in relation to staffing and taking all reasonable steps to protect children's safety, health and wellbeing while they are being educated and cared for.

Staff Arrangement Policy helps to ensure the service is compliant with staff/child ratios and qualified educators to provide high quality education and care to all children. The policy is used to provide information in relation to the Act and Regulations to ensure staffing arrangement are being fulfilled.

Objective

To set out:

- Our requirements to ensure supervision and staffing practices keep children safe at all times and enable the delivery of high-quality education and care for children
- How to provide a safe and positive workplace for staff, students and volunteers.
- determining the responsible person at the service
- This policy helps us to comply with child safety and work health and safety laws and the National Quality Standard, quality area 4: Staffing arrangements.

Related Policies

Service Philosophy
Statement of Commitment to Child Safety
Governance of Service, Management and Educator Policy
Child Safe Standards Policy
Child Safety, Protection & Wellbeing Policy
Child Safe Environment Policy
Safeguarding Children: Look, Do & Tell Framework Policy
Continuity of Education and Care Policy
National Quality Framework Policy
Ethical Conduct and Code of Ethics Policy

To avoid duplications please refer to the Governance of Service, Management and Educator Policy for other information related to Staffing Arrangements.

Definitions

The following definitions apply to this policy and related procedures:

"Approved provider": An organisation or individual authorised to operate an early childhood education and care service, responsible for meeting required standards, providing quality care, facilities, and qualified staff.

"Early childhood teacher": Early childhood teachers are responsible for carrying out educational and care activities.

"Educational Leader": A professional with a leadership role, overseeing educational programs, collaborating with room leaders and educators, providing guidance, mentoring, and professional development opportunities for staff.

"Educators": Educators implement our educational program, carry out routine care and supervision, ensure the health, safety and wellbeing of children.

"Fit and proper": Meaning a person is competent (they have the skills, knowledge and experience needed to comply with Family Assistance Law), of good character (they are diligent, honest and have good judgement and integrity), law-abiding (not disqualified by law from performing their role) and free from conflicts of interest.

"Nominated Supervisor": An individual designated to ensure compliance with regulations and The National Quality Standards, playing a key role in its management and administration.

"Person in day-to-day charge": A person nominated by the approved provider (other than a nominated supervisor or the approved provider) to be in day-to-day charge of the service.

"Persons with management or control": PMCs are responsible for managing the delivery of our service, including making or contributing to making decisions that affect the whole, or a substantial part, of our service or our financial position. A PMC has the same legal responsibilities as the approved provider and must exercise due diligence to ensure that the approved provider has complied with all legal obligations, including putting into place effective governance systems and operational processes.

"Room Leader": A qualified individual responsible for overseeing a specific room or group of children, planning educational programs, supervising children, and maintaining a safe environment.

"Student": An individual studying early childhood education, gaining practical experience or completing a placement under the guidance of qualified educators. However, they are not counted in ratio.

"Volunteer": A person who offers their time and services without payment to assist teachers, organise events, or engage with children under supervision, however, they are not counted in ratio.

Implementation

Our service will maintain compliance to the following in relation to the everyday practicalities of the service's operations:

- A nominated supervisor or responsible person will be on the premises at all times when children are being educated or cared for.
- Our service will nominate a qualified and experienced educator, co-ordinator or other individual as the service's Designated Educational Leader. This person is responsible to lead the development and implementation of the service's educational programs.
- Our service will ensure that any educator that is under eighteen years of age does not work alone at the service and is supervised at all times by an educator who is over eighteen.
- We will only include educators in the educators to child ratio who are working directly with the children.

- Educators rostering and routines will at all times make sure enough educators are available for the adequate supervision of children.
- Supervising educators give their attention to the children and not to any other duties. Supervision plans may be created to assist in communicating of supervision strategies.
- At no time will students or volunteers be included in the ratio of adults supervising children.
- There will be more than one educator present in the service when children are in attendance. No child will at any time be in the care of a sole educator at the service.
- Students and volunteers will never be left alone with a child or a group of children.
- In any situation where adequate supervision of children is threatened, any educators on a meal-break must be prepared to return to duty to supply adequate supervision.
- The Approved Provider or Nominated Supervisor will ensure that regulations in relation to the supervision of children are adhered to.
- Educators supervising outdoors, should position themselves to see as much of the play area as possible.
- One educator should be positioned close to the climbing frame as often as possible.
- Any water activity should be closely supervised by one educator at all times.
- Except for necessary discussions or concerns regarding children or matters relating to the Centre, educators will not congregate together outside.
- When children are resting or sleeping they will be supervised.
- During hand washing and/or toilet times children will be supervised in the bathroom area.
- Toddlers and children undergoing toilet training will not be left unsupervised in the bathroom.
- No child is to be left unattended at the table when eating.
- Rosters will be designed and implemented to ensure that children receive continuity of care.
- Our service will, when possible and to the best of our ability, make use of a regular pool of relief staff.

National Early Childhood Worker Register

The service must ensure that we record the required information in the National Early Childhood Worker Register (*National Regulations* reg 154A-154B) for:

- All educators
- Non-educator staff – e.g., administrative staff, cooks
- Teachers
- Nominated supervisors
- Temporary staff, including those employed by an employment agency or labour hire company
- Volunteers
- Students

Nominated supervisor

- The approved provider must nominate at least one nominated supervisor at our service.
- We must display the name of each nominated supervisor so that it is visible from the service's main entrance.
- A person who accepts a nominated supervisor position must consent in writing using the [ACECQA notification form NS01](#) which is then used to notify the Regulatory Authority through the NQA ITS.
- See 'Procedure- Appointment of Nominated Supervisor' (Appendix A) and 'Appointment of Nominated Supervisor form' (Appendix B) attached.
- We must continue to provide the regulatory authority with the written consent of other individuals appointed as a nominated supervisor. This can be included in the application for service approval or by submitting a Nominated supervisor consent form ([form NS01](#))

Person in day-to-day charge

- The approved provider must ensure there is a person in day-to-day charge of the service at all times when the service is educating and caring for children.
- An approved provider does not have to appoint a person in day-to-day charge if a nominated supervisor/s or approved provider is to be the responsible person.
- The approved provider or the nominated supervisor may appoint a person in day-to-day charge of the service in the absence of the approved provider or nominated supervisor, see 'Procedure-Appointment of Person in Day-to-day Charge' (Appendix C) and Appointment of Person in Day-to-day Charge' form (Appendix D) attached.
- Being in day-to-day charge of a service does not place any additional legal responsibilities on a person under the National Law. The responsibilities relevant to educators under the National Law continue to apply.

Educational Leader

- The approved provider must designate a suitably qualified and experienced educator, co-ordinator or other person as 'educational leader' to lead the development and implementation of our educational program (*National Regulations* reg 118).
- The approved provider must ensure that the educational leader has been advised of, can understand, and can explain the section of the *Regulations* that establishes their role, the purpose of the role (*National Regulations* reg 118), and their obligations under the National Quality Framework.
- The educational leader must agree to the appointment in writing (*National Regulations* reg 118), documented according to our record keeping procedures, and available for inspection by the regulatory authority on request (*National Regulations* reg 148).
- The educational leader must be supported and lead the development and implementation of the educational program and assessment and planning cycle (National Standard 7.2.2).

First Aid Officer

- The Nominated Supervisor or Responsible Person in Charge is the appointed First Aid Officer for the service and is responsible for coordinating the first aid response to serious incidents, ensuring appropriate medical assistance is obtained, notifying families where required, and completing all required reporting and regulatory notifications.
- Qualified Room Leader team members will assist in caring to children and team members as required. They will provide immediate assistance as required until the Nominated Supervisor, Responsible Person in Charge or emergency services assume responsibility.

Supervision of Services

Services must have at least one "responsible person" present at all times when caring for and educating children. A responsible person is:

- an approved provider
- a nominated supervisor
- person in day-to-day charge of the service

If more than one person at the Service is a "responsible person", services will develop a roster to ensure at least one is present whenever children are present. The name of the responsible person will be clearly displayed in the main entrance of the service.

Record of educators working directly with children

A reliable, detailed record provides the information needed to act quickly and decisively to protect children. The record of educators working directly with children is one of the most important protective records your service maintains and in a serious matter may be used by regulatory authorities, the approved provider, and law enforcement agencies to conduct thorough and accurate investigations.

The service must keep a record of educators working directly with children that, under *National Regulations* reg 151, includes the following information:

- (a) the name of each educator who works directly with children being educated and cared for by the service;

- (b) the hours that each educator works directly with children being educated and cared for by the service;
- (c) the rooms and times that each educator is allocated to work directly with children being educated and cared for by the service;
- (d) the Working with Children Check number of each educator. A list of staff and the WWCC numbers should be included/attached with the roster record (ie use the WWCC/VIT Central Register to be attached to the roster). Digital records also need to include the WWCC/VIT numbers.

Each Service will have in place for Record of educators working directly with children

Use of OWNA Digital Sign in and out of Service and Room Checkin Tool

- Use OWNA App (Public Mode)
 1. Click Centre In/Out button > Enter PIN > Press Access
 2. Click Room Checkin > Select Room > Enter PIN > Click Sign In.
 3. If changing rooms, access Room Checkin > Select new room > Enter PIN > Click Sign In.
 4. Repeat process for each change of room.
- Select / recording Responsible Person Role in OWNA App accordance with roster:
 1. Select "Responsible Person" on the OWNA App (Public mode);
 2. Click Tick Box for Sign-in as Responsible Person and sign the form.
 3. Enter PIN
 4. Sign in or out.

Please ensure Responsible person is recorded on Roster and also a sign is displayed in foyer of your is the Responsible Person In Charge of the Service.

Roster and WWCC/VIT Central Register

- A roster record with sufficiently detailed that advise what rooms or groups of children they have worked directly with. The roster record needs to include the names, qualification, role and WWCC numbers of educators. It needs to include detailed information on who is working with which children at any point in the day (i.e. including staff breaks and beginning and end of day arrangements). The roster need to document who is relieving staff members. ie covering breaks, lunch and non-contact time.
- Responsible person in charge of service should be recording on the Roster and also a sign displayed in the foyer.
- To be an accurate roster record, deviation from the roster (i.e. if arrangements shift in the course of the day, so that different educators are working with different children/rooms) must be recorded. Services should carefully consider how any deviations will be reliable and consistently recorded. The roster should be marked in pen to document the changes
- Educators who float between rooms during day: Record each period working with a different group of children. If you spend meaningful time with more than one group across the day, that should be reflected in your entries. Meaningful change: A lunch break, administrative time, or a sustained period working with a different group are all meaningful. A quick trip to get a supply from another room is not.
- What matters is keeping a record of which educators are working directly with which children throughout the day. Services make detailed forward-plans or rosters that include all meaningful changes (e.g. lunch cover, end of day groupings etc). If these are annotated through out the day with any unexpected changes, they can meet the requirement of Regulation 151.
- It is not a requirement to record brief or routine movements, such as:
 - stepping away to use the bathroom
 - briefly speaking with a parent or colleague
 - collecting materials or answering the phone.
 - every transition between physical indoor and outdoor spaces when moving with the same group of children.

- At the end of the week the roster (with breaklists) and WWCC/VIT Central Register should be uploaded as a PDF in allocated electronic folder. ie in Teams Folder – Rosters.

Educator to Child Ratios

Our educator to child ratios will always meet the minimum requirements as stated below:

- For children aged from birth to 24 months the educator to child ratio will be 1 educator to 4 children.
- For children aged over 24 months but less than 36 months the educator to child ratio will be 1 educator to 4 children.
- From 1 January 2016, for children aged over 36 months but not over preschool age is 1 educator to 11 children.
- If children being educated and cared for at the service are of mixed ages the minimum number of educators for the children must meet the requirements above at all times.
- When an early childhood teacher (ECT) is required to be in attendance at the service as per the licensed places of our service, that teacher is counted as an educator at the service for the purposes of this regulation.
- If the service is required to have access to an ECT for a period of time as per the licensed places, the ECT must be added to the minimum number of educators required for that service for that period.
- If the service is a preschool program in a school in a class or classes where a full-time education program is also being delivered, educator ratio and qualification requirements do not apply.

Access to an Early Childhood Teacher (ECT)

We must engage or have access to an early childhood teacher (ECT) based on the number of children in attendance at the service. Under *National Regulations* regs 130-134, this is as follows:

- **Less than 25 approved places, or 25 or more approved places but fewer than 25 children in attendance-** access to an ECT working for at least 20% of the time the service provides education and care (may be calculated on a quarterly basis). Access may be via information communication technology.
- **For 25 to 59 children** in attendance on any given day there are two options to meet ECT requirements-
 - The service must have one ECT in attendance for at least 6 hours on the day if operating 50 or more hours a week OR
 - 60% of the operating hours on that day if operating for less than 50 hours per week OR
 - The service has 25 – 59 approved places and employs a full-time or full-time equivalent ECT.
- **For 60 to 80 children** in attendance on any given day there are two options to meet ECT requirements-
 - The service must have one ECT in attendance for at least 6 hours on that day if operating 50 or more hours a week OR
 - 60% of the operating hours on that day if operating for less than 50 hours per week
 - **The service must also have** a second ECT or a 'suitably qualified person' in attendance for at least 3 hours on that day if operating for 50 or more hours a week OR
 - 30% of the operating hours on that day if operating for less than 50 hours a week OR
 - The service has 60-80 approved places, and employs a full-time or full-time equivalent ECT, and employs a second ECT or 'suitably qualified person' for half of the full-time or full-time equivalent hours.

- **For more than 80 children** in attendance on any given day there are two options to meet ECT requirements-
 - The service must have one ECT in attendance for at least 6 hours on that day if operating 50 or more hours a week OR
 - 60% of the operating hours on that day if the service operates for less than 50 hours per week.
 - **The service must also have** a second ECT or 'suitably qualified person' in attendance for at least 6 hours on that day if operating 50 or more hours a week OR
 - 60% of the operating hours on that day if operating for less than 50 hours per week OR
 - The service has more than 80 approved places, and employs a full-time or full-time equivalent ECT, and employs a second full-time ECT or 'suitably qualified person'.

Note: a 'suitably qualified person' is a person:

- actively working towards an approved ECT qualification and who has completed at least 50% of it or
- actively working towards an approved ECT qualification and who holds an approved Diploma qualification or
- registered as a primary or secondary school teacher in Australia and also holds an approved Diploma qualification or higher approved qualification.
- The numbers of children do not include children being cared for in an emergency for no more than two consecutive days the service operates.
- To be read in conjunction with ACECQA's 'Recognition as a suitably qualified person'.

The numbers of children do **not** include children being cared for in an emergency for no more than two consecutive days the service operates.

Record of access to ECT(s)

The approved provider must keep a record to prove that children have access to the required number of ECTs.

A service that provides education and care to fewer than 25 children preschool age or under must ensure that a record is kept of the following-

- (a) the period that an early childhood teacher is working with the service
- (b) the periods that the early childhood teacher is working directly with children and is not working directly with children.

A service that provides education and care to 25 or more but not more than 59 children preschool age or under must ensure that a record is kept of the period that an early childhood teacher is in attendance at the service.

A service that provides education and care to 60 or more children preschool age or under must ensure that a record is kept of the period that each early childhood teacher and each suitably qualified person is in attendance at the service.

ECT illness or absence

If an ECT (or suitably qualified person) is absent due to short term illness, on a required practicum placement, on leave or resignation, they may have their absence covered by a person who holds an approved Diploma level qualification or a qualification in primary teaching. This does not apply in case of a period of leave exceeding 12 weeks (*National Regulations* reg 135).

The record regarding replacement of ECT or suitably qualified person must include reference to:

- Name of the ECT/suitably qualified person being replaced;
- Role;
- Qualifications(s) of the substitute educator;
- Dates of replacement;
- Reason(s) for absence.

The nominated supervisor will ensure a record is maintained of the hours each ECT is in attendance (or service has access to ECT).

A notice needs to be displayed in the foyer of the service to inform everyone the absence of the kindergarten teacher and that a “Learning Program” will be delivered by a Diploma Qualified Educator. (ie not a “Kindergarten Program”).

Early Childhood Teachers (ECT)

An individual can be counted as an ECT if they:

- appointed and hold an approved early childhood teaching qualification, OR
- hold a former approved early childhood teaching qualification that commenced before 1 January 2012, OR
- hold a qualification that ACECQA has determined to be an [equivalent early childhood teacher qualification](#), OR
- hold a qualification that ACECQA has assessed as equivalent to an approved early childhood teaching qualification.

Teacher accreditation through the Victorian Institute of Teaching is a requirement for ECTs in Victoria.

The roles and responsibilities of an ECT may include but are not limited to:

- designing, planning, implementing and evaluating educational programs and practices, including following and implementing an approved learning framework
- leading and supporting the wellbeing, learning and development of children
- providing pedagogical leadership
- coaching, mentoring and supervising staff
- building and maintaining strong relationships with children and their families
- complying with, and supporting other educators to comply with, service policies and legislation
- working with community members and external agencies.

Educators – Diploma level qualified or higher

Under regulation 126(1)(a), 50% of educators required to meet the relevant educator to child ratios in centre-based services with children preschool age and under, must have, or be actively working towards, an approved diploma level education and care qualification (or higher).

Under regulation 364, regulation 126(1)(a) *does not* apply in Victoria if an educator had been employed:

- full-time and continuously as a staff member at a licensed children's service or outside school hours care service for a period of at least 5 years immediately preceding 25 May 2009 OR
- at least part-time and continuously as a staff member at a licensed children's service or outside school hours care service for a period of at least 10 years immediately preceding 25 May 2009 OR
- before the scheme commencement day and completed a professional development course approved by the Secretary under the Children's Services Regulations 2009 of Victoria.

Under the NQF, a person is considered a diploma qualified educator if they:

- hold an approved diploma level qualification, OR
- hold a former approved diploma level qualification completed before 1 January 2012 OR
- hold a qualification that ACECQA has assessed as equivalent to an approved diploma level educator qualification.

If an individual is 'actively working towards' an approved diploma level qualification, they **may** be counted as a diploma level educator, if they meet the requirements under the 'actively working towards' provisions. For more information visit ACECQA's [actively working towards web page](#).

The roles and responsibilities of a diploma level educator may include, but are not limited to:

- supporting and leading children's learning and development
- designing, planning, implementing and evaluating educational programs and practices, including following and implementing an approved learning framework
- working with community members and external agencies
- complying with, and supporting other educators to comply with, service policies and legislation.

The National Regulations do not specify what roles or responsibilities a diploma level educator must undertake. This is up to the approved provider of each education and care service.

Educators – Certificate III level qualified

All other educators required to meet the relevant educator to child ratios at the service must hold, or be actively working towards, at least an approved certificate III level education and care qualification.

Under the NQF, a person is considered a certificate III level educator if they:

- hold an approved certificate III qualification, OR
- hold a former approved certificate III level qualification completed before 1 January 2012, OR
- hold a qualification that ACECQA has assessed as equivalent to an approved certificate III level educator qualification.

If an individual is 'actively working towards' an approved certificate III level qualification, they may be counted as a certificate III level educator, if they meet the requirements under the 'actively working towards' provisions. For more information visit ACECQA's ['actively working towards' web page](#).

The roles and responsibilities of a certificate III level educator may include, but are not limited to:

- supporting children's learning and development, including contributing to the educational program and practice
- assisting the delivery of an approved learning framework
- developing positive relationships with children and families
- supporting the daily routines of children
- complying with service policies and legislation.

The National Regulations do not specify what roles or responsibilities a certificate III level educator must undertake. This is up to the approved provider of each education and care service.

Qualifications and responsibilities

The approved provider must take reasonable steps to ensure that staff who have a role in educating children at our service hold the required qualifications. See other policies for information on our ongoing child safe employment practices, including our procedures for ensuring staff are suitably trained and hold the required qualifications.

National Child Safety Training

Child safety training is mandatory for persons with management or control, nominated supervisors, persons in day-to-day charge, all other staff, students and volunteers (*National Law* s 162B).

The approved provider must ensure that each person listed above completes mandatory child protection training and mandatory 'Foundation' child safety training **before** they start working directly with children. If the person will not be working directly with children, they must complete the training within 14 days of starting at our service and ongoing staff need to complete the training every 2 years.

Certain people must also complete advanced training. Advanced training will be available from July 2026.

Child protection training – EC Protect Training

The approved provider and nominated supervisor make sure that all staff (including volunteers and students) who work with children are advised of and understand child protection laws and their obligations under them. In practice, staff can explain their mandatory reporting obligations, including the circumstances that will trigger a report and how to submit a report (*National Regulations* reg 84).

The approved provider and nominated supervisor must ensure that the nominated supervisor, every person in day-to-day charge and all other staff and volunteers successfully complete Victoria's child protection training module, EC PROTECT (*National Law* s 162A).

Working with Children Check

Everyone over 18 years old working or volunteering with children must hold a current non-prohibited Working with Children Check (WWCC).

Under *National Regulations* reg 358 [Vic] the approved provider must ensure they, the nominated supervisor or the person in day-to-day charge has read a person's Working with Children Check prior to that person being engaged as an educator or permitted to be a volunteer at the service. The exceptions to this are registered teachers whose registration has been verified by the approved provider, nominated supervisor or person in day-to-day charge, and volunteers who are working under the immediate supervision of an educator who holds a Diploma qualification or higher.

Staff and volunteers under 18 years of age do not require a WWCC, but they must be under the direct and immediate supervision of an educator aged over 18 years.

Protected disclosures (whistleblowers) training

The approved provider and our service will provide regular training and awareness sessions to our staff (including volunteers and students) about the importance of, and protections provided to, persons making protected disclosures (*National Law s 300(1)(b)*). See 102 - Protected Disclosures (Whistleblowers) Policy.

Other Qualifications:

First Aid Qualifications

- The approved provider of a centre-based service must ensure that the following persons are in attendance at any place where children are being educated and cared for by the service and immediately available in an emergency at all times that children are being educated and cared for by the service:
- At least one educator who holds a current approved first aid qualification. Please note our services have all staff holding this qualification.
- If the service is situated within a school's grounds it is sufficient for educators to have an individual that has completed an approved first aid course in attendance at the school site and so long as that person is immediately available in an emergency.
- An educator is taken to hold an approved first aid qualification if they meet the following:
 - If immediately prior to the 1st of January 2012 a person held a first aid qualification or had completed first aid training that met the requirements under former education and care services laws of a participating jurisdiction.
 - Holds an approved qualification as published on <http://www.acecqa.gov.au/home/>

Anaphylaxis Qualifications

- The approved provider of a centre-based service must ensure that at least one educator who has undertaken anaphylaxis management training is in attendance at any place where children are being educated and cared for by the service and immediately available in an emergency at all times that children are being educated and cared for by the service. Please note all our staff have this qualification.
- If the service is situated within a school's grounds it is sufficient for educators to have an individual that has completed an approved anaphylaxis training in attendance at the school site and so long as that person is immediately available in an emergency.
- If immediately prior to the 1st January 2012 the service was not required to have an educator present with anaphylaxis management training under previously education and care service laws, the service will implement this by 1st January 2013.

Emergency Asthma Management Training

- The approved provider of a centre-based service must ensure that at least one educator who has undertaken asthma management training is in attendance at any place where children are being educated and cared for by the service and immediately available in an emergency at all times that children are being educated and cared for by the service.
- If immediately prior to the 1st January 2012 the service was not required to have an educator present with asthma management training under previously education and care service laws, the service will implement this by 1st January 2013.
- If the service is situated within a school's grounds it is sufficient for educators to have an individual that has completed an approved asthma management training in attendance at the school site and so long as that person is immediately available in an emergency.

Food Safety Training/Food Supervisor training

Under Standard 3.2.2A [Australia New Zealand Food Standards Code](#) anyone who handles unpackaged potentially hazardous food that is used in the preparation of ready-to-eat food to be served to children or adults at our service must either complete a food safety training course or must have adequate skills and knowledge in food safety and hygiene to do that activity correctly and keep food safe (e.g. they have extensive relevant experience in the food industry, have completed in-house education or a food safety induction course with another employer etc).

Food safety training courses must cover, at a minimum, each of the following topics: a) safe handling of food; and b) food contamination; and c) cleaning and sanitising of food premises and equipment; and d) personal hygiene

Under the same Standard, our service's appoints a food safety supervisor/s who oversees the food handling and manages the day-to-day food handling operations must have approved food safety supervisor certificate from a registered training organisation that is recognised by food regulatory authority.

Fitness and Propriety

Everyone leading or working for us must be safe, suitable and legally allowed to be in their job.

The approved provider, PMCs, and persons responsible for the day-to-day operations must demonstrate they are 'fit and proper' persons (see definition) to be involved in managing or controlling our service (*National Law s 12*) (*Family Assistance Law*).

We must have child safe recruitment and ongoing employment practices in place.

The approved provider must ensure that:

- The nominated supervisor and person/s in day-to-day charge satisfy the minimum requirements related to fitness and propriety
- Educators, persons with management or control, and persons responsible for the day-to-day operations have the [background checks](#) required under *Family Assistance Law*
- All staff (including the approved provider, nominated supervisor, educators, other paid staff, contractors, volunteers and students) are thoroughly screened and verified as safe, suitable and not-prohibited to work with children and in an education and care service, and hold a valid, non-prohibited working with children check unless they are exempt under the law
- Staff complete mandatory approved child protection training and child safety training, as required under the *Law* (*National Law ss 162A, 162B*). They are made aware of, and understand, current child protection laws and can explain any obligations they have under them, including for mandatory reporting
- We continue to check and maintain accurate records about the fitness and propriety of the approved provider, PMCs, staff, volunteers and students according to our procedures
- We notify the regulatory authority and the Australian Department of Education if there is any change relevant to whether the approved provider/PMCs are fit and proper persons within 7 days (*National Law s 174, Family Assistance Law*)
- We provide information to the regulatory authority and Australian Department of Education, on request, for their assessment as to whether a person involved in our service is a fit and proper person (*National Law s 174, Family Assistance Law*)
- We notify the appropriate authorities (e.g., regulatory authority, Australian Department of Education, child protection services etc) within the required time of any relevant changes to the suitability of a staff member, volunteer or student (e.g., negative notices, working with children checks, charges or convictions, incidents etc)

- We comply with any suspension notices, supervision notices, prohibition notices, training notices or enforceable undertakings issued to current staff and volunteers (*National Law s 178A*) and make reasonable enquiries about whether current and prospective staff are subject to any such notices

Child Care Subsidy and PRODA

Any person with management or control of the Provider and persons responsible for the day-to-day operation of the service must be registered with the Federal Government's Provider Digital Access (PRODA) for administering Child Care Subsidy/Additional Child Care Subsidy.

Any staff member nominated by a person who meets these criteria may also be registered as a service contact with PRODA. In addition to obtaining a Working with Children Check if required to hold one, the Approved Provider will ensure all registered persons meet the fitness and propriety requirements under the Family Assistance Law as follows:

For person with management or control of the Provider:

- a certified copy of an Australian National Police Criminal History Check dated no more than six months before the application for approval
- an extract from the National Personal Insolvency Index Bankruptcy Search service provided by the Australian Financial Security Authority dated no more than three months before the application
- a current and historical personal name extract search of the Australian Securities and Investments Commission records dated no more than three months before the application
- evidence (computer printout) the person does not appear on the banned and disqualified register held by the Australian Securities and Investments Commission dated no more than three months before the application.

For persons responsible for the day-to-day operation of the service:

- a certified copy of an Australian National Police Criminal History Check dated no more than six months before the application for approval.

APPENDIX A

Procedure- Appointment of Nominated Supervisor

If an approved provider is an individual and has nominated themselves to be a nominated supervisor, there is no need for written consent.

Minimum requirements for nominated supervisors:

The approved provider must determine whether a person is suitable to be a nominated supervisor before nominating the person. We record this information, with sufficient evidence, and keep it on file to demonstrate compliance to the Regulatory Authority. This will help to show that we have taken reasonable steps to comply with our obligations under the National Law.

The person must:

- be 18 years or older
- have adequate knowledge and understanding of the provision of education and care to children
- have an ability to effectively supervise and manage an education and care service.

The approved provider must have regard to:

- the person's history of compliance with the National Law and other relevant laws
- any decision under the Law to refuse, suspend, refuse to renew, or cancel a licence, approval, registration, certification or other authorisation granted to the person under the National Law and other relevant laws
- considering their age, qualifications, skills and experience to determine their suitability for educating and caring for children and supervision and management abilities

The approved provider is responsible for:

- checking their WWCC clearance is current
- ensuring they are advised of and aware of their child protection obligations under the law, and have completed approved child protection training and approved child safety training
- getting a statement from person about their compliance history. Use 'compliance history statement' template on ACECQA website
- getting declaration from person that they're not a 'prohibited person'. Use 'prohibition notice declaration' on ACECQA website.

Notifying the regulatory authority

The approved provider and nominated supervisor are required to comply with all the notification requirements relating to staff changes and staff details as outlined in the *Governance and Management Policy*, including:

- Notifying the regulatory authority via the NQS ITS 7 days prior to the nominated supervisor starting, or if that is not possible, within 14 days after the nominated supervisor has commenced in the role.
- Notifying the regulatory authority via a separate notification if the nominated supervisor:
 - changes their name or contact details
 - is no longer employed or engaged by the service
 - has been removed from the role or withdraws their consent to the nomination
 - Receives a negative notice in relation to their WWCC or is subject to disciplinary proceedings under an education law.

APPENDIX B

Procedure- Appointment of Person in Day-to-Day Charge

Appointing a person in day-to-day charge enables our service to have a responsible person at the centre-based service at all times that the service is educating and caring for children.

For example, they can be a point of contact for parents and staff in the absence of the nominated supervisor.

The person appointed to be person in day-to-day charge must give their written consent. This is completed in the Staff Record form.

Minimum requirements for a person in day-to-day charge

- The approved provider or nominated supervisor must determine whether a person is suitable to be a person in day-to-day charge before nominating the person. We record this information, with sufficient evidence, and keep it on file to demonstrate compliance to the Regulatory Authority. This will help to show that we have taken reasonable steps to comply with our obligations under the National Law.
- A person in day-to-day charge must be 18 years or older.
- The approved provider or the nominated supervisor must take reasonable steps to ensure the person:
 - has adequate knowledge and understanding of the provision of education and care to children
 - has an ability to effectively supervise and manage an education and care service.

The approved provider or the nominated supervisor must have regard to:

- the person's history of compliance with the National Law and other relevant laws
- any decision under the Law to refuse, suspend, refuse to renew, or cancel a licence, approval, registration, certification or other authorisation granted to the person under the National Law and other relevant laws.

The approved provider is responsible for:

- checking their WWCC clearance is current
- ensuring they are advised of and aware of their child protection obligations under the law, and have completed approved child protection training and approved child safety training
- getting a statement from the person about their compliance history. Use 'compliance history statement' template on ACECQA website
- getting declaration from the person that they're not a 'prohibited person'. Use 'prohibition notice declaration' on ACECQA website.

APPENDIX C

Procedure- Appointment of a Responsible Person

If the nominated supervisor is present when service opens, they will:

1. Sign in on a responsible person sign in/out record
2. Make sure their name and role (nominated supervisor) is clearly displayed in the main entrance
3. Before they leave the service, handover the responsible person role to either the approved provider or person in day-to-day charge by:
 - talking directly to the approved provider or person in day-to-day charge
 - signing out of the responsible person record
 - making sure the approved provider or person in day-to-day charge signs in on the responsible person record
 - changing the name and position of the responsible person displayed in the main entrance to match that of the new responsible person

(The nominated supervisor will **not** leave the service if the approved provider and person in day-to-day charge are both absent. Another Responsible Person need to be appointed to this role before leaving.)

When they return to the service, resume the responsible person role by:

- talking directly to the person who took on the role when they were absent ie the approved provider or person in day-to-day charge
- signing in on the responsible person record

- making sure the approved provider or person in day-to-day charge signs out on the responsible person record
- changing the name and position of the responsible person displayed in the main entrance to their name and role

If the approved provider or person in day-to-day charge takes on the responsible person role while the nominated supervisor is absent, they will:

- Sign in on a responsible person sign in/out record
- Make sure their name and role is clearly displayed in the main entrance
- Stay at the service until the nominated supervisor returns and resumes the responsible person role, or before they leave the service, handover the responsible person role to another responsible person i.e approved provider or person in day-to-day charge by:
 - talking directly to that person
 - signing out of the responsible person record
 - making sure the new responsible person signs in on the responsible person record
 - changing the name and position of the responsible person displayed in the main entrance to match that of the new responsible person

The approved provider or person in day-to-day charge will **not** leave the service if there is not another responsible person present to take on the role.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/06/2026

Next Policy Review: 15/10/2026

Reference:

Education and Care Services National Regulations 2011
National Quality Standard

NQS

QA4	4.1.1 (Pre: 1 February 2018:4.1.1)	The organisation of educators across the service supports children's learning and development. Educator-to-child ratios and qualification requirements are maintained at all times.
QA7	7.1	Governance supports the operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Regulations numbered 240 and higher are state or transitional regulations

Reg	122	Educators must be working directly with children to be included in ratios
Reg	123	Educator to child ratios—centre-based services
Reg	126	Centre-based services—general educator qualifications
Reg	130	Requirement for early childhood teacher—centrebased services—fewer than 25 approved places
Reg	131	Requirement for early childhood teacher—centrebased services—25 or more approved places but fewer than 25 children
Reg	132	Requirement for early childhood teacher— centre-based services—25 to 59 children
Reg	133	Requirement for early childhood teacher—centre-based services—60 to 80 children
Reg	134	Requirement for early childhood teacher—centre-based services—more than 80 children
Reg	135	Early childhood teacher illness or absence
Reg	136	First aid qualifications
Reg	150	Staff record must include name of responsible person at service each time children being educated and cared for by the service.
Reg	173(2)(c)	Offence not to clearly display name of responsible person in the main entrance
Reg	240	Qualifications for educators—centre-based service applies until 31.12.15 Applies to reg 126
Reg Reg	241 242	Persons taken to hold an approved early childhood teaching qualification Persons taken to be early childhood teachers applies from 1.1.14 to 1.1.16 Applies to regs 130-134
Reg	243	Persons taken to hold an approved diploma level education and care qualification
Reg	244	Persons taken to hold an approved certificate III level education and care Qualification
Reg	245	Person taken to hold approved first aid qualification. Applies until 31.12.12 or qualification expires.
Reg	246	Anaphylaxis training
Reg	247	Asthma management training
Reg	355	Educator to child ratio—children over preschool age Applies to reg 123(1)(d)
Reg	356	Qualifications for educators—children over preschool age Applies to reg 126(2)
Reg	357	Educator to child ratio—children aged over 24 months but less than 36 months Applies to reg 123(1)(b)
Reg	358	Working with children check to be read

Reg	360	Educator to child ratios—children aged 36 months to preschool age Applies to reg 123(1)(C) until 31.12.15
Reg	361	General qualifications—centre-based service Applies to reg 126(1) until 31.12.15
Reg	362	Early childhood teacher in attendance—fewer than 60 children Regs 130-132 do not apply until 1.1.14.
Reg	363	Early childhood teacher in attendance—60 or more children Reg 133(1)(a) and (b) does not apply until 1.1.14 Reg 134(1)(a) and (b) does not apply until 1.1.20
Reg	364	Educational qualifications—centre-based services— saving of existing experience and qualification Applies to reg 126(1)(b)

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80 – STAFF INJURY MANAGEMENT POLICY & PROCEDURE

Position Statement

For the workplace to be a safe and healthy environment. The centre is committed to fostering and incorporating Occupational Health and Safety (OHS&W) / Risk Management initiatives that provide and maintain a safe work environment.

In accordance with the legislation, we are committed to providing staff members with support and assistance following a work-related injury, and promoting effective and sustainable rehabilitation assistance.

The injury management process will commence as soon as possible after a work-related injury/illness is reported and will take into account the injured staff members' recovery.

The organisation will endeavour to assist staff to remain at work while they recover or return to work at their earliest opportunity and will, where possible, provide modified pre-injury employment or offer other suitable employment to an injured staff member.

If an injured staff member is unable to ever return to their pre-injury duties, the organisation may consider offering alternative employment at the organisation, within the staff member's ongoing medical restrictions. Any alternative employment offered will be in accordance with the organisation's operational requirements. Should no suitable alternative employment be available, the organisation will engage a Rehabilitation Provider to assist the staff member in finding alternative employment with another organisation.

The organisation may provide, in some instances and subject to operational requirements, support and assistance in returning to work to a staff member who has sustained a personal injury, illness or disease upon the presentation of a medical certificate certifying them fit to undertake modified pre-injury employment.

The success of the Injury Management Policy and the Injury Management Procedure is dependent on a cooperative and proactive environment being created to enable successful outcomes to be achieved for both the injured staff member and the organisation.

Objective

To advise the organisations commitment to the effective management of work-related injuries and to the provision of appropriate support and assistance to ill or injured staff members.

Scope

This policy applies to all staff members employed.

Definitions

Alternative Employment/Alternative Workplace:	Employment that is different from the staff member's usual pre-injury position, as detailed in their position description. In certain circumstances this may mean the staff member is placed in an alternative Centre where the suitable duties are available.
Current work capacity	In relation to a staff member, means a present inability arising from a work-related injury or illness such that the staff member is not able to return to their pre-injury employment but is able to return to work in suitable employment.
Employment Obligation Period:	The duration of the organisation obligation to provide Suitable or Pre-Injury employment to an injured Staff Member.

Insurer:	The Insurer is an agent appointed by WorkSafe Victoria. The Insurer is authorised by WorkSafe Victoria to: - pay benefits to injured workers; - collect premiums; - manage compensation claims; and - provide return to work and risk management advice; in accordance with the Accident Compensation Act 1985 and associated legislation and regulations.
Modified Duties:	Duties that a Staff Member undertakes as part of their usual employment that have been modified or restricted to take into account the Staff Members current medical condition, as detailed on a WorkSafe Certificate of Capacity.
Occupational Rehabilitation Program:	A documented process to assist injured/ill staff members to return to work from a work-related illness or injury
Offer of Suitable Employment (OSE):	A written offer detailing the duties offered and hours of work for an injured staff member who is able to return to work.
Personal Illness or Injury:	An injury, illness or disease that is not related to employment. A medical certificate(s) may be submitted.
Pre-Injury Duties:	Duties undertaken by a staff member, as per their position description. Duties undertaken prior to a work-related injury occurring.
Rehabilitation Provider:	An external organisation approved by WorkSafe Victoria that provides independent support to injured workers and their employer by facilitating the return to work process.
Return to Work Plan (RTW):	A written plan detailing how the organisation will assist a staff member stay at work/return to work while recovering from a work-related injury or illness
Staff Members:	Any employee of the organisation. Any person who holds a current contract of employment with the organisation.
Suitable Employment:	Suitable employment is work that is suited to the Staff Members medical condition, capacity for work, skills and experience. It is work the injured Staff Member can do without the risk of further injury. The aim is to support the injured Staff Member so they can remain at or return to work while they recover from their injury and return to their normal work when possible.
Supervisors:	Managing Director, Support Manager, Centre Directors, or Supervisor who has the responsibility for the management of staff.
Work Related Injury:	Any injury, illness or disease where employment is a significant contributing factor as defined by the Accident Compensation Act 1985.
WorkCover Claim:	If a staff member has a work-related injury or illness, they may be able to receive compensation from WorkSafe Victoria. To do this they must lodge a Worker's Injury Claim Form with the Centre Director and Management.
WorkSafe Victoria:	WorkSafe Victoria is the manager of Victoria's workplace safety system. Broadly, the responsibilities of WorkSafe are to: - help avoid workplace injuries occurring - enforce Victoria's occupational health and safety laws

	provide reasonably priced workplace injury insurance for employers help injured workers back into the workforce manage the workers' compensation scheme by ensuring the prompt delivery of appropriate services and adopting prudent financial practices.
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Responsibility

This Section Outlines the Responsibilities of All the Parties Required to be Involved in the Occupational Rehabilitation Program

Responsible Officer:

The Managing Director, Human Resources and Centre Director has responsibility for the review and implementation of this policy. The Coordinators are responsible for:

- managing, monitoring and reviewing this policy;
- the ongoing management and monitoring of the WorkCover claims;
- consulting with staff members, supervisors, treating health practitioners and rehabilitation providers to develop sustainable return to work options;
- providing injured Staff Members with clear, accurate and current details of their return to work arrangements;
- monitoring progress of return to work plans;
- assisting injured/ill staff members in returning to work from their work-related injury/illness as soon as they are medically able to do so.

The Co-ordinator can be the Centre Director, Support Manager or Managing Director. (The Co-ordinator role at the centre this roles is normally handled by the Centre Director.) The Co-ordinator is required to:

- manage, monitor and review the Injury Management Policy and Procedure;
- ensure that the confidentiality of information is maintained and appropriate disclosure authorities are obtained before releasing any information to third parties.
- develop sustainable working relationships with the relevant insurer case manager, supervisors, injured staff members, treating health practitioners, rehabilitation and other service providers;
- ensure staff members are aware of their rights and responsibilities in the event of a work related injury or illness;
- consult, to the extent that is reasonable, with the injured Staff Member, supervisors, treating doctors and rehabilitation providers in planning return to work options;
- provide injured Staff Members with clear, accurate and current details of their return to work arrangements;
- ensure staff members who are participating in return to work plans have suitable and productive duties that make a contribution to the organisations operations and are within their identified medical capacity;
- manage, monitor and review return to work plans and suitable duties in consultation with the injured staff member, their supervisor, treating health practitioner(s) and rehabilitation providers;
- participate in the Return to Work Issue Resolution Process should it be required;
- ensure compliance with legislative requirements in relation to employee compensation and return to work processes;
- ensure staff members receive compensation entitlements for a work related injury or illness;

- promptly process all paperwork relating to a worker's compensation injury; and
 - liaise with staff members who have a personal illness or injury and their supervisors, where appropriate, to facilitate their return to the workplace.
- Supervisors (Person on Duty, Assistant Centre Director, Educational Leaders, Assistant or any person to who the staff member reports to).

Supervisors are required to:

- ensure an injured staff member receives appropriate first aid or medical treatment;
- promptly advise the Co-ordinator, Workplace Support via telephone or email as soon as they become aware of any work-related injuries that require medical treatment and/or time off work;
- promptly forward any paperwork received (including a copy of the Injury Report) relating to a workplace injury to the Co-ordinator, Centre Director;
- facilitate and/or implement changes as needed to provide a workplace that is safe and free of any risks to health and safety;
- facilitate the necessary advice and/or training to the staff members with regard to preventing work-related injuries;
- stay in contact with the injured staff member while they are away from the workplace;
- participate in return to work planning discussions with an injured staff member and the Co-ordinator, Centre Director;
- participate in Return to Work Issue Resolution Processes should they be required;
- ensure that staff under their supervision observe organisation's Policies and Procedures; and
- provide appropriate supervision to all staff members.

Injured Staff Members

Injured Staff are required to:

- report any workplace injuries/illnesses to their supervisor immediately or if immediate reporting is not possible as soon as reasonably practicable and within the requirement of the **Occupational Health and Safety Incident Reporting Procedure**;
- advise their supervisor within three hours (3) of the commencement of their usual working day if they are unable to attend work due to a work place injury/illness.. If notification within the specified time period is not possible, notification should be made as soon as practicable. Sick Policy procedures should be followed and adhered to at all times;
- promptly complete all necessary paperwork relating to their workplace injury/illness;
- If a claim is wishing to be made a Worker's Injury Claim Form must be completed and forwarded to the Co-ordinator, Centre Director, when a staff member receives medical treatment and/or has time off work for a work related injury/illness. Worker also need to be made aware of the procedure and implementation of making a claim.
- provide ongoing WorkSafe Victoria approved medical certificates for the duration of the injury/illness;
- make reasonable efforts to return to work in Suitable or Pre-Injury Duties;
- make reasonable efforts to actively participate in return to work planning and any ongoing return to work discussions;
- discuss with their treating health practitioner any duties they will be able to complete while injured/ill;
- submit all medical accounts for work related injuries to the Co-ordinator, Centre Director for payment;

Medical accounts relating to a workplace injury should not be paid for by the staff member.

Payment of any medical and travel expenses will only be made at the scheduled fee as specified by WorkSafe.

- abide by the doctor's medical restrictions both at work and at home;
- actively participate and co-operate in assessment of your capacity to work, rehabilitation progress or future employment prospects;
- If unable to attend an assessment appointment, the staff member must contact the Insurer to request an alternative date. If a staff member unreasonably refuses to attend a medical examination arranged by the insurer, the insurer has the right to suspend access to compensation payments.
- make any medical appointments concerning their injury outside of work hours where possible. Where it is not possible to do so, provide their supervisors with reasonable notice of their absence from work prior to the absence occurring;
- regularly communicate with their Supervisor and/or the Co-ordinator, Centre Director in relation to the status of their injury and their return to work program;
- make reasonable efforts to participate in a Return to Work Issue Resolution Process should it be required; and
- immediately advise their Supervisor or the Co-ordinator, Centre Director if they have an aggravation or re-occurrence of their injury.

Actions and Procedures

This Section Outlines the Steps Taken in the Injury Management Procedure:

Step 1	Injury reported by staff member - time off work and/or medical treatment received for injury. Refer to Occupational Health and Safety Incident Reporting Procedure for further detail.
Step 2	Staff member advises their Supervisor and Centre Director (or Person on Duty) of their capacity to work within 3 hours of the commencement of their usual working day or as soon as reasonably practicable. Sick Leave Policy needs to be followed and adhered to at all times.
Step 3	Centre Director or Person on Duty contacts the Support Manager and/or Managing Director (Co-ordinator) to advise of the injury as soon as they become aware an injury has occurred. Support Manager and/or Managing Director will appoint a Return to Work Coordinator. (The Return to Work Coordinator will typically be the Centre Director or the Assistant Centre Director.)
Step 4	The Co-ordinator telephones or writes to the injured Staff Member to provide contact details and outline the organisations processes. The injured Staff Member is provided with a claim form if required. Written contact will be initiated if the Co-ordinator is unsuccessful in establishing contact by telephone or telephoning may be deemed inappropriate.
Step 5	HEI007b_ Incident Report Form_For Staff Member * and WorkCover Claim Form** are completed promptly by staff member. Assistance may be provided if the staff member is unable to complete the forms personally. ** WorkCover paperwork is to be sent/provided to the Co-ordinator and Centre Director.
Step 6	Co-ordinator collates completed claim form and associated medical certificates and provides to Managing Director and/Support Manager for review. Then forwards documentation within ten (10) days of receipt, to the insurer for recording or determination of liability. The Co-ordinator confirms the receipt of the claim form to the Staff Member.
Step 7	Co-ordinator or the Centre Director begins a consultation process with the injured Staff Member, their Supervisor, treating health practitioners and Rehabilitation Provider (if appointed). The organisation will commence planning a staff members return to work, to the extent that it is reasonable to do so, as soon as: - the first WorkSafe Certificate of Capacity is received; or - when a Worker's Injury Claim Form for weekly payments is received, or - are advised by our WorkSafe Agent that they have received either of these documents from a staff member. In initiating discussions regarding returning to work, the Co-ordinator or Centre Director will make all reasonable efforts to consult with the parties involved. When initiating return to work discussions with the injured Staff Member, the Co-ordinator, will take into consideration the nature of the Staff Members injury or illness and begin consultation at an appropriate time. Advice on the whether or not it is appropriate to commence return to work discussions may be sought from the treating doctor prior to commencement. During the consultation process, the Staff member, Supervisor and Co-ordinator, Centre Director will discuss potential medical restrictions and available return to work options that can be offered within those restrictions. These discussions can be done individually, however discussion as a group is the preferred method of consultation. The injured Staff Member may be assisted by a representative during any consultation. The injured Staff Member can request the participation of an approved Rehabilitation Provider at any time during the consultation process.
Step 8	The Co-ordinator, Centre Director will develop a Return to Work Plan based on the information gathered as part of the consultation process and provide this to the staff member, treating health practitioners, and supervisor for review and approval. The Return to Work Plan will

	detail identified suitable employment, medical restrictions and return to work goals. The Return to Work Plan must be signed by all parties to the agreement. A Return to Work Plan cannot commence without medical approval and the appropriate Certificate of Capacity being provided. Any issues arising from the Return to Work process may be dealt with under the grievance procedures of this policy.
Step 9	Co-ordinator, Centre Director will monitor and update the Return to work plan in accordance with the Certificates of Capacity provided by the injured Staff Member. The Co-ordinator, Centre Director will maintain contact with injured staff member, Supervisor, treating health practitioner(s) and Rehabilitation Provider and continue the consultation process as the Return to Work Plan progresses.
Step 10	The injured Staff Member completes Return to Work process and returns to pre-injury duties*. This is confirmed by providing a final Certificate of Capacity from the treating doctor which indicates the Staff Member is fit to return to normal duties. * If an injured staff member is unable to ever return to their pre-injury duties, the organisation may consider offering alternative employment, within the staff member's ongoing medical restrictions. Any alternative employment offered will be in accordance with the organisation operational requirements. Should no suitable alternative employment be available, the organisation will engage a Rehabilitation Provider to assist the staff member in finding alternative employment with another organisation.

Legislative Context

- Accident Compensation Act 1985 and Associated Regulations

Associated Documents

- Injury Management Procedure
- Occupational Health and Safety Policy
- Hazard Management Policy
- Health and Safety Management Procedure

Forms

- Occupational Rehabilitation Program
- Workers Injury Claim Form

Forms/Record Keeping.

Title	Location	Responsible Officer	Minimum Retention Period
Injury Report Form	Risk, Health & Safety	Coordinator,	50 Years
Injury Report Form (Copy 1)	Staff Member	Centre Director	
Injury Report Form (Copy 2)	Human Resources/Payroll		
WorkCover Employee Claim Form	Insurer	Insurer Case Manager	50 Years
WorkCover Employee Claim Form (Copy 1)	Staff Member	Coordinator,	
WorkCover Employee Claim Form (Copy 2)	Human Resources/Payroll	Centre Director	

Return to Work Plans/Programs	Staff Member	Insurer Case Manager	50 Years
Return to Work Plan (Copy 1)	Insurer		
Return to Work Plan (Copy 2)	Human Resources/Payroll	Coordinator, Centre Director	
Associated certificates & reports relating to an individual claim	Insurer	Insurer Case Manager	50 Years
Associated certificates & reports relating to an individual claim (Copy 1)	Human Resources/Payroll	Coordinator, Centre Director	

Implementation

The Staff Injury Management Policy is to be implemented throughout the organisation via:

1. Inclusion on the Centre Policy Handbook; and
2. Electronic or hard copies provided to supervisors and injured staff members.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/9/2025

References:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>

107	<i>Space requirements—indoor</i>
108	<i>Space requirements—outdoor space</i>
109	<i>Toilet and hygiene facilities</i>
110	<i>Ventilation and natural light</i>
111	<i>Administrative space</i>
112	<i>Nappy change facilities</i>
113	<i>Outdoor space—natural environment</i>
114	<i>Outdoor space—shade</i>
115	<i>Premises designed to facilitate supervision</i>
116	<i>Assessments of family day care residences and approved family day care venues</i>
117	<i>Glass (additional requirement for family day care)</i>
73	<i>Educational programs</i>
74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
75	<i>Information about the educational program to be kept available</i>
76	<i>Information about educational program to be given to parents</i>
80	<i>Weekly menu</i>
86	<i>Notification to parents of incident, injury, trauma and illness</i>
99	<i>Children leaving the education and care service premises</i>
102	<i>Authorisation for excursions</i>
111	<i>Administrative space (centre-based services)</i>
168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
	<i>Children respond to diversity with respect</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Policy Statement

The organisation has determined that hazards management will form an essential element of its incident and injury prevention program. Therefore, all team members must integrate the management of hazards associated with the areas and activities under their control within their day-to-day responsibilities.

The centre will:

- develop, publish, and maintain procedures for the identification of hazards, and the assessment and control of risks for existing, new and proposed elements of the work environment;
- develop and conduct training sessions for Supervisors, Managers and Health and Safety Representatives;
- allocate resources for the implementation of risk control measures;
- monitor the implementation of the policy and related procedures; and
- assign responsibilities in relation to the implementation of this policy and related procedures.

Purpose

To affirm the commitment of the organisation to the integration of hazards management into the core responsibilities of Educators, Supervisors and Managers of the Centre.

To provide safe and healthy conditions for employees, contractors, students, customers and visitors whilst on centre premises.

Definitions

Hazard: The potential to cause harm to a person or to the natural environment.

Hazards management: The structured process of hazard identification, risk assessment and control aimed at providing a safe and healthy environment for employees, contractors, students, customers and visitors whilst on premises or whilst engaged in service-endorsed activities.

Risk: A combination of the likely frequency and severity of harm arising from a hazard.

Risk assessment: The process of evaluating likely frequency and severity of harm arising from a hazard.

Risk control: The process of implementing measures to reduce as far as reasonably practicable the risk associated with a hazard. The control process must follow the control hierarchy, in order, as prescribed in some health and safety legislation. It is important that control measures do not introduce new hazards, and that the ongoing effectiveness of the controls is monitored.

Reasonably practicable: Means practicable having regard to:

- the severity of the hazard or risk in question;
- the state of knowledge about that hazard or risk and any ways of removing or mitigating it;
- the availability and suitability of ways to remove or mitigate that hazard or risk; and
- the cost of removing or mitigating that hazard or risk.

Supervisor / Manager: Any employee of the organisation who:

- plans, organises or supervises the activities of other employees, contractors, students or visitors on behalf of the organisation; or
- designs or organises the design of new/refurbished facilities for the organisation.

In this policy and associated documents, the terms "Supervisor" and "Manager" include Centre Directors,

Responsibility

The Manager - Risk, Health and Safety will:

- develop, publish and maintain procedures for the implementation of this policy;
- develop and conduct training sessions for Supervisors, Managers and Health and Safety Representatives; and
- advise Supervisors and Managers in implementing the policy and related procedures.

Centre Directors will:

- attend training sessions organised in relation to this policy and related procedures; and
- implement a hazards management program in the area under their control and for all the activities they plan, organise or supervise, in accordance with this policy and related procedures. (This requirement may include developing and implementing procedures adapting organisation-wide documents for the local conditions.)
- Record repairs and maintenance in the OWINA Hazard and Maintenance Log.

Health, Wellbeing and Safety Representatives will:

- attend training sessions organised in relation to this policy and related procedures; and
- assist Centre Director and Person on Duty in implementing a hazards management program in the area and for the work group they represent.
- Record repairs and maintenance in the OWINA Hazard and Maintenance Log.

Contractors will, upon request from the Centre Director or Management, give details of the hazards relating to their activities, and of the measures they have adopted to control the risks. They will comply with the risk control measures specified by the organisation that apply to their activities.

The Health and Safety Policy Committee will monitor the implementation of the policy and related procedures.

Legislative Context

- Occupational Health and Safety Act 2004 (Victoria) and subordinate regulations
- Dangerous Goods Act 1985 (Victoria) and subordinate regulations
- Common Law principles of (a) general duty of care, and (b) duties of occupiers of premises

Associated Documents

- *Hazard Identification, Risk Assessment and Control (HIRAC) Procedure*

Implementation

Policy Handbook – P004. 7.2026

The Hazard Management Policy is to be implemented throughout the organisation via:

3. Inclusion on the Centre Policy Handbook; and
4. Electronic or hard copies provided to supervisors and injured staff members.

Hazard Identification, Risk Assessment and Control (HIRAC) Procedure

Purpose

To describe the measures to be implemented within Centres for the identification of hazards, and for the assessment and control of health and safety risks. It outlines methods for these hazards management activities, and sets minimum performance standards centres.

Scope

It applies to:

- planned workplace changes that may affect the health or safety of employees, students, contractors, visitors or customers of the centre prior to their implementation; and
- existing activities, work practices, equipment, items of plant, materials, substances, facilities, premises, buildings, areas, etc.

Definitions

Hazard: The potential to cause harm to a person or to the natural environment.

Hazards management: The structured process of hazard identification, risk assessment and control aimed at providing a safe and healthy environment for employees, contractors, students, customers and visitors whilst on premises.

Risk: A combination of the likely frequency and severity of harm arising from a hazard.

Risk assessment: The process of evaluating likely frequency and severity of harm arising from a hazard.

Risk control: The process of implementing measures to reduce as far as reasonably practicable the risk associated with a hazard. The control process must follow the control hierarchy, in order, as prescribed in some health and safety legislation. It is important that control measures do not introduce new hazards, and that the ongoing effectiveness of the controls is monitored.

Reasonably practicable: Means practicable having regard to:

- the severity of the hazard or risk in question;
- the state of knowledge about that hazard or risk and any ways of removing or mitigating it;
- the availability and suitability of ways to remove or mitigate that hazard or risk; and
- the cost of removing or mitigating that hazard or risk.

Risk control hierarchy: Ranks risk control measures in decreasing order of effectiveness:

- elimination of hazard;
- substitution of hazardous processes or materials with safer ones;
- engineering controls;

- administrative controls; and
- personal protective equipment.

The risk control measures implemented for the hazards identified should always aim to be as high in the list as practicable.

Managing Director / Support Manager / Centre Director / Person on Duty / Educator / Staff Member: Any employee of the organisation who:

- plans, organises or supervises the activities of other employees, contractors, students or visitors on behalf of the organisation; or
- designs or organises the design of new/refurbished facilities for the centre or organisation.

New: Used to describe plant or substances that have not been previously introduced within the organisation under the proposed conditions. It covers plant and substances that have never been introduced on site or have been introduced, but for different purposes, in different quantities, in different areas, etc.

Plant: All machinery and fixed appliances. It does not include road-registered vehicles, household consumer goods (eg kitchen refrigerators or microwave ovens), common hand-held tools (drills, sanders, angle grinders, etc), portable items (mobile phones, hand-held instruments, etc), and general office furniture and equipment (eg desktop computers, printers), except where these items contain or generate a substance as defined below.

Substance: All chemicals and materials, in any physical form (liquid, solid, powder, gas, mixtures, etc), used in the course of employees' or contractors' work. It includes, but is not limited to, compressed gases, solvents, radioactive substances, building materials, pesticides, laboratory chemicals, and cleaning chemicals. It excludes cooking ingredients used in food preparation, first-aid products and pharmaceuticals used under qualified supervision.

Actions

All Centres must:

New plant or substance:

Prior to the introduction of any new plant or substance within the Centre:

1. comply with the requirements of the OHS Procedure
2. complete a **New Plant or Substance Checklist**; and
3. perform a HIRAC review in accordance with the **HIRAC Guidelines** and document the results on a **HIRAC Report** form.

Workplace change other than introduction of new plant or substance:

1. determine in consultation with relevant employees whether the change may reasonably be expected to affect the health or safety of any person; and
2. if a potential health or safety impact is identified:
 1. perform a HIRAC review in accordance with the **HIRAC Guidelines** or any of the various guidelines provided to assist reviewers (e.g. for manual handling tasks, for children in centre activities/locations and document the results on a HIRAC Report form; and
 2. refer to the **Consultation for Workplace Changes Procedure** for any other requirement.

Existing activities, work practices, equipment, items of plant, materials, substances, facilities, premises, buildings, areas, etc:

1. in consultation with the centres Health and Safety Representative(s), develop an annual schedule of HIRAC reviews as part of the **Annual Health and Safety Plan**;
 2. conduct the HIRAC reviews in consultation with the relevant Health and Safety Representative(s) according to the schedule, using the **HIRAC Report** to document their findings (various guidelines are provided to assist the reviewers: for manual handling tasks, for children in centre's activities/locations;
 3. coordinate the implementation of risk control measures; and
 4. liaise closely with the Health and Safety Improvement Team(s) in their School or Directorate.
- before the end of each calendar year, a copy of the annual schedule of HIRAC reviews for the following year
 - quarterly summary reports that include details of the HIRAC reviews conducted and the risk control measures adopted/implemented. (Refer to the **Quarterly Summary Health and Safety Report** for the required format.)

Centre must keep a central register of completed HIRAC Reports with details of the risk control measures implemented as a result.

Electrical Equipment:

1. develop and maintain a register of the electrical equipment within the centre. This can be a tag and test report.
2. conduct the program of in-service inspection and testing of electrical equipment.

Responsibility

Centre Directors are responsible for the overall implementation of the procedure in their centre (new plant and substances reviews, yearly HIRAC schedules, register of HIRAC Reports, implementation of control measures, quarterly reports, electrical inspection and testing, etc).

Health and Safety Representatives must assist Centre Director in the implementation of this procedure.

Policy Base

Occupational Health and Safety Policy - **Hazards Management Policy**

Associated Documents

- **HIRAC Guidelines.**

Forms

- HIRAC for Children in Centre Locations/Activities
- HIRAC Report
- New Plant or Substance Checklist
- Register for Hazardous Substances/Dangerous Goods/Chemicals

Forms/Record Keeping

Title	Location	Responsible Officer	Minimum Retention Period
HIRAC Report	Centre Directors	Centre Directors	Until superseded by new risk assessment. Most items of legislation specify 5 years as maximum interval between assessments.
Quarterly Summary HIRAC Report	Support Office	Centre Directors	1 year

Implementation

The Hazard Management Procedure is to be implemented throughout the organisation via:

1. Inclusion on the Centre Policy Handbook; and
2. Electronic or hard copies provided to supervisors and injured staff members.

HIRAC Guidelines

Selection

The first step of the HIRAC review is to select an area, task or activity. Priority should be given to areas, tasks or activities thought to present special risks, based on:

- past experience (eg accidents, near-misses, complaints);
- concerns expressed by staff;
- requirements of legislation or policies and procedures (eg workplace changes);
- requests from the various Health and Safety Teams, etc.

Hazard Identification

Hazards can be grouped under various categories, as listed below. The items listed under each category are provided as examples.

Bio-mechanical and Postural

Note: where the main hazards associated with an activity or task relate to manual handling, a special risk assessment form (the HIRAC for Manual Handling Tasks Guidelines) should be used.

- Repetitive or sustained postures, movements or forces as listed below for more than 30 min at a time, or for more than 2 hours over a workday
- Excessive bending or twisting of back or neck in any direction
- Working with hand(-s) above shoulders
- Reaching to front/side more than 30 cm from body
- Reaching behind body
- Squatting, kneeling, crawling, semi-lying or jumping
- Standing with most of body weight on one leg

- Twisting, turning, grabbing, picking or wringing actions with fingers, hands or arms
- Working with fingers close together or wide apart
- Very fast movements Excessive bending of wrist(-s)
- Carrying with one hand or one side of the body
- Pushing, pulling, dragging, holding or restraining
- High force actions as listed below
- Lifting, lowering or carrying heavy loads
- Applying uneven, fast or jerky forces
- Sudden/unexpected forces
- Pushing/pulling objects hard to move or stop
- Awkward grips
- Throwing, catching, hitting, kicking or jumping
- Holding, restraining or supporting person, animal or heavy object

Physical Environment and Workplace Design

- Poor housekeeping, uncontained spillages or wastes
- Uneven or slippery work surfaces
- Obstacles in passageways, near equipment, risk of collision with stationary objects, etc
- Inadequate work platforms, stairs, ladders, guardrails, harnesses, etc for work at height
- Personnel access within 2 metres of edge of roof, high place of work, etc
- Unprotected openings or gaps in walkways & platforms
- Poor lighting
- Exposure to harmful noise levels
- Confusing or inadequate labelling of controls
- Mismatch between plant, workplace, office workstation design, activity or task and user physical characteristics (height, strength, speed, mobility, fitness, etc)
- Body parts coming in contact with hot components during testing, inspection, operation, maintenance, cleaning or repair
- Exposure to camp fires and hot items from fires
- Fall or collapse of ground, materials, plant, structures, etc
- Exposure to extremely cold materials or components (eg dry ice)
- Exposure to radiation (ionising and non-ionising, lasers)
- Entry into cool rooms

Mechanical

- Hair, clothing, jewellery, rags, etc liable to become entangled in moving components
- Uncontrolled or unexpected movement of machinery, components, work pieces, vehicles or loads
- Inability to slow, stop or immobilise machines, vehicles...
- Body parts coming in contact with moving, sharp, hot, or "live" components during testing, inspection, operation, maintenance, cleaning or repair
- Possibility of traffic accident
- Persons or body parts trapped or sheared between moving component & fixed plant, materials or structures

- Persons pushed, pulled or thrown off plant, structures...
- Machines, components, or materials disintegrating (eg grinding wheels)
- Persons being injured by damaged, poorly maintained or unguarded equipment (including electricals)
- Components, work pieces, fluids, etc being ejected

Electrical

- Contact with "live" components during testing, inspection, operation, maintenance, cleaning or repair
- Contact with overhead power lines
- Contact with underground power cables
- Explosion or ignition of electrical components, etc
- Unauthorised access to electrical services, switchboards, controls, etc

Chemicals and Toxicity

- Explosion or ignition of gases, vapours, liquids, dusts, etc
- Exposure to toxic concentrations of chemicals (skin, inhalation, ingestion, etc)
- Exposure to oxygen-depleted atmospheres
- Damage to gas lines, compressed gas cylinders, chemical storage containers, etc

Biological and Human

- Exposure to venomous or dangerous animals
- Exposure to toxic natural substances (plant, mushrooms, gases, etc)
- Exposure to (potentially) infectious substances
- Accidental collision with another person
- Assault by another person

Organisational and Procedural Arrangements

- Insufficient first-aid equipment or trained personnel
- Insufficient evacuation, emergency or rescue planning and facilities
- Access to hazardous equipment by unauthorised or untrained people
- Insufficient job rotation, rest breaks
- Inappropriate, insufficient or poorly maintained personal protective equipment

Psycho-social Environment and Task Design

- Insufficient consideration given to the probability of human error and its consequences
- Lack of clarity in work roles of employee(-s)
- Lack of control or recognition in work roles of employees
- Mismatch between task demands and workers'/people's behaviour or capabilities
- Insufficient consideration given to consultation prior to workplace changes

Natural Environment

- Drowning
- Bushfires
- Persons becoming lost or ill in remote locations

- Possibility of being engulfed in loose or crumbling ground
- Exposure to extreme environmental conditions (hot, cold, dry, wet, snowy, stormy, etc)
- Possibility of tree limbs falling
- Lightning in exposed locations

Risk Assessment

Risk assessments are based on 2 key factors:

- the likely severity of any injury/illness resulting from the hazard and
- the probability that the injury/illness will actually occur.

Table 1: Assessment of risk priority levels based on likely severity and probability of harm

		Probability			
		Very likely Could happen any time	Likely Could happen sometime	Unlikely Could happen, but very rarely	Very Unlikely Could happen, but probably never will
Severity	Death or permanent disability	1 (HIGH)	1 (HIGH)	2 (HIGH)	3 (MEDIUM)
	Long-term illness or serious injury	1 (HIGH)	2 (HIGH)	3 (MEDIUM)	4 (MEDIUM)
	Medical attention and several days off work	2 (HIGH)	3 (MEDIUM)	4 (MEDIUM)	5 (LOW)
	First aid needed	3 (MEDIUM)	4 (MEDIUM)	5 (LOW)	6 (LOW)

Risk Control

Urgent action is required for risks assessed as priority 1. The actions required may include:

- instructions for the immediate end of the work, process, activity, etc
- isolation of the hazard until more permanent measures can be implemented.

Documented control plans with responsibilities and completion dates are required for priority 2 and 3 risks. **(Refer to the HIRAC Report form)**

The risk control hierarchy ranks risk control measures in decreasing order of effectiveness. Risk control measures should always aim as high in the list as practicable. Control of any given risk generally involves a number of measures drawn from the various options (except if option 1 is selected).

Risk Control Hierarchy:

1. **Elimination of hazard:** examples include the proper disposal of redundant items of equipment that contain substances such as asbestos or PCBs, the removal of excess quantities of chemical accumulated over time in a laboratory, etc. The elimination of hazards is 100% effective

2. **Substitution of hazard:** examples include the replacement of solvent-based printing inks with water-based ones, of asbestos insulation or fire-proofing with synthetic fibres or rockwool, the use of titanium dioxide white pigment instead of lead white, etc. The effectiveness of substitution is wholly dependent on the choice of replacement.
3. **Engineering controls:** examples include the installation of machine guards on hazardous equipment, the provision of local exhaust ventilation over a process area releasing noxious fumes, fitting a muffler on a noisy exhaust pipe, etc. The effectiveness of engineering solutions is around 70 - 90%.
4. **Administrative controls:** include training and education, job rotation to share the load created by demanding tasks, planning, scheduling certain jobs outside normal working hours to reduce general exposure (eg planning demolition and building works during summer recess), early reporting of signs and symptoms, instructions and warnings, etc. The effectiveness of administrative controls ranges from 10 to 50%. They typically require significant resources to be maintained over long periods of time for continuing levels of effectiveness.
5. **Personal protective equipment (PPE):** includes safety glasses and goggles, earmuffs and earplugs, hard hats, toe-capped footwear, gloves, respiratory protection, aprons, etc. Research has shown that their effectiveness in realistic work situations is often very limited. However, carefully selected items of PPE are useful as a "last line of defence" when used in combination with some of the more effective measures listed above.

Implementation

The HIRAC Guidelines will be implemented throughout the organisation via:

1. Inclusion on the Centre Policy Handbook; and
2. Electronic or hard copies provided to supervisors and injured staff members.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/9/2025

References:

NQS

QA2	2.2.2	<i>Plans to effectively manage incidents and emergencies are developed in consultation with relevant authorities, practised and implemented</i>
QA3	3.1.1	<i>Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.</i>
	3.1.2	<i>Premises, furniture and equipment are safe, clean and well maintained</i>
	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Policy Handbook – P004. 7.2026

Education and Care Services National Regulations

Reg	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>
	107	<i>Space requirements—indoor</i>
	108	<i>Space requirements—outdoor space</i>
	109	<i>Toilet and hygiene facilities</i>
	110	<i>Ventilation and natural light</i>
	111	<i>Administrative space</i>
	112	<i>Nappy change facilities</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>
	115	<i>Premises designed to facilitate supervision</i>
	116	<i>Assessments of family day care residences and approved family day care venues</i>
	117	<i>Glass (additional requirement for family day care)</i>
	73	<i>Educational programs</i>
	74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
	75	<i>Information about the educational program to be kept available</i>
	76	<i>Information about educational program to be given to parents</i>
	80	<i>Weekly menu</i>
	86	<i>Notification to parents of incident, injury, trauma and illness</i>
	99	<i>Children leaving the education and care service premises</i>
	102	<i>Authorisation for excursions</i>
	111	<i>Administrative space (centre-based services)</i>
	168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
	171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
	<i>Children respond to diversity with respect</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

New Plant or Chemical Checklist – Risk, Health and Safety

Date of Checklist Completed:

Description of new item of plant or chemical introduced to service: _____

Name of Person Requesting New Item of Plant or Chemical: _____

ITEM OF PLANT

HAZARD IDENTIFICATION	YES	NO	If YES, give details.
Mechanical hazards (entrapment, entanglement, etc.)			
Electrical hazards (accessible live components, etc)			
Postural, ergonomic or manual handling hazards			
Inadequate space to locate or store the item of plant			
Hazards to personnel servicing or maintaining the item			
Hazards to personnel cleaning the item or near it			
Hazardous noise levels			
Ventilation requirements			
Hazardous temperatures (hot or very cold components)			
Radiation hazard			
Hazardous pressures (compressed gases, hydraulics, etc.)			
Inadequate, poorly labelled or inaccessible controls			
Relevant Australian Standards (state AS No)			

CHEMICAL

HAZARD IDENTIFICATION	YES	NO	If YES, give details.
Safety Data Sheet obtained and reviewed			
Chemical classified as 'Dangerous Goods'			
Chemical classified as Workplace Hazardous Chemical			
Dangerous Goods signage			
Revision of Chemicals Register			
Access to Safety Data Sheets			
Any specialised first aid equipment or training			
Stored in safe location			

GENEAL CONSIDERATIONS

HAZARD IDENTIFICATION	YES	NO	If YES, give details.
Guarding / emergency stops / safety devices			
Safety signage			
Safety instructions, information & training for users			
Training for those who install, use, maintain or dispose			
Special spill/clean up equipment or procedures			
Special fire protection/emergency requirements			
Protective equipment: footwear, eye, face, head, respirators, hearing, gloves, clothing, harness			
Ergonomics, workplace layout and seating			

Based on the health and safety risk assessment the new plant or chemical is acceptable to use: YES or NO

Name of OH&S Representative: _____ Signature: _____

Name Centre Director: _____ Signature: _____

Position Statement

Our service needs to promote the physical, social, emotional, language, and cognitive development of all children in a safe, nurturing environment that meets the individual needs of the children and achieves the learning outcomes in accordance with the National Quality Framework.

Objective

Our service participates in the National Quality Framework (NQF). The service aims is to provide the highest quality education and care available across all areas.

Implementation

Our Service participates in and values the National Quality Framework (NQF), including the National Quality Standard (NQS), the Early Years Learning Framework (EYLF – 2022 v2), Victorian Early Learning Years and Development Framework (VEYLDF) and the National Regulations – an Australian Government initiative linked to the funding of the Child Care Benefit for parents. This is conducted through the Australian Children's Education and Care Quality Authority (ACECQA) and the state licensing department through scheduled site assessment visits and where appropriate, spontaneous visits.

The NQS provides standards of quality practices for care provided in our Service as well as guidance and support from the Service's self evaluation through our Quality Improvement Plan (QIP). The system also allows educators to continually improve practices by identifying the quality aspects of care the Service is already providing and assisting the Service in developing goals for further improvement through our QIP. The Service is required to complete and submit a comprehensive QIP.

The Service will ensure that all educators and management are informed about current practices and requirements in the NQF process by attending appropriate in-service/training, accessing any other publications and information about the accreditation process that may be of benefit – including those published by ACECQA.

Educators will involve parents, families, community and management in each stage to seek their input and views into practices and care in our Service – this includes having parent input into policy reviews, parent meetings and providing updates in newsletters about the Service's current stage in the process.

The seven Standards under the NQS are –

1. Educational program and practice
2. Children's health and safety
3. Physical environment
4. Staffing arrangements
5. Relationships with children
6. Collaborative partnerships with families and communities
7. Governance and Leadership

The Ratings are as follows –

- Excellent
- Exceeding National Quality Standard
- Meeting National Quality Standard
- Working towards National Quality Standard
- Significant improvement required.

We will access regular updates on the ACECQA website – www.acecqa.gov.au

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

References:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Other References:

National Quality Standard
Early Years Learning Framework

Position Statement

Our service participates in the National Quality Framework (NQF). The service aims is to provide the highest quality education and care available across all areas.

Objective

As a part of our commitment to the National Quality Framework (NQF), our service will annually review our policies and procedures to ensure excellence and compliance. Our review processes also provides an important opportunity for families to offer their valuable input into the practices at the service and how best to meet the needs of each child being educated and cared for.

Implementation

- All policies and procedures will be made available to families during the enrolment and orientation period for their child.
- Educators will notify families of how to access policies and procedures and where they are located in the service.
- Our educators will ensure that all policies and procedures are reviewed annually or more often if required. This gives both families and educators opportunities to suggest elements that need to be improved.
- Management is responsible for developing and changing policies and procedures relating to regulatory guidelines set by government agencies and recognised organisations.
- For educators and management this will occur:
 - At educators meetings.
 - At the policy review points.
 - In family meeting.
- For families this will occur:
 - Via newsletters.
 - At the policy review point.
 - At parent/educators meeting.
- However, at any time of the year educators and family members are invited to enquire and have input into the policies and procedures.
- All policies will be signed, sourced and dated at each review and educators will continuously seek out relevant information to provide the best possible environment.
- All stakeholders at the service must be informed of any changes to policies. This will occur in writing and be provided to families, educators, management, the committee and any other relevant individuals.
- The service will ensure that parents of children enrolled at the service are notified at least 14 days before making any change to a policy or procedure that may have a significant impact on—
 - (a) the service's provision of education and care to any child enrolled at the service; or
 - (b) the family's ability to utilise the service
 - (c) any change that will affect the fees charged or the way in which fees are collected.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/1/2026

References:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

NQS

QA4	4.2	<i>Management, educators and staff are collaborative, respectful and ethical.</i>
	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>
QA7	7.2.3	<i>An effective self-assessment and quality improvement process is in place.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Regulations

Regs	31	<i>Condition on service approval - quality improvement plan</i>
	55-56	<i>Quality improvement plans</i>
	168	<i>Education and care service must have policies and procedures</i>
	170	<i>Policies and procedures to be followed</i>
	171	<i>Policies and procedures to be kept available</i>
	172	<i>Notification of change to policies or procedures affecting ability of family to utilise service</i>

84 – RECORD KEEPING AND RETENTION POLICY

Position Statement

Our service participates in the National Quality Framework (NQF). The service aims is to provide the highest quality education and care available across all areas. This policy outlines the procedures for the safe and responsible management and destruction of documentation, photos and video recordings to protect the privacy and confidentiality of individuals.

Objective

To ensure that our service maintains appropriate records in line with our regulatory requirements. To define the retention policy and destruction methods for documentation, photos and video recordings produced and held at the service.

Related Policies

Child Safe Standard Policy
Child Safety, Protection & Wellbeing Policy
Technology and Digital Use Policy
Education, Curriculum and Learning Policy
Ethical Conduct and Code of Ethics Policy
Governance of Service, Management and Educator Policy
Excursion Policy
Family Violence Safety Policy
Complaints and Grievance Policy
Incident, Injury, Trauma and Illness Policy
Parent Involvement Policy
Privacy and Confidentiality Policy
Employment Policy
Internet, Email & Social Networking Policy
National Model Code Policy

Implementation

Note record keeping requirements also apply in relation to Child and Family Violence information Sharing Schemes – refer Child Safety, Protection & Wellbeing Policy and Family Violence Safety Policy.

Records to be Kept in Relation to Staff

The following records must be kept in relation to the service's Nominated Supervisors:

- The full name, address and date of birth.
- Evidence of any relevant qualifications held by the Nominated Supervisor.
- If applicable, evidence that the Nominated Supervisor is actively working towards that qualification. If this is the case, the following must be recorded:
 - Proof of enrolment.
 - Documentary evidence that the Nominated Supervisor has commenced the course, is making satisfactory progress towards the completion of the course, is meeting the requirements of maintaining the enrolment.
 - For Nominated Supervisors who are working towards the completion of a Diploma level education and care qualification, proof that they hold an approved Certificate III level education and care qualification or has completed the units of study in an approved Certificate III level education and care qualification determined by ACECQA.

- Evidence of any approved training (including first aid training) completed by the Supervisor.
- If applicable, a Working with Children Check.

The following records must be kept in relation to other staff members:

- The full name, address and date of birth.
- Evidence of any relevant qualifications.
- If applicable, evidence that the staff member is actively working towards that qualification. If this is the case, the following must be recorded:
 - Proof of enrolment.
 - Documentary evidence that the staff member has commenced the course, is making satisfactory progress towards the completion of the course, is meeting the requirements of maintaining the enrolment.
 - For staff members who are working towards the completion of a Diploma level education and care qualification, proof that they hold an approved Certificate III level education and care qualification or has completed the units of study in an approved Certificate III level education and care qualification determined by ACECQA.
- Evidence of any approved training (including first aid training) completed by the staff member.
- If applicable, a Working with Children Check.

The following records must be kept in relation to the Educational Leader:

- The name of the educator who is designated at this role.

The following records must be kept in relation to Students and Volunteers:

- The full name, address and date of birth of each student or volunteer.
- The Approved Provider must also keep a record for each day on which the student or volunteer participates in the service, the date and hours of participation.

The following records must be kept in relation to the Responsible Person:

- The staff record must include the name of the responsible individual at the centre-based service for each time that children are being educated and cared for by the service.

The following records will be kept in relation to educators working directly with children:

- The name of each educator.
- The hours that each educator works directly with children. Please note that a staff roster or time sheet is sufficient record of this.

The following records will be kept in relation to access to early childhood teacher/s:

- The approved provider of a centre-based service that provides education and care to fewer than 25 children preschool age or under must ensure that a record is kept of the following:
 - The period that an early childhood teacher is working with the service in accordance with the time frames set out in the Staffing Arrangements Policy.
 - The periods that the early childhood teacher is working directly with children and not working directly with children.
- The approved provider of a centre-based service that provides education and care to 25 or more children preschool age or under must ensure that a record is kept of the period that an early childhood teacher is in attendance at the service.

Records Relating to Enrolled Children

The following records will be kept in relation to enrolled children:

- Documentation relating to child assessments or evaluations for delivery of the education program, including:

- Assessments of the child's developmental needs, interests, experiences and participation in the education program.
- Assessments of the child's progress against the outcomes of the educational program.
- **An Incident, Injury, Trauma and Illness Record** (within Incident, Injury, Trauma and Illness Policy), including:
 - **Details of any incident** in relation to a child or injury receive by a child or trauma to which a child has been subject while being educated and care for by the service. The following must be included:
 - The name and age of the child.
 - Gender
 - The circumstances leading to the incident, injury or trauma.
 - The time and date the incident occurred, the injury that was received or the child was subjected to the trauma.
 - **Details of any illness** which becomes apparent while the child is being educated and care for by the service. The following must be included:
 - The name and age of the child.
 - The relevant circumstances surrounding the child becoming ill and any apparent symptoms.
 - The time and date of the apparent onset of the illness.
 - **Details of the action taken** by the service in relation to any incident, injury, trauma or illness which a child has suffered while being educated and cared for by the service. The following must be included:
 - Any medication administered or first aid provided.
 - Any medical personnel contacted.
 - Details of any individual who witness the incident, injury or trauma
 - The name of any individual who the education and care service notified or attempted to notify of any incident, injury trauma or illness a child has suffered at the service and the time and date of the notification and notification attempts.
 - The name and signature of the individual making an entry in the record and the time and date that the entry was made.
 - This record must be recorded as soon as is practicable, but not later than 24 hours after the incident, injury, trauma or onset of illness occurred.
- **A medication record** which includes the following:
 - The name of the child
 - The authorisation to administer medication (including self-administration is applicable) signed by a parent or an individual named in the child's enrolment record as authorised to consent to administration of medication.
 - The name of the medication to be administered.
 - The time and date the medication was last administered.
 - The time and date or the circumstance under which the medication should be next administered.
 - The dosage of the medication to be administered.
 - The manner in which the medication is to be administered.
 - If the medication is administered to the child:
 - The dosage that was administered.
 - The manner in which the medication was administered.

- The name and signature of the individual who administered the medication.
- If another individual is required to check the dosage, the name and signature of that individual.
- **A record of attendance for enrolled children**, including:
 - The full name of each child attending the service.
 - The date and time each child arrives and departs.
 - The signature (including electronic signature/PIN) of:
 - the person who delivers and collects the child when he or she arrives and departs; or
 - the nominated supervisor or educator. (this is not preferred; signature (including electronic signature/PIN) should be obtained from parent of child for CCMS purposes)
 - For preschool programs provided by a school, services are not required to comply with this is the service keeps attendance records in accordance with the education or of Government education department policy.
- **Child enrolment records** which include the following:
 - The full name, date of birth and address of the child.
 - The name, address and contact details of:
 - Each known parent of the child
 - Any individual who is to be notified of any emergency involving the child if any parent of the child cannot be immediately
 - Any individual who is an authorised nominee
 - Any individual who is authorised to consent to medical treatment of, or to authorise administration of medication to the child.
 - Any individual who is authorised to authorise an educator to take the child outside the education and care service premises.
 - Details of any court orders, parenting orders or parenting plans provided to the approved provider relating to powers, duties, responsibilities or authorities of any individual in relation to the child or access to the child.
 - Details of any other court orders provided to the approved provider relating to the child's residence or the child's contact with a parent or other individual.
 - Gender of the child
 - Language used in the child's home
 - Cultural background of the child and parents (if applicable)
 - Any special considerations for the child (e.g. cultural, religious, dietary requirements or additional needs)
 - Authorisations signed by a parent or a individual named in the enrolment record as authorised to consent to the medical treatment of the or nominated supervisor to seek:
 - Medical treatment for the child from a registered medical practitioner, hospital or ambulance service.
 - Transportation of the child by any ambulance service.
 - Authorisation to take the child on regular outings.
 - The name, address and telephone number or the child's registered medical practitioner or medical service.
 - The child's Medicare number if available.
 - Details of any specific healthcare needs of the child including any medication conditions or allergies including whether the child has been diagnosed as at risk of anaphylaxis, including details of any medical management plan.

- Details of any dietary restrictions for the child
 - The immunisation status of the child
 - A notation that states that a staff member or approved provider has sighted a child's health record.
- **A record of the services compliance with the law**, including:
 - Details of any amendments of the service approval made by the Regulatory Authority including:
 - The reason stated by the Regulatory Authority for the amendment.
 - The date on which the amendment took, or takes, effect
 - The date (if any) that the amendment ceases to have effect.
 - Details of any suspension of the service (other than a voluntary suspension) including:
 - The reason stated by the Regulatory Authority for the suspension.
 - The date on which the suspension took, or takes, effect.
 - The date that the suspension ends.
 - Details of any compliance direction or compliance notice issued to the approved provider in respect of the service, including:
 - The reason stated by the Regulatory Authority for issuing the direction or notice
 - The steps specified in the direction or notice.
 - The date by which the steps specified must be taken.
 - This information must not include any information that identifies any individual other than the approved provider.
 - A record of certified supervisors placed in day to day charge of the education and care service.

The approved provider must ensure that the documents referred to above in relation to a child enrolled at the service are made available to a parent of the child on request. In line with this, if a parent's access to the kind of information referred to in this documentation is limited by an order of a court, the approved provider must refer to the court order in relation to the release of information concerning the child to that parent.

The record of compliance referred to above must be available for access on request by any individual.

Length of Time Records must be Kept

Our service will keep records for the following periods:

Record	Duration
If the record relates to an incident, illness, injury or trauma suffered by a child while being educated and cared for by the service,	Until the child is aged 25 years.
If the record relates to an incident, illness, injury or trauma suffered by a child that may have occurred following an incident while being educated and cared for by the service,	Until the child is aged 25 years.
If the record relates to the death of a child while being educated and cared for by the service or that may have occurred as a result of an incident while being educated and cared for,	Until the end of 7 years after the death.
If the case of any other record relating to a child enrolled at the education and care service.	Until 3 years after the last date on which the child was educated and cared for by the service.

If the record relates to the Approved Provider.	Until the end of 3 years after the last date on which the approved provider operated the education and care service.
If the record relates to the nominated supervisor or staff member of an education and care service.	Until the end of 3 years after the last date on which the nominated supervisor or staff member provided education and care on behalf of the service.
In the case of any other record.	3 years after the date on which the record was made.

Storage of Records

Records made by our service will be stored in a safe and secure location for the relevant time periods as set out above and only made accessible to relevant individuals.

If a service is transferred under the law, documents relating to a child must not be transferred without the express consent of the child's parents.

Confidentiality and Storage of Records

The approved provider will ensure that information kept in a record is not divulged or communicated through direct or indirect means to another individual other than:

- The extent necessary for the education and care or medical treatment of the child to whom the information relates.
- A parent of the child to whom the information relates, except in the case of information kept in a staff record.
- The Regulatory Authority or an authorised officer.
- As expressly authorised, permitted or required to be given by or under any Act or law.
- With the written consent of the individual who provided the information.

Archiving Procedure

Archiving of service documentation will be completed on the following basis:

Record Keeping and Storage Schedule

TYPE OF RECORD (Electronic records)	LEGISLATIVE RESPONSIBILITY AND OPERATIONAL PROCEDURE	CONSOLIDATED RECORDS	PERIOD AND LOCATION FOR STORAGE	RELEVANT LEGISLATION OR OTHER REQUIREMENT
Child's personal enrolment information: -Enrolment form - Copy of birth certificate, passport or citizenship certificate -Medical details on enrolment form (above) -Immunisation records -Medicare details & Private -Health Insurance details if held -Administration of Medication -Special diet form -Food allergy management instruction form -Medical management plan	-Centre responsible for ensuring that records are made and kept up to date -Enrolment form – completed prior to commencement. -Parents requested to update information annually & as changes occur. Immunisation details – sighted & copied at enrolment. -Immunisation updated as required (reviewed 4 times per year – copy of review maintained) Health information to be reviewed annually	Electronically Held: OWINA Stored in the Child's file at Centre	Until the end of 3 years after the last date on which the child was educated and cared for by the service. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(d): in the case of any other record relating to a child enrolled at the education and care service, until the end of 3 years after the last date on which the child was educated and cared for by the service;

-Re-enrolment form -Electronic records – child details; family details				
Child's health: record relates to incident, illness or trauma suffered by child -Illness/minor accident form -Accident report form -Serious incident, accident or injury form	-Centre responsible for ensuring that records are made and kept up to date	Electronically Held: OWNA Stored in the Child's file at Centre	Until the child is aged 25 years. Location: 2 years at the centre, then archived at an approved location until child reaches 25 years of age	Reg183(2)(a), (b)
Child Death: -record relates to the death of a child while being educated and cared for by the education and care service or that may have occurred as a result of an incident while being educated and cared for		Electronically Held: OWNA Stored in the Child's file at Centre	Until the end of 7 years after the death. Location: 7 years at the centre.	Reg183(2)(c)
Attendance: -Attendance sheet -Receipts for fees paid by parents -Written notice of withdrawal -Supporting documentation for approved absences for each child -Electronic records – contracts; paid hours	-Centre staff & parents responsible for ensuring that attendance sheet is accurate, and signed by the person who brings and collects the child at times of handover -Staff to ensure additional days are highlighted on attendance sheets -Service responsible for maintaining an up-to-date record of attendance	Electronically Held: OWNA Stored in individual folders within the rooms	3 years from the end of the calendar year in which the relevant care was provided (Funding Agreement) Until the end of 3 years after the last date on which the child was educated and cared for by the service. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(d)
Excursions: -Non routine excursion forms signed & dated by parents	-All families informed about all aspects of upcoming excursions out of the service. -Room Staff responsible for ensuring parents sign excursion form as required. -Service responsible for ensuring no child participates in an excursion without written permission.	Electronically Held: OWNA Stored in the child's file at the Centre	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)
Complaints & Grievances: -Complaints Register -Electronic record – complaints register -Grievance Register and Documentation	Authorised Supervisor All complaints will be recorded and	Electronically Held: OWNA Stored in the Complaints file (hard managed in accordance with the Grievance Policy)	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)
Visitors to the Centre: -Visitor's Register maintained	-Service responsible to ensure that all visitors sign in to the service	Electronically Held: OWNA Stored at the centre	Until the end of 3 years after the date on which the record was made.	Reg183(2)(g)

			Location: 3 years at the Centre or archived at an approved location after 2 years.	
Children Curriculum: Children's programs, developmental records, daily routine, weekly record of service	-Service ensures records are maintained and kept up to- date -Staff maintain all records; ☐ Developmental records or other child records to be placed on child's file ☐ Program information	Electronically Held: OWNA Child Developmental records stored in child's file at Centre	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)
Mandatory report of child at risk	Dept of Community Services Staff maintain written records of circumstances and all relevant information about report in accordance with Interagency Guidelines	Stored on child's file at coordination unit (reporter may on request not be specified)	Until the child is aged 25 years. Location: 2 years at the centre, then archived at an approved location until child reaches 25 years of age	Reg183(2)(a), (b)
Insurance	Approved Provider responsible for ensuring policy current.	Copy on file by Approved Provider	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)
Training: -Mandatory training -First Aid -Staff first aid currency (stored electronically, hard copy on file)	Service responsible to ensure currently first aid on premise	Stored in the Staff file and electronically	<i>Not specified –</i>	Reg183(2)(f)
Staff Records File: Staff Qualifications & Staff Record Form	Service applicant to provide evidence to Approved Provider /Docs of qualifications Provided as part of recruitment process, or for inclusion with A/S Application; copy kept on personnel file	Kept on personnel file centre & HR.	Until the end of 3 years after the last date on which the nominated supervisor or staff member provided education and care on behalf of the service. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(f)
Staff records: Immunisation information, time sheets, leave information, rosters, training. Staff files stored electronically	Service responsible for the collection and storage of consolidated records	Records stored in Staff file at Centre, & HR Electronically Held: OWNA	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(f)
Staff records: -time sheets, leave form, sick letters, project and actual budget rosters, staff rosters. (These are stored electronically and	Service responsible for the collection and storage of consolidated records	Records stored in Staff file at Centre, & HR Electronically Held: OWNA & Wageloch	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location	Reg183(2)(f)

emailed to payroll email address)			after 2 years.	
Staff Records: Working with Children Checks (Clearance letters stored Electronically)	Approved Provider Employer Prohibited Persons Declarations, Working With Children Check stored in a central file HR	Stored centrally HR Electronically Held: OWNA	Until the end of 25 years after the last date on which the nominated supervisor or staff member provided education and care on behalf of the service. (Reason why 25 years retention is to hold staff record in case incident is raised in relation to child) Location: 2 years at the centre, then archived at an approved location until 25 years.	Reg183(2)(f)
Allegations of reportable conduct against a staff member: ☐ Details of planning process for investigation ☐ Risk assessment records ☐ Details of information provided ☐ Records of interview ☐ Statements and written submissions ☐ Findings ☐ Action ☐ Final correspondence ☐ Any other documentation	Head of Agency Records kept in accordance with Child Protection in the Workplace Guidelines	Stored at Human Resources & Payroll Department	Required to be reported to Department and Ombudsman and registered permanently . Until the end of 25 years after the last date on which the nominated supervisor or staff member provided education and care on behalf of the service. Location: 2 years at the centre, then archived at an approved location until 25 years.	Reg183(2)(a), (b), (c)
OHS&W -Workplace Safety Audit -Daily Safety Checks -Evacuations -Material Safety Data Sheets -Work method statements	Centre Director & Staff required to carry out hazard identification, risk assessment & elimination or control of risks. Written documentation of audits, hazard reports, evacuations, checks of first aid, WMS, MSDS	Current audits and other required documentation kept in Audit file. Electronically Held: OWNA and MS Teams	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years..	Reg183(2)(a), (b)
Emergency and evacuation procedures	Approved Provider Service to maintain evacuation practice records/evaluation. Stored in office.	Electronically Held: OWNA and MS Teams Stored in the office.	Until the end of 3 years after the date on which the record was made. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)
Financial accountabilities for Child Care Benefit (scheme) ☐ Statement of child care usage forms (copies) ☐ Child Care Benefit Statement for payment summary forms (copies) ☐ documents to support claims for Special Child	Authorised service representative ie service manager (coordinator) Service operator or delegate authorized to enter into a legal obligation All required documentation stored as both hardcopy and electronically (CentreCalc)	Stored in categorized folders at the Centre & electronically	3 years from the end of the calendar year in which the relevant care was provided. Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)

Care Benefit where appropriate ☐ Family Assistance Office CCB Statements ☐ certificates for approval of more than 20 hours care per week or more than 50 hours care for a child at risk Electronic storage of attendance, financial records, claims	in nominated files in accordance with established administrative processes; supporting documentation stored in relevant file.			
Where applicable: ☐ state or territory licence ☐ Special Needs Subsidy Scheme documentation ☐ Disabled Supplementary Services (DSUPS) claims ☐ requests for extension of 24-hour care (if used) ☐ Jobs, Education and Training (JET) Child Care document's Electronic storage of financial records and reports	Authorised service representative le service manager (coordinator) Maintained and stored by Centre in accordance with established administrative processes	Stored in categorized folders at the centre	3 years from the end of the calendar year in which the relevant care was provided Location: 3 years at the Centre or archived at an approved location after 2 years.	Reg183(2)(g)

Data Storage:

- All data, including enrolment information, child and family information, learning documentation, photos, and video recordings, will be stored securely on the OWNA platform and Office 365 using encrypted storage solutions.
- Access to stored data will be restricted to authorised personnel only, based on their roles and responsibilities.
- After the retention period, data will be reviewed for deletion, destruction or archiving.

Methods of Destruction:

- Documentation, digital images and videos will be permanently deleted using secure software that ensures they cannot be recovered.
- Physical copies will be shredded or incinerated to ensure complete destruction.
- Private and Sensitive Documentation: A record of destroyed materials will be maintained, including the date of destruction and the method used.
- Compliance: Destruction procedures will comply with applicable laws and regulations regarding data protection and privacy.

Document Management Procedures & Timetable

Documents	File name convention	Filing Order	Triggers	Archiving Cycle - When to Complete
Child Information	Child ID_Last Name	Order document as per stated archive cover page	Enrolment Ends	Weekly
Individual Staff Information	Last Name First Name_End Date	Staff Last Name_First Name	End of Employment	Monthly
Room Attendance Records	Start Date	Date	Completion of Weekly Rolls	Weekly
OHS&W	"Start Period Date Date" to "End Period Date"_OHS	Date	Completion of Checklists and OHS&W Records	Monthly
People Management Information	"Start Period Date Date" to "End Period Date"_People	Date		Monthly
Compliance and Licensing	"Start Period Date Date" to "End Period Date"_Compliance	Date		Monthly
Education Records - Room	"Start Period Date Date" to "End Period Date"_Room Name Eg. 2011 04 91 to 2011 04 30_Koala	Date_Room		Monthly
Financial Records	"Start Period Date Date" to "End Period Date"	Date	End of Week Tasks Completion	Weekly

Procedure for Scanning of Documents

1. Collations of information for scanning into filing subject
2. Prepare Archiving Cover Sheets for each document batch
3. Place document batch in scanner
4. Select relevant pre-set button for document type
5. Scan → Go
6. Confirm document has been scanned to directory
7. Name file in accordance with file convention (this is important step to be able to find the document in the future).
8. Repeat from step 1.

Saving of Documents to Directory: Create a year folder for each new financial year

Year Folder	Sub-Folders
2024	Child Information
	Individual Staff Information
	Room Attendance Records
	OHS&W
	People Management Information
	Compliance and Licensing
	Education Records - Room
	Financial Records

1	ARCHIVING COVER SHEET FOR:	
	1.CHILD INFORMATION	
	ENROLMENT ID:	
	CHILD LAST NAME:	
	CHILD FIRST NAME:	
	DOB: / /	
	LAST DAY OF CARE (DATE): / /	
	DATE FILED: / /	
1A	INCLUDES: Child Enrolment Enrolment form Sighted copy of birth certificate, passport or citizenship certificate Re-enrollment form Orientation Checklist Written notice of withdrawal Holidays Notice Form Supporting documentation for approved absences for each child Electronic records – child details; family details	
1B	Child Health Medical details on enrolment form (above) Immunisation records Doctor Certificates Medicare details & Private Health Insurance details if held Administration of medication Illness/minor accident form Accident report form Insurance Claim Forms Special diet form Food allergy management instruction form Medical management plan	
1C	Excursions Non routine excursion permission forms signed & dated by parents	
1D	Child Development Records Chart of Achievement Records Transition Statement Not art and crafts required to be kept on file.	
1E	Mandatory report of child at risk (if applicable)	
1F	Child Subsidy, Grants and Funding Information Special CCB Letter: Applications, Claims, Corro and Approvals JET Information: Applications, Claims, Corro and Approvals ISS: Applications, Claims, Corro and Approvals	

2	ARCHIVING COVER SHEET FOR:	
	1.INDIVIDUAL STAFF INFORMATION	
	STAFF ID:	
	STAFF LAST NAME:	
	STAFF FIRST NAME:	
	LAST DAY OF EMPLOYMENT (DATE):	
	DATE FILED: / /	
2A	INCLUDES: Staff Records Employee Information Form Letter of Employment/Contract Resignation / Termination Letters Immunisation information	
2B	Qualifications (staff)	
2C	Staff Leave Leave form information Doctor Certificates	
2D	Mandatory training Induction Checklist First Aid Anaphylaxis Child Protection Training Working With Children Checks Police Check Staff first aid currency stored electronically, hard copy on file	
2E	Other training documentation	
2F	Staff Development Staff Appraisals & Professional Development	
	Allegations of reportable conduct against a staff member ☐ Details of planning process for investigation ☐ Risk assessment records ☐ Details of information provided ☐ Records of interview ☐ Statements and written submissions ☐ Findings ☐ Action ☐ Final correspondence ☐ Any other documentation ☐ Notification to Authority	

3	ARCHIVING COVER SHEET FOR:	
	2.ROOM ATTENDANCE RECORDS	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
3A	INCLUDES: Attendance Information Attendance sheets – sign in and out Receipts for fees paid by parents – receipt book Electronic records – contracts; paid hours	

4	ARCHIVING COVER SHEET FOR:	
	3.OHS&W	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
4A	INCLUDES: Workplace Safety Audit Daily Safety Checklists Cleaning Checklists Evacuations Checklists Material Safety Data Sheets Work method statements	
4B	Visitors to the Centre Visitor's Register maintained	
4C	Fire Testing Reports	
4D	Maintenance Reports	

5	ARCHIVING COVER SHEET FOR:	
	4. PEOPLE MANAGEMENT INFORMATION	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
5A	INCLUDES: Timesheets Emailed to <u>payroll@mychildehc.com.au</u> each week Timesheet Summary and Employee Weekly Timesheets	
5B	Rosters – Emailed to <u>payroll@mychildehc.com.au</u> each week Staff Rosters (Presented to Staff) Roster Management Spreadsheets	
5C	Staff Registers for Rooms	
5D	Staff Meeting Agendas, Attendance & Minutes	
5E	Training Attendance Registers & Feedback Forms	

6	ARCHIVING COVER SHEET FOR:	
	5.COMPLIANCE & LICENCING	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
6A	INCLUDES: Child Care Licence	
6B	Licence Compliance Matters	
6C	Licence Correspondence	
6D	Food Licensing External Audits Internal Audits Checklists Monitoring Schedules	
6E	Complaints Register Complaints Register and Written Complaints and Responses	

7	ARCHIVING COVER SHEET FOR:	
	6. EDUCATION RECORDS - ROOM	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
	INCLUDES:	
7A	Education and Curriculum Records Room programs Room education management	
7B	Child Routine Information - Room Records Routine Records Nappy and Toilet Records Sleep Records	

8	ARCHIVING COVER SHEET FOR:	
	7.FINANCIAL RECORDS	
	PERIOD FROM: / / TO / /	
	DATE FILED: / /	
	INCLUDES:	
8A	Tax Invoices Expenses - Suppliers	
8B	Petty Cash Reports and Tax Invoices	
8C	Grants, Subsidies and Funding ISS Subsidies: applications, claims forms, approval letters Kindergarten Funding: applications, claims forms, approval letters	
8D	CCS	

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/6/2026

Next Policy Review: 15/6/2027

References:

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations
Privacy Act 1988

NQS

QA4	4.1.1	<i>Educator-to-child ratios and qualification requirements are maintained at all times.</i>
	4.2.1	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.2	<i>Educators, co-ordinators and staff members work collaboratively and affirm, challenge, support and learn from each other to further develop their skills, to improve practice and relationships</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>
QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Regulations

Reg	55	<i>Quality improvement plans</i>
	125	<i>Application of Division 4</i>
	126	<i>Centre-based services – general educator qualifications</i>
	158	<i>Children's attendance record is to be kept by approved provider</i>
	161	<i>Authorisations to be kept in enrolment record</i>
	162	<i>Health information to be kept in enrolment record</i>
	167	<i>Record of service's compliance</i>
	173	<i>Prescribed information to be displayed</i>
	177	<i>Prescribed enrolment and other documents to be kept by approved provider</i>
	180	<i>Evidence of prescribed insurance</i>
	181	<i>Confidentiality of records kept by approved provider</i>
	183	<i>Storage of records and other documents</i>
	184	<i>Storage of records after service approval transferred</i>

85 – DIABETES POLICY

Position Statement

Diabetes can be life threatening. We are committed to providing, as far as practicable, a safe and healthy environment in which individuals at risk of diabetes can participate equally in all aspects of the service program and experiences. We will aim to minimise the risk of a diabetes illness for those attending our service by implementing the following policy and procedures.

Diabetes is one of the most common chronic diseases of childhood and affects 1-2 per 1000 children and adolescents under 20 years. Appropriate diabetes care in the child care setting is important for the immediate and long term welfare of the child and to optimise their behavioural and academic development.

Objective

The centre will facilitate effective care and health management of children who have diabetes and the prevention and management of acute episodes of illness and medical emergencies.

Diabetes training is required to be arranged by the guardian/parents of the child and the training delivered by a professional medical consultant before the commencement of enrolment or at the time child is diagnosed of diabetes. Further, a Diabetes Care Plan and Blood Glucose Monitoring Plan must be completed by a medical practitioner/ specialist and updated on a periodic basis (annually or change of medical condition) to ensure adequate care can be provided to the child.

Services are required to ensure that each student with type 1 diabetes has a current individual diabetes management plan prepared by that child's medical specialist team, usually consisting of an endocrinologist, diabetes nurse educator and other allied health professionals.

Definitions

Type 1 diabetes

Type 1 diabetes is an autoimmune condition which occurs when the immune system damages the insulin producing cells in the pancreas. This condition is treated with insulin replacement via multiple injections or a continuous infusion via a pump. Type 1 diabetes is fatal without insulin therapy. Type 1 diabetes affects between 10 and 15 per cent of people with diabetes and half of those are diagnosed before the age of 16.

Type 2 diabetes

Type 2 diabetes occurs when either insulin is not working effectively (insulin resistance) or the pancreas does not produce sufficient insulin (or a combination of both). Type 2 diabetes affects between 85 and 90 per cent of all cases of diabetes and usually develops in adults over the age of 45 years, but it is increasingly occurring at a younger age. Type 2 diabetes is initially managed with a healthy life style and often medication that could include tablets and/or insulin.

Endocrinologist

The medical doctor who specialises in diabetes and conditions of the endocrine system.

Glucagon

A hormone, naturally produced by the pancreas, which causes an increase in the blood glucose level. A manufactured Glucagon injection can be administered to counteract severe hypoglycaemia where a child cannot eat or drink sugar.

Hypoglycaemia

Hypoglycaemia (low blood glucose) is a blood glucose level that is lower than normal, ie. below 4mmol/l, even if there are no symptoms. Neurological symptoms can occur at levels below 4mmol/l. Symptoms include sweating, tremor, headache, pallor, poor co-ordination and mood changes. It is important to treat hypoglycaemia promptly and appropriately to prevent the blood glucose level from falling even lower, as very low levels can lead to loss of consciousness and convulsions. The child may need assistance with treatment. Hypoglycaemia can impair concentration, behaviour and attention. Hypoglycaemia is often referred to as a 'hypo'.

Hyperglycaemia

Hyperglycaemia (high blood glucose levels) occurs when the blood glucose levels rise above 15 mmol/L. Hyperglycaemia can cause tiredness, irritability, urinating more frequently. High blood glucose levels can also affect thinking, concentration, memory, problem solving and reasoning.

Insulin

A protein hormone, produced on the pancreas. It allows cells to utilise glucose for energy.

Ketones

Acids formed in the body due to insulin deficiency. Checking for ketones in the blood or urine while at school will be included in the individual student's diabetes management plan if required.

Diabetes Management

While at a service diabetes management tasks can include:

- **Checking blood glucose levels** using a blood glucose monitor and finger pricking device. The child's diabetes management plan will state the times and the method of relaying information about any changes in blood glucose levels between the family and service. Depending on the child's age, services often choose to establish a communication book system to relay health information.
- **Administration of insulin during service hours** may or may not be required in the child's diabetes management plan.
- Should you have a child whose health condition(s) requires additional care and attention during service hours, consultation is required with the parents/guardians and health professionals to ensure that Educators are undertaking tasks within their scope of practice and training. Educators are under no obligation to administer insulin or glucagon.
- **Infection control procedures** must be followed which includes having instruction about ways to prevent infection and cross infection when checking blood glucose levels and administering insulin, hand washing, one child one device, disposable lancets and syringes and the safe disposal of all medical waste.
- **Treating hypos** promptly and appropriately.
- Most **meal requirements** will fit into regular classroom routines.
- Preparation for **physical activities**.
- A diabetes management plan is required to document each time a children attends a **excursion**. The excursion plan is prepared by the child's diabetes management team
- **Special event participation** including classroom parties can include children with type 1 diabetes in consultation with their parents/guardians

Strategies for Managing Diabetes

When a service has a child with type 1 diabetes, service staff are encouraged to:

- Communicate directly with the parents/guardians to ensure the child's individual diabetes management plan is current. This should also include a separate excursion plan if required

- Discuss, document and allocate any diabetes tasks that require the help of and/or supervision by service staff
- Depending on the age of the student, establish a home- to-school means of communication to relay health information. For example a communication book, emails or text messaging
- Support teachers and school staff to seek additional information and attending training about type 1 diabetes.
- Follow the child's diabetes management plan
- Follow the services procedures for medical emergencies for children with type 1 diabetes
- Ask the parents/guardians to notify the services of any changes to the child's individual diabetes management plan
- Contact the parents and/or members of the child's diabetes management team and/or DA–Vic should you require additional support or information to support a student with type 1 diabetes.

Implementation:

To facilitate effective care for a child with diabetes it is necessary to form a partnership between the centre and the child's family with responsibilities for both, and centres should:

• Ensure the family, parent or guardian provides the centre with the following information on the care plan:

1. Child information. The name and a photograph of the child. The details of the child's health problem, treatment, medications and allergies. Who to contact in the case of medical emergency and their current contact details (this will also be on child's enrolment forms).

2. Their doctor's name, address and phone number, and a phone number for contact in case of an emergency

3. A Diabetes Care Plan following enrolment and prior to the child starting at the centre which should include:

- when, how and how often the child is to have finger-prick or urinalysis glucose or ketone monitoring
- what meals and snack are required including food content, amount and timing
- what activities and exercise the child can or cannot do
- whether the child is able to go on excursions and what provisions are required.

4. A Diabetes First Aid or Emergency Medical Plan following enrolment and prior to the child starting at the centre which should include:

- what symptoms and signs to look for that might indicate hypoglycaemia (low blood glucose) or hyperglycaemia (high blood glucose)
- reportable levels of hyperglycaemia and the need for ketone testing
- If the child is able to self- care for all the day-to-day diabetes management tasks or requires help and/or supervision
- what action to take including emergency contacts for the child's doctor and family or what first aid to give.

• In any medical emergency involving a child with diabetes, the centre staff should immediately dial 000 for an ambulance and notify the family in accordance with the Regulation and guidelines on emergency procedures, **and administer first aid or emergency medical aid according to the** child's Diabetes First Aid or Emergency Medical Plan, or a doctor's instructions, or if these are not available, use the First Aid Plan for Diabetic Emergency from *Australian First Aid*, St. John Ambulance Australia, 2020, on the following pages within this policy.

• Ensure at least one staff member who has completed accredited training in emergency diabetes first aid is present in the centre at all times whenever children with diabetes are being cared for in the service. Parents of child are responsible for arranging diabetes first aid training for staff in consultation with service.

• Ensure the family supplies all necessary glucose monitoring and management equipment.

- **Ensure the family and centre staff know it is not the responsibility of the centre staff to administer a child's insulin**, or to administer parenteral injections of glucose or glucagon in an emergency. Ensure the family understands that a child's insulin should be administered before or after care in the centre.
- **Ensure there is a staff member who is appropriately trained to perform fingerprick blood glucose or urinalysis monitoring** and knows what action to take if these are abnormal.
- **Ensure there are glucose foods or sweetened drinks readily available to treat hypoglycaemia (low blood glucose)**, eg. glucose tablets, glucose jelly beans, Lucozade.
- **If a child has had an episode of hypoglycaemia and needed glucose food or drink**, also provide the child with a slow-acting carbohydrate food to help maintain blood glucose levels, eg. milk, raisin toast, yoghurt, fruit.
- **Ensure a location in the centre for privacy for the child to do their own glucose monitoring or insulin administration** if the child is able.
- **Ensure availability of meals, snacks and drinks** that are appropriate for the child and are in accordance with the child's Diabetes Care Plan.
- **Ensure opportunity for the child to participate in any activity, exercise or excursion** that is appropriate and in accordance with their Diabetes Care Plan.

Please refer to 87- Medical Conditions Policy: Appendix 1 for the procedures to complete a 'Medication Record' and an 'Incident, Injury, Trauma and Illness Record' in OWINA when child's Medical Management Plan is enacted and medication is given to child.

St. John First Aid for Diabetic Emergency

Source: from St. John Ambulance Australia,

© St. John Ambulance Australia, *Australian First Aid*, April 2020

DRSABCD action plan

In an emergency call triple zero (000) for an ambulance



D DANGER

Ensure the area is safe for yourself, others and the patient.

R RESPONSE

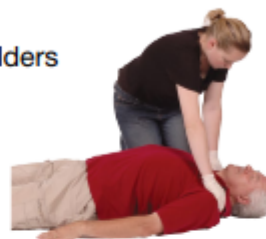
Check for response—ask name—squeeze shoulders

No response

- Send for help.

Response

- make comfortable
- check for injuries
- monitor response.



S SEND for help

Call Triple Zero (000) for an ambulance or ask another person to make the call.

A AIRWAY

Open mouth—if foreign material is present:

- place in the recovery position
- clear airway with fingers.

Open airway by tilting head with chin lift.



B BREATHING

Check for breathing—look, listen and feel.

Not normal breathing

- Start CPR.

Normal breathing

- place in recovery position
- monitor breathing
- manage injuries
- treat for shock.



C CPR

Start CPR—30 chest compressions : 2 breaths

Continue CPR until help arrives or patient recovers.



D DEFIBRILLATION

Apply defibrillator if available and follow voice prompts.

© St John Ambulance Australia. St John encourages first aid training as this information is not a substitute for first aid training.

Learn First Aid | 1300 360 455 | www.stjohn.org.au

Diabetic Emergency

A diabetic emergency may result from too much or too little insulin in the blood. There are two types of diabetic emergency – very low blood sugar (hypoglycaemia, usually due to excessive insulin); or very high blood sugar (hyperglycaemia, due to insufficient insulin). The more common emergency is hypoglycaemia. This can result from too much insulin or other medication, not having eaten enough of the correct food, unaccustomed exercise or a missed meal.

Signs and symptoms:

If caused by **low blood sugar**, the person may:

- Feel dizzy, weak, trembly and hungry
- Look pale and have a rapid pulse
- Be sweating profusely
- Be numb around lips and fingers
- Appear confused or aggressive
- Be unconscious

If caused by **high blood sugar**, the person may:

- Be excessively thirsty
- Have a frequent need to urinate
- Have hot dry skin, a rapid pulse, drowsiness
- Have the smell of acetone (like nail polish remover) on the breath
- Be unconscious

TIP – If unsure whether attack is caused by low or high blood sugar, give a sweet (sugar-containing) drink. Do not use 'diet' soft drinks. This could save the person's life, if blood sugar is low, and will not cause undue harm if blood sugar is high.

Management of Diabetic Emergency

Caused by Low Blood Sugar

If casualty unconscious:

1. Follow **DRSABCD**
2. Give nothing by mouth
3. Call **000** for an ambulance

If casualty conscious:

1. Give sugar, glucose or a sweet drink (e.g. soft drink or cordial – do not use 'diet' soft drinks or diabetic-type cordials).
2. Continue giving sugar every 15 minutes until medical aid arrives or casualty recovers.
3. Loosen tight clothing.
4. Seek medical aid if required.

Caused by High Blood Sugar

If casualty unconscious:

1. Follow **DRSABCD**
2. Give nothing by mouth
3. Call **000** for an ambulance

If casualty conscious:

1. Allow casualty to self-administer insulin (do not administer it yourself, but help if needed). **A casualty who has diabetes may carry a NovoPen™ to inject insulin.**
2. Seek medical aid if required. If help delayed, encourage casualty to drink sugar-free fluids.

Diabetes Management, Communication & Risk Minimisation Plan

CONFIDENTIAL

To be completed by the RELEVANT DIABETES SPECIALIST and the PARENT/GUARDIAN. This information is confidential and will be available only to supervising staff and emergency medical personnel.

Attach Photo of Child

Name of child:
First Name and Family name (please print)

Date of birth:

Medic Alert number (if relevant):

Review date:

Parent Name:
First Name and Family name (please print)

Parent Contact Telephone number: Mobile:
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ROUTINE SUPERVISION FOR SAFETY

Staff members routinely support safe diabetes self-management in the following ways:

- Ensure supervising staff know of the child's/student's diabetes and his or her routine and emergency support plans
- Encourage, in consultation with the child/student, a supportive buddy system with peers
- Enable the child/student to eat at additional times, especially in relation to physical activity
- Enable ready access to the toilet
- Ensure supervision if unwell
- Ensure privacy if testing for blood glucose levels/injecting of insulin is required at school
- Provide a written log, as requested, of any 'hypos' and the action taken while supervised by education/care staff.

Individual routine support needs

Is this child usually able to self-manage his or her diabetes care? ☐ Yes ☐ No (Please tick)

If no, please detail assistance requested from staff to support safety and developing self-management.

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If staff or the child/student is concerned, emergency contact(s) will be informed. A health professional may be nominated by the family to be an emergency contact person as relevant. Please nominate emergency contact and any different/additional steps in relation to this child's/student's management.

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FIRST AID

Observable signs of hypoglycaemia (a 'hypo')—low blood glucose

A 'hypo' can be caused by too much insulin, not enough food or a missed meal and unplanned or unusual exercise. The signs can progress from mild to severe.

Treatment is needed promptly to prevent a mild 'hypo' from progressing to a severe 'hypo'.

Mild - Sweating, paleness, trembling, hunger, weakness - Changes in mood and behaviour (crying, argumentative, outbursts, aggressiveness) -Inability to think straight, lack of coordination	Moderate -Inability to help oneself -Glazed expression -Being disorientated, unaware or seemingly intoxicated -Inability to drink and swallow without much encouragement -Headache, abdominal pain or nausea	Severe - Inability to stand - Inability to respond to instructions - Extreme disorientation - Inability to drink and swallow (leading to danger of inhaling food into lungs) -Unconsciousness or seizures (jerking or twitching of face, body or limbs)
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FIRST AID in response to these observable signs of low blood glucose

Give sugar immediately	to raise blood sugar level (eg half a can of normal with sugar—soft or fruit drink, or 5–6 jellybeans).
Wait	5 minutes.
If no improvement	repeat soft drink/jelly beans and wait further 5 minutes.
If condition improves	follow up with a snack of one piece of fruit or one slice of bread or dried biscuits only when recovered (usually 5 minutes).
If still no improvement	call ambulance. State clearly that the person has diabetes, and whether he or she is conscious. Inform emergency contacts.
If unconscious	maintain A irway B reathing C irculation (ABC) while awaiting ambulance.

Can this child usually tell that he or she is developing a 'hypo'? ☐ **Yes** ☐ **No** (Please tick)

If yes, please detail how early he or she is likely to recognise the 'hypo' and the action he or she typically takes.

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Observable signs of hyperglycaemia—high blood glucose

Hyperglycaemia (high blood glucose levels) can be caused by insufficient insulin, too much food, common illness (eg a cold) and stress.

Signs for this condition emerge over two to three days and can include:

-Frequent urination -Weight loss -Change in behaviour (usually irritability)
 -Excessive thirst -Lethargy

FIRST AID in response to these observable signs of high blood sugar levels

Staff members often become aware of these signs when the child/student is constantly going to the toilet. Emergency contacts should be informed if these signs are observed.

An ambulance should be called immediately if any of the following is observed:

-Rapid, laboured breathing -Abdominal pains -Vomiting
 -Flushed cheeks -Sweating acetone smell to the breath - Severe dehydration

If you anticipate this child will require something other than this standard first aid response, please provide detailed written recommendations so individual care arrangements can be discussed with staff.

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TREATMENT

Will any visiting/consulting nurses be supporting this child? ☐ **Yes** ☐ **No** (Please tick)

(If yes, please provide details)

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Will child require blood glucose monitoring be taken using a bloodletting device ? ☐ **Yes** ☐ **No** (Please tick)

(bloodletting device is a fringer prick test)

Can the child use the bloodletting device independently? ☐ **Yes** ☐ **No** (Please tick)

(If yes, please provide details)

Has training been provided to Educators on the use of bloodletting device and glucose monitoring? ☐ **Yes** ☐ **No** (Please tick)

(If yes, please provide details, including date of training and names of educators)

Name of Educators:

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Date of Training: Training performed by:

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RISK MINIMISATION PLAN

Predominant Diabetes Trigger/s:

Other Triggers:

Risk	Strategy	Who is Responsible?

AUTHORISATION AND RELEASE

Diabetes specialist: Professional role:

Name of agency/address:

..... Telephone

Signature Date

I have read, understood and agreed with this plan and any attachments indicated above. I approve the release of this information to education/childcare staff and emergency medical personnel.

Further, I provide permission for the staff member to assist with the bloodletting finger prick test required for blood glucose testing ☐ Yes ☐ No (Please tick) ☐ Not required (Please tick)

Parent/guardian: Signature: Date
First name (please print) Last name (please print)

Blood Glucose Monitoring Guidelines & Plan

Name of child: Date of birth:

Medic Alert number (if relevant): Review date:

Children and adolescents with diabetes may be required to test their blood glucose level, for a variety of reasons, during service hours.

Equipment

It is recommended that all equipment relating to blood glucose monitoring procedure is kept in a safe place in order to minimise the risk of any other person/child being exposed or having access to the invasive blood letting devices (finger prick test device).

Supervision and assistance

Children and adolescents may be able to undertake the blood letting device (finger prick test device) procedure independently. However, all children and adolescents need to be supervised and require assistance with the decision-making process outlined in the guidelines, based on the test results obtained. Educators and other staff MUST be trained to be permitted to actively participate in the use blood letting device (finger prick testing device). If NOT trained, Educator are NOT allowed to perform procedure. The provision of training must be documented on the Care Plan.

Guidelines

Testing times (please tick)

- ☐ Before recess
- ☐ Before lunch
- ☐ Before sporting/physical activities
- ☐ Any other time, please specify:

Hands should be washed prior to blood glucose testing, the equipment should be stored safely and the blood glucose test result recorded in the communication book.

Test result outcome and action	
4 and below	Treat as per hypoglycaemia protocol.
4–15	NO ACTION, unless the person is complaining of symptoms, OR (please specify)
Above 15	No symptoms —record and inform parent or guardian at the end of the day. Symptomatic (ie complaining of feeling unwell, thirsty and/or vomiting)—follow hyperglycaemic protocol and inform parent or guardian immediately as the person may require additional insulin and/or medical attention.

Diabetes specialist: Professional role:

Name of agency/address:
..... Telephone

Signature Date

I have read, understood and agreed with this plan and any attachments indicated above. I approve the release of this information to education/childcare staff and emergency medical personnel. Further, I provide permission for the staff member to assist with the finger prick test for blood glucose monitoring.

Parent/guardian: Signature: Date:
First name (please print) Last name (please print)

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and staff verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

EYLF

Reg	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

Handbook on Child Care Licensing

Sources and Further Reading:

- Information source from *Australian First Aid*, 10/2002 and is the copyright of St. John Ambulance Australia. This information is not a substitute for first aid training. St. John recommends that everyone is trained in first aid. For more information on St. John first aid training and kits visit www.stjohn.org.au or call toll free 1300 360 455.
- Health and Safety in Children's Centres: Model Policies and Practices: 2003.
http://www.community.nsw.gov.au/docswr/assets/main/documents/childcare_model_policies.pdf pages 76 - 80
- Diabetes; Planning and Support guide for education and childcare services; Department of Education and Children Services, Government of South Australia, DECS Publishing.
http://www.decd.sa.gov.au/speced2/files/pages/chess/hsp/Pathways/DIABETES_BOOK.pdf
- Diabetes Australia – Victoria. Draft model policies for Diabetes Management in Schools and Services – 30 May 2012.

86 – ACCEPTANCE AND REFUSAL OF AUTHORISATION POLICY

Position Statement

Having policies and procedures for authorisation makes sure children are safe while being educated and cared for and helps the service be confident that it is meeting its legal obligations.

Objective

Services must obtain authorisation from parents and authorised nominees to document the acceptance and refusal of authorisations. Further, it is the authorised parent's responsibility to provide the acceptance and refusal of authorisation to the service upon request.

Procedure

The following procedure must be adhered to at all times to ensure the safety of the children:

- Services must obtain authorisation from authorised parents or authorised nominees for events, tasks and procedures requiring permission or consent.
- Acceptance and refusal of authorisation can only be obtained from authorised nominees or parents who enrolled the child. Update of enrolment record or change form for updating authorised nominees.
- Un-authorised people cannot provide acceptance or refusal authorisations.
- The authorisation must include a 'signature from authorised parent or authorised nominee' and a 'date' of signing. For instructions, please refer to the specific form requiring authorisation. The signature can be electronic. I.e using OWNA forms.
- When completing forms, all fields are required to be completed. If the field or question is not applicable please write Not Applicable or N/a.
- If unable to read or write, please consult the Nominated Supervisor or Responsible Person in Charge of Service to discuss this matter and they will provide assistance in completing acceptance and refusal of authorisation.
- If acceptance or refusal authorisation (no response) is not provided for an excursion the child will be unable to participate in the excursion and will participate in other educational programs.
- Further, if authorisation or refusal authorisation is not provided by the authorised person the service reserves the right to terminate or suspend the enrolment of the child on the basis that the service is unable to provide a safe environment and/or practices for the child when being educated and cared for or the services is unable to meet its legal obligations.
- Nominated Supervisors, Responsible Person in Charge of Service and Educators are responsible for implementing this policy on a day-to-day basis.
- Authorisations may be provided away of defined service forms or applications and received by hand, fax or email. The forms must contain an authorised parents signature and date.

Example of Practices

Examples of when accepting and refusal of authorisations are required:

- administering medication to children (regulation 92)
- children leaving the premises in the care of someone other than their parent (regulation 99)
- children being taken on excursions (regulation 102)

Common Practices

1. Enrolment of Child: Completing the enrolment form requires the authorised person to answer questions and provide information about the child's enrolment. All questions are required to be answered and information provided for the enrolment form to be effective.

2. Excursion Permission Form: Completing this form requires a signature and date on a printed excursion form but a parent only gives verbal consent. You can refer them to this policy and explain that you are NOT able to accept authorisation until they sign and date the form. Without a written acceptance authorisation, a child will not be allowed to attend or participate in the excursion. Instead, they will participate in other learning activities.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/8/2025

References:

NQF

QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
QA7	7.1	<i>Governance supports the operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	168 (2)	<i>Policies and procedures are required in relation to the following – (m) the acceptance and refusal of authorisations</i>
Reg	182	<i>children leaving the premises in the care of someone other than their parent (regulation 99)</i>
Reg	102	<i>children being taken on excursions (regulation 102)</i>
Reg	115	<i>administering medication to children (regulation 92)</i>

87 – MEDICAL CONDITIONS POLICY

Position Statement

Medical conditions can be life threatening. We are committed to providing, as far as practicable, a safe and healthy environment in which individuals at risk of medical conditions can participate equally in all aspects of the service program and experiences.

Objective

The service and all educators can effectively respond to and manage medical conditions including asthma, diabetes and anaphylaxis at the service to ensure the safety and wellbeing of children, staff and visitors. The following policy and procedure provides guidance in relation to medical condition management, risk minimisation and communication plans.

Related Policies and Procedures

Please refer to the following policies and procedures for guidance on specific medical conditions and practices:

- Anaphylaxis Policy – 35
- Asthma Policy – 36
- Diabetes Policy – 85
- Illness Policy - 11
- Medication Policy – 33
- Coronavirus Policy - 98

Medical Management Plan: a medical management plan, risk minimisation and communication Plan. The risk minimisation plan and communication plan for each medical condition is documented in our specific policy and procedures (see above) and below policy and procedure.

Implementation

The service will involve all educators, families and children in regular discussions about medical conditions and general health and wellbeing throughout our curriculum. The service will adhere to privacy and confidentiality procedures when dealing with individual health needs.

A copy of the Medical Conditions Policy must be provided to all educators and volunteers at the service. The Policy must also be provided to parents of children enrolled at the service. Educators are also responsible for raising any concerns with a child's parents about any medical condition known to the service, or any suspected medical condition that arises.

No child enrolled at the service will be able to attend the service without medication prescribed by their medical practitioner. Families are required to provide this information on the Enrolment Form as outlined below and are responsible for updating the service on any new medication, ceasing of medication, or any changes to their child's prescription.

Please refer below to Appendix 1 (below): Procedures to complete 'Medication Record' and 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Management Communication Plans

Forms of management communication plans for medical conditions include:

- Parents to continuously and periodically update enrolment information; through updating current enrolment form or completing a new enrolment form.

- Completion of Care Management Plans and Forms by parents including Long Term Illness Notification Form, Allergy Action Plan, Asthma Action Plan, Medication Form (HEI002 and OWNA), HEI003a_341 NAC-Asthma Action Plan, HEI003c_Action Plan Ana Pen, HEI003d_Action Plan Epi pen, HEI003f_Anaphylaxis_Event_Record proforma, HEI003g_Allergy Action Plan
- In some cases child health communication book will be created for parents and educators to communicate and manage child health and medical condition.

It is the responsibility of parents and authorised nominees to provide the service with information in relation to medical conditions.

The service aims at reminding parent through the use of service newsletters, parent notice board and other correspondence.

Medical Condition Communication Plan - Educators

- Each room displays medical conditions register (summary of children's medical conditions) and maintains a folder containing each child's medical condition management plan, risk assessment, and communication plan.
- Family grouping rooms will hold all children's medical condition management plan, risk assessment, and communication plans.
- Staff and educators receive information about the Service's procedures and policies regarding the management of children with diagnosed healthcare needs, allergies, and medical conditions during their induction and orientation programs. This is documented in the Educator and Staff Orientation Checklist and Buddy Checklist, which is signed by the educator and the Centre Director.
- All staff and educators are made aware of and familiar with the medical management plans, risk minimisation strategies, and communication plans through visible displays within the service. The Centre Director (or Responsible Person) ensures that all medical condition management plans, risk assessments, and communication plans are current and correctly stored in the folders and on Medical Condition Register Summary Display.
- Whenever there are new, edited, or updated Medical Condition Management Plans, the Centre Director (or Responsible Person in Charge of Service) will visit the rooms to inform educators about the changes.
- The Centre Director or Responsible Person will utilise the OWNA Message Board to communicate any new or updated children's medical condition management plans, risk assessments, and communication plans to all educators in the service.
- When a new child with a medical condition starts, the Centre Director (or Responsible Person) will notify all rooms via email, providing a copy of the medical management plan, update the medical conditions register and folder in each room.
- Educators will use a routine and mealtime checklist to ensure they have reviewed the medical conditions displays, allowing them to identify children's medication management, risk assessments, and communication plans before providing any food or drink.
- Educators review the Room Compliance and Risk Assessment Checklist to identify all children in care with medical conditions. They are required to read the child's medical condition management plan, risk assessment, and communication plan to ensure familiarity with the plans. The checklist also prompts educators to check that medical management plans are up to date and that any medications are not expired.
- Room educators discuss any changes to children's medical condition management plans, risk assessments, and communication plans during the morning minute meeting.

- Educators access the OWNA child profile to view the child's medical conditions management, risk assessments, and communication plans.
- Discussions about children's medical condition management, risk assessments, and communication plans take place during staff and room meetings.
- If an educator learns of a new or emerging medical condition from a parent, they must immediately notify the Centre Director (or Responsible Person). The Centre Director (or Responsible Person) will explain which medical condition management plan form needs to be completed and outline the relevant procedure. The child should not attend the service until this form has been completed by a parent.

Information that must be provided on Enrolment Form

The service's Enrolment Form provides an opportunity for parents to help the service effectively meet their child's needs relating to any medical condition.

The following information must be completed on the Enrolment Form, and any information will be attached to the Enrolment Form as necessary and kept on file at the service –

- Asthma
- Diabetes
- Allergies
- Anaphylaxis
- Diagnosed at risk of anaphylaxis
- Any other specific medical condition(s) mentioned by a child's parents or registered medical practitioner using the Enrolment Form.
- Any other specific medical condition(s) mentioned by a child's parents or registered medical practitioner at any point during the child's education and care at the service.
- Any Medical Management Plan put forward by a child's parents and/or registered medical practitioner. The Medical Management Plan must be used to inform the Medical Conditions Risk Minimisation Plan. Parents are responsible for updating their child's Medical Management Plan as necessary and will be regularly reminded by the service as per the Medical Management Communications Plan.

Identifying Children with Medical Conditions

- Any information relating to the above medical conditions will be shared with the Nominated Supervisor, educators, volunteers and any other staff member at the service. Individuals will be briefed by the Nominated Supervisor on the specific health needs of each child.
- Information relating to a child's medical conditions, including the child's Medical Management Plan, Medical Conditions Risk Minimisation Plan, and the location of the child's medication will be shared with all educators and volunteers and displayed in the following areas of prominence to ensure all practices and procedures are followed accordingly. The location of the information will be located in the classroom for reference by the Educators and also a copy will be held in the service office.

- All educators and volunteers at the service must follow a child's Medical Management Plan in the event of an incident related to a child's specific medical conditions requirements.
- All educators and volunteers at the service must be able to identify a child with medical conditions easily.
- All educators and volunteers at the service must be able to locate a child's medication easily.
- updated on the child's treatment along with any regulatory changes that may affect practices for specific medical conditions.
- Our service will display information about a child's medical management plan, risk minimisation plan, and the location of each child's medication in an area near a telephone that is visible and easily accessed by all educators eg food preparation or serving area to ensure all procedures are followed. We will ensure the display of information meets privacy guidelines and is not accessible to visitors or other families. We will explain to families why this is important for the safety of the child and obtain parental consent.
- Where a child has been diagnosed at risk of anaphylaxis, a notice stating this must be displayed at the service so it is clearly visible from the main entrance. The privacy and confidentiality of the child will be maintained at all times and the public notice will not name the child. The rooms will have a displayed schedule to inform educators of children's medical conditions.

Medical Conditions Management, Communication & Risk Minimisation Plan

Medical Condition Management Plans for the child must be developed, reviewed and updated by a medical practitioner (with the child's parent). Educators and parents will use this to create the risk minimisation and communication plans. Medical Condition Management Plan are required to be reviewed and updated by a medical practitioner every 12 months or when a review is advised by a Medical Practitioner (this maybe greater or less than 12 months). At the end of the renewal period, the parent will provide an updated or reviewed Medical Condition Management Plan (Including the ASCIA Action Plan, Asthma Action Plan) for the child signed and dated by the medical practitioner. The medical action plan will be provided by the parent to the service Centre Director (Nominated Supervisor) and this will be used to assess any changes to the communication and risk assessment plan.

Using a child's Medical Management Plan, the service will develop a Medical Conditions Risk Minimisation Plan (forms part of the Management Plan) in consultation with a child's parents. The Medical Conditions Risk Minimisation Plan must ensure that any risks are addressed and minimised. Our policies and procedures forms part of these risk minimisation plans. The Plan must be developed with the child's parents and medical professionals and these individuals must inform the Medical Conditions Risk Minimisation Plan. To promote consistency and ensure the welfare of all children using the service, we will follow all health, hygiene and safe food policies and procedures.

It is recommended that parents are given a reminder of 6 weeks if plans and medication is expiring. It is the responsibility of parents to ensure Medical Management Plans are keep current.

Any allergens that may be present at the service will be communicated to parents and addressed through the Medical Conditions Risk Minimisation Plan.

Whilst developing the Medical Conditions Risk Minimisation Plan and to minimise the risk of exposure of children to foods that might trigger severe allergy or anaphylaxis in susceptible children, the service will consider and implement the following –

- While not common, anaphylaxis is life threatening. Anaphylaxis is a severe allergic reaction to a substance. While prior exposure to allergens is needed for the development of true anaphylaxis, severe allergic reactions can occur when no documented history exists.
- Be aware that allergies are very specific to the individual and it is possible to have an allergy to any foreign substance.
- Anaphylaxis can be caused by insect bites such as bees or wasps but is usually caused by a food allergy. Foods most commonly associated with anaphylaxis include peanuts, seafood, nuts and in children eggs and cow's milk.
- Other common groups of substances which can trigger allergic reaction or anaphylaxis in susceptible children include:
 - All types of animals, insects, spiders and reptiles.
 - All drugs and medications, especially antibiotics and vaccines.
 - Many homeopathic, naturopathic and vitamin preparations.
 - Many species of plants, especially those with thorns and stings.
 - Latex and rubber products.
 - Band-Aids, Elastoplast and products containing rubber based adhesives.
- Educators should be on the lookout for symptoms of an allergic reaction, as per their training. Educators should be on the lookout for symptoms as they need to act rapidly if they do occur. Educators should immediately call 000 if symptoms arise. If you know an educator or child is prone to anaphylaxis reactions, and they carry an EpiPen® it should be injected by an educator trained in first aid. CPR should be initiated should the educator or child stop breathing.
- However, steps should be taken to prevent anaphylaxis occurring as outlined below:

Upon enrolment, seek medical information from parents about any known allergies. Ask parents for supporting documentation as well as a Medical Management Plan. This Medical Management Plan should include a photo of the child, what triggers the allergy, first aid needed and contact details of the doctor who has signed the plan. This should be kept on the child's enrolment file and also be displayed in the service, in an area where all educators can easily access near a telephone. A copy should also be kept where the child's medication is stored. If the child is taken on an excursion, a copy of the management plan should be taken on the excursion. Should a child be known to have allergies requiring medication if a reaction occurs, the parents will be asked to provide the medication. Furthermore, should the child's treatment change, families are asked to provide the service with a new Medical Management Plan from their child's medical practitioner. Documentation will then be updated at the service.

- If displaying personal information about children's or staff member's allergies in food preparation or serving areas, do so in accordance with privacy guidelines, such as displaying in an area accessible to staff and not easily accessible to visitors or other families. Explain to families the need to do so for purpose of safety of the child and obtain parental consent. Discuss with families where information will be displayed.

- Risk minimisation practices will be carried out to ensure that the service is to the best of our ability providing an environment that will not trigger an anaphylactic reaction in a child. These practices will be documented and reflected upon, with any practice that may be discovered amended to decrease risk. For example, a procedure to ensure that the child is never at the service without their EpiPen or relevant medication.
- The service will display an Australasian Society of Clinical Immunology and Allergy inc (ASCIA) generic poster called Action Plan for Anaphylaxis in a key location at the service, for example, in the children's room, the staff room or agreed location.
- Ensure that no child who has been prescribed an adrenaline auto-injection device is permitted to attend the service or its programs without the device.
- Develop an ongoing communication plan with the child's parents and with educators at the service to ensure that all relevant parties are updated on the child's treatment, along with any regulatory changes that may change the service's practices in regards to anaphylaxis.
- Provide support and information to the service's community about resources and support for managing allergies and anaphylaxis.
- The service will ensure that the auto-injection device kit is stored in a location that is known to all staff, including relief staff; easily accessible to adults (not locked away); inaccessible to children; and away from direct sources of heat.
- Routinely, the service will review each child's medication to ensure it hasn't expired.
- The service will not allow children to trade food, utensils or food containers.
- Ideally, children who have severe allergies should only be served food prepared at their homes. If it is decided that the child will have food prepared for them at the service, this will be prepared in line with their management plan and family recommendations.
- The service will use non-food rewards with children. For example, stickers for appropriate behaviour.
- Families are requested to label all bottles, drinks and lunchboxes etc with the child's name that they are intended for.
- The use of food products in craft, science experiments and cooking classes may need to be changed in order to allow children with allergies to participate.
- Food preparation staff will be instructed on the necessity to prevent cross contamination.
- Parents will be asked not to send food with their children that contain high allergenic elements even if their child does not have an allergy. For example, a sign in the foyer reminding families not to send food with high allergenic elements to the service even if their child doesn't have an allergy.
- If appropriate, a child with allergies may have to sit at a different table if food is being served that he/she is allergic to. This will always be done in a sensitive manner so that the child with the allergy does not feel excluded. If a child is very young, the

family may be asked to provide their own high chair to further minimise the risk of cross infection.

- When the child diagnosed at risk of anaphylaxis is allergic to milk, non-allergic babies will be held when they drink formula/milk.
- Restrict the use of foods likely to cause allergy in craft and cooking play.
- Always follow correct health, hygiene and safe food policies and procedures.
- Food preparation personnel (staff and volunteers) should be instructed about measures necessary to prevent cross contamination between foods during the handling, preparation and serving of food – such as careful cleaning of food preparation areas and utensils.
- All children need to be closely supervised at meal and snack times and consume food in specified areas. To minimise risk children will not be permitted to 'wander around' the service with food.
- Meals prepared at the service should not contain ingredients such as nuts. (If children have an allergy to eggs; appropriate cleaning and cross contamination strategies need to be implemented.)
- Risk minimisation plans provided by families will be consulted by the service when making food purchases and planning menus.
- The service will ensure that body lotions, shampoos and creams used on allergic children have been approved by their parent.
- Where a child is known to have a susceptibility to severe allergy or anaphylaxis refer to the "anaphylaxis policy", which would exclude children or other individuals visiting the service from bringing any foods containing nuts or nut products such as :
 - peanuts, brazil nuts, cashew nuts, hazelnuts, almonds, pecan nuts
 - any other type of tree or ground nuts, peanut oil or other nut based oil or cooking product, peanut or any nut sauce, peanut butter, hazelnut spread, marzipan
 - any other food which contains nuts such as chocolates, sweets, lollies, nougat, ice creams, cakes, biscuits, bread, drinks, satays, pre-prepared Asian or vegetarian foods
 - foods with spices and seeds such as mustard, poppy, wheat and sesame seeds
 - nut and peanut material is also often in cosmetics, massage oils, body lotions, shampoos and creams such as Arachis oil.
- Be aware that a child may have a number of food allergies or there may be a number of children with different food allergies, and it may not be possible to have an allergy free policy for all those foods involved. Nut allergy is the most likely to cause severe reaction and should take precedence.

- In the situation where a child who has not been diagnosed as allergic, but who appears to be having an anaphylactic reaction, staff will:
 - Call an ambulance immediately by dialling 000
 - ensure the first aid trained educator/educator with approved anaphylaxis management training provides appropriate first aid which may include the injection of an auto immune device EpiPen® in line with the steps outlined by the Australian Society of Clinical Immunology and Allergy <http://allergy.org.au/health-professionals/anaphylaxis-resources/ascia-action-plan-for-anaphylaxis> and CPR if the child stops breathing
 - Contact the parent/guardian
 - Contact the person to be notified in the event of illness if the parent/guardian cannot be contacted.
- Educators should be educated to recognise how serious anaphylaxis is and undertake the steps that need to be taken in order to minimise the possibility of occurrence. The service will maintain the following in relation to educator qualifications for anaphylaxis:
 - The service will ensure that all educators have completed first aid and anaphylaxis management training. After this, educators will complete training at least every 3 years from the date their qualification was issued.
 - The service will ensure that all educators in all services whether or not they have a child diagnosed at risk of anaphylaxis undertakes training in the administration of the adrenaline auto-injection device and cardio- pulmonary resuscitation every 12 months.
 - The service will also ensure that all relief educators used by the service adhere to these qualification requirements.
- Practice of adrenaline auto-injection devices will be undertaken regularly, preferably quarterly, and recorded. All licensed children's services are provided with Anaphylaxis Resource Kits by Victorian authorities, which will be used to practice with. This includes a training auto-injection device. This will be stored separately to a child's medication to ensure that there is no confusion.

Supervised Self-Administration of Medication by Children over Preschool Age

The service does not permit a child of any age to self-administer medication.

Appendix 1: Procedures to complete 'Medication Record' and 'Incident, Injury, Trauma and Illness Record' in OWNA when child's Medical Management Plan is enacted and medication is given to child.

Three step procedure for the administration of Medication in accordance with a Child Medical Management, Risk and Communication Plan. Ie for medical condition such as Asthma, Allergy, Anaphalaxis, etc.

Step 1: Setting up Medication Record in OWNA to Administer Medication per Medical Management, Risk and Communication Plan

Step 2: Complete Medical Record to document the administer of Medication per the Medical Management, Risk and Communication Plan.

Step 3: Complete Incident / Illness Record (see separate procedure for completion of this form). 'Incident, Injury, Trauma and Illness Record'

See below guidance on how to complete each of the above steps.

STEP 1: MEDICATION REPORT – SETTING UP MEDICATION RECORD TO ADMINISTER MEDICATION PER MEDICAL MANAGEMENT PLAN

1. In OWNA App or Portal educator created a "Medication Report"
2. If not an emergency, educator contacts parent to inform them medication is needed to be given to child per Medical Management, Risk and Communication Plan held at service.
3. Complete the Medication Report form below in OWNA Portal or App.

CREATE IN OWNA PORTAL

Add Medication Report

Home / Medication Reports / Add Medication Report

Should always be enrolment form. Can in addition be "verbal" to indicate spoke to parent also.

Parent spoken to.

Comments to include:

- Situation
- Confirm dosage to give per plan
- Confirm if medication given in last 24 hrs

Information per Medical Mgt Plan & checked to medication label for child. Exact Info.

Add new "Medication Report" details below:

Child*

- select child -

Type of authorisation received*

☐ Verbal ☐ Face to Face ☐ Written ☒ Enrolment Form

Name of Parent/Carer*

Date/Time Contacted*

 e.g. 2019-10-31



Comments

e.g. We rang mum and left voicemail but got in contact with dad.

Name of Medication*

Dosage to be Administered*

Method of Administration*

When to Administer? *

 e.g. 2019-10-31



Set expiry date to be the date administering medication per Medical Mgt Plan. ie 1 day expiry.

Expiry Date*	 e.g. 2020-10-31
Comments/Reason	e.g. Samuel had a temperature.

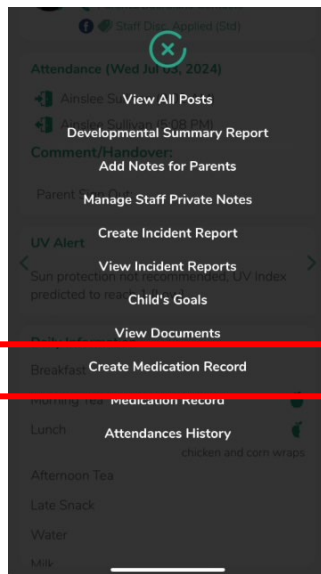
Upload Images, Video or PDF (up to 10)

Comments /Reason: Describe symptoms observed and reference actions noted in Child's Medical Management Plan

Drop files here or click to upload.
(Wait for files to be uploaded before submitting)

Add Medication to App

CREATE MEDICATION RECORD USING OWNA APP



Create Medication Record

A screenshot of the 'Medication Records' form in the OWNA app. The form has a green header bar with a back arrow, the text 'Medication Records', and a forward arrow. Below the header, there is a 'Select Child' section with a dropdown menu showing '<Select Child Name>'. The 'Type of authorisation received' section has four options: 'Verbal', 'Face to Face', 'Written', and 'Enrolment Form'. The 'Name of Parent/Carer' section has a dropdown menu showing 'Parent/guardian'. Below this are fields for 'Date/Time Contacted' and 'Comments'. A red rectangular box highlights the 'Verbal' and 'Enrolment Form' options.

Tick 'verbal' if contacted parent to discuss medical condition and enacting the medical management, risk & communication plan

Tick Enrolment Form: Medical Management, Risk & Communication Plan should be listed on child enrolment form.

A screenshot of the medication information fields in the OWNA app. The fields are: 'Name of Medication*', 'To Be Administered*', 'Dosage to be Administered*', and 'Method of Administration*'. A red rectangular box highlights these four fields.

Medication information: this is per the Medical Management Plan and Medication Label. (these should match and be checked by two educators they are the same)

Expiry Date*

Reason:
e.g. Samuel had a temperature. We rung mum and she gave us permission to give him Panadol

☐ Is the medication in its original container and bearing the original label with the name of child whom the medication is to be administered and where the medication must be stored.

Upload Images 



Expiry Date = date medication given. Set expiry date on this form to be the date administering medication. If it not 1 day will continue displaying in OWNA

Reason: Eg. Describe symptoms observed. Follow Medical Management Plan instructions.

Submit Medication Record. Go to Step 2 below.

STEP 2: MEDICATION RECORD – RECORD OF ADMINISTRATION OF MEDICATION

Person who administers medication should be person logged into OWNA App.

(STEP 1 is needed to be completed before step 2 is completed)

< Medication Records >

Medication Details

Name of Medication

Name of Medication (From Medication Record created in Step1)

Last Administered

This will be blank if no medication has been given for the same ‘Medication Record’.

Expiry Date

Expiry Date for Medication Record (from Medication Record created in Step 1)

Parent/Carer Authorisation Details

Authorisation Type

Eg Enrolment Form (selected from Medication Record created in Step 1)

Contact Date/Time:

(date/time: created from Medication Record created in Step 1)

Parent/Carer Contacted:

(Parent Contacted: created from Medication Record created in Step 1)

Comments:

(Comment are created from Medication Record created in Step 1)

Date and time, or the circumstances under which, the medication should be next administered

Date/Time

Time given medication to child

Dosage To Be Administered

Dosage per medical management plan.

(Information comes from when created “Medication Record” created in step 1.

Method of Administration

Method per Medical Action Plan

Upload Images

Add Medication Administered

Date/Time

Dosage Administered

Method of Administration

Person administering medication (and completing this record)

Signature

Witness

Name of Witness

Signature

☒ Is the medication in its original container and bearing the original label with the name of child whom the medication is to be administered and where the medication must be stored.

☐ I checked the "five rights of medication administration" – right child, right medication, right dose, right time and right manner – with a witness before administering the medication.

Comments/Reason

Medication initially created by Staff - Scott Zannes.Following anaphylaxis management plan.

(Information comes from when created "Medication Record" created in step 1. This is per medical management plan.

INFORMATION INPUT SECTION:
 Educator who is administering medication need to note information on what is being given to child.

Person who administering medication signature. Eg. Centre Director, Responsible Person, Room Leader, Qualified Educator.
 In Method of Administration (see above) list persons name who gave medication. Person who administers medication should be person logged into OWNA App.

Witness and person who completed second check of medical management plan, dosage, label on medication, etc.

Is name of Person who initially created Medication Record. Pre-filled from step 1.

Notes added from Medication Record "Comments / Reason from form created in step 1.

SUBMIT RECORD.

STEP 3: COMPLETION OF AN INCIDENT / ILLNESS REPORT

As a Medical Management, Risk & Communication Plan has been enacted this typically would be the result of a child illness occurring at the service (ie Allergic Reaction, Anaphylaxis Reaction, Asthma attack, etc). Therefore, an Incident / Illness Report (*'Incident, Injury, Trauma and Illness Record'*) need to be completed to document this information and inform parents/guardian (this is in addition to the completion of the Medication Record).

Assess if considered a 'Serious Incident / Illness' and needed to be reported to Regulatory Authority via the NQS ITS within 24 hours. Typically, this would be required if a medical management plan has been enacted.

Questions on form:

General Activity at the time incident/injury/trauma/illness

Detail the activity, experience or routine

Cause of injury/trauma:

Consider: Eg. Child has potentially come in contact with allergen: List possible items.

Circumstances leading to and surrounding any injury or illness (including symptoms)

Detail

Actions taken section of form

- **Details of action taken (including first aid, administration of medication, etc)**

Describe actions completed and reference to Medical Management, Risk and Communication Plan

- **Did emergency services attend? Yes or No**
- **Was medical attention sought from registered practitioner / hospital ?**
- **Have any steps been taken to prevent this type of incident in the future?**

Complete assessment of circumstances and if a risk assessment is needed to be completed and/or Medical Management, Risk and Communication Plan is needed to be reviewed or edited.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/6/2026

Next Policy Review: 15/6/2027

Reference:

Policy Handbook – P004. 7.2026

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

Reg	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement—anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

Handbook on Child Care Licensing
QIAS Principle 5.1, 5.2, 5.3
Standards Australia
Privacy Act 1988
Public Health and Wellbeing Act 2008
Occupational Health and Safety Act 2004
CCB – Federal Funding Guidelines
Drug Regulations

88 – CHILD SAFE ENVIRONMENTS POLICY

Position Statement

To have a safe environment that facilitates learning, development and care of children.

Objective

To provide an environment that is safe, appealing, constructive, well-maintained and welcoming to all individuals who use it.

Related Policies and Procedures

29 – Food Safety & Hygiene Policy
81- Hazard Risk Management Policy Procedure and Program
26 – High Chair Policy
37 – Biting Policy
12 – Child Safety, Protection & Wellbeing Policy
34 – First Aid Policy
29 – Food Safety & Hygiene Policy
27 - Health & Hygiene Policy
19- Incident, Injury and Trauma Policy
10 – Occupational Health & Safety Policy
33 – Medication Policy
14 – Safety & Maintenance Policy
11 – Supervision Policy
23 – Toxic & Potential Dangerous Products Policy
24 – Toxic Plant Policy
13 – Water Safety Policy
42 - Sun Protection and Safety Policy
65 - Delivery and Collection of Children
67- Back Care and Manual Handling

Implementation

PHYSICAL ENVIRONMENT

In relation to our physical environment, the service will:

Provide an environment that fosters children's learning, development and safety and the safety of others in the environment by:

- Providing appropriately sized furniture and equipment in both the indoor and outdoor settings for the age ranges represented in the service.
- Providing challenges in relation to indoor and outdoor elements in the environment to encourage appropriate challenges and risk taking as per each child's developmental level.
- Providing an environment that is well organised so children, educators and others in the environment can generally move around without having to disrupt other children's activities.
- Incorporating any specific requirements of children with additional needs as seamlessly as possible.
- Incorporating soft fall material into our environment and placing any climbing play equipment, swings or slides on soft fall materials.
- Undertaking regular risk assessments of the service's environment to ensure that risk is minimised at all times.

- Providing an environment that allows children in various moods to exist in the same space (e.g. quiet play areas and loud play areas)
- Providing experiences for children to interact with nature by including natural elements (e.g. plants, trees, gardens, rock, mud or water) in our outdoor environment
- Providing adequate and ongoing training for new and existing educators and volunteers in relation to the children's learning and relevant safety considerations.
- As part of our sun protection strategy, providing adequate shading as per the recommendations of recognised authorities.
- Establishing the environment to allow children to be appropriately supervised at all times.
- Fencing the area in order to provide safety.
- Engaging the children in a wide variety of indoor and outdoor experiences.
- Enabling the children to access appropriate furniture, resources, materials, toys and equipment. These resources will be adequate in number for the amount of children attending our service and be developmentally appropriate.
- Providing an environment where children can explore, solve problems, create, construct and engage in critical thinking that is developmentally appropriate for the ages represented in the service.
- Providing an environment that allows children to engage in activities independently or work in small groups and access resources independently.
- Providing an environment that encourages large and small group and independent activities.
- Providing an environment that incorporates commercial, natural, recycled, home-made and real resources that can be used in a variety of ways to encourage children's learning.
- Providing adequate and accessible hand-washing, toileting, eating and sleeping facilities as per the amount of children attending our service. In relation to toileting and hand-washing, these will be accessible from both our indoor and outdoor environments.
- If children attending the service use nappies, the service will provide adequate and appropriate hygienic facilities for nappy changing. If children at the service are under 3 years of age, there will be at least 1 properly constructed nappy changing bench. The nappy change facility are designed, located and maintained to ensure that children cannot access them without being directly supervised.
- Establishing an environment that incorporates natural and artificial lighting, appropriate ventilation, heating and cooling and fresh air.
- Provide appropriate areas for food preparation.
- Provide quiet areas where children can sleep or rest.
- Providing a separate indoor space for children who are under two years of age (yards permitting).
- Providing a space for administrative functions, consultation with children's parents and for private conversations to take place.
- Ensuring children's safety by ensuring that power points not in use have safety caps, all double adaptors and power-boards are out of reach of children and all electrical cords are secured.
- Discussing with children the safety aspects of using toys and equipment. When circumstances allow, we will involve the children in setting these rules.
- Communicating with families about the latest available safety information and making them information available to families.
- Compiling details of maintenance logs and records of building and equipment.
- Ensuring all equipment, including resources, equipment, car seats, booster seats etc meets relevant Australian Standards and educators are trained in correct maintenance and assembly.

Arrangements for Laundering of Soiled Items

- The laundry area includes a clothes dryer, washing machine, clothes dryer and trough with hot & cold water supply for the laundering of soiled cloths and linen.

Choosing Appropriate Resources and Equipment

- The Approved Provider will be ultimately responsible for any purchases of equipment.
- Educators will compile a list for the Approved Provider of equipment which needs maintenance on a prioritised basis on a periodic basis.
- Resources and equipment will be chosen to reflect the cultural diversity of the service's community and the cultural diversity of contemporary Australia, including the incorporation of the Aboriginal and Torres Strait Islander community.
- The service may actively seek the input of parents/guardians regarding toys and equipment at the service.
- All new equipment will be checked against Australian Safety Standards.
- Children will be carefully introduced to new toys & pieces of equipment and taught how to use and care for them appropriately.
- Equipment that should only be used under supervision will be stored in a safe place pursuant to the relevant policies.
- The use of pools and toys or equipment which involves the use of water will be used under the direct supervision of educators. All equipment will be emptied of water when not in use, and stored in such a manner that it cannot collect water.
- Children will only use a trampoline whilst under the direct supervision of an adult educator.
- Equipment will be checked regularly by the educators to ensure it is in a clean and safe condition.
- The Approved Provider will advise the educator and parents about the purchase of new equipment and ensure a risk assessment is carried out.
- All equipment purchased for the service will be within budget limitations.

Rearranging, Adding or Removing Furniture

- Plans may be kept regarding the arrangement of the rooms and outdoor spaces. These plans may show how the service aims to create inviting learning spaces.
- The service may keep a record of any changes that is made to the physical environment of the service, such as rearranging of rooms etc.
- The service will document the links between the arrangements and choice of resources and equipment and the children's learning.

On-going Maintenance

- The service will consistently reflect on its environment and put in place a plan to ensure that the environment continuously reflects our ideology of providing an environment that is safe, stimulating and engaging for all who interact with it.
- The Approved Provider/Nominated Supervisor will put together an ongoing prioritised maintenance plan for the service. This will then be implemented throughout the year in accordance with any relevant policies.

- It is the responsibility of the Approved Provider/Nominated Supervisor to complete a Building Safety Checklist of the service and its grounds bi-annually and ensure any works deemed necessary are done to Australian standards. The Approved Provider/Nominated Supervisor will also ensure that the service and its grounds comply with Local Government and BCA regulations in regards to fire ventilation, natural and artificial lighting and safety glass.
- Should the service undertake major renovations, management plans will be put in place to ensure that the safety of educators, children, families and others at the service is not compromised.

The Children's Groupings, Education and Care

Our service groups the children throughout the day in rooms according to their age and/or developmental stage. Within that space, there are different areas where the children can explore and experience their own moods. For example, each room allows for a quiet play space, such as our book corner and a loud/physical play space such as the block area. Our outdoor environment also allows for children in different moods to be in the environment. Our outdoor table and chairs allows for children to engage in quiet activities while our climbing equipment and sandpit allows for children to engage in physical activities.

In order for children to interact with the children and educators from other rooms in the service, family grouping occurs at the start and end of each day.

The service believes children safety is paramount importance. Safety education is incorporated into the curriculum through intentional and spontaneous teaching. We believe that educators, staff and families should role model safe behaviours. This extends to when children participate in walking excursions, safe active travel is promoted. To further support our safe initiatives, the service engages with a number of community agencies and health professionals to support safety initiatives. Educators, staff and families.

Safety Checks

Please refer to 11 - Supervision Policy for active supervision strategies and headcount procedures.

A daily inspection of the premises will be undertaken before children begin to arrive. This inspection will include the:

- Perimeters
- Fences
- Gates
- Paths
- Buildings
- All rooms accessible by children
- Fixed equipment.

This must be done in order to premeditate any dangerous objects in the grounds ranging from sharps to poisonous or dangers flora and fauna.

In the event of a sharp object being found (for example a syringe) educators will wear gloves and use tongs to pick up the object and place it in the 'sharp object box'. This box will be disposed of as per the recommendations of our local council.

Similarly, trees in the grounds must be checked regularly for overhanging, dead or dangerous looking branches as well as check for any infestations or nests.

In regards to non-fixed play equipment in the service grounds it can be no more than one metre high and will not obstruct the sightlines of supervisory educators.

The service will also regularly have pest inspections carried out by an accreditation pest control company. Documentation of these inspections will be kept and any findings from the pest control check will be carried out in line with the recommendation of the pest control company.

The Indoor and Outdoor Daily Safety Checklists will be used as the procedure to conduct these safety checks. A record of these will be kept by the service. Any required maintenance will immediately be reported by the Approved Provider/Nominated Supervisor who will make the appropriate arrangements to have repairs carried out.

The following is a guideline for service Checklists.

Outdoor items for Review

- **Building maintenance** – regularly maintain and check for hazards, check building is in a safe, clean and hygienic condition. Records of any damages and subsequent repairs are kept.
- **Doors** – have finger jam protectors (as required)
- **Dust mites, pet allergens** – regular dusting and vacuuming.
- **Fence** off securely and effectively all sides of outdoor play areas from roads, water hazards, and driveways. Maintain fences have correct height. Install childproof self locking devices on gates.
- **Garbage** – safe and prompt disposal. Use lidded secure bins that prevent child access and maintain in a clean and safe condition. Encourage recycling.
- **Garden** and renovation debris removed. Regularly trim branches and bushes.
- **Garages and sheds** - keep locked.
- **Heating, cooling, ventilation, lighting** – comfortable, safe, maintained, guarded and are kept out of reach of children.
- **Hygienic**, regularly cleaned and maintained conditions protect against vermin, bacteria, mildew, lead, asbestos and other dust allergens.
- **Non-slip** floors, stairs, steps, grounds and nonporous indoor floors for easy cleaning.
- **Renovation** dangers e.g. lead, asbestos, holes and excavations – reduce risks.
- **Pesticide** residue - dangerous chemicals should not be used to remove vermin.
- **Safety glass** is installed according to the Australian Standards on all glass doors and windows accessible to children, and safety decals on both sliding doors and plate glass doors at child and adult eye level.
- **Security** - minimising unauthorised access with appropriate fencing and locks.
- **Spills** – clean away as they occur.
- **Under Service access** (including buildings on stilts and footings) – lock or block access.
- **Window fly screens** securely fitted, maintained and permanent.

Indoor Items for Review

- **Access for children and adults with disability** - ensure safe access into, within and out of the Service, security, toilet and washing facilities, and check for hazards for wheelchairs and individuals with impaired sight, hearing or mobility.
- **Barriers** - age appropriate, child proof, self locking barriers to balconies, stairways, kitchen, bathroom, laundry, garage, other levels in the Service, front and back garden.

- **Children at risk** – maintain extra security and supervision for children at special risk.
- **Choking hazards** e.g. small toy parts, beads, nuts, blind and curtain cords, plastic bags, sandwich bags and balloons.
- **Decorations and children's artwork** – do not place near ceiling fans, air conditioners or heaters. Avoid use of tacks, pins, and staples.
- **Emergency evacuation** – develop an evacuation plan and emergency contact numbers display, inform families, and practice evacuation procedures.
- **Fire** – fire blanket, extinguisher, fire exits, smoke detectors, electrical safety switch.
- **First aid kit** with approved contents is maintained and accessible. Ensure First Aid certificates are current for relevant educators.
- **Furniture and nursery equipment** - stable, maintained and meets safety standards.
- **Guard and make inaccessible to Children:** heaters, coolers, fireplaces, stoves, microwaves, power points, and office equipment. Ensure heaters are away from children's cots.
- **Hazardous indoor and outdoor plants** identify, remove or make inaccessible to children.
- **Heaters** – ensure that children cannot come in contact with hot surfaces. It is preferable to use heating where combustion products are ducted outside. If gas heating is used ensure there is adequate ventilation while the heater is operating.
- **Hot water** - ensure the hot water supply is regulated so as to keep it below the temperature at which a child can be scalded (the current KidSafe recommendation is below 43.5°C).
- **Machinery, tools and equipment** – ensure all engine operated or other hazardous equipment, tool or machinery are stored securely and are inaccessible to children.
- **Noise** – reduce excessive exposure.
- **Non-slip, non-porous** floors, stairs.
- **Pets and animals** – inform families of pets being kept on premises and plans to obtain new pets. Ensure pets are vaccinated, wormed, don't have fleas, clean, and healthy. Keep pet accessories such as pet food, litter boxes, pet toys away from children. Exclude dogs from children's play areas. Keep children-pet interactions minimal and supervise interaction times.
- **Record details** and notify parents of any child accident.
- **Safe play rules and adequate play spaces:** discourage running indoors and safe furniture layout to avoid collisions.
- **Safety glass used and installed** according to Australian Standards, and Australian Building Codes on all glass doors and windows accessible to children, **safety decals** on sliding doors and plate glass doors at child and adult eye level.
- **Security** – ensure all entry doors are locked at all times and place bells on doors.
- **Smoke free environment** in all areas.
- **Educators personal items** – ensure educators' personal items such as bags, sharp instruments, toiletries and medicines are kept secure and are inaccessible to children.
- **Stairways**, ramps, corridors, hallway, external balcony must be enclosed to prevent a child falling.

- **Store in locked cabinet** any unsafe items, e.g. chemicals, medicines, razors, knives and electrical equipment.
- **Supervision and visibility of children** – ensure children are visible and supervised at all times. High risk areas are children in high chairs, playpens and play areas, on change tables, and in nappy change and toilet areas. Have at least two educators on premises at all times with vision of each other and the children, have two educators present or in view when changing nappies or washing children.
- **Toys** – meet safety standards, age appropriate, maintained, and non-toxic.

CLEANING OF BUILDINGS, PREMISES, FURNITURE AND EQUIPMENT

General Cleaning

The service will use structured cleaning schedules to ensure that all cleaning is carried out regularly and thoroughly. Educators will clean the service at the end of each day and throughout the day as the need arises. Accidents and spills will be cleaned up as quickly as possible to ensure that the service always maintains a high level of cleanliness and hygiene.

When purchasing, storing and/or using any dangerous chemicals, substances, medicines or equipment, our service will:

- Select and make use of the least hazardous substance or equipment.
- Only purchase and make use of substances which have child resistant lids or caps. Educators will ensure that such lids or caps are properly fixed at all times.
- Adhere at all times to manufacturer's advice and instructions when using products to clean furniture and equipment at the service.
- Store all dangerous chemicals, substances and medicines in their original containers provided by the manufacturer. All labels and/or use by dates should be kept intact at all times. Any substance or found to be stored in a different container than originally provided, or with destroyed labels and/or unknown use by dates where appropriate will not be used under any circumstances. Containers should be disposed of correctly following local council guidelines, and not reused under any circumstances. Labelling of container is permitted to be completed with permanent marker to clearly label the name of the contents and the words "Poison – not for consumption see MSDS".
- All dangerous chemicals, substances and equipment must be stored in a locked place or facility which is labelled, secure and inaccessible to children. These materials may include, but are not limited to, all cleaning materials, detergents, poisonous or dangerous substances, dangerous tools and equipment including those with sharp and razor edges and toiletries.
- Educators should follow the instructions of manufacturers, particularly of any product which may need to be stored in a refrigerated environment pursuant to the aforementioned directives.
- Particularly dangerous and hazardous materials such as pesticides, herbicides, petroleum, kerosene, solvents and equipment which is operated by an engine or hazardous to children, will be stored in a locked facility external to the main building of the service. The facility must have a bonded floor and be inaccessible

to children and clearly labelled as storing dangerous substances and/or equipment. Such facilities should be separate from children's play or outdoor environments.

- All hazardous containers and equipment should be properly discarded pursuant to local council guidelines.
- Any substances that need to be refrigerated must be stored in a labelled, child resistant container, preferably in a separate compartment or in a part of the refrigerator inaccessible to children.
- If bulk chemicals or non-domestic products and quantities are used or stored, have a Hazardous Substances Register and Risk Assessment in accordance with the OHS Act and Regulation. This should record product name, application, whether the product is labelled, whether a MSDS (Material Safety Data Sheet) is available, what class risk the chemical is, the controls for prevention of exposure that are required and what first aid, medical or safety action should be taken if a individual is exposed.
- The manufacturer's instructions for use, storage, and first aid instructions will be followed and recorded on an MSDS (Material Safety Data Sheet).
- The Service should keep a register of all hazardous chemicals, substances and equipment used at the Service. Information recorded should include where they are stored, their use, any risks, and first aid instructions.
- Appropriate personal protective clothing should be worn pursuant to the manufacturer's instructions when using and disposing of hazardous substances or equipment.
- Seek medical advice immediately if poisoning or potentially hazardous ingestion, inhaled, skin or eye exposure has occurred, or call the Poisons Information Line on 131126, or call an Ambulance on 000.
- In the case of any child or educator becoming injured by a chemical, substance or equipment, the service will initiate our emergency, medical and first aid procedures, notify the appropriate authority that administers workplace health and safety immediately and any other individual or authority as required by regulations or guidelines.
- In any major emergency involving a hazardous chemical or equipment, a hazardous gas or a fire or explosion hazard, call the emergency services, dial 000 and notify the appropriate authority that administers workplace health and safety and any other individual or authority as required by regulations or guidelines.
- The Poison Safety Checklist will be used in order to ensure we are consistently meeting requirements.

Minimising Potentially Dangerous Substances

Our service also implements the concept of minimising the use of potentially dangerous substances. Ordinary detergents will be used to help removed dirt from surfaces. Colour-coded sponges (per service schedule) will be used in order to not cross-contaminate areas. Different rubber gloves will also be used in each room then hung out to dry and air. Before returning to the children educators will wash and dry hands.

Disinfectants

Disinfectants are usually unnecessary as very few germs can survive exposure to fresh air and natural light. In an outbreak situation, public health units may specify the use of a particular disinfectant. In this situation, for the disinfectant to work effectively, there still needs to be thorough cleaning using a detergent beforehand.

Essentially, there is no ideal disinfectant. Disinfectants cannot kill germs if the surface is not clean. It is more important to make ensure surfaces have been cleaned with detergent and warm water than to use a disinfectant. To kill germs, any disinfectant needs:

- A clean surface to be able to get to the germ.
- To be able to act against those particular germs.
- To be of the right concentrate on.
- Enough time to kill the germs, this is at least 10 minutes.

Even when all of these conditions are met a disinfectant will not kill all the germs present.

Detergents

Proper cleaning with detergent and warm water, followed by rinsing then drying and airing time kills most germs from surfaces as they are unable to multiply in a clean environment. Cleaning equipment should be stored and taken care of so it can dry between uses and not multiply germs itself.

Poison Safety Checklist

Week beginning: _____

Checklist	Yes	No	Action required
Have all chemical products been checked to determine if they need to be stored in a lockable cupboard?			
Have all chemical products been checked to determine if they require disposal (out of date or no longer required)?			
Are all chemicals labelled correctly?			
Are food and chemicals stored separately?			
Are all Material Data Forms available for all chemicals in the Service and placed with the chemical?			
MSDS Register available?			
MSDS Register Current?			
Are containers for soiled nappies securely covered and the contents inaccessible to children?			
Are low toxicity products used whenever possible?			
If any handbags contain medication, are they stored safely?			
Is the Poisons Information Centre number available at every phone? (13 11 26)			
Is there any paint flaking of the walls that may contain lead?			
Have all poisonous plants been removed from the Service's premises?			
Have families been provided with information on poison safety?			
Have chemical storage areas been labelled with chemicals stored here signs?			
Medication is stored in locked containers/cupboard?			

Educators Name: _____

Educators Signature: _____

Cleaning Schedule Guidelines – (Services will have custom schedules)

	Wash DAILY plus when visibly soiled	Wash WEEKLY plus When visibly soiled
Bathrooms. Wash tap handles, toilet seats, toilet handles and door knobs. Check the bathroom during the day and clean if Obviously soiled.	✓	
Toys and objects put in the mouth	✓	
Surfaces the children have frequent contact with, for example, bench tops, taps, cots and tables.	✓	
Vinyl mattress beds must be cleaned with disinfectant. If soiled should be cleaned with warm soapy water and disinfectant. Left to dry.	✓	
Mattress covers and linen.		✓
Door knobs	✓	
Floors.	✓	
Low shelves.		✓
Other surfaces often touched by children		✓

Special areas for Cleaning - nappy change area

Nappy change areas need to be cleaned after each use with disinfectant. If faecal matter spills onto the area wipe down with disinfect / detergent solution and warm soapy water and leave to dry. Please refer to procedure.

Clothing

It is recommended Educators clothing should be wash daily. Educators may wear aprons or something similar to cover clothing that cannot frequently be washed. Educators should also have a change of clothes available in case of accidents. Dress-up and play clothes should be washed once a week (pending amount of use) in hot water and detergent.

Toy Cleaning

Educators will regularly clean the children's equipment and toys. Toys, especially in the rooms of the younger children, are to be washed daily so as to avoid the spread of disease. Educators will wash a toy immediately if it has been sneezed on, mouthed, soiled or if it has been discarded after play by a child who has been unwell. The service will have washable toys for the younger children.

Refer to the following toy cleaning schedule for toys on display in the children's rooms.

Recommended cleaning materials:

Please use child friendly cleaners such as Bicarb, vinegar diluted 1:10 in water, tea tree, lavender or eucalyptus oil diluted 5 drops to 500ml water, lemon juice or plain water. Please rinse in water after cleaning.

Other items that may be necessary include a sponge cloth, an old toothbrush or cotton wool.

General:

- Most toys can be washed with normal dishwashing liquid, rinsing with clean water.
- Get into corners with a toothbrush. Please dry well.
- Leaving items such as LEGO and construction blocks to drain on a clean tea-towel overnight is ideal.

Wooden Toys:

- Should be wiped over with a damp cloth – please do not immerse in water.

Play Dough

Service will reduce the risk of the spread of disease when playing with play dough by:

- Encouraging hand washing before and after using play dough
- Making up individual play dough in zip lock bags for each child to participate in sensory experience.
- Sanitizing surfaces that children will be playing with play dough.
- Storing the play dough in a sealed container in the refrigerator between uses
- Making a new batch of play dough each week, and
- If there is an outbreak of vomiting and/or diarrhoea, discarding the play dough at the end of each day during the outbreak.

Rattles and Baby Toys:

- Must not be immersed in water as it can get inside, rendering the toy useless.
- Wipe thoroughly with hot water or a cloth with diluted vinegar or specialised cleaning solution.

Ride-on Vehicles and Outdoor Toys:

- Must be cleaned.
- Please take care not leave them exposed to the elements as this reduces their lifespan.

Puzzles and Games:

- Wooden puzzles as per 'Wooden Toys' above.
- Cardboard should be wiped over with a slightly damp cloth.

Packaging:

Please help us by:

- Packing small pieces in bags provided. This makes them easier to count when returned
- Using elastic bands to contain similar items (like railway tracks).
- Completing puzzles before returning where practical.
- Alerting Volunteers to plastic bags which may be deteriorating.

Sun Protection

It's important to have a healthy balance of ultraviolet radiation (UV) exposure. Too much of the sun's UV can cause sunburn, skin and eye damage and skin cancer. Overexposure to UV during childhood and adolescence is a major factor in determining future skin cancer risk. Too little UV from the sun can lead to low vitamin D levels. Vitamin D is necessary for the development and maintenance of healthy bones and muscles, and for general health.

This SunSmart Policy has been developed to:

- Ensure all children and educators get some UV for vitamin D.
- Encourage children and educators to use a combination of sun protection measures whenever UV Index levels reach 3 and above to minimise skin and eye damage and skin cancer risk.

- Work towards a safe outdoor environment that provides shade for children and educators at appropriate times.
- Assist children to be responsible for their own sun protection.
- Ensure that families and new educators are informed of the service's sun protection policy.

Refer to the Sun Protection and Safety Policy.

Kitchens

- Children must not gain access to any harmful substance, equipment or amenity
- The kitchen has a door, half-gate or other barrier to prevent unsupervised entry by children into the kitchen.
- The preparation of bottles for children under the age of 2 years is both safe and hygienic at all times and separate from nappy change area.

Maintenance of Fire Equipment

All fire equipment at our centre will be maintained as per the legal standards. Our equipment will be checked as required as per the timeframes below.

Key	
Inspection Procedure of Fire Safety Installations (F.S.I)	1. Inspect for obvious visual faults. 2. Inspect for faults and witness test of F.S.I by a competent person 3. Inspect for faults where possible and accept logbook details of F.S.I 4. Check Building file for details of any extra requirements.
Required Record of Keeping Fire Safety Installations (F.S.I)	L = log book required R = record of maintenance required T = Metal tag on F.S.I or service details/service label (Y) = Weekly test may be omitted refer AS 1851-2005

Fire Fighting Equipment Maintenance Schedule

Special Fire Service	Inspection Procedure for FSI	Required Record Keeping for FSI	Maintenance Schedule							Annual Survey of Installation	Maintenance Standard or Building Preference
			Wk	Mth	3Mth	6Mth	12Mth	3Yr	5yr		
Fire Mains	1					Y	Y	Y			1851-Section 2 & 4

Fire Hydrants (including internal & external hydrants, boosters connection/s and water storage tanks)	2	L-T				Y	Y		Y	Y	1851-Section 4
Fire Pumpsets	2	L-T	(Y)	Y		Y	Y		Y	Y	1852 – Section 3
Fire Hose Reels	2	R-T				Y	Y			Y	1851-Section 14
Fire Extinguishers (Portable)	2	R-T				Y	Y		Y	Y	1851-Section 15

External agencies will be employed to assist the centre with this maintenance if no currently employed staff or educators are qualified to complete the maintenance checks.

Links to other Policies and Company Resources

In conjunction with this policy please read the Staff Handbook (POL002), the Workplace Health, Wellbeing and Safety Handbook (POL003).

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/5/2026

Next Policy Review Due: 15/5/2027

References:

NQS

QA2	2.2.1 (Pre-1 Feb 18: 2.3.2)	At all times, reasonable precautions and adequate supervision ensure children are protected from harm and hazard.
QA3	3.1.1	Outdoor and indoor spaces, buildings, fixtures and fittings are suitable for their purpose, including supporting the access of every child.
	3.1.2	Premises, furniture and equipment are safe, clean and well maintained

	3.2.1	<i>Outdoor and indoor spaces are organised and adapted to support every child's participation and to engage every child in quality experiences in both built and natural environments.</i>
	3.2.2	<i>Resources, materials and equipment allow for multiple uses, are sufficient in number, and enable every child to engage in play-based learning.</i>
QA6	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.1	<i>Families are supported from enrolment to be involved in the service and contribute to service decisions.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.1.2	<i>The expertise, culture, values and beliefs of families are respected and families share in decision-making about their child's learning and wellbeing.</i>
	6.1.3	<i>Current information is available to families about the service and relevant community services and resources to support parenting and family wellbeing.</i>
	6.2.2	<i>The service builds relationships and engages with its local community.</i>
	6.2.1	<i>Continuity of learning and transitions for each child are supported by sharing information and clarifying responsibilities.</i>
	6.2.2	<i>Effective partnerships support children's access, inclusion and participation in the program.</i>
	6.2.2	<i>The service builds relationships and engages with its local community.</i>

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

<i>Regs</i>	<i>168(2)(h)</i>	<i>Policies and procedure; providing a child safe environment</i>
	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>
	107	<i>Space requirements—indoor</i>
	108	<i>Space requirements—outdoor space</i>
	109	<i>Toilet and hygiene facilities</i>
	110	<i>Ventilation and natural light</i>
	111	<i>Administrative space</i>
	112	<i>Nappy change facilities</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>
	115	<i>Premises designed to facilitate supervision</i>
	116	<i>Assessments of family day care residences and approved family day care venues</i>
	117	<i>Glass (additional requirement for family day care)</i>
	73	<i>Educational programs</i>
	74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
	75	<i>Information about the educational program to be kept available</i>
	76	<i>Information about educational program to be given to parents</i>
	80	<i>Weekly menu</i>
	86	<i>Notification to parents of incident, injury, trauma and illness</i>

	99	<i>Children leaving the education and care service premises</i>
	102	<i>Authorisation for excursions</i>
	111	<i>Administrative space (centre-based services)</i>
	168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
	171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
	<i>Children respond to diversity with respect</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Other References:

Occupational Health & Safety Act 2004

Australian Standards 1851-2005 “Maintenance of Fire Protection Systems and Equipment”

89 – EDUCATION, CURRICULUM AND LEARNING POLICY

Position Statement

Our service needs to promote the learning and development of all children in a safe, nurturing environment that meets the individual needs of the children and achieves the learning outcomes in accordance with the National Quality Framework, Early Years Learning Framework and Victorian Early Year Learning and Development Framework.

Objective

Educators aim to create positive learning environments and guide experiences for each child in conjunction with their family. Educators will observe children and facilitate their learning to provide each child with an individualised portfolio by documenting their learning throughout the year. Children and their families will be encouraged to participate in the ongoing process to promote engaged learning.

Implementation

Our Educational Leaders are appointed by the Approved Provider and Nominated Supervisor of the service. Our service has a Head Educational Leader and a committee of Educational Leaders. Please refer to centre notice board for the Educators appointed to these roles.

The role of the educational leader is to work with educators to provide curriculum direction and to ensure children achieve the outcomes of the approved learning framework.

Our service is committed to the Early Years Learning Framework (EYLF, 2022 v2.0) and the Victorian Early Years Learning and Development Framework (VEYLDF).

Our service uses OWNA an online curriculum software for the planning and documentation of children learning and development. Observations of all children enrolled in our service will be documented and kept for future reference and reflection, through use of OWNA Curriculum Software and online portfolios. Portfolios will be added to regularly by educators, families and children and reflected upon by educators to ensure programming for each child remains relevant to their interests and developmental stage.

Please refer to the service's Curriculum Guide for guidance on how to deliver the curriculum, educational program and the tools used to document learning and development in accordance with the NQS, EYLF and VEYLDF.

Other Approved Learning Frameworks

- **Victoria:** The Victorian Early Years Learning and Development Framework
- **Australian Capital Territory:** Every Chance to Learn—Curriculum framework for ACT schools preschool to Year 10
- **Tasmania:** The Tasmanian Curriculum, the Department of Education of Tasmania, 2008
- **Western Australia:** The Curriculum Framework for Kindergarten to Year 12 Education in Western Australia.

Early Years Learning Framework

- Each child's learning will be based on their interests and strengths and guided by our educators.

- Educators must work in collaboration with families to provide relevant learning experiences for each child, based on their interests and family experiences.
- Every child will be equally valued and their achievements and learning celebrated.
- Educators will observe and record the strengths and learning of each child.
- Educators will work closely with children and families to generate ideas for the curriculum.
- Learning Outcomes will be linked to the curriculum during and after each child's learning has occurred.
- The curriculum will be based on the children's interests, educators extending children's interests, spontaneous experiences and family input.
- Where appropriate, the service will liaise with external agencies and support persons to best educate and care for children with additional needs.
- Where appropriate, the curriculum (play and learning experiences) will build and develop each child's Learning Stories, Portfolio, Observations of each child's strengths and achievements, Formative Assessments and Summative Assessments. Child Parent Educator Planning Reflection and Evaluation Schedules will be prepared by parents and educators.
- The curriculum will be evaluated and reflected upon each week by educators.

Learning and Play

- Children are encouraged to express themselves creatively through a wide variety of indoor and outdoor activities.
- Engage in project work. This allows children to be involved in making decisions about the topic focus, research questions and provides opportunities for children to take agency of their own learning.
- Children's fine and gross motor skills are strengthened and developed through a wide variety of both indoor and outdoor activities including manipulative play, block play, sensory play, dramatic play, drawing and other physical activities such as running and skipping.
- Mathematics and science concepts along with exploration of natural aspects of our environment are encouraged through block play, building, cooking, water play, sensory play, collecting natural materials such as leaves and rocks and gardening.
- Language development is encouraged through educators modelling language, show and tell, story time, games, poems and dramatic play experiences.
- Social/emotional and independence skills are strengthened through activities such as role-play, dramatic play, group games and self-help tasks. Further protective behaviours and behaviour guidance skills will also be incorporated into the curriculum.
- Music and movement activities encourage physical, social and creative areas of a child's development.

- Road safety, hygiene, dental care and nutrition will all be built into the weekly curriculum.

These activities will be supervised and guided by educators to find out how child responds as an individual and also as part of a group. Educators will work in conjunction with families to provide learning experiences that are relevant to each child and tailored to their specific needs. A child's home language, culture and religious practices will be accepted and included in the program.

From this, educators will assess the child's needs and plan ways to meet these needs. We evaluate this program every week in order to make sure we stay on target and help each child to reach their full potential. The weekly educational curriculum will be displayed in the room it takes place in. We welcome any suggestions and are happy to answer questions from family members at any time.

EYLF Learning Outcomes

1. Children have a strong sense of identity.
2. Children are connected with and contribute to their world.
3. Children have a strong sense of wellbeing.
4. Children are confident and involved learners.
5. Children are effective communicators.

VEYLDF Learning Outcomes

The VEYLDF identifies five key Learning and Development Outcomes for children:

1. Children have a strong sense of identity.
2. Children are connected with, and contribute to, their world.
3. Children have a strong sense of wellbeing.
4. Children are confident and involved learners.
5. Children are effective communicators.

Evaluation and Review

- This policy will be reviewed at least annually.
- Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.
- Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

References:

NQS

QA's	1-7	
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	73	<i>Educational programs</i>
	74	<i>Information about the educational program to be kept available</i>
	75	<i>Information about educational program to be given to parents</i>
	76	<i>Documenting of child assessments or evaluations for delivery of educational program</i>

EYLF

LO1 – LO5	All Learning Outcomes under the Early Years Learning Framework will be addressed through our Policy and practices.
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Other References:

National Quality Standard

Early Years Learning Framework

90 – ENVIRONMENTAL SUSTAINABILITY POLICY

Position Statement

To have a learning environment that facilitates sustainability.

Objective

Our service aims to help children learn about and implement sustainable practices and foster respect and care for the living and non-living environment.

Implementation

Children develop positive attitudes and values about sustainable practices by engaging in learning experiences, joining in discussions that explore solutions to environmental issues, and watching adults model sustainable practices. Children learn to live interdependently with the environment.

The service has a Sustainability Officer to assist with the coordination of sustainability practices.

Environmental Sustainability and our Curriculum

- Our educators will promote a holistic, open-ended curriculum which explores ideas and practices for environmental sustainability and helps children understand the interdependence between people and the environment by:
 - connecting children to nature through art and play and allowing children to experience the natural environment through natural materials like wood, stone sand and recycled materials, plants including native vegetation, nesting boxes, a potting bench with gardening tools and watering cans.
 - developing education programs for water conservation, energy efficiency and waste reduction.
 - healthy ecosystems and environments being necessary to the survival of humans, animals and other organisms.
 - celebrating childrens' environmental knowledge and sustainable activities.
 - involving children in nature walks, education about plants and gardening and growing plants and flowers from seed.
 - engaging children in learning about the food cycle by growing, harvesting, and cooking food for our service kitchen.
 - using resource kits and information on environmental issues from the Better Business Partnership or resources targeted at early childhood services such as "The Little Green Steps" Resource kits on Water, Waste and Wildlife.
 - enlisting the help of groups with expertise in environmental issues, for example bush care groups, wildlife rescue groups, Clean Up Australia, to deliver elements of our sustainability program
 - acknowledging and celebrating environmental awareness events like Clean Up Australia Day and Walk to School Day.

The Role of Educators

- Our educators will model sustainable practices by embedding sustainability into all aspects of the daily running of our service operations including:
 - recycling materials for curriculum and learning activities
 - minimising waste and effectively using service resources
 - turning off equipment and lights when not in use
 - using the least hazardous cleaning substance appropriate for the situation, for example, ordinary detergent for cleaning dirt from tables and other surfaces
 - Potential examples of sustainability activities include composting, maintaining a worm farm and maintaining a no dig vegetable/herb garden (sustainable practices will be ensure health and safety standards of children and staff are maintained and are practical for the service environment)
 - incorporating water wise strategies such as drip irrigation and ensuring taps are turned off and leaks fixed
 - using food that we have grown in meals on our weekly menu
 - Our service will provide training to our Sustainability Officers when possible and feasible.

Partnerships with Families and the Community

- Our educators will facilitate collaborative partnerships with local community groups, government agencies and private companies to enhance and support childrens' learning about sustainable practices. We will share their brochures and fact sheets on sustainable practices like recycling, saving water and power and green cleaning with our children and their families. Families will be encouraged to participate in decision making and information sharing about environmental sustainability through our newsletters, parent input forms, wall displays, meetings.

As at May 2012, the NSW Early Childhood Environmental Education Network is developing a tool to assist Early Childhood Services to identify and work towards an Environmentally Sustainable Service with the NQS. The Network's website has links to many organisations and Government agencies that provide information on sustainable practices at <http://www.eceen.org.au/links.htm>

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 15/6/2026

Next Policy Review: 15/6/2027

References:**NQS**

QA3	3.3.	<i>The service takes an active role in caring for its environment and contributes to a sustainable future.</i>
	3.2.3	<i>The service cares for the environment and supports children to become environmentally responsible.</i>

EYLF

LO2	<i>Children become socially responsible and show respect for the environment</i>
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National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

Regs	168(2)(h)	<i>Policies and procedure; providing a child safe environment</i>
	182	<i>Tobacco, drug and alcohol free environment</i>
	156	<i>Relationships in groups</i>
	103	<i>Premises, furniture and equipment to be safe, clean and in good repair</i>
	104	<i>Fencing and security</i>
	105	<i>Furniture, materials and equipment</i>
	106	<i>Laundry and hygiene facilities</i>
	107	<i>Space requirements—indoor</i>
	108	<i>Space requirements—outdoor space</i>
	109	<i>Toilet and hygiene facilities</i>
	110	<i>Ventilation and natural light</i>
	111	<i>Administrative space</i>
	112	<i>Nappy change facilities</i>
	113	<i>Outdoor space—natural environment</i>
	114	<i>Outdoor space—shade</i>
	115	<i>Premises designed to facilitate supervision</i>
	116	<i>Assessments of family day care residences and approved family day care venues</i>
	117	<i>Glass (additional requirement for family day care)</i>
	73	<i>Educational programs</i>
	74	<i>Documenting of child assessments or evaluations for delivery of educational program</i>
	75	<i>Information about the educational program to be kept available</i>
	76	<i>Information about educational program to be given to parents</i>
	80	<i>Weekly menu</i>
	86	<i>Notification to parents of incident, injury, trauma and illness</i>
	99	<i>Children leaving the education and care service premises</i>
	102	<i>Authorisation for excursions</i>
	111	<i>Administrative space (centre-based services)</i>
	168(2)(k)	<i>Policies and procedures are required in relation to enrolment and orientation</i>
	171	<i>Policies and procedures to be kept available</i>

EYLF

LO2	<i>Children develop a sense of belonging to groups and communities and an understanding of the reciprocal rights and responsibilities necessary for active community participation</i>
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	<i>Children respond to diversity with respect</i>
	<i>Children become socially responsible and show respect for the environment</i>
LO4	<i>Children develop dispositions for learning such as curiosity, cooperation, confidence, creativity, commitment, enthusiasm, persistence, imagination and reflexivity</i>
	<i>Children develop a range of skills and processes such as problem solving, enquiry, experimentation, hypothesising, researching and investigating</i>
	<i>Children transfer and adapt what they have learned from one context to another</i>
	<i>Children resource their own learning through connecting with people, place, technologies and natural and processed materials</i>

Other References:

National Quality Standard

Early Years Learning Framework

Climbing the little green steps 2007: Gosford and Wyong Councils

Environmental Education in Early Childhood (Victoria) Inc

NSW Early Childhood Environmental Education Network

Green Kinders: A guide to Reducing the Environmental Footprint of Child Care Centres

91 – EPILEPSY POLICY

Position Statement

We are committed to providing, as far as practicable, a safe and healthy environment in which individuals with epilepsy can participate equally in all aspects of the service curriculum and experiences.

Objective

Our aim is to provide a safe environment and to adopt inclusive practices to cater for the additional requirements of children with epilepsy in a respectful and confidential manner.

Implementation

With parent assistance our service will ensure all educators are aware of the enrolment of a child with epilepsy and have an understanding of the condition and the additional requirements of the individual child.

Epilepsy and Learning

Epilepsy refers to recurrent seizures where there is a disruption of normal electrical activity in the brain that can cause disturbance of consciousness and/or body movements.

The effects of epilepsy can vary. Some children will suffer no adverse effects while epilepsy may impact others by affecting, for example, their comprehension, expressive language, visual perception, concentration and memory. Some children with epilepsy may have absence seizures where they are briefly unconscious. Our educators will ensure they go over any learning or activity a child may have missed during a seizure.

The level of expectation for each child has a significant influence on performance. Our educators will facilitate a positive environment of encouragement, stimulation and reassurance.

Behaviour Support

Our educators will ensure that any routine management of a child's epilepsy, including the administration of any medication, occurs with minimal disruption to their education and care (please see our other policies in relation administration of medication).

As for all children, behaviour expectations for children with epilepsy should be consistent and predictable, and also sufficiently flexible to accommodate periods of stress and any emotional difficulties a child with epilepsy may be experiencing.

Our educators will nurture the self-esteem of all children, including those with epilepsy, and create a positive environment of inclusiveness and acceptance for all children.

Information Sharing: Confidentiality and privacy

Our service will adhere to privacy and confidentiality principles when dealing with each child's health and safety needs.

The sharing of information, including the amount and type of information, will be assessed and negotiated for each child with epilepsy. Educators need information about routine and predictable emergency care because it affects the child's learning, access to the curriculum and their safety. Information exchange between the family, health professionals and the service is also essential to support the child emotional health and enhance their peer support.

Young children, for example often enjoy sharing the news and their experiences of living with epilepsy with their classmates. This should be discussed with parents so that they can support their child in this process.

Medical Management Plan

Children with epilepsy will have a Medical Management Plan provided by their doctor and /or parents. This Plan should include information about:

- the type of seizures the child has
- their severity and timing
- whether there are any warning signs before a seizure
- any first aid requirements in addition to standard first aid
- action and procedures required if seizure occurs
- known triggers
- emotional needs of the child
- the level of participation, supervision and protection required for the child during activities, whether the child's safety may be compromised during an activity.
- how to minimise factors that might trigger seizures both at the service premises and on excursions
- what post-seizure support the child may need
- any sensory needs to consider in comforting the child
- how the service will communicate with staff and the family if there is an event
- how to administer medication if the service has a trained staff member, or when to call an ambulance.

Medical Conditions Risk Minimisation Plan

Our service will prepare a Medical Conditions Risk Minimisation Plan outlining procedures we will implement to minimise the incidence and effect of a child's epilepsy. The Plan will cover the child's known triggers and where relevant other common triggers which may cause an epileptic seizure. These include:

- missing medication for non-epileptic conditions
- suddenly stopping anti-convulsant medication or missing a dose
- infection or illness, especially if associated with a temperature
- lack of sleep
- extreme emotions, such as excitement about an excursion, stress or boredom
- hyperventilation/over-breathing
- head injury
- flickering lights (computers are not usually a problem)—only with certain kinds of epilepsy
- missing meals
- dehydration
- significant changes in temperature or extreme temperatures, eg on a hot day sitting on the sunny side of a bus with no air conditioning.

Our service will encourage children with epilepsy to participate in all activities at our service unless any are specifically excluded by the child's doctor or parents. Independence and social acceptance are important to all children. The Risk Minimisation Plan will cover whether any adjustments need to be made to an activity to ensure the child can participate. These may include the child wearing protective gear and providing increased supervision of the activity.

First Aid

Our service will ensure our qualified first aid educator maintains up to date training in epilepsy, and where required, training in the administration of epileptic medication. If a child is having an epileptic seizure, our first aid trained educator will:

- Protect the child from injury
- Not restrain the child or put anything in their mouth
- Gently roll them on to the side in the recovery position as soon as possible (not required if, for example, child is safe in a wheelchair safe and airway is clear)

- Monitor the airway.
- Call an ambulance if necessary. This may include when:
 - a seizure continues for more than three minutes
 - another seizure quickly follows the first
 - it is the child's first seizure
 - the child is having more seizures than is usual for them
 - certain medication has been administered
 - they suspect breathing difficulty or injury
- complete the Incident, Injury, Illness and Trauma Record in OWINA, including the time the seizure started and stopped and observations of the seizure, as soon as possible but within 24 hours of the seizure
- contact the parent/guardian or the person to be notified in the event of illness if the parent/guardian cannot be contacted.

The first aid trained educator may not call an ambulance when the seizure stops within three minutes and there are no complications (ie injury). Please refer to medical management plan or risk minimisation plan for guidelines. The child will be kept in the recovery position until conscious. Educators will always call an ambulance if required under the Medical Management Plan.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/11/2025

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

QA3	3.2.3	<i>The service cares for the environment and supports children to become environmentally responsible.</i>

National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

Regs	90	Medical conditions policy
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	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement—anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

Epilepsy planning and support guide for education and children's services DECS SA 2007

Epilepsy Foundation of Victoria

Epilepsy Action Australia

Early Years Learning Framework

92 – HEALTH AND WELLBEING POLICY

Position Statement

We aim to create a workplace environment where the health and wellbeing of employees is highly valued. Our workplace environment encourages and supports employees to maintain or adopt healthy lifestyles. We:

- are committed to providing team members with a safe, healthy and supportive environment in which to work;
- recognise that the health and wellbeing of our educators/staff is important;
- recognise that physical activity can be a strong protector against a range of medical conditions including cardiovascular disease and type 2 diabetes; and
- will commit to providing a supportive workplace culture where healthy lifestyle choices are valued and encouraged.

Definitions

Smoking: refers to tobacco, e-cigarette and vaping smoke.

Objective

Our policy objectives include:

- To raise awareness within the workplace about issues that impact on health and wellbeing, including the health benefits of physical activity, active travel, healthy eating, oral health, tobacco, e-cigarettes, vaping and sun protection.
- To promote a positive and equitable workplace environment where mental health and wellbeing is supported.
- To promote and encourage participation in workplace health and wellbeing initiatives within and outside the workplace.
- To educate employees about issues relating to health and wellbeing.

Implementation

- The service will achieve our objectives by abiding by and updating regularly, the following policies and procedures.
 - Asthma Policy
 - Anaphylaxis Policy
 - Dental and Oral Health Policy
 - Epilepsy Policy
 - First Aid Policy
 - Health and Hygiene Policy
 - Infectious Diseases and Immunisation Policy
 - Food Safety and Hygiene Policy
 - Smoke, Alcohol and Drug Free Environment Policy
 - Sun Protection and Safety Policy
 - Diabetes Policy
 - Nutrition, Food Beverage & Dietary Requirements Policy
 - Workplace Health, Safety and Wellbeing Handbook
 - Food Safety Program
- Hosting healthy eating and oral health education sessions and providing information sheets.
- Participate and complete the Health Together Achievement Program.

- Encourage active travel to work days
- Encourage physical activity challenge eg. Walk to school/childcare/work day.
- Promote the Quitline service to all employees.
- Provide all employees with information on how to support and maintain positive mental health in the workplace.
- We involve community members to support the children's learning. For example: dentists.

Policy Communication

- All employees will be made aware of this policy and its provisions via mediums such as staff notice board, verbal communication, intranet, e-mail, staff meetings and staff induction.
- It is important that all educators have the opportunity to become active participants in workplace policies, and these are shared with everyone in the company/organisation.
- All educators receive a copy of workplace policies during the enrolment and induction process.

Related Policies

- 35 - Anaphylaxis Policy
- 92 - Health and Wellbeing Policy
- 36 - Asthma Policy
- 85 - Diabetes Policy
- 88 - Child Safe Environment Policy
- 38 - Clothing Policy
- 30 - Dental and Oral Health Policy
- 28 - Nutrition, Food, Beverage, Dietary and Oral Health Requirements Policy
- 27 - Health & Hygiene Policy
- 41 - Smoke, Vape, Alcohol & Drug Free Environment Policy
- 42 - Sun Protection and Safety Policy

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 17/2/2026

Next Policy Reviewed: 15/2/2027

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
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	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>
	2.2	<i>Each child is protected.</i>

QA3	3.2.3	<i>The service cares for the environment and supports children to become environmentally responsible.</i>

National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

Regs	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement—anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

- **WorkSafe - Work Health “ Writing a healthy workplace policy”**
- **Staying Healthy: Preventing infectious diseases in early childhood education and care services (5th edition)**

93 – PHYSICAL ACTIVITY PROMOTION POLICY

Position Statement

Active play is about moving, being and doing. Children may engage in active play by themselves or with other children. Active play uses large muscles and provides children with a range of physical, emotional and social benefits. Outdoor play is very important as it helps develop gross motor and fundamental movement skills. Active play also helps develop a strong and healthy body, builds skills, creates feelings of wellbeing and helps protect from disease.

The educators, staff and management acknowledge the importance of active play and physical activity behaviours that contribute to good health and overall wellbeing.

As a health promoting service we will promote active play and physical activity for children, educators, staff and families through learning, policies, creating a safe and healthy physical and social environment and developing community links and partnerships.

Our service will promote physical activity to meet individual needs and also achieve the learning outcomes in accordance with the National Quality Framework and Early Years Learning Framework.

Objective

This policy confirms our commitment to:

- encourage children to engage in a range of child-initiated and adult-guided physical activities within the educational program
- promote the importance of a healthy lifestyle, which includes being physically active every day.

Implementation

The service will access the Get Up & Grow, Healthy Eating and Physical Activity for Early Childhood resources which promote the key messages included in the healthy eating and physical activity guidelines. The resources can be found at the below web address: www.health.gov.au

The Department of Health provides the following National Physical Activity Recommendations for Children (0-5 years):

Physical Activity Recommendations

- For health development in **infants (birth to one year)** physical activity – particularly supervised floor-based play in safe environments – should be encouraged from birth.
- **Toddlers (1 to 3 years) and pre-schoolers (3 to 5 years)** should be physically active every day for at least three hours, spread throughout the day.

Sedentary Behaviour Recommendations

- **Children younger than 2 years of age** should not spend any time watching television or using other electronic media (DVDs, computer and other electronic games).
- **For Children 2 to 5 years of age**, sitting and watching television and the use of other electronic media (DVDs, computer and other electronic games) should be limited to less than one hour per day.
- **Infants, toddlers and pre-schoolers (all children birth to 5 years)** should not be sedentary, restrained, or kept inactive, for more than one hour at a time, with the exception of sleeping.

In line with this, our service will implement the following promotion of physical activity as per the age and development stage of each child in attendance:

Educators will:

- Encourage children to participate in physical activities through programming and spontaneous experiences.
- Encourage and support children to undertake and participate in new or unfamiliar physical activities.
- Participate in physical activity with the children.
- Show enthusiasm for participation in physical activity and organise play spaces to ensure the safety and wellbeing of all individuals in the environment.
- Set up and plan for physical play activities and equipment and where appropriate encourage the children to help with the set-up.
- Listen to children's suggestions on what physical activities they would like to participate in and where appropriate incorporate them into the program
- Ensure that electronic media (ipads, DVDs, computers, projector screen, etc) are used at times by educators and children as part of intentional and spontaneous learning. The devices are used for educational purposes to deliver a program and provide additional resources for learning to take place. The use of the electronic media is to be used within the recommended guidelines.
- Set up indoor and outdoor areas in a manner that promotes and encourages safe physical play for all age groups and developmental abilities represented in the service.
- Actively encourage children to accept and respect each other's range of physical abilities.
- Consult with families and resource agencies on providing physical experiences that reflect diverse backgrounds and abilities.
- Role model appropriate footwear and clothing for physical activity (Educators can change footwear to sport shoes to perform physical activity session).
- Will ensure a balance of active and sedentary activities throughout the child's day and minimize sedentary behaviours unless the child is tired or ill.

The service will support the children in:

- Learning to use increasingly complex motor skills and movement patterns in order to combine gross and fine movement and balance skills, spatial awareness and problem-solving skills.
- The development of their physical skill set by providing regular opportunities for outdoor play.
- The development of their physical skill set by talking with children about how the human body and how important physical activity is for an individual's health and wellbeing.
- The development of their physical skill set by providing experiences for the children that draw on elements of dance, dramatic play and creative movement.
- The development of their physical skill set by providing babies with encouragement and safe areas to practice rolling over, sitting, crawling, standing and walking.

Engaging children, educators, staff and families

- Educators, staff, children and families are key partners in developing a healthy and active physical environment.
- Educators and staff engage children in developing active play initiatives through discussions and idea sharing.
- Educators, staff and families are provided with information, ideas and practical strategies on a regular basis to support active play in the service and at home.

- Educators encourage children to be active participants in setting up active play areas and spaces each day. This can be initiated through children, family and educator input into the curriculum.
- Educators are to plan outdoor and active play environment on curriculum documentation to ensure that active play guidelines are achieved and evaluated. Planning of physical environment should take into consideration professional development learnings and child support agency strategies.
- Families' experiences, expertise and interests are drawn upon to support active play initiatives.
- Families and children from culturally diverse backgrounds are engaged to ensure cultural values and expectations about physical activity are respected.

Community partnerships

- The service works with local health professionals, services and other organisations to increase educator and staff capacity to support children's physical development and active play initiatives.
- Information about community physical activity opportunities is made available to families through discussions, newsletters and visual displays, for example a service could promote information about local community clubs and walking groups.

Road safety, Active Travel & Movement

- Road safety education is incorporated into the educational program.
- The service engages in walking excursions within the local community that promote physical activity and safe active travel.
- Active travel, such as walking, riding a bike or scooting to and from the service is encouraged for educators, staff, children and families.
- The service aims to identify and address barriers for active travel within the local environment.
- Space is provided at the service for children to leave active travel equipment.
- The service promotes active travel and movement through participating in various walking excursions and fund raising events (e.g. Relay for Life). Where possible the service will promote the physical activity to attend and participate in excursions and events.

Related Policies

- 35 - Anaphylaxis Policy
- 92 - Health and Wellbeing Policy
- 36 - Asthma Policy
- 85 - Diabetes Policy
- 88 - Child Safe Environment Policy
- 38 - Clothing Policy
- 30 - Dental Care Policy
- 28 - Nutrition, Food, Beverage, Dietary and Oral Health Requirements Policy
- 27 - Health & Hygiene Policy
- 41 - Smoke, Vape, Alcohol & Drug Free Environment Policy
- 42 - Sun Protection and Safety Policy

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 19/3/2026

Next review date: 15/03/2027

Endorsed by: Approved Provider, Centre Director, Educators & Parents

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted</i>
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National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

EYLF

LO3	<i>Children become strong in their social and emotional wellbeing</i>
	<i>Children take increasing responsibility for their own health and physical wellbeing</i>

Other References:

- National Physical Activity Recommendations for Children (0-5 years)
<http://www.health.gov.au/internet/main/publishing.nsf/content/health-pubhlth-strateg-phys-act-guidelines#npa05>
- Healthy Together Achievement Program for early childhood services: Sample active play and physical activity policy

94 – CYSTIC FIBROSIS POLICY

Position Statement

We are committed to providing, as far as practicable, a safe and healthy environment in which individuals can participate equally in all aspects of the service program and experiences. We will aim to minimise the risks associated with illness for those attending our service by implementing the following policy and procedures.

Objective

The service and all educators will effectively provide care for children with Cystic Fibrosis. The service and all educators will ensure the safety and wellbeing of all children and will adopt inclusive practices to cater for the additional requirements of children with Cystic Fibrosis in a respectful and confidential manner.

Implementation

The service will ensure all educators are aware of the enrolment of a child with Cystic Fibrosis (CF) and have an understanding of the condition and the additional requirements of the individual child. The service will adhere to privacy and confidentiality procedures when dealing with individual health needs.

Confidentiality, privacy, dignity and safety

Young children often enjoy sharing the news and their experiences of living with CF with their classmates. The degree and nature of this sharing should be discussed with parents so that they can support their child in this process.

Information exchange between the family and health professionals and the service is essential to support the child's learning and enhance peer support. The sharing of information needs to be assessed and negotiated for each child with CF, with due consideration to their needs. Educators need information about routine and predictable emergency care as it affects the child's access to the curriculum, and their safety.

Health Support Plan

For each individual child enrolled in the service with CF, a Health Support Plan will be developed by the Nominated Supervisor in conjunction with the child's family. It will be based on the child's health support needs as identified in their CF care plan and other care information (for example if the child also has asthma or diabetes).

A Health Support Plan for a child with cystic fibrosis should address the following components:

- overall wellness
- diet
- therapy and care
- internal body temperature control
- curriculum participation issues and
- potential emergency/first aid situations.

The information should focus on what educators need to know to provide routine and emergency care. It will be used by educators in planning support for the child.

In addition, a health support plan documents individualised support which educators have agreed to provide in the areas of:

- first aid

- supervision for safety
- personal care, including infection control
- behaviour support and
- additional curriculum support to enable continuity of education and care.

Educator and staff are not permitted to perform medical procedures (this does not include performing First Aid).

Overall Wellness

- The service and educators need to know if recent/frequent hospitalisation and/or general unwellness mean additional care and consideration. They also need to know of any infection control issues in addition to standard precautions.
- It is important to the future health of a child with CF, as with all children, to minimise the risk of cross infection of bacteria and viruses from others. This must be balanced with efforts to encourage children with CF to lead as normal lives as possible.
- Educators will alert the family of a child with CF when a particularly virulent strain of virus is present in the service, as parents may wish to keep their child with CF at home.
- All children in the service should be encouraged maintain hygienic practices. If possible, a child with CF should, discretely, not be partnered or sit next to another child with an obvious cold or cough.
- If child's with CF or other children's health and wellbeing is compromised and effected or educators can not adequately provide a safe and healthy environment for all children, the service may need to be assess the child care arrangements and enrolment. This may include but not limited to reducing hours of attendance, reducing days of attendance or seeking alternative care arrangements.

Diet

- Children with CF have difficulty maintaining their weight and growth patterns as they cannot absorb essential vitamins, minerals, fat and proteins. For this reason educators need to be aware of each individual child's dietary requirements as prescribed by a medical professional.
- Children who need additional food supplements may receive them through a gastrostomy button located in their stomach. There are no routine care issues associated with a gastrostomy button for educators however if the area becomes red or inflamed, parents should be informed as soon as possible
- Children with CF will often have non-prescription medication such as enzyme tablets, as well as prescription medication such as antibiotics, which the service and educators need to be aware of.

Therapy and Care

- Some children with CF may require complex/invasive health support, such as physiotherapy, while attending the service. This support should be provided by a visiting nurse or therapist.
- Some children with CF require nebulised medication prior to physiotherapy. While educators can supervise child while Health Worker or Parent provides nebulised medication, this will be managed by a visiting health worker or Parent. Health workers and information will be coordinated and supplied by the family to deliver these medical procedures.

Body Temperature Control

- Children may need to be reminded to adjust their clothing to help maintain their internal body temperature control.

- A child with CF will have problems with internal temperature control and should be kept at a steady temperature in winter and summer. It is beneficial to place the child with CF in rooms that have heating and cooling where practical.
- Salt tablets may be required during warm weather. Educators should be informed about the required timing and amount of salt tablets and ensure the child has access to fluids at all times. Medical advice will be considered.

Participation in Education and Care Experiences

- An increase in fatigue or feeling tired is common for a child with CF. A lot of effort is required of a person with CF, on top of normal childhood activities, to maintain their health. Educators will be aware of this and provide adequate opportunities for rest.
- During the onset of infections, children with CF may experience difficulty breathing or catching breath. Educators should be aware that, as with other children, breathing difficulties also can be asthma related.
- Children with CF are continually battling infections or recovering from them, thus resulting in low energy levels and reduced concentration. Educators will be mindful of this when planning daily activities.
- A regular exercise program is very beneficial to children with CF as it helps loosen mucus, stimulates coughing and helps build up strength and endurance of the breathing muscles. Children with CF will be encouraged to take part in physical activity and exercise, following guidelines from the child's medical practitioner.
- Children with CF can become dehydrated much more quickly than other children. In relation to this educators will:
 - encourage frequent drinks during and after exercise, and on warm days
 - ensure salt tablets are taken either before or after exercise on warm days with consideration of medical advice
 - avoid scheduling physical activity during temperature extremes
 - ensure children with CF remain, as far as is practical, in a fairly constant temperature, neither too hot nor too cold.

Potential Emergency Situations

Emergency situations associated with CF are rare.

If children have an intravenous line for medication, there are specific standard first aid responses which may be anticipated:

- Child reports discomfort, nausea, rashes or general unwellness.

Call family emergency contact. If they cannot be reached, call the nominated cystic fibrosis nurse for advice.

- Child reports redness, pain, inflammation or swelling at site.

Call nominated cystic fibrosis nurse for advice, and then advise family emergency contact.

- There is a leakage of some sort from the site.

Call nominated cystic fibrosis nurse for advice, and then advise family emergency contact.

- A needle or line falls out.

Use standard first aid and apply pressure to stop any bleeding, call nominated cystic fibrosis nurse for advice, then advise family emergency contact.

Supervision for Safety

The child's Health Support Plan may include a range of routine accommodations so they can continue to access learning programs while effectively managing their health care.

Accommodations could include:

- provision of additional time (through approved funding channel) to support children managing their dietary requirements
- access to fluids and food, and the toilet, as needed
- rescheduling of physical activity to support body temperature control
- supportive and sensitive encouragement to participate in physical activity
- targeted social skills programs: frequent absences mean that some children with cystic fibrosis have difficulty making and retaining friends
- modification of the program and activities in response to the demands of therapy and treatment

Infection Control Consideration

Educators should be aware that, where there is more than one family in the service with CF, cross-infection is a serious health risk. For this reason, our service will only accept the enrolment of one child with CF at any given time. This is based on guidelines developed under the Cystic Fibrosis in Education and Children's Services Planning and Support Guide for Education and Children's Services 2008.

Behaviour Support

As for all children, behaviour expectations for children with CF should be consistent and predictable, and also sufficiently flexible to accommodate periods of stress and other potential mental health issues.

Policy Communication

- All employees will be made aware of this policy and its provisions via mediums such as staff notice board, verbal communication, intranet, e-mail, staff meetings and staff induction.
- It is important that all educators have the opportunity to become active participants in workplace policies, and these are shared with everyone in the company/organisation.
- All educators receive a copy of workplace policies during the enrolment and induction process.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing. (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 16/4/2026

References:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
	2.1.2	<i>Effective illness and injury management and hygiene practices are promoted and implemented.</i>

National Act and Regulations

Education and Care Services National Law Act
Education and Care Services National Regulations

Regs	90	<i>Medical conditions policy</i>
	91	<i>Medical conditions policy to be provided to parents</i>
	92	<i>Medication record</i>
	93	<i>Administration of medication</i>
	94	<i>Exception to authorisation requirement—anaphylaxis or asthma emergency</i>
	95	<i>Procedure for administration of medication</i>
	96	<i>Self-administration of medication</i>

EYLF

LO3	<i>Children are happy, healthy, safe and connected to others.</i>
	<i>Educators promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community</i>
	<i>Educators discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all</i>

Other References:

- **Cystic Fibrosis in Education and Children's Services Planning and Support Guide for Education and Children's Services 2008**
- **Early Years Learning Framework**

95 – NON-COMPLIANCE POLICY

Position Statement

All members of the service, including families, educators, students, visitors, management and approved provider aim to work in partnership to ensure the best outcomes for the children in their education and care. In addition, families and staff agree at the time of enrolment or employment to familiarise themselves with and comply with our Philosophy, Code of Ethics and all policies and procedures.

Despite the best efforts of all, inevitably there will be times when conflict or simply differences of opinion arise regarding a particular policy or procedure. The Non-Compliance Policy and Procedure is to be used as appropriate, in conjunction with the Grievance Management Policy and Procedures or Staff Grievance Management Policy and Procedures.

This policy uses the term ‘family’ generically to encompass enrolled children’s parents and/or guardians and includes extended members involved in the care of the child at the service.

Related Policies

Family and Visitor Code of Conduct Policy – 8
Behaviour Guidance Policy - 9
Enrolment Policy – 1
Ethical Conduct and Code of Ethics Policy – 70
Complaints and Grievance Policy - 7
Occupational Health & Safety Policy – 10
Exclusion Policy – 43
Health and Wellbeing Policy – 92
Child Safe Environment Policy – 88
Education, Curriculum and Learning Policy – 89
Governance of Service, Management and Educator Policy – 78
Team Member Grievances - 54

Objective

The non-compliance Policy and Procedure has been created to formally recognise, promote and protect the rights of all persons in the service.

If, after due process (refer to: Complaints and Grievance Policy – 7 or Team Member Grievances - 54), the Centre Director or Approved Provider or other nominated party deems that a person in the service community has not complied with the services policies or procedures, the Non-Compliance Policy & Procedure will be applied as appropriate.

Rationale

Our service recognises it has a duty of care to take all reasonable practicable steps to provide the service’s community with a safe and healthy work environment (Work Safety Act, 2008). In addition, this policy has been developed to comply with:

- The Philosophy and Code of Ethics.
- The Education and Care Services National Law Act, 2010
- The Education and Care Services National Regulations, 2011
- The Australian Children’s Education and Care Quality Authority’s (ACECQA) National Quality Standard for Early Childhood Education and Care and School Age Care, 2012.

Implementation

It is expected that all persons who access our service will abide by all policies and procedures at all times.

1. Should a grievance arise relating to a particular policy and/or procedure, the relevant Grievance Management Policy and Procedure will be followed. Refer to: Complaints and Grievance Policy – 7 or Team Member Grievances – 54 and the Grievance Register and Procedure Handbook.
2. If the grievance is unable to be settled through informal discussions, the Centre Director, Approved Provider or other nominated party, will investigate and mediate formal grievance meetings according to the guidelines in the relevant grievance management policy.
3. Where the Mediator decides there has been an infringement of a policy and/or procedure by a staff member, they may instigate disciplinary action. Refer to: Employment Policy – 52 and People Management Handbook.
4. Where the Mediator decides there has been an infringement of a policy and/or procedure by a family member, student, volunteer or visitor, the Committee may instigate disciplinary action. Refer to: Non-Compliance Procedure given below.

Non-compliance Procedure

Step 1: Verbal Warning

The Centre Director, in consultation with the Approved Provider will issue a verbal warning.

Step 2: First and Final Written Warning

If the infringement continues, a first and final written warning will be issued which will:

- outline the infringing behaviour;
- request immediate cessation of this behaviour;
- warn of termination of the person's access to the service should the infringement continue (eg, termination of the non-compliant family member's child/ren's place at the service).

Step 3: Termination of the person's access to the Service

If the infringement continues, the Centre Director, in consultation with the Approved Provider, at their discretion:

- terminates the person's access to the service. This includes immediate termination of the non-compliant family member's child/ren's place at the service.

A copy of the warning will be kept on the individual's file and noted in the Grievance Register.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 15/4/2025

Reference:

NQS

QA4	4.2	Management, educators and staff are collaborative, respectful and ethical.
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	4.2.2	<i>Professional standards guide practice, interactions and relationships.</i>
	4.2.1	<i>Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.</i>

QA7	7.2.3	<i>Educators, co-ordinators and staff members' performance is regularly evaluated and individual plans are in place to support learning and development.</i>
	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

National Act and Regulations

Education and Care Services National Law Act 2010
Education and Care Services National Regulations

96 – TECHNOLOGY & DIGITAL USAGE POLICY

Position Statement

The safety of children attending early childhood education and care services is crucial. Monitoring how children's images or videos are taken, stored and shared is difficult on personal devices. Using only service-issued devices helps services manage the risks to children.

We feel technology and computers are an essential learning tool for today's child, and it is incorporated into the child's learning environment. As we embrace the digital age, it is crucial to integrate technology into learning environments in a manner that supports academic growth, creativity, and critical thinking. However, this integration is guided by child-safe standards to ensure a safe, secure and effective learning experience.

Related Policies

Child Safety Standard Policy - 12A
Child Safety, Protection & Wellbeing Policy - 12B
Employment Policy – 52
Internet, Email and Social Networking Policy – 62
Education, Curriculum and Learning Policy – 89
Record Keeping and Retention Policy - 84
National Model Code Policy – 100

Objective

This policy outlines the guidelines for the use of digital devices by children, employee and volunteers in our childcare setting. Our aim is to ensure that technology is used appropriately and safely, enhancing learning and development while safeguarding children's well-being.

The Service will provide use of information technology devices as an extension to the curriculum assisting in development of social, physical, emotional, cognitive, language and creative potential of each child in accordance with Australian electronic media children usage guidelines.

This policy should be read with other related policies.

Definitions

Photo/Photograph/Video: any visual recording of children, either still or moving.

Audio: means a sound recording of children, including voices and other sounds made by children.

Service-issued devices: devices provided by our service for professional use, including phones, tablets, cameras, and computers.

Digital Devices: Computers, tablet, smartphones, e0-readers, educational robots, wearable technology, gaming consoles, etc.

Restricted Personal Electric Devices under the National Model Code are:

- personal devices that can take images or videos and
- personal storage and file transfer media.

Examples of restricted Personal Devices:

- phones

- tablets
- digital cameras
- smart watches
- wearables, such as camera glasses
- SD cards
- USB drive
- hard drives

Implementation

Guidelines for Use of Technology and Digital Devices by Children

In relation to children, Educators will:

Assess Age-Appropriate Content

- Digital devices should only be used for activities and applications that are suitable for young children, promoting cognitive, social, and emotional development.
- All applications and programs should be vetted for suitability and educational value.
- Educational content must be reviewed and approved by educators to ensure it aligns with learning objectives.
- Technology is predominantly used where play-based and 'hands-on' experiences cannot provide the same information (e.g. investigating planets or dinosaurs). It will not replace appropriate experiences nor professional pedagogy.
- Teach children basic digital safety messages in age and developmentally appropriate ways.

Supervise Access and Interaction

- Children will use digital devices only under the supervision of qualified educators to ensure safe and meaningful interactions.
- Educators will facilitate activities that encourage exploration and learning through technology.
- Children are not permitted to bring personal digital devices to the service.
- Be alert to potential signs of exposure to inappropriate material or online harm (e.g. changes in behaviour, secrecy).
- Model respectful and safe online behaviour, including not oversharing, and asking before taking photos.
- Encourage children to talk to a trusted adult if they see or experience something online that makes them feel uncomfortable.

Set and Review Time Limits

- Screen time for young children will be limited to short, intentional periods, ensuring that it complements rather than replaces physical play and face-to-face interactions.
- Specific time limits will be established based on age and developmental needs in accordance with timeframes for 'screen time' according to Australia's Physical Activity and Sedentary Behaviour.

Encourage Active Engagement

- Activities using digital devices will promote active engagement, such as interactive games, storytelling apps, and creative projects, rather than passive consumption of content.
- Educators will encourage discussions and reflections on digital experiences.

Digital Literacy and Safety Education

- Children will be introduced to concepts of digital literacy, including understanding technology use, recognising appropriate content, and practicing respectful communication.
- Safety guidelines will be reinforced, teaching children about online privacy and the importance of not sharing personal information (In an age appropriate manner).

Involve of Families

- Parents and caregivers will be informed about the digital practices within the early childhood setting and encouraged to engage in discussions about technology use at home.
- Talk with families about how online safety is promoted at the service and how they can support it at home.
- Resources will be provided to families to support healthy technology habits. For example resources from the e-Safety Commissioner- Early Years Program.

Regularly Evaluate and Reflect

- The use of digital devices in the early childhood setting will be regularly evaluated to assess its impact on children's learning and development.
- Feedback from educators, parents, and children will inform ongoing improvements to the policy.

Computer and Related Technology Usage

- Computers and devices at the service may only be used for work relevant to the operations and activities of the service. Examples of these activities include administration, research, programming and professional development.
- If relevant to the children's learning, child appropriate websites may be accessed. However, children will only access the computers when directly supervised by educators.
- Similarly, music, videos etc may be streamed from the computer if it is relevant to the children's learning or relevant to research or professional development undertaken by educators. However, streaming of this kind will only take place from websites where this can legally take place.

SERVICE-ISSUED DEVICES

- Service-issued devices are used solely for work-related tasks, such as:
 - Documenting children's learning and activities
 - Communicating with families about children's progress and daily activities
 - Accessing educational resources and child development information
 - Completing administrative tasks such as reporting and record-keeping
 - Participating in professional development and training
 - Authorised promotional and marketing purposes
 - Internal communications
 - Documenting incidents or child safety matters
- Only service-issued devices can be used for taking images or videos of children in our care.
- Personal electronic devices will not be approved as service-issued devices.
- The use of the internet on service-issued devices is permitted for accessing child-appropriate educational websites, professional development materials, and communication tools. Inappropriate digital content (e.g., websites, apps, videos) should not be used on service-issued devices.
- Access to social media platforms is not to be used unless approved for service-related purposes.
- AI tools that have been approved by the approved provider or nominated supervisor may be used on service-issued devices (see guidance below on AI tools).
- Staff must not use any unapproved storage media to store or access content or data on service devices.
- Shared devices will be allocated according to the educational needs, age group, and specific activities in each room, the frequency of use, the need for specific applications, and the ability to support learning objectives.

- Where possible, our service provides dedicated devices in each room to ensure availability and accountability. Where sharing is necessary, room leaders will share according workflow plans or rosters.
- Shared devices should be handled with care, wiped clean after use, properly maintained by the staff using them and stored in our secure storage when they are not in use and at the end of every day.
- Staff must not take any service-issued devices home with them (permission must be obtained from nominated supervisor or approved provider).
- Staff may only take, use, store images, videos and audio recordings of children on our service devices (not their personal devices), and only with the appropriate parental authorisation.

Unacceptable Use of Service Technology

The following actions are strictly prohibited at our service:

- Using technology in any way that breaches our Service Policies such as our Child Safe Code of Conduct
- Accessing systems, data or networks without authorisation
- Installing or using software without authorisation
- Misusing the service's equipment or resources
- Circumventing security measures
- Sharing passwords or login details
- Introducing viruses, malware or other malicious code
- Hacking or trying to gain unauthorised access to systems
- Spamming or sending out unsolicited mass emails
- Using the IT systems to harass, threaten or bully other users
- Sharing or creating inappropriate, discriminatory or offensive content
- Violating intellectual property (e.g., using pirated content or software)
- Using the service's network for illegal activities
- Disrupting or damaging our systems, networks, equipment or resources
- Uploading personal or sensitive information onto an unauthorised AI tool
- Disclosing confidential information without authorisation

Identification of Service Use Devices

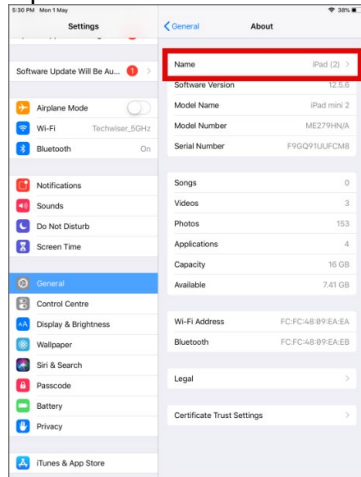
- The service has stickers that are placed on all service-issued devices. Service logo – with the Words “Service-issued Device”. The ‘Service-issued Device’ Sticker will be placed on the device and also a second sticker on the case (if applicable).
- Service-issued devices are identifiable from a distance: Sticker placed on device size – 9.5cm x 9.5cm. “Service-issued Devices” are the only devices that are permitted to be used when educating and care for children while working with children. (no other devices should be used in these environments).



- The Service-issued device sticker will be held securely by the nominated supervisor and approved provider. Upon purchase of new service devices the nominated supervisor or responsible person in charge of the service will affix the ‘Service-issued device’ sticker before provided device to a staff member. If the sticker becomes damaged or removed a new sticker will be affixed to the service-issued device.
- Service-issued devices have a device identification code unique to the device. I.e Device serial number. The following are the device identification codes:
 - Laptop and Computers: Serial Number of Device. For example:



- Ipad: Serial number is found in Settings > General. For example:



Service-Issue Device Register

- A 'Service-Issued Device Register' will be maintained by the nominated supervisor, responsible person in charge of service and approved provider of the service.
- Registers history will be kept and uploaded as a scanned document and sent via email to the Approved Provider and uploaded to OWNA.
- The Service-Issue Device Register will record the following information for all devices:
 - Device Type (eg laptop, ipad, etc)
 - Brand
 - Model Name
 - Serial Number
 - Location of device (Room, Office)
 - Device issued to: Name of staff member
 - Date issued to staff member
 - Date returned by staff member
- Use Service Issue Device Register template to record all service-issued devices.
- The 'Service-Issue Device Register' is kept in the office in the Central Compliance Register folder. The Register is updated and reviewed on change of devices or personnel information by the nominated supervisor or responsible person in charge of service

Minimising service-issued device access by non-authorized people risk

- Updating software on all devices to install any security updates.
- On service-issued devices, the service will install and update virus, spam and security software.
- Not sharing or allowing access to service devices to unauthorised people.
- Using password protection and multifactor authentication, and regularly updating passwords. Passwords are to be kept confidential.

- Regularly check who can access IT systems and devices and promptly remove access for people who leave the service. Removing access to online platforms when families cease their child's enrolment or staff leave the service.
- Share information about children with families in a way that manages risks, only using service-issued devices and approved online platforms, like such as OWNA and Office 365.
- Consider whether it is appropriate to share information with people and how to do so safely in accordance with the service policies and the National Model Code guidelines.
- Understanding the terms and conditions of online platforms, including how information is stored. Using approved software and apps only.
- Follow privacy laws when using online tools to share information with families.
- Keep a record of service-issued devices, who can access them, and their login details.
- Service will enable internet browser security and regularly check internet usage history on all service issued devices/each device that will be used by children and staff.
- Check privacy settings and recommended age restrictions on all service issued devices/each device that will be used by children.
- Do not download, delete, turn off non-authorised chat functions, apps and games. Select appropriate app settings, which could include turning off location settings and enabling privacy controls on each device.
- Actively supervise children's use of electronic devices, including the content they engage with, while at the service. Children are to be directly supervised when using service-issued electronic devices.
- Following the device security storage policy and procedure for service-issued devices to ensure their safety stored and use managed, including when staff move between locations.

Securely store service-issued devices when not in use

- Each service-issued device will have a designated locked area for storing devices when not in use (e.g., a secure cabinet, cupboard or storeroom).
- Outside service hours devices will be stored in secure designated locations.
- This location will only be accessible by authorised personnel.
- If travelling for work purposes service-issued devices should be locked in boot of car or stored out of sight.

Using AI tools safely

- If using AI, Approved Provider, Nominated Super or Responsible Person in charge of service will give clear instructions to service leaders, educators and staff about using AI tools for certain tasks. Be specific about when and how AI can and can't be used at the service. Before using AI tools instruction should be obtained by educator or teacher.
- Do not input information into an AI tool that could reveal personal details, including about children (like names, birthdates, or other identifying information).
- Please manage data by turning off/resetting chat history and not allowing our data to be used to train AI models.
- It is essential to fact check content generated by AI tools and consider whether it has taken into account all relevant information.
- Please assume any information being input into generative AI tools could become public and think about what is appropriate to include. There maintain confidentiality and privacy at all times.

PERSONAL DEVICES

Personal Devices, Mobile Phones and Smart Watches Restrictions

Please refer to the following policies and procedures for further guidance on this topic:

- 100 – National Model Code Policy
- 12A - Child Safe Standards Policy
- 12B - Child Safety, Protection & Wellbeing Policy
- 16 – Privacy & Confidentiality Policy
- 19 - Incident, Injury, Trauma and Illness Policy
- 48 – Excursion Policy
- 62 - Internet, Email and Social Networking Policy
- 84 – Record Keeping and Retention Policy

Restrictions on use of personal devices apply to any person who is providing education and care and working directly with children. Restrictions apply to the following people:

- teachers and educators, including casual and agency staff
- students attending the service as part of a practicum and representatives of tertiary providers who attend the service to assess students
- volunteers, including parent volunteers
- any third parties delivering programs or incursion activities to children in a service, whether paid or unpaid
- allied health and inclusion professionals (Eg (ISS or KIS) attending a service to observe, assess or work with a child at the service
- mentors or coaches attending the service to support teachers or educators working with children or providing education and care
- preschool field officers
- primary school teachers attending a service as part of a school transition program.
- If a third party professional attending the service and working directly with children (such as an allied health or inclusion professional) needs to use a device (for example, to undertake an assessment or take notes) they can use a device that is:
 - issued by their business or institution; and
 - used only for work purposes (and not personal use).(please see section below for detailed guidance)

Our practices align with the National Model Code, which sets out how to implement child safe practices for electronic device use in education and care services. Please see service policy 100 – National Model Code Policy for further guidance.

- Only service-issued electronic devices are to be used when taking images or videos of children while providing education and care.
- Restricted Personal Devices such as phones, tablets, digital cameras, smart watches, wearables (such as camera glasses), SD cards, USB drive, hard drives should not be in the possession of any person or used while they are providing education and care and working directly with children.
These personal devices are able to take images, videos, recordings, transfer information and data and therefore are required not to be used or in possession while providing education and care and working directly with children. Further, smart watches are an unnecessary distraction for active supervision and engagement.
- Mobile phones are required to be turned off or kept silent mode while the team member is working and in the service to minimise noise.
- Centre Directors and/or Responsible Person in Charge of Service are permitted to use mobile phones to perform their management and responsible person in charge duties in the Service's office (a non-children environments) and while not providing education and care or working directly with children. Calls and use of mobile phones

by Centre Director / Responsible Person are restricted to office and not non-child environments to perform service work-related tasks.

- There is no objection to team members use of mobile phones or smart watch while on a lunch break and before or after work while not providing education and care and working directly with children. These must be used in non-children accessible areas (Eg. staff room). Your personal device, mobile phone and smart watch should be left in your bag / locker during work shifts times and when you are educating and caring for children.
- If you need to receive an urgent call that cannot be attended during your break or lunch; please provide the services telephone number and the service team will do their best to accommodate by taking a message or providing you time to take the call in private. If you are expecting this is needed please discuss with the Centre Director / responsible person in charge of service.
- ***Employees using personal devices to Access Arrival or other Service Software:*** Employee cannot use personal devices for multi-factor authentication to access and use Arrival while providing education and care and working directly with children. If not working directly with children, devices can be used. In non-children areas such as the service office.

Use of personal electronic devices

- educators, other staff and volunteers are not to make or receive personal phone calls / communications on personal devices or service-issued devices when working with children as this impacts the active supervision of children.
If an educator, staff member or volunteer is expecting an important call during the day, the call should be taken using a service phone during a scheduled break time. If it is an emergency call this will be coordinated between the staff member and the Centre Director / Responsible Person to ensure the safety and supervision of children is maintained.
- any staff member needing to be contactable during work hours must direct callers to the services contact telephone.
- staff and volunteers are to store and leave their phone, personal devices and other personal belongings in a cupboard or locker. The personal devices are not to be held on their person or left in view in the classroom or storage room. Phones and personal devices may also be stored in the office in a designated container or locker (turned off) during the day.

Failing the policy and procedure, may mean that children are not adequately supervised and breach of service policies which may result in disciplinary action as per the Grounds for Instant Dismissal.

When National Model Code Personal-Device Restrictions Do Not Apply

The restrictions on the possession of personal devices do not apply to people who are not working directly with children.

Examples include:

- parents and carers doing drop-off or pick-up.
- Victorian Regulatory Authority Authorised Officers, police and officers of other regulators, such as environmental health officers.
- third-party contractors who are attending the service but not working with children or providing education or care (for example, maintenance contractors).

Child safe policies and procedures and supervision requirements continue to apply in the above people. Parents, guardian and carers and third-party contractors are not permitted to use personal devices for calls, taking of photos or recording of videos in the location of the service where children are present.

If a third-party contractor (such as a maintenance person) is required to take a photo or use phone to troubleshoot a fault or repair an educator or service staff member will be present in the room or outdoor environment. Photos or video recordings will not contain any children.

Teachers, educators and other staff can access personal electronic devices while taking a scheduled break from work, such as a lunch break and when they are not providing education and care or working directly with children and in child areas.

Personal Devices - Children, Parents, Carers and Families: Calls, Photos and Video Guidelines

- Personal devices are not permitted to be used by children, parents or guardians in children areas or locations of the service. This includes rooms, outdoor environments or hallways.
- Children are not permitted to bring or use personal devices to the service.
- With permission from the Service, parents and families can take photos and videos of their child at our service, but should not take photos or videos of anyone else's child unless they have the consent of the child's parents.
- Parents and families should not share any photos or videos that include identifiable characteristics of other children, staff, visitors or families without getting the relevant consent first. This includes sharing images on social media, websites, or any other platform.

Exceptions to Personal-Device Restrictions

- The National Model Code lists the following essential purposes for which the use or possession of a personal devices may be authorised where access does not impede the active supervision of children and Service phones not available:
 - communication in an emergency situation to ensure safety (lost child, serious injury or accident)
 - emergency communication during excursions and regular outings (for example, when groups of children and educators get split up)
 - emergency communication when children are transported or travel on transport arranged by the service.
 - in the case of a lockdown or evacuation of the service premises
 - technology failure; temporary out of service-issued electronic devices.
- The Victorian Regulatory Authority does not consider routine communication during bush, beach or other nature programs to be an essential purpose for which approved providers can authorise the possession and use of personal devices.
- Permission for use of mobile phone will be obtained from Centre Director or Responsible Person in Charge of Service to use phone for excursion or special event or purpose. Personal Devices are only permitted to be used in emergency situations and should not be kept in person hand (i.e. store in bag). See below guidance.
- For those educators who can access the internet from their mobile phone or smart watches, are not to not access the internet (whether they are using the service's Wi-Fi or their personal data plans) via their mobile phones during their work hours. Staff members should use the service's computers and devices for work relating to their job role.
- Any staff member found to be using the computers inappropriately will face an enquiry by management and other relevant parties to decide a course of action based on the severity of their misconduct. Please to related policy regarding this topic.
- This policy is also incorporative of state and federal laws regarding computer usage. Should a staff member or other relevant individual use the service's computers in a way that breaks a law, the service will take the appropriate required action (eg. Reporting to the police). Furthermore, the staff member or individual will face an enquiry held by management and other relevant parties to assess whether this conduct will affect their role within the service's operations.

Authorisation Personal Device for 'Essential Purposes'

- Authorisations for the possession and use of personal devices for 'essential purposes' is required be documented in writing.
- Advance written authorisation needs to be sought for use of Personal Device for an 'essential purpose' from Approved Provider or Nominated Supervisor. This will be given only in accordance with the National Model Code guidelines and the service policies.
- *Excursions* (See 48 - Excursion Policy): Person devices (such as a mobile phone) are not to be used while on excursions, except in emergencies circumstances. Authorisations for the possession and potential use of personal devices for 'essential purposes' will be documented in the excursion risk assessment and management plan as follows. "Name of person authorised to possess and use personal device in emergency situation while on excursion: <Insert Name>."

Digital devices (laptops or iPads) use by Students, Trainers and Assessors

Students performing placement at the service are subject to the same policies and procedures of employees. Please refer to the policies and procedure of the service for guidance. In summary, no personal devices are permitted to be used in the service by students.

The following outlines the appropriate use of digital devices (laptops or iPads) by trainers and assessors while conducting student observations during vocational placement in our early service.

1. Permitted Use:

- The use of a laptop or iPad during student placement visits is solely for the purpose of digitally recording observational evidence of student performance to meet assessment requirements for the Certificate III, Diploma in Early Childhood Education and Care and Early Childhood Degrees. Devices may only be used to **document student observations and assessment notes**.
- All use must be directly related to course assessment and conducted in a professional, discreet manner.
- Third-party professionals must use a device issued by their business/institution (and not a personal device).

2. Prohibited Activities:

- **Photography, video recording, or audio recording of students, children, or staff is strictly prohibited.**
- The **camera on the device must remain covered at all times** while onsite of the service.
- The device must **not** be used to access personal content, social media, or unrelated applications while onsite.

3. Confidentiality and Data Security:

- For student vocational placement in early childhood education and care services: All observation notes must relate to **student performance only**, with no inclusion of identifiable child information.
- No content or notes are to be shared outside the assessment context or stored on unsecured platforms.

4. Professional Conduct:

- The trainer/assessor must comply with the policies of both Registered Training Organisation and the service.
- The use of devices must not interfere with the daily operations of the early childhood service.
- Devices must be used with respect for the privacy, confidentiality, and safety of all individuals in the service environment.

TELEVISION AND DVD PLAYER USAGE

The T.V will be an additional tool to enhance curriculum activities, not a substitution.

Guidelines for use would be:

- To assist in expanding the content of the daily program and current affairs.
- Be suitable to the needs and development levels of each child watching.
- Chosen programs should hold the interests of the children
- Long Day Care and free activity times can be assisted when inclement weather keeps children indoors.

Programs must be carefully selected with suitable content. Programs depicting violence e.g. graphic news reports should not be shown. Children are to view '**G**' rated video content only.

Educators will sit with the children to monitor and discuss any aspects of the video or television program they are viewing.

SCREEN TIME GUIDELINES

Screen time refers to the amount of time spent watch TV including videos and DVD's; playing computer games on video consoles or on computers and using computer for other purposes. Screen time also refers to using telephones for texting and social networking. Electronic media is a term also used to encapsulate all forms of screen based activities.

Australian Guidelines for Screen Time

- Children aged 2 to 5 years: For children 2 to 5 years of age, sitting and watching television and the use of other electronic media (DVDs, computer and other electronic games) should be limited to less than one hour per day.
- Children aged less than 2 years: Children younger than 2 years of age should not spend any time watching television or using other electronic media (DVDs, computer and other electronic games).

(Source Australian Government Department of Health: Move and Play Every Day: National Physical Activity Recommendations for Children 0-5 years) See appendix.

Developing healthy screen time habits recommendations

Developing healthy screen time habits while they're young will help children make better choices about how to use their free time when they're older. You can help by:

- setting screen time guidelines according to the ages of children in your family
- leading by example, limiting your own screen time
- offering variety, making sure you have a range of activities and objects to entertain and stimulate your children so they don't look to the screen so much
- being choosy about what your younger children watch or play on the computer, and taking an interest in what your older children are doing online
- keeping TVs and computers in family spaces and out of children's bedrooms
- turning the TV off before school and at dinnertime.

(Raisingchildren.net.au, 2016)

TV and DVDs

When young children watch TV and DVDs, they don't see the same things adults do. Children can be negatively affected by scary, violent or sexualised images, as well as advertising. Even having the TV on in the background interferes with children's ability to concentrate on things that are better for their development, like creative or physical play. *(Raisingchildren.net.au, 2016)*

Programs are chosen that are engaging and age appropriate to children: Only 'G' rated television programs and movies will be viewed at the Service

The use of TV and watching DVD's will be kept to a minimum.

Computers

Children and teenagers can use computers to develop their ideas through words, images or music. They can use the internet to explore and find information and to be creative. But children need to learn how to ask questions about the information they find on the internet. Supervise on how long your children spend on the computer.

(Raisingchildren.net.au, 2016)

CLOSED-CIRCUIT TELEVISION SYSTEM (CCTV)

Our services do not have CCTV technology installed. We prioritise privacy, trust, and the importance of human interaction in our approach to child safety. We have put in place several robust measures to ensure the protection and well-being of all children.

The service has measures in place to ensure child safety and well-being in the absence of CCTV. This includes:

- **Child Safe Standards:** Implementation of Child Safe and Protection Policy and Procedures.
- **Staffing levels:** Adequate staffing ratios and trained personnel.
- **Supervision practices:** Clear guidelines on supervision.
- **Reporting procedures:** Well-defined processes for reporting any concerns about child safety and procedures for addressing these concerns promptly.
- **Parent communication:** Regular communication with parents about daily activities and any incidents or concerns.
- **Regular risk assessments:** Ongoing assessment of the environment and activities to identify and mitigate potential risks.
- **Training and professional development:** Ensuring staff are well-trained in child protection, first aid, and emergency procedures.
- **Open communication with families:** Maintaining open communication with parents about the service's practices and any concerns.
- **Privacy Concerns:** The installation of CCTV may raise issues regarding the privacy of children, families, and staff. We believe in fostering an environment of trust where children feel safe and secure.
- **Environmental layout:** Our service has open plan spaces with large viewing windows surrounding our environments.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 16/4/2026

Reference:

NQS

QA2	2.2	<i>Each child is protected.</i>
QA7	7.1.2	<i>Systems are in place to manage risk and enable the effective management and operation of a quality service.</i>

EYLF

LO5	<i>Children use information and communication technology to access information, investigate ideas and represent their thinking.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Reg	73	Education Program
Reg	77	Health, hygiene and safe food practices
Reg	168	Education and care service must have policies and procedures
Reg	168(2)(h)	Providing a child safe environment, including matters relating to the promotion of a culture of child safety and wellbeing within the service
Reg	168(2)(ha)	The safe use of digital technologies and online environments at the service.
Reg	168(2)(i)	Staffing

Other references

www.health.gov.au Move and Play EveryDa, National Physical Activity Recommends for Children 0-5 Years

<http://www.sahealth.sa.gov.au> Opal Give The Screen a Rest. Active Play is the Best.

http://raisingchildren.net.au/articles/screen_time.html

Fact Source

- Australia's Physical Activity Recommendations recommend that 5-18 y.o accumulate no more than 2 hours of screen time a day for entertainment (excluding educational purposes) (*Department of Health and Ageing (2004a) Australia's Physical Activity Recommendations for 5-12 year olds. Commonwealth of Australia.*)
- Guidelines for children under five have also been released and recommend children younger than 2 years do not spend anytime viewing TV or other electronic media and for children 2-5 years less than 1 hour per day. (*Department of Health and Ageing (2009). Get Up and Grow – healthy eating and physical activity for early childhood. Family book.*)

Appendix: Move and Play Every Day Brochure

What about TV and computer games?

TV, DVDs and computer games may be popular with kids, but they usually involve sitting for long periods. Children will benefit more from talking, singing, reading, listening to music or playing with a family member or a friend.

Regardless of how active kids may be at other times, it is still important to limit screen time.

RECOMMENDATIONS

Children aged 2 to 5 years

For children 2 to 5 years of age, sitting and watching television and the use of other electronic media (DVDs, computer and other electronic games) should be limited to less than one hour per day.

Children aged less than 2 years

Children younger than 2 years of age should not spend any time watching television or using other electronic media (DVDs, computer and other electronic games).

A variety of TV programs have been created for children under 2 years, however it is questionable whether TV enhances development in the first two years of life. For more information on the evidence in this area you may wish to visit www.health.gov.au. For healthy development, kids need regular activity and plenty of interaction with others.

TIPS AND IDEAS

Reducing screen time for all children

- Make meal time your family time and turn off the TV.
- Turn the TV off after the program has finished.
- Set limits and have rules around screen time.
- Make your kids' bedrooms TV and computer free.
- Play music or stories on CDs instead.

What about time spent sitting or being inactive?

RECOMMENDATION

All children (Birth to 5 years)

Infants, toddlers and pre-schoolers should not be sedentary, restrained, or kept inactive, for more than one hour at a time, with the exception of sleeping.

'Sedentary behaviour' refers to time spent being physically inactive. All children need some 'down time' but they are not naturally inactive for long periods of time.

Sitting in strollers, highchairs and car seats (being restrained) for long periods isn't good for children's health and development and goes against their natural tendency to be active and play.

The use of baby jumpers and baby walkers is also discouraged. The evidence shows they can restrict the muscle development required for independent walking and may cause injuries.

TIPS AND IDEAS

All children

- Take breaks on long car trips – stop at a park or rest area.
- Give kids a break from the stroller and let them walk for some of the journey.
- Try walking, pedalling or using a scooter for short trips.

What can I do?

Get involved! Remember you are the most important role model in your child's life. By being active with your child you can encourage their lifelong enjoyment of physical activity – and benefit your own health at the same time.

Don't forget that along with lots of play and activity, kids need a variety of good foods for healthy growth and development. For more information on healthy eating for kids see the nutrition resources outlined below.

Healthy living resources for all ages

- Get Up & Go! – Healthy Eating and Physical Activity for Early Childhood (for centre based care, family day care and pre-school)
- Australia's Physical Activity and Sedentary Behaviour Guidelines. Make your move – Sit less – Be active for life!
 - Children (2-12 years)
 - Young People (13-17 years)
 - Adults (18-64 years)
 - Families
- Choose Health Be Active – a physical activity guide for older Australians
- Eat for Health – www.eatforhealth.gov.au

To order resources, free of charge, ph 1800 020 103.

FOR MORE INFORMATION, VISIT www.health.gov.au

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**Australian Government
Department of Health**

Move and Play Every Day

NATIONAL PHYSICAL ACTIVITY RECOMMENDATIONS FOR CHILDREN 0-5 YEARS

These recommendations are for children who haven't started school yet. If your child has started school, please refer to Australia's Physical Activity and Sedentary Behaviour Guidelines for Children (5-12 years).

Children love to play and be active!

Being physically active every day is important for the healthy growth and development of infants, toddlers and pre-schoolers.

Physical activity for children includes both structured activities and unstructured free play, and can be done indoors or out.

THE BENEFITS FOR YOUR CHILD

Being physically active every day is fun and can:

- Help achieve and maintain a healthy weight.
- Build strong bones and muscles.
- Improve balance, movement and co-ordination skills.
- Promote social skills through interactions with people.
- Support brain development.
- Encourage self-confidence and independence.

Whatever their level of ability, children need to be active.

The National Physical Activity Recommendations provide you with support and guidance in relation to your child's physical activity, play, sedentary behaviour (sitting) and television viewing. While meeting these recommendations may seem like a challenge at times, tips and ideas have been provided to help you include more activity in your child's day.

What physical activity does my infant need?

RECOMMENDATION

Infants (Birth to 1 year)

For healthy development in infants, physical activity – particularly supervised floor-based play in safe environments – should be encouraged from birth.

Before infants begin to crawl, physical activity includes reaching and grasping, pulling and pushing, moving their head, body and limbs during daily routines, and supervised floor play, including tummy time.

Once your infant is mobile, encourage them to be as active as possible in a safe, supervised and nurturing play environment.

TIPS AND IDEAS

Infants (Birth to 1 year)

- Encourage them to reach and grasp by placing toys just out of reach.
- Play push and pull games with balls and soft toys.
- Play music to encourage playful movements.
- Encourage movement and play during bath time.

How much physical activity does my toddler or pre-schooler need?

RECOMMENDATION

Toddlers (1 to 3 years) & Pre-schoolers (3 to 5 years)

Toddlers and pre-schoolers should be physically active every day for at least three hours, spread throughout the day.

You'd be surprised by how many opportunities there are throughout the day for toddlers and pre-schoolers to be active. Remember that kids don't need to do their three hours of physical activity all at once. It can be accumulated throughout the day and can include light activity like standing up, moving around and playing.

TIPS AND IDEAS

Toddlers (1 to 3 years) & Pre-schoolers (3 to 5 years)

- Play with different sized balls.
- Twirl and run with streamers.
- Play with balloons – punch, kick or throw them to keep them off the ground.
- Blow bubbles and chase them through the air.
- Play games – try hide-and-seek, obstacle courses, follow the leader, stuck in the mud, or tip/tug games.
- Move to music or try action songs like 'Ring-A-Ring-A-Rose' or 'Head-Shoulders-Knees and Toes'.
- Walk barefoot on different surfaces – try grass, carpet, concrete or sand.
- Pretend to move like different animals.
- Play dress ups and act out different roles.
- Walk to places rather than driving or using the stroller.

Tips for active play!

- Choose 'active' toys. For infants, choose boxes, pots, pans, streamers, hoops and toys that encourage reaching, stretching, crawling and moving. Play materials don't need to be expensive and can be found around the house.
- For toddlers and pre-schoolers, choose toys and play materials that encourage movement and help develop skills like running, kicking, throwing and catching, such as balls, bats, tricycles and kites.
- When you can, involve all of the family – try walking to the park, a visit to the zoo for a special treat, or playing soccer in the backyard.
- Being outdoors is best – just make sure kids have sun protection, such as sunscreen, hats and shade. If the weather is no good, head indoors and build cubby houses or play hide and seek.
- Encourage children to be independent and to explore the world around them. Allow them the freedom to create, imagine and direct their own play while maintaining a safe environment. This will help your child's confidence grow!
 - Competitive sport is not recommended for children under 5 years. Some great alternatives include structured activities like water familiarisation, recreational gymnastics and dance taught by qualified instructors.

Active play is the best way for kids to be physically active.

For more tips, visit www.health.gov.au

97 – OWNA APP ROLLS SYSTEM POLICY

Position Statement

The OWNA App rolls system enhances accuracy and efficiency of the service attendance management system.

Related Policies

Enrolment Policy – 1

Supervision Policy – 62 (This policy include the use of OWNA App and Child Wellbeing Schedule to manage children attendance)

Record Keeping and Retention Policy - 84

Objective

The service will use of information technology devices such as tablets and computer to have an electronic roll system used by parents to sign children in and out of the service. The OWNA enrolment system will allow educators and managers to more accuracy and efficiently the roll management process.

Implementation

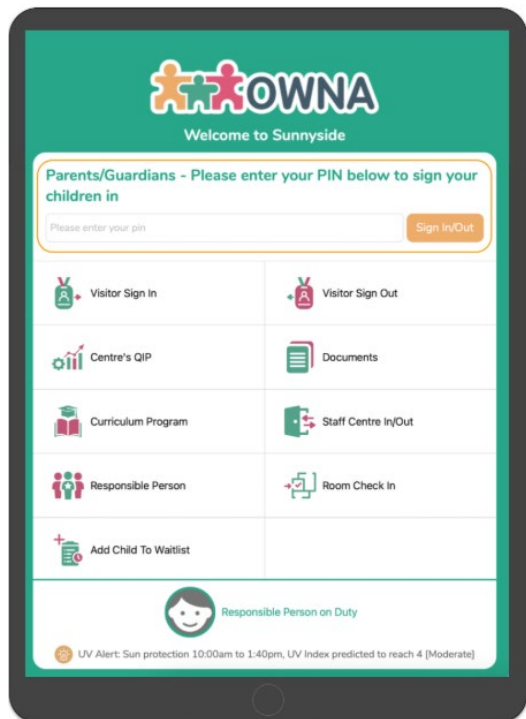
1. PARENT & AUTHORISED NOMINEES (AUTHORISED PEOPLE TO DROPOFF/COLLECT CHILDREN) OWNA APP (PUBLIC MODE)

Throughout the service there will be tablet kiosks set-up for use by parents and authorised nominees to sign in (on arrive) and sign out (on departure) from the service. The signing in and out of children by parent or authorised nominee is a legal requirement under Regulations and Act. The following is a procedure for the parent and authorised nominee to follow to sign in and out a child/ren:

How do I log in?

Step 1 - Enter your PIN

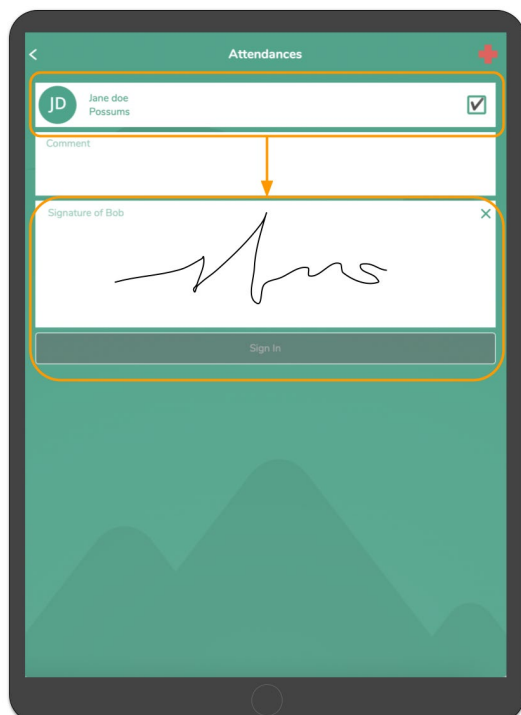
- *Enter your unique PIN by pressing on the field and then use the device's on-screen keypad*
- *Press Sign In/Out to proceed to the next screen*



Step 2 - Sign Children In/Out

- Select the child(ren) you wish to sign in/out using the checkbox next to their name(s)
 - Enter any comments
 - Some Services may also have fields to enter an estimated pickup time, sunscreen application or medication details
- Sign your name
- Click Sign In/Out

If you have forgotten to sign your child out previously, you may be prompted to confirm this first before you proceed. This needs to be signed for each child.



If the OWNA App Kiosk (touchscreen) doesn't accept your phone number or forgotten pin, please see an Educator. We can quickly update / reset pin, then you'll be able to log in. Alternatively, you may be instructed to complete the paper sign in/out form as an interim record of attendance for the day. This information be confirmed digitally when you sign in/out using the OWNA App next time.

 Don't know your PIN?

- Check your OWNA welcome email
- Ask staff for assistance
- Reset your PIN via the App (Tap on the Settings cog > Reset PIN)

2. EDUCATOR / STAFF OWNA APP

This section will show how a staff member manages child sign in/out within the kiosk.

Staff screens – OWNA App

Steps

 Actions > Attendances

1. Tap/Press on the name of the child you wish to sign in or out
2. Click OK to confirm

Attendance Status:

☒ **Not signed in**

☐ **Signed In**

☐ **Signed Out**

☐ **Not Attending**

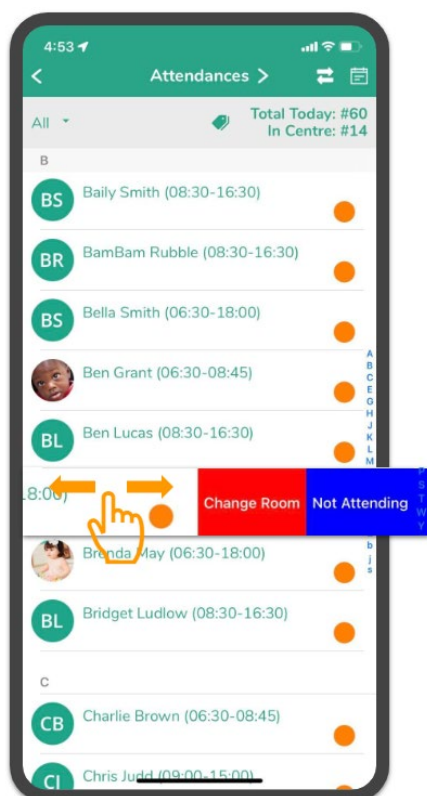
Other Functions

Change Rooms

1. Slide the child's name across
2. Select Change Room
3. Pick a new room for them to move to
4. Press Update

Mark Not Attending

1. Slide the child's name across
2. Select Not Attending
3. Press Yes to confirm



“Other” Room List

For services using “Other” Room Lists, the staff member has the ability to sign children on the “Other” room. The “other” room may be used during excursions when only part of the children are attending.

The staff member can then select and mark children appropriately to the “Other” room list..

KEY PROCEDURES FOR OWNA APP

Room OWNA APP Rolls & Child Wellbeing Schedule

Team members will check room rolls (Using OWNA App) to ascertain which children are in attendance each day and cross checked to the Child Wellbeing Schedule. Using the Child Wellbeing Schedule the columns Sign in time, Sign out time and Educator Initial to confirm cross check to OWNA App is required to be completed for each children as they arrive and depart from the service. The Child Wellbeing Schedule is the schedule that will be used in an Emergency Evacuation to complete headcounts of children present at the service.

Please note if child has been absence this will also be indicated on Child Wellbeing Schedule as “ABSENT” (did not attend the service today) and educator to initial that they have marked the OWNA App roll as ‘ABSENT’. The educator will write “Absent” on the Child Wellbeing Schedule and write their initial as indication of completion of this task and check performed.

Family Grouping Times

When children are grouped together at the beginning and end of the day team members will have all rooms Child Wellbeing Schedule and access to all room rolls via OWNA App for all children present. The rolls will be marked as children arrive and depart by parents using the OWNA App. Educators will check periodically (approximately each 30 minutes during peak children attendance and departure times. I.e. From 8.00am to 9.30am and 3.30pm to 5.30pm. Outside these times each hour) that the parent has signed in and out their child/ren. Also the educators will add the child sign in and out time onto the Child Wellbeing Schedule.

Team members will know how many children are present, their relevant age groups, the required staff – to – child ratios and consistently apply these ratios at regular intervals as child numbers change.

As the groups separate in the morning to go to their rooms, team members will check all children marked on rolls are accompanying them and take the rolls with them to the room. In the afternoon, room rolls will be handed to the late team members for marking as children depart.

Child Electronic Sign In/Out System Sheets – OWNA App

Sign in/out records are a legal requirement and will be maintained as per procedures and regulations. Early and late rostered team members will ensure that parents meet their obligations and sign children in and out using the OWNA App. If parent forgets to sign in or out their child/ren the Qualified Educator will be required to sign in or out the child/ren. Please note the parent will be required to confirm the sign in and out of their children next time they attend the service. Late team members will follow lock up procedure to ensure that all children have been picked up and duly signed out.

Preparation for closing

Team members will be aware of children remaining at the centre towards the end of the day. Children will be supervised and have engaging activities provided for them for the entire time that they are at the centre. Team members will maintain direct supervision of all children remaining as they work through closing procedures at the end of the day.

OWNA App Rolls: Closing educator (qualified) will perform the following duties:

- Review “Children Daily Routine Record” or Books (for nursery rooms) – Sign In and Sign Out Time for each child. Each child should have a sign in time, out time and a educator initial (to indicator that the educator has check child has go and also child is signed out in OWNA App roll). Please note if child has been absence this will also be indicated on Child Wellbeing Schedule as “ABSENT” (did not attend the service today) and educator to initial that they have record the ‘ABSENCE” in the OWNA App roll.
- Before closing service all children will be accounted for on Child Wellbeing Schedule and also in the OWNA App Rolls for all rooms.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 15/4/2025

Reference:

NQS

QA7	7.1.2	Systems are in place to manage risk and enable the effective management and operation of a quality service.

National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations, Regulation 73 Educational program

Other references
OWNA App Help

98 – CORONAVIRUS POLICY

Position Statement

To ensure all employees and families implement appropriate risk management procedures to prevent the spread of coronavirus or reduce the potential for the illness to spread.

Related Policies

Excursion Policy
Fees Policy
Nutrition, Food, Beverage & Dietary Requirements Policy
Health & Hygiene Policy
Health and Wellbeing Policy
Illness Policy
Incident, Injury, Trauma and Illness Policy
Infectious Diseases and Immunisation Policy
Medical Conditions Policy
Interactions with Children Policy
Rest Time/Sleep Policy
Occupational Health & Safety Policy

IMPORTANT – WHEN TO IMPLEMENT SERVICE CORONAVIRUS POLICY

This policy and its associated procedures will be implemented when a health directive relating to Coronavirus is issued by the relevant Government Authority.

At all other times, the *Staying Healthy in Childcare* guide will be followed for the management of infectious diseases, including coronavirus.

Implementation

Coronavirus (COVID-19) can cause illnesses similar to the common cold, but it can also cause more serious respiratory diseases. Most people displaying symptoms such as fever, cough, sore throat, tiredness or shortness of breath are likely suffering with a cold or other respiratory illness—not coronavirus. People at higher risk of catching the virus include older people, those with underlying medical problems and Indigenous Australians. The threats posed by the virus mean serious steps must be taken to stop the spread of the disease.

As outlined in our Infectious Disease and Health, Hygiene and Safe Food Policies, the Approved Provider, Nominated Supervisor, educators and staff implement strict hygiene and infection control procedures at all times to prevent or minimise the spread of contagious illnesses. Hygiene measures and exclusion principles outlined in these policies continue to apply, and will be informed by current guidance on coronavirus issued by the State and Federal Government including:

- [Guidance for Managing a COVID case in ECEC and COVIDSafe settings guidance for ECEC VIC](#)
- [Federal Department of Education, Skills and Employment Information](#)
- [Federal Department of Health information](#)

During times of Government advice, the Approved Provider and Nominated Supervisor will also implement a COVID Safe Plan to ensure the service can provide an environment that's as safe as possible for children, staff and visitors.

What must employees and families do?

Comply with government guidance (required at the time)

The Approved Provider, employees and volunteers and families must:

- **comply with guidance issued by Government agencies, including in relation to attendance and self-isolation.** This includes following the 'Checklist for COVID contacts.' The State Government also advises any staff member or child who is experiencing COVID-19 symptoms, needs to stay at home, even if they are not a positive case or a close contact, get tested and self-isolate until they receive a negative result - unless those symptoms are known to be caused by an underlying health condition or medication. This would also apply to family members
- **comply with all service policies including Infectious Diseases and Immunisation Policy** which requires ill children and adults to remain at home and comply with relevant Exclusion periods. Note employees, volunteers and families must comply with any isolation/exclusion periods in relation to coronavirus implemented by the Approved Provider or Nominated Supervisor including periods which exceed government requirements
- **advise the service** if they develop symptoms of the virus or are confirmed to have the virus. This is particularly important if they have been at the service before a positive test
- **comply with guidance issued by the Government in relation to the wearing of face masks at the Service and on transport, including buses operated by the Service.** **Where the wearing of masks is not mandatory**, it is recommended adult visitors, including families, wear masks when visiting the Service, and staff are requested to wear masks when engaging with other adults, such as during pick-up and drop-off, in administrative areas and staff rooms, and indoors where physical distancing cannot be maintained. Note it is not safe to use a mask on children under two years due to the risk they may choke.

The Approved Provider and Nominated Supervisor will ensure staff, parents and visitors are familiar with best practice infection control measures for wearing masks including:

- washing hands for at least 20 seconds with soap and water or alcohol-based hand sanitiser with at least 60% alcohol before touching mask
- not touching the front of the mask or the face while it's on or when removing it – grasping the ear loops or ties instead
- disposing of single use surgical masks responsibly and putting reusable masks directly into the laundry or a disposable/washable bag for laundering.
- **comply with any Government requirements for vaccination against COVID-19 (as required by law).** This includes vaccination requirements for ECEC staff, volunteers and contractors who attend the Service as outlined on State Government websites and contained in Public Health Directions. Where vaccination is mandated, the Nominated Supervisor will sight evidence of the person's vaccination status and keep a written record of this
- **comply with any Government check-in protocols**
- **comply with the Service COVID Safe Plan (required at the time).** To promote the safety of children, employees, volunteers and families, the Approved Provider or Nominated Supervisor may require staff, volunteers and families to follow stricter measures than those currently required or advised by Government Orders or protocols. This includes vaccination or other requirements for working at or entering the Service. Such as:
 - employees, volunteers and family members providing evidence of COVID vaccination, or medical exemption from vaccination

- when requested by the Nominated Supervisor - children, family members and employees providing evidence of a negative COVID test before entering the service after a household contact tests positive for COVID
- employees/families/visitors/children having their temperature tested before entry to the service
- families/visitors completing a Health Declaration if requested by staff declaring they are healthy and do not have any symptoms of COVID before entering the service.

Implement effective hygiene process

The coronavirus is most likely to spread from person-to-person through droplets of saliva produced when a person coughs or sneezes. Droplets cannot go through skin and people can only be infected if they touch their mouth, nose or eyes once their skin (ie hands) is contaminated. Droplets usually travel no farther than 1 metre through the air. This means the transmission of droplets can occur when people:

- have direct close contact with a person while they are infectious
- have close contact with an infected person who coughs or sneezes
- touch objects or surfaces like door handles or tables contaminated from a cough or sneeze from a person with a confirmed infection, and then touch their mouth or face.

Employees and volunteers will ensure they continue to implement hygiene processes outlined in the Health, Hygiene and Safe Food Policy to ensure high standards of hygiene and infection control at all times. This includes ensuring they and where relevant children:

- wash hands frequently with soap and water including before and after eating or handling food, going to the toilet, changing a nappy, handling play dough, using gloves, after wiping or touching nose and cleaning up spills of body fluids, and where they board transport, including buses operated by the Service
- wash hands in ways that meet the principles recommended by the World Health Organisation in the following videos [wash hands with soap and water](#) and [wash hands with alcohol based sanitiser](#)
- cough and sneeze into their inner elbow, or use a tissue to cover their mouth and nose and placing tissues in the bin immediately after use.

The Approved Provider, Nominated Supervisor and Responsible Person in Charge will implement and ensure the following practices to ensure high standards of hygiene and infection control:

- ensure staff complete the online [COVID-19 Infection Control Training](#) made available by the Federal Department of Health.
- ensure educators engage in regular handwashing with children
- ensure hand hygiene posters are displayed in the kitchen, learning spaces, hand wash and toilet areas, staff room and any other areas which can easily be seen by families, including the front entrance, and require all employees and families to use hand sanitiser provided at service entrances and in rooms. If using alcohol-based hand sanitiser in place of soap it will contain 60-80% alcohol.
- place signs and posters about physical distancing around the Service like those from [Safework Australia](#).
- ensure educators or cleaning staff implement appropriate cleaning practices which are implemented and documented more frequently than before the pandemic. This includes regularly cleaning and disinfecting frequently touched surfaces like door knobs, bathrooms (eg taps, toilets), tables and chairs, phones, tablets, keyboards, playground equipment, children's bags and bottles, and transport operated by the service eg after collecting or dropping off children. Cleaning staff, including contracted cleaning staff, will implement appropriate COVID cleaning procedures like those outlined in the [Information](#)

about routine cleaning and disinfection in the community Information Sheet or VIC Health COVID Cleaning Guidelines. They will, for example:

- wear gloves and use alcohol-based hand sanitiser (and other COVID Approved Sanitizers) before and after wearing gloves
- wear masks if cleaning area where COVID positive person has been or there are spills of body fluids which could be infectious
- disinfect surfaces with an anti-viral disinfectant (made to strength recommended by manufacturer) after cleaning with detergent and water
- ensure all bathrooms are always well stocked with hand soap and towels, and they are sufficient quantities of essential items eg gloves, masks, wipes, sanitiser, disinfectant, soap, detergent and hand towels
- provide hands free sealed bins in bathrooms/rooms and ensure they're emptied daily and when full
- provide detergent/disinfectant surface wipes to clean equipment such as monitor, phone, keyboard and mouse
- replace high touch communal items with alternatives where possible
- open windows to circulate fresh air as much as possible and adjust air-conditioning from recycle to fresh air(unless air quality outside is poor).

Body Temperature Testing Procedure (When directed by Government Authority)

Each child who enters our service will have their body temperature measured, using a non-contact infra-red thermometer. Screening should take place at drop-off time, while the parent/carer is still present.

Any child who records a temperature of **37.5 Degrees Celsius or over** will be not enter their room and refused entry to the service. They will need to seek medical advice. A doctor medical clearance needs to be obtained to return to the service. Parent to provide result of tests performed.

Testing Frequency: on arrival each day of care and as required throughout the day.

General Guidelines – measure surface body temperature and use • Make sure all adults are appropriately physically distanced, 1.5m from each other. • Subject to clean, dry forehead • Hold the device out at arm's length. • Align device 3-5cm from the forehead. • Press trigger to complete temperature check. • If temperature reads 37.5 Degrees Celsius or higher the tester will reperform the test after a 15 minute period with a second device. During this time, the families and child will exit the service to wait to be retested.	PPE must be worn by the person carrying out the test Face mask Face Shield Gloves (if in contact with child or parent) when face masks are required to be worn per guidelines or child has covid symptoms. Ear Thermometer: use of disposable covers will be used for each person. Sanitising of device is required after each use. It is important to be mindful of the location of testing to be performed. For second and third tests it is recommended that these be performed in a	Symptoms of COVID-19 People are not permitted to enter the service with a fever or other COVID-19 related symptoms. Ask parent if child has been provided any medication such as paracetamol; this will affect the temperature of child. • Fever • respiratory symptoms ○ coughing ○ sore throat ○ shortness of breath Other symptoms can include runny nose, headache, muscle or joint pains, nausea, diarrhoea, vomiting, loss of sense of smell, altered sense
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- After the second test is performed use an ear thermometer or oral thermometer to perform a third device temperature test. - If two tests are 37.5 Degrees Celsius or above then the child needs to be sent home immediately and parent needs to provide a doctors clearance to return to care. - It is recommended to wait 15 minutes before measure ear temperature if your child has been outside on a cold day. - If the child is wearing outerwear, the educator should suggest the child remove this once they are indoors. - If high temperature or ill complete Illness form to record information.	different location then room entrance or foyer. Once second and third test performed sanitizing of touch points should be undertaken. Wash hand with soap (20 seconds) or sanitise hands until absorbed. Sanitise clothing with Glen 20. Contact with people with fever and illness should be aimed to be less than 15 minutes in direct contact or 2 hours in confined space or room.	of taste, loss of appetite and fatigue. To stop the spread of COVID-19 people with even mild symptoms of <u>respiratory infection should get tested.</u> (Source: Health.gov.au)
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- teachers and educators who are conducting temperature checks can wear face masks (if service is location of mask wearing requirement or a location of increased risk of COVID-19).
- teacher and educators who are attending to sick or ill children should wear face masks (this apply to all locations.)

Types of Thermometers

 Digital: Quick to use, accurate and suitable for oral, armpit or anal readings. Always use a digital thermometer under the armpit with children younger than 5.	 Ear (tympanic): Placed in the child's ear canal, ear thermometers are quick to use but must be carefully positioned for an accurate reading.	 Touchless (forehead) thermometer: Inaccurate if not placed correctly. This measures the child's temperature using an infrared reading of an artery in the forehead. The thermometer does not touch the skin.
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Face Masks

Please refer to government website for current face mask requirements.

The following face mask guidelines apply when face mask health orders apply:

- It is not compulsory for teachers and educators to wear face masks while teaching or caring for children as this can interfere with the ability to clearly communicate. (Teachers and educators may choose to wear masks while teaching.)
- It is recommended in high risk periods of COVID, teachers and educators who are conducting temperature checks should wear face masks (if service is location of mask wearing requirement or an increased risk of COVID-19).

- teacher and educators who are attending to sick or ill children should wear face masks (this apply to all locations.)
- teachers and educators wear masks when they're not teaching or providing care eg in the staff room (where not consuming food and drink) or during interviews with parents an (at times of increased risk of COVID-19 or mandated requirement).
- non-teaching staff, including office staff, wear face masks while working unless they work alone in an enclosed space (eg office). In this case they must wear a mask if someone enters the space and when they leave it. (at times of increased risk of COVID-19 or mandated requirement).
- parents and carers wear face masks whenever they're in or near the service, including when they deliver and collect their children from the service. (at times of increased risk of COVID-19 or mandated requirement).
- they and all staff implement the following hygiene measures when putting on or removing a face mask:
 - wash hands first for at least 20 seconds with soap and water or alcohol-based hand sanitiser with at least 60% alcohol
 - don't touch the front of the mask or the face while it's on or when removing it

Putting on a mask

- carefully grasp the ear loops or ties. For masks with a pair of ties, fasten the top one first, then the bottom one

Removing a mask

- carefully remove the mask by grasping the ear loops or untying the ties. For masks with a pair of ties, unfasten the bottom one first, then the top one
- if the mask has filters, remove and discard them. Fold the mask and put it directly into the laundry or a disposable/washable bag for laundering. Dispose of single use surgical masks responsibly.

(Source: <https://www.business.vic.gov.au/disputes-disasters-and-succession-planning/coronavirus-covid-19/face-coverings-in-the-workplace>)

Social distancing (When directed by Government Authority)

The Approved Provider or Nominated Supervisor will also implement the following social distancing strategies where possible to limit the potential spread of COVID:

- complying with current capacity limits, if any, and displaying conditions of entry for all families and visitors on social media and entry points
- complying with State Government guidance in relation, for example, to physical distancing and visitors
- ensuring adults, including families, maintain at least 1.5 metres between each other as far as practical, including at the start and end of the program
- avoiding non-essential activities that involve close personal contact
- restricting number of parents in service dropping off or picking up children
- reviewing location of furniture and equipment in children's rooms to promote physical distancing where possible
- keeping educators and children in same rooms where possible to prevent mixing of children and staff
- avoiding situations where children are required to queue, assemble in large groups or hold hands
- staggering lunch /snack times to reduce number of children inside at one time and number of staff in staff room
- reviewing and adjusting work rosters where possible to reduce numbers of staff starting and finishing together or taking simultaneous breaks (while meeting ratio and supervision requirements)

- arranging for deliveries to be dropped away from main entrance or collected from vehicles by one or two staff using contactless acceptance measures, displaying signage for delivery drivers and identifying designated drop off areas away from main entrance
- increasing the use of technology like Skype and Zoom to communicate with families, including during orientation processes, and ensure children can continue to communicate with community members in a protected environment
- increasing supervision in bathrooms
- conducting more learning and activities outside
- ensuring physical distancing is maintained where possible on buses operated by the service
- ensuring where staff need to travel together in same vehicle:
 - passengers and drivers spread out using front and back seats
 - staff only handle their own bags
 - driver cleans vehicle hand touch areas at the end of each journey with detergent/disinfectant
 - air-conditioning set to external airflow rather than recirculation (unless air quality outside is poor).

Information and notification requirements

The Approved Provider or Nominated Supervisor will:

- follow VIC Government Guidance in relation to managing COVID suspected and confirmed positive cases of COVID-19 in ECEC services – and follow the ‘ECEC COVID-19 Response Guidelines – exposure management pack’ which can be downloaded from the site
- comply with notification requirements for serious incidents which include:
 - any incident involving serious illness of a child at the service where the child attended, or should have attended, a hospital
 - any emergency where emergency services attended ie there was an imminent or severe risk to the health, safety or wellbeing of a person at the service
- comply with other notification requirements including notifying:
 - the Regulatory Authority within 7 days about any changes to service days or operating hours
 - the Regulatory Authority within 24 hours if Service is directed to close or closing voluntarily because of COVID – and again when Service reopens. (If closing voluntarily, children cannot be reported as absent and CCS will not be paid unless the closure is determined as a local emergency by VIC Education)
 - the Regulatory Authority within 24 hours if reducing the number of children attending
 - any third party software provider or via operational details in the Provider Entry Point of closures/re-openings or other relevant changes
 - the Regulatory Authority as soon as possible if they’re receiving Kindergarten funding and an ECT is absent and another ECT is not available to deliver the Program
 - WorkSafe VIC if an employee is hospitalised and/or dies as a result of contracting COVID-19 at work
- apply for waivers from ratio and qualification requirements if required where staff are required to self-isolate.

Interactions with Children

Where appropriate, educators will speak with children about the coronavirus in ways that do not alarm them or cause unnecessary fear or distress. Educators may, for example, discuss with children their feelings in relation to the virus, remind children that the risk of catching the illness is very low, review hygiene measures they can take to reduce the risk of infection, discuss some of the good things happening in the world, or implement other strategies.

Educators will be careful not to speak to others in an alarmist way about the coronavirus if children are present or within hearing.

Routine and Experience Changes (When directed by Government Authority)

Practices we are doing to keep children and educators safe within our centres include:

- Option 1: Dropping off and collecting your child from the foyer and we will escort your child to their rooms. This procedure has been recommended by Health Experts as best practice in relation to limiting the germs in the children's environment.

OR

- Option 2: Dropping off and collecting your child at the entrance of the classroom door and not entering the room. This procedure has been recommended by Health Experts as best practice in relation to limiting the germs in the children's environment
- Reduce the number of people entering the service and therefore, during this time of COVID 19, we ask that only one adult drop-off or pick-up their child (if possible, no sibling's should enter the service and if required should remain in the foyer).
- During periods of restrictions advised by Government Health Department guidelines: All non-essential visitors are requested not to attend the centre. Stop excursions and are enforcing social distancing by adults in line with Government guidelines. We're using our extensive educational expertise to implement social distancing practices for children as well.
- Our centres have very well practised infection control procedures and our teams have the training to ensure good hygiene is occurring.
- Our centres are cleaned daily with hospital grade cleaning products.
- We are strictly enforcing rigorous exclusion policies for children and educators in all our centres in line with advice from the public health authorities.
- If children are sick, please keep them home. If educators are sick, we will send them home.

The following will be completed in relation to the experiences and routines:

- **Child drop-off/pickups:** restrict the number of parents in your foyer or reception area.
- **Hand sanitising on entering the service:** Put hand sanitiser or soap at the reception desk and make it a requirement of every person (parent, child or team member) entering the service to clean their hands.
- **Hand washing:** Centre-wide regular handwash times for all team members (and children)
- **Wipe-downs:** three times a day (Eg. 10am / 12noon / 3pm) all door handles, reception desk, pens, keyboards, and other surfaces are wiped down and disinfected.
- Removal of soft furnishings (including rugs, pillows, cloth toys, etc)
- No using materials that increase the spread of germs; including playdough, etc.

What else should families do?

Asthma Australia has advised doctors to ensure all patients with asthma have a current Asthma Action plan and to update it if needed via a phone consultation, with any new plan delivered electronically. If their child has an Asthma Plan, families must consult their doctor and provide the Nominated Supervisor with an updated Plan or written confirmation from the doctor that the current Plan can continue. The Nominated Supervisor will distribute any updated Plans to relevant educators.

Fees

The Approved Provider or Nominated Supervisor will advise families of any relevant fee support which may be provided by the Government to assist families impacted by COVID-19. Without any Government fee support families must pay fees as outlined in our Fees Policy. Please note

families approved for Child Care Subsidy may be eligible for Additional Child Care Subsidy (temporary financial hardship) where their income has dropped because of COVID-19.

Outside Government declared changes to fees and Child Care Subsidy system, fees will be charged as normal in accordance with our Fee Policy.

Families are encouraged to remain enrolled, or to re-enrol to ensure they maintain their eligibility for CCS, and do not have to wait for CCS claims to be assessed in the future.

During lockdowns or other period of Government Health Orders, we maybe required to prioritise enrolments for children of essential workers, vulnerable and disadvantaged children and children who were previously enrolled. Within these categories we must prioritise enrolment within our service capacity and constraints as follows:

- A) Essential workers that provide emergency and essential front-line services like health care workers, police, teachers, etc.
- B) Disadvantaged and vulnerable children that would meet the requirements for the Additional Child Care Subsidy (Child Wellbeing).
- C) Children with existing enrolments who do not have a parent/carer at home and do not have any other care arrangements.
- D) Children with existing enrolments; and
- E) New enrolments for children

For essential services families who have children attending we ask for your children to be only attending for the time you are working. We are providing care for period of time of 'need'. We do not have the capacity to provide care for extended hours and are focused on providing care during core times.

During a pandemic, the service is dealing with new circumstances and having to reconsider available staff, health and other business challenges. This means the service has to make decisions on its own capacity to offer care, based on these new considerations.

We may not be able provide the days of attendance requested; a reduction in attendance days may be offered. Where we do not have the capacity to meet the care needs of priority enrolments, we may suspend the enrolment of any family we subsequently find does not meet the requirements or the service is unable to provide. During this time the service may postpone the commencement of new enrolments.

We must continue to meet the laws and regulations that govern early childhood services. These include meeting educator to child ratios. We may need to adjust our rostering and staffing arrangements to try and maintain the financial viability of our service and also be able to have sufficient educators to educate and care for children attending.

We will ensure the effectiveness of our social distancing strategies is not compromised by excessive numbers of children. The safety and wellbeing of our children, staff and families is always our primary concern, especially during these times of COVID-19. Please ensure you continue to follow all the new procedures to help us maintain high hygiene standards.

Staff Entitlements - Employees are ill or need to care for family member

Permanent employees are entitled to paid sick leave if they're ill with coronavirus. Employees must provide a medical certificate (or test result) confirming their result and medical assessment. Employees who want to stay at home as a precaution against exposure to coronavirus must apply for paid or unpaid leave.

Staff Entitlements - Employees required to self-isolate or wish to stay home as precaution

The Approved Provider or Nominated Supervisor will discuss available employment options with permanent employees who can't return from overseas or are required to enter quarantine

or isolation but aren't sick. Options include taking annual leave or other leave eg long service leave, and taking unpaid leave.

Employees who want to stay at home as a precaution against exposure to coronavirus must apply for paid or unpaid leave.

Staff Entitlements - Stand downs

Under the Fair Work Act, an employee can only be stood down without pay if:

- there's a stoppage of work
- the employees can't be usefully employed (not limited to an employee's usual work)
- the cause of the stoppage is one the employer cannot reasonably be held responsible for (eg service is directed to close by Government).

During stand down periods there is no interruption to continuity of service and leave accruals continue. The Approved Provider may seek legal advice to confirm payment of salaries is not required under the 'stand down' provision of the Fair Work Act if directed to close by the Government.

During this time employees may engage in activities which don't involve children, for example, training, deep cleaning or administration.

COVID Entitlements

Employees may be eligible for financial assistance and should check Government websites (eg [Financial and Other support for COVID-19](#), [Services Australia](#)).

Policy Reviewed: 15/12/2025

The policy will be reviewed annually by the Approved Provider, Supervisors, Employees, Families and any committee members.

References

NQS

QA2	2.1.2	Health practices and procedures Effective illness and injury management and hygiene practices are promoted and implemented.
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National Law

Section	167	Offence relating to protection of children from harm and hazards
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National Regulations

Reg	77	Health, hygiene and safe food practices
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Source

Education and Care Services National Law and Regulations

Fair Work Act 2009

Fair Work Ombudsman 'Coronavirus and Australian Workplace laws'

Federal Department of Health coronavirus information sheets

Federal Department of Education, Skills and Employment coronavirus information sheets

National Quality Standard

Work, Health and Safety Laws and Regulations

Face Coverings – 11.59pm Wednesday 22 July Health and Human Service VIC

99 – FAMILY VIOLENCE SAFETY POLICY

Objective

The safety, health and wellbeing of children is our number one priority. We have zero tolerance for child abuse and are committed to acting in children's best interests. We will ensure our environment and practices are always safe, consistent with best practice and legislative requirements including the Family Violence Information Sharing Scheme.

Related Policies

Child Safety and Protection Policy
Governance of Service, Management and Educator Policy
Incident, Injury, Trauma and Illness Policy
Record Keeping and Retention Policy

Implementation

The Royal Commission into Family Violence found that effective and appropriate information sharing is crucial in keeping family members safe. As a result the Victorian Government has implemented the Family Violence Information Sharing Scheme (the Scheme) to support the effective assessment and management of family violence risk. This Scheme and the Child Information Sharing Scheme complement each other.

Family violence includes physical or sexual violence, emotional or psychological abuse, threatening or coercive behaviour that controls or dominates a family member and makes them fearful for their safety or wellbeing or that of someone else. It also includes behaviour that causes a child to hear, witness or otherwise be exposed to this behaviour.

In this Policy we use the following terms:

- perpetrator – there is a *reasonable belief* the person may commit family violence (eg they've been identified through family violence risk assessment)
- alleged perpetrator – a person who is alleged to pose a risk of family violence
- third party – a person whose confidential information is relevant to assessing or managing family violence
- victim survivor – there's a *reasonable belief* the adult or child may be at risk of family violence.

The Approved Provider or Nominated Supervisor are responsible for ensuring compliance with the Scheme. They are protected from liability if they share information in good faith and with reasonable care. If in doubt about their obligations or Scheme requirements, they will refer to the Family Violence Information Sharing Guidelines or other resources supporting the MARAM Framework. They will never use information obtained under the Scheme to prevent a child enrolling or continuing at the service.

As our Service is an 'Information Sharing Entity (ISE) under the Scheme, the Approved Provider or Nominated Supervisor will voluntarily share information with other ISEs about a perpetrator, victim survivor or third party, or respond in a timely way to requests from ISEs to share information, if the information sharing meets the legal requirements of the Scheme. ISE's may share any personal information (including opinions) for protection purposes ie they *reasonably believe this will help manage an established risk* of a perpetrator committing family violence, or an established risk of a victim survivor being subjected to family violence. Some ISE's, including our service, are also Risk Assessment Entities (RAEs) who can voluntarily share information with, or request information from, other RAEs about a perpetrator, alleged perpetrator, victim survivor or third party, to *establish and assess the risk* of family violence.

The Approved Provider or Nominated Supervisor will confirm a person or organisation requesting information is an ISE or RAE before sharing, for example by asking for an official work email or calling an organisation's switchboard. They will also be aware of local service providers and professionals who can support children and their families. They will also ensure

information to be shared is not excluded and consent to share information is first obtained where required.

Information cannot be shared if it's excluded, or consent requirements have not been met.

Excluded information includes information that might endanger a person's life or result in physical injury, prejudice legal proceedings or police investigation, contravene a court order, or is subject to legal professional privilege.

Consent must be obtained from adults victim survivors and third parties to share their information *unless we reasonably believe sharing the information is necessary to lessen or prevent a serious threat to the person's life, health, safety or welfare*. Consent is not required from a perpetrator or alleged perpetrator. Consent is also not required to share anyone's personal information, including that of a victim survivor or third party, where a child is at risk of family violence. However, the Approved Provider or Nominated Supervisor will consider the child's views and/or those of a parent who is not a perpetrator about sharing the information *where it's safe, appropriate and reasonable to do so*.

Where consent is required, it must be given freely and the person fully informed about what information will be collected, who it might be shared with, how it might be used, the consequences of giving or not giving consent, how long their consent is valid for, that they may withdraw their consent any time, and may decline or receive services without their information being shared.

Collect and Sharing Information

If collecting or sharing the information of *victim survivors or third parties* (other than a perpetrator or alleged perpetrator), the Approved Provider or Nominated Supervisor will notify them about the information collected, who it will be shared with and why, how they can access it, who it might be further shared with, when it might be shared without their consent, and the potential outcomes of sharing the information *unless doing so would pose a serious threat to their life or health*. They will also advise the information sharing is allowed under the Scheme, and that complaints about the sharing of personal information may be made to the Victorian Information Commissioner. The Approved Provider or Nominated Supervisor may refuse perpetrators access to their confidential information if they reasonably believe doing so would increase the family violence risk.

When sharing information the Approved Provider and Nominated Supervisor will ensure:

- only information relevant to assessing or managing the risk of family violence is shared
- information shared about an alleged perpetrator, or to assess a risk of family violence, is only shared with another RAE
- information is shared in a way which protects a person's anonymity if personal details are not needed to assess or manage family violence risks
- the ISE requesting information does not have a conflict of interest eg there is no personal/family relationship with another person involved in an assessment
- sharing information will not increase the threat to a victim survivor or any other person (it will not be shared in this case)
- information is shared where possible in a way that preserves and promotes positive relationships between a child and family members
- any discussions about collecting, using or sharing information is done in an accessible, inclusive and culturally sensitive way (eg provided in home languages), and for Aboriginal people in a way that considers their family and community connections.

The Approved Provider or Nominated Supervisor may share non-excluded information about a perpetrator with a victim survivor where they believe this will help manage a risk to the victim survivor's safety. The perpetrator's consent is not required. (Information will never be shared with a perpetrator or alleged perpetrator.) However, where there is not an immediate threat to the victim survivor's safety, the Approved Provider or Nominated Supervisor may

refer them to an ISE with expertise in this area, including expertise from a culturally sensitive perspective.

Record Keeping

The Approved Provider or Nominated Supervisor will ensure appropriate records are kept securely and confidentially when information is requested and shared including where relevant:

- if the service received a request for information, who this was from, what was requested and the date of the request
- if the service disclosed information voluntarily, who the information was shared with, what was disclosed and the date disclosed
- if the service requested information, who this was made to and the date, what was requested and the date of the request
- how a disclosure was consistent with the Scheme
- reasons any information sharing request is declined (these will also be provided to the ISE)
- copies of any copies of a family violence risk assessment and/or safety plan made as a result of the information sharing
- any consent provided by an adult victim survivor or third party, or why information was shared without their consent
- the views of a parent who is not a perpetrator in relation to sharing a child victim survivor's information, or why their views were not considered, and if they were advised the information would be shared
- details of any complaint received including what it's about, date made, action taken to resolve the complaint and prevent similar complaints and time taken to resolve.

Policy Reviewed: 15 June 2024

The policy will be reviewed annually by the Approved Provider, Supervisors, Employees, Families and any committee members.

References

NQS

QA2	2.2.3	Child Protection - Management, educators and staff are aware of their roles and responsibilities to identify and respond to every child at risk of abuse or neglect.
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National Regulations

Regs	84	Awareness of child protection law
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Source

Education and Care Services National Regulations
Family Violence Protection (Information Sharing and Risk Management) Regulations 2018 (includes ISEs in Schedule 1 and RAEs in Schedule 2)
Family Violence Protection Act 2008 (includes Part 5A Family Violence Information Sharing Scheme)
Family Violence Information Sharing Guidelines: VIC Govt
National Quality Standard

100 – NATIONAL MODEL CODE POLICY

Our Statement of Commitment

The organisation has a strong commitment to child safety, wellbeing and protection through implementing and adherence to the Child Safe Standards, Information Sharing Scheme and the National Model Code guidelines.

Objective

To adopt the National Model Code to promote a child safe culture. To ensure everyone associated with the organisation takes responsibility to protect children from any type of harm through adhering to and supporting the National Model Code (the 'Code').

Definitions

Photo/Photograph/Video: any visual recording of children, either still or moving.

Audio: means a sound recording of children, including voices and other sounds made by children.

Service-issued devices: devices provided by our service for professional use, including phones, tablets, cameras, and computers.

Digital Devices: Computers, tablet, smartphones, e0-readers, educational robots, wearable technology, gaming consoles, etc.

Restricted Personal Electric Devices under the National Model Code are:

- personal devices that can take images or videos
- personal storage and file transfer media.

Examples of restricted Personal Devices:

- phones
- tablets
- digital cameras
- smart watches
- wearables, such as camera glasses
- SD cards
- USB drive
- hard drives

Related Policies

- Child Safety Standards Policy
- Child Safety, Protection and Wellbeing Policy
- Governance of Service, Management and Educator Policy
- Family Violence Safety Policy
- Privacy and Confidentiality Policy
- Record Keeping and Retention Policy
- Ethical Conduct and Code of Ethics Policy
- Technology and Digital Use Policy
- Internet, Email and Social Network Policy
- Recording Keeping and Retention Policy
- Incident, Injury, Trauma and Illness Policy

Related Documentation

- ACECQA National Model Code Guidelines

- Child Safe Standards

Policy Context

Child safety reforms arising from the Review of Child Safety Arrangements under the NQF.
Currently, the National Model Code and Guidelines are voluntary.

When used intentionally as part of implementing an educational program, electronic devices can be utilised safely and effectively when providing education and care, including to help document a child's learning. To minimise risks to children, child safe practices need to be clearly set, understood and implemented when taking images or recording videos of children.

Child Safety Risks

Documentation of children's learning is a requirement of the Education and Care Services National Regulations (regulation 74, 177(1)(a) and 178(1)(a)). Digital photos and videos have become the most common tools used to document children's learning and engagement, and many providers submit these images to families through a range of applications (often restricted for use by authorised persons such as parents and carers) to allow them to view their child's participation in early learning experiences. This type of communication can and does play a role in engaging families and carers in a child's education and care experiences.

There is a risk that persons within a service could use their own personal electronic device to take or record images or videos of children for inappropriate purposes. This risk includes taking and sharing inappropriate or even illegal images, where physical or sexual abuse is occurring to the child, or when sexualised images of the child are produced.

Managing and monitoring what images are being taken and where, or to whom they are being sent, is very difficult when educators and other staff use personal electronic devices while providing education and care.

Australia's National Principles for Child Safe Organisations collectively illustrate that a child safe organisation is one that creates a culture, adopts strategies and takes action to promote child wellbeing and prevent harm to children and young people. Principle 8, which focuses on physical and online environments, promoting safety and wellbeing, and minimising the opportunity for children and young people to be harmed, is particularly relevant to taking images and videos of children.

Implementation

The National Model Code addresses the use of service-issued electronic devices and personal electronic devices while providing children's education and care, including taking images or videos of children.

Below is guidance on how our service incorporates the National Model Code in policies, procedures and everyday practice.

National Model Code, Part 1

Only service-issued electronic devices should be used when taking images or videos of children while providing education and care. The appropriate use of service-issued electronic devices for taking, sending and storing images or videos of children should be clearly outlined in policies and procedures.

To avoid duplication, please refer to 96 – Technology & Digital Usage Policy for guidance in Service-issued devices.

- *Service-issued devices:* The Service has devices (iPads and cameras) to be used for taking images or videos of children while providing education and care.

- Identification of Service Use Devices: The service has stickers that are placed on all service-issued devices. Service logo – with the Words “Service-issued Device”. The ‘Service-issued Device’ Sticker will be placed on the device and also a second sticker on the case (if applicable).
- A ‘Service-Issued Device Register’ will be maintained by the nominated supervisor, responsible person in charge of service and approved provider of the service.
- A personal electronic device is not a service-issued device. Permission must be obtained to have or use a personal electronic device in the service (or while educating and caring for children) in accordance with the service policies.

Oversight, control and access to images and videos of children

This includes, but is not limited to, the following:

- only service-issued devices can be used for taking images or videos of children in our care.
- staff and educators are not to use personal electronic devices for taking images or recording videos of children and communicating with families and carers.
- staff are not permitted to access digital and hard copy files of images and videos of children, including the movement of these onto devices and platforms that are not approved or monitored by the service.
- images, videos and content posted online are completed through service approved applications (OWNA and Email). Post of images and videos to applications are completed using service-devices to share with a child’s family / carer.
- images, videos or content are not to be inappropriately posted online or shared through non-approved applications without consent.
- posts on Facebook (or other social media or applications) are completed with consent by the child’s parent or guardian and via a service devices only.
- service-issued electronic devices will be regularly monitored to ensure that they are being used appropriately.
- service management will monitor the use of devices at the service, including ensuring staff and educators do not use personal electronic devices for taking images or recording videos and communicating with families and carers.

Taking images or recording videos guidance is as follows:

- Who can take or record images or videos of children: educators and teachers of the service.
- Where images or videos cannot be taken: The service has off limits for taking or recording images or videos of children.
 - in bathrooms
 - nappy change facilities
 - cot rooms
 - places where another educator or other staff member cannot see the image or recording being taken. E.g. a cubby house that has no visibility.
- The purpose of recording images and videos is for the documentation of children’s learning per the requirement of the Education and Care Services National Regulations (regulation 74, 177(1)(a) and 178(1)(a)). Images and video are also taken to document child incidents and injuries.
- **Nappy change & toilet support policy and procedure.** Devices (eg iPads or another other device with a camera) are not to be used or taken into bathrooms or nappy change areas. The recording of children wellbeing information (nappy change or toileting information in OWINA) is to be recorded in a location of the service other than toilet and nappy change areas. Further, educators should be aware of the background images when taking a photo or recording a video. Procedure: after a nappy change or toilet support is performed by an educator all health and hygiene procedures are completed, then educator records the information in OWINA in a location outside the bathroom and nappy change area.
- Consent to take photos and videos is obtained by the service through the children’s enrolment form or another service form from the parent or guardian of the child.
- Personal electronic devices cannot be used to take or record images or videos of children without prior permission in accordance with the policy.

- Inappropriate images or videos are any that are not directly relevant to the child's participation in the activities or learning. Inappropriate images or videos should not be taken using personal or service-issued devices.
- Inappropriate (and potentially illegal) images or videos should not be taken and is considered a serious incident (consider mandatory reporting requirements). These include where a child is:
 - not appropriately dressed, for example, in their underwear, in a state of undress, completely undressed or with their genitalia exposed.
 - in a position that could be perceived as sexualised in nature.
 - in distress or anxious / experiencing or demonstrating distress or dysregulation.
- **Photo and Video Guidelines:** Incident and illness forms in OWNA allow for photos and videos of a child's injury, but certain rules apply:
 - Avoid taking photographs or videos in the moment when a child is upset/injured.
 - Do not capture images or recordings of a child's private body parts (E.g. genital/bottoms/breast) that are injured.
 - Do not take photos or videos of private body parts or in situations that may compromise a child's privacy (e.g., nappy rash, genitals, bottoms, chests).
 - Do not take photos of private parts, even if requested by a parent or guardian for external viewing.
 - Parental Guidelines: If parents or guardians need to see an injury of a private body part, they should come to the service to attend to their child directly.
 - These guidelines ensure the child safety, privacy, and dignity of the children in care.

Inappropriate sharing of images or videos

- It is our service policy that it is inappropriate for an image or video of a child to be shared by a staff member to platforms beyond the intended educational purpose of the image or video without written consent from the parent or guardian and the Centre Director.
- Any image or video recording of a child can become inappropriate if shared in the wrong context or for an unintended purpose. This includes if an individual transfers images to a personal account or device either directly or via the cloud. For example, to post images or videos on social media or other applications / software platforms that were not its intended purpose. Permission will be obtained from parent or guardian if posting images or videos on social media or other applications (other than OWNA).
- Parents or guardians should not post images or videos of other family's children without their knowledge and consent.

Digital devices (laptops or iPads) use by Students, Trainers and Assessors

Students performing placement at the service are subject to the same policies and procedures of employees. Please refer to the policies and procedure of the service for guidance. In summary, no personal devices are permitted to be used in the service by students.

The following outlines the appropriate use of digital devices (laptops or iPads) by trainers and assessors while conducting student observations during vocational placement in our early service.

1. Permitted Use:

- The use of a laptop or iPad during student placement visits is solely for the purpose of digitally recording observational evidence of student performance to meet assessment requirements for the Certificate III, Diploma in Early Childhood Education and Care and Early Childhood Degrees. Devices may only be used to **document student observations and assessment notes**.
- All use must be directly related to course assessment and conducted in a professional, discreet manner.

2. Prohibited Activities:

- **Photography, video recording, or audio recording of students, children, or staff is strictly prohibited.**

- The **camera on the device must remain covered at all times** while onsite of the service.
 - The device must **not** be used to access personal content, social media, or unrelated applications while onsite.
3. **Confidentiality and Data Security:**
- For student vocational placement in early childhood education and care services: All observation notes must relate to **student performance only**, with no inclusion of identifiable child information.
 - No content or notes are to be shared outside the assessment context or stored on unsecured platforms.
4. **Professional Conduct:**
- The trainer/assessor must comply with the policies of both Registered Training Organisation and the service.
 - The use of devices must not interfere with the daily operations of the early childhood service.
 - Devices must be used with respect for the privacy, confidentiality, and safety of all individuals in the service environment.

Parental consent/authorisation for taking, use, storage of images and video recordings

- We have child safe systems in place for obtaining authorisation from parents to take, use and store images and videos of children in our care (National Regulations s 168(2)(ha)(ii)).
- We provide this policy with parents at the time of their child's enrolment.
- We get written consent from parents to take, use and store photographs or videos of their child via the enrolment form.
- Parents may choose not to consent to their child being photographed or videoed in any circumstances, or they may choose to give consent for a limited number of situations.
- We notify parents in advance of any school photographers, researchers and students on practicum placements who intend to take photographs/videos of their children, and give parents the opportunity to withdraw their consent.
- We only engage professional photographers who agree to follow the service Policy and Procedure in relation photographs and video recordings.
- External/third-party professionals (e.g., NDIS providers, allied health workers) who are supporting children while they are at our service, must not take photographs or videos of children unless they have the written authorisation of the child's parent, and they are supervised by a staff member while taking the image or recording (note, third parties who are conducting child-related work at our service must comply with our policies).
- Parents may refuse to authorise our service's taking, use or storage of images or videos of their child.
- Parents may withdraw prior authorisation at any time by notifying the nominated supervisor in writing and updating their child's enrolment form.

Parent and Guardian consents/authorisations for photos and video recordings

- Parent and Guardians agree to the enrolment terms and conditions and the service policies and procedures of the service upon enrolment.
- The enrolment form contains specific consent/authorisations from parent/guardians for the photos and video recordings by which will be taken, used and stored.

Extract: Child Enrolment Form: Photo and video recording authorisations

I authorise for photographs and video recordings of my child to be taken, used and stored for service-related purposes and staff training in accordance with service policies.

Please Choose* ☐ Yes ☐ No

I authorise for my child to be included in group photos and video recordings by which these will be taken, used and stored in accordance with the service policy.

Please note that if you select 'No,' this will limit the photos, videos, and learning documents we can provide to you. In that case, you will only receive personal photos and learning documents of your child. Choosing 'No' means your child's photo will not be included in group learning documentation and posts.

Please Choose* ☐ Yes ☐ No

(If you select "No" please consult your Nominated Supervisor/Centre Manager as this will restrict the learning documentation prepared for your child.)

I authorise for photos and video recordings of my child to be taken, used and stored for student training purposes in accordance with service policy. (Photos and video footage may leave the Service for students to present to lecturer and class for viewing and marking).

Please Choose* ☐ Yes ☐ No

I authorise for photos and video footage of my/our child to be taken, used and stored on Service website, social media platforms and other promotional materials.

Please Choose* ☐ Yes ☐ No

The above authorisations/consents on the enrolment form result in the following information displaying on OWNA App and Portal.



(Red camera icon) Denotes Photos / Video should not be put on the App OR needs to be made Private at all times.



(Facebook icon) Denotes Photos/videos can be shared on Social Media (Such as Facebook, Instagram, etc)

National Model Code, Part 2

Personal electronic devices that can take images or videos (such as tablets, phones, digital cameras, smart watches) and personal storage and file transfer media (such as SD cards, USB drives, hard drives and cloud storage) should not be in the possession of any person while providing education and care and working directly with children.

Any exceptions to this should be for limited, essential purposes that are authorised in writing (or through another means if written authorisation is not reasonably practicable) by the approved provider at the service, and where that access does not impede the active supervision of children.

To avoid duplication, please refer to 96 – Technology & Digital Usage Policy for guidance in personal devices.

Restrictions on use of personal devices apply to any person who is providing education and care and working directly with children. Restrictions apply to the following people:

- teachers and educators, including casual and agency staff
- students attending the service as part of a practicum and representatives of tertiary providers who attend the service to assess students
- volunteers, including parent volunteers
- any third parties delivering programs or incursion activities to children in a service, whether paid or unpaid
- allied health and inclusion professionals (Eg (ISS or KIS) attending a service to observe, assess or work with a child at the service
- mentors or coaches attending the service to support teachers or educators working with children or providing education and care
- preschool field officers
- primary school teachers attending a service as part of a school transition program.

- If a third party professional attending the service and working directly with children (such as an allied health or inclusion professional) needs to use a device (for example, to undertake an assessment or take notes) they can use a device that is:
 - issued by their business or institution; and
 - used only for work purposes (and not personal use).
(please see section below for detailed guidance)
- Only service-issued electronic devices are to be used when taking images or videos of children while providing education and care.
- Restricted Personal Devices such as phones, tablets, digital cameras, smart watches, wearables (such as camera glasses), SD cards, USB drive, hard drives should not be in the possession of any person or used while they are providing education and care and working directly with children.
- These personal devices are able to take images, videos, recordings, transfer information and data and therefore are required not to be used or in possession while providing education and care and working directly with children. Further, smart watches are an unnecessary distraction for active supervision and engagement.
- Mobile phones are required to be turned off or kept silent mode while the team member is working and in the service to minimise noise.
- Centre Directors and/or Responsible Person in Charge of Service are permitted to use mobile phones to perform their management and responsible person in charge duties in the Service's office (and non-child environments) and while not providing education and care or working directly with children. Calls and use of mobile phones by Centre Director / Responsible Person are restricted to office and non-child environments to perform service work-related tasks.
- If you need to receive an urgent call that cannot be attended during your break or lunch; please provide the services telephone number and the service team will do their best to accommodate by taking a message or providing you time to take the call in private. If you are expecting this is needed please discuss with the Centre Director / responsible person in charge of service.
- *Employees using personal devices to Access Arrival or other Service Software:* Employee cannot use personal devices for multi-factor authentication to access and use Arrival while providing education and care and working directly with children. If not working directly with children, devices can be used. I.e in non-children areas such as the service office.

Use of personal electronic devices

- educators, other staff and volunteers are not to make or receive personal phone calls / communications on personal devices or service-issued devices when working with children as this impacts the active supervision of children.
If an educator, staff member or volunteer is expecting an important call during the day, the call should be taken using a service phone during a scheduled break time. If it is an emergency call this will be coordinated between the staff member and the Centre Director / Responsible Person to ensure the safety and supervision of children is maintained.
- any staff member needing to be contactable during work hours must direct callers to the services contact telephone.
- staff and volunteers are to store and leave their phone, personal devices and other personal belongings in a cupboard or locker. The personal devices are not to be held on their person or left in view in the classroom or storage room. Phones and personal devices may also be stored in the office in a designated container (turned off) during the day.

National Model Code, Part 3

Essential purposes for which use and / or possession of a personal electronic device may be authorised for purposes other than taking images or recording videos of children include:

- communication in an emergency situation involving a lost child, injury to child or staff member, or other serious incident, or in the case of a lockdown or evacuation of the service premises
- personal health requirements, e.g. heart or blood sugar level monitoring.
- disability, e.g. where a personal electronic device is an essential means of communication for an educator or other staff member
- family necessity, e.g. a worker with an ill or dying family member (please see service procedure for managing this scenario)
- technology failure, e.g. when a temporary outage of service-issued electronic devices has occurred
- local emergency event occurring, to receive emergency notifications through government warning systems, for example, bushfire evacuation text notification.

Unacceptable use of personal electronic devices

Staff should not use a personal electronic device to photograph children or record audio or visual images of children for any purpose, including to document children's learning. There are a list of exceptional circumstances that allow personal devices to be used.

When National Model Code Personal-Device Restrictions Do Not Apply

The restrictions on the possession of personal devices do not apply to people who are not working directly with children.

Examples include:

- parents and carers doing drop-off or pick-up.
- Victorian Regulatory Authority Authorised Officers, police and officers of other regulators, such as environmental health officers.
- third-party contractors who are attending the service but not working with children or providing education or care (for example, maintenance contractors).

Child safe policies and procedures and supervision requirements continue to apply in the above people. Parents, guardian and carers and third-party contractors are not permitted to use personal devices for calls, taking of photos or recording of videos in the in location of the service where children are present.

If a third-party contractor (such as a maintenance person) is requires to take a photo or use phone to troubleshoot a fault or repair an educator or service staff member will be present in the room or outdoor environment. Photos or video recordings will not contain any children.

Teachers, educators and other staff can access personal electronic devices while taking a scheduled break from work, such as a lunch break and when they are not providing education and care or working directly with children and in child areas.

Personal Devices - Children, Parents, Carers and Families: Calls, Photos and Video Guidelines

- Personal devices are not permitted to be used by children, parents or guardians in children areas or locations of the service. This includes rooms, outdoor environments or hallways.
- Children are not permitted to bring or use personal devices to the service.
- With permission from the Service, parents and families can take photos and videos of their child at our service, but should not take photos or videos of anyone else's child unless they have the consent of the child's parents.
- Parents and families should not share any photos or videos that include identifiable characteristics of other children, staff, visitors or families without getting the relevant consent first. This includes sharing images on social media, websites, or any other platform.

Exceptions to Personal-Device Restrictions

- The National Model Code lists the following essential purposes for which the use or possession of a personal devices may be authorised where access does not impede the active supervision of children and Service phones not available:
 - communication in an emergency situation to ensure safety (lost child, serious injury or accident)
 - emergency communication during excursions and regular outings (for example, when groups of children and educators get split up)
 - emergency communication when children are transported or travel on transport arranged by the service.
 - in the case of a lockdown or evacuation of the service premises
 - technology failure; temporary out of service-issued electronic devices.
- The Victorian Regulatory Authority does not consider routine communication during bush, beach or other nature programs to be an essential purpose for which approved providers can authorise the possession and use of personal devices.
- Permission for use of mobile phone will be obtained from Centre Director or Responsible Person in Charge of Service to use phone for excursion or special event or purpose. Personal Devices are only permitted to be used in emergency situations and should not be kept in person hand (i.e. store in bag). See below guidance.
- For those educators who can access the internet from their mobile phone or smart watches, are not to not access the internet (whether they are using the service's Wi-Fi or their personal data plans) via their mobile phones during their work hours. Staff members should use the service's computers and devices for work relating to their job role.
- Any staff member found to be using the computers inappropriately will face an enquiry by management and other relevant parties to decide a course of action based on the severity of their misconduct. Please to related policy regarding this topic.
- This policy is also incorporative of state and federal laws regarding computer usage. Should a staff member or other relevant individual use the service's computers in a way that breaks a law, the service will take the appropriate required action (eg. Reporting to the police). Furthermore, the staff member or individual will face an enquiry held by management and other relevant parties to assess whether this conduct will affect their role within the service's operations.

Authorisation Personal Device for 'Essential Purposes'

- Authorisations for the possession and use of personal devices for 'essential purposes' is required be documented in writing.
- Advance written authorisation needs to be sought for use of Personal Device for an 'essential purpose' from Approved Provider or Nominated Supervisor. This will be given only in accordance with the National Model Code guidelines and the service policies.
- *Excursions* (See 48 - Excursion Policy): Person devices (such as a mobile phone) are not to be used while on excursions, except in emergencies circumstances. Authorisations for the possession and potential use of personal devices for 'essential purposes' will be documented in the excursion risk assessment and management plan as follows. "Name of person authorised to possess and use personal device in emergency situation while on excursion: <Insert Name>."

Monitoring and review of authorisation

The service management team will monitor and review any authorised use of a personal electronic device to ensure use of the device is consistent with what is permitted, and the authorisation remains current.

National Model Code, Part 4

Approved providers and their services should have strict controls in place for the appropriate storage and retention of images and videos of children.

Access to images and videos

Images and videos should only be saved and stored on service issued devices. These images and videos are stored securely through password protections. The images and videos are kept in accordance with the service Record Keeping and Retention Policy. Please refer to this policy for detailed guidance.

In Accordance with the Australian Privacy Principles:

- our organisation takes reasonable steps to protect personal information it holds from misuse, interference and loss, and from unauthorised access, modification or disclosure. See service policy for more information.
- our organisation takes reasonable steps to destroy or de-identify personal information when no longer needed for the purpose it was collected. Service issued devices and storage drives will be destroyed to protect access to information, images and videos.

Creating a Child Safe Culture

- all staff are to maintain / create a culture of child safety and to speak up if they see personal electronic devices being used (outside the policy guidelines), or service-issued electronic devices being used inappropriately.
- The service has off limits for recording images or videos of children e.g. in bathrooms, toilets and nappy change facilities, cot rooms, a cubby or places where another educator or other staff member cannot see the image being taken.
- Consent to having a child's image taken or recorded while providing early childhood education and care is held by the service. ie enrolment form or another form of permission form.
- All staff and educators should know children's photo and video consents. Knowledge of this information determines if child photos and video can be taken and used in learning documentation and displayed in service approved applications.
- It is best practice to ask the child's permission to take a photo or video and provide them the reason why you are taking the image. This practice supports empowering consent; it shows respect for their autonomy and teaches them about consent.

Parental consent/authorisation

- We have child safe systems in place for obtaining authorisation from parents to take, use and store images and videos of children in our care (National Regulations s 168(2)(ha)(ii)).
- We provide this policy with parents at the time of their child's enrolment.
- We get written consent from parents to take, use and store photographs or videos of their child via the enrolment form.
- Parents may choose not to consent to their child being photographed or videoed in any circumstances, or they may choose to give consent for a limited number of situations.
- We notify parents in advance of any school photographers, researchers and students on practicum placements who intend to take photographs/videos of their children, and give parents the opportunity to withdraw their consent.
- We only engage professional photographers who agree to follow the service Policy and Procedure in relation photographs and video recordings.
- External/third-party professionals (e.g., NDIS providers, allied health workers) who are supporting children while they are at our service, must not take photographs or videos of children unless they have the written authorisation of the child's parent, and they are supervised by a staff member while taking the image or recording (note, third parties who are conducting child-related work at our service must comply with our policies).
- Parents may refuse to authorise our service's taking, use or storage of images or videos of their child.
- Parents may withdraw prior authorisation at any time by notifying the nominated supervisor in writing and updating their child's enrolment form.

Parent and Guardian consents/authorisations for photos and video recordings

- Parent and Guardians agree to the enrolment terms and conditions and the service policies and procedures of the service upon enrolment.

- The enrolment form contains specific consent/authorisations from parent/guardians for the photos and video recordings by which will be taken, used and stored.

Extract: Child Enrolment Form: Photo and video recording authorisations

I authorise for photographs and video recordings of my child to be taken, used and stored for service-related purposes and staff training in accordance with service policies.

Please Choose* ☐ Yes ☐ No

I authorise for my child to be included in group photos and video recordings by which these will be taken, used and stored in accordance with the service policy.

Please note that if you select 'No,' this will limit the photos, videos, and learning documents we can provide to you. In that case, you will only receive personal photos and learning documents of your child. Choosing 'No' means your child's photo will not be included in group learning documentation and posts.

Please Choose* ☐ Yes ☐ No

(If you select "No" please consult your Nominated Supervisor/Centre Manager as this will restrict the learning documentation prepared for your child.)

I authorise for photos and video recordings of my child to be taken, used and stored for student training purposes in accordance with service policy. (Photos and video footage may leave the Service for students to present to lecturer and class for viewing and marking).

Please Choose* ☐ Yes ☐ No

I authorise for photos and video footage of my/our child to be taken, used and stored on Service website, social media platforms and other promotional materials.

Please Choose* ☐ Yes ☐ No

The above authorisations/consents on the enrolment form result in the following information displaying on OWNA App and Portal.



(Red camera icon) Denotes Photos / Video should not be put on the App OR needs to be made Private at all times.



(Facebook icon) Denotes Photos/videos can be shared on Social Media (Such as Facebook, Instagram, etc)

Consequences of Failing to Comply

- Services that fail to put appropriate mechanisms in place to establish child safe practices for the use of electronic devices in education and care services, resulting in children being at increased risk of harm, may be failing to comply with the conditions of service approval.
- Failing of the service policies and procedures, may mean that children are not adequately supervised, a breach of service policies and National Model Code which may result in disciplinary action as per the Grounds for Instant Dismissal.
- Any staff member found to be using the computers inappropriately will face an enquiry by management and other relevant parties to decide a course of action based on the severity of their misconduct. Please to related policy regarding this topic.
- This policy is also incorporative of state and federal laws regarding computer usage. Should a staff member or other relevant individual use the service's computers in a way that breaks a law, the service will take the appropriate required action (eg. Reporting to the police). Furthermore, the staff member or individual will face an enquiry held by management and other relevant parties to assess whether this conduct will affect their role within the service's operations.
- Failure to put appropriate mechanisms in place to establish child safe practices for the use of electronic devices in education and care services, resulting in children being at increased risk of harm, is may be assessed as failing to comply with the conditions of service approval. Penalties for any Service and approved provider found to have failed to comply with the conditions of their service approval include fines and potential suspension or cancellation of their service approval.

Evaluation and Review

The policy will be reviewed annually and will be conducted by:

Policy Handbook – P004. 7.2026

- Management
- Employees
- Families
- Interested Parties

Policy Reviewed: 16/4/2026

Next Policy Review: 15/4/2027

References:

NQS

QA2	2.2.3	<i>Child protection - Management, educators and staff are aware of their roles and responsibilities to identify and respond to every child at risk of abuse or neglect.</i>
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National Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

Regs	84	<i>Awareness of child protection law</i>
Reg	168	<i>Safe use of digital technologies and online environments at the service</i>

Legislation References:

Child Wellbeing and Safety Act 2005 (includes Child Safe Standards and Child Information Sharing Scheme)

Child Wellbeing and Safety (Information Sharing) Regulations 2018

Child Information Sharing Scheme Ministerial Guidelines VIC Govt

Children, Youth and Families Act 2005

Crimes Act 1958

Family Violence Protection (Information Sharing and Risk Management) Regulations 2018

Family Violence Protection Act 2008

Family Violence Information Sharing Guidelines: VIC Govt

Licensed Children's services and Victorian Schools "Protecting the Safety and Wellbeing of Children and Young People"

Worker Screening Act 2020

101 – NAPPY CHANGE AND TOILET SUPPORT POLICY

Position Statement

We view nappy changing as a routine caregiving task that provides opportunities for one-on-one interaction, relationship building, and supporting children's learning and independence. We ensure that all nappy changes are carried out in a way that protects children's health and safety, maintains hygiene and infection control, Promotes children's agency and participation and upholds each child's dignity, privacy, and cultural needs.

We recognise toilet training as a significant developmental milestone that requires a respectful, child-centred, and collaborative approach. Our service is committed to supporting each child's emerging independence in self-care in a way that honours their individual readiness, cultural background, and emotional wellbeing.

Objective

To ensure that all nappy changes are conducted safely, hygienically, respectfully, and in a manner that supports the child's wellbeing, dignity, and developmental needs.

To provide consistent, respectful, and supportive toilet training practices that promote children's independence, dignity, and confidence while working in close partnership with families.

Related Documents

- Nappy Change Procedure
- 27 - Health and Hygiene Policy
- 100 - National Model Code Policy
- Hygiene and Infection Control Policy
- OWNA Daily Information > Toileting

Guiding Principles

- Child-led and developmentally appropriate approach
- Collaboration with families
- Promoting independence and self-care skills
- Ensuring hygiene, safety, and emotional wellbeing
- Maintaining the dignity and privacy of children
- Child Safety

Supportive Practices for Nappy Change and Toileting Experiences

There are a range of practical strategies to support children's positive nappy changing and toileting experiences. These include:

- allowing children to take their time, avoiding pressuring them
- when asking children if they're ready for a nappy change, it's often helpful to check back in 5 minutes once they've finished playing
- have on display in toilet area steps for going to toilet and also washing hands (see PDF Supplies displays)
- responding to children's cues and allowing them to be active participants in the process
- where possible, using the correct terms for going to the toilet - ask families what words they use at home, as consistent language between home and care will help children to understand and learn more easily

- being sensitive to individual children's needs and styles, and tailoring individual nappy change and toileting procedures to each child
- prompting children by asking or reminding them about using the toilet
- taking full advantage of every opportunity for a focused, positive interaction with children during toileting and nappy change times
- always being positive about toilet training so that your encouragement is reinforced in your language and actions
- being respectful and sensitive to children's dignity and rights to privacy (r 155 Education and Care Services National Regulations).

Child Safety Photo and Video Guidelines:

- Devices (eg iPads or another other device with a camera) are not to be used or taken into bathrooms or nappy change areas. The recording of children wellbeing information (nappy change or toileting information in OWINA) is to be recorded in a location of the service other than toilet and nappy change areas. Further, educators should be aware of the background images when taking a photo or recording a video. Procedure: after a nappy change or toilet support is performed by an educator all health and hygiene procedures are completed, then educator records the information in OWINA in a location outside the bathroom and nappy change area.
- Incident and illness forms in OWINA allow for photos and videos of a child's injury, but certain rules apply:
 - Avoid taking photographs or videos in the moment when a child is upset/injured.
 - Do not capture images or recordings of a child's private body parts (E.g. genital/bottoms/breast) that are injured.
 - Do not take photos or videos of private body parts or in situations that may compromise a child's privacy (e.g., nappy rash, genitals, bottoms, chests).
 - Do not take photos of private parts, even if requested by a parent or guardian for external viewing.
 - Parental Guidelines: If parents or guardians need to see an injury of a private body part, they should come to the service to attend to their child directly.
 - These guidelines ensure the child safety, privacy, and dignity of the children in care.

Collaborating with Families

Children should arrive with a clean nappy and underpants. They need to be changed out of their overnight nappy prior to attending the service.

We believe the following assists in working collaboratively with families and children:

Consistency

Maintaining consistency between home and a service has the potential to make the process of toilet training easier for children. When a child starts toilet training at home, educators can support this progress in our early learning environment.

Communication

Working with families to pick up on readiness cues will maximise children's chances for success. This is an opportunity to discuss cultural practices and beliefs and how they align with service policies and procedures.

We encourage families to provide children with clothes that assist them to toilet independently, for example, elastic waisted pants that are easy to pull up and down.

Documentation

The service uses OWNA App to record information about nappy changing and toileting. This information is recorded in the OWNA App by Educators and can be viewed by parents.

When child become confident and competent with toileting (can go to the toilet and clean themselves hygienically without assistance). The selection of "Toilet Training" will be deselected in OWNA. This will result in the Toilet Training symbol not showing on the OWNA App. This indicates child can independently go to the toilet without support. Once this occurs the child toileting routine will not be recorded in the OWNA app.

Procedure: Nappy Change

Please follow the following nappy change procedure and guidelines:

Nappy Change Procedure

Children **MUST NOT** be left unattended on the change bench at any time during this procedure.

Devices are not to be kept or used in toilet or nappy change areas.

- Collect nappy and cream needed. *Check each child's individual nappy cream requirements.*
 - Wash or sanitize your hands (*See hand washing procedures*).
 - Place layer of paper on the change mat.
- Encourage child to climb the stairs provided. *Staff to assist children that can walk to use the stairs provided, when getting on and off the change bench. For children who cannot walk, staff are to use the correct lifting techniques and steps at all times.*
 - Put on gloves.
 - Remove any soiled clothing with urine or faeces on them and place in a plastic bag (remove faeces from clothing and rinse before storing).
 - Using the nappy, remove any excess faeces and remove the nappy from the child and roll up.
 - Clean child with wipe from front to back free from urine or soiling.
 - Dispose of nappy, wipes and paper in bin.
 - Remove gloves by peeling back from the wrist.
 - Put new gloves on if cream is required, remove as above.
 - Replace nappy.
- Dress child and assist child to wash and dry hands. *(a) Non-Walkers wash child's hands with clean wipe (explaining why we wipe our hands). (b) Walkers remove child from bench, provide them with a clean wipe or follow hand washing procedure.*
 - Return child to play area.
- Attend to child's soiled clothing. Put on disposal gloves. Remove any faeces from clothing and rinse clean. Place clothing in a sealed plastic bag and write the child's name and description of contents. Remove gloves. Place bag in soiled clothing container for collection by parent.
- Clean change table with disinfectant/detergent solution and paper towel, place paper towel in bin (If the change table or vinyl change mat has been soiled with faeces or urine wash with disinfectant/detergent solution until surface is clean).
 - Staff to wash and dry hands or sanitize hands thoroughly following the hand washing and sanitizing procedure.
 - Record nappy change, time changed and initials of staff member that completed the change on the Nappy Change/Toileting Record or Daily Routine Schedule in OWNA. (Devices are not kept or used in Toilet or Nappy Change areas.)



This procedure shall be displayed in the Nappy Change Area. This Procedure **MUST** be followed by all staff. This procedure is in conjunction with centre policies. Children's nappies are checked every two hours and are changed as required. The Centre Director completes regular 'Nappy Change Compliance' checks to ensure all staff correctly implements this procedure during their daily practices.

Source:

Staying Healthy - Preventing infectious diseases in early childhood education and care services, 6 Edition
National Childcare Accreditation Council http://www.ncac.gov.au/factsheets/qias_factsheet_14_toileting_and_nappy_changing.pdf and
http://www.ncac.gov.au/factsheets/fdcqa_factsheet_16_toileting_and_nappy_changing.pdf
International Scientific Forum on Home Hygiene, Information & Hygiene Advice Sheet; "Cleaning and Disinfection: Chemical Disinfectants Explained", August 2008
Legislative references:
Educational and Care Services National Regulations: Regulation 77, 106, 109, 112, 155, 168-172
Educational and Care Services National Law
Last Reviewed: 29 July 2025

Nappy Change Guidance:

Routine and Preparation

- Review OWNA App: Toileting records to assess what children require nappy changes. The order of nappy changes is based on a number of considerations:
 - if a child nappy is soiled
 - the time since last changed or arrival time
 - planned transition times (ie sleep time, meals, etc).

Routine and Periodic checking:

All children are to have their nappy changed or checked at least every 2 hours.

Children who are toilet training are to be reminded and encourage to go to the toilet

When a nappy is checked but not changed please use the "Dry" selection in the OWNA app to indicate the nappy of the child has been checked.

- Assess during nappy changes if sunscreen cream is applied at the end of .
- It is recommended that an educator change only 4-5 children in one nappy change period. This practice supports having engaging and respectful interactions with children during nappy changes. Nappy changing duties is shared between all educators and teacher of the service.
- Ensure all supplies are ready before bringing the child to the change area:
 - Clean nappy
 - Wipes
 - Disposable gloves
 - Nappy cream (with parental permission form)
 - Bin with lid to dispose of soiled nappy.
- Wash hands thoroughly with soap and water or use hand sanitiser before starting.

Collection and Supervision

- Always supervise children during nappy changing.
- Greet the child warmly, explain the nappy change process in age-appropriate language, and encourage participation.

Changing the Nappy

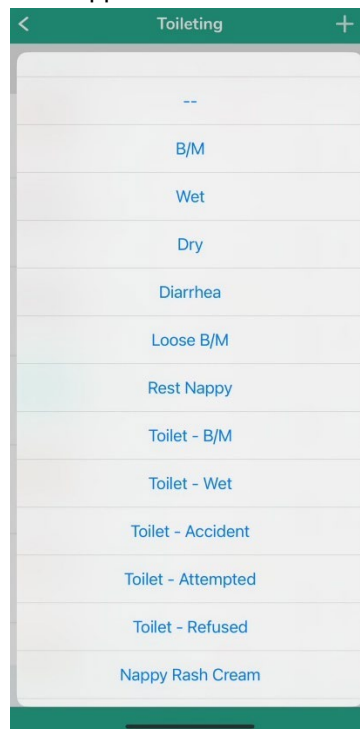
- Put on disposable gloves.
- If the child is old enough, assist the child to use the steps to get onto the change table
- Place child gently on the nappy change mat/bench (never leave child unattended)
- at all times keep one hand on the child to prevent them falling from the change table
- Remove the soiled nappy carefully.
- Clean the child's bottom thoroughly, front to back, using disposable wipes.
- Apply barrier cream if needed and authorised.
- Place the used nappy, wipes, and gloves into the sealed bin.
- Put on a clean nappy, ensuring it fits comfortably.

Post-Change Clean-Up Guidance:

- Dress the child and help them down safely from the nappy change steps.
- Assist or supervise the child to wash their hands using soap and running water.
- Clean and disinfect the nappy change area immediately after each use.
- Wash your hands thoroughly after disposing of waste and disinfecting the area.

Documentation and Communication

- Record the nappy change in OWNA on completion of each nappy change. This will record:
 - Person completed nappy change (please ensure educator is logged into OWNA app to record name of educator)
 - Time of change (automatically recorded when entered)
 - Whether it was wet/soiled (use the selection from OWNA to best describe)
 - Any observations (e.g. rash, unusual bowel movement)
- Notify families of any concerns as appropriate.
- Toileting selection in OWNA App:



Infection Control and Hygiene

- Change gloves between each child.
- Disinfect change mat/bench after each change.
- Maintain strict hand hygiene before and after each change.

Respecting Children's Rights

- Always talk to the child during nappy changes.
- Ensure privacy and dignity are maintained.
- Allow older children to assist with their own care needs where appropriate to promote independence.
- Be sensitive to cultural preferences and family routines.

Procedure: Toileting Support

Toileting occurs at any time of the day and is specific to individual needs. Educators will communicate with parents and guardians to develop consistency with their child's toileting habits.

1. Initiating Toilet Training

- Educators and families will communicate openly about signs of toilet readiness, including:

- Staying dry for longer periods
 - Showing interest in the toilet
 - Verbalising the need to go
 - Discomfort in wet/soiled nappies
 - Families are encouraged and recommended to begin toilet training at home before the service starts the process. Allow sufficient time for child to become comfortable and also show interest in the toilet.
 - A toilet training plan will be developed collaboratively with the family and documented.
- Toilet training plan: The service will use the OWNA Goal Tool to document strategies and notes in relation to toileting training plan.

Example of toileting plan narration in OWNA Goal Tool:

Goal Title: Toileting Support <Child Name>

Start Date: <Date toilet plan commenced>

End Date: <when goal achieved>

Initial Comments Information to cover:

- When parent discussed when at home they are start toilet training (<date>)
- Discussed what parent is doing at home to learn about and support using the toilet? Summarise notes.
- Discussed with parent the following if they noticed: child showing interest in toilet, verbalising wanting to go, shows discomfort in wet/soiled nappy, staying dry for longer periods in nappies
- Is child wear underpants?
- Clothing needs at service. ie spare clothes. Clothes that provide ease of access to go to toilet.

Goals

Goal 1 Example: To support <child name> with getting comfortable and familiar with the toilet at the early learning centre.

Goals 2 Example: For <Child Name> to recognise and respond to toileting needs.

Goal 2 Example: Support <Child Name> to use the toilet independently and be able clean bottom and hands.

Initial Comments / Strategies: (General and specific strategies to support child with toileting)

- Using positive encourage.
- Reading group time books
- Regularly inviting child to visit the toilet.
- Offering child to sit on the toilet before a nappy change. ie 2-3 times per day.
- For <child name> to wipe his/her bottom correctly and independently when doing bowel motions.
- Assisting and encouraging child to wash hands after use of toilet.
- Optional: Assess if child is more comfortable to use person potty used at home (if not comfortable to use service toilet).

EYLF Outcome Link: 3.3

(It is noted the Toilet Training Plan is not required to be sole and primary goal for the child. The toilet training plan is best documented in the OWNA goal tool as it is best reference point for both educators and families.)

- Parents and guardian can use a room communication book or the OWNA App message tool to communicate to educators.
- Once toileting support plan has been mutually agreed the service management team will update the child profile to indicate Toileting; "Toilet Training" and update routine

records to indicate child is toileting training. The child's toileting plan

Toileting ☒

Select Toileting Type

Nappy

Nappy

Toilet Training

Sleep Nappy

Own Sunscreen

Selection in OWNA	Symbol Shown
If nothing selected	No symbol will show in app.
Nappy	
Toilet Training	
Sleep Nappy	

- When child become confident and competent with toileting (can go to the toilet and clean themselves hygienically without assistance). The selection of "Toilet Training" will be deselected in OWNA. This will result in the Toilet Training symbol not showing on the OWNA App. This indicates child can independently go to the toilet without support. Once this occurs the child toileting routine will not be recorded in the OWNA app.
- If child requires Nappy Rash Cream. The following selection is needed in OWNA child profile. The parent or guardian is required to complete a Form via the OWNA App "Ongoing / Non Prescribed Cream & Medication Form" before any creams can be applied to child's body.

☒ Click if child requires Nappy Rash Cream

Selection in OWNA	Symbol in App
Nappy Rash Cream	

2. Routine and Practice

- Children are offered regular toilet times, such as:
 - Upon arrival
 - Before and after meals
 - Before rest time
 - Before outdoor play
- Educators will respond promptly to children's cues and verbal requests.
- Children are supported to:

- Pull down/up clothing
 - Sit safely and comfortably on the toilet or potty
 - Wipe themselves with guidance (encouraging them to wipe from front to back)
 - encourage the child to flush the toilet
 - Wash hands thoroughly: encourage and assist the child to wash their hands, then dry their hands on a clean washer and dispose in the correct bin.
- Positive reinforcement and encouragement are used; no punishment or shaming is permitted.

3. Partnership with Families

- Families are informed daily of progress and any accidents.
- A consistent approach between home and the service is encouraged (e.g., similar language, routines).
- Families are to provide sufficient spare clothing and underwear (4-5 sets)

4. Accidents and Clean-Up

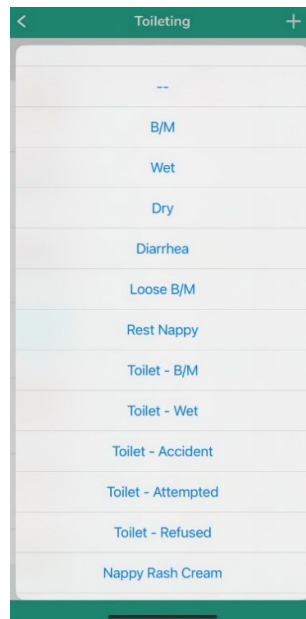
- Accidents are treated calmly and discreetly.
- Clean and dry the child
- Assist the child going to the toilet (see routine above)
- Assist the child to get dressed
- Encourage and assist the child to wash and dry their hands
- Children are involved in changing when appropriate, promoting self-help skills.
- Ask the child to leave the bathroom and resettle the child back into the program
- Disinfect the toilet and clean any spills (following the procedure for cleaning spills of body fluids)
- Soiled clothing is emptied into the toilet and flushed. Soiled clothing is placed in a sealed bag, placed in soiled clothing container, message provided to families to collect soiled clothing.
- Educators follow infection control and hygiene procedures (handwashing, glove use, cleaning protocols).

5. Health and Safety

- Toileting areas are clean, safe, and child-accessible.
- Educators supervise and support children without compromising privacy.
- Regular checks ensure cleanliness of facilities.

6. Documentation

- Significant changes or concerns are discussed with families and, where necessary, management.
- Notes on children's toilet training progress are recorded and communicated with families using the OWNA app. The following are examples of selections that can be made by educators in the OWNA App Toileting tool.
- When an educator check a nappy or a child bottom (when toileting training) and is clean. The selection of "Dry" should be used/
- During toileting training when a child is offered to sit on the as part of the toilet support plan the use of "Toilet - Attempted" or "Toilet – Refused" should be used.



Roles and Responsibilities

Role	Responsibility
Nominated Supervisor	Ensure all staff are trained in procedure and hygiene
Educators	Implement procedure daily, communicate with families
Families	Work in partnership, provide required supplies.
Children	Encouraged to participate in self-care to their ability

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters). Policies will also be discussed at staff meetings and changes to policies recorded on the Record of Changes to Centre Policies.

Policy Reviewed: 22/8/2025

Reference:

NQS

QA2	2.1	<i>Each child's health and physical activity is supported and promoted.</i>
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National Act and Regulations

Education and Care Services National Law Act 2010

Education and Care Services National Regulations

EYLF

Regs	77	<i>Health, hygiene and safe food practices</i>
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LO3	<i>Actively support children to learn hygiene practices.</i>
	<i>Promote continuity of children's personal health and hygiene by sharing ownership of routines and schedules with children, families and the community.</i>
	<i>Discuss health and safety issues with children and involve them in developing guidelines to keep the environment safe for all.</i>
	<i>Model and reinforce health, nutrition and personal hygiene practices with children.</i>

102 – PROTECTED DISCLOSURES (WHISTLEBLOWER) POLICY

Position Statement

This policy supports a culture of integrity and accountability by providing a safe and confidential process for reporting wrongdoing, misconduct, or any matter that may pose a risk to the safety, welfare or wellbeing of children within our Service. We believe that children's safety, rights, and best interests are the paramount consideration for all Service operations, decisions and functions. Our Service ensures that child safety, wellbeing and best interests take priority over all other considerations, including financial interests or other obligations of management, and are embedded in our daily practices, policies and procedures.

Objective

This policy encourages employees, volunteers, contractors, subcontractors, parents and carers to report any concerns or instances of wrongdoing or misconduct without hesitation. It also reinforces the Service's strong commitment to supporting and protecting individuals who make protected disclosures, ensuring they are safe from any form of detrimental action, which will not be tolerated under any circumstances. When the service receives a protected disclosure, we have obligations to protect the person who made the disclosure and to appropriately manage and address the disclosure.

This policy outlines how the service manages protected disclosures, identifies reporting contacts, explains the reporting process and details the safeguards available to individuals who make such disclosures.

Definitions

Protected Disclosure/Qualifying Disclosure: A disclosure of information where the reporter (whistleblower) has reasonable grounds to suspect the information concerns:

- conduct that endangers or may endanger the safety, health or wellbeing of children
- misconduct or an improper state of affairs
- illegal activity or breach of law
- inappropriate conduct toward children
- fraudulent activity or corruption
- negligence
- systemic practices posing serious risk to health and safety
- breach of child protection requirements
- failure to comply with the National Law or Regulations
- taking or threatening to take detrimental action against a person who has made a disclosure, is suspected to have made or is planning to make a disclosure.

Not all concerns or complaints are protected disclosures. A disclosure will only be protected where the person making the report does so honestly and on reasonable grounds, and the information tends to show wrongdoing, an offence under the National Law, or a risk to the safety, health or wellbeing of a child or children. Personal work-related grievances that do not involve broader wrongdoing or risks to children are not protected disclosures.

Reasonable grounds to suspect: The discloser's reasons for suspicion must be reasonable, when considered objectively. Even if the disclosure proves incorrect, protection applies if reasonable grounds existed.

Detrimental Action: Any disadvantage to a person including:

- dismissal, demotion or suspension
- harassment, intimidation or victimisation
- discrimination or adverse treatment
- damage to reputation, property or financial position
- injury or harm including psychological

- threats of any of the above

Personal work-related grievances: Generally, these do not qualify for whistleblower protection unless they:

- include information about misconduct beyond personal circumstances
- involve breach of employment laws punishable by 12+ months imprisonment
- relate to detrimental action for making a protected disclosure
- represent danger to public safety

Serious Detrimental Action: any act or omission that causes, involves or encourages detriment to the person, or the threat of detriment (whether express or implied). However, the following are not considered serious detrimental action:

- lawful action taken by a person or body to investigate a possible contravention of the National Law (NSW Services only)
- prosecuting a person for a criminal offence
- referring a matter about a person to the independent arbiter
- making a disciplinary agreement or disciplinary order in relation to a person
- giving a disciplinary notice to a person

Whistleblowing: a whistleblower, also known as a reporter, is a person who provides information and exposes serious wrongdoing within an organisation in the hope of stopping it. Whistleblowing plays a crucial role in managing risk and cultivating an ethical culture and is an effective way of uncovering potential serious wrongdoing including misconduct, fraud and corruption.

Related Policies

7- Complaints and Grievance Policy
 12A - Child Safe Standards Policy
 12B - Child Safety, Protection & Wellbeing Policy
 12C - Safeguarding Children: Look, Do & Tell Framework Policy
 19 - Incident, Injury, Trauma and Illness Policy
 78 - Governance of Service, Management and Educator Policy
 84 - Record Keeping and Retention Policy
 70 - Ethical Conduct and Code of Ethics Policy

Implementation

Our Service promotes a culture of transparency and will provide safe, confidential channels for disclosures, investigate fairly, and protect disclosers from detrimental action. Our Service encourages anyone who suspects illegal, unethical, corrupt or fraudulent behaviours, wrongdoing, inappropriate conduct or inappropriate discipline to report those concerns.

THE APPROVED PROVIDER/NOMINATED SUPERVISOR/ MANAGEMENT WILL ENSURE:

- that obligations under the Education and Care Services National Law and Education and Care Services National Regulations are met
- educators, staff, students, visitors and volunteers understand and adhere to this policy and associated procedure and are advised on how and where the policy can be accessed
- copies of this policy are readily accessible and available for inspection
- families are aware of this *Protected Disclosure (Whistleblower) Policy* and are advised on how and where the policy can be accessed
- a safe environment is created where employees and volunteers feel supported to report any wrongdoing
- a culture of transparency and integrity is promoted within the Service
- regular training and policy reviews about whistleblower protections and processes are provided to all staff, educators and volunteers and that records of training are maintained.
- multiple reporting channels including internal and external options
- appoint eligible recipients to receive disclosures

- any whistleblower reports are promptly investigated
- feedback is provided to whistleblowers on the progress and outcomes of their reports, where appropriate.

EDUCATORS/VOLUNTEERS/STUDENTS WILL:

- immediately report all incidents, allegations and complaints relating to child safety to the approved provider (see: Child Safe Standards Policy, Child Safety, Protection & Wellbeing Policy and Safeguarding Children: Look, Do & Tell Framework Policy)
- be encouraged to report any concerns, even if you are unsure whether they meet the definition of a protected disclosure. All reports will be assessed by the Service in accordance with this policy and relevant legislation.
- cooperate with investigations into disclosures
- maintain confidentiality about disclosures and investigations
- support colleagues who make protected disclosures
- not engage in or tolerate detrimental action against whistleblowers
- complete required training on whistleblower protections and procedures, including during induction and at regular intervals

FAMILIES WILL:

- be informed of their right to make protected disclosures about serious wrongdoing
- have access to information about how to make a disclosure
- be protected if they make a qualifying disclosure
- have confidence that disclosures will be taken seriously and investigated appropriately.

HOW TO MAKE A DISCLOSURE

Disclosures can be made by current and former employees and educators, volunteers, students, contractors, families, persons with management or control at the Service. Disclosures may be made:

- in person
- in writing (letter or email)
- by telephone
- through a discussion (face-to-face, via telephone or virtually)
- through a legal practitioner

Anonymous disclosures will be accepted; however, this may limit the Service's ability to investigate the matter fully or provide feedback.

When making a disclosure, the whistleblower should provide:

- description of the conduct or issue
- names of persons involved in suspected wrongdoing, their role, title and how they are involved
- dates, times and locations of key events
- any witnesses
- supporting documentation or evidence
- contact details (unless anonymous)

PROTECTION AND REMEDIES FOR REPORTERS

Individuals making qualifying disclosures are protected from:

- civil, criminal or administrative liability
- contractual remedies or rights being enforced
- admissibility of disclosed information in proceedings against them
- detrimental actions or threats

Individuals who make a protected disclosure in accordance with legislation are entitled to significant protections. These protections include:

Protection from detrimental action: the Service will take all reasonable steps to ensure that no person suffers detriment for making, or intending to make, a protected disclosure. Detrimental action includes dismissal, injury, damage, intimidation, harassment, discrimination or adverse treatment. Taking serious detrimental action against a person because of a protected disclosure is a criminal offence.

Preventing harmful actions: the Service will undertake a risk assessment upon receiving a disclosure and implement strategies to minimise the risk of detrimental action.

Recovery of damages: a person who suffers loss, damage or injury as a result of serious detrimental action may seek compensation through legal proceedings.

Ability to seek an injunction: a person may apply to a court to prevent or stop detrimental action.

This may include orders to:

- stop harmful behaviour
- reinstate employment
- require corrective action

Immunity from liability: a person making a protected disclosure is protected from civil, criminal or administrative liability for making the disclosure. This includes protection from breach of confidentiality obligations. This protection does not extend to any wrongdoing by the person making the disclosure.

Immunity from costs: a person who brings proceedings in relation to a protected disclosure is generally not liable for legal costs unless the claim is made without reasonable cause.

REPORTING DETRIMENTAL ACTION

If an individual has experienced or is at risk of detrimental action as a result of making a disclosure, they should report this immediately.

Reports can be made to:

- the approved provider
- the nominated supervisor
- the responsible person in charge of the service
- the Designated Whistleblower Protection Officer (during service opening hours this role will be held by nominated supervisor or the responsible person in charge of the service. Or the Approved Provider at all other times and if the nominated supervision or responsible person is not available.)
- or an external authority

The Service will take all reasonable steps to:

- investigate the concern
- prevent further detrimental action
- support the affected person
- take appropriate disciplinary action where required

FALSE OR MALICIOUS DISCLOSURES

While protection extends to incorrect disclosures made within reasonable grounds, deliberately false or malicious reports will be treated seriously and may result in:

- disciplinary action
- termination of employment
- legal action
- referral to authorities

INTERNAL RECIPIENTS OF A DISCLOSURE

The contact information for Approved Provider is found at the service foyer display.

EXTERNAL RECIPIENTS OF A DISCLOSURE

If a staff member, volunteer or other person would prefer to make an external protected disclosure

outside the Service, they may contact the regulatory authority, Ombudsman or the statutory child protection authority.

Insert details applicable to your state/territory

VICTORIA

Regulatory Authority - Victorian Early Childhood Regulatory Authority

○ 1300 307 415

email: vecra@education.vic.gov.au

Independent Broad-based Anti-corruption Commission (IBAC)

○ 1800 806 314

website: www.ibac.vic.gov.au

Victorian Ombudsman

○ (03) 96136222	website: www.ombudsman.vic.gov.au
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QUEENSLAND

Regulatory Authority - Queensland Early Childhood Regulatory Authority, Department of Education

○ 137 468

email: eccec@ged.qld.gov.au

Crime and Corruption Commission Queensland

○ 1800 061 611

website: www.ccc.qld.gov.au

Queensland Ombudsman

○ 07 3005 7000

website: www.ombudsman.qld.gov.au

RECEIVING AND MANAGING DISCLOSURES

Our Service is committed to ensuring that all protected disclosures are handled appropriately, fairly, and confidentially. The person managing the disclosure will, on behalf of the Service:

- acknowledge receipt of a disclosure within 5 business days (where the discloser can be contacted)
- assess whether the disclosure qualifies for protection under relevant legislation
- determine appropriate investigation procedures while maintaining confidentiality
- take all reasonable steps to protect the discloser from detrimental action
- ensure procedural fairness for any person mentioned in a disclosure
- provide a summary of outcomes, where appropriate and lawful
- document all disclosures, investigations and outcomes securely and confidentially

RISK ASSESSMENT AND MANAGEMENT

Upon receiving a disclosure, the Service will conduct a risk assessment to identify any risk of detrimental action. This assessment will consider risks to:

- the person making the disclosure
- the person who is the subject of the disclosure
- witnesses
- individuals involved in the investigation

The Service will implement strategies to minimise identified risks, which may include:

- adjusting reporting lines
- providing support services
- ensuring secure communication channels
- ongoing monitoring throughout the process

INVESTIGATING DISCLOSURES

Before commencing an investigation, the Service will assess whether the disclosure qualifies as a

protected disclosure under relevant legislation and determine the appropriate response, including whether the matter should be referred to an external authority.

The person managing the disclosure will:

- conduct thorough, fair and impartial investigations into qualifying disclosures
- determine the nature, scope and appropriate investigator (internal or external)
- complete investigations as soon as practicable
- keep the discloser informed of progress where appropriate and lawful
- take corrective action when wrongdoing is substantiated
- report to relevant authorities as required- regulatory authority, Police, Child Protection, Social Services Regulator, VECRA, etc.

MANAGING DISCLOSURES CONFIDENTIALLY

Following a disclosure, the Service will ensure that the discloser is protected from detriment.

The person managing the disclosure will:

- maintain strict confidentiality of the discloser's identity (subject to legal exceptions)
- limit the number of people who are aware of the discloser's identity or information that could identify them
- ensure that any person who becomes aware of the discloser's identity is reminded of their obligation to maintain confidentiality

- restrict access to records and information to authorised personnel only
- ensure secure storage of all documentation, including electronic and hard copy files
- protect disclosers from victimisation, harassment or detrimental action
- provide access to support services including Employee Assistance Programs where available
- monitor and assess risks of detriment to disclosers throughout the process
- take disciplinary action against anyone who takes detrimental action against a whistleblower
- maintain confidentiality of the discloser's identity except where disclosure is required by law or necessary to prevent a serious risk to a child or another person

In some circumstances, it may be necessary to disclose certain information during an investigation to ensure procedural fairness. In these cases, all reasonable steps will be taken to minimise the risk of the discloser being identified. If it becomes likely that the discloser's identity may be revealed, the Service will, where practicable:

- inform the discloser in advance
- review and update the risk assessment
- implement additional strategies to minimise the risk of detrimental action
- provide appropriate support to the discloser.

Evaluation and Review

This policy will be reviewed at least annually.

Family and team member feedback will be considered in the review process. Changes in legislation, regulations and standards will be considered.

Any changes to this policy will be communicated to families and team members verbally and in writing (Notice Boards and Newsletters)

Policy Reviewed: 26/6/2026

Next Policy Review: 15/6/2027

The policy will be reviewed annually by the Approved Provider, Supervisors, Employees, Families and any committee members.

References

NQS

QUALITY AREA 2: CHILDREN'S HEALTH AND SAFETY		
2.2.3	Child Safety and Protection	Management, educators and staff are aware of their roles and responsibilities regarding child safety, including the need to identify and respond to every child at risk of abuse or neglect.
QUALITY AREA 4: STAFFING ARRANGEMENTS		
4.2.1	Professional collaboration	Management, educators and staff work with mutual respect and collaboratively, and challenge and learn from each other, recognising each other's strengths and skills.
4.2.2	Professional standards	Professional standards guide practice, interactions and relationships.
QUALITY AREA 7: GOVERNANCE AND LEADERSHIP		
7.1	Governance	Governance supports the operation of a quality service that is child safe.
7.1.2	Management systems	Systems are in place to manage risk and enable the effective management and operation of a quality service that is child safe.
7.2	Leadership	Effective leadership builds and promotes a positive organisational culture and professional learning community.

EDUCATION AND CARE SERVICES NATIONAL LAW AND NATIONAL REGULATIONS

S. 2A	Paramount consideration – safety, rights and best interests of children
S. 3A	Paramount consideration [NSW]
S.5AA	Meaning of inappropriate conduct [NSW]
S. 162A	Child protection training Offence relating to child protection training [NSW]
S. 166A	Offences relating to inappropriate conduct Offence to subject child to inappropriate conduct [NSW]
S. 174AA	Educators and other staff members of education and care service to notify certain information [NSW]
Division 7	Serious detrimental action
S. 300E	Requirement for policy about protected disclosures [NSW]
84	Awareness of child protection law
168	Education and care services must have policies and procedures
170	Policies and procedures to be followed
171	Policies and procedures to be kept available
172	Notification of change to policies or procedures
175	Prescribed information to be notified to the regulatory authority
181	Confidentiality of records kept by approved provider

Source

Australian Government. [Work Health and Safety Act 2011](#) (Cth).
 Australian Securities & Investments Commission. [Whistleblower rights](#)
[Children \(Education and Care Services\) National Law \(NSW\)](#) (NSW services only)
[Education and Care Services National Law Act 2010](#)
[Education and Care Services National Regulations 2011](#)
[Education and Care Services National Regulations \(NSW\) \(2025\)](#) (NSW services only)
 Justice and Connect. (2025). [Whistleblower protection laws for not-for-profit organisations](#).
 NSW Department of Education. (2024). [Public interest disclosures procedures](#)
 NSW Early Learning Commission. (2026). [Child safety reforms](#)
 NSW Early Learning Commission. (2026). [Protected Disclosures Model Policy](#)
 NSW Ombudsman. (2025). [Public Interest Disclosures](#)
[Western Australian Legislation Education and Care Services National Law \(WA\) Act 2012](#) (WA Services only)
[Western Australia Legislation Education and Care Services National Regulations 2012](#) (WA Services only)